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WAVE COUNTRY REPORT 2025

The status and value of women's specialist services
in tackling gender-based violence against women

IMPRINT

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List of abbreviations

EU	European Union
GBV	gender-based violence
GBVAW	gender-based violence against women
GONGO	government-organised non-governmental organisation
LGBTQI+	lesbian, gay, bisexual, transgender, queer, intersex and others
NAP	National Action Plan
NGO	non-governmental organisation
RCC	rape crisis centre
SVRC	sexual violence referral centre
TFVAWG	technology-facilitated violence against women and girls
VAW	violence against women
VAWG	violence against women and girls
WSS	women's specialist services

Foreword

The WAVE Country Report is our Network's primary publication—a biennial, data-driven overview of the status of women's specialist services in 46 European countries. As a key advocacy tool, the Country Report tracks progress, identifies persistent gaps, and measures the extent to which European countries meet the minimum standards set out by the Istanbul Convention, providing evidence for necessary legislative reforms and policy adjustments at the national and European levels.

The report focuses on women's specialist services, including national helplines, women's shelters, women's centres, and specialist support for survivors of sexualised violence. Additionally, it provides insights into primary prevention work and femicide. For the first time, the 2025 edition also includes a chapter on the rise in technology-facilitated violence against women and girls (TFVAWG), and the strategies adopted by women's specialist services to counter it. Women's specialist services are operating in an increasingly complex and often openly hostile environment. Political backsliding, the rise of restrictive 'foreign agent' laws, and the spread of gender-neutral policies are eroding hard-won women's rights and shrinking the democratic space in which feminist civil society can act. At the same time, TFVAWG is accelerating at an alarming pace, adding new forms of digital abuse to the already widespread physical, sexual, and psychological violence faced by women and girls. These developments are not isolated incidents – they reflect a broader backlash against women's rights defenders and the fundamental European values of democracy and equality.

In the face of these significant challenges, women's specialist services continue to demonstrate remarkable resilience, providing vital and life-saving support across Europe. There are also signs of progress. Since the last WAVE Country Report, the number of European Union (EU) countries operating helplines in line with the Istanbul Convention has risen from 19 to 21, and among non-EU countries, the figure has increased from 13 to 16. Yet, the provision of shelters and women's centres has not improved, especially in non-EU countries, which were hit hardest by the recent cuts to USAID funding. These growing disparities highlight the urgent need for sustainable funding and genuine political commitment to ensure life-saving support for all women and girls, as well as to secure the long-term, effective prevention of violence against women and girls (VAWG).

The rising challenge of tech-facilitated violence against women and girls

A fast-emerging and rapidly evolving challenge is TFVAWG. Digital technologies and artificial intelligence (AI) have opened new frontiers of abuse, with at least 40 forms of online violence now recognised – from cyberstalking and harassment to deepfake pornography¹. Deepfake incidents alone increased by more than 550% between 2019 and 2023, disproportionately targeting women and forcing many out of digital and public spaces². 20% of young women (18–29 years) in the EU have already experienced cyber-harassment.³ Women human rights defenders,⁴ journalists,⁵ and politicians are subject to highly targeted online attacks designed to silence and discredit them. Perpetrators benefit from anonymity, lack of regulation, and the borderless nature of digital platforms – often acting with complete impunity. TFVAWG deepens existing structural inequalities, pushes women and girls out of online and offline public life, and ultimately threatens democracy itself.

Despite the scale of these harms, legal and institutional responses across Europe remain slow and systematically underfunded – leaving many women and girls exposed and unprotected. Addressing this crisis requires action from European governments and funders to invest in women's and girls' safety and the protection of their human rights. This includes sustained funding for women's specialist services responding to TFVAWG, comprehensive training for relevant government and justice-sector professionals, and urgent legal reforms backed by effective implementation and enforcement. At the same time, feminist advocacy must increasingly focus on reclaiming digital spaces – ensuring that technology becomes a tool for empowerment and safety, rather than one that fosters control, surveillance, and exclusion. Critically, however, states bear primary responsibility for protecting people and their data, regulating online spaces, and mandating safety-by-design standards for digital platforms. Any moves toward regulatory flexibility must not compromise Europe's commitment to safety and digital rights protection. States need stronger enforcement mechanisms and greater commitment to maintaining high standards, rather than converging toward weaker frameworks that privilege platform interest over user safety.

- 1 United Nations Population Fund (2025) An Infographic Guide to Technology-facilitated Gender-based Violence (TFGBV), p. 6. <https://www.unfpa.org/publications/infographic-guide-technology-facilitated-gender-based-violence-tfgbv>
- 2 Keepnet (2025) Deepfake Statistics & Trends 2025: Growth, Risks, and Future Insights, available at: <https://keepnetlabs.com/blog/deepfake-statistics-and-trends> (Accessed 24 November 2025).
- 3 European Parliament's Policy Department for Citizens' Rights and Constitutional Affairs (2018) Cyber violence and hate speech online against women. Available at: [https://www.europarl.europa.eu/RegData/etudes/STUD/2018/604979/IPOL_STU\(2018\)604979_EN.pdf](https://www.europarl.europa.eu/RegData/etudes/STUD/2018/604979/IPOL_STU(2018)604979_EN.pdf)
- 4 UN Women (2024) How women human rights defenders are under threat worldwide, available at: <https://www.unwomen.org/en/articles/explainer/how-women-human-rights-defenders-are-under-threat-worldwide> (Accessed 24 November 2025).
- 5 Freedom House (2023) We Must Do More to Address the Online Harassment of Women Journalists, available at: <https://freedomhouse.org/article/we-must-do-more-address-online-harassment-women-journalists>; <https://www.euronews.com/next/2025/08/04/women-politicians-face-more-personal-attacks-online-who-is-to-blame> (Accessed 24 November 2025).

The importance of primary prevention

Despite the persistence of VAWG and the emergence of new forms of violence, institutional responses remain widely reactive – managing harm and the aftermath of violence after it occurs, rather than preventing violence in the first place. This must change.

WAVE continues to call for a fundamental shift towards long-term, transformative prevention strategies, with a particular focus on primary prevention: acting before violence happens and addressing the root causes of gender-based violence (GBV). Primary prevention is the most strategic and sustainable approach – not only to reducing harm, but to ending VAWG altogether. Achieving this requires sustained, cross-generational political will and financial commitment to dismantle the structural inequalities, harmful policies, and gender stereotypes that enable violence. Primary prevention of VAWG is not only a human rights obligation – it is an urgent economic and social necessity that must no longer be sidelined.

Violence against women costs the EU an estimated €289 billion every year,⁶ yet the evidence is unequivocal: every €1 invested in prevention saves up to €86 in long-term costs.⁷ Despite this, prevention remains chronically underfunded and politically deprioritised. The gap between what we know and what we fund is no longer just a policy failure – it is a deliberate choice with profound human, social, and economic consequences.

Real change requires far more than political rhetoric or surface-level awareness campaigns. It demands sustained commitment and predictable, adequate funding for both primary prevention initiatives that address the root causes of VAWG and for feminist civil society organisations – especially women's specialist services – that provide essential support to survivors.

This means supporting feminist civil society organisations doing this critical work on the ground, engaging genuinely with communities to transform the deep-rooted attitudes and norms that allow violence to persist. To be effective, these prevention strategies must be genuinely inclusive and intersectional, centring the voices and lived experiences of survivors – not as an add-on, but as the foundation of all policy and practice.

Celebrating women's specialist services

In closing, we pay tribute to the invaluable work of WAVE members, to whom this report is dedicated. Despite growing adversity and shrinking resources, women's specialist services continue to show extraordinary resilience, innovation, and creativity. Whether operating under authoritarian

restrictions, providing shelters in conflict zones, running multilingual helplines, or pioneering digital advocacy, their work ensures that women's voices, in their full diversity, continue to shape policy and decision-making on VAWG. For over three decades, the WAVE Network and its members have been at the forefront of efforts to secure women's rights and build a future free from violence. Their courage, expertise, and solidarity inspire hope that a world without VAWG is possible.

This report is both a celebration of what has been achieved and a call to action. By documenting the current realities and identifying where progress is urgently needed, the WAVE Country Report aims to strengthen and unite women's rights advocates across Europe in the shared task of building a safer, more just society for all women and girls.

Stephanie Futter-Orel (WAVE Executive Director) and
Susana Pavlou (WAVE President)

Vienna, December 2025

6 European Institute for Gender Equality (2021) The costs of gender-based violence in the European Union, available at: https://eige.europa.eu/gender-based-violence/costs-of-gender-based-violence?language_content_entity=en

7 Nectoux et al. (2010). *Évaluation économique des violences conjugales en France*, « Santé Publique » 2010/4 Vol. 22 | pp. 405 à 416.

Executive summary

The WAVE Country Report 2025 provides an overview of women's specialist services (WSS) in 46 European countries, covering national women's helplines, women's shelters, women's centres, specialist services for survivors of sexualised violence, and primary prevention. It also includes a dedicated section on femicide and, new to this edition, technology-facilitated violence against women and girls (TFVAWG). The term 'European countries' in this context refers to the 46 Council of Europe member states represented in the WAVE Network, while recognising that this does not cover all states in geographical Europe. Data availability varies across countries, and where relevant, differences between EU Member States and non-EU countries are highlighted. A detailed description of data collection processes and limitations is provided in the methodology chapter.

The Council of Europe Convention on preventing and combating violence against women and domestic violence (the Istanbul Convention) remains the most comprehensive regional framework for assessing state responsibilities in the areas of prevention, protection, prosecution, and coordinated policies. It continues to serve as the main benchmark for evaluating the adequacy and quality of specialist services in Europe.

Status of signatures and ratifications of the Istanbul Convention (as of November 2025)

- **44 countries have signed the Convention:** Albania, Andorra, Armenia, Austria, Belgium, Bosnia and Herzegovina, Bulgaria, Croatia, Republic of Cyprus, Czech Republic, Denmark, Estonia, Finland, France, Georgia, Germany, Greece, Hungary, Iceland, Ireland, Italy, Latvia⁸, Liechtenstein, Lithuania, Luxembourg, Malta, Republic of Moldova, Monaco, Montenegro, Netherlands, North Macedonia, Norway, Poland, Portugal, Romania, San Marino, Serbia, Slovakia, Slovenia, Spain, Sweden, Switzerland, Ukraine, United Kingdom.
- **39 countries have ratified the Convention:** Albania, Andorra, Austria, Belgium, Bosnia and Herzegovina, Croatia, Republic of Cyprus, Denmark, Estonia, Finland, France, Georgia, Germany, Greece, Iceland, Ireland, Italy, Latvia, Liechtenstein, Luxembourg, Malta, Republic of Moldova, Monaco, Montenegro, Netherlands, North Macedonia, Norway, Poland, Portugal, Romania, San Marino, Serbia, Slovenia, Spain, Sweden, Switzerland, Ukraine, United Kingdom.
- One country has denounced and withdrawn from the Convention: **Türkiye** (effective 1 July 2021).

- The **European Union** (EU) signed the Convention in June 2017 and acceded in June 2023.

Despite not being universal, the Convention remains the central reference point for assessing specialist service provision and broader state obligations across Europe.

EU Directive on combating violence against women and domestic violence

A significant development in 2024 was the adoption of the EU Directive on violence against women and domestic violence (Directive (EU) 2024/1385), the first EU-wide legislation addressing violence against women (VAW). The Directive establishes minimum standards across Member States in the area of prevention, protection and support, including the criminalisation of female genital mutilation, forced marriage, and key forms of TFVAWG. Member States must transpose the Directive into national law by June 2027.

While recognising the Directive as a significant step forward, several gaps remain. Most notably, the Directive does not include a fully consent-based definition of rape, and its coordination model risks marginalising WSS within general victim-support systems. Furthermore, protections for particularly vulnerable groups remain insufficient, and negotiations led to weakened language in several areas.

The Istanbul Convention remains the main policy framework against which this report assesses regional progress and ongoing challenges in protecting women and girls from all forms of violence.

National women's helplines

National women's helplines are one of the most important basic services for survivors of violence against women and girls (VAWG). Specialist helplines are essential because survivors frequently face disbelief, minimisation, or victim-blaming in broader society. A national women's helpline, staffed by trained professionals or volunteers who believe survivors and understand the gendered dynamics of violence, offers the validation and solidarity that women need in order to seek support.

Article 24 of the Istanbul Convention, as well as the Directive (EU) 2024/1385, require state-wide, 24/7, free helplines that address all forms of VAW, and which are confidential, anonymous, and multilingual. Both instruments also require adequate and sustainable state funding (Istanbul

⁸ On 30 October 2025, Latvia's Parliament voted to withdraw from the Istanbul Convention, with 56 votes in favour and 32 against. As of December 2025, Latvia has paused its withdrawal from the Istanbul Convention following a parliamentary vote in October 2025, with the President returning the withdrawal bill for further review due to legal and human rights concerns. The process remains unresolved, with ongoing public debate and the possibility of a national referendum, while international organisations continue to express concern over the potential rollback of women's rights protections.

Convention, Article 8; EU Directive, Article 36). Without long-term investment, helplines cannot reliably function 24/7, train and retain staff, or expand accessibility.

For many women, calling a helpline is the very first step in seeking help. A free phone number, available 24 hours a day, helps them break the isolation and offers access to information and support. A national women's helpline is also an entry point to the broader WSS network. Calls are often referred to local shelters, counselling centres, legal aid providers, or rape crisis centres (RCCs).

For decades, women's NGOs have been at the forefront of establishing, running, and sustaining national helplines. Their expertise is rooted in direct work with survivors, feminist research, and international networking. They have developed standards of practice that reflect survivors' real needs, and they have consistently advocated for states to recognise helplines as part of basic services. Helplines run by women's NGOs differ fundamentally from generic services: they are survivor-centred, rights-based, and aware of the gendered structures underlying violence. Over the years, many helplines have become more professional and now receive state support or funding. However, their feminist roots remain essential, as they continue to guide the focus on survivors' needs, safety, and rights.

Overview of findings on national women's helplines in Europe

By 2024, 39 of 46 European countries (85%) report having at least one national women's helpline for survivors of VAWG. This means that seven countries (15%) still do not have a national women's helpline. Of these, four are EU Member States, and three are outside the EU. In some countries, such as **Armenia** and **Belarus**, there is no state-wide service. In others, provision is fragmented or gender-neutral, as in **Belgium**, where separate helplines operate for Flemish- and French-speaking regions, of which one is for women and the other one is not exclusively for women, and in **Latvia**, where helplines exist but the 24/7 services are not women-specific and the specialised helpline does not operate 24/7.

In 2024, most European states continue to operate a single national women's helpline, which is the recommended model. Twenty-nine countries (63%) follow this approach, giving women one well-known number that can be reached from anywhere in the country.

In summary:

- 29 countries operate one helpline
- Seven countries operate two helplines
- Two countries operate three helplines
- The United Kingdom has four helplines, one for each of its nations

By 2024, 37 countries (80%) meet the recommended standards for national women's helplines: free of charge, available 24 hours a day, and grounded in a gendered

understanding of VAW. This marks an increase from 32 countries in the 2023 report.

Language accessibility has continued to improve. In 2024, 29 countries (63%) report that their national women's helpline offers multilingual support. Women's NGOs run most national women's helplines. A smaller number are operated by state institutions, and some countries use shared or changing models. Most helplines now receive at least some state funding, although seven countries still report having no state funding for their national services.

Across the 32 countries that provided figures or estimates, national women's helplines received over 900,000 calls in 2024. This highlights the scale of demand and the central role helplines play in providing information, guidance and support.

Across Europe, many other helplines exist that do not meet the standards for a national women's helpline but still provide information and support to women. These fall broadly into three categories:

- Specialist helplines offering support for specific groups of women or particular forms of violence (e.g. sexualised violence, trafficking, forced marriage, legal advice).
- Gender-neutral national gender-based violence (GBV) helplines, which provide support to all survivors and do not take a gender-specific approach.
- Helplines for related issues, such as homelessness, substance use, migration or asylum, which may intersect with women's experiences of violence.

Alongside national services, there are many regional, local and specialised helplines. These may be linked to shelters, women's centres or RCCs, or they may focus on particular forms of violence. They are especially important in countries without a national women's helpline or where coverage is limited.

KEY FINDINGS

85% of EU Member States and 84% of countries outside the EU have at least one national women's helpline.

39 out of 39 national women's helplines in Europe operate free of charge.

37 out of 39 national women's helplines in Europe are available 24/7.

37 countries (80%) meet the Istanbul Convention standards – the helplines are both free of charge and available 24/7.

Recommendations are included at the end of the chapter on national women's helplines.

🏠 Women's shelters

Women's shelters are a core part of any effective response to VAWG. For women whose homes are no longer safe because of fear, control, or physical harm, having access to secure accommodation can be lifesaving. Shelters are not only emergency places to stay; they are specialist services that protect women's rights, dignity, and autonomy. They give survivors the chance to leave abuse, find safety, and start rebuilding their lives.

The first shelters were created by women's grassroots initiatives, driven by solidarity and political awareness. Today, states have clear duties under international human rights standards to ensure that adequate shelter services exist for women survivors of violence. Article 23 of the Istanbul Convention requires states to establish appropriate, easily accessible shelters in sufficient numbers, offering safe accommodation and proactive outreach to survivors. The Convention's Explanatory Report recommends one shelter place per 10,000 inhabitants. Although this is a minimum benchmark, evidence shows that actual demand often exceeds it, especially during periods of economic hardship, conflict, or broader social crises. Adequate funding, good geographic distribution, and high-quality support are as necessary as meeting the required number of beds.

The report distinguishes between women-only shelters and shelters open to women but not exclusively for them, and highlights the number of women-only shelters in each country. This distinction is important to track, particularly as WSS in some countries are losing funding or service contracts to gender-neutral or generic providers.

Research from across Europe consistently shows that women-only, feminist-run shelters offer more holistic and longer-term support. Their approaches focus on empowerment, self-determination, and rights-based advocacy. These shelters also play a key role in driving policy change and raising awareness about VAWG.

Overview of findings on women's shelters in Europe

Data from 46 European countries show that there are 3,224 shelters accessible to women, of which 2,365 are women-only shelters. Together, they provide 48,860 bed spaces for a population of more than 847 million people. This remains far below the Istanbul Convention's minimum requirement of one family place per 10,000 inhabitants, with only 12 countries (26%) currently meeting this standard.

Compared with the WAVE Country Report 2023, the latest data show a modest increase in both shelters and bed spaces. The total number of shelters accessible to women rose from 3,087 to 3,224, and the number of available beds increased from 39,130 to 48,860. The number of countries meeting the Convention standard grew from 10

to 12. **Montenegro** now meets the minimum requirement following an increase in both shelters and bed spaces, and **Croatia** now exceeds the standard due to an expansion in capacity. **France** now meets the Istanbul Convention standards, although the consistent increase in bed spaces is also due to a different way of presenting data. On the contrary, **Liechtenstein** does not meet the Istanbul Convention standards, while in the previous report, it did. This is due to a difference in data collection: the previous reports erroneously included children's beds in the overall count.

In the EU, the number of shelters accessible to women increased from 2,232 to 2,371, and the number of bed spaces increased from 27,124 to 33,450. The number of EU countries meeting the benchmark rose from seven to nine. In non-EU countries, bed spaces grew from 12,006 to 15,410, though some of this increase reflects new data previously unavailable from **Russia** and **Ukraine**.

Women-only shelters are accessible around the clock in most European countries. Thirty-one of 46 countries (67%) report that all or most women-only shelters offer 24/7 access. Most women-only shelters continue to offer free services, but in 11 countries (24%), women are also asked to contribute financially for their stay, according to their financial capacities, or through access to municipal housing funds or other financing programmes.

Access remains uneven for women who face additional obstacles. Women with uncertain or undocumented residence status can access all or most shelters in 41% of countries. Accessibility for disabled women remains limited: only 37% of countries report that all or most shelters are accessible to women with mobility impairments, and 26% for women with sensory disabilities. Access for women aged 65+ is better, with 61% of countries reporting that all or most shelters accept them. Refugee women can access all or most shelters in 50% of countries, while homeless women can access all or most shelters in 46% of countries. Shelters are more inclusive for women with children with disabilities (43%), while access for women with older sons (14+) is lower (26%). Unaccompanied girls under 18 remain largely excluded, with only 20% of countries allowing them full or broad access.

Specialised shelter provision remains limited but present. Shelters for survivors of trafficking exist in 12 countries, and nine countries have dedicated services for LGBTQI+ survivors. Eleven countries report shelters for migrant and ethnic minority women.

Sustainable funding remains a significant challenge. Forty-four of 46 countries (96%) report some form of state or municipal funding, but **Belarus** has none. In **Moldova**, funding is available but does not extend to the **Transnistrian region**, where shelters rely entirely on international and private support. Many shelters still depend on short-term or project-based funding, affecting stability and long-term

planning. In contrast, a few countries, such as **Sweden**, **Slovenia** and **Iceland**, offer multi-year partnership models that provide greater security.

Women's NGOs remain the leading providers of women-only shelters, running such services in 38 of 46 countries (82%). Other NGOs operate them in 20 countries (43%), faith-based organisations in 14 countries (30%), and state authorities in 17 countries (37%), as well as in the northern part of Cyprus. Four countries report other management models, including government-organised non-governmental organisation (GONGOs) or hybrid partnerships.

Women-only shelters are available across all regions in 16 countries (35%), while in 11 countries (24%), they cover most regions. In 13 countries (28%), they are concentrated mainly in major cities, and in **Armenia**, the **northern part of Cyprus**, and the **Transnistrian region** of the Republic of Moldova, they exist only in the capital. In addition, 36 countries (78%) report other types of shelters that can accommodate women survivors of violence, totalling 859 such facilities, including mixed-gender crisis centres, family shelters, anti-trafficking facilities, and shelters for vulnerable groups.

Women-only shelters remain essential for ensuring safe, specialist support for survivors. Their gender-specific approach cannot be replaced by generic or mixed services. As they are the leading providers in most countries, securing their long-term funding and availability is crucial so that all women can access protection when they need it.

KEY FINDINGS

In Europe, there are 3,224 shelters accessible to women, providing approximately 48,860 bed spaces for women and their children.

Of 46 European countries, 12 (26%) meet the Istanbul Convention's recommended standard of one bed space per 10,000 inhabitants.

18 out of 27 EU Member States (67%) and 16 out of 19 states outside the EU (84%) are still failing to meet the minimum Istanbul Convention standards on the provision of bed spaces.

Recommendations are included at the end of the chapter on women's shelters.

Women's centres

Women's centres are a vital part of specialist support for women and girls survivors of VAWG. They provide non-residential, gender-sensitive services that combine emotional support, legal information, practical assistance, advocacy, and empowerment. Rooted in feminist principles, women's centres respond not only to survivors' immediate needs but also work to challenge and change the social and institutional conditions that allow VAWG to continue. Their support is holistic, long-term and grounded in trust, prioritising women's safety, dignity and autonomy. For the purposes of this report, the term 'women's centre' includes all specialist, non-residential services that work predominantly or exclusively with women survivors of violence and their children.

Across Europe, women's centres operate in varied contexts. Some are long-standing independent feminist NGOs, while others form part of coordinated networks or are connected to shelters and crisis services. Despite this diversity, their core purpose is shared: to offer safe, confidential, women-centred spaces where survivors are believed, supported, and empowered to rebuild their lives. They are often the first place where women seek help, especially when they are not ready or able to enter a shelter. Women's centres are also key actors in prevention work, community outreach, awareness-raising, and policy advocacy, ensuring that survivors' experiences shape national and local responses to violence.

The Istanbul Convention requires states to ensure accessible, well-funded and coordinated specialist services for women survivors of violence. Although it does not name women's centres directly, the Convention's guidance makes clear that such specialist, women-focused services are essential to fulfilling these obligations. It recommends one women's centre offering non-residential support per 50,000 women to ensure adequate availability and access.

Overview of findings on women's centres in Europe

Women's centres providing non-residential specialist support for survivors of VAWG were reported in 44 of 46 (96%) European countries. Only **Belarus** and **Hungary** remain without any such services. Across Europe, at least 3,401 women's centres are operating: 2,510 in EU Member States and 891 in non-EU countries. This represents a decrease compared with the previous report, which reported a total of 4,085 women's centres.

Out of the 3,401 women's centres reported in 2024, 562 centres (16%) are run by women's NGOs, 197 (6%) by other NGOs, 195 (6%) by state authorities, six by faith-based organisations, and seven by other or mixed entities. For 2,434 centres (72%), the managing organisation was not reported, highlighting persistent gaps in national statistical systems. Despite this, women's NGOs remain the

leading providers of specialist support, while state-managed and mixed-service models appear to be increasing in some countries.

Women's centres across Europe provide a broad range of support. Counselling and psychological services are the most widespread, available in 39 countries (85%), often through trauma-informed and empowerment-based approaches. Legal advice is offered in 38 countries (83%), while representation before courts, police or social services is available in 30 countries (65%). Support for social welfare, benefits, or financial issues is available in 29 countries (63%), and housing advice in 31 countries (67%), reflecting the importance of stability and safety for survivors rebuilding their lives. Employment-related support is reported in only 22 countries (48%).

Women's centres also play a crucial coordinating role. Referrals and cooperation with other services are reported in 38 countries (83%), making women's centres central actors within multi-agency responses. Emergency or short-term accommodation linked to women's centres is available in 25 countries (54%), and in some cases, women's shelters also function as centres, offering both residential and non-residential support.

Access to women's centres for those facing additional barriers varies across Europe. About three-quarters of countries report that all or most women's centres are open to specific groups such as migrant, refugee, disabled or older women, but gaps remain. Refugee women can access all or most centres in 29 countries (63%), and undocumented or insecurely documented women in 25 countries (54%). Accessibility for women with disabilities is uneven: 22 countries (48%) report full or broad access for women with mobility impairments, while 20 countries (43%) report similar access for women with sensory or cognitive disabilities. Older women (65+) have the best access, with 30 countries (65%) reporting all or most centres open to them. Access for transgender women varies significantly: 15 countries (33%) report that most or all centres are accessible. Only 15 countries (33%) report full or broad access for unaccompanied girls under 18.

Specialisation within women's centres continues to grow. Centres focusing on trafficking in persons are the most widespread, present in 30 countries (65%). Many countries also report specialised services for other groups or forms of violence.

Reliable, sustainable funding remains essential. Thirty-six of 46 countries (78%) report that public funding is the primary source of support for women's centres. However, funding remains inconsistent and often short-term in many contexts. Stable, multi-year arrangements, where they exist, remain the exception rather than the norm.

❖ KEY FINDINGS

There are 3,401 women's centres in Europe.

44 countries have at least one women's centre; two countries have none (Belarus and Hungary).

74% of all women's centres are in EU Member States, and 26% are in countries outside the EU.

There is one centre per 129,329 women in Europe, reflecting a gap of 61% in the recommended number of women's centres.

The majority of women's centres complying with WAVE's definition are run by women's NGOs.

Recommendations are included at the end of the chapter on women's centres.

☂ Specialist services for survivors of sexualised violence

Sexualised violence is one of the most widespread and harmful forms of VAWG. It covers a broad range of acts that violate women's physical, psychological and sexual integrity and is overwhelmingly perpetrated by men against women and girls. At the centre of its definition is consent. The Istanbul Convention states that sexual violence, including rape, includes any sexual act carried out without a woman's freely given agreement, and that consent cannot be assumed from silence, passivity or lack of resistance.

Although many countries have adopted consent-based legal definitions, survivors still face significant barriers in practice. The burden of proof continues to fall heavily on them, and harmful myths and stereotypes persist. Survivors are often questioned about their behaviour, credibility and character, and must frequently explain why their reaction during an assault did not amount to consent. These practices remain re-traumatising and obstruct access to justice.

Survivors of sexualised violence have immediate and long-term needs that require coordinated, specialist and trauma-informed responses. Immediately after an assault, women may need emergency medical treatment, crisis intervention and, where available, forensic examination. Longer-term needs include trauma counselling, peer support, legal information and advocacy to navigate institutions that can feel intimidating or unsafe.

The Istanbul Convention requires states to criminalise all non-consensual sexual acts (Article 36) and to ensure accessible specialist support through rape crisis centres (RCCs) or sexual violence referral centres (SVRCs) (Article 25). These services must be available regardless of whether a survivor reports the assault. The Convention also recommends at least one RCC or SVRC per 200,000

inhabitants, with services distributed across regions so that women in rural and remote areas can access them.

Specialist services take different forms. RCCs provide immediate, medium- and long-term emotional and practical support, grounded in feminist, survivor-centred principles. They offer counselling, therapy, advocacy and accompaniment in dealings with police, courts and other institutions, and many also run specialised helplines. SVRCs, often based in hospitals, focus on urgent medical and forensic care, crisis intervention and referrals. Other crisis or medical services provide emergency contraception, sexually transmitted disease prevention and trauma-informed psychological care. Specialised helplines offer confidential support, information and referrals, often serving as the first point of contact.

WSS for survivors of sexualised violence work on principles of belief, autonomy, confidentiality and empowerment. Their independence is essential, allowing them to advocate for survivors' rights, challenge systemic shortcomings and press for structural change while maintaining cooperation with state institutions to ensure coordinated support.

Overview of findings on specialised services for survivors of sexualised violence in Europe

The 2024 data show a persistent and significant gap in the availability of specialist services for survivors of sexualised violence across Europe. To meet the Istanbul Convention requirement of one RCC or SVRC per 200,000 inhabitants, 4,248 centres would be needed in the 46 reporting countries. Only 633 centres are currently in place, leaving a gap of 3,615 services, or 85% of the total required.

In the 27 EU Member States, 2,243 services are needed, but only 334 exist, leaving 85% of required RCCs and SVRCs missing. In the 19 non-EU countries, 299 centres operate despite a need for 2,005, leaving an 85% gap.

Only four countries — **Luxembourg, Iceland, Norway and Switzerland** — meet or exceed the Istanbul Convention standard. **Wales** meets the standard, but the **United Kingdom** as a whole does not, with 65% of services missing. **Norway** and **Switzerland** exceed the minimum standard: Norway has 46 centres (compared with 28 required), and Switzerland has 63 centres (surpassing the 45 needed).

Across the 46 countries, the data show 208 RCCs, 221 SVRCs, 204 crisis or medical services and 50 specialist helplines. Provision remains uneven: 14 countries have no RCCs or SVRCs at all, and another 14 have only one or two. Countries with more developed provisions tend to prioritise either RCCs or SVRCs rather than both.

In countries without dedicated specialist centres, WSS often provide support. In **Italy**, for example, feminist women's centres in the D.i.Re – National Women's Network

against Violence (*Donne in Rete contro la Violenza*) offer a full range of specialist services for survivors, including collaboration with forensic and medical services within the national health system.

A wide range of support is available across Europe. Forensic and medical care is reported in 29 countries (63%), and needs assessments are conducted in 31 countries (67%). Specialist psychological care is the most common service, available in 35 countries (76%). Advocacy and legal advice are provided in 27 countries (59%), and 28 countries (61%) report community awareness and prevention activities.

State funding is the main source of financial support for these services in 31 countries (67%), although many still rely on unstable or short-term funding arrangements. Twenty-two countries (48%) receive donations from foundations, trusts or individuals, and 12 (26%) countries report international funding.

Most specialist sexualised violence services are designed for women and girls. Thirty-two countries (70%) provide services for adult women, and 31 (67%) offer support to young women and girls under 18. Twenty-four countries (52%) also provide services for other groups, including men and boys, LGBTQI+ individuals and people with disabilities.

KEY FINDINGS

32 countries out of 46 (70%) have at least one RCC, SVRC, or equivalent specialist service.

A total of 633 specialist centres (RCCs, SVRCs, crisis or medical services) were reported across Europe.

In both EU Member States and countries outside the EU, 85% of the necessary services are missing.

Only four countries (Luxembourg, Iceland, Norway and Switzerland) meet the Istanbul Convention standard of one specialist centre per 200,000 inhabitants.

Recommendations are included at the end of the chapter on specialist services for survivors of sexualised violence.

👉 Primary prevention

Primary prevention aims to stop VAWG before it occurs by addressing its root causes. At the same time, primary prevention also means creating the conditions for equality, safety and respectful behaviour in all areas of life. In its 2024 publication *A Life Free from Violence*, the WAVE Network identifies primary prevention as the most strategic approach to ending VAWG and achieving lasting gender equality. It works at several levels: by educating women, girls, boys and men; engaging communities, including activists, educators and civil society; and seeking change in policies, laws and institutions. Primary prevention can target different groups, such as survivors, potential perpetrators, bystanders, and professionals, through activities such as school-based workshops, feminist self-defence, peer education, and professional training. Together, these measures aim to shift attitudes, behaviours and wider social norms.

Research identifies several primary prevention measures with promising or proven results. Evidence supports school-based programmes that prevent dating violence and help children recognise abuse, as well as home-visitation and health outreach during pregnancy. Empowerment-focused and participatory approaches, including feminist self-defence and community-based initiatives that address structural gender inequality, have also shown positive effects. In contrast, some commonly used approaches, such as general awareness-raising campaigns or confrontational rape-prevention sessions, have not been shown to be effective primary prevention measures. It is essential to distinguish between primary, secondary and tertiary prevention, as these terms are often used interchangeably in state reporting. Primary prevention focuses on stopping violence before it occurs; secondary prevention targets early identification and intervention; and tertiary prevention addresses crisis response, recovery and long-term support.

Article 12 of the Istanbul Convention requires states to prevent VAW by promoting social change, particularly by eliminating the stereotypes and inequalities that sustain it. It calls for gender-sensitive and intersectional measures, attention to vulnerable groups, the promotion of women's empowerment, the rejection of "honour"-based justifications, and active engagement of men and boys in prevention efforts.

Despite strong evidence and cost-effectiveness, primary prevention continues to receive limited and inconsistent investment. Governments across Europe still devote far more resources to responding to violence after it has occurred than to preventing it. Without sustained funding for primary prevention, violence will continue to cause profound harm to women and children and impose high social and economic costs.

Overview of findings on primary prevention

Survey responses show that 32 countries (70%) have an official document that includes primary prevention, such as a national action plan (NAP) or a strategy. This includes separate plans for three of the four nations of the **United Kingdom**, and for **Belgium** at both national and regional levels. Ten countries (22%) reported having no such document. In some cases, such as **Norway**, multiple action plans address different forms of violence, but only 18 countries (39%) include a clear definition of primary prevention. This indicates that even where plans exist, the concept is not always clearly understood or applied.

Funding for primary prevention remains limited and inconsistent. Only 16 countries (35%) reported having government funding for primary prevention activities, and where funding exists, it is often small-scale, regional, or linked to specific interventions. Even when funding is available, it is frequently unstable and insufficient for sustained, long-term work.

A wide range of activities are reported across Europe. Awareness-raising campaigns remain the most common, implemented in 39 countries (85%), despite evidence that they are not effective as a primary prevention tool. More promising approaches are also reported: 31 countries (67%) implement empowerment activities for women and girls, 29 (63%) run school-based primary prevention programmes, and 24 (52%) provide training for primary prevention practitioners. Feminist self-defence activities are present in 20 countries (43%), and 19 countries (41%) report programmes specifically targeting men and boys. Bystander intervention training and self-care activities (both implemented in 13 countries, 28%) are the least widespread. While the diversity of activities is encouraging, the current data do not show how far these interventions reach, and limited funding suggests that coverage is likely to remain modest.

Some governments also create barriers to primary prevention efforts, restricting the development or delivery of programmes. Monitoring and evaluation remain weak: only ten countries (22%) reported that public authorities collect data on primary prevention, and only four countries reported any publicly funded evaluation. In 28 countries (61%), no evaluation was conducted, and 21 (46%) provided mixed or unclear answers. This lack of systematic monitoring makes it difficult to assess effectiveness or identify gaps in provision.

👤 KEY FINDINGS

32 out of 46 countries (70%) have an official document that includes primary prevention of VAWG.

Only 18 countries (39%) include a clear definition of primary prevention in their national framework.

16 countries (35%) report having government funding for primary prevention activities.

Awareness-raising campaigns remain the most common activity (39 countries), followed by empowerment programmes (31), school-based prevention (29), practitioner training (24), feminist self-defence (20), programmes for men and boys (19), bystander training (13) and self-care activities (13).

Recommendations are included at the end of the chapter on primary prevention.

🌟 Technology-facilitated violence against women and girls (TFVAWG)

TFVAWG has expanded rapidly over the past decade as digital technologies have become embedded in everyday life. While online platforms offer opportunities for connection, information and activism, they have also become spaces where abuse can spread quickly and with significant impact. Women and girls are disproportionately targeted, and many forms of TFVAWG remain difficult to detect, prove or sanction, leaving survivors with limited protection.

TFVAWG includes any form of GBV carried out, enabled, or intensified through digital technologies or online environments. Its particular severity lies in its scale, reach and persistence: perpetrators can act anonymously, abusive content can be endlessly replicated and shared, and once online, it is often impossible to entirely remove. These dynamics multiply harm, amplify threats and may lead to further abuse both online and offline.

TFVAWG is closely connected to offline violence. Online abuse frequently escalates into physical harm, while violence that begins offline often continues digitally through the non-consensual recording or sharing of intimate images, digital surveillance or cyberstalking during and after abusive relationships. The combined impact can include fear, self-censorship, mental health harms and withdrawal from public or professional life. Women in public roles face particularly high levels of targeted online abuse aimed at silencing them.

GREVIO's General Recommendation No. 1 (2021) confirms that the obligations under the Istanbul Convention

apply fully to TFVAWG. The Directive (EU) 2024/1385 criminalises the non-consensual sharing of intimate or manipulated material, cyberstalking, cyber harassment and cyber incitement to violence or hatred.

Overview of findings on TFVAWG

Delegates' responses show a marked rise in TFVAWG across Europe. Thirty-seven out of 46 countries (80%) reported an increase over the past two years. In nine countries, respondents were unsure, often because no official data is collected. Some noted that the rise may reflect increased visibility and willingness to report rather than an actual increase in incidents. Even so, the evidence points to growing levels of TFVAWG across most of Europe.

The most common forms of TFVAWG reported were cyberstalking, surveillance and spyware, identified in 30 countries (65%), and online harassment, hate speech and threats, also reported in 30 countries (65%). Image-based abuse, including non-consensual intimate image sharing, was reported as increasing in 26 countries (57%). Emerging forms are also becoming more visible: deepfakes and other AI-generated abuse were noted in 10 countries (22%), while sextortion and doxxing were each reported in nine countries (20%). Despite this, TFVAWG remains under-reported. Barriers include stigma, fear of institutional mistrust, and lack of awareness that specialised support exists. Marginalised groups such as migrant and minoritised women, as well as teenage girls, face even greater obstacles to reporting.

The data consistently highlighted that TFVAWG disproportionately affects women in public roles. Politicians, journalists, human rights defenders and feminist activists are frequent targets of coordinated online attacks intended to silence or discredit them. Young women and girls were also repeatedly identified as being at high risk, particularly from cyberbullying, grooming and image-based abuse.

Most countries reported new strategies from WSS to address TFVAWG. Thirty countries (65%) described concrete initiatives, while 12 countries (26%) indicated that they have no specific strategy yet. Innovation across Europe includes new apps, chatbots and digital safety tools. Examples include the Bright Sky app in **Albania** and the **Czech Republic**; the Viktoria WhatsApp chatbot in **Poland**; TinyCheck in **Luxembourg** for identifying stalkerware; Spain's 24/7 TFVAWG chatbots; and the SafetyNed digital safety service in the **Netherlands**. Many WSS have developed new safety protocols to address online monitoring, spyware, and coercive control carried out through digital means. Capacity-building efforts are also expanding, with WSS investing in staff training and developing new resources.

Prevention and awareness-raising initiatives have grown, and in several countries, WSS have directly shaped national legal and policy responses. **Albania's** national helpline

integrated TFVAWG protocols and proposed a draft digital violence law; the **Republic of Moldova** introduced legal reforms recognising online abuse; and in **England**, WSS advocacy contributed to the 2023 Online Safety Act. Other work focuses on content removal and platform accountability, such as the support in **Belgium** for women seeking to remove non-consensually shared images.

KEY FINDINGS

37 out of 46 countries (80%) reported an increase in TFVAWG over the past two years.

The most common forms of TFVAWG were cyberstalking and digital surveillance (30 countries), and online harassment, hate speech and threats (30 countries).

Image-based abuse increased in 26 countries, while emerging forms such as deepfakes, sextortion and doxxing were also reported.

30 countries reported new strategies by WSS to address TFVAWG, including digital safety tools, apps, chatbots and updated support protocols.

Recommendations are included at the end of the chapter on TFVAWG.

Femicide

Femicide refers to the gender-related killing of women and girls. There is still no single, internationally agreed-upon definition, and states, institutions, and researchers use the term differently, which makes comparisons difficult. In many countries, femicide is narrowly understood as the killing of a woman by a current or former partner. Feminist organisations and international mechanisms, including the UN Special Rapporteur on Violence Against Women and Girls, argue that this approach is far too limited and that femicide includes many forms of gender-related killings, including so-called honour killings, dowry-related murders, killings linked to witchcraft accusations, conflict-related deaths, killings connected to trafficking or sexualised violence, and cases where state institutions fail to protect a woman from known violence.

Reliable and comparable data on femicide remain extremely difficult to obtain. The lack of a shared concept affects how cases are recorded and classified. Most countries collect homicide data rather than femicide data, and when gender is documented, it is usually as a demographic detail rather than as a contextual or motivating factor. As a result, many gender-related killings of women never appear as such in official statistics. In some contexts, only intimate partner or family-related homicides are counted as femicide, while in others even these are recorded without reference to gender.

Across Europe, feminist organisations have taken the lead in documenting femicide. They gather and verify cases using media reports, legal records and information from WSS. Their work goes beyond counting deaths: they analyse patterns of coercive control, highlight systemic failings in state responses, and link individual cases to wider inequalities. Many publish reports, speak to the media, run campaigns and organise protests to draw attention to institutional shortcomings and the preventable nature of these killings. For many organisations, documenting femicide is a feminist act of remembrance and resistance, restoring visibility to women's lives and demanding accountability.

At the international level, the UN Special Rapporteur on Violence Against Women and Girls continues to advocate for national femicide watches or observatories to be established in every country. These mechanisms are intended to record every case of gender-related killing of women and girls, analyse risk factors, identify institutional failures and strengthen prevention efforts.

Overview of findings on femicide

Information provided by WAVE delegates from 35 out of 46 European countries indicates that at least 2,871 women were victims of femicide in 2024. Data were gathered from a mix of official statistics, media reports, research, and monitoring by WSS. This figure includes estimates from national experts and feminist organisations where official data were missing. No data were available from nine countries, and in several others, only partial or unofficial figures could be collected. Of the total number reported, 969 femicides occurred in EU Member States and 1,902 in countries outside the EU. In many cases, these numbers represent minimum estimates due to the continued lack of systematic monitoring across Europe.

Only 23 countries provided any disaggregated information, meaning that the figures below represent partial totals based on 1,120 cases where additional details were available. Among these cases, 843 women (76%) were killed by a current or former intimate partner, 217 women (19%) were killed by a family member other than a partner, 36 women (3%) by an acquaintance, and 24 women (2%) by other perpetrators, including strangers or unknown offenders.

Information on whether victims had previously received specialist support remains very limited. Of 46 countries, only 16 (35%) provided any information. Among these, 13 (81%) reported that some victims had received support, two (13%) reported that most had, and one (6%) reported that none had. Twenty-two countries reported not knowing, and eight provided no data. This means that in nearly half of European countries, there is no systematic information on whether women killed by men had prior contact with women's services, shelters or other specialist support.

Many women who were later killed had previously contacted state institutions. Out of 46 countries, 24 (52%) reported that some or most femicide victims had earlier sought help from authorities, most commonly the police. These cases highlight significant gaps in protection and follow-up when women report violence.

Use of the term 'femicide' varies widely. It is officially used in nine countries (20%), under discussion or in development in four (9%), and utilised informally or mainly in public debate in 22 (48%). Respondents in 11 countries (24%) reported that state institutions do not use the term at all. In many cases, feminist NGOs and activists have driven the adoption of the term in public, academic and media discourse to highlight the gendered patterns behind the killings and to advocate for accountability.

Data collection remains inconsistent. Only six (13%) countries reported that the state officially collects femicide data. NGOs are the most active in documenting femicide, compiling data in 29 countries (63%), often through dedicated databases or reports. Police collect relevant data in 21 countries (46%), usually under general homicide statistics without identifying gender-related motives. Other state bodies collect related information in 19 countries (41%), though rarely under a specific femicide category. Media monitoring is a key source in 10 countries (22%), while academic research is a key source in 12 (26%).

Only a minority of countries have mechanisms to analyse femicide cases and identify lessons for prevention systematically. Twelve (26%) countries reported having such processes in place or under development, indicating early but uneven progress towards more coordinated and accountable responses.

Recommendations are included at the end of the chapter on femicide.

KEY FINDINGS

There were at least 2,871 femicides in 2024, based on data and estimates from 35 of 46 countries.

The term 'femicide' is officially used in nine countries, is under discussion in four, and appears mainly in public or academic discourse in 22 countries. In 11 countries, it is not used by state authorities.

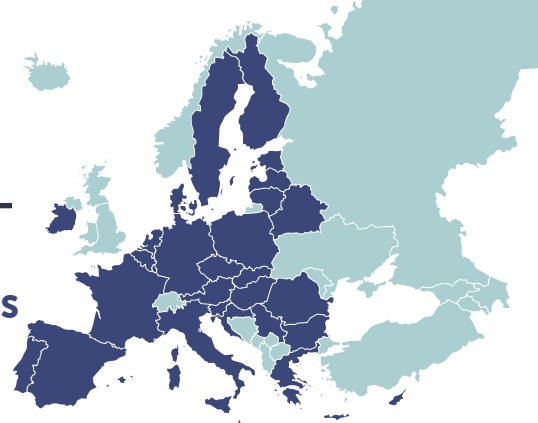
In 29 countries, feminist NGOs are the primary collectors of femicide data; police or other state agencies collect homicide data, but these statistics often do not identify gender-related motives. Many killings of women are recorded without reference to gender.

Only six countries officially collect femicide data at the state level, and 12 countries have a femicide watch or similar mechanism, though most are not yet integrated into national prevention systems.

Tables



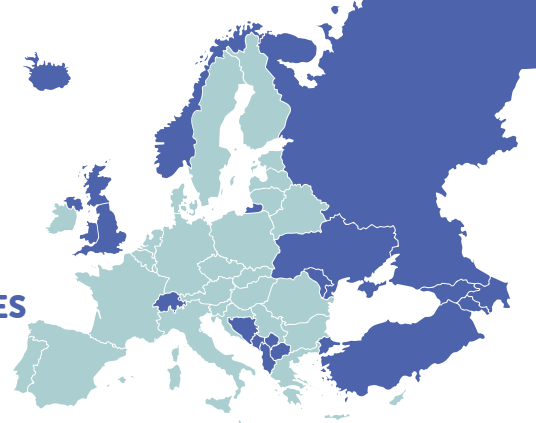
**TABLE 1: NATIONAL WOMEN'S HELPLINES
IN EU MEMBER STATES (2024)**



	Existence of a National Women's Helpline	National Women's Helpline Free of Charge	National Women's Helpline operating 24/7	National Women's Helpline Free of Charge and Operating 24/7 (=meeting the standards of the Istanbul Convention)
Austria	Yes	Yes	Yes	Yes
Belgium	No	—	—	No
Bulgaria	Yes	Yes	Yes	Yes
Croatia	Yes	Yes	Yes	Yes
Republic of Cyprus	Yes	Yes	Yes	Yes
Czech Republic	Yes	Yes	Yes	Yes
Denmark	Yes	Yes	Yes	Yes
Estonia	Yes	Yes	Yes	Yes
Finland	Yes	Yes	Yes	Yes
France	Yes	Yes	Yes	Yes
Germany	Yes	Yes	Yes	Yes
Greece	Yes	Yes	Yes	Yes
Hungary	Yes	Yes	No	No
Ireland	Yes	Yes	Yes	Yes
Italy	Yes	Yes	Yes	Yes
Latvia	No	—	—	No
Lithuania	Yes	Yes	Yes	Yes
Luxembourg	Yes	Yes	No	No
Malta	No	—	—	No
Netherlands	No	—	—	No
Poland	Yes	Yes	Yes	Yes
Portugal	Yes	Yes	Yes	Yes
Romania	Yes	Yes	Yes	Yes
Slovakia	Yes	Yes	Yes	Yes
Slovenia	Yes	Yes	Yes	Yes
Spain	Yes	Yes	Yes	Yes
Sweden	Yes	Yes	Yes	Yes
TOTAL (27)	23 yes 4 no	23 yes	21 yes 2 no	21 yes 6 no



**TABLE 2: NATIONAL WOMEN'S HELPLINES
IN EUROPEAN COUNTRIES OUTSIDE
THE EU (2024)**



	Existence of a National Women's Helpline	National Women's Helpline Free of Charge	National Women's Helpline operating 24/7	National Women's Helpline Free of Charge and Operating 24/7 (=meeting the standards of the Istanbul Convention)
Albania	Yes	Yes	Yes	Yes
Armenia	No	—	—	No
Azerbaijan	Yes	Yes	Yes	Yes
Belarus	No	—	—	No
Bosnia & Herzegovina	Yes	Yes	Yes	Yes
Georgia	Yes	Yes	Yes	Yes
Iceland	Yes	Yes	Yes	Yes
Kosovo	Yes	Yes	Yes	Yes
Liechtenstein	Yes	Yes	Yes	Yes
North Macedonia	Yes	Yes	Yes	Yes
Republic of Moldova	Yes	Yes	Yes	Yes
Montenegro	Yes	Yes	Yes	Yes
Norway	Yes	Yes	Yes	Yes
Russian Federation	Yes	Yes	Yes	Yes
Serbia	Yes	Yes	Yes	Yes
Switzerland	No	—	—	No
Türkiye	Yes	Yes	Yes	Yes
Ukraine	Yes	Yes	Yes	Yes
United Kingdom	Yes	Yes	Yes	Yes
England	Yes	Yes	Yes	Yes
Northern Ireland	Yes	Yes	Yes	Yes
Scotland	Yes	Yes	Yes	Yes
Wales	Yes	Yes	Yes	Yes
TOTAL (19)	16 yes 3 no	16 yes	16 yes	16 yes 3 no



**Find the full list of women's helplines
in all 46 countries on our website:**

wave-network.org/list-of-helplines-in-46-countries/





**TABLE 3: SHELTERS ACCESSIBLE TO WOMEN
IN EU MEMBER STATES (2024)**

	Total Population	Total number of women- only shelters	Total number of shelters accessible to women	Existing number of beds	Number of beds needed	Number of beds missing	Percentage of beds missing
Austria	9,158,750	32	32	505	916	411	45%
Belgium	11,817,096	27	44 ⁹	1,847 ¹⁰	1181	0	0%
Bulgaria	6,445,481	13	13	150	645	495	77%
Croatia	3,861,967	11	26	402	386	0	0%
Republic of Cyprus	966,365	4	5	79 ¹¹	97	18	18%
Czech Republic	10,900,555	7	N/A	200	1090	890	82%
Denmark	5,961,249	79	95	966	596	0	0%
Estonia	1,374,687	19	19	170	137	0	0%
Finland	5,603,851	2	28	228 ¹²	560	332	59%
France	68,467,362	52	250	11,000	6847	—	0%
Germany	83,456,045	380	380	2,720 ¹³	8346	5626	67%
Greece	10,400,720	20	N/A	400	1040	640	62%
Hungary	9,584,627	4	63	460 ¹⁴	958	—	52%
Ireland	5,351,681	20	20	159	535	376	70%
Italy	58,971,230	464	N/A	3,200 ¹⁵	5897	2697	46%
Latvia	1,871,882	17	N/A	N/A	187	N/A	N/A
Lithuania	2,885,891	2	N/A	28	289	261	90%
Luxembourg	672,050	7	7	196	7	0	0%
Malta	563,443	3	7	90	56	0	0%
Netherlands	17,942,942	20	N/A	986 ¹⁶	1794	808	45%
Poland	36,620,970	2	40	605 ¹⁷	3662	3057	83%
Portugal	10,639,726	62	62	1,024	1064	40	4%
Romania	19,067,576	144	144	1,260	1907	647	34%
Slovakia	5,424,687	10	N/A	273	542	269	50%
Slovenia	2,118,697	29	32	473	212	0	0%
Spain	48,619,695	307	N/A	3,810	4862	1052	22%
Sweden	10,551,707	N/A	257	2,219 ¹⁸	1055	0	0%
TOTAL (27)	448,696,087	1,737	2,371¹⁹	33,450	44,870	18,117	40%

9 This number does not include the 82 residential support modules accessible to women in Flanders.

10 This number only includes the beds in shelters in Brussels and Wallonia, as the number of beds in Flanders is not available.

11 Number of beds in the four women-only shelters operated by SPAVO.

12 This is the total number of beds accessible to women in all shelters. The 23 spaces in the two women-only shelters are also included in this figure.

13 2,720 is the number of family places. One family place corresponds to one bed for the woman and 1.54 beds for children. This is based on Germany's average birth rate. Data from WAVE Country Report 2023.

14 This figure excludes the six bed spaces in the protected accommodation only for male survivors of trafficking.

15 Data from the Country Report 2023, as the 2024 national statistics were not published at the time of writing.

16 This includes only the number of beds in women-only shelters.

17 This excludes the beds in the women-only shelter in Krakow, as the number of beds is not available.

18 This number only includes the beds in the 190 shelters which answered the questionnaire for Sweden's National Board of Health and Welfare shelter mapping report.

19 Number includes also the women-only shelters in the countries without an available number of all shelters accessible to women.


**TABLE 4: SHELTERS ACCESSIBLE TO WOMEN
IN EUROPEAN COUNTRIES OUTSIDE THE EU (2024)**

	Total Population	Total number of women- only shelters	Total number of shelters accessible to women	Existing number of beds	Number of beds needed	Number of beds missing	Percentage of beds missing
Albania	2,761,785 ²⁰	N/A	26	153	276	123	45%
Armenia	2,991,201	2	4	24 ²¹	299	275	92%
Azerbaijan	10,180,770	2	10	85 ²²	1018	933	92%
Belarus	9,475,174 ²³	3	3	20	948	928	98%
Bosnia & Herzegovina	3,531,159 ²⁴	8	9	213	353	140	40%
Georgia	3,694,608	6	6	N/A	369	N/A	N/A
Iceland	383,567	2	2	28	38	10	27%
Kosovo	1,773,971 ²⁵	10	11	180	177	0	0%
Liechtenstein	40,015	1	1	3	4	1	25%
North Macedonia	2,423,287	13	13	235	242	7	3%
Republic of Moldova	623,680	4	4	64	6	0	0%
Montenegro	1,826,247	14	16	67 ²⁶	183	116	63%
Norway	5,550,217	0	41	969 ²⁷	555	0	0%
Russian Federation	143,666,931 ²⁸	30	134 ²⁹	2,423 ³⁰	14,367	11,944	83%
Serbia	6,605,168	11	12	184	661	477	72%
Switzerland	8,962,258	23	N/A	462	896	434	48%
Türkiye	85,372,377	150	150	3,677	8,537	4,860	57%
Ukraine	40,997,698 ³¹	60	60	1,035	4,100	3,065	75%
United Kingdom ³²	68,322,005	289	328	5,588	6,832	1,392	20%
England	57,690,323 ³³	221	280	4,551	5,769	1,218	21%
Northern Ireland	1,920,382 ³⁴	14	14	340	192	0	0%
Scotland	5,546,900 ³⁵	34	34	425	555	130	23%
Wales	3,164,400 ³⁶	20	N/A	272 ³⁷	316	44	14%
TOTAL (19)	398,620,806	628	853	15,410	39,862	24,705	62%

20 The latest available population data on Eurostat is from 2023.

21 This number only includes the beds in the two women-only shelters run by the Women's Support Centre (WSC).

22 This figure only refers to the beds in the two women-only shelters.

23 The most recent available data on Eurostat is from 2019.

24 Agency for Statistics of Bosnia and Herzegovina, 2023. Latest population data from 2013: https://bhas.gov.ba/data/Publikacije/Bilteni/2024/DEM_00_2023_TB_1_BS.pdf

25 The latest available Eurostat population data for Kosovo is from 2022.

26 Number of beds in the women-only shelters.

27 Number of available beds based on state data from 2019.

28 Latest available data on Eurostat's database is from 2014.

29 This figure comes from the Department of Social Protection and Social Services of the Ministry of Labor of Russia and is repeatedly cited by officials in 2024-2025. However, it is not possible to know if this figure includes the estimated 30-40 women-only shelters based on unofficially information collected by NGOs. Additionally, this figure does not include the crisis centres run by the Russian Orthodox Church.

30 Ibid.

31 The most recent available data on Eurostat is from 2022.

32 United Kingdom encompasses England, Northern Ireland, Scotland and Wales.

33 Office for National Statistics (ONS), Estimates of the population for the UK, England, Wales, Scotland, and Northern Ireland, mid-2023 edition, <https://www.ons.gov.uk/peoplepopulationandcommunity/populationandmigration/populationestimates/datasets/populationestimatesforukenglandandwalesscotlandandnorthernireland>

34 Northern Ireland Statistics and Research Agency, population estimates for mid-2023, <https://www.nisra.gov.uk/publications/2023-mid-year-population-estimates-northern-ireland-and-estimates-population-aged-85>.

35 National Records of Scotland, mid-year population estimates 2024, <https://www.nrscotland.gov.uk/publications/population-estimates-time-series-data/>

36 Office for National Statistics (ONS), Estimates of the population for the UK, England, Wales, Scotland, and Northern Ireland, mid-2023 edition, <https://www.ons.gov.uk/peoplepopulationandcommunity/populationandmigration/populationestimates/datasets/populationestimatesforukenglandandwalesscotlandandnorthernireland>

37 This figure is from the WAVE Country Report 2023.



**TABLE 5: WOMEN'S CENTRES
IN EU MEMBER STATES (2024)**

	Number of Centres	Current number of female population per centre	Beneficiaries	Entities running women's centres (including number of centres per entity when available)
Austria	65	71,445	N/A	N/A
Belgium	44	135,934	Women-only (10 centres) All survivors (34 centres)	Women's NGOs (4 centres); Other NGOs (34 centres); State (6 centres)
Bulgaria	20	167,389	Women-only (0 centres) All survivors (20 centre)	Women's NGOs (12 centres); Other NGOs (5 centres); State (3 centres)
Croatia	61	32,567	Women-only (30 centres) All survivors (31 centres)	Women's NGOs (50 centres); Other NGOs (11 centres)
Republic of Cyprus	1	490,771	Women-only (0 centres) All survivors (1 center)	Women's NGO
Czech Republic	26	213,767	Women-only (5 centres) All survivors (21 centres)	Women's NGOs (5 centres); Other NGOs (N/A); State (N/A)
Denmark	6	499,593	Women-only (4 centres) All survivors (2 centres)	Women's NGOs; State
Estonia	19	38,057	Women-only (19 centres) All survivors (0 centres)	Women's NGOs (4 centres); Other NGOs (12 centres); State (3 centres)
Finland	28	101,070	Women-only (3 centres) All survivors (25 centres)	Women's NGOs (2 centres); Other NGOs (26 centres)
France	260	135,659	N/A	Women's NGOs; Other NGOs
Germany	526	80,407	N/A	Women's NGOs; Welfare organisations; Other
Greece	44 ³⁸	120,542	N/A	Women's NGOs, State (44);
Hungary	0	—	Women-only (0 centres) All survivors (0 centres)	—
Ireland	37	143,347	N/A	NGOs
Italy	404	74,566	N/A	Women's NGOs; Other NGOs; State
Latvia	3 ³⁹	334,381	N/A	Women's NGOs (3 centres)
Lithuania	24	63,333	Women-only (0 centres) All survivors (24 centres)	Women's NGOs (15 centres); Other NGOs (2 centres); State (7 centres)
Luxembourg	8	41,722	N/A	Women's NGOs (4 centres); Other NGOs (1 centre); Other (3 centres)
Malta	8	33,087	N/A	Women's NGOs; Other NGOs; Police
Netherlands	20	451,109	N/A	Women's NGOs (20 centres)
Poland	38	497,763	N/A	Women's NGO (1 centre); Other NGO State; Faith-based organisation (3 centres)
Portugal	140	39,687	N/A	Women's NGOs; Other
Romania	15	653,080	N/A	Women's NGOs (6 centres); Other NGOs (4 centres); State (6 centres); Faith-based organisation (1 centre) ⁴⁰
Slovakia	22	125,976	Women-only (13 centres) All survivors (9 centres)	Women's NGOs (13); Other NGOs (7); State (1); Other
Slovenia	16	65,970	Women-only (3 centres) All survivors (13 centres)	Women's NGOs (8 centres); Other NGOs (3 centres); State (2 centres); Faith-based organisations (3 centres)
Spain	475	52,195	N/A	Women's NGOs; Other NGOs; State; Private companies
Sweden	200	26,196	N/A	Women's NGOs; Other NGOs; State
TOTAL (27)	2,510	92,465	Women-only (87 centres) All survivors (180 centres) N/A (15 countries)	Women's NGOs (147 centres); Other NGOs (105 centres); State (72 centres); Other (3); Faith- based organisations (4 centres)

38 Number of state-run women's centres. Data about NGO-run women's centres is not available.

39 This figure is from the WAVE Country Report 2023.

40 Some women's centres are run by multiple entities (e.g., an NGO that is also a religious institution).



**TABLE 6: WOMEN'S CENTRES
EUROPEAN COUNTRIES OUTSIDE OF THE EU**

	Number of Centres	Current number of female population per centre	Beneficiaries	Entities running women's centres (including number of centres per entity when available)
Albania	12	116,239	Women-only (5 centres) All survivors (5 centres) ⁴¹	Women's NGOs; State; Faith-based organisations
Armenia	11	14,334	N/A	Women's NGOs (11 centres)
Azerbaijan	11	464,869	N/A	State (11 centres)
Belarus	0	N/A	N/A	N/A
Bosnia & Herzegovina	11	163,535	N/A	Women's NGOs (9 centres); Other NGOs (2 centres)
Georgia	10	192,014	N/A	Women's NGOs; State
Iceland	7	26,716	Women-only (2 centres) All survivors (5 centres)	Women's NGOs (3 centres); Collaboration between state, municipalities, and NGOs (4 centres)
Kosovo	3	301,181	N/A	Women's NGOs (3 centres)
Liechtenstein	2	10,078	N/A	Women's NGOs (2 centres)
North Macedonia	11	83,773	Women-only (5 centres) All survivors (6 centres)	Women's NGOs (11 centres)
Republic of Moldova	16	80,862	Women-only (10 centres) All survivors (6 centres)	Women's NGOs (11 centres); Other NGOs (1 centre); State (4 centres)
Montenegro	7	45,265	N/A	Women's NGOs (7 centres)
Norway	41	67,183	N/A	Women's NGOs; Other NGOs; State
Russian Federation	150 ⁴²	514,134 ⁴³	N/A	N/A
Serbia	42	80,781	Women-only (25 centres) All survivors (17 centres)	Women's NGOs (32 centres); Other NGOs (10 centres)
Switzerland	24	187,947	N/A	Women's NGOs; State
Türkiye	157	271,582	Women-only (73 centres) All survivors (84 centres)	Women's NGOs (60 centres); State (97 centres)
Ukraine	N/A	—	N/A	N/A
United Kingdom	376	92,653	Women-only (152 centres) All survivors (193 centres)	Women's NGOs (266 centres); Other NGOs (79 centres); State (11 centres)
England	308	95,478	Women-only (116 centres) All survivors (192 centres)	Women's NGOs (218 centres); Other NGOs (79 centres); State (11 centres)
Northern Ireland	11	87,913	N/A	Women's NGOs (11 centres)
Scotland	37	77,074	Women-only (36 centres) All survivors (1 centre)	Women's NGOs (37 centres)
Wales	20	80,574	N/A	N/A
TOTAL (19)	891	231,574	Women-only (272 centres) All survivors (316 centre) N/A (12 countries)	Women's NGOs (415); State (123); Other NGOs (92 centres); Collaboration between state, municipalities, and NGOs (4 centres)

41 There are two extra centres in Albania that engage in advocacy activities or social services, supporting broader gender equality efforts.

42 This figure is from the WAVE Country Report 2023. Mappings estimate that there around 200 services for survivors of violence (state/faith-based organisations/NGOs) available nationwide, but this figure lumps all services together including helplines, counselling centres, and shelters.

43 Estimate based on the latest available data on female population in 2014.

Methodology

Introduction

The WAVE Country Report is the primary publication of the WAVE Network, published biennially. The Country Report aims to survey the situation of women's specialist services (WSS) in 46 European countries and identify current progress and gaps concerning service provision, emphasising five key areas: women's helplines, women's shelters, women's centres, specialist services for survivors of sexualised violence, and primary prevention services. To do so, it draws on both quantitative and qualitative data to assess the number of services available in the surveyed countries and to provide insights into their characteristics and accessibility. Additionally, the Report provides a chapter on femicide, with insights into the related data collection, and for the first time, a chapter about technology-facilitated violence against women and girls (TFVAWG).

This approach is in line with relevant provisions on specialist support services enshrined in the Council of Europe Convention on preventing and combating violence against women and domestic violence, also known as the Istanbul Convention. Not all countries assessed in the Report have

signed and ratified the Istanbul Convention. Nevertheless, its provisions are considered in this report to assess service availability across Europe and enable comparison across years and countries. Moreover, other legal instruments are referenced in the chapters, such as the Directive EU 2024/1385 on combating violence against women (VAW) and domestic violence, and the Convention on the Elimination of All Forms of Discrimination against Women (CEDAW).

The purpose of the WAVE Country Report is to depict the situation of WSS in Europe from a feminist civil society perspective and to reaffirm why WSS are best placed to provide support for women, their children and girls who are experiencing gender-based violence (GBV) and to prevent violence from happening. As the only report of its kind, the WAVE Country Report regularly collects and compares data, enabling monitoring of states' compliance with international standards for service provision and serving as a crucial tool for advocating improvements at the national and European levels and, eventually, for combating VAW in Europe.

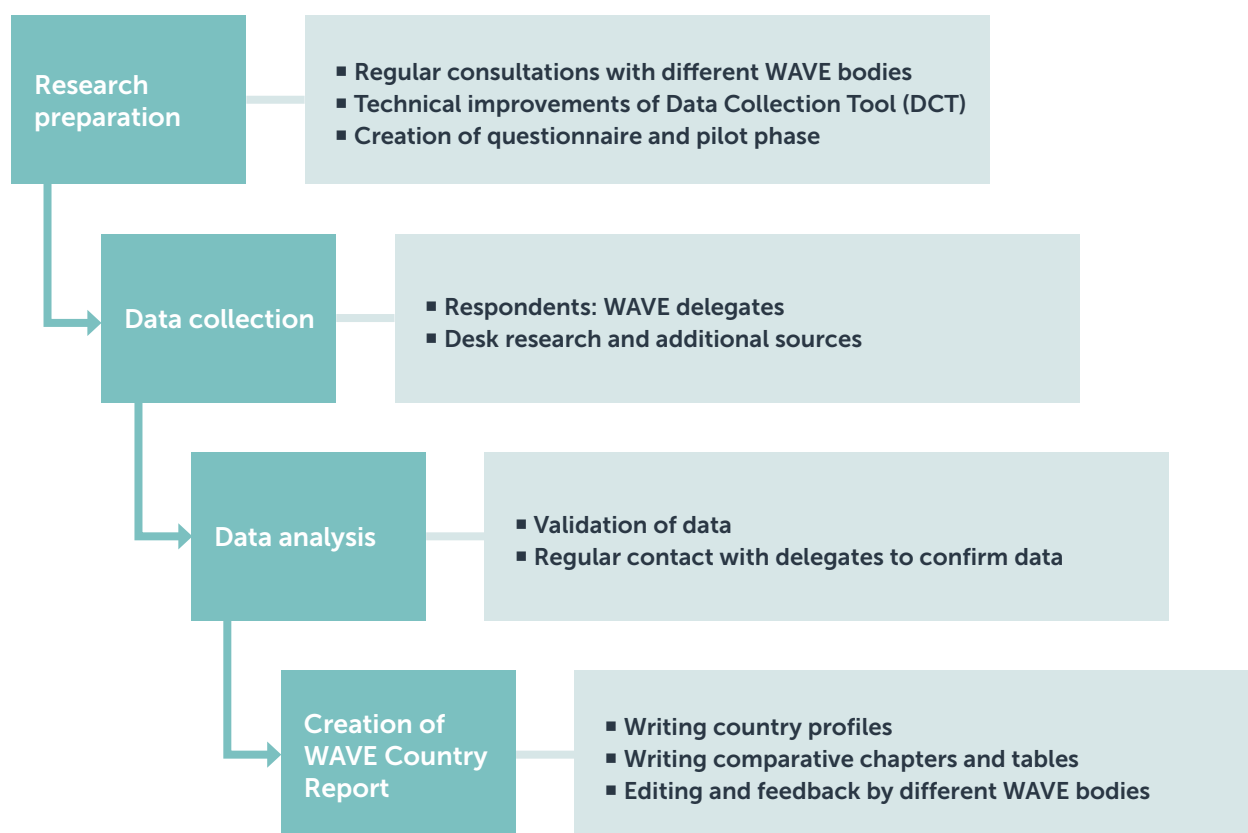


Figure 1: Methodological process of WAVE Country Report 2025

Research preparation

A collective effort

Multiple WAVE bodies are involved in the development process of the WAVE Country Report.

The Editing Group was established in 2021 to support the WAVE team in the improvement of the WAVE Country Report and questionnaire and seeks to implement knowledge and expertise from WAVE's diverse membership into the Country Report. The Editing Group involved in the research preparation of the Country Report 2025 comprises 12 members, including WAVE Advisory Board Members, Board Members, and WAVE Team Members. The members' responsibilities include providing their expert advice and support on key decisions regarding the methodology and participating in the editing of the Report.

In 2023, the WAVE Country Report included, for the first time, a focus on primary prevention activities. In 2025, after evaluating the responses, the questionnaire section on primary prevention was further developed to improve data collection and reliability. This was done with the support of experts from the former WAVE Thematic Working Group on primary prevention, who provided feedback on the questionnaire and were involved in editing the related chapter.

Additionally, the WAVE Country Report is presented and discussed regularly with the WAVE Advisory Board during its in-person meetings. The Advisory Board consists of the same delegates and co-delegates who are responsible for the data collection at the national level. Presenting the latest published report and discussing its strategic aims help guide its development over the years, based on the experience and input of WAVE delegates from 46 countries. In May 2024 and May 2025, the Advisory Board met in Vienna, and sessions on the Country Report were held to present data from the previous report and gather input for the current one.

Lastly, the WAVE Board and management discuss key methodological and advocacy decisions as needed and guide the research and publications of the WAVE Network.

The online Data Collection Tool

The Data Collection Tool (DCT) is an online platform run by the WAVE Network to collect data on WSS in Europe. It includes a questionnaire in English divided into a set of general questions and six core sections: national women's helplines, shelters accessible to women, women's centres, support services for survivors of sexualised violence, primary prevention activities, and femicide. In 2025, the general questions section also included two open questions on TFWAWG. The questionnaire consists of both closed- and open-ended questions, and collects both quantitative and

qualitative data. In 2025, the DCT was further developed to improve the quality of collected information, make it more user-friendly for respondents, and mitigate differences in support systems and data-collection processes. Each country included in the report has dedicated access to provide the data. Additionally, a number of countries complete multiple questionnaires to facilitate data collection in regions with specific forms of administration or where states do not exercise control over specific territories.

Data collection process

Pilot phase and data collection process

Before starting the data collection process for the current Country Report, the updated DCT was piloted by Editing Group members from five countries (Italy, Serbia, Romania, Russia, and the United Kingdom) in March 2025. The goal was to gather feedback and assess the DCT's user-friendliness, identifying areas for improvement. This pilot phase further improved the structure and conciseness of the questionnaire while ensuring comprehensive quantitative and qualitative data collection. The actual data collection took place between April and June 2025 and focused on data from 2024.

The data presented in this report were collected from WAVE member organisations across Europe, represented by one delegate and one co-delegate from each country. When filling out the questionnaire, country delegates were encouraged to consult with other WAVE Members in their countries and relevant government agencies as needed to provide reliable, verified data. The data in the report not only refer to services offered by WAVE members but also to all services available in each country.

The data collected through the DCT was complemented with desk research when, for example, information was lacking or the data provided was not precise. Additional sources also used to prepare the country profiles include Group of Experts on Action against Violence against Women and Domestic Violence (GREVIO) baseline evaluation reports, CEDAW periodic reports, and reports published by national statistical institutes.

Countries included in the data collection

The data analysed in the report cover 46 European countries (27 European Union (EU) Member States and 19 European countries outside the EU) and were provided by WAVE delegates from each country, with support from WAVE Team members. In some countries, more delegates were involved in data collection, each providing data for their own region or nation. This is the case for the United Kingdom (UK), Belgium, Bosnia and Herzegovina, the Republic of Cyprus (including the northern part of Cyprus) and the Republic of Moldova (including the Transnistrian

region of the Republic of Moldova). In the case of three countries (Latvia, Liechtenstein, and Norway), data collection was conducted by the WAVE Team through desk research, as it was not possible to obtain the necessary information from these countries during the data collection period.

Regarding the UK, delegates from each constituent country (England, Northern Ireland, Scotland, and Wales) completed a separate questionnaire. Country profiles were drafted for each of the UK countries; however, when examining service provision in Europe and in countries outside the EU, the UK was counted as a single entity. Two questionnaires were also completed by delegates from Belgium, one covering Flanders (the Flemish-speaking part) and another covering Brussels and Wallonia (the French-speaking part). One country profile was drafted for Belgium, covering the overall situation of service provision in the country and providing specific information for each of the two regions. Similarly, delegates from Bosnia and Herzegovina completed two questionnaires: one for the Federation of Bosnia and Herzegovina and the other for Republika Srpska. One country profile was drafted to illustrate the overall situation in Bosnia and Herzegovina and its specificities. In the case of the Republic of Cyprus, since the government does not exercise direct control over the territory of the northern part of Cyprus, administered by Turkish Cypriot authorities, two questionnaires were filled out by WAVE delegates; therefore, the country profile presents the data from the northern part of Cyprus in a different section, at the end of the profile. A similar approach was undertaken for the Republic of Moldova, where the government does not exercise direct control over the Transnistrian region. In this case, two questionnaires were collected, and the profile presents data from the Transnistrian region of Moldova in a separate section.

Sources for population data

Population data presented in the country profiles and tables were retrieved from Eurostat's database and cover the year 2024,⁴⁴ unless otherwise specified. If 2024 data was unavailable for certain countries at the time this report was finalised, the latest year with available data was selected.

Data analysis process

Collaboration with WAVE delegates

Once WAVE received data from a country, the WAVE team reviewed the submission and compared it with the data

from previous Country Reports. In case of notable inconsistencies, the respondents were contacted to confirm the reliability of the information provided. Regular communication with them improved the quality of the information and the precision of the country profiles, which were corrected and approved by the respondents before publication.

Methodological approach to collect and assess data

This report presents data and information on WSS for 2024. Where data for 2024 were unavailable, older data have been provided (referenced in the footnotes).

To assess the provision of **national women's helplines**, the standards provided by Article 24 of the Istanbul Convention were applied. Specifically, a 24/7, free-of-charge national women's helpline is necessary to meet the Istanbul Convention standards. Multilingual support is also strongly recommended and, therefore, included in the relevant tables. While Article 24 recommends that national helplines should provide advice in relation to all forms of violence, this report considers helplines offering support mainly or only in relation to domestic violence as meeting the standards for the provision of national women's helplines. WAVE's methodology aims at recognising helplines across Europe that provide a good and gender-competent service to survivors, even if limited to domestic violence, to welcome current promising practices, and eventually encourage states to ensure adequate helpline provision for survivors of all forms of violence.

When assessing the capacity of **women's shelters** to accommodate women and their children, the standards provided by Article 23 of the Istanbul Convention were applied. For this methodology, one family shelter place is considered to be equivalent to one shelter bed space. When referring to family places, the Istanbul Convention and its Explanatory Report set a minimum requirement of one family place (covering two beds, one for a woman and one for a child) per 10,000 population.⁴⁵ This standard is a valuable tool for assessing the availability of beds in shelters accessible to women and for comparing data across countries, and it is applied in the related chapter and in the tables. Nevertheless, as the specific needs of each country may vary, as acknowledged in the Istanbul Convention Explanatory Report,⁴⁶ compliance with this standard is not included in the country profiles; rather, it is used to assess its availability at the European level.

The Country Report distinguishes between women-only shelters and shelters accessible to all survivors, including

44 Eurostat, Population on 1 January by age and sex, [https://ec.europa.eu/eurostat/databrowser/product/page/demo_pjan\\$defaultview](https://ec.europa.eu/eurostat/databrowser/product/page/demo_pjan$defaultview), data extracted on 22/07/2025.

45 Council of Europe. (2011) *Explanatory Report to the Council of Europe Convention on preventing and combating violence against women and domestic violence*, p. 80-81.

46 Council of Europe, *Explanatory Report to the Council of Europe Convention on preventing and combating violence against women and domestic violence* (2011), art. 23 par. 135.

women. WAVE considers it essential to underscore the importance of shelters by and for women that apply a feminist, gender-specific approach and to monitor the availability of gender-specific services, as women's NGOs providing specialist support are increasingly losing funding or contracts to gender-neutral or generic providers.⁴⁷ While the overall availability and gaps in beds availability at the European level include all shelters accessible to women, the quality and key aspects of shelters were analysed separately in the country profiles and shelters chapter. A separate column in the country profiles' table is included to record the number of women-only shelters in each country. When presenting information about other shelters that support not only women, the groups of survivors supported by these shelters are specified whenever respondents provide this information. These might include shelters supporting women, men, non-binary people, transgender people, or other specific groups of survivors.

Similarly, the availability of an adequate number of beds in a specific country does not necessarily reflect the accessibility of shelters for different groups of survivors. Therefore, respondents were asked if the shelters in their country are accessible to the following groups of survivors: women with uncertain residence status, undocumented women, refugee women, homeless women, women with physical, cognitive or sensory disabilities, women with substance abuse issues, older women, transgender women, women with older sons, women with children with disabilities and unaccompanied girls under the age of 18. Additionally, it was assessed if there are any shelters specialised in supporting particular groups, e.g., migrant/undocumented women, women with disabilities, Black and minority ethnic women, LGBTQI+ people, older women, or women survivors of specific forms of violence such as trafficking, 'honour-based' violence, forced marriage and female genital mutilation (FGM). This information, wherever available, is included in the country profiles in order to provide a clearer picture of the accessibility of shelter services in Europe.

The definition of **women's centres** includes all women's services providing non-residential specialist support to survivors and can encompass a broad range of women's centres. Many women's centres focus on counselling; however, WAVE recognises that not all women's centres necessarily provide these services, e.g., some focus primarily or solely on prevention work and advocacy. The relevant tables of the Report include the proportion of the female population per centre, and highlight that in most cases, there are not enough centres to comply with the Council of Europe recommendation of one counselling centre for VAW survivors for every 50,000 women. Secondly, the tables relating to women's centres indicate who the beneficiaries of such centres are, as in some cases they are women-only, in other cases they support all survivors of GBV; finally, the tables include a column to show whether centres are run by women's NGOs, other NGOs,

the State, or other providers. This information is relevant because it indicates the general approach to service provision in each country. WAVE recognises that women's NGOs mostly have a gendered understanding and analysis of VAW, and that their service provision is based on a feminist, gender-sensitive approach that aims to empower women.

As in the shelters section, additional accessibility information was gathered regarding women's centres. Whenever available, the country profiles provide information on specialist support for particular groups of survivors and women survivors at risk of specific forms of violence. Article 25 of the Istanbul Convention makes specific reference to the provision of support for survivors of sexualised violence. The requirements are that states provide for the establishment of "*appropriate, easily accessible rape crisis or sexual violence referral centres*" for survivors, with the recommendation being one centre available for every 200,000 inhabitants. Therefore, data on the availability of women's centres and **support services for survivors of sexualised violence** are presented in different sections of the Country Report. Although sexualised violence can also be a form of domestic violence, specialised services for survivors of sexualised violence should be analysed separately from domestic violence services. Importantly, sexualised violence can be committed irrespective of the relationship between the perpetrator and the survivor, in any setting, including but not limited to home and work. Another key feature of specialised sexualised violence services is holistic support for survivors of child sexual abuse and its long-term psychological and physical impact.

Some women's centres provide support for women survivors of sexualised violence. However, this support is not always specialised and may therefore result in repeated referrals, as many survivors have unique needs, such as immediate forensic testing, complex trauma support and medical services, which general women's centres supporting all survivors of VAW cannot necessarily provide. Services responding to survivors' needs should ensure specific sexualised violence training and specialisation of service provision.

It is worth noting that different European countries have varied approaches to the provision of sexualised violence services. In some countries, women's centres are offering specialised holistic support for all forms of VAW, including sexualised violence. The aim is to avoid multiple referrals and re-traumatisation, as well as ensure adequate and feminist support to survivors. Other countries have consolidated networks of rape crisis centres (RCCs), with trained staff offering immediate and long-term confidential survivor-led counselling and therapy and providing various services such as advocacy and prevention. To reflect these national differences, the questionnaire completed by WAVE delegates included the same multiple-choice

⁴⁷ WAVE Network (2020) WAVE Handbook 2020: *How Gender Neutral Policy and Practice is Dismantling Women's Specialist Support Services and Ways to Counteract It*, p. 25. Currently available only for WAVE members.

question in both sections on women's centres and sexualised violence support services, listing the types of support provided to survivors of sexualised violence. In the section about support services for survivors of sexualised violence, the questionnaire collected data about RCCs, sexual violence referral centres (SVRCs), crisis and medical services, and specialised sexualised violence helplines. The availability of such services is not summarised in tables in this Report, to avoid comparison between countries that apply different approaches.

Questions about **primary prevention** were included in the questionnaire for the second time in this edition of the WAVE Country Report. The information gathered in the previous data collection was evaluated, and the questionnaire was revised to better reflect the availability of primary prevention policies and activities. It is key to distinguish between primary, secondary, and tertiary prevention, especially as states tend to conflate the three when presenting their prevention efforts and reporting on violence prevention. Based on the approach promoted by the World Health Organisation in their 2002 World Report on Violence and Health, the chapter focuses on primary prevention as the most strategic and transformative approach to ending violence against women and girls (VAWG) and achieving lasting gender equality.

For the first time, the questionnaire included two open questions about **technology-facilitated violence against women and girls**. One question inquired into the rise in such forms of violence, while the second assessed what strategies have been adopted by WSS to tackle such violence. This assessment is a starting point for the WAVE Network to gather a clearer picture of the needs and challenges across Europe and to guide future work to build the capacity of WSS to prevent and counter TFVAWG.

The questionnaire section on **femicide** was also reviewed to assess the status of data collection in each country and the state initiatives to prevent femicide, for instance, through systematic analysis of femicide (e.g., femicide reviews). An additional question was included in 2025 to assess whether data is collected about women who committed suicides that appear to be related to GBV. The results are presented in a dedicated chapter that sheds light on the lack of harmonised definitions and data collection. At the end of the chapter, a table summarises some key findings: the number of reported femicide victims in each country, and additional details related to data collection.

Limitations of collected data

While all country delegates made efforts to ensure the data and information provided through the questionnaire were accurate, the figures presented should be interpreted with caution. In some cases, respondents were unable to obtain all relevant information and data. This was either due to limited capacity or because government agencies

either do not collect this data or do not make it available to the public; therefore, they could not present the complete picture of specialist services available in their country or detailed qualitative information in the different questionnaire sections. Due to this limitation, only limited comparisons with previous Country Reports are included in the analysis; instead, specific examples are used to showcase improvements or declines.

One limitation of the questionnaire concerns respondents' interpretations of the definitions, the types of services they reported, and how they classified them. Moreover, not all countries have the same systems in place to support survivors of VAW. While some models have been successfully implemented in certain countries, they may not be effective in other contexts, making the comparability of country situations difficult. This is the case with support services for survivors of sexualised violence, as mentioned in the previous section. To mitigate such differences, the respondents were provided with definitions embedded in each questionnaire section, and the WAVE Team held regular open online sessions and ad hoc meetings to clarify any doubts about the questionnaire.

As mentioned above, although the report analyses 46 countries, five of them provided multiple questionnaires. This poses a challenge in data analysis and makes it more difficult to provide an accurate national overview. When presenting the data at the European level, information from different regions of the same country was combined, and internal differences were explained when relevant.

The tables in the report aim to summarise the results of the data collection, facilitate comparisons across countries, and provide an overview of the situation at the European level. When consulting the tables, footnotes should be considered, as the lack of data in some countries does not always allow for consistent comparisons across years or between data from different editions of the Country Report.

As emerged in previous reports, there are serious gaps in the availability and comparability of data concerning the recorded number of femicide victims in the 46 countries surveyed, as there is no Europe-wide data collection system or agreed use of the term femicide. These figures should be treated with caution. Data on the number of femicide victims were not available in nine of the countries surveyed, and the data available in the surveyed countries are not statistically robust and should be regarded as estimates based on the best available sources. The same applies to the number of survivors supported by WSS in 2024: in many cases, these numbers are estimates, while in others, no data were available. Therefore, the real number of survivors supported is likely much higher.

Why women's specialist services matter

The foundation of survivor support

Women's Specialist Services (WSS) are the cornerstone of Europe's response to violence against women and girls (VAWG). These services provide the first critical entry point for survivors seeking safety, support, and a path to reclaiming their lives. Unlike generic victim support services, WSS operate from a distinct rights-based, intersectional feminist approach. They are women-led, run by women for women, and fundamentally grounded in the principle of survivor agency, meaning women themselves determine the pace and direction of their recovery journey.

WSS adopt a gender-transformative methodology that distinguishes them from all other support structures. Their mission extends beyond individual support to address the societal systems that enable VAWG to persist. This dual mandate encompasses both prevention and response. Through primary prevention initiatives, WSS challenge gender stereotypes that devalue women and girls, promote bystander intervention when violence is witnessed, and deliver comprehensive sexuality education.

Through secondary prevention, the frontline response after violence occurs, WSS provide immediate, specialised support via helplines, shelters, rape crisis centres (RCCs), and women's centres. These services create secure environments where all women can access compassionate, expert support and develop strategies to prevent and recover from violence, free from judgment or reprisal. This is why WSS are not only essential for crisis response but are also leaders in the primary prevention work this report identifies as critical to ending VAWG.

The expertise and approach that WSS bring cannot be replicated by generic services. Research consistently demonstrates that women experiencing sexual violence often withhold disclosure unless they perceive service providers as specialists who are open, knowledgeable, and trustworthy. Generic providers frequently lack this expertise, creating risks of disbelief, minimisation, or mishandling of disclosures that can cause women to withdraw from services entirely, leaving them unsupported and more vulnerable. WSS are trauma-informed, independent from state institutions, and possess a deep understanding of gendered violence dynamics that enables them to provide consistent, high-quality care tailored to each woman's specific circumstances and intersecting needs.

The threat of gender neutrality

Despite decades of evidence that gender-responsive specialist services are more effective than generic approaches in supporting survivors of VAWG, Europe faces a growing threat from gender-neutral policies. This shift has moved beyond far-right politics into mainstream discourse, making it more insidious.

Politically, gender neutrality serves to diminish the perceived impact of VAWG by claiming gender equality has been achieved. It falsely equates VAWG with violence against men, ignoring that approximately 90% of perpetrators of gender-based violence against women (GBVAW) are men, and women face disproportionate risks across every form of violence.

Gender neutrality also ignores that societal gender inequality persists across employment, leadership, pay equity, care responsibilities, political representation, and online violence. True progress requires equity, investing more in women's rights and dismantling structures that undervalue women. We cannot reach equality by treating everyone identically when starting conditions are fundamentally unequal.

Economically, gender-neutral approaches are presented as cost-saving measures in times of austerity, allowing governments to consolidate services and reduce spending. This logic treats WSS as expendable, positioning generic victim support as adequate. It is not. Evidence shows that this apparent cost-saving is a false economy that will ultimately cost Europe far more.

The economic case for investment

Far from being a luxury, investing in WSS makes economic sense. Violence against women and girls costs the EU € 289 billion annually.⁴⁸ Yet, economic modelling conducted in France suggests that each additional € 1 allocated to prevention could save up to € 87 in societal costs, including € 30 in direct expenditures.⁴⁹ Evidence from the United Kingdom shows that each £1 invested in WSS generates up to £ 11 in social value.⁵⁰ Similarly, an Italian study estimates a social return of € 7,15 for every euro invested in specialised services.⁵¹ Yet despite this proven return on investment, WSS across Europe face an unprecedented funding crisis. Women's organisations, particularly women-led and by-and-for organisations serving racialised,

48 European Institute for Gender Equality, *Costs of Gender-Based Violence*, accessed July 2025. https://eige.europa.eu/gender-based-violence/costs-of-gender-based-violence?language_content_entity=en

49 Nectoux, Marc & Mugnier, Claude & Baffert, Sandrine & Albagly, Maité & Thélot, Bertrand (2010). *Évaluation économique des violences conjugales en France*. Santé Publique. 22. 10.3917/spub.104.0405.

50 Women's Resource Centre (2011) *Hidden Value: Demonstrating the Extraordinary Impact of Women's Voluntary and Community Organisations*, London, <https://www.wrc.org.uk/Handlers/Download.ashx?IDMF=4222e0eb-c528-48c8-a56c-d10db4fd3fb0>

51 WeWorld (2017) *Violenza sulle Donne. Non c'è più tempo. Quanto vale investire in prevenzione e contrasto. Analisi SROI delle politiche d'intervento*. <https://www.weworld.it/cosa-facciamo/pubblicazioni/violenza-sulle-donne-non-ce-piu-tempo>

migrant, and underserved communities, are under growing financial pressure due to global and European funding cuts.

The reinstatement of the Global Gag Rule, USAID funding freeze, and US cuts to UN agencies have severely impacted gender equality and intersectional civil society organisations. Simultaneously, several European governments are diverting funds toward security and defence, further destabilising organisations that prevent and respond to VAWG.

This financial weakening is compounded by increased funding for anti-rights actors. Between 2019 and 2023, anti-gender organisations invested €339.6 million⁵² in lobbying to reverse progress on gender equality and women's rights, ultimately aiming to undermine EU values of fundamental rights, democratic principles, and the rule of law. If Europe is serious about upholding fundamental rights and preventing VAWG, it must provide adequate, sustainable, long-term funding for WSS. A significant budget increase for programmes like CERV+ and its DAPHNE strand, exclusively dedicated to preventing and combating VAWG, sends a strong political message: protecting EU values, fundamental rights, and democracy is vital. This requires that funding reach frontline organisations, human rights defenders, and WSS directly.

52 Neil Datta (2025) *The Next Wave: How Religious Extremism Is Regaining Power*, European Parliamentary Forum for Sexual and Reproductive Rights (EPF), <https://www.epfweb.org/node/1147>

National women's helplines



Introduction

National women's helplines are among the most vital and life-saving services for women and girls survivors of gender-based violence (GBV). They provide immediate access to confidential advice, emotional support, and information about rights and services. Women's helplines in Europe emerged from the feminist movements of the 1970s and 1980s, simultaneously with the creation of women's shelters and rape crisis centres (RCCs). In the United Kingdom, feminist activists established local women's refuges and volunteer-run telephone lines where women could seek safety and advice. By the mid-1980s, these grassroots initiatives had grown into the first national helpline structures.⁵³

Similar developments took place across Europe. In Austria, women's organisations that had founded shelters in the late 1970s went on to set up hotlines that connected women across the country to immediate support.⁵⁴ In Germany, feminist groups created local helplines which were gradually consolidated into wider networks.⁵⁵ In the Nordic countries, helplines were established as part of the broader infrastructure of gender equality and welfare policies, often linked to women's shelters or RCCs.

In the Balkan region (former Yugoslavia), the first SOS helpline was founded in 1988 in Zagreb, Croatia, followed by the first SOS hotlines in Ljubljana, Slovenia, and in Belgrade, Serbia.⁵⁶ Soon, women's helplines were opening across Europe. Decades before the Istanbul Convention was adopted and came into force, mandating State Parties that have ratified it to provide 24-hour hotlines for all forms of violence against women and girls (VAWG), women's specialist services (WSS), NGOs, and feminist grassroots organisations and initiatives had already begun establishing these services.

These early helplines in Europe were deeply rooted in women's self-organised activism. They reflected an understanding that women needed independent and safe spaces to speak freely about their experiences and to access practical, confidential, empathetic victim-centred support. Over time, many became professionalised and state-supported, but their feminist origins remain vitally important. They continue to remind us that helplines are not neutral specialised services, but part of a broader struggle for women's human rights. But more than this, they represent a recognition that VAWG is not random or isolated, but a structural problem rooted in gender inequality.

This gendered understanding is what makes a national women's helpline different from generic or gender-neutral services. Violence against women, whether physical, sexual, psychological, economic, or technology-facilitated, is not simply 'individual conflict' or 'family problems'. It is a manifestation of unequal power relations between women and men, a form of discrimination, and a violation of human rights. For this reason, survivors need support that acknowledges the reality of gender inequality, rather than treating violence as genderless or incidental.

By contrast, where governments invest instead in generic or gender-neutral helplines, survivors are often left without the expertise and sensitivity they require. Gender-neutral services risk overlooking patterns of coercive control and may present violence as symmetrical between women and men. This can lead to inappropriate advice, unsafe referrals, and secondary victimisation. Moreover, in some political contexts, the promotion of gender-neutral services reflects a backlash against gender equality and a refusal to acknowledge VAWG as a structural problem requiring feminist solutions.

A national women's helpline must therefore be specialist and comprehensive, covering all forms of violence against women (VAW). Women rarely experience violence in neatly separated categories, and the same survivor may be subjected to multiple forms of abuse, often simultaneously. A helpline that recognises this continuum of violence and responds from a gender-sensitive standpoint is better equipped to understand women's experiences and provide meaningful assistance.

Specialist helplines are also essential because survivors frequently face disbelief, minimisation, or victim-blaming in broader society. A national women's helpline, staffed by trained professionals or volunteers who believe survivors and understand the gendered dynamics of violence, offers the validation and solidarity that women need in order to seek support.

What makes national women's helplines unique

→ Accessibility and anonymity

For many women, calling a helpline is the very first step in seeking help. The relative anonymity of a phone call allows survivors to tell their story without necessarily taking steps to report and prosecute violence; it gives them an opportunity to see that they will be believed. It further shows them that information is available and that they

53 See Women's Aid (n.d., "History").

54 See Wiener Frauenhäuser (n.d., "24-Stunden Frauennotruf Wien") and WAVE (2018, "The situation of specialist women's support services in Austria").

55 See Hilfetelefon (n.d., "About the helpline") and Hilfetelefon (n.d., "Violence against women – facts and figures").

56 See Ženska grupa Trešnjevka (n.d., "SOS telephone for women and children victims of violence") and Miškovska Kajevska (2013) for early SOS helplines in the former Yugoslavia.



have options. Women who may not yet be ready to leave a violent relationship can explore safety planning, while others in acute crisis can be directed immediately to emergency protection and shelter.

→ Breaking isolation

Violence against women often isolates survivors, cutting them off from family, friends, and community. A free phone number, available 24 hours a day, can break that isolation and offer a bridge to information and support to women who otherwise have no one to turn to. This is especially important in rural or remote areas, where local specialised services may be scarce or entirely absent.

→ Inclusion of all women

The principle of non-discrimination, enshrined in the Istanbul Convention, requires that national helplines are accessible to all women, including women with disabilities, migrant and refugee women (including undocumented women), queer women, women from minority communities, older women, and young women. This is not merely about availability but also about competence to provide specialised help through multilingual support and to respond sensitively to intersecting forms of oppression and discrimination.

→ Referral and coordination

A national women's helpline is also an entrance to the broader network of women's specialist support services. Calls are often referred to local shelters, counselling centres, legal aid providers, or RCCs. In this way, helplines form part of an integrated chain of protection and support, ensuring that women are not left alone at any point in their journey.

→ The role of women's NGOs

For decades, women's NGOs have been at the forefront of establishing, running, and sustaining national helplines. Their expertise is rooted in direct work with survivors, feminist research, and international networking. They have developed standards of practice that reflect survivors' real needs, and they have consistently advocated for states to recognise helplines as part of basic services.

Helplines run by women's NGOs differ fundamentally from generic services: they are survivor-centred, rights-based, and aware of the gendered structures underlying violence. This expertise cannot be replicated by services that treat violence as gender-neutral or that fail to understand its structural causes.

International obligations

The Istanbul Convention (Article 24)⁵⁷ reflects this principle by requiring state-wide, 24/7, free helplines that address all forms of VAW. It emphasises that such services are not optional, but a legal obligation.

The Council of Europe Istanbul Convention sets out the minimum standards for national women's helplines:

- State-wide coverage
- 24/7 availability
- Free of charge
- Confidential and anonymous
- Addressing all forms of VAW
- Providing services in relevant languages

These standards are echoed and reinforced by the EU Directive on Combating Violence against Women and Domestic Violence,⁵⁸ which requires that helplines are free, universally accessible, and inclusive of women with disabilities. The Directive specifies that multiple communication formats must be available, including national sign language and simplified language.

Both instruments also require adequate and sustainable state funding (Istanbul Convention Article 8, EU Directive Article 36). Without long-term investment, helplines cannot reliably function 24/7, train and retain staff, or expand accessibility. Funding through short-term projects or competitive tenders, while common, undermines the continuity and safety of services.

WAVE framework for data collection on national women's helplines

The data presented in this chapter are based on WAVE's 2025 data collection tool, which included a dedicated section on national women's helplines. Respondents were asked to indicate whether a state-wide women's helpline exists in their country, and to provide details about its operation, based on data from 2024. Specifically, questions covered:

- The number of national women's helplines and their names
- Which organisations run the helpline(s)
- Whether the helpline(s) is/are free of charge and available 24/7
- Whether multilingual support is provided, and if so, which languages are covered
- Which forms of VAW are addressed
- Main sources of funding (state, donations, international funding, or other)
- Call volumes in 2024, including total contacts and total individual callers, when available
- The three most common forms of violence reported
- Other helplines in the country (regional or specialised), including details on their scope and operation

This framework ensures consistent and comparable data collection across countries, and highlights both progress and remaining gaps in the provision of women's helplines.

⁵⁷ See Council of Europe, Istanbul Convention, Article 24.

⁵⁸ See European Union, Directive (EU) 2024/1385, Article 27.

Findings on the situation of national women's helplines in Europe

	Europe (46)	EU Member States (27)	European countries outside the EU (19)
Total number of countries with NWH	39	23	16
Free of charge	39	23	16
Operating 24/7	37	21	16
NWH free of charge and 24/7 (meeting the Istanbul Convention standard)	37	21	16

Table 7: Overview of service provision – national women's helplines in Europe

Countries with at least one state-wide helpline

By 2024, 39 of 46 European countries (85%) report having at least one national women's helpline supporting survivors of VAWG. The remaining 15% (seven countries) have some form of helpline provision at the national and/or regional level.

Seven countries⁵⁹ (15%) across Europe remain without a free, 24/7 national women's helpline. In some cases, no statewide service exists at all, such as in **Armenia** and **Belarus**. In others, provision is fragmented or gender-neutral, as in **Belgium**, where separate helplines operate for Flemish- and French-speaking communities, none exclusively for women, and in **Latvia**, where helpline services exist, but those that are 24/7 are not specifically for survivors of VAW, and the one that is specialised does not operate 24/7.

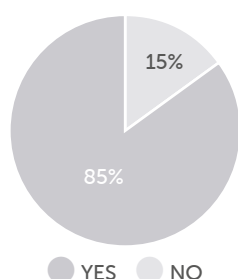


Figure 2: Percentage of European countries with at least one national women's helpline

The progress since 2023 demonstrates the continuing recognition of women's helplines as a cornerstone of state

responsibility under the Istanbul Convention. Yet the absence of proper provision in seven countries still leaves women without a guaranteed right to immediate, specialist, and accessible support.

Over half of European states continue to operate a single national women's helpline, which is the model recommended by the Istanbul Convention. In 2024, 29 countries fall into this category. For women, this means one well-known number that can be called from anywhere in the country at any time.

A smaller group of countries reports two national helplines, such as in **France**, where the Violence Femmes Info line coexists with specialist lines on forced marriage and workplace harassment. In total, seven countries reported having two helplines.

In **Türkiye**, there are three main helplines providing support in cases of violence. The first is the national helpline, operated and financed by the Ministry of Family and Social Services. The hotline is free of charge and available 24/7, providing support in Turkish, Arabic, and Kurdish, open to various groups, but not specialised in supporting women survivors of violence. The two other helplines are the Emergency Hotline for Domestic Violence, run by the Federation of Women Associations of Türkiye, providing support to survivors of all forms of VAW, especially domestic violence, and Woman Support Line, run and funded by the Istanbul Metropolitan Municipality, providing support in Turkish, Kurdish, English and Arabic for all forms of VAW.

Spain has a national helpline and multiple regional helplines. The national helpline is called Service of attention to all forms of VAW, run by the Ministry of Equality, free of charge and available 24/7, offering multilingual support in Spanish, Catalan, Basque, Galician, English, French, German, Portuguese, Mandarin Chinese, Russian, Arabic, Romanian, Bulgarian, Tamazight and 39 other languages through a tele-translation service. It provides help for all forms of VAW and is funded by the state.

The data collected shows a high number of regional helplines in **Serbia**, with a total of nine licensed helplines listed. There is one national helpline and eight regional ones. This suggests that women are far more likely to rely on local or regional helplines run by women's NGOs and shelters. For survivors, the reality is that multiple services may be essential to fill gaps left by the state.

In summary:

- 29 countries operate one helpline
- Seven countries operate two helplines
- Two countries operate three helplines
- The **United Kingdom** has four helplines, one for each of its nations

⁵⁹ Armenia, Belarus, Belgium, Latvia, Malta, the Netherlands, and Switzerland.



National women's helplines meeting Istanbul Convention standards

The Istanbul Convention calls for national women's helplines to be free of charge, available 24 hours a day, and operating from a gendered understanding of VAW. As of 2024, 37 countries across Europe meet these standards, representing a significant increase from the 32 countries reported in the Country Report 2023.

In the **United Kingdom**, each of the four nations (**England, Scotland, Wales, and Northern Ireland**) continues to operate its own fully compliant national helpline.

CASE STUDY: United Kingdom

Across the **United Kingdom**, all four nations, **England, Wales, Scotland, and Northern Ireland**, operate national 24/7 domestic violence or VAW helplines, and each also has a separate, specialised sexual violence helpline or support line. This structure is relatively unique, as it ensures that in a comparatively small geographical area, all jurisdictions maintain two fully functional, distinct helplines addressing different forms of VAWG.

England: The National Domestic Abuse Helpline (Refuge) offers 24/7, free, multilingual support to women affected by domestic violence. In addition, the Rape and Sexual Abuse Support Line (Rape Crisis England & Wales) provides nationwide 24/7 support to anyone aged 16+ affected by sexual violence.

Wales: The Live Fear Free Helpline (Welsh Women's Aid) offers 24/7, free, bilingual (Welsh/English) and multilingual support on all forms of VAW. Survivors of sexual violence in Wales can also access the Rape and Sexual Abuse Support Line run by Rape Crisis England & Wales.

Scotland: Scotland's Domestic Abuse and Forced Marriage Helpline (Scottish Women's Aid) operates 24/7 and provides multilingual support. A separate Rape Crisis Scotland Helpline offers daily evening support to survivors of sexual violence.

Northern Ireland: The Domestic and Sexual Abuse Helpline (Nexus NI) is 24/7, free, and multilingual, offering support to survivors of all genders. In addition, the Rowan Sexual Assault Referral Centre operates a dedicated sexual violence helpline alongside forensic and medical services for survivors across Northern Ireland.

This dual-helpline model, with specialised services for both domestic/VAW and sexual violence, is a distinguished feature of the **United Kingdom** context, demonstrating strong institutional recognition of the need for specialised, parallel response systems within each nation.

Most compliant helplines also ensure accessibility for diverse groups of women by providing services in multiple languages. In some countries, this is delivered by multilingual staff, in others, through structured interpretation services. Increasingly, helplines are also expanding accessibility by offering interpretation for deaf women and those with hearing difficulties, reflecting a commitment to inclusion.

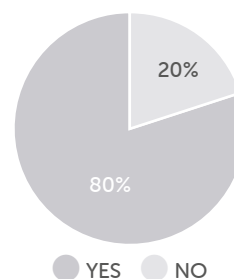


Figure 3: Percentage of European countries meeting the Istanbul Convention standards for the provision of a national women's helpline

Helplines that are not yet compliant with the Istanbul Convention

While the overall picture shows evident progress, nine countries still lack a national women's helpline that fully meets the requirements of the Istanbul Convention.

In the EU, seven states fall short due to limitations in their provision. In **Belgium**, there is no national women's helpline due to structural and linguistic differences within the country. Instead, there are distinct helplines for each of the two major linguistic communities (Brussels-Wallonia and Flanders). In **Latvia**, there is no national helpline, but there are three other helplines supporting survivors of VAW. There is no specialised national women's helpline in **Malta**, but two generic helplines that can provide support also to women survivors of GBV. There is no national women's helpline in the **Netherlands**, but there is a national helpline for domestic violence and child abuse, Safe Home, available 24/7 and free of charge, providing support to all survivors of violence as well as for professionals who want to report cases of child abuse and/or domestic violence.

Outside the EU, three additional countries remain non-compliant. In **Armenia**, 12 different helplines exist, most of them local or specialised, but no state-wide service is available, although there is a process underway to establish a national helpline. The Ministry of Social Affairs runs a national helpline which is not specialised but can refer domestic violence cases to regional helplines, according to the location of the survivor. In **Switzerland**, discussions continue on establishing a national service, but provision remains limited to cantonal or regional levels, with the introduction of a national 24-hour helpline planned for May 2026. The only national women's helpline in **Belarus** was the National Hotline for victims of family violence, which

was run by the international public association 'Gender Perspectives' from 2012 to 2021. As a result of state repressions against Belarusian civil society, 'Gender Perspectives' was compulsorily liquidated, and the helpline was indefinitely suspended in July 2021 when the office was raided by law enforcement. This situation has created significant challenges and limitations in accessing information on WSS in Belarus.

Taken together, these examples highlight the different ways in which gaps appear. In some contexts, helplines are under-resourced or restricted in scope, preventing them from offering the full level of accessibility required. In others, the absence of a state-wide women's helpline reflects deeper structural or political barriers, including hostility to feminist organisations or the persistence of gender-neutral approaches that don't take into account women's specific experiences of violence.

Multilingual support

The Istanbul Convention emphasises that national women's helplines should be accessible to all women, regardless of their language. This is particularly important for migrant and refugee women, many of whom face additional obstacles in seeking protection and support.

In 2024, 29 countries report that their national women's helpline offers multilingual support. This represents the overwhelming majority of European states with helpline provision, showing that language accessibility has become a standard expectation. By contrast, five countries reported that their helplines are not multilingual, and in the remaining countries, information was not available. In **North Macedonia**, the national helpline offers support in Macedonian, Albanian, Roma and Turkish, although multilingual support is not available 24/7, but only when an operator who speaks one of these languages is on duty.

There are considerable examples of good practice. In **Spain**, the 016 helpline provides support in more than 50 languages through professional interpretation services, making it one of the most comprehensive models in Europe. The national helpline in **Germany** offers counselling in 18 languages, while in **France**, interpretation is available in more than 200 languages through a partnership for interpretation. In smaller states, multilingual services are often less available. In **Slovenia**, support is primarily available in Slovenian, English and neighbouring languages (Croatian and Bosnian), while in **Denmark**, two national women's helplines provide multilingual support. One of them is the state-funded Live Without Violence's National Hotline, providing multilingual support in any language provided by the interpretation service. The other one is Danner's National Crisis Hotline, which operates in Danish and English.

Several countries also report efforts to make helplines more accessible for women with disabilities. In **France**, for

instance, the national helpline has developed video relay services and adapted online chat functions for deaf and hard-of-hearing women.

CASE STUDY: Multilingual support in Germany's national women's helpline

The German national women's helpline, the Violence against Women Helpline (*Hilfetelefon 'Gewalt gegen Frauen'*), demonstrates how multilingual provision can be embedded into a national service. Operating 24/7 and free of charge, the helpline offers support in 18 languages through professional interpretation. This system enables migrant and refugee women to access specialist advice and support without language barriers, which is a crucial factor in ensuring equal access to protection.

In addition to spoken languages, the helpline has developed specific accessibility measures, including sign language support, Easy language and online chat, so that women with hearing impairments or communication difficulties can also reach the service. Germany's model illustrates how national women's helplines can extend their reach by integrating multilingual and inclusive communication as primary elements of service provision.

At the same time, the absence of multilingual provision in some countries leaves significant gaps. Women who do not speak the majority language may struggle to access vital information and support or may be unable to call the helplines entirely. This disproportionately affects migrant, refugee and minoritised women, who are often among the most vulnerable to violence.

Overall, the 2024 data confirms that multilingual accessibility is becoming the norm across Europe, but the quality and scope vary greatly. The strongest models are those that combine broad language provision with interpretation for women with disabilities, ensuring that all women can reach specialist feminist support when they need it.

Entities running national women's helplines

The majority of national women's helplines across Europe are run by women's NGOs. This reflects the long history of feminist organisations in creating and sustaining these services, often decades before states assumed any responsibility. Examples include **Austria**, where the national helpline is operated by the Association of Autonomous Austrian Women's Shelters, **Croatia**, where the Autonomous Women's House Zagreb runs the line, and the **Czech Republic**, where the feminist organisation ROSA is now operating the European harmonised number for helplines for victims of VAW. In **Hungary**, the helpline is operated by NANE, while in **Ireland**, the national service is run by



Women's Aid Ireland. These services are firmly rooted in a gendered understanding of VAW, which is central to their effectiveness.

In a smaller but still significant number of countries, national women's helplines are operated directly by government ministries or state agencies. For example, in **Greece**, by the General Secretariat for Gender Equality; in **Germany**, by the Federal Office for Family and Civil Society Tasks; and in **Georgia**, by the Ministry of Internal Affairs. In **Türkiye**, three parallel helplines operate, one of which, ALO 183, is managed by the Ministry of Family and Social Services.

There are also mixed models, where responsibility is shared between state institutions and NGOs, or where contracts have shifted between providers. In **Finland**, the helpline is run by the Institute of Health and Welfare in partnership with NGOs.

Despite the differences in who runs them, the crucial factor is whether helplines operate from a gendered perspective and ensure feminist, survivor-centred support. Where women's NGOs remain at the centre of service provision, helplines are most clearly grounded in this expertise.

In some countries, more than one national women's helpline operates in parallel. While the state may fund or manage the official line, additional services are frequently run by women's NGOs. These often have a more specialised focus, reflecting the diverse needs of survivors. For example, in **Bosnia & Herzegovina**, *Medica Zenica* operates a state-wide helpline for survivors of war-related sexual violence. In **Denmark**, the feminist organisation *Danner* runs a national service, operated by volunteers, alongside the general helpline. In **Finland**, the Women's Line (*Naisten Linja*) provides a feminist helpline complementing the state's service through the Institute for Health and Welfare. In **Albania**, the NGO *Psycho-Social Center Vatra* operates an additional helpline, while in **Croatia**, *Women's Help Now* provides another women's helpline for survivors of VAW.

Funding for national women's helplines

The Istanbul Convention and the EU Directive on VAW and domestic violence both make clear that governments have the responsibility to ensure adequate and sustainable funding for women's specialist support services, including national helplines and that they should not have to rely on insecure resources such as donations or short-term international projects.

In 2024, the majority of national women's helplines across Europe reported receiving at least some state funding. This represents substantial progress in recognising helplines as part of the state's core duty to protect women's rights. However, five countries still lack state funding for their national helplines: **Bosnia & Herzegovina, Hungary, Poland, Russia, and Slovakia**. Additionally, in Serbia, two of the nine helplines receive state funding, while the others

rely on donations and international financing. In these contexts, women's NGOs continue to provide services with limited or no public support, often at significant cost to their sustainability.

Alongside state support, 17 countries report that their national helplines depend at least partly on donations from foundations, trusts, or individuals. While donations can indeed supplement resources, relying on them to cover core costs undermines the principle that specialist services are a state obligation.

International funding continues to play a crucial role. Fourteen countries report dependence on international donors and projects, including **Albania, Bosnia & Herzegovina, Croatia, Hungary, Georgia, Moldova, Poland, Portugal, Romania, Russia, Serbia, Slovakia, Türkiye, and Ukraine**. In these contexts, helplines are often sustained by EU projects or international humanitarian aid, rather than secure state budgets.

In **Russia**, the highly complex and threatening environment severely limits communication among women's NGOs. It is often impossible to verify how organisations operate or what strategies they employ. Information on funding sources is particularly sensitive for security reasons and therefore cannot be publicly disclosed. In the case of the POLINA Centre, while efforts are made to diversify funding, the organisation's main sources of support for many years have been international donors and private contributions.

In **Bosnia & Herzegovina**, NGOs in the Federation continue to operate SOS lines on a volunteer basis with no state financing, despite new legislation recognising women's right to helpline support. In **Hungary**, both NANE and PAT-ENT depend on international grants and private donations, with much of the work carried out by volunteers. In the **Netherlands**, the government funds a generic, gender-neutral helpline (Safe Home), while feminist NGOs continue to campaign for an independent women's helpline that meets Istanbul Convention standards. In **Serbia**, only two out of nine national helplines receive state support, while the rest rely on international projects and donations. In **Switzerland**, no state-wide helpline exists, and provision is left to individual cantons, where services are volunteer-based. These examples demonstrate that while state funding has increased since 2023, it is still not adequate, secure and sustainable.

Information on the number of calls

Across Europe, national women's helplines received hundreds of thousands of calls in 2024; 32 countries were able to provide either exact numbers or estimates of the number of calls, and helplines in these countries received over 900,000 calls. The highest numbers were reported in larger states such as **Spain**, where the national helpline received nearly 59,000 calls, and **France**, where the state line recorded over 152,000 calls. Smaller states also show

significant demand: in **Austria**, 5,425 calls were recorded, while in **Finland**, the *Nollalinja* registered over 15,000 calls. In **Hungary**, the combined number of calls for the two helplines run by NANE and PATENT is over 25,000. A total of 48,369 calls were answered by Sweden's *Kvinnofridslinjen*'s staff in 2024.

The number of calls to the helplines is also substantial in non-EU states. In **England**, the national helpline run by Women's Aid reported over 135,000 callers. In **Armenia**, while there is no single state-wide helpline, twelve regional helplines together handled 3,877 calls, with most linked to domestic violence. In **Albania**, the national helpline for women and girls received 4,658 calls, a 12% increase compared to the previous year, indicating a growing trust in the support system available to survivors of VAWG. **Montenegro** received 2,114, and **North Macedonia** 760 calls.

It is important to note that statistics are not directly comparable across states. Some helplines count total contacts, including repeat calls, while others record only the number of individual callers. Some also provide support through chat. The extent of public awareness of the helpline, its visibility in national campaigns, and levels of state support also shape the number of calls received. Nevertheless, the overall picture confirms that women's helplines are among the most widely accessed specialist support services in Europe.

Most common forms of violence reported

When asked about the three most common forms of violence reported, the majority of helplines identified domestic violence as the most common. Where domestic violence was not reported as the most common was in the countries that did not have general data on the calls. Information was also collected for different types of violence that occur within domestic violence. In about two-thirds of the countries, psychological and physical violence were also reported among the three most common, while sexualised violence was included in the three most common in 11 countries. Economic violence was among the most prevalent in seven countries, while stalking was mentioned only in the **Czech Republic**. Violence directly or indirectly affecting children was reported as one of the most common forms of violence in the **Czech Republic**, **Azerbaijan**, **Estonia**, **the Netherlands** and **Cyprus**.

Other frequently reported issues included child abuse and child protection concerns, often linked to family violence, followed by cases of sexual abuse and assault, as well as reports of human trafficking and exploitation. In **Moldova**, the most commonly reported form of violence was physical, often occurring in combination with other types, accounting for 65% of all cases. This is followed by psychological violence (22%), economic violence (7%), and sexual violence (4%). These figures highlight that many survivors face multiple, overlapping forms of abuse, with physical violence being the most prevalent among those who sought help.

It is worth noting that domestic violence means all acts of physical, sexual, coercive, psychological, or economic violence and that different forms of VAWG intersect. For example, in **Hungary**, most callers reach out to NANE due to domestic violence, which typically includes a combination of verbal, psychological, economic, and physical abuse and in many cases, sexual violence as well. A significant proportion of these situations also involves children. Most women contacting the PATENT helpline seek legal guidance on divorce proceedings resulting from domestic violence and matters of child custody.

Other national helplines in Europe

In many countries, additional helplines exist together with or instead of the official national line. These may be regional, local, or dedicated to particular forms of VAWG or GBV. They play a vital role in reaching survivors who might not otherwise access support.

In **England**, for example, 181 local domestic abuse services run helplines. They are mostly for local residents and callers to receive support over the phone, sometimes before accessing other support services. Other national helpline services for other forms of VAWG include Modern Slavery Helpline, Revenge Porn Helpline, and Honour-Based Abuse Helpline. Similarly, in **Croatia**, around 20 separate helplines exist, many operated by women's organisations that specialise in areas such as sexual violence, human trafficking, and domestic violence. In **Estonia**, there are 16 regional helplines run by women's support centres and operating 24/7. In addition, the organisation Lifeline runs a specific helpline for survivors of human trafficking and sexual exploitation, available during limited hours.

Specialised helplines also respond to specific forms of violence. In **Austria**, women have access to helplines focused on sexual violence, while in **Greece**, there is a helpline for supporting survivors of trafficking in persons, offering multilingual support in over 200 languages through tele-interpretation, and available 24/7. In **Bosnia & Herzegovina (Federation)**, the organisation *Medica Zenica* runs a unique helpline for survivors of war-related sexual violence and their families, which is free of charge and available 24/7. In **France**, there is the Family Planning hotline, a reference centre for issues related to contraception, abortion, sexually transmitted infections, violence and sexuality, as well as a helpline for advocacy against discrimination towards women with disabilities.

In some countries, there are parallel helplines that reflect different institutional approaches. In **Denmark**, several helplines operate in addition to the national one, including targeted support for children and youth exposed to domestic or dating violence, medical and psychological support after rape/assault, therapeutic support for families affected by violence, general support for victims of violent crime, specialised support for stalking and others.



In some European countries, the only state-wide helpline available is a gender-neutral service. These helplines respond to all survivors of domestic violence or child abuse, and sometimes to broader issues of violence or social welfare. While they provide an important point of contact, they do not meet the Istanbul Convention's requirement for specialist helplines for survivors of all forms of VAWG.

One example is the **Netherlands**, where the government funds Safe Home (*Veilig Thuis*), a 24/7 service covering domestic violence and child abuse. Although it is widely accessible and free of charge, the helpline operates from a gender-neutral framework. Women's NGOs in the country continue to campaign for an independent women's helpline, grounded in a gendered understanding of violence against women.

Conclusion

The latest findings on national women's helplines in Europe show both important progress and persistent gaps. Thirty-nine out of 46 countries now have at least one national women's helpline, and 37 of these meet the minimum standards set out in the Istanbul Convention by being free of charge, available 24/7, and operating from a gendered understanding of VAW.

Helplines continue to play a crucial role as an entry point for women into protection and support. They provide confidential, immediate assistance, often serving as the first contact for women survivors of VAWG. They are also the key point of contact, referring women survivors to other essential specialised services and institutions. They are also accessible to women who might otherwise be isolated, including those in rural or remote areas. The increase in provision of multilingual services further strengthens accessibility for diverse groups of women, including migrants and refugees, and highlights the principle of non-discrimination in service provision.

Despite this progress, significant challenges remain. Some countries still lack a fully compliant national women's helpline, and in others, provision is limited to only certain forms of violence, such as domestic violence, or they are delivered through gender-neutral services. Funding also continues to be a concern, with many helplines reliant on short-term projects, donations, or international grants rather than secure and sustainable state support. This instability undermines both the continuity of services and states' obligations under international standards.

Across Europe, women's helplines receive nearly a million calls each year, most commonly concerning domestic, physical, and psychological violence, but also sexualised, economic, and technology-facilitated violence. Each call is a vital step in breaking isolation and seeking safety, confirming the indispensable role of helplines in comprehensive systems of protection.

Recommendations

- Each European country must establish at least one national women's helpline providing specialist, gender-specific support on all forms of VAWG, operating 24/7, free of charge, and grounded in feminist, survivor-centred principles.
- States must guarantee adequate, secure, and long-term public funding for national women's helplines, avoiding reliance on volunteers or private donations. Public funding should prioritise women's NGOs with demonstrated expertise, while allowing them to access complementary international or philanthropic resources without reducing state obligations.
- National women's helplines should be equipped to provide referrals to other relevant services, including WSS. Governments must strengthen co-operation frameworks to ensure smooth referral pathways and coordinated support across sectors.
- States should develop standardised data collection and reporting practices for national women's helplines, in consultation with WSS, in order to better understand the scope of VAWG, track helpline effectiveness, and improve service provision.
- National women's helplines must ensure that all survivors can access support by ensuring the availability of multilingual services, when possible, by staff with lived knowledge of the challenges and barriers experienced by migrant and refugee women. Accessibility of helplines can be improved with the use of technology, e.g., including chat options, mobile apps, and integration into social media platforms for wider reach.
- States should guarantee nationwide visibility of helpline information, ensuring that the national helpline number is widely promoted across the country, including in media coverage related to VAWG.
- States must recognise the specific contribution of WSS in providing helpline services and refrain from replacing or prioritising gender-neutral helplines over women's specialist helplines, as they risk depriving survivors of a gender-specific service.
- Helpline staff must receive comprehensive training informed by a gender-specific understanding of VAWG, including knowledge of legal rights, regional contexts, and all relevant provisions for women survivors of violence. The different needs of diverse groups of women need to be addressed by helpline staff in collaboration with specialised women's organisations catering to specific groups.

Women's shelters



Introduction

Shelter is one of the most fundamental components of a comprehensive response to violence against women (VAW). For women whose homes

have become places of fear, control, and physical danger, access to secure accommodation can mean the difference between life and death. Shelters are not merely emergency spaces but essential specialist support services that uphold women's human rights, autonomy, and dignity. They enable survivors of violence to escape abuse, recover from trauma, and rebuild their lives in safety and independence.

The term 'women-only shelters' refers to shelters exclusively for women survivors of violence and their children (if any), and ensures immediate access to safe accommodation. These provide empowering support based on a feminist and gender-specific approach, focusing on the human rights and safety of survivors. The functions of women's shelters go beyond providing a safe place to stay in an emergency. Shelters also offer long-term support in order to provide women and their children with the opportunity and resources necessary to resume their lives free from violence. Some examples of services provided by women-only shelters include: counselling, legal advice and assistance throughout legal proceedings, support to enter/re-enter the labour market, and move-on support to find long-term accommodation after staying in the women's shelter.

Besides women-only shelters, accommodation for women survivors of VAW can also be provided by other shelters. Shelters included under this category do not have to be specialised or gender-specific, but they should still provide some level of support to all survivors of violence, such as women, men, and people who identify as LGBTIQ+. Shelters under this category also include services that provide temporary housing and general shelters.

The European women's shelter movement has its roots in feminist activism of the 1970s, when women across Europe and beyond organised collectively to challenge male VAW that was normalised and hidden within the family. The first shelters were founded by women for women as grassroots initiatives born out of solidarity and political consciousness. Feminists who founded the first shelters were also the first ones to recognise and claim that VAW was not a private problem but a structural manifestation of gender inequality.

These early shelters were often established with no state support and against prevailing social and political norms that denied the existence of violence within the home. They were spaces of resistance, providing physical safety

alongside awareness-raising, collective empowerment, and advocacy for legal and social change. Their feminist approach was revolutionary in its insistence that intimate partner violence was rooted in unequal power relations and must be addressed as a matter of justice and human rights.

Over time, these pioneering efforts evolved into national networks and international alliances, among which the WAVE Network plays a leading role. WAVE and its members uphold the legacy of the feminist shelter movement, defending the rights of women survivors, promoting high-quality, women-centred services, and challenging trends that undermine the gendered understanding of violence.

Shelters have thus been both practical responses to violence and engines of systemic change. They have exposed the pervasiveness of male violence, influenced public awareness, shaped national and international policies, and contributed to the recognition of VAW as a human rights violation.

The Istanbul Convention and WAVE's standards define what constitutes an adequate and rights-based shelter service. A shelter must provide more than a bed: it must ensure physical safety, confidentiality, comprehensive support, and empowerment.

At a minimum, shelters should:

- Be accessible to all women, including those with disabilities, migrant women, or those from rural areas, who may face additional barriers or discrimination in accessing support services.
- Be accessible 24/7 and located throughout the country, including in rural areas.
- Be secure and confidential, with clear safety and protection protocols.
- Employ trained female staff with expertise in VAW.
- Provide individualised needs assessments, risk management, and safety planning.
- Offer counselling, advocacy, legal and psychological support, and assistance in accessing justice, health care, social protection, and housing.
- Support economic empowerment and long-term reintegration through employment and education initiatives.
- Be free of charge for women without means.
- And operate on the basis of women's empowerment, non-victimisation, and self-determination.



Shelters should also guarantee child-friendly environments and employ trained childcare workers to support children's recovery.

Quantitative standards are equally important for ensuring that the number of shelter beds matches the population and its needs. In its Explanatory Report, the Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention), recommends one shelter place per 10,000 inhabitants.⁶⁰ In WAVE's methodology, *one family place equals one bed space*.

While this standard represents a minimum benchmark, experience across Europe shows that real demand often exceeds these numbers, particularly in times of economic crisis, conflict, or social disruption. Sustainable funding, geographic coverage, and high-quality standards are just as significant as meeting the required number of shelter places. In addition to ensuring an adequate number of shelter places, many countries also highlight the importance of removing perpetrators from the home so that women and children do not have to relocate. Depending on the national context, some systems prioritise expanding shelter capacity, while others emphasise perpetrator removal measures, thereby reducing overall demand for shelter accommodation.

Women-only shelters embody the feminist principles that shaped the movement and ensure services are grounded in a gendered understanding of violence. Women-only environments are vital for survivors' safety and healing. Many women have experienced violence from men, including intimate partners, relatives, traffickers, or others. The presence of men in shelters can re-trigger trauma, reinforce fear, and undermine women's sense of security. In women-only spaces, survivors can rebuild trust, regain autonomy, and experience solidarity with other women.

Evidence across Europe demonstrates that women-only, feminist-led shelters provide higher-quality, more holistic, and longer-term support. Their methodologies prioritise empowerment, self-determination, and rights-based advocacy. They also act as critical advocates for systemic change, contributing to policy reforms and awareness-raising about gender-based violence (GBV).

PRINCIPLES OF A WOMEN'S SHELTER

According to *Away from Violence – Guidelines for Setting Up and Running a Women's Refuge* (WAVE, 2004),⁶¹ a women's shelter (or women's refuge) provides safe accommodation for women and their children who have experienced domestic violence, enabling them to live without fear of further abuse. Its primary purpose is safety, but its role extends far beyond providing a temporary place to stay.

Grounded in a feminist and human rights-based approach, a shelter is guided by the principles of women's empowerment, solidarity, and advocacy for women's rights. The central aim is to help women overcome the powerlessness created by violence, regain self-esteem, and rebuild the capacity for a self-determined and independent life.

Shelters provide crisis intervention and ongoing support, including counselling, legal and practical assistance, housing and employment advice, and help throughout legal procedures. Support must be offered by trained female counsellors who understand the gendered dynamics of violence and believe women's stories without prejudice. Women are seen as experts in their own situations, and counselling focuses on empowerment rather than pathology.

Shelters are also communal spaces, offering an environment where women and children can experience new, non-violent ways of living and relating. They serve as models of equality and shared responsibility, helping women overcome internalised gender stereotypes and rediscover their strength and agency.

Beyond their direct support role, women's shelters fulfil an essential public and political function. They raise awareness about domestic violence, challenge social norms that perpetuate male violence, cooperate with professionals and institutions, and engage in prevention, education, and advocacy. They act as agents of social change, exposing the structural and political roots of male violence and demanding accountability from perpetrators and society.

Fundamentally, shelters are built on the principles of:

- **Safety** – secure, confidential, and protective accommodation
- **Empowerment** – supporting women to regain control over their lives

⁶⁰ See Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention): Explanatory Report, para. 135.

⁶¹ See WAVE Network (2004) *Away from Violence – Guidelines for Setting Up and Running a Women's Refuge*.

- **Solidarity and advocacy** – believing survivors and standing beside them without neutrality toward violence
- **Feminist understanding** – recognising violence as rooted in gender inequality and power imbalances
- **Collectivity** – creating a supportive community that models equality and cooperation
- **Social responsibility** – raising public awareness, influencing policy, and preventing violence

Only services that fulfil these functions, ensuring safety, empowerment, advocacy, feminist values, and societal engagement, can rightly be called a women's shelter.

However, the trend toward gender-neutral service models threatens these achievements. Increasingly, public authorities allocate funding through competitive procurement that prioritises cost-efficiency over expertise. Generic or commercial providers often lack the gendered understanding necessary to support survivors effectively. As a result, feminist NGOs are losing funding or being excluded from public contracts, leading to a loss of specialised knowledge, continuity, and trust from survivors.

Maintaining women-only shelters is therefore not merely a question of service design, but a matter of human rights, equality, and feminist principles. They represent both a safe haven and a political commitment to ending male VAW.

International obligations

States have binding obligations under international and regional human rights instruments to establish and maintain adequate shelter services for women survivors of violence. The Istanbul Convention, in Article 23, requires that:

“Parties shall take the necessary legislative or other measures to provide for the setting-up of appropriate, easily accessible shelters in sufficient numbers to provide safe accommodation for and to reach out proactively to victims, especially women and their children”.⁶²

To support the implementation of Article 23, the Council of Europe published its *Quality Guidelines for Shelters for Victims of Violence against Women and Domestic Violence*

in 2021.⁶³ These Guidelines specify that shelter systems should be independent, women-led, and non-profit, and that the safety and autonomy of survivors must be the guiding principles of all operations. Services should be comprehensive, encompassing immediate protection, psychological and legal support, advocacy, and assistance in securing long-term housing and economic stability. Staff should be trained women professionals with expertise in GBV, trauma, and intersectional approaches. States must ensure sustainable, long-term funding rather than project-based or competitive procurement systems to guarantee continuity and prevent the erosion of women's specialist services (WSS). The Guidelines also emphasise that all women must be able to access shelters, including migrant and refugee women, women with disabilities, older women, and those without residence status or financial resources.

The Convention on the Elimination of All Forms of Discrimination against Women (CEDAW) imposes obligations on States Parties to eliminate discrimination and ensure women's right to live free from violence.⁶⁴ General Recommendation No. 35 (2017)⁶⁵ affirmed that States Parties must exercise due diligence to prevent, investigate, punish, and provide reparation for acts of VAW, including by ensuring access to safe shelters and support services.

Within the European Union (EU), several legal and policy instruments reinforce these obligations. The EU Gender Equality Strategy 2020–2025 prioritises the eradication of GBV and the strengthening of WSS.⁶⁶ The EU Victims' Rights Directive (2012/29/EU)⁶⁷ already requires that all victims of crime have access to confidential, free-of-charge support, but it does not explicitly guarantee specialised women-only services.

In 2024, the EU Directive on combating violence against women and domestic violence obliges Member States to ensure the availability of shelters and interim accommodations to all survivors of violence. It requires that services shall be provided in sufficient numbers, be easily accessible and can accommodate the specific needs of women, including by providing women-only shelters with room for children.⁶⁸

While the Preamble to EU Directive 2024/1385 (paragraphs 58–59) recognises women's specialist support services as providing tailored, integrated assistance to survivors, it problematically frames them as optional supplements to general victim support rather than as essential, autonomous components. Crucially, the Preamble lacks the

62 See Article 23 of the *Council of Europe Convention on preventing and combating violence against women and domestic violence* (Istanbul Convention, 2011).

63 See *Council of Europe Quality Guidelines for Shelters for Victims of Violence against Women and Domestic Violence* (2021).

64 See *Convention on the Elimination of All Forms of Discrimination against Women* (1979).

65 See *CEDAW General Recommendation No. 35 on gender-based violence against women, updating General Recommendation No. 19* (2017).

66 See *European Commission Gender Equality Strategy 2020–2025* (2020).

67 See *Directive 2012/29/EU of the European Parliament and of the Council establishing minimum standards on the rights, support and protection of victims of crime* (2012).

68 See *Articles 30 of Directive (EU) 2024/1385 of the European Parliament and of the Council on combating violence against women and domestic violence* (2024).



binding legal force that specialist services hold under the Istanbul Convention, in line with WAVE standards. Article 25 on specialist support to victims compounds this issue by allowing specialist services to be subsumed within general services, undermining the Istanbul Convention's clear distinction and treating women's support services merely as potential referral options rather than foundational support systems. This approach contradicts Article 22 of the Istanbul Convention, which establishes that specialist women's services, ideally provided by feminist, survivor-centred women's organisations, must be independent and are vital for effectively combating GBV.

Across Europe, provision remains inadequate, uneven, and often under threat from gender-neutral policies and market-driven service models. Upholding the feminist principles on which the shelter movement was built is crucial to ensuring that shelters remain places of refuge, transformation, and freedom.

WAVE framework for data collection on shelters

For the 2025 report, WAVE collected information on shelters through a structured framework completed by member organisations across Europe. The data collection tool distinguishes between women-only shelters, which are specialist feminist services serving exclusively women and their children, and other shelters, which may provide accommodation for a broader range of survivors.

The questionnaire examined the number and capacity of shelters, measured against the Istanbul Convention standard of one shelter place per 10,000 inhabitants. Respondents reported on accessibility, geographical coverage, length of stay, funding sources, the entities running the shelters, and inclusivity for diverse groups of women. It also collected data on the main forms of violence reported, services provided within shelters, and reasons for declining referrals. Most data refer to women-only shelters, while an overview of other shelters is provided as a separate section.

Findings on the situation of women-only shelters in Europe

Data collected for the WAVE Country Report 2025 from 46 European countries show that there are 3,224 shelters accessible to women, of which 2,365 are women-only shelters. Altogether, these shelters provide approximately 48,860 bed spaces for a total population of more than 847 million people. This remains far below the Istanbul Convention's minimum standard of one family place per 10,000 inhabitants, requiring an estimated 84,732 bed spaces across Europe. Only 12 countries currently meet this standard, as well as **Northern Ireland**, as separate data was collected for the four **United Kingdom** entities.

Within the 27 EU Member States, there are 2,371 shelters accessible to women, including 1,737 women-only shelters, offering 33,450 bed spaces. This represents a 40% shortage in provision, with nine EU Member States meeting the Istanbul Convention benchmark. In non-EU European countries (19 in total), there are 853 shelters accessible to women, including 628 women-only shelters, providing 15,410 bed spaces. The gap is even more severe, with 62% of the required beds missing, and only four countries meeting the Istanbul Convention standard.

	Europe (46)	EU Member States (27)	European countries outside the EU (19)
Total population	847,316,893	448,696,087	398,620,806
Total number of women-only shelters	2,365	1,737	628
Total number of shelters accessible to women	3,224	2,371	853
Number of existing bed spaces	48,860	33,450	15,410
Total number of bed spaces needed	84,732	44,870	39,862
Number of countries that meet the Istanbul Convention standard	12	9	3

Table 8: Overview of service provision – women's shelters in Europe

Compared with the findings presented in the WAVE Country Report 2023, the data indicate a modest overall increase in both the number of shelters and the number of available beds across Europe. The total number of shelters accessible to women rose from 3,087 to 3,224, and the total number of bed spaces increased from 39,130 to 48,860. While this is a positive trend, the recorded increase may be due to differences in data collection and national statistics and doesn't necessarily reflect an actual increase in the number of beds. One country influencing this increase is **France**: in the previous Country Report, only the shelters and beds belonging to the FNSF Network were included, whereas the current report includes overall national statistics, resulting in an increase from 111 shelters accessible to women to 250 shelters accessible to women, and from 4,905 beds to 11,000 beds. The number of countries meeting the Istanbul Convention minimum standard grew from 10 to 12, reflecting gradual but uneven progress across regions. In **Montenegro**, the number of shelters increased from three to four, expanding capacity from 42 to 64 bed spaces and enabling the country to

meet the Istanbul Convention standard for the first time. In **Croatia**, the number of beds also increased from 346 to 402, exceeding the requirement of the Istanbul Convention. As mentioned, **France** reported an increase in the number of bed spaces and now meets the Istanbul Convention standards. On the contrary, **Liechtenstein** does not meet the Istanbul Convention standards, while in the previous report, it did. This is due to a difference in data collection: the previous reports erroneously included children's beds in the overall count.

In the EU Member States, the number of shelters accessible to women increased from 2,232 to 2,371, while the number of available beds rose from 27,124 to 33,450. The number of EU countries meeting the Istanbul Convention benchmark increased from seven to nine.

In non-EU European countries, shelter availability also improved, though significant gaps remain. The number of shelters accessible to women remained stable, and bed spaces increased from 12,006 to 15,410; however, this is most likely due to data availability, as in 2023, there was no data available for **Russia** and **Ukraine**. The number of non-EU countries meeting the Istanbul Convention standards remained the same (three countries).

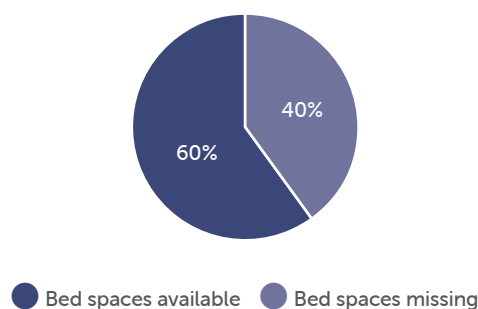


Figure 4: Percentage of bed spaces available and missing in EU Member States

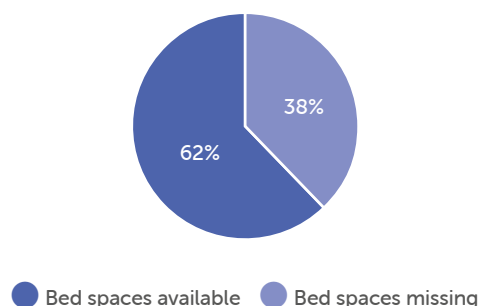


Figure 5: Percentage of bed spaces available and missing in European countries outside the EU

COUNTRY EXAMPLE: Republic of Moldova

In 2024, the Republic of Moldova reported 13 women-only shelters providing 235 bed spaces for women and their children, a significant increase from eight shelters and 181 beds in 2022. With this expansion, Moldova now nearly meets the Istanbul Convention's minimum standard, with only seven beds (3%) still missing. Shelters operate across most regions, though coverage remains uneven, with rural and remote areas still lacking adequate access to services.

Shelters are operated by a mix of public authorities, NGOs, religious organisations, and national institutions, reflecting a hybrid model of service provision. Local public authorities manage the majority of shelters under the coordination of the Ministry of Labour and Social Protection, while a smaller number are run by non-governmental organisations. Among them, Casa Mărioarei remains the only feminist-run shelter in the country, offering a comprehensive, empowerment-based approach grounded in women's rights and survivor-centred support.

Funding for women's shelters primarily comes from state and local budgets, complemented by international donors and occasional religious or private contributions. Despite progress in expanding capacity, challenges persist, particularly in ensuring stable funding, retaining qualified staff, and meeting the needs of women with disabilities or from rural areas. Overall, Moldova's shelter system demonstrates significant growth and institutionalisation, yet continued investment and stronger coordination with feminist NGOs are essential to ensure full compliance with international standards.

Findings of women-only shelters

Across 32 countries that could provide this information, women-only shelters accommodated an estimated **160,566 women and children in 2024**.⁶⁹ While this figure provides an important indication of the scale of shelter provision, it should be treated as approximate due to significant gaps and inconsistencies in national data collection. Some countries, such as **Türkiye, Austria, Portugal, and the Netherlands**, publish detailed annual statistics, whereas others rely on estimates from women's NGOs or administrative records at the local level.

⁶⁹ Four countries provided estimates from previous years, as data for 2024 was not available.



Accessibility of women's shelters

→ 24/7 accessibility

Data collected for the WAVE Country Report 2025 show that women-only shelters are accessible around the clock in most European countries. In total, 67% of countries (31 out of 46) report that all or most women-only shelters offer 24/7 access, ensuring immediate availability for women escaping violence at any time. However, in 22% (10 countries), only some shelters provide 24/7 access, and two countries (4%), Belarus and Greece, report no round-the-clock access. In three countries, the situation remains unknown or unclear, often due to fragmented data or decentralised service provision.

This unevenness highlights a continued geographical and policy gap: while some states maintain comprehensive access, others restrict entry to office hours or rely on referral mechanisms that delay safety and protection.

Comments from the WAVE delegates who provided the data further reveal that 24/7 access often depends on interpretation and capacity. In several countries, such as **Finland and the Netherlands**, all shelters or emergency crisis centres provide full 24-hour access, ensuring immediate admission at any time. In others, including **Belgium (Flanders) and Iceland**, 24/7 accessibility refers primarily to the availability of hotlines or on-call staff, rather than guaranteed physical admission.

In countries like **Luxembourg, North Macedonia, and Poland**, shelters are reachable at night only through police or emergency referrals, which can create delays or exclude women who seek help independently. Some comments also noted that funding limitations or local administrative rules restrict night-time intake. In **Romania**, several categories of shelters exist, differentiated by the range of services provided and the duration of accommodation. Emergency reception centres offer immediate, short-term refuge for women survivors of violence. These centres are accessible 24/7 and typically accommodate women for a period of two to six months, providing essential services such as food, basic medical care, and psychological support. Recovery centres, on the other hand, provide a more stable and supportive environment with comprehensive assistance, including social services, legal counselling, and rehabilitation programmes. Their primary aim is to support women in regaining autonomy and rebuilding their lives free from violence, with stays lasting up to one year or longer, depending on individual needs.

→ Women's shelters accessible free of charge

Across Europe, most women-only shelters continue to provide free accommodation and services, ensuring that survivors can access safety without financial barriers. However, the latest data show that 11 out of 46 countries (24%) report that women may be expected to contribute

financially for their stay. These countries are: **Belgium, the Czech Republic, Denmark, France, Germany, Ireland, Luxembourg, the Netherlands, Slovakia, Slovenia, and parts of the United Kingdom (Scotland and Wales)**.

In **Belgium** and the **Netherlands**, payment is systematically required, structured as a regulated co-payment model linked to income or social welfare rules. In both contexts, women are routinely expected to contribute a proportion of their accommodation costs, often calculated in accordance with municipal or social housing regulations. In the **Netherlands**, these costs are usually paid for through social welfare or benefits, which the women's shelters help women apply for with the municipalities. Women who already have income from a salary or otherwise pay from this income. In no case are accommodation costs a barrier to accessing women's shelters. In **Brussels and Wallonia**, women are expected to pay for their accommodation in the shelter, meaning undocumented women without sufficient income have limited access to women-only shelters, whereas in **Flanders**, accommodation is free for the first 12 weeks of a stay in a shelter with a secret address. After this twelve-week period, women are expected to cover the accommodation costs either through their own means or through a contribution from the Public Centre for Social Welfare.

In the remaining countries, payment is conditional or occasional. Women may be asked to contribute only if they have an income or receive social welfare benefits, while those without financial means are accommodated free of charge. In **Germany**, for instance, co-payment practices vary regionally depending on local funding arrangements. In **France**, it is based on the woman's income. In Ireland, women are expected to pay from approximately €30–€60 per week, but the inability to pay is commonly considered by services. In **Denmark and Luxembourg**, shelters may apply small daily or weekly fees for women with income, which are waived in hardship cases. In **Slovenia**, shelters charge women 30% of the financial social assistance, but this can be adjusted according to income, whereas in **Slovakia**, accommodation is either free for the first few months or subject to only symbolic payments. Any contributions required later are typically adjusted based on the woman's individual financial situation, including her income and essential expenses. In the **Czech Republic**, women are also expected to pay a fee for their stay. In **Wales**, the majority of the cost is met by Housing Benefit, but survivors also pay a minimal weekly service charge. In **Scotland**, women are expected to pay for their accommodation in shelters, but most do so through housing benefits.

By contrast, the majority of countries, 35 out of 46 (76%), confirm that shelter services are entirely free of charge. In **Russia**, church-affiliated shelters may ask women to help with domestic chores or community work, but no monetary payment is requested.

→ Accessibility for diverse groups of women

Overall, inclusion has improved in recent years, but many women still face structural obstacles to accessing shelter services. Aggregated data show that across Europe, women with uncertain or undocumented residence status can access all or most women's shelters in 41% of countries (19 out of 46), while 35% (16 countries) report partial access and three countries, **Bulgaria, Serbia and Russia** (7%) have no women's shelters that can accommodate this group. These figures confirm that immigration and residency status remain among the strongest determinants of exclusion, particularly in states with 'no recourse to public funds' rules or restrictive welfare eligibility.

Refugee and homeless women encounter similar but slightly better conditions. Refugee women can access all or most shelters in 23 countries (50%), and they have partial access in an additional 12 countries (26%), while homeless women can access all or most shelters in 21 countries (46%). These proportions suggest that most shelters can accommodate women in unstable housing situations, but the absence of targeted support and the lack of long-term housing options continue to limit their safety and recovery.

Accessibility for women with disabilities remains inconsistent across Europe. Only 37% of countries report that all or most shelters are physically accessible to women with mobility impairments, while 41% report that some shelters are accessible. For women with cognitive or psychosocial disabilities, 26% of countries report all or most shelters being accessible, and 41% partial accessibility. Shelters designed or renovated without full consideration of accessibility standards often rely on ad hoc adaptations or the goodwill of staff rather than systematic provision. Accessibility for women with sensory disabilities (blind or deaf women) is similar, as 26% of countries state that all or most shelters are accessible and 48% report partial accessibility.

Women with substance abuse issues are among the most excluded groups, with only 17% of countries reporting all or most shelters being accessible, while 17% have no available shelters for this group, and in 48% of the countries, some shelters are available to them. Shelters often cite safety, limited staff capacity, and lack of addiction specialists as reasons for exclusion. This points to an ongoing tension between safety management and inclusivity, where rigid risk-assessment protocols may unintentionally exclude some of the most vulnerable survivors.

Older women (65+) have comparatively better access to shelters, with 61% of countries reporting all or most shelters being open to them, reflecting recognition that violence persists in later life and that ageing survivors require specific support. Older women don't have access to shelters in only three countries (0.7%): **Belarus, Greece and**

Russia. Transgender women in 28% of countries have full or broad access, and 35% partial, while in 11% of the countries they are excluded. These disparities are often linked to policy ambiguity, inadequate guidance on inclusive practice, or resistance from funders or authorities to recognising transgender women as legitimate service users.

Shelters are generally more inclusive toward women with children with disabilities, wherein 43% of countries, all or most shelters accept them, and in 30% of the countries, some shelters accept them. Regarding women with older sons (14+), they have full or broad access to shelters in 26% of countries, and access to some shelters in 35% of countries. Unaccompanied girls under 18, however, remain largely excluded: only 20% of countries report that all or most shelters can accommodate them, while 24% provide partial access and 30% none. In most cases, unaccompanied minors fall under child protection rather than GBV frameworks, leaving a protection gap for adolescent girls exposed to violence.

According to the WAVE data, 23 countries report the existence of specialised shelters or accommodation programmes for particular groups of women, such as migrant or undocumented women, women with disabilities, LGBTQI+ survivors, older women, or women affected by specific forms of violence, including trafficking, forced marriage, or so-called honour-based violence. While these services represent essential steps toward inclusivity, they remain limited in number, unevenly distributed, and often dependent on short-term or project-based funding.

The most widespread form of specialised provision across Europe remains shelters for victims of trafficking, reported in 12 countries. In nine countries, there are dedicated services for LGBTQI+ survivors. In eleven countries, there are also shelters for migrant and ethnic minority women. In **Austria**, the women's counselling centre 'Orient Express' in Vienna runs a shelter/emergency apartments, which can accommodate girls and women between the ages of 16 and 24 from all over Austria, who are affected by forced marriage.⁷⁰ In **Sweden**, there are specific shelters for women with substance abuse issues, women subjected to prostitution, pornography and trafficking, and women survivors of honour-based violence. Dedicated facilities for women with disabilities, older women, or women with substance abuse issues are comparatively rare and usually operate through partnerships between NGOs and specialised service providers rather than as stand-alone shelters. For example, in **Albania**, the Albanian Disability Rights Foundation offers specialised support, including technology-based tools and peer mentoring and partners with shelters to improve accessibility for women with disabilities. In **Slovenia**, there is one shelter specifically designed for women who have substance abuse issues and are survivors of violence.

70 In Sweden, prostitution is considered a form of men's violence against women by law.



Funding for women-only shelters

Reliable, sustainable funding remains one of the most critical determinants of whether women-only shelters can provide consistent, quality, and survivor-centred support. According to the data collected, 44 of 46 countries report state or municipal funding as a primary source, whereas there is none in **Azerbaijan** and **Belarus**. In **Moldova**, state funding is provided, but not extended to the **Transnistrian region**, where shelters rely exclusively on international and private sources.

In several Western and northern European countries, such as the **Netherlands, Denmark, Germany, Austria, and France**, shelters are integrated into public social protection systems and receive funding from national, regional, and municipal budgets. In **Austria**, women's shelters are funded by the respective federal states and local governments, resulting in differing funding schemes across regions, and women are usually asked to enter shelters in their own area unless they are facing a high-risk situation. In **Finland**, for example, all three women-only shelters have been fully state-funded since 2015, following ratification of the Istanbul Convention. In **Denmark**, municipalities finance shelter accommodation at daily rates between DKK 1,000–3,000 (ca. €130–400), with 50% reimbursement from the state. However, in other contexts, public funding covers only basic operational costs, and shelters must actively seek supplementary resources to provide comprehensive support. In **Bosnia and Herzegovina**, for example, a 2025 amendment to the Law on Protection from Domestic Violence establishes a framework for funding shelters from federal, cantonal, and local budgets, but the exact financing percentages were removed, leaving implementation uncertain. Similarly, in **Serbia**, women-only shelters are financed from local government budgets, which mainly cover basic operations, but are also supported by donations from foundations and individuals. In **Slovenia**, the Ministry of Labour, Family, Social Affairs and Equal Opportunities is the primary funder of the 16 women-only shelters, covering around 70% of their costs.

Beyond state support, donations from foundations, private individuals, and trusts are among the primary funding sources in 31 countries. Donations remain particularly important for feminist NGOs, which use them to cover staff salaries, rent, utilities, and emergency assistance when state funding is delayed or insufficient. In **Armenia**, for instance, state funding typically covers only 10% of women's shelters' budgets, with the remainder coming from foundations, international donors, and individual contributions. International funding was reported in 16 states, and additionally in the **northern part of Cyprus**. It plays a vital role in sustaining shelters in non-EU countries when domestic resources are limited. Examples include support from UN agencies, foreign embassies, and private foundations, often providing lifelines for independent women's NGOs that cannot access national funding. In **Türkiye**, for instance, the *Mor Çatı* Women's Shelter Foundation operates the

only independent feminist shelter, financed exclusively through donations and international projects.

Religious institutions contribute to shelter funding in nine countries: **Albania, Germany, Italy, Malta, Portugal, Romania, Russia, Switzerland, and Ukraine**. In most cases, this support is provided through church-affiliated charities or faith-based NGOs, rather than feminist women's organisations. In **Russia**, for example, some shelters run by the Orthodox Church provide accommodation and basic aid, though not necessarily gender-specific or rights-based services.

A particularly detailed report from **Albania** shows an expanding network of state-funded services. In 2024, the Ministry of Health and Social Protection financed 11 services for victims of violence, including six multifunctional emergency centres, three shelters for victims of violence and trafficking, one national counselling line, and one day residential service for women and girls in vulnerable situations, reaching over 4,000 beneficiaries with a budget increase from 51.8 million ALL to 79.4 million ALL (€ 515,000 to € 789,000). Crowdfunding was also reported as an emerging supplementary source in some contexts.

Despite progress, funding remains fragmented, project-based, and precarious in much of Europe. Many shelters operate under annual budget renewals or competitive tenders, which undermine long-term planning and the ability to retain experienced staff.

In contrast, a few countries, such as **Sweden, Slovenia, and Iceland**, offer multi-year, partnership-based agreements between the state and women's NGOs, which guarantee stability and preserve independence.

Entities running women-only shelters

The data collected show that women-only shelters across Europe are most frequently operated by women's NGOs with a gender-specific or feminist approach. Such organisations run shelters in 38 out of 46 countries, confirming that the women's movement continues to form the backbone of protection and support for women and their children escaping violence. In addition, 20 countries report women-only shelters operated by other NGOs, while faith-based organisations provide such services in 14 countries. State authorities run women-only shelters in 17 countries, as well as in the **northern part of Cyprus**, and four countries report that shelters are managed by other entities, such as government-organised non-governmental organisations (GONGOs) or hybrid partnerships between public and private actors.

In total, WAVE delegates reported 760 shelters run by feminist women's NGOs, 180 by other NGOs, 181 by faith-based organisations, and 449 by state institutions. These figures should be interpreted with caution, as several countries were unable to provide disaggregated data or

could only offer estimates. In some contexts, data are not systematically collected at the national level, and in others, shelter management is decentralised, with responsibilities shared between local authorities and contracted service providers. This makes it difficult to establish an exact breakdown of shelter operators, particularly in countries with mixed or hybrid models.

Despite these challenges, the data clearly demonstrate that women's NGOs continue to play a leading and irreplaceable role in ensuring that women-only shelters operate in line with feminist principles and international standards. However, the growing involvement of state and non-feminist actors marks an important structural shift. In several countries, the expansion of state-run and faith-based services has been accompanied by a reduction in the influence of feminist NGOs, often linked to competitive funding models or changes in policy frameworks that favour gender-neutral or welfare-oriented service provision. There has been an increase in the proportion of shelters run by private, profit-driven actors in **Sweden**. In 2012, only 8% of shelters were run by private, profit-driven actors. This figure rose to 37% in 2019, and in the latest mapping report, it stands at 47%. At the same time, the proportion of non-profit shelters has decreased from 54% in 2019 to 46% in the first quarter of 2024.

Examples from national contexts illustrate the diversity of organisational models. In **Albania**, a mixed system exists in which seven shelters are run by NGOs, including *Edlira Haxhiymeri*, *Different and Equal*, *Woman to Woman*, and *Vatra*, while others are operated by faith-based organisations (*Papa Giovanni XXIII* in Shkodër and *Rozalba Home* in Lezhë), and by the state (including the *National Centre for the Treatment of Victims of Domestic Violence* and the shelter for victims of human trafficking). In Austria, the long-established Association of Austrian Autonomous Women's Shelters (*Autonome Österreichische Frauenhäuser – AÖF*) continues to provide gender-specific support, although some shifts have taken place, such as in Salzburg, where the former women's shelter was replaced by a mixed model jointly managed by Association *VIELE* and *Jugend am Werk*. In **Finland**, several municipal and national agencies operate shelters, but *Turvakoti Mona*, run by a women's NGO focusing on cultural and ethnic diversity, remains an example of feminist practice within a publicly funded framework. In **Moldova**, *Casa Mărioarei* remains the only feminist-run shelter, offering empowerment-based, trauma-informed support to survivors and representing a model that should be expanded nationwide. In **Slovenia**, feminist organisations such as SOS Helpline for Women and Children, the Association for Nonviolent Communication, and *Ženska svetovalnica* operate alongside state and faith-based services. In **Türkiye**, the Ministry of Family and Social Services runs 112 shelters, municipalities manage 35, and the Directorate of Migration oversees two, reflecting a highly centralised, state-led model.

Geographical coverage

The data collected by WAVE show that women-only shelters are available across all regions in 16 countries, while in 11 countries, they cover most regions (over 50%). In 13 other countries, shelters are concentrated in major cities; in Armenia, the northern part of Cyprus, and the Transnistrian region of the Republic of Moldova, they are located only in the capital. For five countries, no information was provided.

This distribution highlights persistent regional disparities in access to safe accommodation for women and their children. In several countries, particularly those with centralised or urban-based service networks, women living in rural or remote areas continue to face significant barriers to reaching protection.

Comments from WAVE delegates underline that these gaps are not merely geographic but also reflect broader inequalities in resource allocation and local governance capacity. In federal or decentralised systems, such as those in **Austria and Spain**, regional authorities have varying levels of responsibility and funding for shelter provision, resulting in uneven coverage. In smaller countries like **Slovenia** or **Denmark**, the concentration of shelters in urban centres is partly offset by well-developed outreach and referral mechanisms, while in others, such as **Poland and Serbia**, large regions remain without any women-only shelter.

The WAVE data reveal considerable variation in how long women usually stay in women-only shelters across Europe. In eight countries, the average length of stay is limited to up to three months, while in 15 countries, women remain for three to six months. In 12 countries, stays of up to one year are possible, and in three countries, women may stay for more than a year, often in transitional or reintegration shelters designed to support longer-term recovery. In some countries, shelters offer varying lengths of stay. Some shelters have a fixed length of stay, sometimes connected to funding models, while others are more flexible. Eight countries did not provide any data on this question. These figures suggest that while most countries provide mid-term accommodation (up to six months), only a smaller group offers the extended stays necessary for survivors to rebuild stability and autonomy. In practice, the duration of stay often depends on the type of shelter: emergency shelters typically accommodate women for a few weeks to several months, while reintegration or recovery shelters may allow stays of one year or more. For example, in some systems, emergency shelters provide accommodation under court-issued protection orders, after which women are referred to long-term reintegration services, though these remain limited in number.



Data on the provision of services in women's shelters

According to data collected for the WAVE Country Report 2025 on the three most common forms of violence reported by survivors accommodated in women-only shelters, domestic violence remains by far the most common form of violence, cited by 35 countries. This broad category encompasses various overlapping forms of abuse, most frequently physical and psychological violence, which were reported by 32 and 23 countries, respectively.

Other forms of violence were reported less frequently as one of the three most common forms. Sexualised violence was identified in nine countries, while economic violence and violence directly or indirectly affecting children were mentioned in five and seven countries, respectively. Stalking and technology-facilitated violence against women and girls (TFVAWG) were rarely captured, reported in only three and two countries, reflecting both their underreporting and the absence of systematic data collection in shelter services.

The most common forms of in-house support are counselling and psychological assistance, reported in 41 countries, and referrals or collaboration with other services, reported in 39 countries. These findings underline the dual role of shelters as both safe spaces and coordination hubs within national support systems.

Casework, which includes safety planning, risk assessment, and individual support plans, is available in 39 countries, demonstrating that most shelters work through structured, personalised approaches tailored to each survivor's situation. Practical advice on housing, social benefits, healthcare, or employment is offered in 35 countries, reflecting the recognition that long-term independence requires economic and social reintegration.

Finally, legal advice is provided in 33 countries, often through partnerships with women's rights NGOs or legal aid providers. Access to legal counselling remains uneven, however, with some shelters relying on external referral systems rather than in-house expertise.

When it comes to declining referrals, the most frequently cited reason was no space or capacity to support the survivor, reported in 31 countries, followed by survivors being ineligible for support (for example, the woman was an undocumented migrant, or the woman did not experience VAW) in 21 countries. Insufficient capacity to accommodate women with children was cited as a reason for declining referrals in 16 countries. Shelters in 23 countries also reported having to decline referrals because they were not adequately equipped to meet the specific needs of some survivors, for instance, women with physical or mental disabilities, or those requiring specialised accessibility adaptations.

Qualitative comments reveal that capacity shortages remain a significant obstacle across Europe. In some countries, such as **Austria and Moldova**, shelters operate at or near full occupancy year-round, leaving many women unable to find a place when they need it. For example, in **Austria's** AÖF network, approximately 58% of women declined in 2023 could not be accommodated due to a lack of space. The situation is particularly acute for women with children, as many facilities have limited family rooms.

In addition to capacity issues, several WAVE delegates reported that infrastructural barriers and restrictive admission criteria contribute to a decline in referrals. Shelters often lack facilities for women with disabilities or mental health conditions, and in some cases cannot accommodate women with substance use issues or severe psychiatric disorders due to safety and staffing limitations. Accessibility is further constrained where shelters operate in privately rented buildings that cannot be modified for physical access.

Findings on other shelters

Across Europe, 36 out of 46 countries reported the existence of shelters other than women-only facilities that can accommodate and support survivors of VAW. In total, these countries identified 859 such shelters. The range of services and target groups varies widely, reflecting differences in welfare systems, funding structures, and national approaches to violence prevention and protection.

These shelters include mixed-gender crisis centres, family shelters, anti-trafficking facilities, faith-based safe houses, and centres for specific vulnerable groups, such as LGBTQI+ individuals, teenage mothers, or people with substance abuse issues. In several countries, including Belgium, Poland, and Albania, such facilities are run by local authorities or large social welfare organisations, while NGOs often complement their work by providing specialised counselling or referral services.

However, many of these mixed or general shelters lack the specialised, gender-sensitive approach necessary for addressing the specific needs of women survivors of violence. For example, in Belarus, survivors may share accommodation with men, including potential perpetrators, or may be housed in facilities without confidentiality safeguards. Others are placed in 'crisis rooms' or temporary accommodation units under social welfare systems, which do not always ensure safety, privacy, or trauma-informed care.

Among the 36 countries reporting the existence of mixed or non-specialised shelters, the majority indicated that these facilities are open to multiple gender groups and survivor categories. According to the WAVE delegates, 23 countries confirmed that such shelters accommodate cisgender women, 19 accept cisgender men, 17 include

transgender women, 15 accept transgender men, and 16 are open to non-binary people. Additionally, 11 countries noted that shelters also support other groups of survivors, such as people with disabilities, refugees, and survivors of trafficking and those affected by homelessness.

While these figures suggest a growing trend toward inclusivity, several delegates emphasised that the mere openness of shelters does not necessarily ensure safety or gender-competent care. In many cases, the shelters do not apply gender- or trauma-informed approaches, nor do they have trained staff or infrastructure to meet the specific needs of LGBTQI+ individuals or survivors with disabilities. In some countries, admission is formally unrestricted, but services remain de facto gender-neutral, meaning that women survivors of male violence may lack adequate protection, confidentiality, and support tailored to their experiences.

Moreover, respondents highlighted that data on access for transgender and non-binary people are rarely disaggregated or systematically recorded, and that many facilities assume inclusivity without explicit policies or safeguards. Out of the 36 countries that reported the existence of other shelters supporting survivors of VAW, only six countries stated that all such shelters offer 24/7 access, while three reported that most do. In eight countries, only some shelters are available around the clock, and in one country, Belarus, it was indicated that none of their mixed or general shelters provide continuous access.

COUNTRY EXAMPLE: Croatia

Croatia's network of women's shelters has a long feminist history rooted in the activism of the women's movement. The first shelter was opened in 1990 by the Autonomous Women's House Zagreb (AWHZ), marking the beginning of organised, rights-based protection for women survivors of violence. Throughout the 1990s and 2000s, six additional autonomous women's shelters were established by women's NGOs across the country. In 2024, Croatia reported 26 shelters accessible to women.

The shelter system operates under a mixed model, combining women's shelters run by feminist NGOs with state, church, city, and county-run homes that accommodate survivors of domestic violence.

Funding for autonomous women's shelters has deteriorated over time. In the early years, the state, city, and county governments jointly financed around 90% of their annual budgets, each contributing approximately 30%. Today, funding is provided through three-year contracts following public tenders, but each of these authorities now covers less than 30% of operational costs.

Concerns have also been raised about the inclusion of religious organisations in state tenders for shelters. For example, the St. Michael shelter, operated by the Association Betlehem, a faith-based organisation known for its opposition to abortion and sexual and reproductive rights, received state funding under the same mechanism as feminist-run shelters. This raises questions about value-based consistency and adherence to women's human rights principles in state-funded service provision.

Overall, Croatia's shelter system reflects both the legacy of feminist leadership in establishing life-saving support for women and the increasing institutionalisation and bureaucratisation of these services. Safeguarding the feminist model that built Croatia's first shelters remains critical to ensuring that protection services uphold survivors' rights, empowerment, and safety.

Conclusion

The latest WAVE findings confirm what the feminist shelter movement has long known: women-only shelters are indispensable, life-saving, and rights-affirming services, and Europe still lacks enough of them. Across 46 countries, 3,224 shelters accessible to women offer 48,860 bed spaces for a population of 847 million, far short of the Istanbul Convention's recommendation of one place per 10,000 inhabitants, which corresponds to 84,732 beds needed. Only 12 countries (as well as Northern Ireland) currently meet this baseline. Capacity gaps are compounded by uneven geographic distribution, with many rural and remote areas lacking timely access and partial 24/7 availability, which can be the difference between safety and grave risk.

The Council of Europe Quality Guidelines and Article 23 of the Istanbul Convention are clear: shelters must be safe, confidential, survivor-centred, free of charge for women without means, staffed by trained women professionals, and available for as long as needed. Yet practice remains inconsistent. Many countries report co-payment regimes or ad-hoc fees; women with undocumented status, disabilities, substance use needs, or older sons face structural barriers; and unaccompanied girls are frequently excluded due to system silos. The most common reasons for declining referrals are a lack of space and an inability to meet specific needs, revealing gaps in capacity and accessibility. The data also underscore the irreplaceable role of feminist women's NGOs: they operate shelters in 38 countries and anchor survivor-centred, empowerment-based methodologies. However, procurement and gender-neutral service models risk displacing women's NGOs with generic providers, diluting expertise and trust.



🏠 Recommendations

- States should establish clear minimum quality standards for women's shelters in line with the Istanbul Convention and the Council of Europe's Quality Guidelines for Shelters for Victims of Violence against Women and Domestic Violence. These should cover safety, confidentiality, staffing ratios, survivor participation, and accountability mechanisms.
- Governments must guarantee adequate, long-term, multi-year core funding for women's shelters and their umbrella organisations to ensure long-term sustainability and reduce dependence on short-term grants or competitive procurement systems that undermine continuity and autonomy.
- States should establish or strengthen coordination between women's shelters and other key sectors such as law enforcement, justice, health, social welfare, housing, and employment services, ensuring survivors can access adequate support through integrated referral mechanisms, including long-term and affordable housing for survivors after leaving shelters.
- Governments must create or strengthen national databases and monitoring systems to track the number, capacity, and occupancy of women's shelters, ensuring data are disaggregated by gender, age, disability, and residence status while maintaining confidentiality.
- Women's shelters must be accessible to all women, including women with disabilities, older women, migrant and refugee women, queer women, and those with substance-use or mental-health needs, supported by staff trained in inclusive and intersectional practice. Access to women's shelters should be possible irrespective of whether women are undocumented or have a precarious migrant status.
- States must ensure adequate geographical distribution of women's shelters, including in rural and remote areas, and promote public awareness on the right of women to access safe accommodation and available services through accessible, multilingual information.
- Governments should integrate women's shelters into national emergency-response protocols and crisis-response plans. These protocols should be incorporated into National Action Plans on VAWG and ensure continuity of protection and access to shelters for women and children during crises, conflicts, displacement, and natural disasters.
- States should protect the feminist, survivor-centred character of women's shelters by prioritising WSS with proven gender-specific expertise in policy frameworks. Regular consultation with survivors and women's NGOs must be institutionalised, ensuring that those directly affected shape shelter policies and monitoring mechanisms.
- Women's shelters must be staffed by professionals who have received adequate training on how to prevent and respond to VAWG and must put the survivor and her needs at the centre of all interventions. Ongoing training and support for shelter staff, including mental health advice, must be guaranteed to ensure that they can provide the best care to survivors and manage the emotional toll of their work.

Women's centres



Introduction

Women's centres are a cornerstone of specialist support for women and their children survivors of violence against women and girls (VAWG). They provide non-residential, gender-sensitive services that combine emotional, legal, and practical support with empowerment and advocacy. Rooted in feminist principles, these centres not only respond to the immediate needs of survivors but also work to transform the social and institutional conditions that sustain gender inequality and VAWG.

Across Europe, women's centres operate in diverse contexts, some as independent feminist NGOs, others as part of integrated service systems or networks connected to shelters. Despite this diversity, their fundamental mission is consistent: to offer safe, confidential, and women-centred spaces where survivors are believed, supported, and empowered to rebuild their lives. Women's centres are also key actors in prevention, awareness-raising, and policy advocacy, ensuring that women's lived experiences inform national and local responses to violence.

According to WAVE's methodology for this data collection, the term 'women's centre' includes all women's services providing non-residential specialist support to survivors, serving only or predominantly women survivors of violence and their children (if any). These centres provide empowering short and long-term support, based on a feminist and gender-specific approach to violence and focusing on the human rights and safety of survivors. The following services are subsumed under the term:

- Women's counselling and women's crisis centres supporting women survivors of all forms of gender-based violence (GBV)
- Services offering support to survivors of sexualised violence
- Regional crises centres on domestic violence centres
- Pro-active intervention centres serving victims as a follow-up to police interventions
- Specialist services for particular groups such as Black and minority ethnic women, migrant and refugee women survivors of violence
- Outreach services

- Services providing independent domestic or sexual violence advisors
- Other newer types of services

These centres usually provide information, advice, advocacy and counselling, practical support, court accompaniment, proactive support, outreach, and other services.

Since the WAVE Country Report 2023, the landscape of women's centres has continued to evolve under the combined pressures of gender equality backsliding, austerity measures, and increasing institutionalisation of services. In many countries, feminist NGOs face shrinking funding opportunities, competition from gender-neutral or state-run services, and challenges in sustaining their autonomy and quality standards. At the same time, the demand for specialist, long-term, and trauma-informed support remains high, particularly among women facing multiple forms of discrimination or displacement.

International obligations

The Istanbul Convention sets out clear obligations for states to ensure that services for women subjected to all forms of violence are accessible, adequately funded and well-coordinated. Article 18 provides the overarching framework, requiring states to establish a comprehensive and effective network of general and specialist services to protect and assist survivors and their children.⁷¹ It emphasises that all measures must be based on a gendered understanding of violence, respect women's human rights, and prioritise their safety and empowerment. Article 22 builds on this by requiring immediate, short-term and long-term specialist support services for all women survivors of violence and their children. Although the Convention⁷² does not explicitly refer to women's centres, the Explanatory Report makes clear that such specialist women's services, providing counselling, legal and practical assistance, advocacy and empowerment support, are central to meeting these obligations.⁷³ It also recommends that one women's centre providing non-residential support should be available for every 50,000 women in the population, ensuring adequate coverage and accessibility. Women's centres, therefore, represent a key pillar of the Convention's implementation, putting its standards into practice at a community level.

The European Union (EU) Directive 2024/1385 acknowledges the importance of women's specialist support services in its Preamble (paragraphs 58–59), emphasising their role in providing targeted, integrated support tailored

⁷¹ See Council of Europe (2011) *Convention on Preventing and Combating Violence against Women and Domestic Violence* (Istanbul Convention), Article 18.

⁷² See Council of Europe (2011) *Convention on Preventing and Combating Violence against Women and Domestic Violence* (Istanbul Convention), Article 22.

⁷³ See Council of Europe (2011) *Explanatory Report to the Convention on Preventing and Combating Violence against Women and Domestic Violence* (Istanbul Convention), para. 85.

to survivors' needs. However, the Directive's framing problematically positions these services as optional add-ons to general victim support rather than as essential, autonomous pillars, and the Preamble lacks the binding legal status accorded to specialist services under the Istanbul Convention and in line with WAVE standards. Article 25 perpetuates this ambiguity by suggesting specialist services can form part of general services, blurring the clear distinction established by the Istanbul Convention, and merely references women's support services as potential referral points rather than as foundational systems of support. This limited recognition fails to align with Article 22 of the Istanbul Convention, which asserts that specialist women's support services, best delivered by women's organisations rooted in feminist principles and survivor-centred approaches, are independent and critical for effectively addressing GBV.

The UN Committee on the Elimination of Discrimination against Women (CEDAW), in its General Recommendation No. 35 on gender-based violence against women (2017),⁷⁴ calls on States to ensure that survivors have access to shelters, counselling, legal aid, and support from women's organisations. This establishes a clear expectation that States should fund and enable women-led, gender-sensitive services, including women's centres, as part of comprehensive protection systems.

WAVE framework for data collection on women's centres

This chapter provides an updated overview of women's centres in Europe in 2024, based on data and qualitative input from WAVE members. Respondents provided data on the number and type of women's centres in their country, distinguishing between those working only with women survivors of violence against women (VAW), those open to all survivors of GBV, and other forms of provision. The questionnaire also collected information on the range of services available, including counselling and psychological support, legal advice, advocacy, and practical assistance such as housing or employment support. It explored accessibility for diverse groups of women, the existence of specialised services for survivors of specific forms of violence, management and funding structures, and geographical coverage.

This comprehensive approach enables comparison across countries and identifies progress and remaining gaps in meeting the Istanbul Convention's standards for specialist support. The 2024 data reaffirm the central role of feminist women's centres in ensuring effective, survivor-centred responses to VAWG.

Findings on the situation of women's centres in Europe

Women's centres providing non-residential specialist support for survivors of VAWG were reported in 44 of the 46 European countries covered by WAVE. Only **Belarus** and **Hungary** remain without any such provision. According to data from WAVE members, at least 3,401 women's centres operate across Europe: 2,510 in EU member states and 891 in non-EU countries. This represents an overall decrease compared with the previous report, which reported a total of 4,085 women's centres (2,995 in the EU, 1,090 in non-EU countries).

	Europe (46)	EU Member States (27)	European countries outside the EU (19)
 Total female population	439,847,480	232,087,594	207,759,886
Number of women's centres	3,401	2,510	891
Number of women's centres needed	8,797	4,642	4,155
Number of women's centres missing	5,396	2,132	3,264
Number of female population per centre	129,329	92,465	233,176

Table 9: Overview of service provision – women's centres in Europe

Applying the Council of Europe's recommendation of one counselling centre for every 50,000 women, the current figures indicate a continued gap. Across Europe, there is approximately one women's centre per 129,329 women, meaning that provision still falls far short of the minimum standard, and 61% of the women's centres recommended at the European level remain missing. While many countries report progress in maintaining or expanding their networks, significant geographical gaps persist, particularly in rural or remote areas.

⁷⁴ See CEDAW Committee (2017) *General Recommendation No. 35 on gender-based violence against women, updating General Recommendation No. 19*, paras. 31(b) and 32.

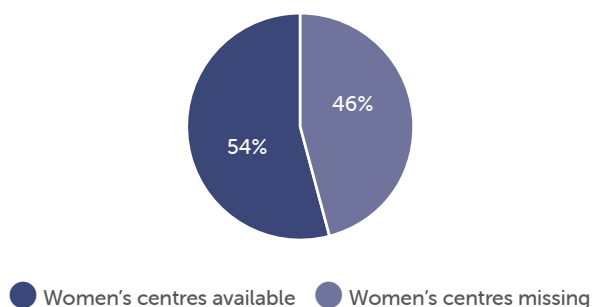


Figure 6: Percentage of women's centres available and missing in EU Member States

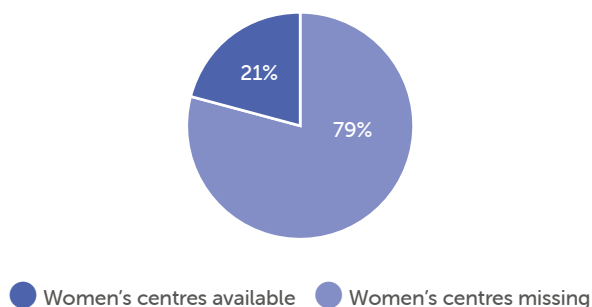


Figure 7: Percentage of women's centres available and missing in European countries outside the EU

The women's centres across Europe differ in their scope and target groups, reflecting different national policy frameworks, funding models and feminist traditions. Based on the data collected, most countries distinguish between four broad categories of provision:

- Centres serving women survivors of VAW – explicitly women-only and feminist in orientation
- Centres serving all survivors of GBV or domestic violence – open to women, men and sometimes children
- Other centres – including general victim support services or centres without a defined gender focus
- Undefined – where no information was provided on the primary target group

Across Europe, 1,425 centres (42%) work with women survivors of VAW and their children. According to the data, 710 centres (21%) are open to all survivors of gender-based or domestic violence, while a smaller group of 43 centres (1%) provide other forms of support not directly connected to VAW, and for 1,223 centres (36%), the target group was not specified in the data available.

In the Federation of **Bosnia and Herzegovina**, all eight women's centres provide support specifically for women survivors of violence, offering counselling, legal assistance, and representation at courts and other institutions.

The same is true for **Greece**, where all 44 centres operate as services for women, although this data is for state-run women's services, while data for NGO services is not available. In **Estonia**, all 19 identified centres follow this model, providing specialist, women-only support.

In **Scotland**, 36 out of 37 women's centres work only with women survivors, while in **Wales**, all 20 centres are women-only. These services are deeply rooted in feminist practice and provide comprehensive, gender-specific support, including counselling, legal advice and advocacy, often delivered through national women's aid networks.

In **Switzerland**, among the 24 women's centres, the majority serve only women survivors, with a small number open to all survivors of GBV. Each canton has a state-funded information centre for survivors of violence and gender-related issues, alongside NGO-run counselling centres specifically for women.

Of the 710 centres open to all survivors of gender-based or domestic violence, some are run by the state and some by women's NGOs. In **Finland, Denmark, Poland, Norway, Türkiye and Azerbaijan**, state-run or mixed centres provide assistance to all survivors. In **Slovakia**, women's NGOs have been legally required to offer services to all domestic violence survivors to obtain accreditation. In Germany, the network includes both feminist women's counselling centres and mixed-gender support organisations.

In **Albania**, there are twelve women's centres. Five work with women survivors of violence, and five support all survivors of GBV, while the remaining two mainly engage in advocacy activities. The women-only centres, many of which are part of the Albanian Women Empowerment Network (AWEN) or the Monitoring Network Against Gender-Based Violence, provide legal, psychological, and social support grounded in feminist practice. Centres open to all survivors, such as the Psycho-Social Centre *Vatra*, provide counselling, reintegration and advocacy services. Two other centres operate within broader social service or gender equality frameworks.

In **Denmark**, six women's centres operate nationwide: four work with women only and two are open to all survivors. *"The Mother's Aid – Out of the Shadow of Violence"* programme supports women and their children after leaving abusive relationships, offering counselling, legal advice and reintegration services. The Joan Sisters Centre provides peer-led, trauma-informed counselling for women affected by sexual violence, while Tell Someone offers individual and group counselling for survivors of partner violence and their families.

In **Finland**, there are 28 women's centres, most of which are run by the Federation of Mother and Child Homes and Shelters. Under state accreditation rules, 24 centres are open to all survivors, though women make up the vast



majority of those they work with. This system reflects Finland's integrated, gender-neutral approach to responding to violence.

In **Austria**, 65 counselling centres for women and girls operate within the national Network of Austrian Counselling Centres for Women and Girls, along with 17 independent centres. All are feminist and state-funded, providing legal, psychological and practical support across all nine federal states. In addition, 32 Violence Protection Centres work with all survivors of GBV, with women representing about 80% of those supported.

While different services can play an important role in ensuring geographical coverage, they often lack a gendered understanding of violence. WAVE members continue to express concern that gender-neutral approaches risk erasing women's specific experiences of male violence and weakening specialist feminist provision of services.

In the group of centres that provide other forms of support not directly connected to VAW, there are general 'crisis rooms' or 'violence prevention centres' that provide casework and referrals but are not specialised in VAW. In **Bulgaria**, services for survivors of violence mainly focus on domestic violence. The law defines victims broadly, including both women and men. Women's NGOs provide counselling and other support to all survivors of domestic violence, though their work primarily focuses on women. In **Hungary**, there are non-residential services for the support of survivors of domestic violence, which are called 'crisis ambulances'. Currently, there are seven crisis ambulances in the country, with regional spread. Most of them are operated by the Hungarian Interchurch Aid. These services, however, do not categorise themselves as women's centres; they are not operated by a feminist approach to violence. NANE and PATENT Associations provide in-person victim support services with a feminist approach, but they do not operate as centres; their scope and outreach are determined and limited by available human and financial resources. The 'Integrated Client Service' – a joint NANE-PATENT initiative – offers trauma-aware support to survivors: legal assistance, psychological and social counselling, provided by a team of experts who regularly consult with one another to ensure balanced and coordinated cooperation. The 'Helper's Network' – also a joint NANE-PATENT initiative – was operational in 2024 and provided victims/survivors with technical and practical support for legal and administrative processes, in a trauma-informed and victim-centred manner.

Entities running women's centres

Women's centres across Europe are managed by a diverse range of entities, reflecting national policy frameworks, funding systems and institutional traditions. According to WAVE's data collection for this report, five main types of service providers operate across the region: women's

NGOs, other NGOs, state-run services, faith-based organisations, and other or mixed providers.

Out of a total of 3,401 women's centres reported in 2024, 562 centres (16%) are run by women's NGOs, 197 centres (6%) by other NGOs, 195 centres (6%) by state authorities, six centres by faith-based organisations, and seven centres by other or mixed entities. For 2,434 centres (72%), no information was provided on the managing entity.

Women's NGOs remain the leading providers of women's centres across Europe. They manage more than half of all identified centres and are the leading providers in **Austria, Croatia, Italy, Portugal, Slovenia, Spain, Serbia, North Macedonia, and Kosovo**, where services are defined as feminist, women-only and survivor-centred. In these countries, women's NGOs offer integrated services that combine counselling, legal and practical support, and advocacy and empowerment. For instance, the Austrian Association of Autonomous Women's Shelters and the Central Information Office of Autonomous Women's Shelters (*Zentrale Informationsstelle Autonomer Frauenhäuser, ZIF*) in Germany both reported operating extensive non-residential counselling and outreach services led by feminist professionals.

In **Germany and Sweden**, a mix of women's NGOs and larger social welfare organisations operate centres, reflecting the decentralised structure of service provision. Respondents indicated that women's NGOs often provide specialist, feminist services within these broader systems. State-run services account for 603 centres (18%) across Europe. These are most common in **Finland, Denmark, Poland, Türkiye and Azerbaijan**, where governments have established national networks of crisis response, violence prevention or family support centres. Respondents from these countries confirmed that such centres usually serve all survivors of violence and are managed by public authorities such as ministries or municipal social services. In **Türkiye**, for example, the state network of Violence Prevention and Monitoring Centres (*ŞÖNİMs*) operates in every province, while in **Finland and Denmark**, crisis centres are largely coordinated through local municipalities.

Faith-based organisations manage six centres in **Albania, Romania and Slovenia**. A further seven centres are operated by other or mixed entities, including four such centres in Iceland.

For the majority of centres (72%), the managing entity was not reported, reflecting ongoing limitations in national data monitoring systems. The 2024 findings indicate that while feminist women's NGOs continue to lead the provision of gender-specific, survivor-centred support, there is also a growing presence of state-managed and mixed-service models. This diversification expands geographical coverage but can create challenges in maintaining gender competence and consistent quality standards across Europe.

Services provided by women's centres

Women's centres across Europe provide a wide range of specialist, non-residential services that address the diverse needs of survivors of VAWG. Data collected for the WAVE Country Report 2025 show that most countries have at least one women's centre offering counselling, legal and practical support, crisis intervention and community outreach. Although the level and quality of provision vary, women's centres remain central to ensuring survivors' safety, empowerment and recovery.

It is important to note that the figures below refer to the existence of women's centres providing these services within each country, not to the proportion of all individual centres that offer them. In other words, a 'Yes' response means that the service exists in that country, not that every identified women's centre in the country provides it.

Service	Yes (countries)	No (countries)	Do not know
Counselling/ psychological support	39	1	6
Legal advice	38	2	6
Financial and social welfare support/ advocacy	29	6	11
Housing advice	31	4	11
Employment	22	10	14
Referrals/collaboration with other services	38	1	7
Representation at court/ police/social services/ etc.	30	7	9
Residential support services (e.g., emergency accommodation)	25	9	12

Table 10: Services provided by women's centres

According to the 2024 data, counselling and psychological support are available in 39 countries (85%), making this the most widespread service provided by women's centres. Both individual and group counselling are provided, often grounded in trauma-informed and empowerment-based approaches.

Legal advice is provided in 38 countries (83%) by at least one women's centre. Representation at court, police or social services is available in 30 countries (65%). Financial and social welfare support or advocacy services exist in 29 countries (63%), while housing advice is provided in 31 countries (67%), reflecting the importance of women's centres in helping survivors achieve safety and stability after leaving violent situations. Employment-related support

is reported in fewer than half of countries (22, or 59%), usually in the form of job counselling, skills development, or referrals to employment services.

Women's centres also play an important coordinating role. Referrals and collaboration with other services are reported in 38 countries (83%), underscoring their central role in multi-agency response systems. Residential support services, such as emergency accommodation linked to non-residential women's centres, are available in 25 countries (54%), demonstrating how integrated service models can provide continuity of care across the crisis and recovery stages. In some cases, women's shelters also act as centres and so offer both residential and non-residential support services.

The latest data collection also captured information on how many women's centres in each country provide specific services. While this level of detail was not reported consistently across all countries, the available information offers valuable insight into how services are organised and where strong integrated models exist.

In **Austria**, for example, women's centres report providing a comprehensive range of support. Counselling and legal advice are available in all centres, while many also offer advocacy, outreach and housing advice. Several Austrian centres combine individual and group counselling with longer-term empowerment programmes and play an active role in prevention work and in coordinating with local institutions.

In **Italy**, the Anti-violence centres (*centri antiviolenza*) reflect one of the most developed national networks. They provide psychological counselling, legal advice, housing and employment assistance, and proactive contact with survivors referred by the police or social services. Many centres also offer empowerment and reintegration programmes for women rebuilding their lives after leaving violent relationships.

Similarly, **Spain** has a broad and well-coordinated system of Women's Centres (*centros de la mujer*) operating at regional and municipal levels. Almost all centres offer psychological and legal counselling, and many extend their services to include institutional mediation, childcare support and awareness-raising initiatives. Coordination between local women's centres and shelters ensures that survivors receive continuous support from crisis response to long-term recovery.

In many other countries, even where detailed quantitative data are not available, descriptive information confirms that counselling, legal assistance, and practical advocacy remain the backbone of support provided by women's centres across Europe.

The data from 2024 shows encouraging progress in mapping the diversity of women's centres and the scope of



their services. Future WAVE data collection will build on this foundation, aiming for even greater consistency and comparability in reporting. This will help strengthen the evidence base for assessing compliance with Article 22 of the Istanbul Convention, which calls for immediate, short- and long-term specialist support services for survivors of VAWG.

CASE STUDY: Austria – integrated and specialist network with stable public support

Austria represents one of the most comprehensive and well-established models of women's specialist service provision in Europe. The country has a nationwide network of counselling centres for women and girls, combining state funding with the autonomy of feminist NGOs. As of 2024, the Network of Austrian Counselling Centres for Women and Girls includes 65 centres operating in all nine federal states, with at least 17 additional independent women's counselling centres.

Women's centres in Austria provide a wide range of services including counselling, psychological support, legal advice, social welfare assistance, employment counselling, housing advice and referrals to other services. Many also provide representation before courts and social institutions. Services are confidential, free of charge and available in person, by phone or online. Several centres also provide residential support or collaborate closely with shelters.

Austria's women's centres play an active role in prevention, awareness-raising and advocacy. They conduct empowerment workshops, school-based programmes and community campaigns against VAWG, often in partnership with local authorities. A new National Action Plan currently being developed includes specific funding lines for prevention.

Specialisation is another defining feature of the Austrian model. Many centres have developed expertise on specific forms of violence or particular groups of survivors. For instance, *Orient Express* and *LEFÖ* in Vienna support women affected by forced marriage and trafficking; *DIVAN* in Graz focuses on survivors of honour-based violence; and the *African Women Organisation* operates the 'Bright Future' project addressing female genital mutilation. The centre *Ninlil* provides counselling for women with cognitive and physical disabilities, while several centres explicitly welcome transgender and non-binary survivors.

Accessibility and inclusion are enshrined in Austrian legislation: since 2016, all recognised women's centres must be barrier-free.

Funding for women's centres comes primarily from the state and local governments, complemented by donations and small-scale international or faith-based support. Although funding is sometimes project-based and bureaucratic, the overall model ensures institutional stability and national coverage.

Accessibility for groups of women at the intersection of multiple forms of discrimination

Women's centres across Europe are founded on the principle of equality and non-discrimination, aiming to ensure that all women survivors of VAWG can access specialist support. Data from the WAVE Country Report 2025 show that women's centres in most countries are open to a wide range of women, although the degree of accessibility and inclusivity varies across groups.

Most countries report that women's centres are accessible to women with different backgrounds and needs. Across the 46 reporting countries, about three-quarters indicate that all or most women's centres are open to specific groups such as migrant, refugee, disabled or older women. However, barriers remain in ensuring equal access for women with disabilities, undocumented women, transgender women and unaccompanied girls, whose needs often require specialised responses or additional institutional coordination.

Access for refugee women is relatively covered, with 29 countries (63%) reporting that most or all women's centres are accessible to them. Women with uncertain residence permits or undocumented women also have some degree of access in the majority of countries, though only 25 countries (54%) confirmed that all or most centres provide support without restrictions.

In many cases, access depends on legal or funding frameworks. For example, in **Italy and Spain**, services are available to all women regardless of residence status, and staff receive training on supporting migrant and refugee survivors. In contrast, some countries provide access through specialised centres or by cooperating with NGOs supporting migrants, rather than through all women's centres.

Women with physical disabilities have access to most or all centres in 22 countries (48%), while accessibility for women with sensory disabilities (e.g., blind or deaf women) and women with cognitive disabilities (e.g., intellectual and/or psychosocial impairments) is reported at slightly lower levels (20 countries, 43%).

These differences often reflect physical barriers in older buildings, limited availability of sign language interpreters, and insufficient resources for tailored support. Nonetheless, several countries, including **Austria, Germany, and Sweden**, report targeted efforts to improve accessibility

through structural adaptations, accessible communication tools, and partnerships with disability organisations. Access for older women (aged 65 and above) is the most comprehensive among all groups. Thirty countries (65%) report that all or most women's centres are accessible to older survivors of violence. This reflects growing awareness of violence against older women and recognition that they may experience specific barriers, such as dependency on family members or economic vulnerability. In **Portugal and Finland**, for example, awareness campaigns and outreach programmes have been developed to reach older survivors through community services and health systems.

Access for transgender women varies significantly between countries. About one-third (15 countries, 33%) report that most or all centres provide support to transgender survivors. Some centres are open to everyone but do not have the training or clear procedures needed to meet different survivors' needs, while others refer transgender survivors to general victim support or LGBTQI+ organisations.

For unaccompanied girls under 18, accessibility depends largely on national child protection frameworks. Around a third of the countries (15, or 33%) report that all or most women's centres can support unaccompanied girls, often in collaboration with youth services or guardianship systems. In **Spain and North Macedonia**, for example, coordination between women's centres and social welfare institutions ensures that adolescent survivors of violence receive both protection and psychosocial support.

Specialised centres for specific forms of violence

Specialisation within women's centres reflects efforts to address the complex realities of intersecting forms of VAWG and discrimination. The 2024 WAVE data indicate that across Europe, there is a growing network of women's centres offering support based on dedicated expertise on specific forms of violence or with particular groups of survivors.

Centres specialised in trafficking in persons are the most prevalent, reported in 30 countries (65%). These centres often combine individual counselling, legal aid, and reintegration support, and many work in collaboration with law enforcement or national anti-trafficking coordinators. Strong examples come from **Austria, Romania, and Serbia**, where women's NGOs lead comprehensive anti-trafficking services, often with specialised shelters or safe houses. Specialised services for undocumented and refugee women are present in less than half the countries, in 18 (39%) and 20 (43%) countries, respectively. In **Italy and Spain**, for instance, national women's networks have developed expertise in migration-related violence, providing language support, legal advice on residence status, and access to safe accommodation. In **Germany and Sweden**, feminist organisations offer targeted counselling and

advocacy for refugee and asylum-seeking women affected by sexualised and domestic violence, often in cooperation with refugee support agencies.

Centres addressing forced marriage, honour-based violence, and female genital mutilation remain less common but significant. Twenty-two countries (48%) report specialised centres for survivors of forced marriage, while 19 (41%) have services addressing honour-based violence, and 15 (33%) for survivors of female genital mutilation. These forms of violence are most frequently addressed in Western and Northern Europe, where migration patterns have prompted targeted policy responses. In **France, the Netherlands, and Belgium**, women's centres work in partnership with migrant women's organisations to provide culturally sensitive and rights-based support.

Specialised services for Black and minority ethnic women are reported in 15 countries (33%), reflecting a continued need for intersectional approaches that address racism and structural exclusion alongside VAWG. In the **United Kingdom, Germany, and Finland**, Black and minority ethnic and migrant women's organisations have been instrumental in developing community-based advocacy and counselling that combine feminist and anti-racist frameworks.

According to the data collected, 16 countries (35%), as well as **England and Northern Ireland**, have women's centres specialised in supporting women with physical disabilities, while 15 countries (33%) report specialised services for women with cognitive or sensory disabilities. These centres often focus on accessible counselling environments, adapted communication tools and coordination with disability services. Examples include **Austria**, where specialist counsellors receive training in disability-sensitive approaches, and **Germany**, where there are women's centres specialised in supporting women survivors with physical, cognitive and/or sensory disabilities.

Centres for older women (65 years and above) are available in 18 countries (39%) as well as in **Northern Ireland and Wales**, which report dedicated services. In **Portugal and Finland**, for instance, some women's centres have integrated elder abuse prevention into their programmes, combining community outreach with support for older survivors within existing counselling services.

There is also growing recognition of the need for age-appropriate, gender-sensitive services for girls under 18. Eighteen countries (39%), as well as Wales, report specialised centres or programmes for girls, often linked to broader child protection systems. In **Spain and Croatia**, women's NGOs operate dedicated counselling and prevention projects targeting adolescent girls affected by dating violence or sexual abuse. In **Moldova**, there are 10 centres specialised in supporting girls under 18.

Specialisation for transgender women and non-binary people remains more limited, though it is expanding. Fourteen



countries (30%), as well as **Wales**, have at least one women's centre with specific training or protocols to support transgender survivors, and ten countries (22%), as well as **Wales**, report initiatives inclusive of non-binary people. These centres often collaborate with LGBTQI+ organisations to ensure safe and informed support. In **Sweden and Belgium**, feminist organisations have introduced inclusive training and adapted intake procedures to respond sensitively to survivors' gender identity and lived experiences of discrimination.

Although these specialised services remain unevenly distributed, their presence across more than half of reporting countries represents progress towards meeting the Istanbul Convention's requirement to ensure equal access to protection and tailored support. However, as in previous years, the availability of such centres is often dependent on project-based or international funding, raising concerns about their sustainability and national recognition within state frameworks. At the same time, countries should aim to go beyond the minimum requirements of the Istanbul Convention by strengthening specialised and intersectional service provision, ensuring that support effectively reaches all women, including those facing multiple and intersecting forms of discrimination.

Specialist support for survivors of sexualised violence

Sexualised violence remains one of the most underreported and traumatising forms of VAWG, and the availability of specialist, trauma-informed services is a critical indicator of compliance with the Istanbul Convention's Articles 22 and 25. European countries vary in their approach: in some cases, women's centres offer specialised holistic support for all forms of VAW, including sexualised violence, while others maintain consolidated networks of RCCs with trained staff providing immediate and long-term survivor-led counselling. To reflect these national differences, the WAVE questionnaire included the same multiple-choice question about types of sexualised violence support in both the women's centres and the sexualised violence support services sections. This section presents the results about the support provided by women's centres to survivors of sexualised violence.

Data collected for the WAVE Country Report 2025 show that women's centres in most countries provide at least some form of specialist support for survivors of sexualised violence, though the scope and quality of these services vary significantly. The most widely available service is specialist psychological care, reported in 30 countries (65%). This includes trauma therapy, counselling, and peer support groups provided by trained professionals. Countries such as **Austria, Germany, Italy, and Spain** have developed strong networks of rape crisis and sexual violence counselling centres integrated within broader women's NGO structures. In **Austria**, the women's helpline (*Frauenhelpline*) and women's shelters (*Frauenhäuser*) collaborate

with regional crisis centres to provide long-term trauma counselling, while in **Spain**, regional crisis centres (*centros de crisis 24 horas*) offer 24/7 emergency psychological and legal assistance.

Specialist advocacy services for survivors of sexualised violence are reported in 23 countries (50%), and often include accompaniment to medical and legal appointments, court support, and coordination with police and health services.

Legal advice related to sexualised violence is available in 29 countries (63%), often as part of comprehensive support packages offered by women's centres. Legal services typically include assistance in filing complaints, representation in court, and support in obtaining protection measures. However, only 10 countries (22%) report that women's centres provide specialist forensic or medical care, which remains primarily the responsibility of state or hospital-based services. In some contexts, such as **Ireland and Finland**, partnerships between women's NGOs and hospital-based sexual assault units ensure coordinated survivor care.

Needs assessment mechanisms, such as evaluating survivors' entitlement to special measures in court or in asylum processes, are in place in 20 countries (43%). These services are crucial in ensuring that survivors' safety and psychological well-being are prioritised throughout legal and administrative procedures.

Finally, community education and prevention work related to sexualised violence is conducted in 26 countries (57%). Many women's centres engage in prevention campaigns, often targeting young people and professionals. Examples include the 'No is No' (*No es No*) campaign in **Spain**, and school-based sexual violence prevention programmes implemented by women's centres in **Croatia and Italy**.

Despite these positive examples, the overall picture shows that comprehensive sexual violence response systems remain uneven. Many countries lack integrated forensic and medical services linked to feminist counselling centres, and the sustainability of specialist psychological and advocacy services is often dependent on short-term funding. Ensuring a consistent, nationwide network of trauma-informed, survivor-centred services, therefore, remains an urgent priority for compliance with the Istanbul Convention's standards.

Funding and geographical coverage of women's centres

Women's centres across Europe continue to rely on a combination of state funding, donations and international funding to sustain their work. The data collected show that the majority receive some form of state funding, whether from national, regional or local authorities. In 36 out of 46 countries, public financing represents the main source

of support. In several countries, including **Austria, Spain, Italy, Portugal and Finland**, such funding has enabled the development of stable national or regional networks of women's centres. However, even where state funding is well established, it is often project-based or short-term, requiring frequent reapplication and limiting service continuity and staff retention.

Donations and international funding continue to play an important role, with 34 of 46 countries reporting receiving donations and 25 receiving international funding. This is particularly important for feminist NGOs operating in political or financial environments that restrict public funding for gender equality. In the Western Balkans, Eastern Europe and the South Caucasus, many women's centres rely on grants from the European Union, United Nations agencies or private foundations. This is especially the case in **Serbia, Kosovo, Georgia and Armenia**, where international support has been crucial in maintaining feminist service provision in the absence of adequate state backing.

Only a small number of countries report funding from religious institutions, notably **Albania, Austria, France, Germany, Portugal, Switzerland and Ukraine**.

Overall, the funding landscape reveals a clear imbalance: feminist NGOs that offer the most specialised, gender-specific and empowerment-based support are often the least financially secure. Meanwhile, larger state-run or generic providers tend to benefit from more stable public budgets, even when their services are less specialised or gender-sensitive.

The geographical distribution of women's centres remains uneven across Europe, creating persistent disparities in women's access to specialist support. Seventeen countries (37%) report that women's centres are available in all regions, a situation found mainly where there is a strong national coordination mechanism or statutory funding model, as in **Austria, Spain, Italy, Sweden and Portugal**, but also in smaller countries such as **Croatia and Montenegro**. In a further group of thirteen countries (28%), women's centres operate in most regions, while in ten countries (22%) the centres operate only in major cities or only in the capital city in the case of **Cyprus**.

In many parts of Eastern and South-Eastern Europe, provision is concentrated in major urban centres, leaving survivors in rural, island or mountainous areas without in-person access to counselling or legal assistance. A few smaller or more centralised states report that women's centres exist only in the capital city, which severely limits accessibility, particularly for women who face financial or mobility constraints, or who need to maintain confidentiality when seeking help.

While the overall network of women's centres continues to expand, geographical inequalities persist. Several countries are attempting to address these gaps through mobile

outreach, online counselling and local referral mechanisms. Examples include **Finland, Croatia, and Spain**, where women's NGOs have developed regional outreach models and worked with local authorities to expand access for women outside major cities. However, without stable, long-term funding and decentralised service planning, achieving genuine national coverage remains difficult.

CASE STUDY: Serbia – feminist-led provision sustained through international support

Serbia provides an instructive example of a feminist NGO-led system functioning with limited state involvement and significant reliance on international and donor funding. As of 2024, there are 42 women's centres in Serbia, of which 32 are run by women's NGOs and 10 by other NGOs. Among them, 25 serve women survivors of violence, while 17 provide services to all survivors regardless of gender.

Women's centres in Serbia offer counselling and psychological support, referrals and coordination with other services, legal advice, and advocacy. Some also provide representation before courts and social services, as well as financial or social welfare support. Specialist services for survivors of sexualised violence are available in a small number of centres, including needs assessment, psychological care and advocacy.

The network is geographically uneven: most centres are concentrated in cities, while some regions have no centres at all. This urban concentration reflects the absence of systemic state funding and coordination. Instead, women's NGOs sustain their work largely through international donors and project-based grants. The principal funding sources are international foundations, UN programmes and bilateral donors, with only minimal state contributions.

Despite financial insecurity, women's centres in Serbia demonstrate strong feminist principles and survivor-centred approaches. They offer essential support in a context of limited public infrastructure, bridging the gap between institutional systems and women's actual needs. Many centres also engage in prevention and empowerment activities, including running workshops, awareness campaigns, and advocating for policy reform.

A few centres provide specialist or intersectional services, including to women with uncertain residence permits or undocumented women, women with physical and sensory disabilities, and Black and minority ethnic women and survivors of forced marriage. Some centres also support refugee women, transgender women, non-binary people and girls under 18, though in very small numbers.

Women's centres in Serbia have a crucial role in a system where public services often lack the necessary gender competence or accessibility. However, without state-backed financing or national coordination, sustainability remains a major challenge.

Conclusion

The data collected for the WAVE Country Report confirm that women's centres remain the cornerstone of specialist, non-residential support for survivors of VAWG in Europe. Despite diverse governance models, their defining feature is the provision of gender-specific, feminist and empowerment-based services. Across 46 countries, women's centres continue to ensure that survivors are believed, supported and empowered, yet their reach, sustainability and recognition vary considerably.

Progress since 2023 is visible in the gradual expansion of networks, especially in countries with stable public funding frameworks such as Austria, Spain, and Italy. The persistence of feminist NGO-led provision in regions such as the Western Balkans demonstrates the resilience and expertise of civil society, even where state involvement is limited. However, the overall availability of centres still falls short of the Council of Europe's recommended standard of one counselling centre per 50,000 women, with coverage remaining uneven and rural areas particularly underserved. The findings highlight an ongoing tension between the institutionalisation of services and the preservation of feminist principles. While state-run or mixed models have improved geographical reach, they often lack gender-specific understanding and survivor-centred methodologies. Feminist NGOs, which provide the most comprehensive and empowering forms of support, continue to face precarious, project-based funding, limiting their ability to retain staff and maintain quality standards.

The data also reveal growing diversification in the services provided. Counselling, legal support and advocacy remain universal, but more centres now offer specialist services for survivors of sexualised violence, trafficking, forced marriage and other intersecting forms of discrimination. There is visible progress in accessibility for women with disabilities, migrant and refugee women, and older women, though significant gaps remain for undocumented and transgender women as well as unaccompanied girls. Overall, women's centres continue to bridge the gap between survivors and institutions, translating the principles of the Istanbul Convention into practice. Their future sustainability depends on stable and adequate funding, state recognition of feminist expertise, and continued advocacy to ensure that every woman in Europe has access to specialist, rights-based, and survivor-centred support.

❖ Recommendations

- Women's specialist services should develop a shared definition for 'women's centres' grounded in feminist, gender-specific, rights-based and survivor-centred principles and methodologies. A shared definition would aid clarity and consistency in data collection and facilitate a more accurate representation of specialist services in line with the Istanbul Convention.
- Governments should guarantee institutional, multi-year core funding for feminist women's centres to ensure long-term sustainability, staff retention and high-quality support. Public funding should be prioritised for feminist NGOs applying gender-specific, survivor-centred methodologies, and procurement systems should include gender competence and feminist principles, not only organisational size or administrative capacity.
- States must strengthen cooperation and partnerships between public authorities and women's NGOs. Efforts should focus on expanding the provision of specialist services for women survivors and their children, particularly in countries or regions where such centres are absent or limited.
- States must establish and maintain comprehensive, national data collection systems that capture available women's centres and service providers, geographical coverage, target groups, and number of survivors supported, ensuring public accessibility and reducing the burden on under-resourced feminist civil society organisations.
- The availability of adequate specialist services for minoritised and marginalised groups must be ensured, including support for survivors of forced marriage, honour-based violence, female genital mutilation, human trafficking, and for refugee, migrant and undocumented women, which remains non-existent or very limited in many countries.
- States must establish comprehensive geographical coverage of women's centres, including coordinated outreach in rural, remote and island areas where in-person services are scarce.
- States should promote the integration of intersectional and trauma-informed approaches in service provision through training, quality standards and monitoring in order to strengthen gender-specific and survivor-centred practice.



Specialist services for survivors of sexualised violence



Introduction

Sexualised violence is one of the most pervasive and harmful forms of violence against women and girls (VAWG), causing deep and lasting trauma to women and girls across Europe and beyond. It encompasses a continuum of behaviours that violate physical, psychological, and sexual integrity. It includes rape and sexual assault, coerced or unwanted sexual acts, sexual harassment, sexual exploitation, forced or non-consensual sexual touching, non-consensual image sharing, online sexual abuse, stalking with sexual intent, grooming, and child sexual abuse. It can occur in intimate relationships, within families, in workplaces and institutions, in public spaces, online, and in situations of conflict, displacement, or detention.

The prevalence of sexualised violence in Europe remains alarming. According to the latest European Union gender-based violence (EU GBV) survey, around 30% of women in the EU (one in three) have experienced physical and/or sexual violence since the age of 15, and approximately 17% of women report having experienced sexual violence (including rape and other unwanted sexual acts).⁷⁵ Sexualised violence is often experienced at an early age: one in four women reports some form of sexual harassment before the age of 15, and young women are disproportionately affected by online and image-based abuse. These figures are alarming, yet despite its gravity and the long-term traumatic consequences for survivors, sexualised violence remains among the most underreported of all crimes. Shame, fear of disbelief, stigma, and distrust in institutions frequently prevent survivors from seeking help or justice.

Sexualised violence is not a random or private act but a deeply gendered violation that reflects and reinforces unequal power relations. It is in the overwhelming majority of cases committed by men against women and girls. On a societal level, it functions as a mechanism of control, intimidation, and subordination. Feminist research has long shown that the threat and reality of sexualised violence shape women's daily lives by restricting their freedom of movement, limiting participation in public life, and undermining equality. Liz Kelly's concept of the *continuum of sexual violence* further demonstrates that women's experiences of sexualised aggression, from seemingly minor acts of harassment to rape, are connected through the same structures of male power and control. This framework

exposes how everyday intrusions and coercive behaviours sustain an environment in which more severe forms of sexualised violence can occur.⁷⁶

Central to the issue of sexualised violence and its definition is the concept of consent. The Istanbul Convention defines sexual violence, including rape, as any sexual act without the freely given consent of the victim. Consent must be given voluntarily, as the result of free will, assessed in the context of the surrounding circumstances. It cannot be inferred from silence, passivity, or lack of resistance.⁷⁷

Despite notable legislative developments, international standards, and case law emphasising the need to integrate consent-based definitions and adopt an affirmative consent approach (the yes-means-yes model), in practice, the presumption of innocence continues to place a heavy burden of proof on the prosecution, and indirectly on the survivor. Whereas the focus once lay on proving active resistance, survivors are now expected to demonstrate the absence of consent. This requirement remains challenging, often humiliating, and re-traumatising for survivors of sexualised violence. The situation is further exacerbated by persistent myths and stereotypes surrounding sexualised violence, which lead to intrusive questioning of survivors' behaviour, credibility, and character. Although the phrasing of such questions may have evolved since the introduction of consent-based definitions, their underlying implications of blame remain unchanged. Consequently, the responsibility continues to fall on the survivor to prove that their reaction during an assault did not constitute consent, and ultimately to demonstrate their own victimisation.

Understanding the centrality of consent means recognising that survivors often do not resist physically, as trauma and fear can trigger freezing or dissociation responses. Legal and institutional responses that fail to understand these dynamics risk reproducing myths and stereotypes that blame survivors for violence perpetrated against them. The move towards consent-based legal definitions across Europe represents progress in aligning law with lived experience. Yet this must be matched by specialist services that support survivors holistically, uphold their autonomy, and ensure that consent and bodily integrity are respected in all interventions. Survivors of sexualised violence have a range of immediate and long-term needs that require coordinated, specialist, and trauma-informed

⁷⁵ FRA, EIGE, Eurostat (2024), *EU gender-based violence survey – Key results. Experiences of women in the EU-27*, Publications Office of the European Union, Luxembourg.

⁷⁶ See Kelly, L. (1988). *Surviving Sexual Violence*. Cambridge: Polity Press.

⁷⁷ See Council of Europe, *Convention on preventing and combating violence against women and domestic violence* (Istanbul Convention), Article 36.



responses. Immediately after an assault, women may need urgent medical care, psychological first aid, and forensic examination, where available. In the long term, many require trauma counselling, peer support, specialised legal information, and advocacy to navigate systems that often re-traumatise or intimidate them.⁷⁸

Above all, survivors need to be believed, supported, and treated with dignity.⁷⁹ When survivors receive insensitive or discriminatory responses from police, prosecutors, medical staff, or family members, they may experience secondary victimisation, which deepens the trauma of the original assault. Specialist services are designed to prevent this by offering informed, survivor-centred, and gender sensitive support. Survivors also need systems that are accessible to all, including women living in rural areas, migrant and refugee women, women with disabilities, Roma women, and other marginalised groups.⁸⁰

According to WAVE definitions, specialist services for sexualised violence include several types of provision, each addressing different aspects of survivors' needs.

Rape crisis centres (RCCs) are specialist services offering immediate, medium-, and long-term support to survivors of rape, sexual assault, or any form of sexualised violence. Their work is rooted in feminist, survivor-centred principles. They provide counselling, therapy, advocacy, and accompaniment throughout legal proceedings and in contact with other institutions. RCCs also engage in community education, awareness-raising, and inter-agency cooperation to improve institutional responses and prevent sexualised violence. Many operate specialised helplines that provide counselling, information on rights, and referrals to other services.

Sexual violence referral centres (SVRCs) usually focus on the immediate aftermath of an assault. Often based in hospitals, they provide emergency medical and forensic care, evidence collection and storage, crisis intervention, and referrals to longer-term community-based services such as RCCs.

Crisis or medical services offer urgent medical treatment, emergency contraception, prevention of sexually transmitted infections, and trauma-informed psychological support.

Specialised helplines for sexualised violence provide confidential, accessible, and immediate emotional support, information, and referral. They are often the first point of contact for survivors who are not yet ready to engage in person.

Many women's centres also provide counselling, advocacy, and accompaniment to survivors of sexualised violence as part of a broader holistic response to VAWG. In countries where RCCs or SVRCs do not exist, these centres often fill critical gaps, even without formal recognition or funding.

Historically, specialist services for sexualised violence were pioneered by women's organisations grounded in feminist theory and activism. They were created not as extensions of state systems but as acts of resistance to a culture that silenced survivors and normalised male violence. The feminist model sees survivors not as passive recipients of aid but as agents in their own recovery. It understands sexualised violence as a social and political problem rooted in gender inequality, and its aim is not only individual healing but systemic transformation.

Key principles of feminist specialist services include belief, autonomy, confidentiality, and empowerment. They use trauma-informed methods that understand the complexity of trauma responses and the importance of restoring control and trust. They provide both direct support and advocacy for structural change. It is essential for feminist services to maintain independence to fulfil this role effectively. While cooperation with state institutions is necessary to ensure coordination and funding, independence allows them to criticise systemic failings and defend survivors' rights without political or institutional pressure.

International obligations

The Council of Europe Convention on Preventing and Combating Violence against Women and Domestic Violence (the Istanbul Convention) establishes that all non-consensual sexual acts must be criminalised (Article 36) and that states must provide accessible, specialised support through RCCs or SVRCs (Article 25). These services must be available regardless of whether survivors report the crime, ensuring that support and justice are not conditional on legal action. It further recommends that at least one RCC or SVRC should be available per 200,000 inhabitants and that they should be geographically distributed to ensure accessibility, including for women in rural and remote areas.⁸¹ Despite these standards, the current provision across Europe remains far below the recommended level, with significant disparities in access, quality, and sustainability. The Group of Experts on Action against Violence against Women and Domestic Violence (GREVIO), in its midterm horizontal report, found that while immediate medical and trauma care for sexual violence is generally well developed across parties, long-term psychological counselling remains insufficient. Yet such long-term services are essential, as many victims do not seek help immediately due to stigma, shame, guilt, fear of not being

⁷⁸ See WAVE Network (2019) *Mapping of Sexual Violence Services in the Western Balkans and Turkey*.

⁷⁹ Ibid.

⁸⁰ Ibid.

⁸¹ See Council of Europe (2011) *Explanatory Report to the Council of Europe Convention on preventing and combating violence against women and domestic violence*, para. 138.

believed, or the emotional difficulty of disclosing sexual violence. As a result, the effects of trauma often surface weeks, months, or even years later, making sustained psychological support crucial for recovery and for addressing the long-term consequences of sexual violence.⁸²

The Convention on the Elimination of All Forms of Discrimination against Women (CEDAW) has played a defining role in framing sexualised violence as a violation of women's human rights. In its General Recommendation No. 35 on gender-based violence against women, the Committee explicitly requires States to ensure that sexual assault, including rape, is recognised *"as a crime against the right to personal security and physical, sexual and psychological integrity and that the definition of sexual crimes, including marital and acquaintance or date rape, is based on the lack of freely given consent and takes into account coercive circumstances"*.⁸³

As recognised in the Directive (EU) 2024/1385 on combating violence against women and domestic violence, *"the traumatic nature of sexual violence, including rape, requires a particularly sensitive response by trained and specialised staff"*. The Directive further underlines that *"victims of sexual violence need immediate trauma support combined with immediate forensic examinations for the safe-keeping of evidence needed for future prosecution"*. It calls on Member States to ensure that *"rape crisis centres or sexual violence referral centres should be available in sufficient numbers and adequately spread over the territory of each Member State, taking into account the geography and demographic composition of the Member State concerned, and notes that such centres can form part of the existing healthcare system in the Member State"*.⁸⁴ These provisions reaffirm the obligation of EU Member States to provide accessible, specialist, and survivor-centred services for all women and girls survivors of sexualised violence. The Directive emphasises accessibility for survivors with disabilities, migrant and refugee women, and those in rural areas, and underlines that services must be available irrespective of whether survivors report the crime. It is important to note, however, that while the Directive indeed establishes an important framework, significant gaps remain, specifically the absence of a consent-based definition of sexual violence and insufficiently detailed requirements on the nature and scope of support services. While the Directive broadens access to general victim services, it risks relegating specialist women's services to a subordinate role rather than ensuring direct access to long-term, tailored support.⁸⁵

WAVE framework for data collection on sexualised violence services

For the 2025 report, WAVE gathered information on specialist services for survivors of sexualised violence, asking whether services for women and girls survivors of sexualised violence exist in each country, and which types are available, including RCC, SVRC, crisis or medical services, and specialised helplines. Respondents were asked to provide the number of each type, their availability, and their location. The questionnaire also explored the kinds of support these services offer, including medical and forensic care, trauma counselling, advocacy, legal advice, and community awareness work.

Information was also collected on how services are funded, where they are distributed geographically, and which groups of survivors they support, including women, girls, and other affected groups. Respondents could add comments about country-specific systems, such as whether women's centres offer support to survivors of sexualised violence or where forensic evidence is collected. This allows for consistent comparison between countries while recognising national differences in service models.

Findings on the situation of specialist services for survivors of sexualised violence in Europe

The data show a severe shortage of specialist services for women and girls survivors of sexualised violence across Europe. To meet the Istanbul Convention standard of one RCC or SVRC per 200,000 inhabitants, 4,248 services would be required across 46 countries. However, only 633 centres are currently reported, leaving a shortage of 3,615 services, or 85% of the total required.

In the 27 EU Member States, 2,243 centres are needed, yet only 334 exist. This leaves a shortage of 1,909 centres, meaning that 85% of the required services are not in place. In the 19 European countries outside the EU, there are 299 centres compared to the 2,005 required, leaving an 85% gap. Data on the availability of sexual-violence-specific services remain incomplete, as not all countries were able to provide figures, meaning the actual number of services is likely higher. This is particularly evident in contexts such as Germany, where services exist but no comprehensive data could be obtained; however, these gaps do not substantially alter the overall finding of a significant shortage of specialised support.

82 See Council of Europe (2023) *Mid-term Horizontal Review of GREVIO Baseline Evaluation Reports*, p. 86, para. 285.

83 See Committee on the Elimination of Discrimination against Women (2017) *General recommendation No. 35 on gender-based violence against women, updating general recommendation No. 19*, para. 29(e).

84 See Directive (EU) 2024/1385 of the European Parliament and of the Council on combating violence against women and domestic violence, Preamble, recital 64.

85 See WAVE Network (2024) *WAVE Statement on the Proposed EU Directive on Combating Violence Against Women and Domestic Violence*.



	Europe (46)	EU Member States (27)	European countries outside the EU (19)
Total population	849,677,812	448,696,087	400,981,725
Number of services required to meet Istanbul Convention Standard	4,248	2,243	2,005
Number of services available	633	334	299
Number and percentage of services missing	3,615 (85%)	1,909 (85%)	1,706 (85%)

Table 11: Overview of findings on service provision – sexualised violence support services

These figures reveal a severe and consistent failure to meet international standards for service provision. In practical terms, this means that most women and girls in Europe do not have access to an RCC or SVRC in their area. The data underline the urgent need for states to prioritise the establishment, expansion, and sustainable funding of specialist, feminist services so that survivors everywhere can receive the support, safety, and justice they are entitled to under the Istanbul Convention.

Of the 46 countries covered by the data collection, only **Luxembourg, Iceland, Norway, and Switzerland** meet or exceed the required level of one RCC or SVRC per 200,000 inhabitants. This represents just four out of 46 countries (9%). In addition, **Wales** meets the standard within the **United Kingdom**, although when the United Kingdom is considered as a whole, it falls short, with 65% of the services missing. Both **Norway and Switzerland** significantly exceed the Istanbul Convention standard. Norway operates 46 centres, compared with the 28 required by its population, while **Switzerland** has 63 centres, far surpassing the 45 needed to meet the minimum standard.

RCC or SVRC have not been established in **Austria, Cyprus, Hungary, Latvia and Türkiye**. On the other hand, **Greece, Italy, Slovakia, Azerbaijan, Belarus, Georgia, Kosovo, Liechtenstein, and Montenegro** have no specialist sexualised violence services at all. This means that nine out of 46 countries (20%) provide no support through specialist sexualised violence services. However, in these countries, other women's specialist services (WSS) provide support to survivors of sexualised violence.

Types of sexualised violence services

Across Europe, the data show that provision of specialist services for survivors of sexualised violence remains fragmented and uneven. A total of 208 RCCs and 221 SVRCs were reported across 46 countries. In addition, there are 204 crisis or medical services and 50 specialised helplines providing support to survivors. The distribution of these services is uneven across Europe: 14 countries have no RCCs or SVRCs, and another 14 have only two or fewer. Both Norway and Switzerland exceed the number required by the Istanbul Convention. In **Norway**, there are 22 Sexual Assault Reception Centres (*Overgrepsmottak*) and two Rape Clinics (*Voldtekstmottaket*), located within hospitals across the country. These centres function as RCC or SVRC, providing immediate medical care, psychological first aid, and forensic examination. Additionally, there are 21 Centres for Victims of Incest and Sexual Abuse (SMI-SO/NOK), which offer long-term psychological and peer support for survivors of incest, sexual abuse, and rape, as well as for their relatives.

Among countries with larger numbers of services, most have prioritised the development of either RCCs or SVRCs, rather than both. In **Switzerland**, there is one RCC, around 60 SVRCs, and two crisis/medical services. In **Belgium**, there is also one RCC and 10 SVRCs. WAVE delegates explained that the classification of services “does not entirely fit the reality on the ground.” The NGO SOS Viol in Brussels provides long-term counselling and support for survivors, while the state-run Sexual Assault Care Centres in **Brussels and Wallonia**, and the Sexual Violence Care Centres (*Zorgcentrum na Seksueel Geweld – ZSG*) in **Flanders**, offer 24/7 comprehensive medical, forensic, and psychological care. These hospital-based centres represent a strong integrated model, although the NGO-led RCC remains the only one offering long-term therapeutic support independent of the health system.

In **Finland**, there is one RCC and 25 SVRCs. The SERI Support Centres form a coordinated, state-funded national structure providing immediate medical, forensic, and psychological support, regardless of whether survivors choose to report to the police. Experts describe this as a coherent and trauma-informed national model integrated into the healthcare system. In **Lithuania**, five major regional hospitals are responsible for providing specialised services, including medical care, forensic services, evidence storage, and crisis intervention. However, the system is still in the process of formation, and many hospitals do not yet provide all necessary services. While survivors receive treatment for physical injuries and prevention of sexually transmitted diseases and pregnancy, psychological support is limited, especially during nights and weekends. The Specialised Complex Assistance Centres (SKPC) also offer psychological and intermediary assistance to survivors of domestic and sexualised violence, and there are 14 organisations running these centres across the country. In **North Macedonia**, three SVRCs operate in Skopje, Tetovo, and

Kumanovo. They operate within the general healthcare system and lack dedicated state funding. They provide medical, psychological, and legal assistance but face significant challenges in accessibility and sustainability due to insufficient funding. In **Romania**, Intervention Centres for Victims of Sexual Violence were established in 2021 and are based in hospitals, where survivors can receive medical services and forensic evidence collection. Access begins through emergency units, after which the coordinator activates a multi-agency response, including the police and social services. Survivors can then access free psychological and legal counselling, temporary accommodation, and social support. In practice, however, the system remains bureaucratic, inconsistent, and often depends on the initiative of individual professionals. In **Serbia**, five SVRCs provide medical, psychological, and legal support, while women's organisations deliver longer-term counselling and advocacy.

Several countries have opted for developing more RCCs. For example, in **Spain**, by June 2025, 54 RCCs operating 24/7 were expected to be established nationwide. According to NGO sources, at least 23 centres are currently operational. **Estonia** has five RCCs located in regional hospitals, providing primarily specialist forensic and medical care for survivors of sexualised violence. The **Netherlands** has 16 RCCs operating as Sexual Assault Centres, offering comprehensive support that includes specialist forensic and medical care, psychological services, advocacy, legal advice, and prevention activities. At these centres, multidisciplinary teams of medical, psychological, social, and law-enforcement professionals work together to provide coordinated specialist care to survivors.

England, on the other hand, has taken an approach to develop both RCCs and SVRCs, albeit still in insufficient numbers. It provides a relatively wide network of sexualised violence services, with 35 RCCs and 48 SVRCs (SARCs) across the country. SARCs handle forensic and medical care and work in coordination with police and women's services, while RCCs and other organisations offer psychological support, advocacy, legal guidance, and prevention activities. Despite the range of services, provision varies significantly between regions, and support for children and rural areas remains limited.

In countries where there are no reported RCCs, SVRCs or even specialist sexualised violence services, WSS still provide them. For example, **Italy** has a longstanding tradition of feminist women's centres which provide a full range of specialised services for survivors of all forms of VAW. The centres belonging to D.i.Re – National Women's Network against Violence (*Donne in Rete contro la Violenza*) are able to offer full support to sexualised violence survivors, including cooperation with the national health system's forensic and medical care procedures, as required by law. In **Italy**, survivors are referred to the local women's centres that are run by women's NGOs and provide specialist and competent support for cases of sexualised violence.

Women's centres can accompany women during medical procedures and judicial proceedings, offer needs assessment, specialist advocacy services and community awareness and education for prevention. Specialist forensic and medical care is provided in medical services. In the case of **Liechtenstein**, which has a total population of about 40,000 inhabitants, women survivors of sexualised violence are referred to facilities in **Switzerland**. In **Hungary**, there are no RCCs or SVRCs. Women survivors of sexualised violence can access basic support through general crisis centres or feminist NGOs, such as NANE and Patent, which offer information, legal advice, and emotional assistance. Forensic examinations are conducted in hospitals upon police request, without dedicated or trauma-informed structures. In **Greece**, there are no specific services, so while women's centres can provide counselling to survivors of sexualised violence, they do not provide specialist support.

CASE STUDY: Ireland

Ireland offers a comparatively strong and evolving model of support for survivors of sexualised violence, with 17 RCCs and six sexual assault treatment units (SATUs) providing a broad range of specialist services. RCCs deliver psychological support, advocacy, legal information, and prevention work, while SATUs ensure high-quality, state-funded clinical and forensic care. Survivors can access RCC services anywhere in the country, reflecting a flexible, survivor-centred approach, even as regional differences in resources and capacity persist.

A notable strength of the Irish system is its integration of formal and community-based supports, including vital grassroots organisations that reach marginalised groups. National helpline provision is robust, with a 24/7 state-funded service complemented by regional and specialist NGO helplines. While challenges persist, particularly in rural accessibility and the uneven availability of court and police accompaniment, the overall structure demonstrates a strong commitment to comprehensive, survivor-centred care and represents a good practice model for further development.

Across Europe, few countries provide the full range of integrated support required under the Istanbul Convention. Twenty-nine countries (63%) reported offering specialist forensic and medical care, and 31 (67%) conduct needs assessments, for example, in relation to protection measures or court procedures. Specialist psychological care is the most common form of support, available in 35 countries (76%), while 27 (59%) provide sexualised violence advocacy services and legal advice. Twenty-eight countries (61%) reported engaging in community awareness and prevention activities.



Regardless of whether countries have prioritised RCCs, SVRCs, or other models, those with specialised sexual violence services generally offer a similar range of support. All six key service types are available simultaneously in 15 countries (33%), including those with a stronger focus on RCCs, such as **France, Ireland, and the Netherlands**, as well as those that primarily operate SVRCs, such as **Belgium, Denmark, Switzerland, and Sweden**. In **Germany**, forensic evidence is collected in specialised violence protection clinics (*Gewaltschutzambulanzen*) or hospitals, depending on regional availability. The nationwide network BAG FORSA coordinates feminist counselling centres that deliver psychological care, advocacy, and prevention work, though medical and forensic services are often separate.

In **France**, Women's Houses (*Maisons des Femmes*), university hospitals, and women's rights centres jointly deliver services including medical and forensic care, psychological counselling, and legal information. However, long-term support remains limited, as most interventions focus on acute cases. In **Malta**, the Care for Victims of Sexual Assault (CVSA) operates 24/7 within the main hospital, providing medical care, counselling, and legal guidance. In **Moldova**, pilot centres integrate forensic, medical, psychological, legal, and advocacy support, offering a holistic and trauma-informed approach. However, access is limited to urban areas, and there is an ongoing need for sustainable funding and expanded outreach.

Funding of sexualised violence services

Across Europe, state funding remains the primary source of financing for sexualised violence services, reported by 31 countries (67%). However, reliance on unstable and short-term funding sources remains widespread. Twenty-two countries (48%) reported using donations from foundations, trusts, or individuals, while 12 (26%) reported receiving international funding. Only one country, **Ukraine**, reported receiving support from religious institutions, while two countries, **Spain and Ukraine**, identified additional or mixed funding sources.

In many countries, state funding is limited to covering basic operational costs and is often project-based rather than sustainable. This leaves services vulnerable to political and economic fluctuations. For example, in **Germany**, RCCs and counselling services such as *Wildwasser* rely heavily on donations to compensate for inadequate institutional funding. Similarly, in **Scotland**, RCCs are primarily funded by the national government, but this model is under threat as authorities consider shifting to localised, commission-based funding.

In contrast, some countries have developed more stable and structured funding models. In **Belgium**, the establishment and expansion of Sexual Assault Care Centres (*Centres de Prise en Charge des Violences Sexuelles – CPVS*) are supported by federal funding from the Secretary of State for Equality and by the national lottery. The 2024 law on

CPVS formalised state responsibility for funding. In **Lithuania**, the five designated hospitals providing forensic and medical care for survivors are financed by state funds. The Ministry of Social Security and Labour also supports the specialised assistance centres (SACs), ensuring continuity of psychological and social support services. However, there is no separate funding stream specifically for sexual violence services.

Several countries continue to depend heavily on international donors. In **Moldova**, for instance, specialised services receive significant support from the European Union, UN Women, UNFPA, and USAID, which fund infrastructure, training, and capacity building. Similarly, projects in Serbia and Romania have been supported through international grants such as Norway Grants and the UN Trust Fund to End Violence against Women.

At the same time, gaps in public investment remain a significant obstacle to comprehensive service provision. In **North Macedonia, Romania, and Poland**, centres often operate without dedicated state funding or rely on health-care institutions' limited budgets, leading to inconsistent or interrupted service delivery. Survivors who are uninsured or lack documentation may face additional barriers to medical assistance when specific funds are not allocated.

CASE STUDY: Germany

Germany has a broad but decentralised system of services for survivors of sexualised violence, combining feminist counselling centres, RCCs, medical and forensic support, and national helplines. Although the overall provision remains uneven across regions, Germany's model demonstrates how long-standing feminist expertise has shaped public and institutional responses to sexualised violence.

Specialised services available to survivors include RCCs, SVRCs, crisis and medical services, and a specialised helpline. These services provide forensic and medical care, psychological counselling, legal advice and advocacy, and community awareness and prevention activities. In addition, around half of Germany's women's centres offer counselling and support related to sexualised violence, extending the reach of assistance beyond specialist facilities.

The development of SVRCs is currently being expanded through a federal initiative coordinated by an RCC in Hesse. This effort aims to ensure more consistent availability of trauma-informed, interdisciplinary services across all federal states. Meanwhile, the Sexual Abuse Helpline (+49 800 2255530) provides free, anonymous support to children, family members, and professionals concerned about sexual abuse, offering both emotional support and practical guidance.

A key coordinating structure is the Federal Working Group of Feminist Organisations against Sexual Violence against Girls and Women (*Bundesarbeitsgemeinschaft Feministischer Organisationen gegen Sexuelle Gewalt an Mädchen und Frauen e.V., BAG FORSA*). Representing 22 member organisations, BAG FORSA promotes a feminist, society-wide approach to combating sexualised violence and supports local centres in their development and sustainability. It works closely with the German Association for Prevention and Intervention (DGPI) and the Federal Coordination of Specialised Centres (BKSF), ensuring coherence across diverse service providers.

Geographical coverage of sexualised violence services

Geographical access to specialist services for survivors of sexualised violence remains uneven across Europe. Seven countries (15%) reported that services are available in all regions, while 10 (22%) indicated that coverage extends to most regions. In contrast, 11 countries (24%) reported that services are available only in major cities, and seven (15%) that they exist solely in the capital. The remaining countries did not provide information.

In **Belgium**, 10 Sexual Assault Care Centres ensure that every region, including more remote areas such as Arlon and Genk, has access to 24-hour, survivor-centred care. Similarly, in **Finland** and the Netherlands, hospital-based models ensure national coverage through coordinated networks. In **Spain**, each autonomous community maintains at least one crisis centre or hospital-based unit operating 24/7, though rural areas remain without such services. Countries such as **Ireland, Germany, and Scotland** also have relatively broad coverage. In **Ireland**, RCCs exist in 17 of 26 counties, and in **Scotland**, centres are present even in remote island regions through outreach and digital support.

Many countries in Central and Eastern Europe continue to face serious accessibility gaps. In **Lithuania**, forensic and medical care is available only in five major cities, while local centres primarily support survivors of domestic violence. **North Macedonia, Moldova, and Serbia** report that specialised centres are concentrated in capitals or a few large cities, leaving survivors in rural areas with limited or no access. In **Moldova**, mobile outreach initiatives attempt to fill this gap, but complete forensic and counselling services remain urban-based. Similarly, in **Romania, Montenegro, and Poland**, survivors outside urban centres often rely on general hospitals or non-specialised services.

Groups of survivors supported by sexualised violence services

Most sexualised violence services in Europe are designed to support women and girls, in line with their primary

mission to address VAWG. Thirty-two countries (70%) reported providing services specifically for adult women, and 31 (67%) offer support to young women and girls under 18. In addition, 24 countries (52%) indicated that services are also available to other groups, such as men and boys, LGBTQI+ individuals, and persons with disabilities.

In several countries, services that were initially established for women have expanded to include other groups. In **Belgium**, the Sexual Assault Care Centres (*Centres de Prise en Charge des Violences Sexuelles – CPVS*) provide support to women, men, transgender people, and children, although women account for over 90% of those using the services. In **Finland**, Sexual Violence Referral Centres are open to survivors over the age of 16, while the NGO-run RCC *Tukinainen* offers support to women and girls of all ages. The **Netherlands** similarly allows access to all survivors, regardless of age or gender.

In many other countries, services remain primarily oriented towards women and girls but have developed the capacity to support other groups within the same framework. In **Ireland**, RCCs are open to survivors regardless of sex, gender or sexuality. In **Scotland**, for example, twelve centres support survivors of all genders, and three centres provide support only to women survivors of sexualised violence, with most RCCs supporting survivors from the age of 13. In **Germany, Serbia, and Spain**, feminist and women's organisations continue to serve women as their primary target group but collaborate with other agencies to ensure that men, boys, and other survivors can receive appropriate care elsewhere. In **Lithuania**, legislative amendments have enabled five designated hospitals to assist women, men, and children. Since the full-scale invasion, **Ukraine** has expanded its support system for survivors of conflict-related sexualised violence, establishing additional services specifically tailored to the needs of women affected by wartime abuses. Delegates also noted the emergence of a small number of services for male survivors, primarily focused on addressing sexualised violence linked to war trauma.

Conclusion

The findings of this report reveal a persistent and alarming gap in the provision of specialist services for women and girls survivors of sexualised violence across Europe. Despite clear obligations under the Istanbul Convention, the vast majority of countries do not fulfil the required standard of one RCC or SVRC per 200,000 inhabitants. Only four countries meet or exceed this requirement. This means that millions of women and girls in Europe still do not have access to specialist, trauma-informed, and gender sensitive services for survivors of sexualised violence.

Where services do exist, they are often underfunded, unevenly distributed, and concentrated in urban areas. Even in countries with more established systems, long-term



psychological counselling and independent feminist advocacy remain limited compared to acute, hospital-based care. The sustainability of services is further undermined by unstable funding structures, with many relying on short-term projects, donations, or international aid rather than secure, institutional financing.

While some countries have broadened access to include all survivors of sexual assault, WSS remain the cornerstone of an effective, long-term feminist response. These services are grounded in the understanding that sexualised violence is a form of structural gender inequality and that survivors need not only medical or legal help but also safety, belief, and empowerment. Their independence is essential to holding institutions accountable and to ensuring that survivors' rights and autonomy remain at the centre of all interventions.

Equally important is the sustained provision of long-term psychological and advocacy support. Many survivors come forward months, years, or even decades after the violence occurred, particularly those who experienced sexualised violence as children, and require stable, trauma-informed services that can accompany them through their healing at their own pace. Ensuring consistent, long-term support across the life course is therefore essential to addressing the full impact of sexualised violence and enabling survivors to rebuild their lives with dignity and autonomy.

To comply with international standards and meet the real needs of survivors, states must urgently invest in expanding, coordinating, and sustainably funding feminist specialist services. Without such commitment, sexualised violence will continue to be met with silence, fragmented care, and systemic neglect rather than justice, healing, and equality.

Recommendations

- European states must prioritise the establishment of sufficient RCCs and SVRCs in all regions, according to the Istanbul Convention standard of one centre per 200,000 inhabitants and guarantee specialist, trauma-informed, gender-specific support.
- National governments and EU institutions should provide sustainable, multi-year funding for specialist sexualised violence services to ensure long-term operational stability, prioritising remote, rural and underserved regions as well as minority and marginalised communities.
- States must strengthen cross-sector cooperation, including healthcare, law enforcement, legal services, and women's NGOs, based on equal partnership and recognition of women's NGOs as independent, specialist actors, ensuring coordinated and comprehensive referral pathways for survivors of sexualised violence.
- States should develop and maintain national coordination mechanisms and data collection systems on sexualised violence, including sex-disaggregated and intersectional data on prevalence, reporting, access to services, and justice outcomes, in line with the Istanbul Convention. This should include regular quality assurance and monitoring to evaluate the effectiveness of sexualised violence services and guide policy development, in collaboration with women's NGOs and survivor representatives.
- Accessibility of sexualised violence services should be guaranteed for all survivors, with particular attention to those from marginalised groups, such as women with disabilities, migrant and refugee women, Roma women, LGBTQI+ survivors, and those living in rural areas.
- States must adopt and enforce affirmative consent-based definitions and legislation on sexualised violence, developed in collaboration with WSS, requiring voluntary, informed and ongoing affirmative consent.
- States must invest in sustained public awareness, educational programmes and prevention campaigns to counter myths, victim-blaming and harmful gender stereotypes associated with sexualised violence, highlighting men's collective responsibility to challenge the norms that enable such violence.
- Gender-specific, survivor-centred, and human-rights-based methodologies should be embedded across all specialist sexualised violence services, including free-of-charge long-term recovery programmes such as trauma-informed therapy, empowerment groups, and economic reintegration support.
- States must establish systematic, initial and continuous training for all relevant professionals (including police, judiciary, healthcare and education) on gender-informed responses to sexualised violence and workplace harassment prevention.

Primary prevention of violence against women and girls



Introduction

Violence against women and girls (VAWG) is one of the most pervasive violations of human rights across all societies. It is rooted in gender inequality and power imbalances between women and men, and it is predominantly perpetrated by men against women and girls.

Although its prevalence and manifestations vary across the world, the underlying cause and intent are the same: to exert power and control over the lives of women and girls.

Under pressure from civil society, particularly women's movements, many countries across Europe and worldwide have developed structural responses to this issue by providing protection to survivors and ensuring the prosecution and punishment of perpetrators. Most countries have developed policies and legislation, as well as specialised support services, mainly through the involvement and advocacy efforts of women's NGOs. While these efforts represent significant progress, the prevalence of VAWG remains alarmingly high and continues to pose a grave challenge.

Besides strengthening protection, prosecution, and the provision of services to women who have experienced violence, it is equally necessary to place greater emphasis on the prevention of VAWG. Without challenging and transforming the underlying attitudes, norms, and structures that legitimise gender inequality and violence, it will not be possible to achieve a lasting reduction in violence. This is where primary prevention plays a crucial role, as it seeks to stop violence before it occurs by addressing its root causes and creating the social conditions in which equality and non-violence can thrive.

What is primary prevention, and why is it important?

Primary prevention means *stopping VAWG before it occurs* by addressing its root causes: unequal distribution of power and resources between women and men, discriminatory norms, stereotypes and harmful attitudes that enable and validate violence, and a lack of prevention skills

in professionals and the general public. At the same time, primary prevention involves promoting gender equality, prosocial behaviour, and safety across all areas of life.

In its 2024 publication *A Life Free from Violence: Primary Prevention as the Key to Eliminating Violence Against Women and Girls*,⁸⁶ the WAVE Network emphasises that primary prevention is the most strategic and transformative approach to ending VAWG and achieving lasting gender equality. According to this report's findings, primary prevention operates at several interconnected levels. It targets women, girls, boys, and men through education and awareness-building. It also works with groups and communities, such as activists, educators, and civil society organisations, as well as at the structural level by influencing policies, laws, and institutions. It addresses different audiences: women survivors of violence and discrimination, potential perpetrators, bystanders, professionals who can intervene, and institutions or individuals who sustain patriarchal systems. Activities can take various forms, such as school-based workshops for children, feminist self-defence for women and girls, peer education programmes, and workshops for professionals. These interventions are needed at all levels of society to create systemic change.

Research has identified several proven or promising primary prevention strategies. Evidence shows that school-based programmes to prevent dating violence and teach children how to recognise and avoid abuse, as well as home visitation and health outreach during pregnancy, can be effective in preventing VAWG.⁸⁷ Empowerment and participatory initiatives addressing structural gender inequality, together with feminist self-defence for women and girls, have also demonstrated positive impacts, remarkably when grounded in feminist and community-based approaches.⁸⁸ In contrast, some commonly used approaches, such as general awareness-raising campaigns or confrontational rape prevention sessions, have been shown to be ineffective as forms of primary prevention.⁸⁹

It is important to distinguish primary, secondary, and tertiary prevention, especially as states tend to conflate the three when presenting their prevention efforts and reporting on VAWG prevention. This distinction was first promoted by the World Health Organisation (WHO) in relation to diseases, accidents, and disabilities and then adapted to

86 See WAVE – Women Against Violence Europe (2024) *A Life Free from Violence: Primary Prevention as the Key to Eliminating Violence Against Women and Girls*.

87 See World Health Organization (WHO) (2010) *Preventing Intimate Partner and Sexual Violence Against Women: Taking Action and Generating Evidence*; Arango, D. J., Morton, M., Gennari, F., Kiplesund, S., & Ellsberg, M. (2014) *Interventions to Prevent or Reduce Violence Against Women and Girls: A Systematic Review of Reviews*, Washington, DC, World Bank Group.

88 See Kelly, L., & Sharp-Jeffs, N. (2016) *Preventing Violence Against Women: A Review of the Evidence for Primary Prevention*, London, London Metropolitan University, Child and Woman Abuse Studies Unit (CWASU).

89 See Arango, D. J., Morton, M., Gennari, F., Kiplesund, S., & Ellsberg, M., *Interventions to Prevent or Reduce Violence Against Women and Girls: A Systematic Review of Reviews*, Washington, DC, World Bank Group, 2014; World Health Organization (WHO), *Preventing Intimate Partner and Sexual Violence Against Women: Taking Action and Generating Evidence*, Geneva, WHO, 2010.



the context of VAWG in their 2002 World Report on Violence and Health.⁹⁰

THREE-PRONGED APPROACH to prevention of VAWG

Primary prevention: *Stops violence before it starts* by tackling root causes like gender inequality and stereotypes. Actions may include community organising for gender equality and against VAWG, school-based programmes on preventing child abuse or dating violence, feminist self-defence, and programmes challenging harmful masculine behaviour and attitudes.

Secondary prevention: *Early action and response to violence when it starts, to stop escalation.* Activities include helplines, risk assessments, early police or health interventions, and safety planning.

Tertiary prevention: *Addressing the consequences after violence has occurred, supporting recovery and preventing recurrence.* Examples include shelters, counselling and therapy, economic empowerment, and perpetrator programmes.

Empowerment lies at the heart of primary prevention. Unlike traditional support approaches, which often reinforce dependency and maintain existing power imbalances, empowerment seeks to restore control, agency, and collective power to survivors. Self-awareness and self-care are also central to feminist primary prevention. Feminist activism often emphasises responding to crisis, leaving little space for reflection, rest, or regeneration. Yet slowing down, restoring self-awareness, and connecting with one's own sensations and emotions are essential acts of resistance against patriarchal conditioning that teaches women to ignore their instincts and sensitivity to injustice. Feminist self-defence exemplifies this principle by combining physical, psychological, and verbal techniques that enhance women's confidence, safety, and agency. Research shows that self-defence training not only reduces fear and avoidance but also strengthens general self-efficacy, supporting women's participation in all areas of society.

Despite the evidence of success and cost-effectiveness, primary prevention remains underfunded and often neglected in favour of secondary and tertiary responses that deal with violence after it has occurred. Governments continue to invest more in crisis management than in prevention, even though prevention is far cheaper and more

effective in reducing both human suffering and economic costs. For instance, one French study found that every euro invested in prevention saves up to eighty-seven euros in costs, including direct expenses.⁹¹

Ultimately, primary prevention allows us to imagine and build a society without violence. It shifts the narrative from counting survivors to celebrating those who will never experience violence. It focuses energy on transformation, empowerment, and collective freedom. By prioritising primary prevention, we can redirect resources and attention from managing the consequences of patriarchy to constructing a safer, fairer, and more equal world for everyone.

International obligations

International frameworks recognise prevention as an essential component of states' human rights obligations. The Istanbul Convention, which has been ratified by 38 Council of Europe member states, was also formally acceded to by the European Union (EU) in June 2023, entering into force across member states in October 2023. This added significant weight to its provisions and implies broader applicability, especially in countries that have not ratified it individually. In its opening preamble, the Istanbul Convention recognises that *'violence against women and girls stems from historically unequal power relations and harmful gender norms'*.⁹² While it does not distinguish between primary, secondary and tertiary prevention, it underlines the importance of prevention by dedicating Chapter III of the Convention to this issue. Article 12 explicitly addresses states' obligations to take measures to prevent violence against women (VAW) through social change, thereby enshrining the states' obligation to engage in primary prevention.

More specifically, Article 12 obliges states to eliminate stereotypes and inequalities that sustain VAW, using legislation and other preventive measures grounded in a gender-sensitive and intersectional approach. Special attention should be given to vulnerable groups, including women with disabilities, minority women, refugees, and displaced persons. States must also promote women's empowerment, reject 'honour' as a justification for violence, and engage men and boys in primary prevention.⁹³

The General Recommendation No. 35 on gender-based violence against women to the Convention on the Elimination of All Forms of Discrimination Against Women (CEDAW) strengthens the focus on prevention. It defines gender-based violence against women (GBVAW) as *'one of the fundamental social, political and economic means by which*

⁹⁰ World Health Organization (2002) *World report on violence and health*, p. 15.

⁹¹ See Nectoux, M., Minodier, C., & Cambon, L. (2010) *Coût social des violences faites aux femmes en France*, Paris, Ministère de la Solidarité et de la Cohésion Sociale.

⁹² See Council of Europe, *Convention on preventing and combating violence against women and domestic violence* (Istanbul Convention), CETS No. 210, Istanbul, 11.V.2011, Preamble.

⁹³ See Council of Europe, *Convention on preventing and combating violence against women and domestic violence* (Istanbul Convention), CETS No. 210, Istanbul, 11.V.2011, Article 12.

the subordinate position of women with respect to men and their stereotyped roles are perpetuated”,⁹⁴ calling for comprehensive responses addressing systemic causes. It recommends integrating gender equality, non-violent masculinities, and evidence-based, age-appropriate comprehensive sexuality education into curricula at all educational levels.⁹⁵

The WHO’s RESPECT Framework, an interactive ecological model of the primary prevention of VAW, organises risk factors on the different levels at which they occur (societal, institutional, community/family, and individual).

The framework outlines seven strategies to prevent VAW (RESPECT):⁹⁶

- **R** – Relationship skills strengthened
- **E** – Empowerment of women
- **S** – Services ensured
- **P** – Poverty reduced
- **E** – Environments made safe
- **C** – Child and adolescent abuse prevented
- **T** – Transformed attitudes, beliefs, and norms

To encourage implementation, WHO and UN Women released the RESPECT Implementation Package in 2020,⁹⁷ a collection of practical resources that includes strategy briefs for each RESPECT pillar, programme examples, mini briefs on enabling environments, a monitoring and evaluation brief, and a workbook for national prevention planning.⁹⁸

Since the last WAVE Country Report in 2023, the new Directive (EU) 2024/1385 on combating violence against women and domestic violence was adopted on May 14, 2024, and entered into force on June 14, 2024. Although WAVE has been critical of the final adopted text, having consistently advocated for a stronger instrument that would classify gender-based violence (GBV) as an Eurocrime,⁹⁹ the Directive still represents a significant step forward. For the first time, a broad range of measures to combat VAWG has become an official part of EU law, providing a binding framework for Member States. This binding international legal document explicitly states that Member States must adopt a comprehensive approach to address violence, including primary, secondary, and tertiary prevention measures, making it the first international instrument to use this language. Member states’ primary prevention obligations under the Directive include public awareness campaigns and integrating primary prevention into formal education

(e.g., improved sexuality education, empathy-building, and healthy relationships).¹⁰⁰

Taken together, these international and regional instruments clearly establish that states’ obligations extend beyond responding to violence once it occurs. They require governments to address the root causes of VAWG by transforming harmful norms, eliminating structural inequalities, and promoting gender equality in all areas of life. Prevention is therefore not an optional policy choice, but an essential human rights obligation.

WAVE framework for collecting data on primary prevention

This is the second time that WAVE has collected, with its members’ help, data and insights on primary prevention of VAWG. Since there is no unified formal definition of primary prevention, with international documents focusing on different components and aspects of all three types of prevention, WAVE has focused on what has been shown to work and what is aligned with the priorities of women’s specialist services (WSS), based on recommendations from the WAVE Primary Prevention Working Group. Therefore, the data collection focused on promising primary prevention strategies to address VAWG, the existence of National Action Plans (NAPs), definitions of primary prevention, and the funding of these programmes at the national level.

Findings on primary prevention in Europe

In its 2025 survey, WAVE collected information from 46 countries:¹⁰¹

- Definitions – whether a national definition of primary prevention exists
- NAPs or other national provisions on primary prevention – if and how prevention is included in official strategies
- Definition of primary prevention
- Funding for primary prevention activities
- Types of activities – including:

94 See CEDAW, *General recommendation No. 35 on gender-based violence against women, updating general recommendation No. 19*, CEDAW/C/GC/35, 14 July 2017, para. 10.

95 See CEDAW, *General recommendation No. 35 on gender-based violence against women, updating general recommendation No. 19*, CEDAW/C/GC/35, 14 July 2017, para. 35(a).

96 See World Health Organization, *RESPECT women: Preventing violence against women* (Geneva: WHO, 2019).

97 See WHO & UN Women (2020) for the RESPECT Implementation Package.

98 Similarly, the INSPIRE Framework developed by WHO provides seven strategies for ending violence against children and promotes primary prevention of violence against girls. Available at: <https://www.who.int/teams/social-determinants-of-health/violence-prevention/inspire-technical-package>

99 See WAVE Network, *Acceptance with reservations: WAVE Network’s stance on the EU Directive addressing violence against women and domestic violence* (1 March 2024).

100 See European Union, *Directive (EU) 2024/1385 of the European Parliament and of the Council of 14 May 2024 on combating violence against women and domestic violence*, OJ L 2024/1385, 24.5.2024, Recital 73.

101 Four countries did not provide information on primary prevention: Latvia, Liechtenstein, Malta and Slovakia.



- feminist self-defence for women and girls
 - empowerment activities for women and girls
 - activities targeted to men and boys
 - school-based programmes
 - training for primary prevention practitioners
 - bystander intervention training
 - self-care activities
 - awareness-raising campaigns or programmes
- Evaluation and monitoring – whether prevention initiatives are assessed for effectiveness

National Action Plans

NAPs are an efficient way to ensure that VAWG is addressed through holistic measures that include primary prevention. The EU Directive 2024/1385 mandates under Article 39 that Member States must establish NAPs to prevent and combat VAW and domestic violence by 14 June 2029.¹⁰² According to a UN Women analysis of its global database on VAWG from July 2025, 34% of UN Member States have not adopted a dedicated NAP to address VAWG.¹⁰³

Based on information from WAVE members, the last WAVE Country Report from 2023 listed 31 countries with some form of NAP or a similar policy document. In this round of data collection, we specifically asked if there was an official document on primary prevention, such as a NAP or other provisions. Responses indicate that 32 countries (70%) reported having an official document on prevention, such as a NAP or a strategy, although it is not known whether they specifically address primary prevention. This includes separate plans for three of the four nations in the **United Kingdom**¹⁰⁴ and in **Belgium**, which has a national as well as two regional plans.¹⁰⁵ No NAP or similar document defining primary prevention was reported in ten countries. **Azerbaijan and North Macedonia** both had a NAP, but they expired at the end of 2023, and a new one hasn't been passed yet. **Austria** is developing a new NAP that will include primary prevention, but the exact definition and measures are not yet known. Some countries do not have an official NAP, but they do have other strategic documents. For example, the **United Kingdom's** approach to addressing VAWG will be set out in the Tackling VAWG Strategy 2025. In the **Netherlands**, primary prevention is listed as one of the four pillars of the *Stop Femicide!* plan, with numerous activities outlined by different ministries.

Some countries, such as **Hungary, Greece, and Moldova**, rely on NGOs and EU projects rather than a national strategy to develop primary prevention programmes. Some countries have multiple NAPs covering different forms of VAWG, such as **Norway**, which has a NAP on domestic

violence, rape, forced marriage, female genital mutilation, and online sexual abuse. These include preventive measures, but they are not defined within a single national framework of primary prevention. In these cases, prevention is carried out operationally through measures and activities, but it is not defined conceptually, and there is no holistic, coordinated primary prevention approach, decreasing the efficacy of individual measures.

Defining primary prevention

The existence of a NAP is often used as an indicator of a country's commitment to implement an integrated policy and a strategic, holistic approach to addressing and eliminating VAWG. However, the findings of the WAVE data collection for the Country Report 2025, similar to the 2023 report, show that the existence of a NAP does not guarantee that it provides a comprehensive, gender-sensitive strategy and a primary prevention framework.

When asked if there is a definition of primary prevention of VAWG in their country's NAP or elsewhere, 18 WAVE country delegates (39%) responded that there is such a definition. Among the countries that responded yes to this question, there is variation in the detail provided in the definitions of primary prevention, and it could not always be verified whether the definitions were specifically about primary prevention or only about prevention. Some countries have very clear definitions (the **Netherlands, Italy, Scotland, and Albania**), while others only refer to prevention in terms of general awareness measures or international standards. Many countries take their definition of prevention directly from the WHO or the Istanbul Convention, while others create their own national definitions based on education or on removing stereotypes. For countries that responded that they do not have a definition of primary prevention in an official document, a common theme is that prevention exists but is implemented through activities such as awareness-raising, education, and campaigns.

For countries that do have a definition of primary prevention in their NAP, some explicitly align it with international definitions. For example, **Belgium** quotes the WHO's health-prevention framing and applies it to VAWG. Some countries have more detailed and specific descriptions of primary prevention. For example, the **Netherlands** defines it as:

"eliminating root causes of violence against women. Preventing gender-based violence (including femicide) can be achieved largely by eliminating gender inequality as a cause of the violence. We want to promote gender equality and equality between individuals and drive a societal culture change. This

¹⁰² See Article 39, Directive 2024/1385 on combating violence against women and domestic violence.

¹⁰³ See UN Women (2025) *From Words to Action: National Action Plans to End Violence Against Women and Girls*, 34%.

¹⁰⁴ The United Kingdom has separate action plans in Scotland, Wales, and Northern Ireland, but not in England (still under development).

¹⁰⁵ Belgium has a federal National Action Plan as well as regional plans in Flanders and Brussels/Wallonia.

*requires raising awareness and changing harmful gender norms into healthy and equal gender norms. To do this, we are committed to gender mainstreaming and supporting civil society organisations.”*¹⁰⁶

Furthermore, the **Netherlands'** *National Action Programme on Sexually Transgressive Behaviour and Sexual Violence* (2022–2026) defines prevention as a central pillar, emphasising the need “to combat sexually transgressive behaviour and sexual violence not only reactively, but also preventively” by ensuring safe working environments and focusing on the role of bystanders, as well as fostering norms and values shared across society.¹⁰⁷

In **Albania**, the National Strategy for Gender Equality 2021–2030 stresses reducing harmful practices and VAWG before they occur, with gender mainstreaming identified as the key tool. In **Italy**, the National Strategic Plan focuses on awareness, communication, and education to counter stereotypes, promote respect, and stigmatise violence. **Scotland's** *Equally Safe* strategy defines primary prevention as “preventing violence before it occurs; this means tackling the root causes of violence and challenging the attitudes and inequalities that enable it”,¹⁰⁸ aligning with the WHO's definition.

For countries that do not define primary prevention, the situation is sometimes similar to that in countries that do. For example, **France, Slovenia, Sweden, and the United Kingdom** have no formal definition, but prevention is implicitly present through awareness-raising and cultural change. In **Moldova**, the policy mentions prevention but does not distinguish between primary, secondary, or tertiary. Hungary has no VAWG policy at all, and Ukraine has no formal definition, but the State Social Programme outlines prevention in terms of attitudes, awareness, and tolerance, with a focus on secondary prevention. The State Social Program for the Prevention and Counteraction of Domestic Violence and Gender-Based Violence until 2025 explicitly includes:

*“formation of an intolerant attitude toward violence in society, strengthening readiness to counteract its manifestations, caring attitude toward victims (especially children), and public awareness-raising about forms, manifestations, causes, consequences of domestic violence, and its disproportionate impact on women, men, persons with disabilities, pregnant women, children, incapacitated persons, elderly persons”.*¹⁰⁹

In countries where delegates responded ‘do not know,’ primary prevention is partially present but unclearly defined. For example, **Estonia** has a violence prevention agreement, but the definition is generic rather than gender-specific. If it is not gender-specific, then it is not a definition of primary prevention that tackles the root causes of VAWG.

In **Finland**, NAPs contain prevention statements, but they are vague and do not constitute a formal definition.

Overall, these results reinforce WAVE's findings from two years ago: most countries still lack a clear, consistent definition of primary prevention in their national frameworks, and, regardless of the presence and quality of their definitions, most invest in programmes that are not, strictly speaking, efficient forms of primary prevention. Additionally, many countries do not clearly distinguish between primary, secondary and tertiary prevention. We therefore conclude that primary prevention remains the poor parent of public policy to combat VAWG.

Funding for the primary prevention of VAWG

Adequate and sustainable funding is essential for effective primary prevention, yet findings from the WAVE data collection show that support for such activities remains inconsistent and, in many cases, has declined. Only 16 delegates (35%) indicated that a specific budget is allocated to primary prevention activities. This is very concerning and aligns with the general cuts to funding for women's and other civil society organisations across Europe and worldwide. It may also be linked to the ongoing wars in Europe and globally, which have prompted governments to redirect significant parts of their budgets towards defence and security, often at the expense of social policies and measures to prevent and address VAWG.

In countries that indicated government funding is available for primary prevention activities, such funding is often small-scale or regional. They are related to specific sets of primary prevention activities, such as school educational programmes or empowerment activities. Even when such funding exists, it is often inconsistent and insufficient. In countries without a specific budget, some funding exists, but it is not gender-specific and is allocated to general prevention activities. In some cases, it is part of budgets for combating domestic violence or other forms of violence, but it is not explicitly allocated to primary prevention. In countries where respondents reported not knowing whether a specific budget is allocated to primary prevention activities, budgeting is opaque. In some countries, primary prevention may be funded at the local or regional level rather than the national level, making it difficult to track. There are also cases in which no single authority oversees primary prevention budgets, which are instead distributed across different government agencies, creating ambiguity.

Albania regularly funds various primary prevention activities against VAWG, including education, community engagement, and capacity-building. In 2023, about 9.6% of the national budget was allocated to gender equality

106 See Government of the Netherlands (2024) *Plan van aanpak “Stop femicide!”*, pp. 12–13, for the definition of primary prevention.

107 See Government of the Netherlands (2023) *Government presents action programme to tackle sexually transgressive behaviour and sexual violence*.

108 See Scottish Government (2023) *Equally Safe: Scotland's Strategy for Preventing and Eradicating Violence Against Women and Girls*, 33.

109 See Cabinet of Ministers of Ukraine (2021) *State Social Program for Prevention and Counteraction of Domestic Violence and Gender-Based Violence until 2025*, 1.



initiatives, covering both prevention and support services, though it is unclear how much was allocated to primary prevention. **Scotland** allocates specific funding for primary prevention activities related to VAWG, primarily through the Delivering Equally Safe (DES) Fund, which is a central component of the Equally Safe strategy. As of the 2024–25 period, the DES fund has allocated approximately £19 million (almost €22 million) per year, supporting over 120 projects delivered by more than 100 organisations across Scotland.¹¹⁰ Another positive example is **Spain**, where a specific budget is allocated to the implementation of primary prevention activities, while autonomous communities and local councils often co-finance prevention projects (such as school programmes and community workshops) with their own resources, which guarantees continuity and regional coverage.

However, despite some positive examples, the limited and inconsistent funding in most countries directly shapes which primary prevention activities are implemented, with countries often prioritising short-term or less resource-intensive measures over comprehensive, evidence-based approaches.

Primary prevention activities

To gain a clearer overview of the types of primary prevention efforts undertaken across Europe, respondents were asked to indicate which primary prevention activities are implemented in their country. This approach allowed identification of both the most widespread forms of primary prevention activities across countries and the less common or emerging ones. The summary table on this page provides information on the number of countries where each type of activity is present.

We observe that awareness-raising campaigns or programmes, although not effective as primary prevention tools, are the most widely implemented activity. About two-thirds of countries also implement empowerment activities for women and girls, as well as school-based primary prevention programmes. Approximately half the respondents also reported training for primary prevention practitioners, feminist self-defence training, and activities specifically targeted to men and boys. Bystander intervention training and self-care activities are the least common, occurring in fewer than a third of countries. While the relatively widespread implementation of various primary prevention programmes is encouraging, the data do not allow us to draw conclusions about the coverage these programmes achieve. Given the data on funding for primary prevention activities, it is likely that primary prevention service provision, while diversified and widespread, covers only a small portion of the population and has not yet

achieved the necessary reach for the social transformation required to end VAWG. In addition, some governments proactively create obstacles for the implementation of primary prevention programmes.

Type of activity	Number of countries
Awareness-raising campaigns or programmes	39
Empowerment activities for girls and women	31
School-based primary prevention programmes	29
Training for primary prevention practitioners	24
Feminist self-defence training	20
Primary prevention activities specifically targeted to men and boys	19
Bystander intervention training	13
Self-care activities	13
Other	6

Table 12: Types of primary prevention activities carried out in Europe

In **Hungary**, for example, organising primary prevention activities in public schools became very difficult after a 2021 amendment to the Child Protection Act. According to the regulation, any person or organisation that co-operates with an educational institution and conducts sessions on topics such as sexuality education, internet safety, or mental health promotion must obtain special authorisation from a state-designated body. However, this body was only established in June 2025, which meant that NGOs and organisations offering primary prevention programmes could not register and therefore could not deliver their activities in schools. As a result, schools became noticeably less open to inviting external organisations, including those providing violence prevention programmes. The public education curriculum still does not provide space or time to address these topics, leaving violence prevention activities largely out of schools.¹¹¹

In **Italy**, the Ministry of Education is pursuing a bill on prior informed consent of families for school activities dealing with issues related to sexuality.¹¹² This follows pressure from conservative family groups, such as the Family Day movement. The proposed legislation is particularly restrictive toward younger students, aiming to exclude children under 10 from such programmes entirely. This concerns programmes on sexuality education that are part of

¹¹⁰ See Scottish Government (2025) *Funding to tackle violence against women and girls*.

¹¹¹ See Hungary, Act LXXIX of 2021 on taking more severe action against paedophile offenders and amending certain Acts for the protection of children (Child Protection Act), Magyar Közlöny, No. 118/2021.

¹¹² See Brussels Signal (2025) *'Italy PM Meloni makes parental consent mandatory for pupils' sex education'*, noting that the bill requires explicit family consent for any school activity addressing sexuality.

primary prevention. By requiring prior family consent and limiting access for younger students, the bill effectively places feminist and LGBTQI+ organisations under increased scrutiny, limiting their work in primary prevention of VAWG.

In most countries, primary prevention activities are organised by women's NGOs, which undertake a variety of initiatives. In **Moldova**, primary prevention of VAWG continues to be implemented by feminist organisations, primarily supported by international donors (such as Oak Foundation). The National Coalition 'Life Without Violence',¹¹³ a network of 24 women's rights organisations, implements many prevention activities, including gender education in schools, empowerment workshops, and training on positive masculinities. The Association Against Domestic Violence *Casa Mărioarei*¹¹⁴ has spearheaded a programme engaging men and boys to challenge harmful gender norms. However, prevention efforts remain predominantly project-based and donor-dependent, without sufficient support from state authorities.

Following **Türkiye's** withdrawal from the Istanbul Convention, providing primary prevention activities against VAWG has become more difficult. It is not possible for NGOs and women's organisations to operate in public schools, as their activities relate to gender equality. In fact, as of May 2025, the Directorate General of Family and Community Services under the Ministry of Family and Social Services has prohibited the use of concepts such as 'gender', comprehensive sexuality education, and gender equality within the relevant departments. This policy effectively restricts the presence and work of civil society and women's groups in state schools by framing their initiatives as incompatible with current educational guidelines.¹¹⁵

Awareness-raising activities: common practice but limited preventive impact

In many countries, women's organisations and NGOs continue to engage in awareness-raising activities to prevent violence. Data collected through this mapping show that such initiatives are widespread and often represent the main or only component of prevention work at the national and local levels. Typically, these activities include public campaigns (online and offline), the production and dissemination of informational materials, and public events aiming to increase knowledge about GBV, domestic violence, or related issues such as forced marriage, female genital mutilation, or harassment in public spaces.

Examples mentioned in the data include the Norwegian campaign "*How Little Should One Tolerate?*" which aims to increase knowledge of domestic violence and the

assistance available from the police, as well as to encourage survivors to seek help. In Iceland, the campaign "*Stand Up, Oppose Harassment in Public Places*" raised awareness of sexual harassment in public spaces and equipped participants with practical tools to respond to such incidents. In addition, several countries, such as **Norway and Azerbaijan**, reported the production of information materials for newly arrived migrants or refugees on topics including negative social control, domestic violence, forced marriage, and female genital mutilation.

Some organisations also report awareness raising through media engagement, social media campaigns, and workshops targeting specific professional or community groups. In **Ireland**, for example, awareness raising accompanies cooperation between Safe Ireland and perpetrator programmes (MEND and MOVE) to promote community accountability.

In a few cases, these activities have been evaluated, with findings showing that awareness-raising can improve public knowledge and recognition of VAWG and, in some contexts, lead to increased help-seeking among survivors. However, evaluations also highlight that such activities rarely influence deep-rooted gender norms or behaviours that sustain violence. They often stop at the level of information and attitude change, without the participatory, empowerment-based, or structural components necessary for genuine violence prevention.

This aligns with WAVE's position and with wider international evidence that awareness-raising alone does not constitute effective primary prevention. While it can play a supportive role, by informing the public, challenging silence, and signalling institutional concern, it does not address the underlying causes of GBV, nor does it transform the power relations and gender norms that perpetuate it.

Data collection on primary prevention

Out of 46 countries, 10 reported that public authorities collect data on primary prevention activities, 20 stated that they do not, nine provided unclear or partial answers, such as 'not systematically', 'planned but not implemented', or 'do not know', and seven did not answer.

Only a small minority of countries systematically collect data on primary prevention activities. Where collection occurs, it is typically limited in scope, often linked to specific projects, action plans, or donor-funded programmes, rather than part of a coordinated national monitoring system. Examples include **Albania**, where data collection is linked to the implementation of the National Strategy for Gender Equality, and **Austria and Azerbaijan**, where ministries

113 National Coalition "Life Without Violence" (2023) *Shadow Report of Women's NGOs, members of the National Coalition "Life Without Violence"*

114 See Devex, *Association Against Domestic Violence "Casa Mărioarei"*

115 See Kaos GL (2025) "A 'warning' from the Ministry of Family to its directorates against LGBTI+: 'Do not use the concepts of gender, gender identity, or sexual orientation'".



or agencies collect partial information through reporting mechanisms on state-funded initiatives.

In contrast, most respondents noted that no formal or systematic data collection is in place. Information on prevention activities, when available, tends to be fragmented, either integrated into broader gender equality, education, or violence response frameworks or gathered sporadically at the local or NGO level without national aggregation.

Several countries, such as **Denmark, Belgium** (Brussels and Wallonia), and the **United Kingdom** (England and Wales), indicated that plans or regional frameworks exist for monitoring prevention efforts, but implementation remains incomplete.

Overall, the responses highlight a clear gap: systematic, state-led data collection on primary prevention is rare, and where it does occur, it is ad hoc, project-based, or sector-specific, rather than embedded in a comprehensive monitoring or evaluation framework.

CASE STUDY: Poland – primary prevention activities

Background

Poland lacks official, state-led programmes explicitly focused on primary prevention of VAWG and GBV. Most initiatives are carried out by feminist organisations and foundations, often with support from international partners or city authorities. Despite limited state involvement, several innovative projects have emerged in recent years.

Activities

- **Safer Cities Project (Juniper Foundation & Plan International Poland, 2024):**¹¹⁶ Focused on the safety of girls and young women in urban spaces, this project combined research with workshops, safety walks, the establishment of a Girls Advisory Group, and peer-led advocacy. Recommendations were developed for institutions on preventing violence in cities, and a national conference, *Safer Cities – Solutions and Recommendations for Institutions and Communities*, was organised on International Day of the Girl. The project highlighted an intersectional approach to urban safety, taking into account LGBTQ+ people, migrants, and women with disabilities.
- **Feminist Self-Defence (Autonomia Foundation):**¹¹⁷ Autonomia has trained around 80 WenDo trainers (from Poland, Ukraine, and minority communities, including LBT women, trans people, migrants, refugees, and persons with disabilities). In 2024

alone, more than 2,000 women and girls participated in WenDo workshops on feminist self-defence, self-determination, and assertiveness.

- **Stand Up Campaign (Women's Rights Centre, Feminoteka, L'Oréal Paris, Right To Be):**¹¹⁸ A nationwide awareness and training campaign on sexual harassment in public spaces. Using the 5D methodology (Deconcentrate, Join Others, Speak Up, Document, Give Support), it trained thousands of participants through schools, universities, companies, and public events. The campaign received broad institutional backing, including the honorary patronage of the Commissioner for Human Rights and the mayors of major cities (Warsaw, Krakow, Gdansk, Wroclaw, Poznan, and Lodz).

Results

- Thousands of women and girls directly participated in empowerment and safety activities, with strong outreach to minority and vulnerable groups.
- Projects gained high-level political and public visibility (e.g., city mayors, Ombudsman patronage, and international partnerships).
- These initiatives combined grassroots feminist activism with corporate and international partnerships, showing creative ways to advance primary prevention even in a restrictive policy environment.

Evaluation of primary prevention

When asked whether there is an evaluation of the impact of primary prevention activities in each country, and if so, whether the evaluation is financed by public authorities, only four respondents answered positively. There is no such evaluation in 28 countries, and 21 gave a mixed or unclear answer.

Albania has conducted evaluations of primary prevention activities against VAWG, funded by both international organisations and public authorities, including national ministries. The assessments helped measure the effectiveness of prevention programmes and guide improvements in policies and services. In **Finland**, primary prevention activities are evaluated for their impact and effectiveness when they are a part of state-level action plans. **Scotland** has also undertaken evaluations of primary prevention activities aimed at eradicating VAWG, funded by public authorities, primarily the Scottish Government. In many cases, evaluations focus only on how a programme was implemented, rather than on its long-term preventive effectiveness, namely the reduction in post-intervention victimisation rates.

¹¹⁶ See Juniper Foundation (2024) *Safer Cities – Policy Recommendations for a Safer Poland: Addressing Gender-Based Violence in Public Spaces*, implemented with Plan International Poland.

¹¹⁷ See Autonomia Foundation (2024) *WenDo – feminist self-defence trainings*.

¹¹⁸ See Feminoteka (2024) *Stand Up Against Harassment campaign carried out with the Women's Rights Center, L'Oréal Paris, and Right To Be*.

In countries where there is no systemic evaluation of the impact of primary prevention programmes, some evaluation still exists. For example, in **Austria**, there is currently no comprehensive evaluation of the impact of primary prevention activities at the national level; however, some individual projects and initiatives include internal or external evaluations, which are not consistently funded or coordinated by public authorities at the national, regional, or local levels. A bigger, Austria-wide study is being prepared and should be released in 2026.

CASE STUDY: Austria – The StoP Project: Neighbourhoods Without Partner Violence

The StoP Project – Neighbourhoods Without Partner Violence (*Stadtteile ohne Partnergewalt*) is a community-based primary prevention model originally developed in Germany in 2010 and later expanded to Austria. The StoP concept was developed in 2006 by Prof. Dr. Sabine Stövesand of the Hamburg University of Applied Sciences, Faculty of Business and Social Sciences, Department of Social Work, and is a copyrighted concept. Since 2010, StoP has been successfully implemented in 12 districts in Germany. The locations in Austria build on the experiences in Germany.

The project has been implemented by the Association of Autonomous Women's Shelters (*Autonome Österreichische Frauenhäuser – AÖF*) and local partners. StoP has become one of Austria's most recognised primary prevention initiatives. By 2024, it was active in around 30 locations across the country.

What is StoP: *"The violence prevention project aims to raise awareness of (domestic) violence/intimate partner violence among people, especially neighbours, and to strengthen social cohesion. By highlighting options for action and support, StoP helps to identify and interrupt violence in neighbourhoods early on. StoP pursues a new and future-oriented approach to violence prevention that combines existing victim protection work with community work".*¹¹⁹

Activities of the project are:

1. A local neighbourhood facility (e.g., community centre, parent school) decides to address the issue by allocating staff, space, and budget.
2. Exploration and activation of the community through social space analysis, network mapping, surveys, and community organising.

3. Formation of neighbourhood action groups, engaging residents through tailored public outreach and community-based workshops.
4. Expansion of community networks and work on shifting cultural norms and collective identities—through activities like film nights or flea markets that spark dialogue.
5. Initiation and strengthening of partnerships at the neighbourhood level, including cross-sector networks, resource pooling, and mutual training.
6. Provision of individual support and person-centred networking, such as establishing local counselling or coordinating regular outreach.
7. Establishment of continuous relational and organisational community work, maintaining engagement and laying the groundwork for a reliable intervention-ready structure.
8. Development of political alliances and advocacy, crafting strategic demands and tactics, and connecting local efforts with professional and broader policy networks.

Independent qualitative studies spanning up to 14 years demonstrate that StoP fosters community cohesion, civil courage, and empowerment. The model has shown real impact internationally, reflecting deepening social change in neighbourhoods and reduced domestic violence.¹²⁰

Several women's organisations in **Kosovo**, such as the Kosovo Women's Network (KWN) and QIKA, actively monitor and evaluate their own primary prevention programmes. These evaluations often focus on awareness-raising campaigns, empowerment initiatives, and educational workshops, assessing changes in public attitudes, knowledge about VAWG, and participation rates, but not on the effectiveness of these measures. These evaluations are usually funded by NGOs themselves or by international donors, rather than by public authorities at national, regional, or local levels.

Evaluation of primary prevention activities remains very limited and uneven across countries, and mostly focuses on programme implementation. Given the complexity and costs of evaluating the preventive impact of primary prevention programmes and the limited overall funding for these programmes, this is not surprising. On the contrary, NGOs should be commended for mobilising their scarce resources to evaluate underfunded programmes.

119 See Über StoP – StoP – Stadtteile ohne Partnergewalt (n.d.)

120 See StoP Toolbox (2023) *Achievements of StoP*.



This situation rather underlines the need for coordinated primary prevention policies, as well as adequate state funding, including for stringent evaluation of the preventive impact of individual programmes. Evaluation is necessary to identify promising and effective programmes that should be the focus of public and private investment in order to end VAWG as quickly as possible.

Conclusion

Primary prevention is essential to address and eliminate the root causes of VAWG, such as gender inequality and harmful attitudes and norms. Every country should develop, as part of its strategy to prevent and combat all forms of VAWG, primary prevention policy and programming. These measures should be based on a normative framework that clearly defines primary prevention, its purpose, and the goal of ending VAWG. International documents such as the Istanbul Convention and CEDAW have enshrined primary prevention as a legal and human rights obligation and not an optional policy choice.

The overall picture is not encouraging, as most states do not meet this obligation. While there is some progress and positive examples in particular states that allocate funding and pilot innovative and structured primary prevention initiatives, the results of the second round of WAVE's investigation into this issue show insufficient government support, both financial and political, for this approach. Many countries still lack a clear definition of primary prevention, and confusion between primary, secondary, and tertiary prevention remains widespread, further eroding attention to and investment in primary prevention. Funding is typically non-existent, not specifically earmarked for primary prevention, short-term and inconsistent, leaving most activities small-scale, project-based, and dependent on external donors, with women's NGOs carrying the main responsibility. Systematic, state-level evaluations of the effectiveness of primary prevention are rare, and funding for NGO evaluations is similarly scarce. This prevents practical preventative impact from being measured or replicated.

Women's organisations remain the backbone of primary prevention efforts across Europe. Their work is innovative, intersectional, and addresses the needs of marginalised groups, but it remains severely underfunded, often unsupported by the state, and therefore unsustainable in the long term. In some contexts, restrictive government policies further limit women's NGOs' space for preventative action by restricting their access to schools or by censoring the very idea of gender equality. Despite these commendable efforts against the grain, without stronger state support, adequate funding, and systematic evaluation, primary prevention cannot achieve systemic impact, and high levels of VAWG will persist across Europe. Moving forward, it will be necessary to ensure that NAPs adopted in the coming years contain clear, human-rights-based, gender-transformative definitions and comprehensive

measures across primary, secondary, and tertiary prevention. Governments must provide sustainable funding, long-term planning, cooperation with women's NGOs, and robust evaluation systems so that primary prevention can meaningfully contribute to the reduction and eradication of VAWG.

Recommendations

- **National governments and EU institutions must strengthen existing legal instruments, such as the Istanbul Convention, in terms of primary prevention. This can be achieved by integrating it into the Group of Experts on Action against Violence against Women and Domestic Violence (GREVIO) baseline questionnaire, monitoring, and reporting; by issuing evidence-based policy recommendations for effective primary prevention resourcing; and by adopting an optional protocol to the Istanbul Convention on primary prevention. Such measures would reinforce states' due diligence obligations to prevent VAWG and safeguard women's and girls' fundamental right to live free from violence.**
- **To effect the societal change needed to end VAWG, policymakers need to capitalise on the combined effectiveness of the full three-pronged public-health approach to violence prevention: addressing primary, secondary and tertiary prevention.**
- **EU institutions should embed primary prevention in policy frameworks and call for long-term political commitment to centre primary prevention on a national level. This should include forthcoming revisions of EU legislation such as the EU Victim's Rights Directive, the EU Directive 2024/1385 on combating VAW and domestic violence, and the EU recommendations on harmful practices (in development), as well as policy documents including the EU Gender Equality Strategy 2026–2030, and the LGBTIQ+ Equality Strategy 2026–2030.**
- **States must secure dedicated long-term funding for primary prevention, ensuring that programmes for women's and girls' empowerment remain adequately resourced. Prevention initiatives targeting men and boys or the wider community should complement, not replace, women- and girl-centred programmes. Priority areas should include scientific evaluation of NGO-run pilot programmes, research into scaling proven prevention initiatives, and development of quality standards.**

- National governments and EU institutions should develop comparable national and EU evaluation systems for primary prevention activities, using clear indicators able to capture long-term changes in attitudes, behaviours, and VAWG prevalence, recognising that progress requires sustained, long-term prevention efforts rather than isolated or short-term activities.
- States must adopt long-term National Action Plans on VAWG that centre primary prevention, include clear definitions, ensure sufficient funding, support evidence-based NGO-run programmes, and report on measurable reductions in VAWG. Such strategies should address intersectionality, responding to the needs of marginalised groups such as migrant women, Roma women, women with disabilities, LGBTQI+ individuals, and rural women.
- States should consider that awareness-raising campaigns and activities are necessary but not sufficient for effective primary prevention. To increase impact, awareness-raising activities and campaigns should be rolled out long-term and include messaging promoting specific prosocial behaviour and transforming gender norms.
- Governments should remove legal and political barriers restricting NGOs' access to schools and public institutions, ensuring women's organisations can fully deliver gender equality and sexuality education as part of their primary prevention initiatives. Additionally, primary prevention must be systematically integrated into education systems by introducing evidence-based gender equality and violence prevention curricula from early childhood onwards.
- States must support WSS to evaluate and upscale their existing primary prevention programmes, including training of primary prevention professionals and the exchange of experiences.



Technology-facilitated violence against women and girls



Introduction

Over the past decade, with the rapid rise of digital technologies and their integration into everyday life, technology-facilitated violence against women and girls (TFVAWG) has also grown significantly. While online spaces and digital tools offer opportunities for connection, information, and activism for women and girls, as seen in movements like #MeToo, they have also created new environments for violence against women and girls (VAWG) to occur. Violence carried out online is often harder to recognise, prove, and sanction, leaving many women and girls exposed to harm without adequate protection. Notably, research shows that TFVAWG affects people differently, with women and girls disproportionately targeted and harmed.¹²¹

TFVAWG can be broadly understood as any act of gender-based violence (GBV) against women and girls that is committed, assisted, aggravated, or amplified by the use of digital technologies or online platforms. It is a continuation of the same structural inequalities and power imbalances that underpin all forms of VAWG, only manifesting through digital means.

FORMS OF TFVAWG

This violence takes multiple and evolving forms, including but not limited to:

- **Cyberstalking** – repeated unwanted surveillance, monitoring, or communication through digital technologies.
- **Online harassment/hate speech** – abusive, threatening, or offensive communications, often persistent and escalating, involving sexualisation, objectification, body-shaming comments, threats of rape.
- **Image-based sexual abuse** – non-consensual creation or sharing of intimate images or videos, including so-called ‘revenge porn’.¹²²
- **Deepfakes** – manipulated images or videos, often sexually explicit, created using artificial intelligence without consent.

- **Doxxing** – publishing private or identifying information online without consent, often with the intent to harm or intimidate.
- **Grooming** – establishing an emotional connection with a child or young person online to manipulate, exploit or sexually abuse them, often carried out through social media, gaming platforms or messaging apps.
- **Spyware and online surveillance** – covert monitoring of a person’s devices, accounts, or online activity, frequently used as a strategy in intimate partner violence.

Context, consequences and data on TFVAWG

What makes TFVAWG particularly harmful is its scale and persistence, meaning that perpetrators can act anonymously, targeting women they have never met, while abusive content can be endlessly reproduced, widely shared, and difficult or even impossible to erase. This multiplies harm, reinforces threats, and can encourage further attacks, both online and offline. The consequences for survivors are serious and long-lasting, ranging from fear and self-censorship to mental health impacts and withdrawal from public life. Certain groups of women, such as politicians, journalists, human rights defenders, and women in public life, face especially high levels of online abuse aimed at silencing their voices.¹²³

Emerging technologies, especially artificial intelligence (AI), have created new and alarming risks. More than 90% of deepfake videos online are pornographic in nature, with women almost exclusively the target.¹²⁴ AI makes it possible to generate fake sexual images, impersonate a woman’s voice or likeness, and allow harassment to spread faster and more widely than ever before. Algorithms can quickly spread misogynistic content to large numbers of people, creating closed spaces where VAWG is normalised and harmful ideas spread, especially among young men. AI tools also make it easier for perpetrators to spy on women, steal their identities, and stalk them, thus giving them new ways to watch, control, and even predict women’s lives, placing survivors at greater risk.

¹²¹ See UN Women (2023) *Technology-facilitated violence against women: Taking stock of evidence and data collection*.

¹²² The term ‘revenge porn’ is used here in quotation marks to reflect its widespread public and media usage, although it is increasingly rejected by feminist and legal experts. The phrase inaccurately suggests consensual pornography and revenge as a motive, rather than the non-consensual nature of the abuse. More accurate terms include *image-based sexual abuse* or *non-consensual sharing of intimate images*, which emphasise the violation of consent and the harm caused.

¹²³ See UNESCO (n.d., ‘Safety of Women Journalists’).

¹²⁴ See European Parliamentary Research Service (EPRS) (2024) *Cyberviolence against women in the EU*, European Parliament, p. 3.

One of the distinct challenges in addressing TFVAWG is its connection and interaction with offline violence. Abuse that begins in online spaces often spills into real-life violence, either by fuelling attacks or by extending patterns of coercive control that are already present in intimate relationships. At the same time, violence that starts offline is increasingly prolonged through digital means, for example, the recording and non-consensual sharing of intimate images, or the threat of such sharing, which traps survivors in cycles of coercion and fear. Stalking through social media is part of the stalking strategy both during an abusive relationship and after it ends. Rather than separate spheres, online and offline VAWG reinforce one another, with TFVAWG deepening the impact of all forms of offline violence and vice versa. Recognising this continuum is essential to understanding the seriousness of TFVAWG and the urgent need for coordinated responses. This also makes it clear that online abuse is not a lesser form of violence, but part of the same system of control and harm that women and girls face in all areas of life.

As TFVAWG has gained more attention in recent years, efforts have been made to define, map and better understand it, including through surveys and data collection. While this issue still remains under-researched and under-recognised, there is some data available on its prevalence and seriousness:

- According to the European Union Agency for Fundamental Rights (FRA), 11 per cent of women across the European Union (EU) and the United Kingdom have experienced cyber harassment and five per cent have experienced cyberstalking since the age of 15, based on interviews with 42,000 women.¹²⁵
- More recent figures from the 2019 FRA Fundamental Rights Survey, cited in a 2024 briefing by the European Parliamentary Research Service, show that 13 per cent of women in the EU reported cyber harassment in the previous five years, and five per cent in the previous year, with prevalence rising to 27 per cent among women aged 16 to 29.¹²⁶
- FRA's 2023 report on online hate and content moderation, with findings based on a keyword search method, demonstrated high rates of misogyny across all major online platforms that the research covered (10). Posts targeting women most often included denigrating language, comparing women to objects or animals. Online VAWG was most often sexualised violence.¹²⁷
- A global survey conducted by Plan International with 14,000 girls and young women aged 15 to 25 across 31 countries found that more than half reported being victims of cyberstalking.¹²⁸
- UNICEF has estimated that by the age of 15, around nine million girls in Europe will have experienced some form of cyber violence.¹²⁹
- The Internet Watch Foundation (IWF) found that in 2021, 97 per cent of child sexual abuse imagery taken down featured girls.¹³⁰
- Seventy per cent of women survivors of cyberstalking also experience at least one form of physical or sexual violence from an intimate partner.¹³¹
- The psychological impacts of TFVAWG include fear, self-censorship, withdrawal from public or online spaces, and serious effects on mental health.¹³²

The evidence shows that TFVAWG is widespread, deeply harmful, and constantly evolving as new technologies emerge. It silences women and girls, reinforces existing inequalities, and often overlaps with other forms of violence. Understanding its context, consequences and scale is therefore essential for shaping laws, policies and services that can respond to it effectively, a task that women's NGOs across Europe have already begun to work on.

International recognition and obligations

Global and European frameworks increasingly recognise TFVAWG as part of the continuum of VAWG. The UN Special Rapporteur on Violence Against Women and Girls *Thematic Report on online violence* defined it as “any act of gender-based violence against women that is committed, assisted or aggravated in part or fully by the use of information and communication technologies”. It mapped responsibilities across prevention, protection and prosecution, calling on states to regulate platforms and on companies to ensure “transparency, accountability and access to effective remedies for women whose rights have been violated online”.¹³³ The report made it clear that online violence is not separate from offline violence, but part of the same structural patterns of inequality.

More recently, the UN General Assembly Resolution 79/152 recognised “the growing impact of violence in digital contexts” and urged states to strengthen measures to

125 See European Union Agency for Fundamental Rights. (2014). *Violence against women: An EU-wide survey: Main results report*.

126 See European Parliamentary Research Service. (2024). *Cyber violence against women in the EU* (PE 767.146), p. 3.

127 See Fundamental Rights Agency (2025) *Fundamental Rights Report 2025*, p. 65.

128 See Plan International (2020) *Free to be online? Girls' and young women's experiences of online harassment*.

129 See UNICEF (2021) *The state of the world's children 2021: On my mind – promoting, protecting and caring for children's mental health*.

130 See Internet Watch Foundation (2022) *Sexual abuse imagery of girls online at record high following pandemic lockdowns*.

131 See European Institute for Gender Equality (2017) *Cyber violence against women and girls*.

132 Ibid.

133 See UN Human Rights Council (2018) *Report of the Special Rapporteur on violence against women, its causes and consequences on online violence against women and girls from a human rights perspective* (A/HRC/38/47), para. 25, 69.



address it. The resolution called for closer cooperation with technology platforms, stronger accountability for perpetrators, and expanded support services for survivors. It reaffirmed that online violence “*undermines women’s full, equal and meaningful participation in all areas of society*” and highlighted the need for coordinated international responses.¹³⁴

The Council of Europe Committee of Ministers Recommendation CM/Rec(2019)1 on preventing and combating sexism provides the first international definition of sexism and sets out explicit measures for the internet and social media, the media and advertising, and the justice sector, offering important guidance to prevent online sexist behaviours that can escalate into TFVAWG.¹³⁵

The Digital Services Act (DSA), adopted in 2022, requires online platforms to take stronger responsibility for what happens on their services. They must put in place clear ways for users to report illegal content and are expected to work with independent organisations recognised by the EU to identify and report harmful content. Importantly, recital 87 refers to very large online platforms used for sharing pornographic content, stressing that they must ensure survivors can have non-consensual intimate or manipulated images removed quickly and without unnecessary delays. These platforms are also obliged to conduct risk assessments to prevent the spread of illegal content and the harm it can cause to human dignity, including risks linked to TFVAWG.¹³⁶

Recognising the serious nature and prevalence, as well as a need to develop a response to this form of VAWG, the Group of Experts on Action against Violence against Women and Domestic Violence (GREVIO) published in 2021 their General Recommendation No. 1 on the digital dimension of violence against women (General Recommendation No. 1) under the Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention). General Recommendation No. 1 defines the “*digital dimension of violence against women*” and affirms that state obligations to prevent, protect, prosecute, and coordinate policies apply entirely in online and digital contexts.

A significant step forward came with the Directive (EU) 2024/1385 on combating violence against women and domestic violence, adopted in May 2024. This Directive criminalised several forms of TFVAWG across the EU, including:¹³⁷

- Non-consensual sharing of intimate or manipulated material (Art 5)
- Cyber stalking (Art 6)
- Cyber harassment (Art 7)
- Cyber incitement to violence or hatred (Art 8)

WAVE framework for mapping TFVAWG

Countries across Europe have begun to respond to these developments, but recognition, resources and responses remain uneven. Women’s specialist services (WSS), often the first to advocate for legal change and support to survivors, remain crucial for monitoring TFVAWG, shaping effective responses and ensuring that survivors’ voices and needs are not overlooked. Within this context, WAVE has, for the first time, collected data from its members on TFVAWG. Although limited to two core questions, this section of data collection represents an important first step in documenting how WSS across Europe are observing the rise of TFVAWG and how they are beginning to adapt their responses. The findings highlight both the urgency of addressing TFVAWG and the innovation of WSS, providing a foundation for future research and stronger policy engagement.

Findings on TFVAWG in Europe

As part of its 2025 data collection, WAVE asked its members two questions: first, whether they had observed an increase in TFVAWG in the past two years and, if so, in which forms; and second, whether WSS in their country had developed new strategies to respond to this form of violence and support survivors, including how such initiatives were funded.

Patterns in TFVAWG

The responses from WAVE members to the question of whether they had observed an increase in TFVAWG over the past two years indicate a rise in such forms of violence, with 37 out of 46 countries (80%) responding ‘yes.’ In nine countries, respondents could not give a definite answer, mainly because such data is not collected.

Among the countries that responded, ‘don’t know,’ some noted uncertainty about whether there has been an increase or whether women are simply speaking out more now that TFVAWG has been more recognised as a form of VAWG. For example, in **Belarus**, survivors of domestic violence report that (ex) partners use stalking apps to follow them and track their movements. As most Belarusian civil

134 See United Nations General Assembly (2024) *Resolution 79/152: Intensification of efforts to prevent and eliminate all forms of violence against women and girls in the digital environment*, paras. 1–2.

135 See Committee of Ministers of the Council of Europe. (2019). *Recommendation CM/Rec(2019)1 on preventing and combating sexism*.

136 See European Union (2022) *Regulation (EU) 2022/2065 of the European Parliament and of the Council of 19 October 2022 on a Single Market for Digital Services (Digital Services Act)*, OJ L 277, 27.10.2022, p. 1–102.

137 See Directive (EU) 2024/1385 of the European Parliament and of the Council of 14 May 2024 on combating violence against women and domestic violence (OJ L 1385), Arts 5–8.

society organisations continue to work in exile and online, there is no opportunity to collect reliable data.

In **Bosnia and Herzegovina, Hungary and Lithuania**, WSS emphasised that while no data are collected that could confirm or provide evidence of an increase, they are receiving requests for support from survivors of TFVAWG. This confirms that such violence is occurring and highlights the urgent need for mapping its patterns, collecting data on the prevalence and designing and providing specialised services and support.

For those who responded ‘yes’ to this question, there are some differences within particular countries. For example, while the **Republic of Cyprus** reported an increase in TFVAWG, the WAVE member from **northern Cyprus** said that they had not observed such an increase. This may be due to a lack of data being collected or women not coming forward with reports of this type of violence. In both England and Iceland, respondents confirmed an increase in TFVAWG, though not all available data reflected this: in **England**, Women’s Aid reported a steady rate of around 10% of women survivors saying they have survived this type of violence, while other indicators showed growth, and in Iceland, women’s shelters observed an increase, while the counselling centre *Stígamót* did not.

Form of TFVAWG	Number of countries that reported it
Cyberstalking/surveillance/spyware	30
Online harassment/hate speech/threats	30
Image-based abuse/non-consensual intimate image sharing	26
Deepfakes/AI-generated abuse	10
Sextortion/blackmail	9
Doxxing	9
Grooming/minors	1

Table 13: Forms of TFVAWG reported in Europe

Looking at specific forms of TFVAWG across Europe, it emerges that the most common forms reported by women and girls are cyberstalking, surveillance, and the use of spyware, mentioned in 30 countries (65%). Online harassment, hate speech, and threats were equally widespread, also reported in 30 countries (65%). In addition, more than half of the countries (26, 57%) reported a rise in image-based abuse and non-consensual intimate image sharing. Emerging forms are also becoming visible:

deepfakes and other AI-generated abuse were noted in 10 countries, while sextortion through blackmail and doxxing were each reported in nine countries.

Across Europe, WAVE members pointed to a clear rise in TFVAWG, with evidence showing both its scale and its severe consequences. In **Albania**, GREVIO’s 2024 evaluation report found an increase in online VAW, warning that online stalking and harassment are on the rise. This type of violence should not be underestimated as it can result in grave consequences. Civil society organisations from Albania reported that there were cases of suicide committed in response to online harassment. In **Austria**, WSS describe a growing number of cases, while the study “(No) Space: Cyber-violence against Women in (Ex-)Partnerships”¹³⁸ stresses the grave impact of digital abuse, especially in situations of separation from abusive partners. **Denmark** has likewise documented a substantial increase in multiple forms of TFVAWG, with national data recording an escalation in sextortion, image-based abuse, and AI-generated deepfakes. This reflects an increase in gendered hate and intimidation in digital spaces. According to WAVE members from Denmark, in 2023–2024, deepfake pornography emerged as a growing threat. *StopChikane* and *Digitalt Ansvar* have received more reports from women whose faces have been digitally inserted into explicit videos or images without consent. Such material often circulates on platforms like Reddit or adult content websites and is difficult to remove.

In **Greece**, the government’s 5th Annual Report on Violence Against Women (2024) presents the Hellenic Police (ELAS) statistics showing that in 2023 women made up 55.3% of victims in online-threat cases, 69.6% in cyberstalking cases, and 73.6% in cases of non-consensual sharing of intimate images, where complaints also increased five times from 2022 to 2023 (from 52 to 266 cases).¹³⁹ Complementing police data, a study on young women (18–24) commissioned by the Research Centre for Gender Equality (KETHI) found that 59.3% reported experiencing some form of online violence, with most facing multiple forms (59.5%).¹⁴⁰ Together, these findings indicate both rising reporting rates and significant exposure to TFVAWG among young women.

A similar pattern is evident in **Poland**, where a 2024 national survey by the Fem Institute found that nearly one in three Polish women, internet users, have personally experienced online violence, which is an increase from one in four in 2017, with overall exposure rising to almost 70% when including those who witnessed such abuse. These data confirm a significant escalation in TFVAWG, such as online harassment, hate speech, cyberstalking, doxxing, and image-based abuse.¹⁴¹ Together, these examples show that TFVAWG is not only rising across Europe but is

138 See Habringer, M., Hoyer-Neuhold, A., & Messner, S. (2022) [No] Space: Cyber-violence against women in (ex-)partnerships.
139 See General Secretariat for Demography and Family Policy and Gender Equality (2024), 5th Annual Report on Violence Against Women.
140 See KETHI – Research Centre for Gender Equality (2024), Survey on online violence against young women (18–24).
141 See Fem Institute (2024), Raport: Cyberprzemoc wobec kobiet w Polsce [Report: Cyberviolence against women in Poland].



also causing profound harm, from silencing women's voices to devastating mental health impacts and, in the most tragic cases, femicide. TFVAWG often intersects with offline abuse, extending patterns of coercion and creating new risks to women's physical safety. This continuum between online and offline violence is illustrated by a case in the **Transnistrian region of the Republic of Moldova**, where a survivor staying in a shelter was targeted by her ex-husband, who posted a fake missing person notice online and spread it through Telegram and Facebook groups in an attempt to track her down and discredit her. The case shows how digital tools are used to extend coercive control into the physical world, continuing to endanger women even after they escape abusive relationships.

Despite the gravity of these forms of VAWG, responses to the questionnaire also show that in many countries, TFVAWG remains under-reported and poorly understood. Barriers to reporting include stigma, fear of not being believed, lack of trust in institutions, or simple unawareness that support services exist for online abuse. In **Belgium**, reports of TFVAWG have increased, with the Institute for Women and Men recording 1,126 cases in 2024, a 6.5% increase from the previous year.¹⁴² However, these figures show only a fraction of the problem, as many survivors never report violence. Shame, fear of retaliation, and the complexity of reporting procedures discourage them from filing complaints, leaving much abuse invisible. Official statistics, such as police data from **Flanders**, confirm long-term growing trends in cyber harassment, grooming, and cyberstalking,¹⁴³ but they too underestimate the scope of TFVAWG, since many survivors are unable or hesitant to report.

Certain groups of women, such as migrant and minoritised women, as well as teenage girls, are even less likely to report experiences of TFVAWG due to additional cultural, linguistic, or social barriers. This uneven recognition means that while services observe growing demand, the true extent of this type of violence is likely higher than what is currently documented. In recent years, there has been an increase in technology-facilitated violence against migrant women in **Malta**. This type of violence includes online harassment, stalking, doxxing, image-based abuse and threats, often occurring alongside offline violence and can have a severe impact on the safety and mental health of these women. The perpetrators frequently originate from the same community or from communities that share similar languages and cultures. The survivors may lack the courage to report these incidents, and many choose to either decrease their online activities or hide their identities to avoid judgment from their community when seeking help.

Across Europe, WAVE members consistently highlighted that TFVAWG disproportionately targets those women who are visible in public life. Politicians, journalists, women's rights defenders, and feminist activists are frequent targets of online harassment, smear campaigns, deepfake pornography, and coordinated hate speech designed to silence or discredit them. In **Ukraine**, a 2025 study by "Women in Media" revealed that an alarming 81% of female journalists surveyed had experienced online violence.¹⁴⁴ In **Serbia**, digital spaces have increasingly become sites of smear campaigns and online abuse directed against female journalists, public figures, and politicians, particularly those engaged in activism or outspoken on social issues. A similar pattern is visible in **Moldova**, where women in politics, journalism, and activism are frequent targets of online harassment and image-based abuse. In **Kosovo**, women who are vocal on social and political issues, including activists, journalists, and public figures, are especially exposed to harassment and threats on platforms such as Facebook, Instagram, and TikTok. This abuse not only threatens individual women but also undermines democratic participation, as it discourages women from engaging in politics, media, and civic activism. The situation is particularly grave for human rights defenders in **Belarus**, where feminist and LGBTIQ+ activists in particular face online harassment, deepfakes and hate speech aimed at censoring and silencing them.

Young women and girls were another group repeatedly identified as highly exposed to TFVAWG. They are especially vulnerable to cyberbullying, grooming, and image-based abuse. In **Denmark**, Save the Children reported that the number of young people experiencing image-based abuse has tripled since 2021.¹⁴⁵ In **Georgia**, both the police and women's organisations documented rising cases of online threats, sexual harassment, and grooming of underage girls, showing how easily perpetrators exploit young people's presence on social media and gaming platforms. In **Sweden**, national crime survey data confirm significant gender differences: over 40% of girls reported receiving unsolicited sexualised images, more than a third had experienced offensive messages online, and one in five had images or videos of them posted without consent.¹⁴⁶ The combination of peer pressure and widespread use of social media means that harms spread quickly, often leaving young women without effective means of protection and access to justice.

New strategies employed by WSS

Most WAVE members reported that WSS have developed new strategies to address TFVAWG. Around two-thirds of countries (30 out of 46) gave a clear 'yes', often describing concrete initiatives such as new digital safety tools,

¹⁴² See Institute for the Equality of Women and Men (2025) *Annual statistics on violence against women*.

¹⁴³ See Belgian Federal Police (2024) *Quarterly crime statistics report*, Q3.

¹⁴⁴ See Women in Media (2025) *Her Voice, Their Target: Gendered Online Violence Against Ukrainian Women Journalists*.

¹⁴⁵ See Save the Children Denmark (Red Barnet) (2023) *Report on image-based abuse among youth*.

¹⁴⁶ See Swedish National Council for Crime Prevention (Brå) (2023) *National Crime Survey*.

survivor support protocols, or awareness campaigns. By contrast, about a third of respondents (12 out of 46) indicated that WSS in their countries had not yet developed specific strategies in this area. Four out of 46 countries provided uncertain or partial responses, including **Bulgaria, Croatia, Italy, and Ukraine**. Here, organisations acknowledged growing awareness of TFVAWG and noted that individual services were taking steps, such as staff training or limited online counselling, but they emphasised the lack of comprehensive strategies or sustainable funding. Furthermore, there are differences within the same countries, for example, services in the **Republic of Moldova** started developing new approaches to address TFVAWG, while the **Transnistrian region** did not. Similarly, while strategies are in place in most of the **United Kingdom**, they have not yet been developed in **Wales**.

In the countries that responded 'yes', WSS have developed a variety of strategies to address TFVAWG. These range from creating digital tools such as apps and chatbots to introducing new safety protocols that help women protect themselves from online stalking and surveillance. Many services have also strengthened their work through training for staff and other professionals, awareness-raising and advocacy for legal and policy reforms. Taken together, these strategies show how women's organisations are adapting their response to new forms of violence and finding innovative ways to support and protect women and girls.

→ Digital tools and apps

Several WSS have created or adapted apps and chatbots to support women survivors of TFVAWG better. In **Albania** and the **Czech Republic**, the *Bright Sky* app was launched, offering information about violence, risk and safety assessment, and access to support services. In **Poland**, the feminist organisation Feminoteka, together with CARE, developed the WhatsApp chatbot *Viktoria*, which provides women and girls with confidential support and counselling, legal information, and referrals. In Luxembourg, WSS introduced *TinyCheck* in shelters and women's centres to detect stalkerware on devices, helping women regain control over their digital safety. **Spain** introduced 24/7 chatbots and virtual assistants for women survivors of TFVAWG, while in the **Netherlands**, the *SafetyNed* initiative provides digital safety advice and direct online support to women survivors of cyberviolence.

CASE STUDY: Albania and the Czech Republic – The Bright Sky App

In both **Albania** and the **Czech Republic**, WSS have co-operated with state institutions and international partners to launch the *Bright Sky* app, adapting a tool originally developed in the United Kingdom. The app provides women with accessible information about different forms of abuse, helps them assess the safety of their relationships, and offers guidance on where

to seek support. Importantly, it also includes safety features to help women increase their digital security and find services in their area.

In **Albania**, the app was developed through a partnership between Vodafone Albania Foundation, UN Women, and government bodies. Besides launching the app, disadvantaged women were given smartphones, internet access, and training to ensure they could make use of the technology. In the **Czech Republic**, the app was implemented by the ROSA Centre for Women, in collaboration with Vodafone Foundation, the Police, and the Ministry of the Interior, and is available in Czech, English, and Ukrainian.

By combining information, safety tools, and service directories in one place, the *Bright Sky* app strengthens women's capacity to recognise and respond to TFVAWG. It also illustrates how partnerships between WSS, technology providers, and public institutions can create innovative resources tailored to women's needs.

→ Safety protocols and protection from surveillance

Women's specialist services in several countries have developed new safety protocols to address online monitoring, spyware, and stalkerware. In **Denmark**, the Danish Stalking Centre designed national guidelines to support women facing online monitoring by abusive partners. In **Norway**, crisis centres updated their protocols to include digital security measures, such as disabling location tracking and securing online accounts. In **Switzerland**, WSS now integrate structured digital safety planning into their services, including spyware checks, account protection, and device security.

CASE STUDY: Türkiye – women's organisations' initiatives

In recent years, women's organisations in **Türkiye** have begun developing strategies to make TFVAWG more visible and to respond to its growing impact, particularly among young women in dating relationships. These efforts include awareness-raising campaigns, workshops on online safety and consent, and publications illustrating how perpetrators use digital tools for monitoring and control, blackmail, and making threats to women's reputation. Some initiatives highlight the overlap between digital and offline abuse, as women report that partners not only share or threaten to share intimate images but also use apps, passwords, and financial accounts to control them. The Federation of Women's Associations of Türkiye has partnered with StopNCII.org to help women prevent and respond to the non-consensual sharing of intimate images. While these initiatives are still relatively new and



face challenges due to limited resources, they mark an important shift towards recognising TFVAWG as a distinct but interconnected form of GBV.

→ Training and professional capacity-building

Many WSS have invested in training and resources to build the capacity of their staff and professionals to respond effectively to TFVAWG. In **Austria**, the *#Gemeinsamgegen-Cybergewalt* project produced factsheets, workshops, and a handbook for practitioners titled *Ist das schon digitale Gewalt?* (Is this digital violence?). In **Germany**, umbrella organisations such as the Federal Association of Rape Crisis Centres and Women's Counselling Centres (*Bundesverband Frauenberatungsstellen und Frauennotrufe – BFF*) and Association of Women's Shelters (*Frauenhauskoordination – FHK*) provided sensitisation and training to WSS staff. In **Ireland**, Safe Ireland partnered with universities to develop a digital badge programme that certifies professionals in recognising and responding to TFVAWG. In **Lithuania**, WSS organised specialised training to build staff capacity to identify and address online VAWG. A local domestic abuse service in **England**, IDAS, partnered with a regional crime unit to run Technology-Facilitated Abuse workshops designed for practitioners working in the VAWG sector and associated professions.

→ Awareness-raising and prevention work

Prevention and awareness-raising initiatives have also grown across countries. In **Albania**, the Children's Human Rights Centre of Albania and the iSIGURT initiative on online safety launched a campaign in 2024 highlighting that over half of reported cases of online violence affected women and girls. In **Greece**, WSS developed EU-funded school-based projects (*SAFENET*, *CyberBuddy*) to educate young people about TFVAWG and build digital literacy. In **Hungary**, the organisation NANE implemented the project *I Can Choose to Say No*, which empowered teenage girls to stand up to online sexual violence in relationships. In **Romania**, organisations such as ANAIS integrated TFVAWG into their campaigns and participated in international feminist networks to raise awareness of AI-based online violence like deepfakes. In **Malta**, the *BeSmartOnline!* programme has been used to educate youth about online risks, including digital forms of violence. In **Belgium** (Brussels-Wallonia), multiple NGOs ran awareness-raising campaigns with government support, including projects targeting sexism and cyberviolence in schools and workplaces.

CASE STUDY: Hungary –

“I Can Choose to Say No. Empowering youth, especially girls, to stand up against cyber sexual and gender-based violence in intimate partner relationships”

In 2019–2021, together with partners from other countries, Serbia, Croatia and Spain, the women's

organisation NANE from Hungary implemented a project entitled: *“I can choose to say no. Empowering youth, especially girls, to stand up against cyber sexual and gender-based violence in intimate partner relationships”*.¹⁴⁷

The project was supported by the European Commission's Rights, Equality and Citizenship Programme. The main objective was to contribute to ending cyber sexual and gender-based violence (SGBV) in youth (aged 15–19) intimate partner relationships, especially by empowering girls and supporting them to stand up to violence.

Key activities included research on the awareness and attitudes of secondary school students and teachers regarding cyber SGBV in intimate relationships, as well as training and capacity-building for project partners. NANE also developed educational resources and conducted awareness-raising activities for both youth and teachers, equipping them with tools to challenge harmful attitudes and support safe online practices.

The project placed strong emphasis on youth mobilisation, encouraging girls to speak out against cyber violence and to build solidarity among peers. Materials developed through the project, such as research reports, leaflets, and a thematic website, continue to be used in schools and communities after the project's completion, extending its long-term impact.

→ Policy and legal reforms

In some contexts, WSS have directly influenced national policy and legal frameworks on TFVAWG. In **Albania**, the national helpline integrated TFVAWG counselling protocols and submitted a draft law on digital violence to parliament in 2024. In the **Republic of Moldova**, legal reforms explicitly recognised online forms of violence and new coordinated support units were set up in hospitals to provide integrated responses. In **England**, WSS advocacy helped create the 2023 Online Safety Act, which requires the Office of Communications to develop guidance, in consultation with the Domestic Abuse Commissioner, summarising measures that can be taken to reduce the risk of harm to women and girls, including measures for online platforms. The national organisation Refuge, in a project funded by Google, launched their technology-facilitated abuse and economic empowerment strategy, which has supported local women's centres across England by raising awareness of tech-facilitated abuse, recognising leaders in the sector in this area, driving legislative change to support survivors, and improving responses to survivors by increasing agency awareness. In **France**, members of the National Federation of Women Solidarity (*Fédération Nationale Solidarité Femmes – FNSF*) network worked to ensure that TFVAWG was included in national policy frameworks on VAWG.

147 See NANE (2021) *I Can Choose to Say No* project resources.

CASE STUDY: Sweden – women’s organisations’ initiatives

In Sweden, WSS have been among the earliest in Europe to integrate TFVAWG into their advocacy and prevention strategies. Over the past decade, organisations have pushed for limits on online pornography, accountability for tech platforms, and the integration of consent and digital violence into sexual education curricula. This advocacy has contributed to legislative changes, including the criminalisation of certain forms of online exploitation. Women’s organisations have also worked with cybersecurity experts to develop practical guidelines for shelters and safe houses, helping women prevent and respond to cyberstalking. Furthermore, WSS successfully influenced the government’s national action plan (NAP) on VAWG to include a dedicated focus on the ‘digital and tech-based dimensions’ of men’s violence against women, with a mapping exercise commissioned by the Swedish Gender Equality Agency to be finalised in 2026. Sweden’s approach demonstrates how sustained advocacy can change national frameworks and connect service provision and frontline responders with systemic change.

→ Removing harmful content and platform cooperation

Another area of work against TFVAWG has focused on helping women remove abusive content and pressuring platforms to act more responsibly. In **Belgium**, the Institute for the Equality of Women and Men offers legal support to women whose intimate images have been shared without consent, helping them instigate and pursue requests for removal of such content. In the **Netherlands**, *Offlimits* and partner organisations published a 2023 report monitoring the implementation of online sexual violence laws, advocating for stronger responsibilities on platforms. In **Serbia**, the NGO SHARE Foundation’s Cyber Intimacy platform provides step-by-step guidance for women to report non-consensual intimate content to tech companies and demand removal.

CASE STUDY: Denmark – women’s organisations’ initiatives

Denmark stands out for its comprehensive, multi-layered response to TFVAWG. Women’s organisations have launched services such as *StopChikane*, a national helpline for online violence that supports women in removing harmful content, conducting risk assessments and safety planning, and securing temporary devices. In cooperation with the Danish police cybercrime unit and tech platforms like Meta and TikTok, the service helps women to seek the removal of non-consensual intimate images and strengthen their online safety. Other specialised initiatives include the

Danish Stalking Centre’s Intervention Against Digital Stalking project, which developed screening tools and guidelines for professionals, and the *Q-Værk Project*, a digital aftercare service supporting women leaving abusive relationships. These projects are supported by a mix of public funds (including the Victim Fund and NAPs) and private foundations such as *Velux*. Together, they illustrate how WSS can combine direct survivor support, platform accountability, and structural funding to create a resilient response to TFVAWG.

Overall, the findings show that while many WSS across Europe are advancing their work to address TFVAWG, progress is uneven. Strategies range from developing apps and digital tools to awareness-raising campaigns, capacity-building, policy advocacy, and cooperation with tech platforms. In some regions, especially Western and Northern Europe, WSS have created comprehensive approaches, while in others, initiatives remain ad hoc and fragmented, or dependent on short-term donor funding. Importantly, many WSS have integrated TFVAWG into existing structures and service provision models, such as helplines, intake forms, or counselling protocols. However, many gaps and challenges still remain. Structural and sustainable funding is rare, coordination at the national level is often missing, and many women remain without adequate protection or access to justice. These dynamics highlight the creativity and resilience of WSS, but also the urgent need for stronger institutional support.

Conclusion

The evidence collected across Europe by WAVE members clearly demonstrates that TFVAWG is a rapidly growing problem. Thirty out of 46 countries reported a rise in TFVAWG, with cyberstalking, online harassment, and image-based abuse among the most common forms. The consequences for women and girls have been grave, with reports of suicides linked to online harassment, a significant increase in non-consensual intimate image sharing complaints, and survey data showing that young women are among the most exposed to multiple forms of online violence. At the same time, newer forms such as deepfakes, sextortion, and doxxing are rapidly spreading throughout Europe, underscoring how quickly perpetrators adapt their methods to rapidly advancing technology. This constant growth and adaptability of different forms of TFVAWG undermines the police and courts’ ability to investigate and punish such crimes, leaving many women and girls without adequate protection and making it difficult to bring perpetrators to justice.

However, recognition of TFVAWG as a distinct and severe form of violence has also increased. WSS and the feminist movement have been the first to respond to the growing demand for support from women and girls facing violence



online. They have taken the lead by integrating TFVAWG responses into helplines, intake forms, and counselling; developing apps and chatbots to improve access to support; implementing awareness-raising and prevention projects; and collaborating with technology platforms to remove harmful content. In many countries, it was women's organisations that first named TFVAWG, collected data, and designed practical solutions that are now influencing national strategies and EU-level debates.

The current state of TFVAWG in Europe can be described, therefore, by the perpetrators' quick adaptation to rapidly developing technology and means of using it to perpetrate violence on the one hand, and feminist WSS and grass-roots organisations' innovation, creativity and resilience on the other. While the challenges are severe, the foundation laid down by WSS shows that solutions are possible. What is urgently needed now is sustained political will, long-term structural funding, and coordinated action at both national and EU levels to effectively respond to technological change and ensure that women and girls are fully protected in online and offline spaces.

Recommendations

- States must ensure widely available, accessible, gender-responsive, survivor-centred and trauma-informed services for survivors of TFVAWG, adapted to the needs of women and girls. These should include legal and psychological counselling, online safety advice, and pathways for long-term recovery.
- States should guarantee sustainable, long-term public funding for specialist TFVAWG services, recognising that WSS continue to lead effective survivor-facing responses through the development of feminist-led digital tools such as apps, chatbots, and online counselling platforms.
- Platforms and internet intermediaries must take responsibility for preventing TFVAWG, including through safety-by-design principles, human rights risk assessments, transparent reporting, and ensuring the timely removal of harmful content and prevention of re-uploads. Key to this is that platforms employ trained moderators with gender expertise and develop ongoing partnerships with WSS, youth groups, survivors, and feminist organisations to ensure that standards, safeguards, and responses address survivors' needs.
- Governments should establish national systems to collect sex- and age-disaggregated data on TFVAWG, complemented by research on survivor experiences, to shape policy development, monitor progress and develop referral pathways.
- Governments and EU institutions must ensure that TFVAWG is fully integrated into national and European frameworks on preventing and combating VAWG, criminalising all forms of TFVAWG while also enabling civil remedies such as restraining and protection orders. In addition, legal frameworks should align with relevant European legislation on data management and digital environments to ensure that apps, gaming spaces, and other online platforms are safe-by-design.
- When drafting or revising laws and policies on TFVAWG, governments must engage in regular, transparent consultations with WSS, youth groups, survivors, and researchers, ensuring their expertise guides evidence-based measures that explicitly recognise and address the compounded forms of abuse experienced by survivors with intersecting identities, including women with disabilities, LGBTQI+ individuals, and women from Black and minority ethnic backgrounds.
- States must ensure that survivors can obtain timely removal of unlawful or abusive online material, access to justice and compensation from perpetrators and complicit platforms.
- Justice systems should adopt survivor-centred, trauma-informed approaches that protect survivors' dignity and privacy, including free legal aid and effective remedies, while fully integrating TFVAWG into broader strategies on intimate partner and sexualised violence.
- States must strengthen accountability by addressing sexism, gender stereotypes, systemic discrimination and norms that normalise abuse, recognising TFVAWG as part of the continuum of VAWG, and ensuring environments where women and girls can safely assert their rights both online and offline.
- Governments must invest adequately in youth-focused prevention and support programmes in schools, ensuring they address cyberbullying, grooming, image-based abuse, and peer-to-peer online harassment. This should include the promotion of comprehensive, accessible, and evidence-based prevention education and digital literacy training involving WSS, feminist and youth organisations, survivors, schools, media, and the private sector.

Femicide



Introduction

Femicide is the gender-related killing of women and girls. These crimes are most often perpetrated by men and are rooted in stereotyped gender roles and historically unequal power relations between women and men, which instil a sense of possessiveness and superiority among men toward women. The term highlights that women are not killed as a matter of chance or coincidence, but within the context of unequal power relations, misogyny, and discrimination. The word was first used publicly by feminist activist Diana Russell in 1976 as *“the killing of females by males because they are female”*.¹⁴⁸ Understanding femicide helps to make visible the structural nature of violence against women and girls (VAWG) and the continuum that connects everyday forms of harassment, control, and abuse with the most extreme outcome, the killing of women.

There is still no uniform or internationally agreed-upon definition of femicide. States, international organisations, and researchers use the term in different ways, making comparisons difficult. Many countries where it is used limit its meaning to the killing of a woman by her current or former intimate partner. Feminist NGOs and international mechanisms, including the United Nations Special Rapporteur on Violence Against Women and Girls (UN Special Rapporteur), have long argued that this is too narrow. The UN Special Rapporteur’s report on gender-related killings of women¹⁴⁹ identified a wide range of forms that femicide can take, including so-called honour killings, dowry-related deaths, killings linked to witchcraft accusations, conflict-related killings, those connected to sexualised violence or trafficking, and cases where a woman’s death results from state failure to protect her from known violence. The report also recognised indirect femicides, such as suicides following domestic or sexualised violence, deaths caused by unsafe abortion or maternal neglect, and other harmful practices. Each of these deaths reflects the continuum of VAWG and the structural discrimination that women experience throughout their lives.

According to the most recent global estimates published by the United Nations Office on Drugs and Crime (UNODC) and UN Women in November 2024, around 140 women and girls were killed every day in 2023 by an intimate partner or family member.¹⁵⁰ The report shows that homes remain the most dangerous place for women, and that the real number of femicides is likely much higher due to poor data and underreporting. At the European level, the

European Institute for Gender Equality (EIGE) notes that women make up 87% of victims of intimate partner homicides and 60% of victims of domestic homicides.¹⁵¹ However, differences in definitions and data systems between countries mean that these figures are incomplete and cannot yet show the whole picture.

Femicide is not an isolated crime. It is the most visible and irreversible expression of the system of male violence that limits women’s freedom and equality in all areas of life. Naming it, documenting it, and understanding its patterns are crucial steps in preventing future killings and achieving justice for all women whose lives have been taken by gender-based violence (GBV).

Feminist organisations have been central to naming, documenting, and politicising femicide. Long before official data collection began, women’s rights groups exposed the scale and patterns of gender-related killings of women and girls, challenging the silence of state institutions and the normalisation of male violence. Their activism continues to drive visibility, accountability, and policy change at local, national, and international levels.

Collecting reliable and comparable data on femicide remains one of the most significant challenges for both governments and civil society. The lack of conceptual clarity directly affects how data are gathered, classified, and reported, making it difficult to measure the full scale of the problem or to compare figures across countries. Most countries record data on homicide rather than femicide. Where gender is noted, it is often only as a demographic variable, not as a motive or contextual factor. As a result, many killings of women that are clearly gender-related remain invisible in official statistics. In some contexts, only intimate partner or family-related homicides are counted as femicide, while in others, even these cases are recorded without reference to gender. This inconsistency limits the accuracy of national and regional estimates and obscures the broader social patterns of male violence against women (VAW).

Across Europe, feminist organisations have developed a wide range of initiatives to document femicide and push for systemic responses. Many collect and verify data on women killed by men, often drawing on media reports, court records, and direct information from women’s specialist services (WSS). Their efforts go beyond counting deaths. They analyse patterns of coercive control, identify failures in protection, and link individual cases to broader

148 See Diana E. H. Russell (2012) *Origin of Femicide*, San Francisco: DianaRussell.com.

149 See United Nations Human Rights Council (2012) *Report of the Special Rapporteur on Violence against Women, Its Causes and Consequences, Rashida Manjoo: Gender-related killings of women (femicide)*, A/HRC/20/16, Geneva: United Nations.

150 See UNODC and UN Women (2024) *Femicides in 2023: Global Estimates of Intimate Partner/Family Member Femicides*. United Nations. https://www.unodc.org/documents/data-and-analysis/briefs/Femicide_Brief_2024.pdf

151 See European Institute for Gender Equality (2025) *Improving the collection of national administrative data on femicide in the EU*, Brussels: EIGE.

institutional and social inequalities. They publish statements, hold press conferences, run media campaigns and organise protests to draw attention to all the systemic failures that often precede the individual cases of femicide. This approach understands systematic data collection as a feminist act of resistance and remembrance, a way to restore visibility to women's lives, holding institutions accountable, and driving prevention efforts.

International obligations

In Europe, there has been growing recognition that femicide must be named, measured, and prevented as a distinct form of GBV. The European Union's ratification of the Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention) in 2023 and the adoption of the European Union Directive (EU) 2024/1385 of 14 May 2024 on combating violence against women and domestic violence (EU Directive) in 2024 mark significant progress. These instruments require EU Member States to strengthen prevention, protection, and prosecution, and to improve the collection and sharing of data. Although the Istanbul Convention does not explicitly use the term femicide, it is implicit in its definition of GBV as violence directed against a woman because she is a woman. The Istanbul Convention obliges Member States to take all necessary legislative and policy measures to ensure the right of women to live free from violence, including through effective risk assessment, coordinated institutional response, and accountability for failure to protect women. The Council of Europe's monitoring body, Group of Experts on Action against Violence against Women and Domestic Violence (GREVIO), has repeatedly noted that the prevention of femicide must be a core component of Member States' implementation of the Convention.¹⁵²

The European Observatory on Femicide (EOF) serves as a central hub for research, monitoring, and data exchange on gender-related killings of women across Europe.¹⁵³ Its aim is to build a comparative knowledge base on femicide, promote shared methodologies, and strengthen collaboration between academic institutions, women's organisations, and policy actors. The EOF develops both quantitative and qualitative research tools and facilitates the exchange of national data and good practices to support the creation of femicide watches and observatories at the national level. Within this framework, the *FEM-United project* (2020–2022)¹⁵⁴ provided an important foundation for developing harmonised indicators and guidelines for data collection and analysis, enabling a more systematic understanding of femicide in Europe.

Globally, the UN Special Rapporteur on Violence against Women and Girls continues to promote the establishment of national femicide watches or observatories. These mechanisms are meant to record and analyse every case of gender-related killing of women and girls, identify risk factors, and ensure accountability of state institutions. The call to “*count femicides so that counting counts for women*”¹⁵⁵ remains as urgent as ever. In her 2021 report, the UN Special Rapporteur set out what a femicide watch/observatory should be: mainly a mechanism that records and analyses every case of gender-related killings of women and girls, disaggregates data by sex, age, perpetrator-victim relationship, motive, and uses the findings to strengthen prevention and ensure accountability.

WAVE framework for collecting data on femicide

For the Country Report 2025, WAVE collected information on whether state institutions used the term femicide in legislation, policy, or data systems. Respondents reported what types of data were collected, who collected them, and whether victim characteristics and relationships with perpetrators were recorded. The questionnaire also gathered numbers of femicide victims in 2024, information on victims' previous contact with authorities or specialist services, and whether systematic reviews or femicide analyses were conducted.

Findings on femicide in Europe

According to information provided by WAVE delegates from 35 out of 46 European countries, at least **2,871 women** were victims of femicide in 2024.¹⁵⁶ This figure includes estimates supplied by national experts and feminist organisations where official data were missing. No data on femicide were available from nine countries,¹⁵⁷ and in several others, only partial or unofficial information could be collected. Of the total, 969 femicides were recorded in EU Member States and 1,902 in European countries outside the EU. In many cases, these figures represent minimum estimates, as systematic monitoring of femicide remains uneven across Europe. The data were gathered from a combination of official sources, media reports, research findings, and tracking by WSS.

152 See Council of Europe (2011) *Convention on preventing and combating violence against women and domestic violence (Istanbul Convention)*, CETS No. 210.

153 See European Observatory on Femicide (2018) About the EOF.

154 See University of Malta (2022) *FEM-United Project: Developing tools for the prevention and investigation of femicide in Europe*.

155 See Dubravka Šimonović (Special Rapporteur on violence against women, its causes and consequences) (2021) *Report of the Special Rapporteur on violence against women, its causes and consequences: Taking stock of the femicide watch initiative (A/76/132)*, New York / Geneva: United Nations.

156 Data primarily from 2024; see footnotes in the tables for exceptions.

157 These are: Azerbaijan, Belarus, Liechtenstein, Luxembourg, Malta, Norway, Romania, Slovakia, and Latvia.

 Estimated number of femicide victims (2024)	Europe (46)	EU Member States (27)	European countries outside the EU (19)
	2,871	969	1,902

Table 14: Estimated number of femicide victims in 2024

Only 23 out of 46 countries reported any disaggregated information in these subsections, meaning that the figures below represent partial totals based on available data, not the full 3,982 femicides recorded across Europe in 2024. Among the 1,120 femicide cases where such information was available:

- 843 women (76%) were killed by a current or former intimate partner¹⁵⁸
- 217 women (19%) were killed by a family member other than a partner¹⁵⁹
- 36 women (3%) were killed by an acquaintance¹⁶⁰
- 24 women (2%) were killed by other perpetrators, including strangers or unknown offenders¹⁶¹

These findings reaffirm that femicide is most often committed by men with a close personal relationship to the victim. However, the limited number of countries providing detailed breakdowns highlights persistent gaps in systematic, gender-sensitive data collection across Europe. Besides these numbers, **Spain** additionally reported that in 2024, 36 children were killed by fathers/current or former intimate partners of the mother.

Information on whether femicide victims have previously received specialist support, from women's organisations or state-run services, remains extremely limited across Europe. Information on this question was available in only 16 of 46 countries. Among them, 13 countries reported that *some* victims had received support, two reported that *most* victims had done so, and one stated that *none* had. In contrast, 22 countries indicated that they *did not know*, while eight countries provided no data at all. This indicates that in nearly half of European countries, there is no systematic information about whether women killed by men had prior contact with WSS, shelters, or other forms of specialist support. The lack of linkage between femicide data, service records, and law enforcement systems severely limits the ability to identify missed opportunities for protection and to learn from fatal cases.

Overall, these findings underscore a critical weakness in national monitoring systems: the absence of mechanisms that systematically record and analyse victims' prior contact with protection and support services. Addressing this gap is essential for understanding institutional failures and for developing effective prevention measures against femicide.

Across Europe, many women killed by men had previously reported violence to the police, prosecutors, or other public authorities, yet their search for help did not prevent their deaths. Out of 46 countries, 24 reported that *some* or *most* femicide victims had earlier contacted institutions, most often the police. In contrast, 15 countries stated that this information is not known, and the remaining countries provided no data. This suggests that while evidence of prior reporting exists in more than half of the countries, it remains fragmented, inconsistent, and poorly integrated into official data systems.

For example, in **Belgium**, most victims reported their perpetrators (in the vast majority of cases, their current or former partner) to the police; however, these reports were not adequately followed up by the police and/or judicial authorities. In **Hungary**, NANE's Silent Witness cases rely on information from police and media reports, meaning the available data depends on what is publicly disclosed. In several femicide cases in 2024, authorities had prior knowledge of the violence, such as situations where victims unsuccessfully requested restraining orders or where perpetrators had been released from prison shortly before the killing. In **Iceland**, victims reported the violence to social services.

In the **Netherlands**, media reports and testimonials from relatives of women who have been murdered indicate that there were early warning signs. Police, healthcare services and other public bodies could have followed up if they had had enough knowledge and awareness of red flags and would have connected incidents to possible patterns of intimate terror or stalking. Across countries, respondents emphasised that information is fragmented among police, courts, and WSS and is rarely analysed collectively. In **Republika Srpska**, in two of the three femicide cases, the domestic violence had previously been reported to the police, and both victims had left the shared household before the murder was committed. In **Spain**, more specific data are available, showing that 15 of the 48 women had previously reported their perpetrators to the police.

¹⁵⁸ Based on information provided in 23 countries.

¹⁵⁹ Based on information provided in 17 countries.

¹⁶⁰ Based on information provided in 6 countries.

¹⁶¹ Based on information provided in 8 countries.

CASE STUDY: the Netherlands – the story of Gea

On 15 May 2020, Barbara Godwaldt's only sister, Gea, was killed by her husband, who then took his own life. A relationship that seemed to start happily—despite early signs that he had been violent with a former girlfriend—gradually turned into a nightmare of intimate terror for her and her daughter, who lived in what initially appeared to be a large, blended family and a dream come true. Over time, arguments became more frequent and more abusive. After fights, he bombarded her with messages, sometimes threatening to harm himself. He made it difficult for her to meet friends. As the violence escalated, she told him she questioned whether the relationship could still be saved. The dynamic shifted: he would cry and apologise. His regrets and promises to return to how things once were convinced her to give him another chance, but her gut feeling that something was terribly wrong remained.

After a long period of abuse, threats, intimidation, and insults, she decided she had to end the relationship—for herself, and to show her daughter that no one should be treated so badly.

She tried to leave the house, but he stopped her. Gea was found dead in the living room, and although the police could not identify the order of events, it was clear she had endured a bloody fight and suffocation. Her husband had hanged himself in the stairwell.

After the murder, Barbara, Gea's sister, and their family were thrown into a rollercoaster of distressing events. Minutes after learning how her sister had died, Barbara had to contact the perpetrator's family to arrange picking up clothes so Gea could be buried in something clean. She was interrogated for hours at the police station. She had to identify her sister; it took her 20 minutes to recognise her. The officer assigned to support her had little experience and began talking about his deceased grandmother—the only dead body he had ever seen. Her niece, who still lived with her mother but had not been home that night, had no clothes, study materials, or letters from her mother, as everything was inside the house, now a crime scene. They had to sneak in to retrieve them. The house had not been cleaned after the murder, so they cleaned the blood and other traces themselves. The police refused to acknowledge that Gea had been killed by her husband. Out of sympathy for his family, they repeatedly called it a 'family tragedy'. Then came disputes over inheritance rights: the aftermath seemed endless.

The guilt, the lack of recognition for what had been done to her sister, and the pain and injustices she witnessed in her own family and among other survivors drove Barbara to become one of the most active and visible advocates against femicide in the Netherlands. Her goal: to prevent this loss for others. After an intense period of traumatic grief, she wrote the Femicide Action Plan and submitted it to the government. Her plan—and the support she mobilised—formed the basis of the Dutch National Action Plan (NAP) *Stop Femicide!*. To this day, campaigning against femicide and advocating for more effective measures has become her daily work. She is one of the leading women and spokespersons of the Federation of Relatives of Victims of Violence (<https://www.nabestaandengeweldslachtoffers.nl/>). She advises several organisations, supports survivors, and tirelessly advocates to improve femicide policies at both national and regional levels. She appears regularly in the media and as a keynote speaker at conferences.

The use of the term 'femicide'

According to information provided by respondents from 46 European countries, in 2024, the term femicide was officially used in nine countries, was under development or legislative consideration in four, and was used only informally or in public discourse in 22. Respondents from 11 countries reported that the term was not used at all by the state or other authorities.

Where femicide is officially recognised, it appears in law, public policy, or national data systems. In **Croatia**, the 2023 amendments to the Criminal Code introduced femicide as a specific aggravated criminal act of murder when the crime is committed against a woman because of her gender. The reform was the result of long-term advocacy by women's organisations, particularly the Autonomous Women's House Zagreb and the Women's Network Croatia, who pushed for legal recognition of gender-related killings as part of the continuum of VAW.

In **Belgium**, a law adopted in June 2023 formally recognised femicide in the Penal Code, introducing legal terminology and state-level monitoring of gender-related killings. The law followed years of feminist campaigning led by the Stop Femicide (*Stop Féminicide*) collective, and it established obligations for the government to collect and publish disaggregated data on femicides annually.

In **Cyprus**, the term was incorporated into legislation in 2022, criminalising femicide as a distinct offence and mandating prevention and awareness measures. **France**

also uses *fémicide* in legal and statistical practice, while **Italy** records 'killing of women for gender-related reasons' (*omicidio di donne per motivi di genere*) in official reports by the Ministry of the Interior.¹⁶² **Spain** officially uses the expression 'fatal victims of gender violence' or 'macho murders' in legislation (LO 1/2004 and its development) and in judicial reports. However, in the Statistical Portal of the Government Delegation against Gender Violence, there is a section entitled 'Feminicides', where the annual records of fatal victims are grouped. In the NGO and media sphere, the term femicide is used with some frequency to refer to the gender-related murder of women.

In the **Netherlands**, the term femicide has been officially endorsed since 2024, following the launch of the National Plan Stop Femicide! by three ministers, and while it is not yet embedded in the legal system, it is now used in national and local policies as well as by the Public Prosecutor's Office. Since August 2024, the term femicide has been included in **Moldovan** legislation through its definition in the Law on the Prevention and Combating of Violence against Women and Domestic Violence (Law No. 45 of 01.03.2007). However, it has not yet been incorporated into the Criminal Code as a specific offence.

In most European countries, however, the term femicide is not officially recognised, even if it is increasingly visible in society. In 22 countries, respondents noted that while the state does not use the term, it has become part of public, academic, or media discourse, often promoted by women's rights organisations. Feminist NGOs and activists continue to lead in naming femicide, using it to highlight the gendered patterns behind killings of women and to demand stronger prevention and accountability measures. In **Hungary and the Czech Republic**, women's organisations such as NANE and proFem monitor cases and organise public awareness campaigns that explicitly name femicide, while official institutions continue to record these cases as homicides or domestic violence. In **Malta**, the term femicide is formally recognised but not a separate offence; instead, since 2022, the Criminal Code treats 'femicidal circumstances' as aggravating factors requiring maximum sentences and excludes 'crime of passion' defences in gender-motivated killings. Malta has also established a Femicide Research and Advocacy Observatory, led by the Women's Rights Foundation with the national Commission on Gender-Based Violence, to improve data collection, awareness, and policy responses.

Finally, 11 countries reported that the term is not used at all, either by the state or within public debate. In these contexts, killings of women remain classified under general homicide or domestic violence categories.

CASE STUDY: Belgium – the #StopFémicide law and initiative

In October 2023, Belgium's Law of 13 June 2023 on the prevention and combating of femicide and gender-based homicides,¹⁶³ commonly referred to as the #StopFémicide Law, entered into force. It establishes legal definitions of femicide, gender-based homicide, GBV, sexualised violence, and coercive control.

The #StopFémicide Law sets out four distinct categories of gender-related killing:

- Intimate femicide, such as the killing of a partner or former partner
- Non-intimate femicide, such as the killing of a woman in prostitution networks
- Indirect femicide, including deaths resulting from forced abortion or female genital mutilation
- Gender-based homicide, encompassing killings of trans, gender-fluid, or non-binary persons committed in the context of GBV

The law establishes an obligation for the annual publication of an official report on femicides in Belgium. This report must include:

- The number of complaints to the police
- The number of femicides recorded
- The characteristics of victims and perpetrators
- The relationship between victim and perpetrator
- The number of convictions of perpetrators

The report is to be reviewed by a consultative committee, which analyses the data and makes recommendations for state action to prevent femicide. However, at the time of reporting, no official report has yet been published. Currently, femicide data in Belgium continue to be collected primarily by civil society initiatives, most notably the Stop Femicide platform, which has long documented cases and advocated for official recognition of the problem.

This legislative progress represents the outcome of sustained feminist mobilisation, particularly through the *Stop Femicide* initiative, which has been instrumental in pushing for state accountability and legal recognition of femicide.

Data collection on femicide

According to information provided by respondents from 46 European countries, only six countries reported that the state officially collects data on femicide. These are **Croatia, France, Italy, Spain, Albania, and the Republic of Moldova**. In these countries, the collection of data on

¹⁶² On 25 November 2025, Italy passed a law introducing the crime of femicide and making it punishable by life imprisonment. The law defines femicide as causing the death of a woman as an act of discrimination or hatred because she is a woman, or with the view to repress her rights, freedoms, or expression of personality.

¹⁶³ See *Droits Quotidiens* (2023), *La loi #StopFémicide a été publiée*.

gender-related killings of women is embedded within state institutions, usually involving the police, justice or equality bodies, as well as national statistical offices.

In 29 countries, respondents reported NGOs collect or compile data, often maintaining national femicide databases or annual reports. Police collect data in 21 countries, usually under general homicide or domestic violence statistics, without identifying the killings as gender related. Other state bodies, including ministries, ombudspersons, and statistical offices, collect related data in 19 countries, but rarely under a specific femicide category. Media monitoring is an important source in 10 countries, while academic research is cited in 12 countries.

In **Hungary**, NANE systematically compiles data each year on women killed by partners or family members, drawing on police reports and media sources. More recently, NANE has also begun recording cases of children killed in the context of domestic or intimate partner violence. Each year, during the 16 Days of Activism campaign, NANE organises the *Silent Witness March* to honour victims and demand more decisive state action to prevent femicide. The march features life-sized red silhouettes representing victims, each inscribed with the woman's name, date and place of death, relationship to the perpetrator, and brief details of the case, including prior violence and institutional responses. In parallel, PATENT Association and the independent online newspaper *Mérce* launched a joint initiative to increase visibility and public accountability: a public femicide counter, displayed on social media under the hashtag #MercePatentCounter (*#MercePatentSzámláló*). Updated regularly, the counter tracks every woman and child killed by gender-based or domestic violence, while also exposing systemic misogyny and state inaction that enable these crimes. Together, these initiatives combine data collection, public awareness, and commemoration, keeping femicide in Hungary visible as both a personal and political issue.

In the **Czech Republic**, the organisation proFem documents femicide cases and uses them to advocate for legal and policy reform. In **Serbia**, the Women Against Violence Network systematically monitors media reports, verifies cases through partners, and publishes annual reports that are recognised by the state as the most reliable national source. In **Austria**, the Association of Austrian Autonomous Women's Shelters (*Autonome Österreichische Frauenhäuser – AÖF*) operates a femicide watch that collaborates with shelters and specialist services. In **Germany**, the initiative *One Billion Rising Deutschland* and academic researchers publish femicide data and commentary, filling the absence of state reporting. Similar initiatives exist in **Greece, Türkiye, Kosovo, and North Macedonia**, where feminist groups monitor killings and raise public awareness. In several of these contexts, media outlets now also use the term femicide in their reporting, showing that feminist advocacy is reshaping the public understanding of gender-based killings even in the absence of state recognition.

Only **Belgium** and the Republic of **Moldova** reported collecting data on suicides linked to GBV. In **Belgium**, femicide and gender-based homicide data collection frameworks also cover indirect femicides, including deaths by suicide resulting from coercive control, forced marriage, or other forms of GBV. This inclusion follows the adoption of the #StopFéminicide Law (2023), which recognises indirect femicide as a category of gender-related killing. While official implementation is still in progress, the law mandates the gathering and publication of data on all forms of gender-based killing, including suicides related to gendered violence.

Systematic analysis and review of femicide cases

Only a minority of European countries have mechanisms in place to analyse femicide cases and identify lessons for prevention systematically. Out of 46 countries, 12 respondents reported that such processes exist or are under development. This means that only about one-quarter of European states have taken any steps towards institutionalising femicide reviews or analytical mechanisms, despite long-standing international calls to establish national femicide watches.

In most countries, femicides are still treated as isolated criminal incidents, investigated case by case without broader institutional learning. Respondents highlighted that authorities rarely assess how victims interacted with services before their deaths or whether intervention could have prevented the killing. This lack of systematic review prevents the identification of recurring patterns of institutional failure, such as ineffective police response, poor risk assessment, or the absence of inter-agency coordination.

Some positive examples show gradual progress. In **Spain**, the Ministry of Equality conducts ongoing monitoring and publishes detailed annual reports on femicide, distinguishing between intimate partner, family-related, and other gender-related killings. **France** has developed the National Observatory on Violence Against Women (*Observatoire national des violences faites aux femmes*), which produces thematic analyses of femicides and issues recommendations for policy improvement. **Croatia** has a state-led femicide watch coordinated by the Ombudswoman for Gender Equality, which analyses cases and publishes statistics in cooperation with the police. **Belgium's** 2023 #StopFéminicide Law obliges the government to publish an annual report on femicide and to review the data through a consultative committee. In **Poland**, the Women's Rights Centre runs a feminist-led Femicide Observatory project that monitors killings of women and advocates for stronger state accountability. In **Finland**, the NGO Women's Line collects information to contribute to the international Femicide Watch initiative. These examples show that while only a handful of countries have established dedicated observatories, feminist organisations remain the main drivers of femicide monitoring across Europe.

In **Malta**, a Femicide Research and Advocacy Observatory was established, led by the Women's Rights Foundation in collaboration with the National Commission on Gender-Based Violence and Domestic Violence. The Observatory collects data, raises awareness, and contributes to policy responses to femicide and gender-related killings.

In **Albania**, the Femicide Observatory records key indicators such as the number of cases, prior reporting to authorities, protection orders, age, gender, marital status, education level, relationship between victim and perpetrator, and municipality. However, data on ethnicity, disability, and socioeconomic status of the victims are still missing, leaving significant gaps in understanding intersecting risk factors. The People's Advocate (Ombudsman), with support from UN Women Albania under the Joint UN Programme End Violence against Women (funded by the Government of Sweden), published the report *"Murder of Women and Girls – Femicide in Albania (2021–2023)"*.¹⁶⁴ The report analyses femicide cases nationwide, evaluates the role of key institutions, including police, prosecutors, courts, municipalities, and health services, and assesses the effectiveness of protective measures, support services, and rehabilitation programs. It provides detailed municipal-level data and information on cases involving protection orders or prior police reports, contributing valuable insight into institutional responses and prevention gaps.

In the Republic of **Moldova**, the *Commission for Monitoring and Analysing Cases of Domestic Violence Resulting in Death or Serious Bodily Injury* was recently renamed the *Commission for the Analysis of Femicide and Domestic Violence Cases Resulting in the Victim's Death or Serious Bodily Injury or Health Damage (Commission)*.¹⁶⁵ The Commission uses a comprehensive data collection form to document all cases of domestic violence leading to death or serious harm. The police provide most of the information, though the Commission is authorised to gather data and to request cooperation from other relevant authorities and institutions. The form collects detailed information on the act itself, including the legal classification and circumstances of the offence, as well as on the victim's background, such as citizenship, nationality, sex, age, education, means of subsistence, and whether the victim had dependents or owned a firearm. It also includes data on any addictions or harmful habits (for adult victims), information about past violations of protection orders within the previous five years, and criminal records. Where applicable, details on minor victims are also recorded. In addition, the form captures information about the relationship between the victim and the perpetrator, the circumstances surrounding the commission of the offence, post-incident information about both parties, and the history of their relationship or other relevant background details. Medical data and social

assistance information related to the case and the individuals involved are also included.

Despite these advances, the majority of countries lack formal, legally mandated femicide review mechanisms. In many cases, feminist organisations, shelters, and researchers have stepped in to fill this gap, conducting independent analyses of femicide cases using media monitoring and court data. These initiatives, such as those led by women's rights networks in **Serbia, Hungary, and Austria**, provide critical insight into systemic failures and have been instrumental in pushing governments toward recognising femicide as a structural problem rather than an individual crime. However, without official status or access to state-held information, their findings often remain outside policy and institutional reform processes.

CASE STUDY: Bosnia and Herzegovina – livestreamed femicide

On 11 August 2023, 35-year-old Nermin Sulejmanović livestreamed the murder of his former partner, Nizama Hećimović, from the town of Gradačac. In the videos posted to Instagram, he announced viewers would "see a murder live", then shot her on camera. He went on to kill two other people and wound several more before taking his own life. The incident triggered immediate national and international outrage, with authorities confirming the sequence of killings and the use of Instagram Live.

The case quickly raised questions about failures in state protection. Media and officials reported that Hećimović had previously reported harassment and violence, and within days, authorities said they would investigate why a restraining order had not been issued and whether police and a local judge failed to act on risks. A year on, observers argued that Bosnia and Herzegovina still had not delivered the legal reforms needed to recognise and prevent femicide, underlining gaps between public pledges and concrete change.

Public reaction was swift and sustained. Thousands marched in cities across Bosnia and Herzegovina, demanding better protection for women, effective police action, and recognition of femicide in law – carrying banners such as "Stop femicide" and "Silence is approval". UN officials condemned the livestreamed killing as horrifying, while national leaders called for stricter penalties and systemic improvements. These mobilisations kept the case on the political agenda, linking it to broader patterns of GBV in the region.

The Gradačac femicide also exposed gaps in social

¹⁶⁴ See People's Advocate (Ombudsman) of Albania (2023), *Murder of Women and Girls – Femicide in Albania (2021–2023)*.

¹⁶⁵ See Government of the Republic of Moldova, Ministry of Internal Affairs (n.d.), *Activitatea Comisiei de monitorizare a cazurilor de violență în familie soldate cu consecințe*.

media platform accountability. Civil society groups wrote to Meta, asking why the video remained online for hours and urged stronger content-moderation protocols to prevent the amplification of GBV, avoid causing distress to viewers and the retraumatisation of survivors. Combined with the state's own failures, this case underscored how both online platforms and public authorities must act swiftly and responsibly to protect women's lives and dignity, including through legal recognition of femicide and systematic post-homicide reviews.

Conclusion

The findings in this chapter demonstrate that femicide is neither isolated nor inevitable: it is the most visible expression of unequal power relations, enabled by institutional gaps and sustained by social norms that tolerate male control over women and girls. While feminist organisations have made femicide visible, naming it, counting it, and insisting on its political meaning, state recognition and response remain uneven. Although legal frameworks have advanced in Europe, the absence of harmonised definitions and gender-sensitive recording practices continues to obscure the scale and patterns of these killings.

WAVE's findings underline three structural weaknesses. First, visibility: only a minority of states collect femicide data in a way that captures gendered motives and relationships, leaving WSS and feminist NGOs to fill critical evidence gaps. Second, protection: many women killed by men had previously reported the violence to public authorities, and some of them sought help from WSS and state-run services, yet weak risk assessment, inadequate inter-agency coordination, and poor enforcement resulted in missed opportunities to prevent escalation. Third, accountability and learning: most countries still lack mandated femicide reviews, which means institutional failures are neither systematically examined nor remedied.

Progress is possible and already underway. Where states partner with WSS, adopt clear definitions, and institutionalise data systems, the picture becomes sharper and prevention more plausible. Country experiences highlighted in this chapter, alongside emblematic cases that mobilised public outrage, demonstrate that reform follows when civil society evidence meets political will. Yet scattered initiatives are not enough; Europe needs consistent, comparable, and legally binding responsibilities to protect women and girls.

Overall, the findings indicate that femicide is foreseeable and preventable when state systems respond adequately to women's reports of violence. Establishing mandatory femicide reviews and integrated national data systems, linking police, judicial, and support service information, would allow governments to identify failures in protection and strengthen preventive responses across Europe.

Recommendations

- States should explicitly integrate femicide into national strategies and policies addressing VAWG, recognising gender-related killings and suicide linked to domestic abuse as femicides, ensuring such cases are addressed in data collection and accountability mechanisms, and concrete actions are identified to eliminate such preventable deaths.
- States should establish and sustain femicide watch mechanisms to monitor and highlight femicide as a fatal manifestation of the continuum of VAWG, in line with the recommendation of the UN Special Rapporteur on VAWG. This should involve close collaboration with the European Observatory on Femicide through sustained funding, sharing of relevant data and strengthened cooperation with national authorities and women's specialist organisations to facilitate effective international collaboration and inform policy.
- Governments should establish mandatory, gender-sensitive review mechanisms for all killings of women, with coordinated intervention and real-time data exchange between WSS, law enforcement, health and judicial institutions to systematically analyse each case, identify risk factors, learn from institutional failures, and support consistent classification of femicide where applicable.
- EU institutions should ensure that minimum comparable homicide data is collected in official statistics of all European countries, including disaggregation by sex of victims and perpetrators and their relationship, to ensure identification of femicide. Additionally, data on complaints and reports of violence previously raised by the victims to the authorities should be recorded, as well as information on which statutory and voluntary agencies were previously involved in supporting the victim to ensure accountability of all involved support agencies and to guarantee the effective protection of women experiencing violence.
- Governments and international institutions should support public awareness and commemoration initiatives led by feminist organisations, recognising the social and symbolic power of naming femicide and remembering women killed by men as a political act against gender inequality.
- States should ensure training and capacity-building on femicide prevention is mandatory for police officers, prosecutors, judges, and healthcare as well as social service professionals, focusing on recognising patterns of coercive control, escalating violence, and early warning signs.



**TABLE 15: ESTIMATED NUMBER OF FEMICIDE VICTIMS
IN EU MEMBER STATES AND DETAILS ON FEMICIDE DATA
COLLECTION**

(Data primarily from 2024, see footnotes for exceptions)

	Reported number of femicide victims	Details on femicide data collection
Austria	27	The term femicide does not exist in the Austrian law, and therefore, data on femicide are not being collected. The Association of Autonomous Austrian Women's Shelters (AÖF) is counting cases of femicide and attempted femicide/cases of severe violence against women each year, according to media coverage and publishes the numbers online.
Belgium	26 ¹⁶⁶	Data collected by the NGO Feminist Platform for Combating Violence Against Women through media reporting.
Bulgaria	20	No systematic data collection on femicide. Available data is collected by NGOs.
Croatia	18	Data is collected by the Ombudswoman for Gender Equality, who runs the femicide watch, and by the police and research bodies. The gender-based killing of women was introduced into the criminal law as a separate offence.
Republic of Cyprus	1	Available data is collected by the state and the police. The police collect data on killings, and not specifically on femicide cases.
Czech Republic	22	Data is collected by the media and NGOs. The police collect data on homicide victims, including those killed by intimate partners or family members, but do not record femicide cases. The term "femicide" is not officially defined within national legislation. Non-governmental organisations play a crucial role in monitoring and documenting femicides in the Czech Republic. For instance, the ROSA Centre for Women runs the annual project Silent Witnesses, which maps femicide cases reported in the media.
Denmark	2	Data on the number of women killed is collected by the state and the police. Available data on femicide is collected by research bodies and institutions, NGOs and the media.
Estonia	5 ¹⁶⁷	Available data is collected by the state, the police and NGOs. The Ministry of Justice and Digital Affairs collects and reports data to the public, but detailed information on femicide is collected by NGOs.
Finland	14 ¹⁶⁸	Available data is collected by the state, the police, the media, NGOs, and research bodies and institutions. More specific data collection is done periodically by Statistics Finland. Data on femicide victims is mainly reported by the media, which is gathered by Women's Line Finland and reported to the Femicide Watch initiative.
France	94	Data on the number of killings of women by current or former intimate partners are collected by the state, the police, NGOs, research bodies and institutions. Quantitative data is published annually by the Statistical Service of the Ministry of the Interior. Different civil society organisations collect femicide data (e.g., <i>#NousToutes</i> , <i>Inter Orga Féminicides</i>).
Germany	360 ¹⁶⁹	The data is collected by the state, the police, NGOs, and research bodies and institutions. Most of the federal states do not collect comprehensive data on any gender-specific motive of the perpetrator or their relationship to the victim. Initiatives such as <i>feminizid.org</i> and <i>One Billion Rising</i> collect data on femicide, and multiple studies are conducted by research institutions.
Greece	27	Available data is collected by the police, NGOs, research bodies and institutions. The term "femicide" is not included in the Penal Code, and data is collected under art. 299 on homicide.
Hungary	27 ¹⁷⁰	Available data is collected by the police, the media, and NGOs. Homicide data is collected by the Unified Investigation and Prosecution Crime Statistics (<i>ENYÜBS</i>). The NGO NANE collects data on femicide based on police reports and related media articles.
Ireland	5	Data collected by the NGO Women's Aid Ireland under its Femicide Watch initiative.
Italy	98	Available data is collected by the National Institute of Statistics and by the Ministry of the Interior, as well as by women's NGOs. The women's organisation <i>Casa delle Donne di Bologna</i> has been publishing data for several years based on media monitoring.
Latvia	N/A	—

166 This number includes one indirect femicide.

167 One case was related to domestic violence; the victim was a four-year old girl.

168 Identified through media reporting by Women's Line, official number still unconfirmed.

169 Number of femicides in Germany in 2023.

170 This figure comes from the NGO NANE's Silent Witness data collection.

	Reported number of femicide victims	Details on femicide data collection
Lithuania	14 ¹⁷¹	Homicide data is collected by the police and transferred to the Register of Crimes, which is operated by the Information Technology and Communications Department under the Ministry of the Interior.
Luxembourg	N/A	No official data on femicide.
Malta	N/A	Malta established a Femicide Research and Advocacy Observatory, led by the Women's Rights Foundation with the National Commission on Gender-Based Violence, to collect data, raise awareness, and shape policy responses to femicide and gender-based killings. In Malta, the term 'femicide' is formally recognised but not as a separate criminal offence. Since 2022, the Criminal Code has included 'femicidal circumstances' as aggravating factors in homicide cases.
Netherlands	45 ¹⁷²	Available data is collected by research bodies and institutions. The University of Leiden collects this data, which comes from several sources, mainly police and other judicial reports. ¹⁷³
Poland	67 ¹⁷⁴	Available data is collected by the media and NGOs. The Women's Rights Center implements the <i>Femicide Observatory project</i> , ¹⁷⁵ and <i>Femicide in Poland</i> is the largest database of materials related to this topic in the country. Between 2017 and 2023, there were an estimated 300 femicides in total during this period. ¹⁷⁶
Portugal	20	Data is collected by the state, the police and NGOs. Data is disaggregated by sex, age and relationship between perpetrator and victim.
Romania	N/A	No systematic data collection on femicide. The National Inspectorate of Police and other institutions collect some (limited) data on victims of domestic violence.
Slovakia	N/A	Available data is collected by the police. This data only includes victims of homicide, disaggregated by sex. The data is not publicly available.
Slovenia	6 ¹⁷⁷	Available data is collected by the state and the police. Additionally, NGOs and research institutions also collect femicide data. The police record the number of victims of domestic violence by gender and age, as well as criminal offences related to sexualised violence, murder, manslaughter, and other violent acts. However, publicly available data is not disaggregated by gender, nor is it possible to determine the relationship between the victim and the perpetrator of the criminal offence.
Spain	48	Data is collected by the state, the police, NGOs, and research bodies and institutions. In the Statistical Portal of the Government Delegation against Gender Violence, there is a section entitled 'Feminicides', where the annual records of femicide cases are collected.
Sweden	23	Available data is collected by the state, the police, and the media. The Swedish National Council for Crime Prevention annually gathers statistics on deadly violence (including deadly violence against women). Aftonbladet, a Swedish daily newspaper, administrates a database with names and stories of women murdered in Sweden since January 1, 2000. ¹⁷⁸
TOTAL (27)	969	

171 According to the State Data Agency on 'Persons Murdered in the Domestic Environment'.

172 Based on various sources and definitions (commonly derived from media reporting), it is estimated that approximately 45 women are murdered by a partner or ex-partner per year (one every 8 days).

173 There is an ongoing discussion on the use of the term 'femicide' for all murders of women and on the methodology and use of data. More information here: <https://www.universiteitleiden.nl/binaries/content/assets/governance-and-global-affairs/isga/factsheet-femicide-in-nederland.pdf>.

174 This figure is for 2023 (estimates for 2024 are not yet available) and comes from the NGO Femicide in Poland.

175 Available here: <https://cpk.org.pl/obserwatorium-kobietobojstwo/>.

176 A Map of all femicides which occurred from 2017 to 2023 can be found on the Femicide in Poland website here: <https://femicideinpoland.pl/polska-mapa-kobietobojstwo/>.

177 Estimate of femicide cases based on NGO information.

178 The database is being regularly updated at: <https://www.aftonbladet.se/nyheter/a/0G3n6A/dodade-kvinnor>.



TABLE 16: ESTIMATED NUMBER OF FEMICIDE VICTIMS IN EUROPEAN COUNTRIES OUTSIDE THE EU AND DETAILS ON FEMICIDE DATA COLLECTION

(Data primarily from 2024, see footnotes for exceptions)

	Reported number of femicide victims	Details on femicide data collection
Albania	8 ¹⁷⁹	Available data is collected by the state, police, and NGOs. In March 2024, the Observatory on Femicide was established to monitor and analyse gender-related killings of women and girls. The term femicide is not explicitly included in Albania's Criminal Code.
Armenia	9	Available data is collected by NGOs. Official statistics by the state do not use the term 'femicide victims'. The NGO Women's Support Center collects ongoing information on femicide, and the Coalition to Stop Violence Against Women publishes periodic reports on femicide.
Azerbaijan	N/A	Available data is collected by NGOs, including the Femicide Azerbaijan Platform (<i>Qadın Qətləri</i>), which compiles case-based lists and reports. Official databases focus on murders and domestic violence, but do not identify gender-motivated killings or track victim details.
Belarus	N/A	No data on femicide.
Bosnia & Herzegovina	12–14 ¹⁸⁰	Available data is collected by NGOs, research bodies/institutions and media through monitoring reports and is not fully reliable. State authorities do not conduct systematic analysis on femicide cases, and there are no official statistics on femicide at the state or entity level. By May 2025, amendments to the Criminal Code of the Federation of BiH introduced 'gender-based killing of a female person' as a criminal offence. The term femicide is mainly used by NGOs, activists and media.
Georgia	14	Available data is collected by the police and NGOs. The Public Defenders Office and NGOs analyse and collect data on cases of femicide.
Iceland	4	Available data is collected by NGOs.
Kosovo	3	Available data is collected by NGOs, media and research bodies such as the Kosovo Women's Network (KWN) and the Centre for Information, Criticism and Action (QIKA). The term femicide is not officially recognised in Kosovo's legislation or by state institutions. Killings of women are recorded as general homicides, without noting gender-based motives. There is no official or gender-sensitive data collection system.
Liechtenstein	N/A	N/A
North Macedonia	2	Available data is collected by NGOs and research bodies, particularly the National Network to End Violence against Women and Domestic Violence. The AIRE Centre and Gender Champions ¹⁸¹ in the Judiciary Network have conducted a regional analysis highlighting inconsistent legal qualification and the absence of femicide-specific data. The term femicide is not officially recognised in North Macedonia's legislation or statistics, though Article 123(2)(2-a) of the Criminal Code treats the murder of a woman related to gender-based violence as an aggravating circumstance. There is no centralised or disaggregated database, and the state and police only collect data on homicide disaggregated by sex.
Republic of Moldova	33	Available data is collected by the police, the state and NGOs. Since 2022, an inter-ministerial Commission has monitored cases of domestic violence resulting in death or serious injury. The Commission collects data primarily from the police and collaborates with medical, social, and educational institutions. While domestic violence-related deaths are tracked, femicide is not included in official statistics. The term femicide was introduced into Moldovan legislation in August 2024 under the Law on the Prevention and Combating of Violence against Women and Domestic Violence, but it is not yet included in the Criminal Code. In July 2025, the Parliament adopted amendments recognising femicide as a serious form of violence against women; the law awaits presidential promulgation.
Montenegro	4	Available data is collected by women's NGOs based on media reports. The term femicide is not officially used in Montenegro. Amendments to the Criminal Code are underway to introduce femicide as a separate criminal offence.

179 Number of femicides in 2023. Data retrieved from "The killings of women and girls – femicide in Albania (2021–2023)", available at: <https://avokatipopullit.gov.al/sq/list/publications/rraporte-speciale-1/>.

180 Due to the lack of official statistics, the number of femicides varies according to different sources. On average, one woman per month is a victim of femicide.

181 Number of deaths resulting from domestic violence in 2024, as collected by the police.

	Reported number of femicide victims	Details on femicide data collection
Norway	N/A	—
Russian Federation	1,174 ¹⁸²	Available data is collected by NGOs. There are only general statistics provided by the Ministry of Internal Affairs on the number of women killed annually. The NGO Consortium of Women's Non-Governmental Associations analyses court sentences relating to women killed since 2011.
Serbia	19	Available data is collected by NGOs, primarily the Autonomous Women's Centre, which has documented 423 femicides (2011-2024) through its online Femicide Memorial. The organisation FemPlatz also tracks femicide cases via the Femicide Map covering Serbia and neighbouring countries, based on media monitoring. There is no mechanism to systematically monitor or classify femicide.
Switzerland	20	Available data is collected by the police and NGOs. The Federal Statistical Office and the police do not keep separate records of femicides, and murders of women are simply categorised as 'domestic violence' or 'relationship crime'. There is no specific legal offence of femicide. Projects such as Stop Femicide document cases and estimate that one woman is murdered in Switzerland every two weeks.
Türkiye	378 ¹⁸³	Available data is collected by the state, the media, and NGOs. The Ministry of Interior typically discloses only the number of femicide cases, without providing any further details or contextual information; such data is not reliable as it is based on partial figures collected from the media. A media outlet (Bianet) collects data on femicide cases by compiling reports from the local press. In addition to the number of cases, the data includes information on the victim–perpetrator relationship, as well as where and how the femicides were committed. Additionally, the We Will Stop Femicides Platform also collects data, and in addition to the media figures, they rely on information gathered from families of victims who approach them.
Ukraine	118 ¹⁸⁴	Available data is collected by the state and the Office of the Prosecutor General. From 2024, the Office of the Prosecutor General collects data and publishes a report on the Results of Law Enforcement Activities Regarding the State of Prevention and Counteraction of offences related to Domestic Violence.
United Kingdom	103 ¹⁸⁵	Available data is collected by the state, the police, the media, NGOs, research bodies and institutions. The Femicide Census charity is the only comprehensive source of Femicide data across the UK (latest available report covers 2022). Other research bodies and institutions research femicide, but it is very much dependent on access to data, funding and the scope of the study.
TOTAL (19)	1,902	

182 Data from 2022, available at <https://www.wmmsk.com/femicid/>. Figures for 2024 are not yet available. Additional available data combines figures from 2022-2023 and reports 2,284 femicide victims. This figure is based on court cases analysed by the NGO Consortium of Women's Non-Governmental Associations for the period 2022-2023. Over the course of these two years, at least 3,436 women were killed in Russia, 2,284 of them were killed in a situation of domestic violence, most of them were killed by their partner (2,123). More info available here: <https://algoritmsveta.com/>

183 Data from media monitoring. According to the Ministry of Interior, 326 femicides were recorded in 2024.

184 According to the Office of the Prosecutor General's report on persons who suffered from criminal offenses related to domestic violence, and child witnesses of criminal offenses, 118 women were intentionally murdered in 2024 (Article 115 of the Criminal Code). In total, there were 474 female victims of obvious intentional murders (Articles 115-118 of the Criminal Code) in 2024. How many murders were gender-based is unknown.

185 Based on data from the Counting Dead Women project which collect details of women killed across the UK by calendar year. This data includes women over 14 who have been killed where a man or men were the principal suspect (but only includes cases where the man or men were charged of homicide). Femicide data is independently collected by NGOs in the respective nations of the UK. In Wales, 80 femicide victims were reported by NGOs and in Northern Ireland, the NGO Women's Aid Federation Northern Ireland reported 7 femicide victims in 2024.

ALBANIA



GENERAL COUNTRY INFORMATION

Population	2,761,785 ¹
Female population	1,394,864 ²
Member of Council of Europe (year)	1995
Member of European Union (year)	No
Member of United Nations (year)	1955
CEDAW ratified (year)	1994
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2013

SUMMARY

There is a national women's helpline in Albania, operating 24/7 and free of charge, offering multilingual support. Additionally, there are other regional helplines and apps supporting survivors of violence against women. There are 26 women's shelters with at least 153 beds, and at least 12 women's centres. One rape crisis centre is available in the capital, and two "one-stop-shop" services for cases of sexualised violence have been integrated in hospitals. Primary prevention is included in the National Strategy for Gender Equality, which has a specific budget allocated to it. The state collects data on primary prevention and on women's specialist services, but data collection is not comprehensive and accurate. Data is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	4,658 calls

Albania has one official state-wide women's helpline, the National Counselling Line for Women and Girls (*Linja Kom-bëtare e Këshillimit për Gra dhe Vajza*, tel.: 116117). This helpline operates 24/7, is free of charge, and therefore, meets the standards of the Istanbul Convention. It is operated by Hotline Albania and offers multilingual support according to the availability of the counsellors. It provides psychological and legal counselling services to women and girls who are survivors of domestic violence, gender-based violence, or sexual abuse, as well as referrals to shelters and protection services. The helpline receives state funding, donations and international funding, and in 2024, it received 4,658 calls, representing a 12% increase compared to the previous year. The most common forms of violence reported by callers were domestic, physical and psychological violence.

In addition to the national helpline, several local organisations across Albania operate regional helplines and provide support services. For instance, the Psycho-Social Centre "Vatra", a non-profit organisation which supports survivors of human trafficking and violence, provides legal and psychosocial counselling through a helpline available 24/7 and free of charge (*Qendra Psiko-Sociale "Vatra"*, tel.: +355 699333523). Digital tools like the Bright Sky Albania and Find Your Voice (*GjejZa*) apps also help women identify abuse and access help. There is also a National Child Helpline for children and adolescents, including survivors of violence (116111), an Anti-Trafficking Helpline managed by the Ministry of Interior (+355 8001212) and a Green Line for Elderly People, which provides referrals and social support.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	N/A
Total number of all shelters accessible to women	26
Existing number of beds	153
24/7 access	All

There are 26 shelters in Albania, offering at least 153 beds.³ These include two state-run shelters (one for survivors of domestic violence and one for survivors of trafficking), eight long-term shelters for survivors of gender-based violence, and 16 emergency shelters, which offer accommodation up to 72 hours. The number of beds available in Albania does not meet the country's needs. All shelters offer 24/7 access, and women are not expected to pay for their accommodation. The length of stay depends on the type of shelter, with limited time in emergency shelters and up to one year in shelters managed by NGOs.

According to law no. 121/2016 "On social care services in the Republic of Albania", in-house services provided by shelters include specialised support, housing, physical security, food, health care, legal assistance, psychosocial

1 The latest available population data on Eurostat is from 2023.

2 Ibid.

3 Number of beds from the WAVE Country Report 2023.

rehabilitation, employment empowerment, and reintegration services. According to Law 22/2018 "On Social Housing", the state must provide survivors of domestic violence with specialised housing, along with specialised social workers and guaranteed protection from the State Police for high-risk cases. Nevertheless, the specialised housing programme foreseen in this law has not been implemented yet.

In terms of accessibility, all shelters can accommodate women with uncertain residence permits and undocumented women, refugee women, transgender women, women with children with disabilities and unaccompanied minor girls. Most shelters can be accessed by and are accessible to women with physical disabilities, homeless women, women with substance abuse issues and older women, while some shelters are also equipped to support women with cognitive and sensory disabilities. There are also several specialised shelters in supporting survivors of trafficking (e.g., in Tirana, Elbasan, Vlorë) offering housing, legal aid, healthcare and vocational support. The *Vatra* Psycho-Social centre specialises in supporting both survivors of trafficking and specific forms of violence, such as forced marriage and honour-based violence, while the shelter *Edlira Haxhiymeri* in Tirana provides support for lesbian, bisexual, and transgender women, in collaboration with the *Streha* Centre. The Albanian Disability Rights Foundation (ADRF) offers specialised support (including technology-based tools and peer mentoring) for women with disabilities and partners with shelters to improve accessibility. While no shelters are exclusively for older women, some social centres provide support groups and services for older survivors of domestic violence.

While the state-operated shelters are financed directly by the ministry, other shelters rely on multiple sources of funding: state funding, donations, international funding, funds from religious institutions and crowdfunding. Women's NGOs with a feminist and gender-specific approach run seven shelters in Albania, while the others are run by other NGOs and faith-based organisations. Shelters are located in urban areas except for the shelter in Lezhë "*Shpija Rozalba*", which is in a rural area. The reasons why shelters had to decline referrals were the lack of space and capacity to support the survivor, the shelter not being adequately equipped to support women and children, if the survivors had severe mental health issues or if no protection order had been issued. The most common forms of violence reported by survivors in 2024 were domestic violence, physical violence and psychological violence.

In Albania, besides women-only shelters, there are other types of shelters that support survivors of violence against women. These include mixed-gender shelters that provide assistance to women, men, children, and LGBTQI+ individuals, particularly in cases involving trafficking or displacement. Municipal social centres across various cities offer emergency housing, legal aid, and psychosocial support to survivors of domestic violence, serving people

of all genders. Additionally, anti-trafficking shelters, such as those operated by the National Coalition of Anti-Trafficking Shelters (NCATS), provide comprehensive services to all survivors of trafficking, many of whom have experienced gender-based violence. Some faith-based and charity-run shelters also offer support to women in crisis, often collaborating with NGOs to provide specialised care. While these are not exclusively women-only shelters, they do assist survivors of violence against women and often refer them to more specialised services when necessary. Most of these shelters offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	12
Entities running women's centres	Women's NGOs State Faith-based organisations

There are at least 12 women's centres in the country, some of which are part of the Women's Empowerment Network in Albania (AWEN), while others are part of the Monitoring Network Against Gender-Based Violence. Five of these centres are exclusively for women, five are open to all survivors of gender-based violence, and two engage in advocacy activities or social services, supporting broader gender equality efforts. Women's centres offer various types of support, including counselling and psychological support, legal advice, financial and social welfare support, housing advice, employment counselling, referrals and collaboration with other services, representation at court and other institutions, and residential support services, although the capacity for this is limited. Women's centres also provide specialist support for survivors of sexualised violence, such as specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention and legal advice.

Women's centres in Albania are accessible to various groups of survivors: women with uncertain residence permits, undocumented women, refugee women, women with physical and cognitive disabilities, older women, unaccompanied girls and transgender women. The Albania Disability Rights Foundation provides specialised support to people with disabilities, and two centres are specialised in supporting transgender women. The *Streha* Centre offers specialised services for LGBTQI+ individuals who face violence, family rejection, and housing insecurity, and besides emergency accommodation, it also provides psychosocial counselling, guidance and referrals to specialised services, as well as support in accessing education, employment, and healthcare.

Women's centres in Albania are run by women's NGOs, the state, and religious institutions, and are funded through state funds, donations, international funding and religious institutions. They are only located in major cities.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There is one rape crisis centre in Albania, the Liliun Centre, based in Tirana, which provides medical, psychological, and legal support to survivors of sexualised violence. This centre also handles forensic evidence collection within a hospital setting. The centre is funded by the state and provides support to women, young women and children, and other survivors. Additionally, there are two new services in the regional hospitals in Fier and Shkodër, dealing with cases of sexualised violence and applying a “one-stop-shop” approach. Besides this, women’s centres and specialist NGOs play a crucial role in supporting survivors of sexualised violence, especially outside of the capital.

5. PRIMARY PREVENTION SERVICES

Public authorities in Albania collect data on primary prevention activities related to gender-based violence through surveys, monitoring referral mechanisms, and UN-supported programmes. However, such data collection is not comprehensive and systematic. Albania includes primary prevention activities in key documents such as the National Strategy for Gender Equality 2021–2030, and various municipal Gender Equality Action Plans. These documents focus on education, awareness, and community engagement to prevent gender-based violence, but the focus on evidence-based primary prevention measures, beyond general awareness raising, is still developing. The National Strategy for Gender Equality 2021–2030 emphasises reducing harmful practices and gender-based violence before they occur, using gender mainstreaming as a key prevention tool. Albania regularly finances various primary prevention activities against violence towards women and girls, including awareness-raising campaigns, education, community engagement, and capacity building. In 2023, approximately 9.6% of the national budget was directed to gender equality initiatives, covering both prevention and support services. The activities conducted in the country include feminist self-defence trainings, empowerment activities for women and girls, primary prevention activities specifically targeted to men and boys, training for primary prevention practitioners, self-care activities and awareness-raising campaigns or programmes. Albania has conducted evaluations of primary prevention activities against violence toward women and girls. These evaluations have been funded by both international organisations and public authorities, including national ministries, to help measure the effectiveness of prevention programmes and guide improvements in policies and services.

ARMENIA



GENERAL COUNTRY INFORMATION

Population	2,991,201
Female population	1,577,675
Member of Council of Europe (year)	2001
Member of European Union (year)	No
Member of United Nations (year)	1992
CEDAW ratified (year)	1993
CEDAW optional protocol ratified (year)	2006
Istanbul Convention signed (year)	2018
Istanbul Convention ratified (year)	No

SUMMARY

Armenia does not have a national women’s helpline, although there are regional helplines supporting women survivors of violence in every region of the country and there is a process underway to launch a national helpline. There are two women-only emergency shelters in Armenia with 24 beds for women and their children, and at least two other shelters which are accessible to women survivors, although the total number of beds is unknown. There are 11 women’s centres operating as domestic violence support centres as well as one sexual assault crisis centre for survivors of sexualised violence. Measures related to primary prevention are included in the country’s Strategy and Action Plan but there is no specific budget allocated to such activities. The state does collect data on women’s specialist services, including primary prevention activities, and this information is available upon request. The Ministry of Labour and Social Affairs partially subsidises domestic violence support centres. These centres are required to provide information on survivor demographics and other relevant information, such as the type of service provided.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

There is no national women's helpline in Armenia although there is a process underway to launch a national helpline. The Ministry of Social Affairs runs a national helpline (tel.: +374 114 and +374 119) which is not specialised but can refer domestic violence cases to regional helplines, according to the location of the survivor. Therefore, Armenia does not meet the Istanbul Convention standards for the provision of a national women's helpline.

There are 12 regional helplines specifically supporting survivors of domestic violence in Yerevan and across the 10 regions in Armenia. The helplines run by the registered domestic violence support centres are as follows: Women's Rights House (tel.: +374 77570870); Martuni Women's Community Council – Martuni (tel.: +374 94876502); Gavar (tel.: +374 94876505); Family Without Violence (tel.: +374 98092094); Sose Women's Issues (tel.: +374 98848453); Resource Center for Women's Empowerment (tel.: +374 77380053); You Are Not Alone – Vayots Dzor (tel.: +374 98886077); Ararat (tel.: +374 77802700); For Equality Human Rights Defender (tel.: +374 099788770); Youth Vanguard (tel.: +374 93574657); Talin-Huys (tel.: +374 91482035); and Women's Support Centre (tel.: +374 99887808). The helplines are funded through donations and international funding. In 2024, the helplines recorded a total of 3,877 calls, of which 1,980 were received by the Women's Support Centre. The most common forms of violence reported by callers to the helplines in 2024 were domestic violence, physical violence, and psychological violence.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	2
Total number of all shelters accessible to women	4
Existing number of beds	24 ⁴
24/7 access	All

There are two women-only shelters in Armenia with a total of 24 beds run by the Women's Support Centre (WSC), a women's NGO with a feminist and gender-specific approach, which operates as emergency shelters for survivors of domestic violence.⁵ Both women-only shelters offer 24/7 access, and women are not expected to pay for their accommodation in women-only shelters. On average, women and their children stay in women-only shelters for

a period of two or three months, which WSC has found to be sufficient for survivors to begin the process of rehabilitation and reintegration into society. However, if there is a serious security concern that warrants a longer stay, WSC offer the service for as long as it is needed. The types of in-house services most often provided by the women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services, practical advice, legal advice, and therapy. Shelters also offer self-help groups, life and parenting skills training.

The number of beds in Armenia is far below the Istanbul Convention standards. However, given the social norms around utilising emergency shelters, as opposed to relocating with relatives or finding other options, at the moment the number of shelter spaces is sufficient. The WSC has never had to turn away any women or children. However, as reporting patterns change and more women seek shelter accommodation, this may become an issue in the near future.

Both women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive or sensory disabilities, older women (65 years and above), women with older sons (14 years and above), women with children with disabilities, and unaccompanied girls under the age of 18. Neither of the women-only shelters can accommodate women with substance abuse issue, women with psychiatric diagnoses, or transgender women. For safety reasons and taking into consideration cultural norms and attitudes, WSC works with a partner organisation that can provide shelter support for transgender survivors. There is also a shelter for unmarried women with children up to the age of two, and one shelter for survivors of trafficking. These shelters can also offer accommodation to all survivors if the other shelters do not have availability.

Funding for the two women-only shelters primarily comes from foundation grants, international funding, and individual donors, as state funding only covers roughly 10% of the budget. The women-only shelters are located in Yerevan, meaning that there are no women-only shelters outside of the capital city and in other regions of Armenia. The main reason why women-only shelters had to decline referrals was the survivor being ineligible for support. The most common forms of violence reported by survivors to women-only shelters in 2024 were domestic violence, with psychological violence being the most prevalent. A large percentage of survivors had also experienced physical and economic violence, and a smaller percentage had experienced sexualised violence. A total of 274 women and children were accommodated in the women-only shelters in 2024.

⁴ This number only includes the beds in the two women-only shelters run by the Women's Support Centre (WSC).

⁵ A transitional shelter for survivors of domestic violence opened in early 2025. This shelter is fully funded by one foundation grant.

There are other shelters in Armenia, that can provide support to women survivors of domestic violence. However, these shelters are for vulnerable families and survivors of trafficking and are not specifically intended to support women affected by violence. These shelters are available to all survivors.

❖ 3. WOMEN'S CENTRES

Total number of centres	11
Entities running women's centres	Women's NGOs (11 centres)

There are 11 women's centres in Armenia which operate as domestic violence support centres and provide services to all survivors of gender-based violence, primarily women and their children. All women's centres provide counselling/psychological support, legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, and representation at court/police/social services. In addition, women's centres also provide hygiene products and medical help. Only one of the domestic violence centres (Women's Support Centre) provides emergency shelter support, and all centres refer to this organisation. However, some of the other centres offer financial support to women for their temporary housing needs. Additionally, all centres also offer specialised support to survivors of sexualised violence in the form of specialist psychological care, community awareness and education for prevention, and legal advice. All women's centres in Armenia can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive or sensory disabilities, older women (65 years and above), transgender women, and unaccompanied girls under the age of 18. There are no specialised centres supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Armenia. However, women's centres do have experience working with women with disabilities, refugee women, undocumented women, transgender and non-binary people.

Women's centres are all run by women's NGOs, and the majority of funding comes from foundation grants and international funds. Some funding for women's centres also comes from national authorities. There is one registered domestic violence support centres in each region of Armenia. In 2024, a total of 1,939 women were supported by women's centres.

In addition to the 11 domestic violence support centres, there are other centres providing support to women, although not specifically survivors of domestic violence, as well as centres supporting LGBTQI+ survivors of violence.

🏠 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There is one sexual assault crisis centre in Yerevan, the Sexual Assault Crisis Centre (SACC), which provides crisis support and also runs a helpline (tel.: +374 77991280). When needed, the SACC can also provide healthcare services. The centre provides needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, as well as legal advice. Funding for the SACC comes from projects financed by international organisations and donors. Besides the centre in Yerevan, SACC also has focal point NGOs in different regions of the country where specialists are trained to provide support to survivors of sexualised violence. The sexualised violence service is available to all survivors of sexualised violence, including women, men, children, LGBTQI+ individuals, and people with disabilities.

👤 5. PRIMARY PREVENTION SERVICES

Public authorities do collect data on primary prevention activities and provisions on primary prevention are included in the 2025–2028 Strategy and Action Plan for the Implementation of Gender Policy in the Republic of Armenia. Section five of this Strategy and Action Plan does include a definition of primary prevention and focuses on creating a safer and more secure environment and society for all women and girls. However, there is no specific budget allocated to primary prevention activities. Available activities include feminist self-defence trainings, empowerment activities for girls and women, school-based primary prevention programmes, training for primary prevention practitioners, self-care activities, and awareness-raising campaigns or programmes. There is no evaluation of the impact of primary prevention activities in Armenia.

AUSTRIA



GENERAL COUNTRY INFORMATION

Population	9,158,750
Female population	4,643,918
Member of Council of Europe (year)	1956
Member of European Union (year)	1995
Member of United Nations (year)	1955
CEDAW ratified (year)	1982
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2013

SUMMARY

There is one national women's helpline in Austria that operates 24/7, is free of charge and offers multilingual support. There are 32 shelters in the country offering 505 beds in total (including 369 in shelters and 136 in temporary flats). The Network of Austrian Counselling Centres for Women and Girls has 65 counselling centres in all nine Austrian federal states as of 2025. There are at least 17 additional independent women's counselling centres in Austria. Currently, a new National Action Plan (NAP) is being developed, including primary prevention actions, and there is a specific budget allocated to such activities. There is no comprehensive and centralised data collection mechanism on these activities. According to the financial agreement between the State and the NGOs, the women's specialist services are required to collect data. All the statistics are published on the website of the National Coordination Office for Violence Against Women.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	5,425 calls/contacts 4,805 callers

There is one state-wide women's helpline in Austria called Women's Helpline Against Violence (*Frauenhelpline gegen Gewalt*, +43 800222555). It is free of charge and available 24/7, offering support in German, English, Arabic, Romanian, Spanish and Turkish. Bosnian-Serbian-Croatian was available until 2024. The helpline complies with the standards of the Istanbul Convention. The helpline is run by the Association of Autonomous Austrian Women's Shelters (*AÖF-Verein Autonome Österreichische Frauenhäuser*), and is funded by the state, as well as by private donations. The helpline provides support for all types of gender-based violence against women, including physical violence, psychological violence, sexualised violence, sexual harassment, cyber violence, stalking, and economic violence. In 2024, it received 5,425 calls/contacts from 4,805 callers. The most common forms of violence reported by callers were domestic violence, sexualised violence and psychological violence.

There are other regional helplines supporting survivors of violence against women in the country, such as the 24-hour Women's Helpline of the City of Vienna (*Frauennotruf der Stadt Wien*, +43 1 71719), the gender-neutral Hotline of the victim protection centres (*Gewaltschutzzentren*, tel.: +43 800700217) and a gender-neutral Victim's helpline for victims of all crimes (*Opfernotruf* +43 800112112).

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	32
Total number of all shelters accessible to women	32
Existing number of beds	505
24/7 access	Most

There are 32 women-only shelters in Austria with 505 beds, including 369 in shelters and the remaining 136 in temporary flats, according to data from 2023. Sixteen of these shelters are part of the AÖF Network. Most women-only shelters offer 24/7 access. Based on the federal state, women who have an income can be expected to pay a small amount for their stay in a shelter, whereas women with lower financial income are not expected to pay. The length of the stay in the shelters depends on individual circumstances and ranges between three and six months. Based on 2023 data of the women's shelters in the AÖF Network, most women stay in the shelters for no longer than 90 days (61%). The in-house services most frequently provided by the women-only shelters in the country are casework and psychological support/counselling.

In terms of accessibility, all shelters can accommodate older women, most can accommodate women with uncertain residence permits, undocumented women and refugee women. Some shelters can be accessed by and are accessible to women with physical, cognitive and sensory disabilities. There are other shelters in the country specialised in supporting specific groups, such as the women's counselling centre "Orient Express" in Vienna, which runs a shelter/emergency apartment to accommodate girls and women between the ages of 16 to 24 who have been

affected by forced marriage, and a shelter for women survivors of trafficking run by the NGO LEFÖ. These shelters do not offer 24/7 services. In 2023, a special women's shelter for young women aged 16 to 25 was opened in Vienna, offering 28 bed spaces.

The funding of women's shelters is a responsibility of the respective federal state and the local government. Therefore, the funding differs in each federal state. Additionally, shelters receive donations. Women's NGOs with a gender-specific and feminist approach run 31 of the women-only shelters. The women's shelter in Salzburg was closed in 2021, and a new concept has been implemented by the associations *VIELE* and *Jugend am Werk*. Women-only shelters are available in all regions of Austria; however, their presence in more remote areas varies according to the regional government. According to data from the 16 shelters of the AÖF Network, in 2023, approximately 200 women were unable to find support due to a lack of space in the shelters, which corresponds to 58% of the rejections. Of these, 7% of the referrals were declined due to homelessness, and another 7% because of the place of residency being in another federal state. The most common forms of violence reported by survivors to the women-only shelters in 2024 were domestic, physical and sexualised violence.

There are no other shelters in Austria, besides the women-only shelters, that can provide support to survivors of violence against women.

❖ 3. WOMEN'S CENTRES

Total number of centres	65
Entities running women's centres	N/A

Due to the lack of public data on women's centres, there are no consolidated statistics on their numbers. According to the Federal Ministry for Women, Science and Research, there are currently 164 counselling centres for women and girls throughout Austria, but some centres are counted twice if they offer different services. The Network of Austrian Counselling Centres for Women and Girls (*Netzwerk Österreichischer Frauen- und Mädchenberatungsstellen*) had 65 counselling centres in all nine federal states in 2025. There are an additional 17 women's counselling centres in Austria that operate independently of the network. These centres offer psychological assistance, legal advice, financial and social welfare support, employment, residential support services, housing advice and referrals to other services. The counselling is confidential, free of charge and takes place at the centres, by phone or online. Some of the centres of the Network of Austrian Counselling Centres for Women and Girls accompany clients to courts, police and social services. Some women's centres in Austria can also offer residential support. Additionally, they also provide specialised support for survivors of sexualised violence, offering psychological care, advocacy services, legal advice, community awareness and education for prevention.

Most women's centres can be accessed by women with uncertain residence permits/undocumented women, refugee women and older women. Some centres can be accessed by and are accessible to women with cognitive, physical and sensory disabilities, as well as transgender women and unaccompanied girls under the age of 18. Many centres also offer specialised support to survivors of specific forms of violence. For instance, at least four centres are specialised in supporting survivors (at risk) of female genital mutilation: the African Women Organisation with the project "Bright Future" in Vienna, FEM SÜD-Frauengesundheitszentrum at Kaiser Franz Spital in Vienna, Orient Express in Vienna and DIVAN in Graz. The new Austria-wide FGM/C Coordination office offers advice, information and support on all issues related to female genital mutilation. Some members of the Network of Austrian Counselling Centres for Women and Girls are specialised in supporting survivors of forced marriage (e.g., Orient Express and LEFÖ in Vienna). The counselling centre DIVAN offers support to survivors of honour-based violence, and LEFÖ has an intervention centre for survivors of trafficking, offering psychological and legal assistance to women and children. The centre Ninlil specialises in supporting women with cognitive and physical disabilities. Some centres provide counselling for migrant women. According to the law, since 2016, all counselling services in Austria that are recognised women's service centres must be barrier-free. This requirement must also be continuously supported with adequate financial resources. The majority of women's centres are welcoming to transgender women and non-binary people, and at least five centres explicitly express this inclusion in the name of the centre.

The majority of women's centres are run by women's NGOs. Women's centres are funded by the state and local governments, donations, international funding and religious institutions. Women's centres often rely on multiple funding sources and must apply every year, making it difficult to ensure long-term financial stability. Funding is often project-based and bureaucratic and does not consider inflation. In terms of geographical coverage, women's centres are located in all regions of Austria.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

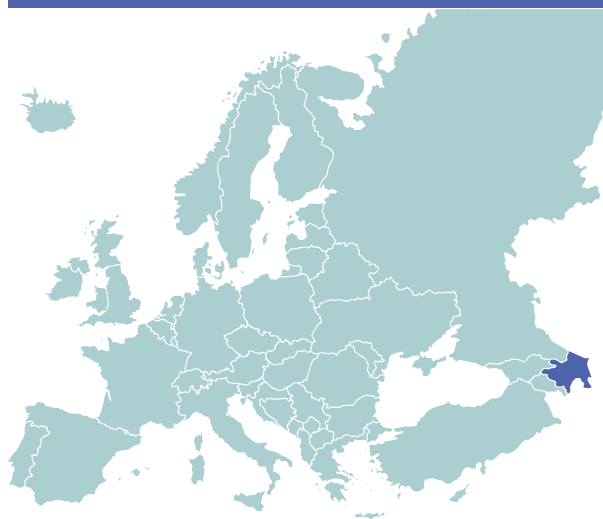
There are currently nine counselling centres for survivors of sexualised violence available in all regions of Austria, which belong to the association BAFÖ (Federation of Autonomous Women's Counselling Centres Against Sexual Violence Austria). These counselling centres also engage in prevention, public awareness-raising campaigns, and advocate for socio-political change against sexism/sexualised violence. Furthermore, there are three Violence Intervention Clinics (*Gewaltambulanzen*) located in Graz, Vienna and Innsbruck; however, they are not specialised in the care of survivors of sexualised violence. Women's centres also provide support to survivors of sexualised violence and refer them to specialised services and medical

professionals. Hospitals have specialist victim protection groups (*Opferschutzgruppen*), which require adequate funding and resources to ensure comprehensive care and effective support. There are also specialised helplines in the country (e.g., *Frauen gegen Vergewaltigung und Frauenberatung Notruf bei sexueller Gewalt*). Services for survivors of sexualised violence provide needs assessment, specialist psychological care, advocacy services, community awareness and education for prevention, as well as legal advice. They are funded by the state/local government and donations from individuals and small businesses, and support both women and young women and children under the age of 18.

5. PRIMARY PREVENTION SERVICES

Public authorities partially support primary prevention activities. However, there is no comprehensive data collection mechanism on these activities. In Austria, a new National Action Plan is being developed at the time of writing, and it will include some aspects related to primary prevention, although details on its scope are not available yet. There is no official definition of primary prevention in the country, and there are small regional funding programmes for primary prevention offered and managed by different ministries and projects. However, there is no unified or comprehensive overview of these budget lines. In the country, there are several primary prevention activities offered (e.g., empowerment activities for girls and women, school-based primary prevention programmes, training for primary prevention practitioners, awareness-raising campaigns and primary prevention activities targeted at boys and men). One example is the Austria-wide project “StoP – Neighbourhoods without partner violence”, which is managed by the Association of Austrian Autonomous Women’s Shelters and funded by the Ministry of Social Affairs, and is externally evaluated. Moreover, women’s and girls’ counselling centres often integrate primary prevention activities into their work, offering workshops and educational programmes in schools and community spaces. Some women’s organisations conduct their own evaluations of these prevention efforts, focusing on issues such as gender-based violence, sexual harassment, and digital violence. There is no comprehensive evaluation of the impact of primary prevention activities at the national level. Some individual projects and initiatives may include internal or external evaluations, but these are not consistently funded or coordinated by public authorities at the national, regional, or local levels. Currently, there is a comprehensive Austria-wide study being prepared of the impact of the StoP projects in Austria, which should be released in 2026.

AZERBAIJAN



GENERAL COUNTRY INFORMATION

Population	10,180,770
Female population	5,113,564
Member of Council of Europe (year)	2001
Member of European Union (year)	No
Member of United Nations (year)	1992
CEDAW ratified (year)	1995
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	No
Istanbul Convention ratified (year)	No

SUMMARY

There is one national women’s helpline in Azerbaijan, which is free of charge, available 24/7 and offers multi-lingual support. There are also other helplines supporting survivors of violence. There are two women-only shelters accessible to women in Azerbaijan, offering 85 beds in total, as well as eight other shelters accessible to women survivors of violence. There are also 11 Family and Child Support Centres operating in various regions of the country and offering a range of support services. There are no rape crisis centres or sexual violence referral centres in Azerbaijan. State authorities do collect data on primary prevention activities, but not in a centralised or fully standardised way, and there is no specific budget allocated to such activities. The National Action Plan on Combating Domestic Violence for 2024-2027 has been drafted but not yet approved. Apart from primary prevention activities, the state does not collect data on women’s specialist services.

WOMEN’S SPECIALIST SERVICES

1. NATIONAL WOMEN’S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	1,623 calls

There is one national women's helpline in Azerbaijan, called Women's Support Line (*Qadın Dəstək Xətti*, tel.: 116 111). It operates free of charge, is available 24/7, and offers support in Azerbaijani, Turkish, English and Russian. The helpline is run by Reliable Future (*Umidli Gələcək*) Youth Organisation with the support of the State Committee for Family, Women, and Children Affairs of the Republic of Azerbaijan, and Azercell Telecom LLC. The helpline provides referral services, initial legal and psychological support, and rehabilitation services to women survivors of violence. The helpline is funded through donations. In 2024, the helpline received 1,623 calls and the most common form of violence reported were physical and domestic violence, as well as violence directly or indirectly affecting children. Azerbaijan therefore does meet the Istanbul Convention minimum standards for the provision of a national women's helpline.

There are other helplines in Azerbaijan supporting survivors of violence against women. This includes the government-run 152 Hotline (tel.: +994 12152) which provides information and consultation services to individuals who have experienced human trafficking crimes and potential or presumed survivors of trafficking, especially women and children. It is funded and operated by the Ministry of Internal Affairs, available 24/7, free of charge, and provides multilingual support. Additionally, *asso 102* is a helpline of the Ministry of Internal Affairs available for all criminal cases including those relating to violence against women. The helpline operates 24/7 and is free of charge within the country. Two NGO-led helplines (+994 116 and +994 111) provide support to survivors of violence, mainly handling calls related to psychological and economic violence. Another helpline is run by Clean World Public Union for Aid to Women for survivors of human trafficking (tel.: +994 125111151).⁶

The 860 Support Line for Gender-Based Violence (*Gender əsaslı zorakılıqla bağlı Dəstək Xətti*) was a joint initiative of the United Nations Population Fund (UNFPA) and the State Committee for Family, Women and Children Affairs of the Republic of Azerbaijan. The helpline, which had been operating since October 2007, is currently not functioning due to UN project implementation suspensions.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	2
Total number of all shelters accessible to women	10
Existing number of beds	85 ⁷
24/7 access	All

There are two women-only shelters in Azerbaijan, with a total of 85 beds. One shelter is run by Clean World Public Union for Aid to Women and has 60 available beds while

the other shelter is run by Temas Regional Development Public Union and offers 25 beds. There were two other women-only shelters operating in Baku, but these are currently not functioning and therefore not able to support women survivors. Both women-only shelters offer 24/7 accessibility, and women are not expected to pay for the accommodation. The average duration of stay in the women-only shelters is up to three months, however the length depends on the individual case. It can range from as little as one week to three–six months. Women can stay longer than six months if there is a danger that is causing harm to the woman's health, and this danger persists or if the woman is pregnant and is due to give birth. In case the survivor does not have anywhere else to go, she can stay in the shelter until the child is two years old. The in-house services most often provided by the women-only shelters include casework, counselling and psychological support, referrals and collaborations with other services, practical advice and legal advice.

Both women-only shelters can be accessed by unaccompanied girls under the age of 18. Most women-only shelters are equipped to support women with uncertain residence permits/undocumented women, refugee women, homeless women and women with substance abuse issues. Some women-only shelters are accessible to women with physical, cognitive or sensory disabilities, older women, transgender women and women with children with disabilities. However, neither of the women-only shelters can accommodate women with older sons (14 years and above). The women-only shelter run by Clean World Public Union for Aid to Women is also equipped to support women survivors of specific forms of violence, such as trafficking, honour-based violence, and forced marriage. There are no specialised women-only shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Azerbaijan.

Funding for women-only shelters mainly comes from donations. The state and international funders also provide financial support, however, in 2025 the women-only shelters received no international funding and limited government support. The two women-only shelters are run by women's NGOs with a gender-specific and feminist approach. They are only present in major cities; the shelter run by the Public Union Clean World Aid to Women is located in Baku, the capital city, and the shelter run by Temas Regional Development Public Union is located in Ganja. The main reasons why women-only shelters had to decline referrals were no space/capacity to support the survivor, no space/capacity to accommodate survivors with children, and the shelter not being adequately equipped for the needs of women and children. The most common forms of violence reported by survivors to the women-only shelters were domestic violence, physical violence and violence directly or

6 Clean World Public Union for Aid to Women also ran a helpline for survivors of domestic violence, which is no longer active. It was closed due to financial constraints and lack of sustainable funding.

7 This figure only refers to the beds in the two women-only shelters.

indirectly affecting children. In 2024, 535 women and children were accommodated in both women-only shelters.

There are eight other shelters accessible to women survivors of violence in Azerbaijan. Two shelters are operated by the Social Services Agency (SSA) under the Ministry of Labour and Social Protection. One of these is the SSA Shelter and Social Rehabilitation Institution located in Baku with 70 beds for vulnerable groups, including survivors of domestic violence. The other shelter is the SSA Assistance Center for Trafficking Victims, which provides temporary shelter and broader reintegration services to survivors of trafficking. One shelter is run by the Ministry of Internal Affairs in Baku and is called the Anti-Trafficking Shelter and Reintegration Center with 50 beds for survivors of human trafficking and potential survivors. These shelters can be accessed by women and men and are available 24/7. In addition, there are four shelters specifically for children run by NGOs which can support those experiencing domestic violence, as well as a one shelter run by the Catholic Church in Azerbaijan (Missionaries of Charity) with 30 beds which can accommodate women survivors of violence.

❖ 3. WOMEN'S CENTRES

Total number of centres	11
Entities running women's centres	State (11 centres)

There are 11 nine Family and Child Support Centres (*Ailə və Uşaq Mərkəzləri*) operating in various regions of Azerbaijan, offering psychological assistance, legal advice, and referrals to other services, such as temporary shelters, court or police representation, and access to social services. The Family and Child Support Centres are financed through government funding and managed by the State Committee for Family, Women and Children Affairs of the Republic of Azerbaijan. Formerly, there were nine Women's Resource Centres run by the United Nations jointly with the State Committee for Family, Women and Children Affairs in the frame of USAID-funded projects. However, with the termination of USAID activities, these centres closed in 2024. Women's centres are located in most regions of Azerbaijan. Three centres provide specialist support to survivors of trafficking and one NGO, called Gender and Development, advocates for the rights of transgender women and non-binary people. There are two centres that provide specialist support to women with uncertain residence permits/undocumented women.

None of the women's centres can be accessed by or are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive or sensory disabilities, older women (65 years and above), transgender women, and unaccompanied girls under the age of 18. There are no specialised centres supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Azerbaijan.

Women's centres provide only non-residential support services and participate in the placement of women in shelters. Women's centres also provide specialist support for survivors of sexualised violence, such as specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services and community awareness and education for prevention.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no rape crisis centres or sexual violence referral centres in Azerbaijan. The country does provide psychosocial and legal support to survivors of sexualised violence, especially through helplines and general shelters, however it lacks dedicated, specialised services for rape and sexual assault survivors. Formally, women's centres across the country did provide services to survivors of sexualised violence including referrals and escorting survivors to hospitals or emergency rooms to receive medical services. However, due to cancellation of international funding, these centres no longer provide such services. Critical services, such as 24/7 forensic exams, rape crisis centres, and gender-sensitive legal support are yet to be established. Additionally, gaps persist in survivor-friendly justice processes.

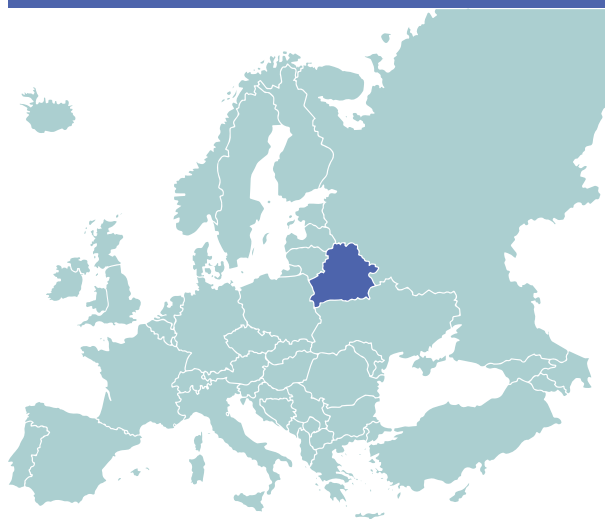
☞ 5. PRIMARY PREVENTION SERVICES

In Azerbaijan, state authorities do collect data on primary prevention activities, but not in a centralised or fully standardised way. The State Committee for Family, Women and Children Affairs operates 11 Family and Child Support Centres and these centres run activities related to preventing domestic violence, early marriage, substance abuse, and promote gender equality through workshops, trainings, and awareness campaigns. However, there is no comprehensive or integrated framework that measures their impact or includes all sectors. Data on the effectiveness and outcomes of these primary prevention activities (e.g., reduced violence rates) is not publicly available or aggregated in a single national report. There is also no centralised system that combines primary prevention data from NGOs, police, health services, or local municipalities. Efforts are ongoing to strengthen multi-sector data systems.

There is no official document on primary prevention and the National Action Plan (NAP) on Combating Domestic Violence for 2024-2027 has been drafted but not yet approved. The previous NAP on Combating Domestic Violence for 2020-2023 included provisions for establishing support centres, shelters, social rehabilitation, and public awareness raising. The NAP focused more on service provision and awareness-raising, not structured primary prevention strategies, and referenced prevention in broad terms but did not offer a specific, operational definition of primary prevention.

Budget lines for domestic violence activities are not disaggregated enough to isolate funds for evidence-driven primary prevention interventions, and it is therefore not known if there is a dedicated budget for such activities. Available primary prevention activities include empowerment activities for girls and women, primary prevention activities targeting boys and men, training for primary prevention practitioners, self-care activities, awareness-raising campaigns and programmes. Empowerment activities were implemented by Women's Resource Centres and NGOs (e.g., vocational training, legal education, confidence-building workshops) before these shut down. Self-care activities are provided by some women's centres and shelters as part of psychosocial support, which is also not always framed as primary prevention. The country relies heavily on awareness-raising and support services, with limited outreach to men and boys or structured gender-transformative programming.

BELARUS



GENERAL COUNTRY INFORMATION

Population	9,475,174 ⁸
Female population	5,059,429 ⁹
Member of Council of Europe (year)	No
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1981
CEDAW optional protocol ratified (year)	2004
Istanbul Convention signed (year)	No
Istanbul Convention ratified (year)	No

SUMMARY

There is no national women's helpline in Belarus, although there is one generic helpline for family and child-related issues which can provide counselling. Belarus has three women-only shelters, but the number of beds is not available. There are no women's centres in Belarus as well as no support services for survivors of sexualised violence. State authorities do not collect or publish data on women's specialist services. In 2023, the Ministry of Internal Affairs stated that law enforcement information on domestic violence was classified as official information of restricted distribution and therefore could not be made available to the public. However, until 2018, statistics on domestic violence offenses were publicly available and regularly updated. Other state bodies publish domestic violence related statistics irregularly, but the data is often not comparable. The absence of women's specialist services and the repression of civil society prevent any comprehensive understanding of violence against women and girls in Belarus.¹⁰

⁸ The most recent available data on Eurostat is from 2019.

⁹ Ibid.

¹⁰ By December 30, 2024, 1,186 NGOs were in the process of forced liquidation, including liquidation claims by authorities. Furthermore, 705 NGOs decided to self-liquidate, primarily due to pressure on members and staff as well as the unfavourable legal environment and the general socio-political situation in Belarus.

WOMEN'S SPECIALIST SERVICES

☎ 1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

There is no national women's helpline in Belarus. The only national women's helpline was the National Hotline for victims of family violence (*Общенациональная горячая линия для пострадавших от домашнего насилия*) which was run by the international public association "Gender Perspectives" from 2012 to 2021. The helpline responded to over 15,000 calls from 2011 to 2021. As a result of state repressions against Belarusian civil society, "Gender Perspectives" was compulsorily liquidated and the helpline was indefinitely suspended in July 2021 when the office was raided by law enforcement. This situation has created significant challenges and limitations in accessing information on women's specialist services in Belarus. The country does not meet the Istanbul Convention minimum standards for the provision of a national women's helpline.

There is one other generic helpline for family and child-related issues which can provide counselling, crisis psychological support, and referral to state crisis rooms for survivors of domestic violence, when requested. This is the 24-hour crisis helpline for family and children's issues (*Городская круглосуточная кризисная телефонная информационная линия по проблемам семьи и детей*) managed by the Minsk City Centre for Social Services for Family and Children (tel.: +375 293673232). The helpline is free of charge and available 24/7, although it provides services mainly for Minsk residents.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	3
Total number of all shelters accessible to women	4
Existing number of beds	20 ¹¹
24/7 access	None

There are three women-only shelters in Belarus operated by the SOS Children Village, a children rights NGO. The two shelters in Minsk and the Minsk region have five beds for women and 10 for children, while the shelter in Mogilev City can accommodate 10 women and 14 children. In 2022, the only specialised shelter for women survivors of domestic violence managed by the NGO Radislava was liquidated by authorities. Available shelters are open to women and children and on average, women stay in the shelters between six weeks and five months. None of the

women-only shelters offer 24/7 access. Women are not expected to pay for their accommodation in women-only shelters. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services, and legal advice.

None of the women-only shelters can be accessed by and are accessible to women with substance abuse issues, older women (65 years and above), women with older sons (14 years and above), and unaccompanied girls under the age of 18. There are also no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Belarus.

Funding for women-only shelters in Belarus comes from donations and international funding. One shelter for women and children offers its services in the Mogilev region and all the others are in Minsk and the Minsk region. The most common form of violence reported by survivors in women-only shelters in 2024 was domestic violence. The two shelters in Minsk and the Minsk region assisted 13 women and 17 children in 2024, while the shelter in Mogilev accommodated 15 women and 29 children.

There are no other shelters in the country that can provide specialised support to survivors of domestic violence. Survivors in need of a safe shelter are provided with temporary shelter services, in one of the 134 available crisis rooms¹² across the country. However, in 2022 and 2023, only about half of those receiving support in crisis rooms were domestic violence survivors. Additionally, women may be asked to share a room with men receiving crisis room care, and in some cases, perpetrators have also been accommodated in crisis rooms. Some crisis rooms do not have a secret address as they are located in homes for veterans and pensioners. The state in Belarus is developing a network of crisis rooms throughout the country, but their effectiveness is low, especially in small towns and rural areas, where the address of the crisis room can easily become public. The majority of crisis rooms are located in a city or near a city, but none offer 24/7 access. According to the Ministry of Social Welfare, in 2023, temporary shelter was provided to 496 citizens, including 236 survivors of domestic violence.

❖ 3. WOMEN'S CENTRES

Total number of centres	0
Entities running women's centres	—

There are no women's centres in Belarus. The 134 crisis rooms however do provide non-residential support such as casework, referrals and collaboration with other services, as well as practical advice. Crisis rooms do not offer

¹¹ This number includes only the shelter beds available to women in the three women-only shelters and excludes the 34 other shelters beds for children.

¹² According to the shadow report on Domestic Violence Problem in Belarus to the CEDAW Committee prepared by the Belarusian Coalition of Civil Society Organisations, Initiatives and Activists (in exile) against Gender-based and Domestic Violence, 2025.

specialised assistance to survivors of violence, or only to women, but to all individuals in different crisis situations. According to the Ministry of Social Welfare, in 2023, 15,400 citizens who had suffered domestic violence sought help from state social centres, and almost all social centres now have crisis rooms available.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no services for women and girls who have experienced sexualised violence in Belarus.

5. PRIMARY PREVENTION SERVICES

Public authorities in Belarus do not collect data on primary prevention activities and there is no official state definition of primary prevention. Additionally, no specific budget is allocated to primary prevention activities. There are no organisations focusing solely on primary prevention in Belarus.

In 2022, amendments were made to the Law of the Republic of Belarus from 2014; No. 122-Z "On the Basics of Activities for the Prevention of Offenses", aimed at preventing domestic violence. It is important to note that the principles of "preserving traditional family values, respecting privacy, not considering customs, beliefs, and traditions as justifications for offenses, and prioritising preventive measures over punitive measures" have been added to Article 4 of the law, which regulates the principles of the activities in the field of offense prevention. Additionally, the amended Prevention Law does not define domestic violence as a gender-specific problem, unlike the Istanbul Convention. The state lacks an understanding of how gender inequality, as a root cause, and gender, as a vulnerability factor for women, is related to domestic violence. The Prevention Law only once mentions NGOs as subjects of interagency cooperation and does not regulate their participation in domestic violence prevention and assistance to survivors.¹³

BELGIUM



GENERAL COUNTRY INFORMATION

Population	11,817,096
Female population	5,981,093
Member of Council of Europe (year)	1949
Member of European Union (year)	1958
Member of United Nations (year)	1945
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2004
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2016

SUMMARY

There is no national women's helpline in Belgium as it is a federal state but there are regional helplines offering support to survivors of violence, both in Brussels and Wallonia and in Flanders. There are approximately 27 women-only shelters in Belgium, as well as 17 other shelters accessible to women survivors in Brussels and Wallonia and approximately 82 additional shelter accommodation options in Flanders (referred to as residential support modules). The number of beds in shelters in Flanders is not available, but in Brussels and Wallonia, there are approximately 1,847 beds in total. There are at least 44 women's centres in Belgium providing different support services. There are services for survivors of sexualised violence in Belgium, including ten sexual violence referral centres and one rape crisis centre which also operates a specialised helpline. Measures related to primary prevention are included in the National Action Plan and there is an official definition, although no specific budget for primary prevention activities. The state in the French-speaking part of Belgium does not systematically collect data on women's specialist support services. In Flanders, data on women's specialist support services is available upon request and sometimes (partially) publicly available on the website of the Flemish Parliament.

¹³ More on the legislative changes which came into force in 2022 can be found in the shadow report on Domestic Violence Problem in Belarus to the CEDAW Committee prepared by the Belarusian Coalition of Civil Society Organisations, Initiatives and Activists (in exile) against Gender-based and Domestic Violence, 2025.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

There is no national women's helpline for the whole of Belgium due to structural and linguistic differences within the country. Instead, there are distinct helplines for each of the two main linguistic communities (the French-speaking part, Brussels and Wallonia, and the Dutch-speaking part, Flanders). Therefore, Belgium does not meet the Istanbul Convention standards for the provision of a national women's helpline.

For the French-speaking part of Belgium, there are two helplines supporting women survivors of violence. The first helpline is the Domestic Violence Helpline and Centre for the Prevention of Domestic Violence (*Écoute violence conjugale et Centre de Prévention des Violences Conjugales et Familiales – CPVCF*, tel.: +32 80003030) which is free of charge and available 24/7. In 2024, the helpline received 13,067 calls. The majority of calls were from survivors of domestic violence, 22% from the survivor's network (family or professionals) and 2% from perpetrators of domestic violence. The three most common forms of violence reported by callers were psychological violence (74%), verbal violence (47%), and tensions (59%).¹⁴ It should be noted that callers reported several forms of violence at the same time. The CPVCF also offers online chat and messaging services and in 2024, the helpline received and responded to just over 500 messages. The second helpline is run by the Collective against Family Violence and Exclusion (*Ligne d'écoute du CVFE – Collectif contre les Violences familiales et l'Exclusion*, tel.: +32 42234567). The helpline is available 24/7 and received 5,475 calls in 2021.¹⁵ There is also a helpline for survivors of forced marriage, My Marriage Belongs to Me (*Mon mariage m'appartient*, tel.: +32 80090901), which is free of charge but does not run 24/7 (it is available on workdays only). Funding for women's helplines in Brussels and Wallonia comes from the state, local authorities, subsidies, and donations.

In Flanders there is a helpline (tel.: +32 1712) to report any kind of violence and (child) abuse, which is available to all survivors of violence, not just women. The helpline is run by the Centres for General Welfare Work (*Centrum voor Algemeen Welzijnswerk – CAW*) and Child Abuse Trust Centres (*Vertrouwenscentra kindermishandeling – VK*) and is accessible via phone, email, and chat. The helpline is

state-funded, free of charge, and offers multilingual support in Dutch, English, and French, but it is not available 24/7. In 2024, the helpline received 9,444 calls concerning 13,281 persons or (potential) survivors of violence or child abuse. Seventy-three percent of callers were women, 26% were men, 0,5% were non-binary or gender non-conforming individuals, and 1,4% unknown. The three most common forms of violence reported were emotional violence (38%), physical violence (22%), and sexualised violence (11%). In addition, 85% of calls were related to domestic violence incidents.

There is another regional helpline in Belgium supporting survivors of violence, Tele-Reception (*Tele-Onthaal/Télé-Accueil*) that is available 24/7 also via chat in both Dutch (tel.: +32 106) and French (tel.: +32 107). The helpline is a general support line run by volunteers for all possible questions, including support to survivors of all forms of violence. In 2023, the helpline received 105,568 calls (68% by women) and handled 78,376 conversations.¹⁶ Other helplines for specific groups of survivors include GAMS Belgium (tel.: +32 22194340) for women and girls (at risk) of female genital mutilation. Different specialised helplines exist for survivors of human trafficking including *Payoke* (tel.: +32 32011690) in Flanders, *Pag-Asa* (tel.: +32 25116464) in Brussels, and *Sûrya* in Wallonia (tel.: +32 42324030). In addition, there is also a group of government-initiated, volunteer-run helplines called Listening Ears (*Luisterende Oren*), which offer support to survivors of domestic violence in 24 languages (individual phone numbers available by language).

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	27
Total number of all shelters accessible to women	44 ¹⁷
Existing number of beds	1,847 ¹⁸
24/7 access	Most

In the French-speaking part of Belgium, there are 21 women-only shelters (five in Brussels and 16 in Wallonia) with a total of 889 beds (203 in Brussels and 686 in Wallonia). Most of the women-only shelters offer 24/7 access. In Brussels and Wallonia, women are expected to pay for their accommodation in a shelter, meaning that undocumented women without sufficient income have limited access to women-only shelters. Concerning women with a low income, most women-only shelters propose a contribution based on the woman's income. On average, women are allowed to stay in a shelter for up to one year. The types of in-house services most often provided by women-only shelters are counselling/psychological support, practical advice, and legal advice.

¹⁴ More "subtle" forms of violence employed by abusers (e.g., sighs, angry looks, silence, veiled reproaches, disapproval, etc.) to reaffirm their power over the relationship.

¹⁵ Figures for 2022, 2023 and 2024 are not available.

¹⁶ Figures for 2024 are not available.

¹⁷ This number does not include the 82 residential support modules accessible to women in Flanders.

¹⁸ This number only includes the beds in shelters in Brussels and Wallonia, as the number of beds in Flanders is not available.

Most women-only shelters in Brussels and Wallonia are accessible to homeless women. Some women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical disabilities, women with substance abuse issues, transgender women, and women with older sons (14 years and above). There are also specialised shelters supporting particular groups of survivors including the Sisters' House in Brussels, which is accessible to refugee women and can accommodate 70 women. The Brussels Refuge (*Le Refuge Brussels*) provides emergency accommodation for up to three months and support for LGBTQI+ people aged 18 to 25 who have been discriminated against because of their sexual orientation and/or gender identity. In 2022, a new shelter reserved for LGBTQI+ youth opened in the Brussels-Capital Region with 14 beds.

Funding for women-only shelters in Brussels and Wallonia comes from the state as well as donations. Six women-only shelters are run by women's NGOs with a feminist and gender-specific approach and 15 are run by other NGOs. In Wallonia, and especially in the shelters run by the Collective against Family Violence and Exclusion, all survivors reported experiencing psychological and verbal violence; three quarters also reported economic violence, while two thirds reported physical violence and/or sexualised violence. The main reasons for declining referrals were no space/capacity to support the survivor and no space/capacity to accommodate survivors with children. While Brussels and Wallonia do meet the Istanbul Convention standards for the provision of women's shelters, the need actually outnumbers what women-only shelters can currently accommodate.

In addition, there are other shelters which can provide support to all survivors, namely 17 shelters (11 in Brussels and six in Wallonia) with a total of 958 beds, 815 in Brussels and 143 in Wallonia. Shelters are located in all regions of Brussels and Wallonia and most offer 24/7 access.

In Flanders, there are six women-only shelters, but the number of beds for 2024 is not known. In recent years, the Centres for General Welfare Work (*Centrum Algemeen Welzijnwerk* – CAW) have focused on transitioning from group accommodation to individual accommodation in studios with support options – referred to as intra-family violence residential support modules. As a result, shelter accommodation capacity is no longer counted per bed but per type of residential support. The CAWs are responsible for providing residential accommodation for women survivors of violence. Four women-only shelters have an anonymous address, while two women-only shelters have a known but secure address. Women-only shelters in Flanders are, in principle, available 24/7, but physical access to the building is limited to those who have been officially admitted. In emergency situations, immediate admission

can be arranged, even outside office hours, but this usually happens via referrals from the police, hospitals, or care providers. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services, practical advice, legal advice, and childcare. Women are expected to pay for their accommodation in a women-only shelter. At the beginning of 2024, the Flemish government decided that during the first twelve weeks, the average length of stay in a shelter with a secret address, no daily fee would be charged. After this twelve-week period, the survivor is expected to cover the accommodation costs either through their own means, if sufficient, or through a contribution from the Public Centre for Social Welfare (OCMW). This measure doesn't count for women-only shelters with a known but secure address. In 2024, the daily rate was €20.28 for persons under 12 years old and €32.98 for persons aged 12 or over. In 2023, the average length of stay in a women-only shelter was 74,9 days.¹⁹

All women-only shelters in Flanders can accommodate older women (65 years and above) and most can accommodate women with older sons (14 years and above). Some women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical disabilities, women with cognitive disabilities, women with sensory disabilities, women with substance abuse issues, transgender women, and women with children with disabilities. Women-only shelters cannot accommodate homeless women, but temporary accommodation is available for women (and their children) who can no longer live at home due to various circumstances or when they have nowhere to stay due to an emergency situation. There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Flanders. However, *Payoke* is a Flemish NGO for survivors of trafficking and exploitation, offering specialised care and support to survivors of human trafficking, including a 45-day stay in a safe space. Women-only shelters in Flanders are run by the CAWs which fund shelters with the subsidies they receive from the Flemish government. However, not all CAWs operate a shelter. Women-only shelters are located in all five Flemish provinces. The main reason why women-only shelters had to decline referrals was because of a lack of capacity for example due to illness of staff members, or because there was no space to accommodate the survivor and/or her children. The most common forms of violence reported by survivors in 2024 were domestic violence, physical violence, psychological violence, sexualised violence, and economic violence. In 2024, a total of 568 survivors were accommodated in women-only shelters.

In addition to the intra-family violence residential support modules, there are other shelter accommodation options for survivors of violence against women in Flanders. These

¹⁹ Data for 2024 is not available.

are also offered by the CAWs and include residential support units with accommodation and crisis shelters, which women and children can also turn to if needed. There are 82 such residential support units and crisis shelters available. Shelters are available to all survivors and some offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres		44
Entities running women's centres	Women's NGOs (4 centres)	
	Other NGOs (34 centres)	
	State (6 centres)	

There are at least 34 women's centres in Brussels and Wallonia, four of which are run by women's NGOs and the rest by other NGOs. Four of the women's centres are only for women survivors of violence against women, and the rest are for all survivors, including women. Almost all centres provide counselling/psychological support, more than half offer legal advice, and some provide financial and social welfare support/advocacy, housing advice, and employment, but no centres provide referrals with other services, representation at court/police, or residential support services. Some centres provide support services for survivors of violence such as discussion groups, trainings, literacy classes or individual psychosocial support. Women's centres also provide specialist support services for survivors of sexualised violence such as specialist forensic and medical care, specialist psychological care, and specialist sexualised violence advocacy services.

Some women's centres in Brussels and Wallonia can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical disabilities, and transgender women. There are at least eight women's centres which support transgender women and non-binary people, the association *Tels Quels* and seven centres of the *Maisons Arc-en-Ciel*. Located in the four corners of Wallonia, the *Maisons Arc-en-Ciel* are local relays for all LGBTQI+ people looking for a friendly and caring place, peer-to-peer activities, and information. In addition, three centres provide specialised support to survivors (at risk) of female genital mutilation, including GAMS Belgium, the group for the Abolition of Female Genital Mutilation. Two centres provide specialised support to survivors (at risk) of forced marriage and honour-based violence. There are also two centres providing specialised support to survivors of trafficking, including the NGO *Payoke* which is located in Flanders but operates throughout Belgium. There is one women's centre providing specialised support to women with uncertain residence permits/undocumented women, refugee women, and older women, respectively.

In Flanders, there are four women's centres offering various services to women, such as socio-cultural activities,

discussion groups, and trainings, though not only for survivors of violence. In addition, the six women-only shelters that are part of the Centres for General Welfare Work (CAW) also offer non-residential support services. Collaboration with the Houses of Justice, the victim support unit of the police and/or the *Veilig Huizen* (Family Justice Centres) is possible. All centres offer counselling/psychological support, information and referrals for legal aid, referrals for financial and social welfare support (some also do advocacy), referrals for housing advice and collaboration with other services.

All women's centres in Flanders are equipped to support women with uncertain residence permits/undocumented women, refugee women, older women (65 years and above), transgender women, and unaccompanied girls under the age of 18. There are two women's centres providing specialised support to survivors (at risk) of forced marriage, honour-based violence, or female genital mutilation, namely GAMS Belgium and INTACT (both of which operate in all of Belgium), as well as support to girls under the age of 18. Additionally, there is one centre, *Persephone*, which supports women with disabilities and chronic illnesses.

Funding for women's centres in Brussels and Wallonia comes from the state as well as donations and centres are located in most regions. In 2024, approximately 2,500 women were supported by women's centres in Brussels and Wallonia. Funding for women's centres in Flanders also comes from the state as well as donations, and centres are located only in major cities.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There is one rape crisis centre in Belgium, SOS Rape (*SOS Viol*), which is the only NGO in Belgium that provides long-term support to survivors of sexualised violence. While the organisation has a feminist background, it also works with male survivors. *SOS Viol* also runs a helpline (tel.: +32 80098100) which is free of charge, available 24/7, and operates both in Brussels and Namur (Wallonia). The helpline received 2,810 calls in 2022,²⁰ an average of 11 calls per day. Approximately one in three calls concerned a new report of sexual assault. In 2022, 88% of survivors who called were women, and 12% were men. The three most common forms of sexualised violence reported were rape (81%), sexual assault (11%), and harassment (3%). *SOS Viol* also offers a chat service and replied to 443 messages in 2022, 92% of which concerned female survivors and 8% male survivors.

In addition, there are five sexual violence referral centres in Brussels and Wallonia called Sexual Assault Care Centres (*Centres de Prise en Charge des Violences Sexuelles – CPVS*). The CPVS are open 24/7 and offer medical and forensic

²⁰ Figures for 2023 and 2024 are not available.

care, psychological support, and follow-up for survivors. Specialised police officers are also present at the centres if the survivor wishes to lodge a complaint. In 2023, the centre in Brussels was extended to offer three examination and consultation rooms, an additional psychotherapy room, rest room, several adapted hearing rooms, as well as a multi-purpose area to provide additional activities for survivors or their families. The centres are located in Brussels, Charleroi, Liège, Namur and Arlon. The CPVS in Namur opened in April 2023, and the one in Arlon in December 2023.²¹ Additional CPVS were planned to open in 2024 covering the provinces Halle-Vilvorde, Eupen, Mons and Walloon Brabant, but these services have so far not been established. Sexual Assault Care Centres are open to all survivors of sexualised violence, more specifically women, men, transgender people and children, although 91% of survivors receiving support in these centres in 2022 were women (including transwomen).

In Flanders, there are also five CPVS which are called *Zorgcentrum na Seksueel Geweld* (ZSG) in Dutch. These centres are located in Antwerpen, Leuven, Limburg (in Genk), Oost-Vlaanderen (in Gent), and West-Vlaanderen (Roeselare). Funding for sexualised violence support services comes from the state, and these services are located only in major cities. Even though they are located in medium-sized and large cities, CPVS cover all regions of Belgium, some of which are more remote than others.

In 2022, the 10 CPVS in all of Belgium supported a total of 3,287 survivors of sexualised violence, with an average of nine people visiting a centre per day. The average age of survivors seeking support at a CPVS was 24, and 70% of survivors reported to the CPVS within seven days of the assault. Furthermore, *Nupraatikerover.be* is an online chat support service for minors who have questions about, or are survivors of, abuse, neglect, or sexualised violence. It is an initiative of the Child Abuse Trust Centres (*Vertrouwenscentra Kindermishandeling – VK*). The helpline 1712 can also provide support to survivors of sexualised violence. Child Focus is another helpline available in Belgium that aims to prevent the sexualised exploitation of minors (tel.: 116000) and which is available 24/7. All Sexual Assault Care Centres can also be contacted seven days a week via phone or chat.

5. PRIMARY PREVENTION SERVICES

Public authorities in Belgium do not collect data on primary prevention, although this was planned as part of the National Action Plan to Combat Gender-Based Violence 2021–2025. This National Action Plan does include specific provisions on primary prevention (in Axis III), focusing on awareness-raising, information, and primary prevention initiatives, as well as a definition of primary prevention in line with the definition used by the World Health

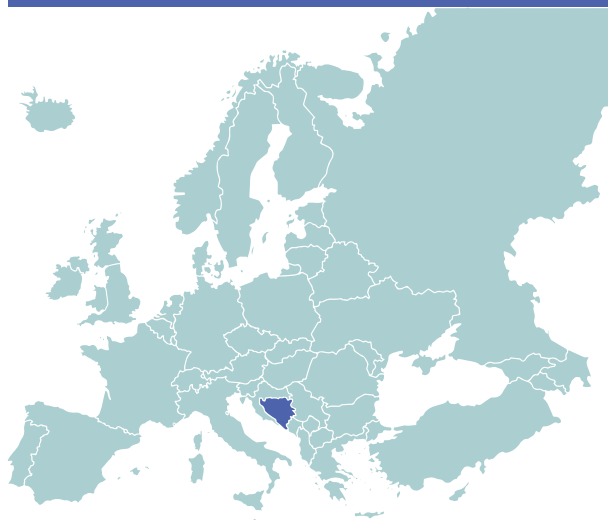
Organisation (WHO). However, no specific budget is allocated to available primary prevention activities which include feminist self-defence trainings, empowerment activities for girls and women, school-based primary prevention programmes, training for primary prevention practitioners, self-care activities, and awareness-raising campaigns or programmes.

In Brussels and Wallonia, only two organisations carry out primary prevention activities, notably by organising feminist self-defence courses (for women, including trans women, non-binary people, and girls aged eight to 18). These are CVFE in Liège and Garance in Brussels. The CVFE offers trainings exclusively for adult women and does not yet offer additional mixed-gender trainings. For almost two years, Garance has expanded its trainings to include courses for more specific audiences, such as a course for bisexual and lesbian women and a course for trans women and intersex people. Garance is the only organisation in Belgium that evaluates its primary prevention activities.

In Flanders, there are several primary prevention initiatives which are run by NGOs and include information campaigns, educational programmes for schools, and training for professionals working with at-risk groups. There is also an increasing focus on gender-based violence and the role of toxic masculinity in the emergence of violence. These organisations collect data for their annual reports, but it is not published by public authorities under the heading of primary prevention.

²¹ The Secretary of State for Equality has supported the creation of new CPVS. It means that it is a federal pilot project supported by the Secretary of State for Equality between Women and Men, subsidised by the national lottery and piloted by the ICRH (International Centre for Reproductive Health) UZ Gent.

BOSNIA AND HERZEGOVINA



GENERAL COUNTRY INFORMATION

Population	3,531,159 ²²
Female population	1,798,889 ²³
Member of Council of Europe (year)	2002
Member of European Union (year)	No
Member of United Nations (year)	1992
CEDAW ratified (year)	1993
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2013
Istanbul Convention ratified (year)	2013

SUMMARY

There are two national women's helplines in Bosnia and Herzegovina, one covering the Federation of Bosnia and Herzegovina and one covering Republika Srpska. They are both free of charge and available 24/7, but do not offer multilingual support. There are eight women-only shelters in the country, offering 208 beds, as well as 11 women's centres. Three rape crisis centres are operating in the Federation of Bosnia and Herzegovina, but they are not fully operational, and there is a helpline supporting survivors of war-related sexual violence. State authorities collect data on domestic violence, and the Federal Ministry of Labour and Social Policy gathers information on the specialised services for survivors of domestic violence in the Federation. National strategies to tackle domestic violence include provisions on prevention, but no budget is allocated, and no evaluation is conducted on existing activities.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	Yes
Multilingual support	No
Total number of calls/callers in 2024	5,963 calls

In the Federation of Bosnia and Herzegovina, the SOS telephone line for domestic violence for the Federation of BiH (*SOS telefon za žrtve nasilja u porodici Federacije BiH*, tel.: +387 1265) is free of charge, operates 24/7, but it does not offer multilingual support. The helpline does meet the standards of the Istanbul Convention. It is run by five women's NGOs, namely Association "Medica" Zenica, Vive Women Tuzla (*Vive žene Tuzla*), Foundation of Local Democracy Sarajevo (*Fondacija Lokalne demokratije Sarajevo*), Women with Una Bihać (*Žene s Une Bihać*) and Women of BiH Mostar (*Žena BiH Mostar*). The helpline provides support primarily for cases of domestic violence, but survivors can receive support and referrals for all types of violence against women. In 2024, the helpline received 2,225 calls. The most common forms of violence reported by callers were domestic violence, physical violence and psychological violence. There is no state funding for the helplines operating in the Federation of Bosnia and Herzegovina. The Gender Centre of the Federation of Bosnia and Herzegovina pays the annual fee for the number to the telecom provider, while the other costs are covered through international funding. Additionally, the Centre for Social Work Jajce runs the helpline in one canton of the Federation, Central Bosnia Canton, but the line is not available 24/7.

In Republika Srpska, the women's helpline is called SOS Telephone for women and children survivors of violence (*SOS telefon za žene i djecu žrtve nasilja*, tel.: +387 1264) and is run by Foundation "United Women" Banja Luka, Foundation "Lara" Bijeljina, and Association "Budućnost" Modriča. The helpline is free of charge and operates 24/7, thus meeting the standards of the Istanbul Convention. The helpline does not offer multilingual support, and it supports survivors of physical, psychological, economic and sexualised violence. In 2024, it received 3,738 calls from 2,617 callers, and within domestic violence, the most commonly reported forms were physical, psychological, economic and violence directly/indirectly affecting children. The organisations running the helpline do so on a voluntary basis, as they do not receive financial support from the state.

In Bosnia and Herzegovina, there is also a helpline that provides help specifically for survivors of war-related sexual violence. It is called Unique telephone line for support

²² Agency for Statistics of Bosnia and Herzegovina, 2023. Latest population data from 2013: https://bhas.gov.ba/data/Publikacije/Bilteni/2024/DEM_00_2023_TB_1_BS.pdf

²³ Ibid.

and help to survivors of war rape and sexual violence and their family members (*Jedinstvena telefonska linija za pomoć i podršku preživjelim ratno silovanje i seksualno nasilje i članovima njihovih porodica*, tel.: +387 80022334). The Association “Medica” Zenica operates the helpline, which is free of charge and available 24/7, but does not offer multi-lingual support. In 2024, the helpline received 64 calls.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	8
Total number of all shelters accessible to women	9
Existing number of beds	213
24/7 access	All

There are eight women-only shelters in Bosnia and Herzegovina, five in the Federation of Bosnia and Herzegovina and three in Republika Srpska, offering 208 beds.

In the Federation of Bosnia and Herzegovina, the five women-only shelters offer 146 beds and are accessible 24/7. They are run by women’s organisations with a gender-specific approach: “Medica” Zenica, *Vive žene Tuzla*, *Žene sa Une Bihać*, *Fondacija lokalne demokratije Sarajevo* and *Žena BiH Mostar*. Survivors can stay in the shelters up to six months, and in exceptional cases, this period can be extended in cooperation with the Centre for Social Welfare. Women are not expected to pay for their accommodation, although the new Law on Protection from Domestic Violence and Violence against Women of the Federation, passed in 2025, foresees in Article 65, item (e), that shelters, among other sources, are to be financed from the personal contribution of service beneficiaries or their relatives. Women’s shelters opposed this provision during the legislative process, but their proposals were not taken into account. This law has not been implemented at the time of writing, and minimum standards for safe houses are being developed. The types of in-house services provided by shelters include casework, counselling/psychological support, referrals/collaboration with other services, and practical and legal advice. Some shelters also offer medical assistance, occupational therapy, and an economic empowerment programme.

Most women’s shelters in the Federation of Bosnia and Herzegovina can be accessed by and are accessible to women with physical and sensory disabilities, older women, women with children with disabilities and unaccompanied girls under the age of 18. Some shelters are equipped to support women with uncertain residence permits/undocumented women, refugee women, homeless women, and women with older sons, while no shelters can support women with cognitive disabilities and with substance abuse issues. Additionally, there is one shelter specialised in supporting survivors from the LGBTQI+ community. This shelter provides five beds, it is accessible 24/7 and is run by the organisation *Krila nade* in Sarajevo.

Article 65 of the new Law on Protection from Domestic Violence and Violence against Women of the Federation also stipulates the funding guidelines for shelters. While in the past the costs were covered by the Federation (70% of the costs) and the Canton (30% of the costs), the new law does not include a fixed percentage. Women’s shelters also receive international funding, but this is limited. Shelters are located in most regions. If they had to decline referrals, the main reason was the survivors being ineligible for support. In fact, shelters define specific ineligibility criteria for admission, which may include, for example, substance addiction, severe mental impairment, and other circumstances that prevent the provision of adequate support and care. The most common forms of violence reported by survivors were domestic, physical and psychological violence, in most cases combined with other forms of violence. In 2024, women’s shelters in the Federation accommodated 347 women.

In Republika Srpska, there are three women-only shelters offering 62 beds. They are accessible 24/7, and women are not expected to pay for their accommodation. On average, women can stay in the shelters up to six months, but according to the complexity and severity of the case, their stay can be extended to several years. Women’s shelters provide different kinds of in-house services, including counselling/psychological support, referrals/collaboration with other services, practical advice and legal advice. The Foundation “United Women” also provides support through the Fund for the economic empowerment of women. This covers different costs incurred by the survivors, such as transportation costs to go to work, certificates and examinations necessary to start a work contract, fees for kindergarten, costs for agricultural seeds and other planting material, and to supplement retirement years.

In terms of accessibility, all shelters can accommodate refugee women, homeless women, women with cognitive disabilities, older women, women with older sons or with children with disabilities, and unaccompanied girls. Most shelters are accessible to women with sensory disabilities and with substance abuse issues, while only some can be accessed by and are accessible to women with physical disabilities. No shelter can accommodate women with uncertain residence permits and undocumented women. Shelters in Republika Srpska are funded through state funding and donations, are run by women’s NGOs with a gender-specific and feminist approach, and are present in most regions. If they had to decline referrals, it was mostly due to the lack of space and capacity to support the survivors, the survivors not being eligible for support, or the shelters not being adequately equipped for the needs of women and their children. The most common forms of violence reported by women were domestic violence, physical violence, and psychological violence. In 2024, 87 women were accommodated in women’s shelters in Republika Srpska.

❖ 3. WOMEN'S CENTRES

Total number of centres	11
Entities running women's centres	Women's NGOs (9 centres) Other NGO (2 centres)

There are 11 women's centres in the country: eight in the Federation of Bosnia and Herzegovina and three in Republika Srpska.

In the Federation of Bosnia and Herzegovina, the eight women's centres can provide referrals and collaboration with other services, while six also provide counselling/psychological support, legal advice and representation at court/police/social services. Additionally, women's centres also provide support to survivors of sexualised violence, including specialist psychological care, specialist advocacy services, community awareness and education for prevention, and legal advice.

All women's centres can be accessed by older women and unaccompanied girls under the age of 18, while most of them can be accessed by and are accessible to women with uncertain residence permits, undocumented women, and women with physical and sensory disabilities. Some centres can also be accessible to women with cognitive disabilities. Additionally, some centres are specialised in supporting specific groups of women: four centres for survivors or those at risk of trafficking, and one for women with uncertain residence permits/undocumented women, and refugee women. Women's centres are funded by donations and international funding, and only centres that also run shelters receive funding from the government. Women's centres are located in most regions.

In Republika Srpska, there are three women's centres supporting survivors of gender-based violence. They offer legal advice and referrals/collaboration with other services, while one also provides counselling/psychological support and representation at court/police/social services. Two centres are run by women's NGOs, while one is run by another NGO. Both centres are accessible to women with physical, cognitive and sensory disabilities, as well as older women. Women's centres in Republika Srpska are funded by donations and international funding, and they are located in cities. In 2024, they supported 45 women.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In the Federation of Bosnia and Herzegovina, three rape crisis centres were established in 2021, following the signing of the Agreement on Cooperation for the Establishment of Crisis Centres for Victims of Rape and Sexual Violence by the Gender Equality Agency. These centres are located within hospitals in major cities and are intended to provide support to women, girls, and children under the age of 18. However, they are not yet fully operational. At present, there is no available data confirming whether

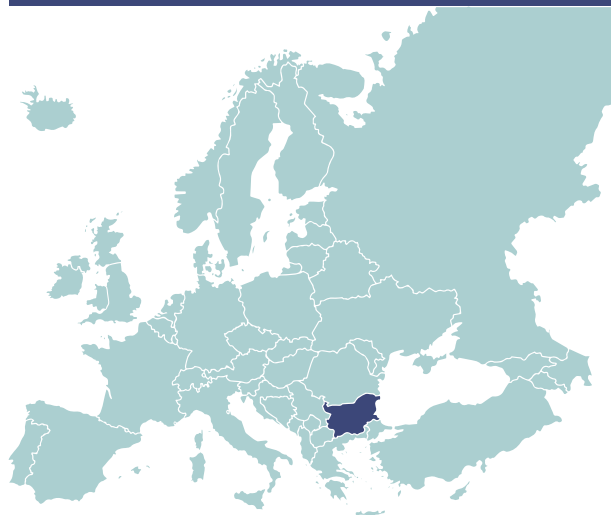
these centres are functioning. There are no services for survivors of sexualised violence in Republika Srpska.

👤 5. PRIMARY PREVENTION SERVICES

Public authorities do not collect data on primary prevention activities in the country. There is a Gender Action Plan of Bosnia and Herzegovina for the period 2023-2027, as well as a Strategy for the prevention and fight against domestic violence 2024-2027 in the Federation and a Strategy for Combating Domestic Violence of the Republic of Srpska (2020-2024). Both strategies include references to prevention. Additionally, the Law on Protection against Domestic Violence and Violence against Women in the Federation of BiH prescribes several obligations for preventive action, from the education of professionals to the promotion of non-violent values.

In the Federation of BiH, there is no specific budget allocated to primary prevention activities. The majority of such activities are conducted by women's NGOs that are financed through international projects; therefore, there is no systemic evaluation of primary prevention activities across the country. Activities delivered include feminist self-defence training, empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, self-care activities and awareness-raising campaigns or programmes. In Republika Srpska, only information and awareness-raising campaigns on domestic violence are funded by the state, and there is no evaluation in place to assess their impact.

BULGARIA



GENERAL COUNTRY INFORMATION

Population	6,445,481
Female population	3,347,783
Member of Council of Europe (year)	1992
Member of European Union (year)	2007
Member of United Nations (year)	1955
CEDAW ratified (year)	1982
CEDAW optional protocol ratified (year)	2006
Istanbul Convention signed (year)	2016
Istanbul Convention ratified (year)	No

SUMMARY

There is one national women's helpline in Bulgaria which is free of charge and operates 24/7 but does not offer multilingual support. There are 13 women-only shelters with 150 available beds as well as 20 women's centres supporting all survivors of violence, predominantly survivors of domestic violence. There is one sexual violence referral centre for survivors of sexualised violence, which also runs a specialised helpline. Measures related to primary prevention are not included in official documents like the National Action Plan and there is no specific budget allocated to primary prevention activities. The state does collect data on women's specialist services, and this data is publicly available. However, data collected relates mainly to social services offering support to women survivors of violence, and separate statistics are not available for specialist services for survivors of domestic violence.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	No
Total number of calls/callers in 2024	2,800 callers ²⁴

24 This figure is for 2023.

There is one national helpline in Bulgaria which is run by Animus Association and is called the National Hotline for Victims of Violence (*Национална гореща телефонна линия за пострадали от насилие*, tel.: +359 80018676/+359 29817686). The helpline is free of charge and available 24/7 but does not offer multilingual support. Bulgaria therefore does meet the Istanbul Convention minimum standards for the provision of a national women's helpline. The helpline provides support to survivors of domestic violence, sexualised violence, human trafficking, as well as other types of violence. Funding for the helpline comes from the state and the telecommunications company Vivacom. The most common form of violence reported by callers to the helpline in 2024 was domestic violence. Over 2,800 people called the helpline in 2023, 81% of which were women survivors of domestic violence.

There are also other helplines supporting survivors of violence against women run by members of the Alliance for Protection against Gender-Based Violence and also other helplines run by other NGOs. There was a second national helpline run by the Alliance for Protection against Gender-Based Violence (*Алианс за защита от насилие, основано на пола*) which was available 24/7 and free of charge, but is no longer active.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	13
Total number of all shelters accessible to women	13
Existing number of beds	150
24/7 access	All

There are 13 women-only shelters in Bulgaria with 150 available beds. All the women-only shelters offer 24/7 access, and women are not expected to pay for their accommodation in shelters. On average, women are allowed to stay in the women-only shelters for up to three months. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services, practical advice, and legal advice.

All women-only shelters are equipped to support homeless women and older women (65 years and above). Most women-only shelters can accommodate unaccompanied girls under the age of 18. Some can be accessed by and are accessible to refugee women, women with physical and/or sensory disabilities, women with older sons (14 years and above), and women with children with disabilities. However, none of the women-only shelters can accommodate women with uncertain residence permits/undocumented women, women with substance abuse issues, and transgender women. There are also no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence.

Funding for women-only shelters in Bulgaria comes predominantly from the state. Eleven women-only shelters are run by NGOs, including women's NGOs with a gender specific and feminist approach, while two are state-run shelters. Women-only shelters are located only in major cities, or in locations close to major cities. The main reasons why women-only shelters had to decline referrals were no space/capacity to support the survivor and the shelter not being adequately equipped for the needs of the women/children. The most common forms of violence reported to women-only shelters in 2024 were domestic violence, physical violence, and violence directly/indirectly affecting children. Approximately 200 women and children were accommodated in women-only shelters in 2024, although there is no official data.

In addition to the women-only shelters, there are other shelters for survivors of human trafficking and crisis centres for abused children, including girls. It is estimated that there are nine such additional services, but their overall bed capacity to accommodate survivors is unknown.²⁵

❖ 3. WOMEN'S CENTRES

Total number of centres	20
Entities running women's centres	Women's NGOs (12 centres) Other NGOs (5 centres) State (3 centres)

There are 20 women's centres in Bulgaria, for all survivors and predominantly supporting survivors of domestic violence. Centres usually provide both non-residential and residential support as they sometimes provide emergency accommodation as well. All women's centres provide counselling/psychological support as well as referrals/collaboration with other services, and 15 centres offer financial and social welfare support/advocacy. Additionally, ten centres offer legal advice, and approximately ten centres provide housing advice and employment. Some centres offer representation at court/police/social services. The counselling services provided by women's NGOs are open to all survivors of domestic violence, even if there is a focus on women survivors. Women's centres also provide specialist support for survivors of sexualised violence including specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice.

All women's centres are equipped to support older women (65 years and above). Most women's centres can be accessed by and are accessible to refugee women and women with physical, cognitive or sensory disabilities. Some women's centres can be accessed by women with uncertain residence permits/undocumented women, transgender women, and unaccompanied girls under the age of 18. There are no specialised centres supporting particular

groups of survivors nor women survivors or those at risk of specific forms of violence.

Funding for women's centres in Bulgaria comes from the state, donations, as well as international funding. Twelve women's centres are run by women's NGOs, five by other NGOs and three by the state. Women's centres are located just in major cities. In 2024, approximately 700 women accessed counselling services from women's centres run by women's NGOs.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There is one sexual violence referral centre for survivors of sexualised violence in Bulgaria, Center "Vselena", which is located in Burgas. The centre opened in 2016, and available support includes specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention as well as legal advice. The service is funded by the state and through donations and is accessible to all survivors of sexualised violence. The centre also runs a 24/7, nationwide specialised helpline for survivors of sexualised violence (tel.: +359 80018017) which is based at the Burgas University Hospital Crisis Intervention and managed by trained psychologist-consultants. The helpline also provides assistance in contacting the Burgas Ministry of Interior Department to receive access to a forensic doctor.

👤 5. PRIMARY PREVENTION SERVICES

Public authorities in Bulgaria do not collect data on primary prevention activities and provisions on primary prevention are not included in official documents like the National Action Plan on violence against women and girls for 2020-2025. There is no official definition of primary prevention in Bulgaria, and the state does not allocate a specific budget to primary prevention activities, nor does it evaluate the impact of such activities. Available activities include empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, training for primary prevention practitioners, and awareness-raising campaigns or programmes.

²⁵ Information from the WAVE Country Report 2021.

CROATIA



GENERAL COUNTRY INFORMATION

Population	3,861,967
Female population	1,986,557
Member of Council of Europe (year)	1996
Member of European Union (year)	2013
Member of United Nations (year)	1992
CEDAW ratified (year)	1992
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2013
Istanbul Convention ratified (year)	2018

SUMMARY

There are two national women's helplines in Croatia, both of which are free of charge, offer multilingual support, and one of them is accessible 24/7. Additionally, there are several other regional helplines supporting survivors of violence against women in Croatia. There are 26 shelters accessible to women in Croatia, with a total of 402 beds provided. Eleven of them are women-only shelters. There are approximately 61 women's centres, 30 of which are exclusively for survivors of violence against women and 31 for all survivors of gender-based violence, as well as specialised services for survivors of sexualised violence. There is no specific budget allocated to primary prevention, and no official documents, such as National Action Plans, include measures related to it. The state collects data on women's specialist services, but not on primary prevention activities, and this data is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	One yes, one no
Multilingual support	Yes
Total number of calls/ callers in 2024	2,065 calls 869 callers

Croatia has a state-wide women's helpline for survivors of violence against women (*Nacionalni pozivni centar za žene koje su preživjele nasilje*), available free of charge and operated by the Autonomous Women's House Zagreb (*Autonomna ženska kuća Zagreb*) under the EU helpline number 116 016. This helpline is not available 24/7 due to a lack of additional state funding, and it offers multilingual support in Croatian, English, French, Italian, and German. There is a second free, 24/7 helpline called Women's Help Now (*Ženska pomoć sada*, tel. +385 800655222), which provides support in Croatian and English. Both helplines provide assistance for survivors of various forms of violence, including domestic violence, partner violence, gender-based violence, and institutional violence. The main funding sources are national and local authorities, donations and international funding, among others. Together, the helplines received 2,065 calls from 869 callers in 2024, with physical and psychological violence being the most reported forms. While the first helpline does not meet the Istanbul Convention standards due to its limited hours, the second helpline complies with the Convention's requirements, being available 24/7 and free of charge. Additionally, there are several other helplines supporting survivors of violence against women in Croatia, specialised in, among others, aiding survivors of sexualised violence, human trafficking and war violence.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	11
Total number of all shelters accessible to women	26
Existing number of beds	402
24/7 access	Some

There are 11 women-only shelters in Croatia offering 130 beds. Some of the women-only shelters provide 24/7 access, and none require women to pay for their accommodation. The average length of stay in these shelters can vary depending on individual circumstances and shelter policies. The most common in-house services offered include casework, counselling and psychological support, legal advice, referrals to or collaboration with other services, and practical advice on issues such as housing, healthcare, employment, and social benefits.

Most women-only shelters in Croatia can accommodate women with uncertain residence permits or undocumented status, refugee women, older women and homeless

women. Some shelters are equipped to support women with physical, cognitive and/or sensory disabilities. Additionally, some shelters can accommodate women with substance abuse issues, transgender women, women with older sons and women with children with disabilities. None are accessible to unaccompanied girls under the age of 18. There are no specific shelters in Croatia dedicated exclusively to particular groups of survivors.

Funding for women-only shelters in Croatia primarily comes from state funding provided by national and local authorities, as well as donations. Shelters are located in most regions of Croatia, and women's NGOs with a gender-specific and feminist approach run seven women-only shelters. In 2024, the main reasons for declining referrals were the lack of space and capacity to support the survivor, and the shelters not being adequately equipped to meet the specific needs of women and their children. The most common forms of violence reported by survivors were physical violence, psychological violence, and violence directly or indirectly affecting children. Around 120 women and children were accommodated in women-only shelters during 2024.

Besides the 11 women-only shelters, Croatia has 15 other shelters providing a total of 272 beds. These are faith-based and government-run (local or state) facilities that are not women-only. They are available to survivors of violence without specifying particular groups served. Some of these shelters offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres		61
Entities running women's centres	Women's NGOs (50 centres)	
	Other NGOs (11 centres)	

Croatia has approximately 61 women's centres, of which 30 are exclusively for women survivors of violence against women and 31 for all survivors of gender-based violence, including women. Most centres provide counselling and psychological support, financial and social welfare support and advocacy, and referrals or collaboration with other services. Some offer legal advice, housing advice, employment support, and residential services, while a few provide representation at court, police, or social services. Several centres specialise in supporting survivors of trafficking, undocumented women, women with disabilities, older women, and transgender women. Women's centres also provide specialist support services for survivors of sexualised violence, including needs assessment, specialist psychological care, advocacy, community awareness, education for prevention, and legal advice.

Some women's centres in Croatia are accessible to women with uncertain residence permits and undocumented women, refugee women, older women, and transgender women. The same applies to the accessibility of women's centres for women with physical, cognitive, or sensory

disabilities. There are three women's centres specialised in supporting survivors at risk of or affected by forced marriage, honour-based violence, female genital mutilation, women with uncertain residence permits/undocumented women, refugee women, and Black and Minority Ethnic women. In Croatia, 50 women's centres are run by women's NGOs and 11 by other NGOs. Funding mainly comes from state funding provided by national and local authorities, donations, and international funding. Women's centres operate in all regions of the country. In 2024, they supported approximately 5,000 women.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

Croatia has specialised services for survivors of sexualised violence, including one centre specialised in supporting survivors of sexualised violence and other centres that offer counselling to survivors of different forms of violence, including sexualised violence. Such services provide needs assessment, specialist psychological care, specialist advocacy services, community awareness and prevention activities, and legal advice. Funding comes mainly from state funding, as well as donations. Services are located only in major cities and support women aged 18 and over, young women and children under 18, and men survivors of sexualised violence. There are two helplines specialised in supporting survivors of sexualised violence in Croatia.

☞ 5. PRIMARY PREVENTION SERVICES

There is no official data collection on primary prevention activities in Croatia, and the country does not have a National Action Plan or an official definition of primary prevention of violence against women and girls. A working group was established to produce a National Plan for Protection against Violence against Women and Domestic Violence, and an Action Plan for its implementation. The state does not allocate a specific budget for primary prevention activities. Existing primary prevention measures include empowerment activities for girls and women, school-based programmes, training for primary prevention practitioners, and awareness-raising campaigns or programmes.

THE REPUBLIC OF CYPRUS



GENERAL COUNTRY INFORMATION

Population	966,365
Female population	490,771
Member of Council of Europe (year)	1961
Member of European Union (year)	2004
Member of United Nations (year)	1960
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2015
Istanbul Convention ratified (year)	2017

SUMMARY

In the Republic of Cyprus, the national women's helpline is free of charge, operates 24/7 and provides multilingual support. Four women-only shelters offer 79 beds, and there is one women's centre which provides several support services, including specialist services for survivors of sexualised violence. The National Action Plan for 2023–2028 includes provisions for primary prevention activities, as well as a dedicated budget. The state collects data on women's specialist support services and on some primary prevention activities, and this data is available upon request.

In the northern part of Cyprus, there is one women's helpline, which is free of charge and available 24/7. Additionally, there is only one women-only shelter with 48 beds available and two women's centres. There are no specialised services for women and girls who have experienced sexualised violence in Northern Cyprus, and there are no specific provisions on primary prevention. In Northern Cyprus, the state does not collect data on women's specialist services.

The Republic of Cyprus does not exercise control over the northern territory of the country, which is administered by Turkish Cypriot authorities. For this reason, the services of the Republic of Cyprus and those of Northern Cyprus are presented in separate sections under this profile.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	6,883 calls/contacts

In the Republic of Cyprus, there is one national women's helpline called National Helpline 1440 (*Εθνική Γραμμή Βοήθειας* 1440 – for people living in the Republic of Cyprus: tel.: 1440, for people calling from abroad: tel.: +357 22339001). It is run by the Association for the Prevention and Handling of Violence in the Family (SPAVO), is free of charge and operates 24/7, offering multilingual support in Greek and English; thus, it meets the standards of the Istanbul Convention. The SPAVO helpline also offers a live chat (available on www.domviolence.org.cy) and an SMS service (tel.: +357 99984042), which are both free of charge and available 24/7. Through these three channels, SPAVO offers support for all forms of violence against women, domestic violence and gender-based violence. The helpline receives state funding. In 2024, it received 6,883 calls/contacts, and the most common form of violence reported was domestic violence, including in particular psychological, physical and economic violence.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	4
Total number of all shelters accessible to women	5
Existing number of beds	79 ²⁶
24/7 access	All

There are four women-only shelters in the Republic of Cyprus operated by the Association for the Prevention and Handling of Violence in the Family (SPAVO), offering 79 beds; 3 shelters for survivors of domestic violence and 1 shelter for young and underage migrant mothers. Another shelter for survivors of trafficking accessible to women is operated by the Social Welfare Services. Women-only shelters offer 24/7 access, and women are not expected to pay for their accommodation. On average, women stay in the shelters between three and six months, but all cases are evaluated separately, and high-risk cases can receive accommodation for longer. The types of in-house services provided by women-only shelters include casework,

26 Number of beds in the four women-only shelters operated by SPAVO.

counselling, practical advice and referrals/collaboration with other services.

All shelters can accommodate women with uncertain residence permits/undocumented women, refugee women, homeless women, older women, transgender women and women with children with disabilities. Most shelters can be accessed by and are accessible to women with physical, sensory and cognitive disabilities, women with substance abuse issues, women with older sons and unaccompanied girls. Since 2023, Sophie's House has been operating by SPAVO following a Ministerial Decision, to support young and underage migrant mothers who have arrived in Cyprus as asylum seekers or survivors of trafficking. At the time of writing, the programme hosts twelve young women and eleven newborns and children. It provides women with a safe space to live, as well as practical support, counselling, accompaniment, childcare, training on parental skills, vocational support, medical services, and legal counselling.

Women-only shelters are funded through state funding and donations. Shelters are present in most regions, and a fifth shelter is expected to be opened in 2026, covering the last two provinces of Larnaca and Ammochostos, which currently have no shelter. If women-only shelters had to decline referrals, the main reasons were the lack of space/capacity to accommodate survivors with children, survivors being ineligible for support, or the shelters not being adequately equipped for the needs of the women/children. If women-only shelters for domestic violence survivors do not have capacity, SPAVO rents alternative safe accommodations that offer the same services as shelters. The most common forms of violence reported by survivors to the women-only shelters in 2024 were domestic, physical and economic violence, and in that year, 447 women found support in women-only shelters.

❖ 3. WOMEN'S CENTRES

Total number of centres	1
Entities running women's centres	Women's NGO

There is one women's centre in the Republic of Cyprus, called Woman's House, which is only for survivors of violence against women, operating as one-stop-centre where all competent agencies cooperate under one roof, including the Association for the Prevention and Handling of Violence in the Family (SPAVO), Cyprus Police, Social Welfare Services, and Mental Health Services. The centre provides needs and risk assessment, counselling/psychological support, legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, as well as multidisciplinary multi-agency meetings. The Woman's House Crisis Intervention Team provides survivors' accompaniment services, and if a woman needs residential support, she is referred to SPAVO shelters. The centre also provides support for survivors of sexualised violence, including specialist

forensic and medical care, needs and risk assessment, specialist psychological care, specialist advocacy services, legal advice, as well as community awareness and education for prevention.

The Woman's House can be accessed by and is accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive and sensory disabilities, older women, transgender women and unaccompanied girls. Additionally, staff are trained to support survivors from specific groups, including survivors (at risk) of forced marriage, honour-based violence, female genital mutilation, trafficking in persons, as well as women with uncertain residence permits/undocumented women, refugee women and black and minority ethnic women.

The centre is coordinated by SPAVO and is funded by the state. It is located in the capital city, and cases from other regions may also be referred to the Woman's House if deemed necessary by the Police and/or the Social Welfare Services. Survivors of rape are referred to the Woman's House regardless of their province of residence. In 2024, the House supported 405 women.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

The Woman's House provide specialist support for survivors of sexualised violence, including specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention, and legal advice for women, young women and girls under 18. Funding is provided by the state, and the Woman's House is located in the capital city. Additionally, the centre "NIMA" is the first specialised centre in Cyprus that provides psychological support and treatment to adults, including women, who have suffered sexual abuse in their childhood, and their families. It is operated by the Cyprus Family Planning Association following a Ministerial Decision since July 2023. It offers psychological therapy and support for healing trauma, legal advice, a referral and information hub, and prevention through education and community outreach. All of its services are provided free of charge, and the centre operates with a multidisciplinary team.

👤 5. PRIMARY PREVENTION SERVICES

In Cyprus, public authorities collect limited data on primary prevention activities. Cyprus is implementing a National Action Plan for the Prevention and Handling of Violence against Women for 2023–2028. This includes provisions for primary prevention activities (campaigns, educational seminars, capacity-building), which are monitored by various ministries, the Advisory Committee for Preventing and Combating Violence in the Family, and the National Coordinating Body for the Prevention and Combating Violence against Women. The National Action Plan focuses

on priorities arising from both the Istanbul Convention and the law "On the Prevention and Handling of Violence against Women and Domestic Violence", as well as from the results of the GREVIO Evaluation Report on the compliance of Cyprus with the provisions of the Convention. The National Action Plan includes a definition of primary prevention, and a specific budget is allocated to primary prevention activities. These include empowerment activities for girls and women, school-based primary prevention programmes, training for primary prevention practitioners and awareness-raising campaigns or programmes. No evaluation of the impact of primary prevention activities is conducted in the country. In practice, NGOs such as SPAVO and the Mediterranean Institute of Gender Studies play a central role in implementing prevention measures, including EU-funded projects, trainings for professionals and practitioners, school-based programmes and community awareness activities. Cyprus also participates annually in the international '16 days of Activism against Gender-Based Violence' campaign, which raises public awareness but is not systematically evaluated.

NORTHERN CYPRUS

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

There is one women's helpline in the northern part of Cyprus, called Hello 183 Domestic Violence Support Line (*Alo 183 Şiddetle Mücadele Hattı*, tel.: +90 183), which is free of charge, and available 24/7, therefore meeting the standards of the Istanbul Convention; nevertheless, it does not offer multilingual support. The helpline is run by the social services department functioning under the Ministry of Employment and Social Security and is funded by central authorities. It is a general violence prevention and support helpline which specifically supports women and children facing violence. The most common forms of violence reported by callers to the state-wide women's helpline in 2024 were domestic, physical and sexualised violence.

The Association of Women to Support Living (KAYAD) previously operated a helpline, which was discontinued due to insufficient resources. There are no other helplines available in northern Cyprus.

2. SHELTERS ACCESSIBLE TO WOMEN

There is one women-only shelter in northern Cyprus, which is located in Nicosia and offers 48 beds. The shelter offers 24/7 access, and women are not expected to pay for their accommodation. Women can stay in the shelter up to three months, and the types of in-house services most often provided are counselling/psychological support and legal advice. The shelter is accessible to women with uncertain residence permits/undocumented women, refugee

women, homeless women, older women, transgender women, women with older sons, and women with children with disabilities. There are no other shelters specialised in supporting particular groups of survivors, nor different shelters accessible to women. The shelter is funded through state funding and international funding, and is run by municipal staff of the Turkish Municipality of Nicosia. If the shelter had to decline referrals, it was mostly due to the survivors being ineligible for support, for instance, in the case of women with substance abuse issues. The most common forms of violence reported by survivors to the women-only shelters in 2024 were domestic, physical and sexualised violence.

3. WOMEN'S CENTRES

There are two women's centres in northern Cyprus that support all survivors of gender-based violence. The one run by the Association of Women to Support Living (KAYAD) provides counselling/psychological support, legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, representation at court/police/social services and residential support services. Additionally, it also supports survivors of sexualised violence through specialist psychological care, specialist advocacy services, community awareness and education for prevention and legal advice. The centre receives international funding and is located in the capital city. Additionally, the Turkish municipality of Nicosia runs a municipal women's centre. Women's centres are open to survivors of specific forms of violence (e.g., forced marriage, honour-based violence, trafficking in persons, etc.) and can support survivors from specific groups (e.g., refugee women, women with disabilities, etc.), but they are not specialised in providing support tailored to their specific needs.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no specialised services for women and girls who have experienced sexualised violence in northern Cyprus.

5. PRIMARY PREVENTION SERVICES

Public authorities do not collect data on primary prevention activities, and there is no budget allocated to conduct them. Primary prevention activities organised in northern Cyprus include feminist self-defence trainings, empowerment activities for girls and women and awareness-raising campaigns or programmes.

THE CZECH REPUBLIC



GENERAL COUNTRY INFORMATION

Population	10,900,555
Female population	5,557,945
Member of Council of Europe (year)	1993
Member of European Union (year)	2004
Member of United Nations (year)	1993
CEDAW ratified (year)	1993
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2016
Istanbul Convention ratified (year)	No

SUMMARY

There is one national women's helpline in the Czech Republic, which is free of charge, available 24/7 and provides multilingual support, as well as other helplines that can support survivors of violence against women. There are seven women-only shelters and centres with crisis beds in the country, with approximately 200 beds, alongside 26 women's centres and two services for survivors of sexualised violence. No data is collected on primary prevention activities, nor is a budget allocated for them. The state does not collect data on women's specialist services.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	3,913 calls/contacts

There is one national women's helpline in the Czech Republic, called Hotline for victims of violence against

women (*Linka pro oběti násilí na ženách*, tel.: 116 016). It is run by the ROSA Centre for women, is free of charge and available 24/7, meeting the standards of the Istanbul Convention. It offers multilingual support in Czech and English, and Ukrainian is available upon request. The helpline supports survivors of domestic violence and sexualised violence, and is financed by the state and donations. This helpline began operating with the European harmonised number in November 2024. Before November 2024, the same service was provided on a different number operated by the ROSA Centre for Women. In 2024, the helpline received 3,913 calls and chat contacts. The most common forms of violence reported by callers were domestic violence (in all its forms, including psychological, physical, sexualised and economic violence), as well as stalking and violence directly/indirectly affecting children.

Other helplines in the Czech Republic provide support to survivors of violence against women. The organisation proFem runs an information hotline for survivors of domestic and sexualised violence, which operates around the clock, but is not free of charge (tel.: +420 608 222 277); proFem also runs a legal advice line, active on Wednesdays between 17:30 and 20:30 (tel.: +420 777 012 555). The organisation Acorus runs a helpline for survivors of domestic violence, which is available around the clock, but not free of charge (tel.: +420 283 892 772). The DONA Hotline is available to survivors of crimes and domestic violence. It is not gender-sensitive, and is available 24/7 and free of charge.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	7
Total number of all shelters accessible to women	N/A
Existing number of beds	200
24/7 access	Some

There are seven women-only shelters and centres with crisis beds in the Czech Republic, with approximately 200 beds.²⁷ Some of them offer 24/7 access, and women are expected to pay a fee for their accommodation. On average, women can stay in the shelters for up to one year. The types of in-house services provided include casework, counselling and psychological support, referrals and collaboration with other services, and practical and legal advice.

All the women-only shelters are accessible to older women, and most of them are accessible to homeless women. Some women-only shelters accommodate women with physical disabilities, women with cognitive disabilities, women with substance abuse issues, as well as women with older sons (14 years and above). There is one crisis accommodation for survivors of trafficking in the country. Funding for women's shelters comes from the state, donations, and international funding, and shelters are located

²⁷ Data from the WAVE Country Report 2023.

only in major cities. The main reasons for declining referrals were lack of space, insufficient capacity to support survivors or to accommodate survivors with children, or ineligibility for support. The most common forms of violence reported by survivors are normally domestic violence (in all its forms, including psychological, physical, sexualised and economic violence), as well as stalking and violence directly/indirectly affecting children.

Other shelters in the Czech Republic can support survivors of gender-based violence and accommodate both women and men. Some of these offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	26
Entities running women's centres	(N/A)

There are approximately 26 women's centres and intervention centres in the Czech Republic, of which five are only for women survivors of Violence Against Women, are run by women's NGOs and have a gender-sensitive approach. Women's centres provide counselling, psychological support, legal advice, housing advice, referrals and collaboration with other services, representation at courts and other institutions, and residential support services. Additionally, they also provide specialised support to survivors of sexualised violence, including specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention and legal advice.

While all women's centres are accessible to older women, most of them are equipped to support women with uncertain residence permits, undocumented women, refugee women and women with sensory disabilities. Some centres are accessible to and can be accessed by women with cognitive disabilities and unaccompanied girls under 18. In fact, there are centres specialised in supporting minor girls and older women. There are no centres specialised in supporting survivors (at risk) of forced marriage, honour-based violence and female genital mutilation.

Women's centres are funded by the state, donations, and international funding, and are located only in major cities.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

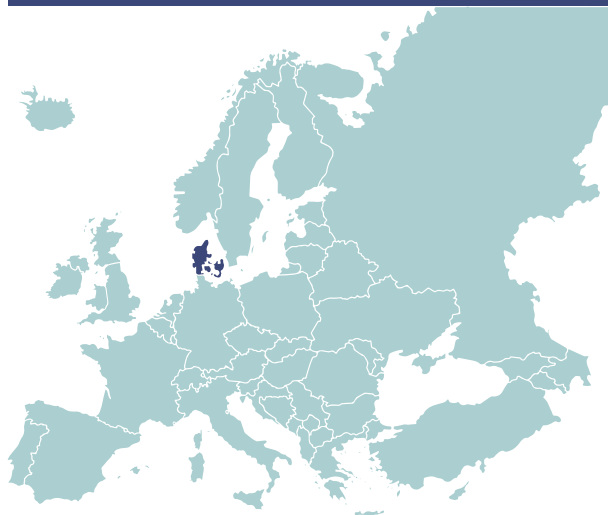
There is one specialised rape crisis centre for survivors of sexualised violence, run by proFem. This centre provides specialist forensic and medical care, needs assessment, specialist psychological care, and specialist advocacy services. Additionally, it also provides a specialised helpline for sexualised violence cases. The funding comes from the state, donations and international funding. It is located in Prague and supports women, young women and children under the age of 18.

Additionally, the centre Persefona, based in Brno, supports survivors of domestic violence and sexualised violence. It offers different services for survivors of sexualised violence, including legal and psychological counselling, self-help groups, accompaniment to police, court and other authorities, and referrals.

☞ 5. PRIMARY PREVENTION SERVICES

Public authorities do not collect data on primary prevention activities, and there is no official definition of primary prevention included in the national strategies to counter violence against women. No budget is allocated to this field by public authorities. Activities conducted in the Czech Republic include feminist self-defence trainings, school-based primary prevention programmes and awareness-raising campaigns or programmes. The NGO *Kon-sent* works on the prevention of sexualised violence and focuses on awareness-raising and education activities.

DENMARK



GENERAL COUNTRY INFORMATION

Population	5,961,249
Female population	2,997,558
Member of Council of Europe (year)	1949
Member of European Union (year)	1973
Member of United Nations (year)	1945
CEDAW ratified (year)	1983
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2013
Istanbul Convention ratified (year)	2014

SUMMARY

Denmark has two national women's helplines, which are free of charge, operate 24/7 and offer multilingual support. There are 79 women-only shelters with a total of 775 beds, as well as 16 other shelters available to women with 191 beds. Six women's centres offer a range of services and specialised support, and there are specialised services for survivors of sexualised violence including nine regional Sexual Assault Centres/Centres for Rape Victims. While no systematic data collection specifically focused on primary prevention activities exists, there are several National Action Plans addressing violence against women in Denmark, which include specific provisions on primary prevention. The state collects data on women's specialist services, and this information is publicly available.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	12,612 calls ²⁸

Two national women's helplines operate in Denmark. One of them is the state-funded Live Without Violence's National Hotline (*Lev Uden Volds nationale hotline*, tel.: +45 1888). It is run by Lev Uden Vold, is free of charge, available 24/7 and provides multilingual support in any language provided by the interpretation service. The helpline does not exclusively operate for women as any survivor of domestic or intimate partner violence, as well as relatives, survivors of rape, and perpetrators can call for assistance. In 2024, *Lev Uden Vold's* helpline registered 12,612 calls, of which 8,687 led to counselling conversations, with psychological violence reported in 84% of cases, physical violence in 38%, and 85% of violence occurring in close relationships. The second Danish women's helpline, Danner's National Crisis Hotline (*Danners krisetelefon*, tel.: +45 33330047), also provides its services free of charge, operates 24/7 and offers support in Danish and English. It is mostly funded by donations. Both helplines meet the Istanbul Convention standards for the provision of a national women's helpline.

In addition, there are other helplines offering support to women exposed to violence in Denmark. Key services are, Break the Silence (*Bryd Tavsheden*, tel.: +45 29271717), which offers anonymous counselling via phone and chat to children and youth exposed to domestic or dating violence and Dialogue Against Violence (*Dialog mod Vold*, tel.: +45 35301717), supporting both perpetrators and survivors of domestic violence. Moreover, Mother's Aid – Hold Point (*Mødrehjælpen – Holdepunkt*, tel.: +45 33458600) supports women and families affected by intimate partner violence. Additionally, Medusa – A Life Without Violence (tel.: +45 70277607) offers counselling to women exposed to intimate partner violence and the Danish Stalking Centre (*Dansk Stalking Centre*, tel.: +45 30704047) provides counselling and legal guidance to survivors of stalking.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	79
Total number of all shelters accessible to women	95
Existing number of beds	966
24/7 access	Most ²⁹

Denmark has a total of 79 women-only shelters offering 775 beds with mostly 24/7 access. Women are generally expected to make a small daily co-payment of DKK 101,19

²⁸ These numbers only refer to the Lev Uden Vold national helpline.

²⁹ This applies to women-only shelters, only some of the other shelters accessible to women offer 24/7 access.

(€13,53) although exemptions are available to those with low income or existing housing costs. The average length of stay in women-shelters is more than a year. The types of in-house services most often provided by women-only shelters are casework, counselling and psychological help, referrals and collaboration with other services, as well as practical and legal advice.

In Denmark, all women-only shelters are equipped to support older women, and most can support women with uncertain residence status. Some shelters can be accessed by and are accessible to women with physical, cognitive and/or sensory disabilities, women with substance abuse issues, transgender women, women with older sons, women with children with disabilities and unaccompanied girls under the age of 18. None of the women-only shelters are able to accommodate refugee women. There are specific shelters for particular groups, such as LGBTQI+ people and survivors of honour-based violence.

Women-only shelters are mainly funded by national and local authorities. Accommodation is paid for by the municipality with a 50% state reimbursement. Women-only shelters are located in all regions of Denmark and run by women's NGOs with a gender-specific and feminist approach, followed by other NGOs, the state at various levels, faith-based organisations, and other entities such as government-organised NGOs. In 2024, the most common reasons for declining referrals were lack of space, the survivor being ineligible for support and insufficient equipment to meet specific needs such as accessibility for women with disabilities. The main forms of violence reported by survivors were domestic, physical and psychological violence. In 2024, 5,783 women and children were accommodated in women-only shelters.

In addition to women-only shelters, Denmark has 16 other shelters for survivors of violence, offering a total of 191 beds. These include shelters for both men and women, as well as LGBTQI+ shelters, and men-only shelters. They are available to women, men, transgender women and men, and non-binary people. Some of these shelters offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	6
Entities running women's centres	Women's NGOs State

There are six women's centres in Denmark, from which four are exclusively for women survivors of violence and two support all survivors of violence. These centres follow a feminist and human rights-based approach, offering individualised counselling, legal and social support, and proactive outreach. Organisations such as The Joan Sisters (*Joan-Søstrene*), Tell Someone (*Sig det til nogen*), and Out of the Shadow of Violence (*Mødrehjælpen – Ud af Voldens Skygge*) offer support to women survivors. All these

centres provide counselling and psychological support, three centres offer legal advice, and one offers referrals and collaboration with other services. Moreover, women's centres provide specialist support for survivors of sexualised violence, including specialist psychological care, specialist sexualised violence advocacy services and legal advice.

All women's centres in Denmark are equipped to support older women. Most can be accessed by and are accessible to women with uncertain residence permits, undocumented women, refugee women, women with physical, cognitive, and/or sensory disabilities and transgender women. Some shelters can support unaccompanied girls under the age of 18. There are centres that provide specialised support to survivors of honour-based violence and trafficking.

Four women's centres are run by women's NGOs, one by another NGO and one by the state. Main funding sources for women's centres in Denmark are national and local authorities as well as donations. Geographically, women's centres are present in most regions. In 2024, only one centre, *Mødrehjælpen's Ud af Voldens Skygge*, reported public figures and assisted 168 women and 163 children.

🚰 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

Denmark provides a comprehensive system of specialised services for women and girls who have experienced sexualised violence including rape crisis centres, sexual violence referral centres (SVRCs), crisis/medical services, and specialised helplines. In Denmark, SVRCs exist under the name of Sexual Assault Centres (*Center for Seksuelle Overgreb*) and are integrated into the public hospital system. These centres provide acute, free, and multi-disciplinary support to survivors of sexualised violence, including forensic medical examinations, collection and preservation of evidence (for use in police investigations or legal proceedings), psychological first aid and follow-up, medical treatment (including STI testing, emergency contraception, injury care) and support with police reporting (optional). These services are available without referral and independent of police involvement, and they are accessible 24/7 at hospitals in major cities including Copenhagen (Rigshospitalet), Aarhus, Odense, Aalborg, and Esbjerg. In total, there are nine such regional services which are also referred to as Centres for Rape Victims (*Center for Voldtægtsofre*). They are located throughout the country and cover most regions in Denmark. The country's two largest centres (the centres in Aarhus and Copenhagen) also have a knowledge centre function, which involves the collection and dissemination of knowledge on sexual abuse. These centres also operate a helpline, the phone number varies by hospital (e.g., +45 35457898 at Rigshospitalet in Copenhagen) and provides medical and psychological support after rape or assault.

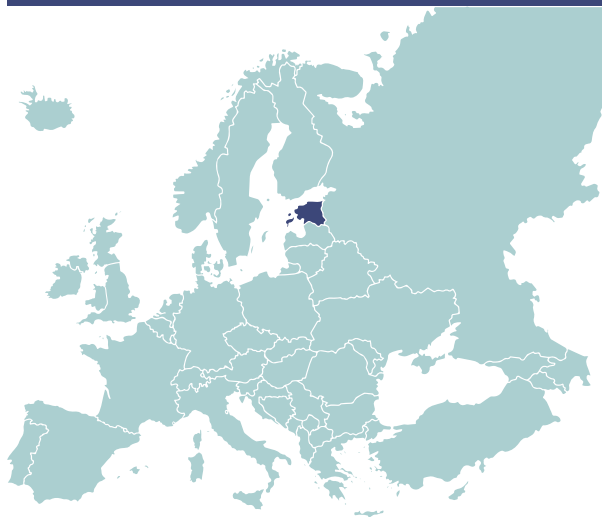
In addition to these medical services, women's centres such as *Joan-Søstrene*, *Danner*, and *Mødrehjælpen* provide non-residential, trauma-informed support to women survivors of sexualised violence. These include psychosocial counselling, legal guidance, peer support, safety planning and empowerment-based recovery. Confidential support to survivors of sexualised violence is offered via national helplines such as 116 006 and Danner's helpline. Sexualised violence services in Denmark are primarily state funded (with additional support from donations), free of charge, and available to adult women and girls under 18.

5. PRIMARY PREVENTION SERVICES

In Denmark, there is currently no systematic or centralised data collection specifically focused on primary prevention activities aimed at preventing violence against women and girls. While national data is generally gathered through administrative registers and surveys coordinated by Statistics Denmark and relevant ministries, these do not specifically track primary prevention efforts. Denmark has adopted several National Action Plans addressing violence in close relationships, which include primary prevention measures such as awareness-raising campaigns, school-based education programmes, and professional training. However, these documents do not include a definition of primary prevention.

Despite the lack of a clear definition or coordinated data system, Denmark does implement various primary prevention initiatives. These include empowerment activities for girls and women, school-based programmes, and public awareness-raising campaigns. There is no centralised system to evaluate the scope or effectiveness of these activities, and it is not known whether a specific budget is allocated to them. Reports by GREVIO and national stakeholders have highlighted the need for more structured and targeted data collection on both the reach and impact of primary prevention activities in Denmark.

ESTONIA



GENERAL COUNTRY INFORMATION

Population	1,374,687
Female population	723,074
Member of Council of Europe (year)	1993
Member of European Union (year)	2004
Member of United Nations (year)	1991
CEDAW ratified (year)	1991
CEDAW optional protocol ratified (year)	2025
Istanbul Convention signed (year)	2014
Istanbul Convention ratified (year)	2017

SUMMARY

There is one national women's helpline in Estonia that is free of charge, operates 24/7 and provides multilingual support. There are 19 women-only shelters with approximately 170 available beds. All women-only shelters also provide non-residential services such as counselling and also operate as women's support centres. Estonia also has five rape crisis centres located in regional hospitals. Public authorities in Estonia do not collect data on primary prevention activities, although provisions on primary prevention are included in official documents such as National Action Plans. There is no specific budget allocated to primary prevention activities. The state does collect data on women's specialist support services, and this information is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/ callers in 2024	5,647 calls 1,299 callers

There is one national women's helpline in Estonia called the Victim's Support Crisis Helpline (*Ohvriabi kriisitelefoni*, tel.: 116006). The helpline is run by the Victim Support Unit of the state-run Social Insurance Board and is free of charge, available 24/7, and offers multilingual support in Estonian, Russian and English. Therefore, Estonia does meet the Istanbul Convention standards for the provision of a national women's helpline. The helpline is funded by the state and provides support to all survivors of violence including domestic violence, physical violence, sexualised violence, psychological violence, human trafficking and can also support survivors of cyberviolence. In 2024, the helpline received 5,647 calls from 1,299 callers, 58% of whom were from women. The most common forms of violence reported to the state-wide helpline were domestic violence, psychological violence, and violence directly or indirectly affecting children.

There are other helplines supporting survivors of violence against women in Estonia, including 16 regional helplines run by women's support centres and operating 24/7.³⁰ In addition, the organisation Lifeline (*NPO Eluliin*) runs a specific helpline for survivors of human trafficking and sexual exploitation, available during limited hours.³¹ Lifeline also runs a helpline providing emotional support to anyone in a crisis situation, including for survivors of violence, in English (tel.: +372 6558088) and in Russian (tel.: +372 6555688), available Monday to Friday from 19:00–07:00.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	19
Total number of all shelters accessible to women	19
Existing number of beds	170
24/7 access	Some

There are 19 women-only shelters in Estonia with approximately 170 beds. One new women-only shelter opened since 2022. The number of beds is approximate, as there is no clear requirement from the state regarding bed spaces needed. There are roughly five bed spaces in shelters in smaller counties and around 10 bed spaces in shelters in bigger counties. The current number of beds in women-only shelters meets the country's needs, and there is in fact an underusage of beds as in 2024, only around 7% of women who came to a women's support centre needed accommodation. Most women-only shelters offer 24/7 access, and the intake of survivors can vary by shelter. In the case of acute need, crisis support and emergency shelter accommodation will be provided. Women are not expected to pay for their accommodation in a women-only shelter, and women can stay up to six months. However, in 2024, the average stay in a women-only shelter was 30 nights for women and 38 nights for women with children. The types of in-house services most often provided by

shelters include casework, counselling and psychological support, referrals and collaboration with other services, practical advice, and legal advice. Usually, the support is tailored to the specific needs of the women and children, and some shelters provide other services such as group work and empowerment workshops. There are also options for referral to other services for longer-term support, in case a women's support centres does not have enough resources.

Most women-only shelters can accommodate women with uncertain residence permits/undocumented women, refugee women, older women (65 years and above), and women with children with disabilities. Some shelters can be accessed by and are accessible to homeless women, women with physical disabilities or sensory disabilities, women with substance abuse issues, transgender women, and women with older sons (14 years and above). However, no women-only shelters are equipped to support women with cognitive disabilities or unaccompanied girls under the age of 18. There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Estonia. As all shelters are located in privately rented spaces, it is not possible to renovate these to meet the needs of survivors with disabilities. As an alternative, women's support centres can visit survivors in their home to provide counselling and support through multistakeholder cooperation.

Funding for women-only shelters comes from the state as well as donations, although state funding remains low and only covers basic support including rent for shelters and counselling centres, crisis and social counsellors' salaries, and any necessary or critical supplies for women and children who stay in shelters. In cases where survivors need more than basis support, women-only shelters must seek additional funding although there is often limited capacity to do so. Twelve women-only shelters are run by NGOs, four by women's NGOs with a gender-specific and feminist approach, and three by the state (two in Tallinn and one in Pärnu). Women-only shelters are located in all regions of Estonia. The main reasons why shelters had to decline referrals were no space/capacity to support the survivor, the survivor being ineligible for support, and the shelter not being adequately equipped for the needs of the women/children, such as women with psychological/mental health issues (as there is a lack of specialists who can be present in the shelter 24/7). However, inability to accommodate women and children in need is extremely rare, as shelters are usually empty. The most common forms of violence reported by survivors in 2024 were psychological violence, economic violence, and physical violence. In 2024, 243 survivors were accommodated in women-only shelters, 103 women and 113 children. Although women-only shelters see fluctuation in numbers year by year,

30 Information on the different crisis helplines run by women's support centres available here: <https://www.palunabi.ee/en/womens-support-centres>.

31 Information on the different operating hours and phone numbers available here: <https://eluliin.ee/inimkaubandusohvrite-ja-prostitutsiooni-kaasatute-noustamine>.

women generally are less in need of shelter accommodation since 2022 due to improved support from their social network. There are no other shelters in Estonia which can provide support to women survivors of violence.

❖ 3. WOMEN'S CENTRES

Total number of centres	19
Entities running	NGOs (12 centres)
women's centres	Women's NGOs (4 centres)
	State (3 centres)

There are 19 women's support centres in Estonia only for women survivors of violence and their children. All women's centres provide referrals and collaboration with other services as well as residential support services. Seventeen centres provide counselling and psychological support as well as legal advice. Some women's centres offer financial and social welfare support or advocacy, housing advice, and representation at court/police/social services. Some women's centres in Estonia are able to base their services on the individual needs of survivors and can also provide further support such as financial literacy, employment practice interviews, and CV writing skills, among others. This however depends on the capacity and resources of the women's centre.

All women's centres are equipped to support older women (65 years and above), and some can support women with uncertain residence permits/undocumented women and refugee women. Some women's centres can be accessed by and are accessible to women with physical, cognitive or sensory disabilities, transgender women, and unaccompanied girls under the age of 18. In terms of specialist support, all women's centres can provide specialist support to black and minority ethnic women as well as women with uncertain residence permits/undocumented women and older women. Two centres provide specialist support to women with disabilities, transgender women and non-binary people, and one centre provides specialist support to survivors of trafficking. Some women's centres also provide specialist support to survivors of sexualised violence, including specialist sexualised violence advocacy series.

Each county in Estonia has a women's support centre, which is almost always connected to a women-only shelter. The Social Insurance Board Victim's Support Services (*Sotsiaalkindlustusameti Ohvriabi*) oversees services provided by NGO's who have won the contract. Women's centres are financed by the state as well as through donations. Twelve centres are run by NGOs, four by women's NGOs with a gender-specific and feminist approach, and three by the state (two in Tallinn and one in Pärnu). In 2024, a total of 1,893 women were supported by women's centres.

🚫 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for survivors of sexualised violence in Estonia, including five rape crisis centres located in regional hospitals. These centres provide mainly specialist forensic and medical care such as examinations, collecting evidence and injury notes, and storing data and information. They can also provide crisis counselling if needed, but for further support, they would refer the survivor to a women's support centre. One rape crisis centre opened in 2024 in Saaremaa. Continuous psychological care and legal advice is provided through referrals by the Social Insurance Board Victims Support Unit or the local women's support centre. Referrals depend on the capacity and availability of regional women's support centres. Funding for rape crisis centres comes from the state as well as donations and are available to all survivors of rape, including women. These centres serve all counties and four are located in each geographical point of the country (North-East and South-West) as well as one on the biggest island (Saaremaa). The reality is that women in peripheral areas do not have an easy access to rape crisis centres and may prefer not to go to such centres.

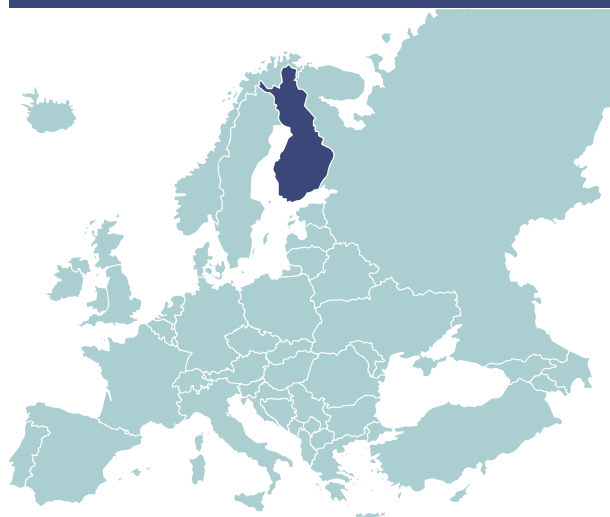
👤 5. PRIMARY PREVENTION SERVICES

Public authorities in Estonia do not collect data on primary prevention activities, although provisions on primary prevention are included in the Violence Prevention Agreement 2021–2025³² as well as in the Action Plan on the Prevention of Domestic Violence for 2024–2027 signed by four ministries: Social, Internal, Justice, and Education.³³ Currently, there are discussions surrounding the development of the agreement for the next period, 2026–2031. Understanding of violence prevention is based on the World Health Organisation's recommendations on a public health model, which emphasises the need to address the root causes of violence rather than just the symptoms. There is, however, no specific budget allocated to primary prevention activities. Available activities include feminist self-defence training, provided by the NGO *Kaidsõq*, and awareness-raising campaigns or programmes conducted in the frame of international project funding or when private businesses want to support such activities. There is also one NGO working on education and public advocacy, *Feministeerium*, and some women's support centres also provide training to specialists in the field of violence against women and girls and conduct awareness-raising campaigns, based on available project funding.

32 Available here: <https://www.justdigi.ee/sites/default/files/documents/2021-11/V%C3%A4givallaennetuse%20kokkulepe.pdf>

33 Available here: <https://www.justdigi.ee/uudised/neli-ministrit-allkirjastasid-lahisuhtevagivalla-ennetamise-kokkuleppe-koos-uue>

FINLAND



GENERAL COUNTRY INFORMATION

Population	5,603,851
Female population	2,829,953
Member of Council of Europe (year)	1989
Member of European Union (year)	1995
Member of United Nations (year)	1955
CEDAW ratified (year)	1986
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2015

SUMMARY

There is one national women's helpline in Finland that is free of charge, operates 24/7, and offers multilingual support. Additionally, there are other helplines supporting women survivors of different forms of violence. There are two women-only shelters in Finland with 23 rooms, as well as 26 other shelters accessible to women with a total of 228 beds. There are 28 women's centres in Finland, as well as one rape crisis centre and 25 sexual violence referral centres, called SERI Support Centres, for survivors of sexualised violence. Public authorities in Finland do not consistently collect data on primary prevention activities and there is no specific budget allocated to such activities. Provisions on primary prevention are, however, included in state Action Plans. The Finnish state does not collect data on women's specialist services as public authorities do not recognise women's specialist services as an entity. Most of the women's specialist services are run by NGOs and there is no data collection on the services provided by these NGOs. Although public authorities do collect numerical data on shelters run by NGOs, this includes all shelters in Finland, of which only two are gender specific.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	15,167 calls

There is one national women's helpline in Finland called Zeroline (*Nollalinja*, tel.: +358 80005005), coordinated by the National Institute for Health and Welfare and serviced by the NGO *Setlementti Tampere*. The helpline is free of charge, available 24/7, and offers multilingual support in Finnish, Swedish, English, Arabic, Dari, Farsi, Somali, Sorani, Spanish, Thai, Ukrainian, and Russian. In addition, survivors can also get in touch with a crisis worker via the *Nollalinja* chat service. The chat is open Monday to Wednesday 9:00–15:00, and Thursday to Friday 14:00–20:00 (excluding public holidays). The *Nollalinja* website offers information about domestic violence and shelters in Finnish (in plain language as well as sign language), Swedish, English, Arabic, Dari, Farsi, Sami (North Sami, Inari Sami, Skolt Sami), Somali, Sorani, Spanish, Thai, Ukrainian, and Russian. The helpline offers support to all survivors of domestic violence in close relationships and for women specifically in relation to violence or threat of violence in and also outside of close relationships. Finland, therefore, does meet the Istanbul Convention minimum standards for the provisions of a national women's helpline. *Nollalinja* is a special government service funded by the Finnish state through victim surcharges paid by offenders in criminal cases. In 2024, the helpline received 15,167 calls, of which 9,878 were answered, as well as 1,076 chat conversations. The most common forms of violence reported by callers in 2024 were psychological violence (76,7%), physical violence (39,5%), threat of violence (33,5%), and coercive control (20,5%).

There are other national helplines supporting survivors of violence in Finland. The Women's Line (*Naisten Linja*, tel.: +358 80002400) is run by an NGO with the same name and provides support to women survivors of all forms of violence. It is free of charge and offers multilingual support in Finnish, Swedish and English. In 2024, *Naisten Linja* received 4,053 calls, of which 1,181 were answered, as well as 324 chat conversations. The Crisis Center Monika's Helpline (*Kriisikeskus Monikan auttava puhelin*, tel.: +358 80005058) is run by Monika's Women Association (*Monika-naiset liitto ry*) and provides support to immigrant women on all forms of violence or threat of violence. The helpline offers multilingual support in Arabic, Dari, English, Farsi, Persian, French, Finnish, Belarusian and Russian. *Suvanto Helpline*, run by *Suvanto Association* (*Suvanto, turvallisen vanhuuden puolesta ry*, tel.: +358 80006766), provides support to survivors of elder abuse and is specialised in supporting elderly women. Funding for this helpline comes from the Centre for Social Welfare

and Health Organisations (STEA). There is also a national helpline for survivors of human trafficking (*Ihmiskaupan uhrien auttamisjärjestelmä*, tel.: +358 0295463177) run by the Assistance System for Victims of Human Trafficking and funded by the Finnish government.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	2
Total number of all shelters accessible to women	28
Existing number of beds	228
24/7 access	All

There are two women-only shelters in Finland which are accessible 24/7 with 23 spaces that can accommodate individuals or family units. In autumn 2025, a women-only unit with six places was also established within the shelter in Oulu (which is not a women-only shelter). One of the women-only shelters is the Shelter Mona (*Turvakoti Mona*) run by the Monika's Women Association, which provides specialised support to migrant women survivors of violence. Women are not expected to pay for their accommodation in the shelters and receive free meals as well as basic necessities such as bed linen and some hygiene products, if needed. The types of in-house services most often provided by the women-only shelters are counselling/psychological support, referrals/collaboration with other services, and practical advice.

The women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, homeless women, women with physical, cognitive, or sensory disabilities, older women (65 years and above), transgender women, and women with children with disabilities. Women-only shelters can accommodate women affected by substance use, provided that no alcohol or drug use occurs during their stay. However, they are not equipped to support unaccompanied girls under the age of 18. The Shelter Mona provides specialised support to women survivors (at risk) of honour-based violence, forced marriage and trafficking. In 2025, three places will be established in Tampere specifically for women with active substance abuse issues. The places will be in addiction treatment units, and shelter workers will work in the substance abuse unit with a maximum of three women.

The women-only shelters are funded by the state, located in Tampere and the capital area, and run by women's NGOs with a gender-specific and feminist approach. The main reason why the women-only shelters had to decline referrals was no space or capacity to support the survivor. Sometimes special needs cannot be met as the shelters have a limited number of, for example, wheelchair accessible rooms. All survivors accommodated in the shelters have experienced domestic violence and the most common forms of violence reported by survivors in 2024 were

physical, psychological, and economic violence. In 2024, 318 women and children were accommodated in the Shelter Mona.

In addition, there are 26 other shelters accessible to women in Finland with a total of 228 beds³⁴ for families and individuals. These general shelters are for all survivors including women, men, non-binary people, transgender women and transgender men. Although these shelters are open to all, they are predominantly accessed by women and apply a gender-sensitive approach. They are all accessible 24/7 and are run by different NGOs, predominantly by the Federation of Mother and Child Homes and Shelters (*Ensi- ja Turvakotien Liitto*). Shelters are coordinated and funded by the National Institute for Health and Welfare, an expert agency working under the Ministry of Social Affairs and Health. All shelters in Finland are free of charge and state-funded since 2015, when the Istanbul Convention was ratified. In general, women can stay in the shelters for a maximum of three months, and the aim is for survivors to stay for a short period until safe accommodation is funded. The average length of stay in 2024 was 18 days. Eighty percent of shelter stays lasted less than one month. Sixteen percent of the stays lasted more than one month but less than two months. A very small portion, only 4% of the shelter stays, lasted more than two months. Shelters are located across the country, but since Finland is large with a small and scattered population, providing services, including specialised and especially gender-specific ones, remains challenging. General shelters are located in rural areas too, but the distances between shelters can be very far, especially in Northern Finland, where the population is extremely scattered.

3. WOMEN'S CENTRES

Total number of centres	28
Entities running women's centres	Women's NGOs (two centres) Other NGOs (26 centres)

There are 28 women's centres in Finland, three of which are women-only and 25 centres are for all survivors of violence. All women's centres provide counselling/psychological advice, financial and social welfare support/advocacy, housing advice, and collaboration with other services. Some centres collaborate with legal counsellors to offer legal support and some also provide representation at court/police/social services, although separate support services also exist for crime victims in criminal investigations and court processes. At least two women's centres provide residential support services. Finally, women's centres also provide specialist support for survivors of sexualised violence including specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice. The Federation of Mother and Child Homes and Shelters (*Ensi- ja Turvakotien Liitto*) has 22 daytime centres for survivors

³⁴ This is the total number of beds accessible to women in all shelters. The 23 spaces in the two women-only shelters are also included in this figure.

of domestic violence and other forms of violence against women. Daytime centres aim to help survivors break free from violence and provide non-residential support services such as information and advice, counselling, advocacy, practical support, empowering support, and legal advice. All women's centres can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive, or sensory disabilities, older women (65 years and above), and transgender women. Some women's centres are equipped to support unaccompanied girls under the age of 18. All women's centres can also support survivors or those at risk of forced marriage, honour-based violence, female genital mutilation, trafficking in persons, and black and minority ethnic women. However, only one women's centre provides specialised support to these groups of women, Crisis Center Monika (*Kriisikeskus Monika*). There are other specialised services supporting particular groups, but these are not necessarily focusing on violence against women. In addition to the 28 women's centres, there are also nine gender-specific services for minors and young adults, so-called "Girls' houses".

Two women's centres are run by women's NGOs and the rest by other NGOs. Women's centres are funded by the state and through donations as well as international funding. Women's centres are located in most regions of Finland, although areas with dispersed settlements may lack service providers close by. However, services are offered by women's centres via remote access (such as phone or video calls). In 2024, 6,749 women and 2,059 underage girls were supported by women's centres in Finland.³⁵

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Finland, there is one rape crisis centre and 25 sexual violence referral centres (SVRCs), called SERI Support Centres which also act as rape crisis centres. The SVRCs are part of a national co-ordination structure and provide support to anyone over 16 years old who have experienced rape in the preceding month. The one and only rape crisis centre is run by an NGO, *Tukinainen ry*, and offers support to all survivors of sexualised violence, regardless of when the violence occurred. The rape crisis centre also runs a specialised helpline (*Raiskauskriisikeskus Tukinainen*, tel.: +358 080097899) as well as a legal helpline staffed by lawyers (tel.: +358 080097895). The rape crisis centre offers crisis and legal support, while SERI support centres provide forensic medical examinations, medical care, trauma support, psychological counselling, and therapy. SERI support centres are located in local and university hospitals and all but one operate their own 24/7 helpline. Specialised psychological care and legal advice is provided; however, the state co-ordinated services offer them on a limited basis. The NGO-run rape crisis centre can only offer these services when funding allows. Services for

survivors of sexualised violence are present in most regions in Finland. More SERI support centres have been established in recent years, some of which are also located in smaller towns. These services are funded by the state and through donations.

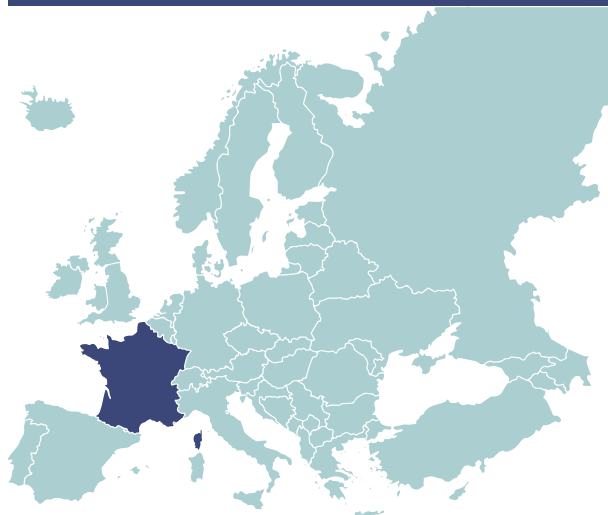
There are other organisations supporting survivors of sexualised violence in Finland. This includes *Nuorten exit*, which seeks to prevent sexualised violence and offers free support to all aged 13–29 experiencing sexualised violence and harassment, *Pro-tukipiste*, which works against violence in the sex industry and provides help and support to survivors of violence and human trafficking in the sex industry (located in Tampere, Turku and Helsinki), and *Delf-ins*, which offers support to adults who experienced sexual abuse as children. All of these services are run by NGOs.

5. PRIMARY PREVENTION SERVICES

Public authorities in Finland do not consistently collect data on primary prevention activities. Finland has an Action Plan on the Istanbul Convention for the period 2022–2025, which does include specific provisions on primary prevention. The measures included in the Action Plan emphasise the gendered nature of violence and highlight the specificity of violence against women. These complement, where appropriate, other ongoing and planned violence prevention work, the activities of which have been included in other national programmes. There is no specific budget allocated to primary prevention activities in Finland. Some primary prevention measures are budgeted, although this depends on the interests of the government and other national decision-makers. Available primary prevention activities in Finland include empowerment activities for girls and women, primary prevention activities specifically targeting men and boys, school-based primary prevention programmes, training for primary prevention practitioners, and awareness-raising campaigns or programmes. These activities are evaluated within the state's Action Plan. The focus of prevention work is related to general awareness-raising and training at different educational levels, including professionals working in those fields. It should be noted that prevention work in Finland is quite general and not gender specific.

³⁵ These figures only include data from 22 out of 28 women's centres.

FRANCE



GENERAL COUNTRY INFORMATION

Population	68,467,362
Female population	35,271,283
Member of Council of Europe (year)	1949
Member of European Union (year)	1957
Member of United Nations (year)	1945
CEDAW ratified (year)	1983
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2014

SUMMARY

There is one national women's helpline operating free of charge, available 24/7 and offering multilingual support, as well as other specialised helplines. There are 52 women-only shelters and approximately 200 other shelters, offering a total of 11,000 bed spaces. There are around 260 women's centres in France, as well as different support services for survivors of sexualised violence. Measures related to primary prevention are included in official documents such as the National Action Plan, but no dedicated budget is allocated to such activities, and there is no unified national data collection system for primary prevention activities. The State collects data on women's specialist services, and this information is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/ callers in 2024	152,220 calls 113,731 callers

There is one national helpline for women, called Violence against Women Info (*Violence Femmes Info*, tel.: +33 3913), which is run by the National Network on Women Solidarity (FNSF - *Fédération Nationale Solidarité Femmes*). The helpline is available 24/7, free of charge, and offers multilingual support in over 200 languages, thanks to an interpreting service. Therefore, it meets the standards of the Istanbul Convention. The helpline provides support for survivors of violence, including domestic violence, sexualised violence, female genital mutilation, forced marriage, sexist and sexual harassment in the workplace, and for survivors of sexual exploitation. The funding for the helpline comes from state funding, as well as donations. In 2024, the helpline received 152,220 calls from 113,731 callers. The most common forms of violence reported by callers were domestic violence, sexualised violence, and sexual harassment in the workplace.

In addition, other national helplines in France offer specialist support for survivors of different forms of violence, such as in forced marriage cases (*SOS Mariages Forcés*, tel.: +33 130310505), and sexual harassment at work (*Violence Faites aux Femmes au Travail – AVFT*, tel.: +33 145842424). These helplines are free of charge, but they are not available 24/7, nor do they offer multilingual support. Other specialised helplines include the Family Planning helpline for questions about contraception, abortion, sexually transmitted infections, and sexualised violence (*Planning Familial*, tel.: +33 800081111), the helpline for women with disabilities (*Femmes pour le Dire Femmes pour Agir*, tel.: +33 140470606), and the National Foundation for Women's and Family Rights Information Centres (*Fondation Nationale des Centres d'Information sur les Droits des Femmes et de la Famille – FNCIDFF*, tel.: +33 142171200).

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	52
Total number of all shelters accessible to women	250
Existing number of beds	11,000
24/7 access	Most

Fifty-two associations affiliated with the FNSF Network run one or more women-only shelters in France and provide more than 5,000 bed spaces. In total, there are 250 shelters in the country offering approximately 11,000 bed spaces, but details about other shelters accessible to women are not available. Most of the shelters offer 24/7 access, but this varies greatly across regions, and there is no centralised approach to help women find accommodation. Women-only shelters in France are financed by state funding and donations, and women, depending on their financial circumstances, are expected to pay for their accommodation. Survivors can normally stay in the shelters for three to six months. The women-only shelters provide casework, counselling, referrals, and practical as well as legal advice.

All women-only shelters in France can accommodate homeless women, while most shelters are equipped to support women with uncertain residence permits/undocumented women, refugee women, and older women. Some shelters can accommodate and are accessible to women with physical, cognitive and sensory disabilities, women with substance abuse issues, transgender women, women with older sons, women with children with disabilities and unaccompanied girls under 18. Additionally, there is a shelter specialised in supporting refugee women who are survivors of sexual exploitation.

In France, different types of organisations run women's shelters. There are organisations with a gender/feminist approach (*FNSF, La Maison des Femmes*), women's NGOs (*Centres for Information on Women's and Families' Rights – Centres d'Information sur les Droits des Femmes et des Familles - CIDFF, SOS Femmes*), faith-based organisations (*Secours Catholique*) and other associations and local services (*Amicale, Accommodation and Social Reintegration Centre - Centre d'hébergement et de réinsertion sociale – CHRS, Integrated Reception and Orientation Services – Service Intégré d'Accueil et d'Orientation – SIAO*). Women-only shelters exist in most regions of France. The main reasons why women-only shelters decline referrals are that no space is available, the survivors are ineligible for support, and/or the shelters are not adequately equipped for the needs of the women and their children. The three most common forms of violence reported by clients to the women-only shelters in 2024 were domestic violence, sexualised violence, and intra-family violence. Within the FNSF Network, between 7,000 and 8,000 women were supported by shelters in 2024.

There are other shelters in France which can provide support to survivors of different forms of gender-based violence, including women, men, non-binary people, transgender women and men, and other groups of survivors. Some of these shelters offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	260
Entities running women's centres	Women's NGOs Other NGOs

There are more than 260 women's centres in France, among which 98 are part of the National Federation of CIDFF (*Fédération nationale des CIDFF – FNCIDFF*), 79 are affiliated with FNSF, and 80 are from Planning Familial. Women's centres provide counselling, psychological support, legal advice, financial and social welfare support, housing advice, referrals, representation at court/police/social services, as well as residential support services. Additionally, women's centres can also provide specialist support for survivors of sexualised violence, including specialist forensic and medical care, needs assessment, specialist psychological care, advocacy services, community awareness and education for prevention, and legal advice.

All women's centres are equipped to support women with uncertain residence permits/undocumented women, refugee women, older women and unaccompanied girls under the age of 18. Most centres are accessible to women with physical, cognitive and sensory disabilities, as well as transgender women. Additionally, there are women's centres specialised in supporting survivors and those at risk of specific forms of violence, such as forced marriage, honour-based violence, female genital mutilation, and trafficking in persons, as well as centres specialised in supporting women with physical, cognitive, and sensory disabilities.

The state, religious institutions and donations are the major sources of funding for women's centres, which are located in most regions.

👤 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In France, support to survivors of sexualised violence is provided by rape crisis centres (between 35 and 50 centres available), sexual violence referral centres (mostly services offered in hospitals with specialised units or consultation services), crisis/medical services (between 90 and 110 services available) and specialised helplines. These support services offer specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention, and legal advice. The services are provided jointly by Women's Houses (*Maisons des Femmes*), university hospitals, CIDFF, and associations such as the Feminist Collective Against Rape (*Collectif Féministe Contre le Viol – CFCV*). Funding for sexualised violence services is provided by the state and donations. Such services are available in most regions, and support different groups: women, young people under the age of 18, as well as transgender women, non-binary individuals, women with physical, cognitive, sensory disabilities, older women, and women with precarious legal status (undocumented migrants, asylum seekers).

Additionally, there is an online portal to support survivors of sexualised violence (*Portail de signalement des violences sexuelles et sexistes*, via the website service-public.fr). It is free of charge and available 24/7 and consists of a chat with police officers trained to support and counsel survivors of sexualised violence. There is a helpline supporting survivors of sexualised violence in the country (*Viols Femmes Informations*, tel.: +33 800059595).

👤 5. PRIMARY PREVENTION SERVICES

There is no unified national data collection system on primary prevention activities in France. However, some local authorities (e.g., departmental councils, regional health agencies, education authorities) and ministries (Education, Gender Equality, Justice) may occasionally fund or assess initiatives in this field. Associations often evaluate their field activities for reporting purposes or funding renewal.

The 2023–2027 National Action Plan to combat violence against women includes measures related to awareness, education on equality, and prevention. While the concept of primary prevention is implicitly present in public policies, there is no standardised or explicitly formulated definition in official documents such as the National Action Plan. There is also no dedicated budget line for primary prevention in public documents and policies. Funding comes from various mechanisms (regional health agencies, local authorities, projects from the Ministry of Equality, etc.). Budgets often relate to awareness-raising, equality education and support to associations, but without precise allocation to primary prevention. The primary prevention activities conducted in the country include feminist self-defence trainings, empowerment activities for girls and women, school-based programmes, training for primary prevention practitioners, bystander intervention training, self-care activities and awareness-raising campaigns or programmes. Some evaluations are occasionally conducted by state services or local authorities, often as part of funded projects. Associations like *En Avant Toute(s)* or CIDFF incorporate evaluation tools into their programmes. However, there is no national framework for systematic evaluation.

GEORGIA



GENERAL COUNTRY INFORMATION

Population	3,694,608
Female population	1,920,139
Member of Council of Europe (year)	1999
Member of European Union (year)	No
Member of United Nations (year)	1992
CEDAW ratified (year)	1994
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2015
Istanbul Convention ratified (year)	2017

SUMMARY

There is one national women's helpline in Georgia which is free of charge, operates 24/7 and provides multilingual support. There are six women-only shelters in Georgia, three of which are run by the state and three by women's NGOs. There are also 10 women's crisis centres which are available to all survivors of violence. There are no specialised services for survivors of sexualised violence in the country, however, state-run crisis centres and shelters support survivors of sexualised violence. There is a National Action Plan on violence against women and girls in Georgia, which includes specific provisions on the primary prevention on violence against women, however, there is no specific budget allocated to such activities. The state does collect data on women's specialist services, including primary prevention activities, and this data is publicly available. The State Welfare Agency, responsible for overseeing shelters and crisis centres, compiles statistics related to the usage of these services. Additionally, separate statistics are gathered by the Ministry of Internal Affairs (some information is publicly available), the Prosecutor's Office, and the Social Agency.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	4,762 calls

There is one national women's helpline in Georgia called the LEPL Agency for State Care and Assistance for the (statutory) Victims of Human Trafficking (სახელმწიფო ბრუნვისა და ტრეფიკინგის მსხვერპლთა, დახმარებულთა დახმარების სააგენტო, tel.: 116 006). The helpline is free of charge, operates 24/7 and offers multilingual support in Georgian, English, Ukrainian, Russian, Turkish, Azerbaijani, Armenian, Persian and Arabic. Georgia, therefore, does meet the Istanbul Convention minimum standards for the provision of a national women's helpline. The helpline is funded by the state as well as through international funding. In 2024, the helpline received 4,762 calls. The most common forms of violence reported by callers were domestic, physical and psychological violence. There are other helplines supporting women survivors of violence in Georgia, including five helplines run by local women's NGOs.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	6
Total number of all shelters accessible to women	6
Existing number of beds	N/A
24/7 access	All

There are six women-only shelters in Georgia, all of which offer 24/7 access, but the number of beds is not available. Since 2022, three shelters had to close. Women are not expected to pay for their accommodation during their stay in a women-only shelter. The average length of stay in the women-only shelters is three to six months, although women can stay more than one year if needed. The maximum length of stay in a state-run shelter is nine months, while accommodation at the shelters run by women NGOs is more flexible. The types of in-house services most often provided by women-only shelters are counselling/psychological support, legal advice and practical advice.

Some women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, homeless women, women with physical, cognitive or sensory disabilities, women with substance abuse issues, older women (65 years and above), transgender women, women with older sons (14 years and above), women with children with disabilities and unaccompanied girls under the age of 18. There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Georgia.

Funding for women-only shelters comes from the state as well as through international funding. Three women-only shelters are run by the state and three are run by NGOs. The shelters are located in major cities in Georgia. The main reason why women-only shelters had to decline referrals was that they were inadequately equipped for the needs of the women/children. In 2024, the most common forms of violence reported by survivors to the women-only shelters were domestic, physical and psychological violence.

There are no other shelters in Georgia, besides the women-only shelters, that can provide support to survivors of violence.

3. WOMEN'S CENTRES

Total number of centres	10
Entities running women's centres	Women's NGO State

There are 10 women's crisis centres in Georgia for all survivors of violence, including women. All centres offer counselling/psychological support, legal advice, housing and employment advice, referrals/collaborations with other services, representation at court/police/social services and residential support services. All centres also provide specialised support for survivors of sexualised violence such as needs assessment, specialist psychological care and legal advice.

Some women centres can be accessed by and are accessible to refugee women, women with physical, cognitive, and sensory disabilities and older women (65 years and above). Additionally, some women's centres can support survivors (at risk) of forced marriage. There are no specialised women's centres supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence.

Women's centres are located only in major cities. Seven women's crisis centres are run by the state and three by women's NGOs. They are funded by the state as well as through donations and international funding.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

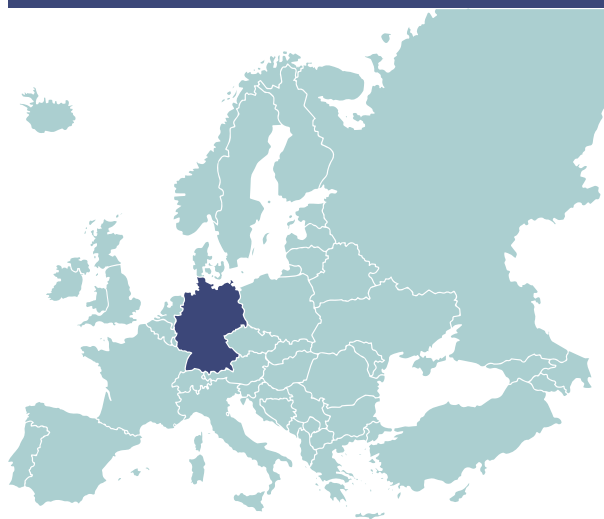
There are no specialised services for women survivors of sexualised violence in Georgia. However, legal counselling and survivor needs assessments are provided by all state-run women's crisis centres and shelters.

5. PRIMARY PREVENTION SERVICES

Public authorities in Georgia collect data on primary prevention activities. There is a National Action Plan (NAP) on violence against women and girls, the National Action Plan on Combatting Violence Against Women and Domestic Violence and Measures to be implemented for the Protection

of Victims 2022-2024, which includes a specific definition of primary prevention. The National Action Plan was developed by the Inter-Agency Commission on Gender Equality, Violence Against Women and Domestic Violence with UN Women technical and financial support within the framework of the "Ending Violence Against Women and Girls in Georgia (EVAWGG)" programme. The programme is funded by the European Union and implemented jointly by UN Women and the United Nations Population Fund (UNFPA). Available primary prevention activities include awareness-raising campaigns and programmes. There is no specific budget allocated to primary prevention activities.

GERMANY



GENERAL COUNTRY INFORMATION

Population	83,456,045
Female population	42,294,114
Member of Council of Europe (year)	1850
Member of European Union (year)	1958
Member of United Nations (year)	1973
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2017

SUMMARY

There is one national women's helpline in Germany, which is free of charge, available 24/7, and offers multilingual support, and there are other regional helplines supporting survivors of violence against women. There are 380 women-only shelters in Germany with a total of 2,720 family places available. There are 526 women's centres, run mostly by women's NGOs and welfare organisations, and different support services for survivors of sexualised violence. Germany does have a definition of primary prevention; it has started collecting systematic data on it, but a budget is allocated mostly on a project basis. Since 2023, the German Institute for Human Rights (DIMR) has acted as the National Rapporteur Mechanism on gender-based violence, mandated to monitor the implementation of the Istanbul Convention. Its first monitoring report was published in 2024. *Frauenhauskoordination e.V. (FHK)* publishes annual shelter statistics (approx. 50% of shelters provide data). The National Helpline also publishes annual reports. Federal states additionally collect data linked to subsidies, but these are generally not public.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/ callers in 2024	61,200 calls/contacts

Germany has one state-wide women's helpline, the Violence against Women Helpline (*Hilfetelefon "Gewalt gegen Frauen"/Bundeshilfetelefon*, tel.: 116016), which is run by the Federal Office of Family Affairs and Civil Society Functions (*Bundesamt für Familie und zivilgesellschaftliche Aufgaben*). It operates free of charge, is available 24/7 and offers support in 18 languages (German, Italian, French, Portuguese, Spanish, Kurmanji, Polish, Serbian, English, Farsi/Dari, Romanian, Ukrainian, Vietnamese, Russian, Polish, Chinese, Arabic, and Bulgarian, as well as German sign language and Easy Language). Therefore, Germany does meet the Istanbul Convention standards for the provision of a national women's helpline. The helpline provides support in cases of domestic violence, including psychological, physical and/or sexualised violence, stalking, forced marriages, sexual harassment in work and public spaces, trafficking in persons, female genital mutilation, bullying, and femicides. It is state-funded. In 2024, it offered 61,200 counselling services (54,300 calls and 7,000 online counselling). The most common form of violence reported was domestic violence, which remained at a similarly high level (59%) as in previous years, followed by sexualised violence (13% of the calls) and psychological violence.

In addition to the national women's helpline, further services are available in Germany. The BIG Hotline, operated by BIG e.V. (*Berliner Initiative Gegen Gewalt an Frauen e.V.*, tel.: +49 306110300), is a regional hotline for Berlin available 24/7, providing multilingual support to women survivors and relatives of women survivors of violence. At the national level, the Sexual Abuse Helpline (*Hilfe-Telefon Sexueller Missbrauch*, tel.: +49 8002255530) is operated by N.I.N.A. e.V. and funded by the Independent Federal Commissioner Against Sexual Abuse of Children and Adolescents. It is available on Mondays, Wednesdays and Fridays from 09:00–14:00, and Tuesdays and Thursdays from 15:00–20:00, and works to improve the protection of children and young people from sexualised violence.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	380
Total number of all shelters accessible to women	380
Existing number of beds	2720 ³⁶
24/7 access	Most

Germany has approximately 380 women's shelters with an estimated 2,720 family places, which corresponds to approximately 6,800 beds. No federal state³⁷ meets the number of family places recommended by the National Rapporteur Mechanism on gender-based violence, which corresponds to one family place per 10,000 inhabitants; one family place corresponds to one bed for the woman and 1.54 places for children. The website www.frauenhaus-suche.de indicates free capacities in shelters on a nationwide level. The website is run by ZIF, the coordination of the Autonomous women's shelters. Most shelters are open 24/7; according to the first periodic report for the implementation of the Istanbul Convention in Germany published by the German Institute for Human Rights (DIMR), 297 shelters declared round-the-clock accessibility. In several cities (e.g. Hamburg, Heidelberg, Hannover), 24/7 emergency shelters operate for short-term stays, transferring women later to permanent facilities. Funding of shelters remains highly fragmented and insufficient. Only around one-third of shelters receive stable institutional or mixed funding. The majority rely on a daily rate system under which women must cover accommodation and support costs either personally or through welfare assistance. This excludes certain groups—such as students, pensioners, EU migrants, and undocumented women—who may not qualify for public benefits. Depending on the shelter, women may bear none, part, or all of the costs: in 11% of shelters, costs always apply, in 60.8% only under certain conditions, and in 26.8% women must contribute directly.

The length of stay varies considerably. Autonomous shelters generally follow a principle of self-determination, giving women the space and necessary time to take back control of their lives and recover from violence. However, funding arrangements and bed shortages often impose limitations, with some shelters restricting stays to one, three, or six months, while others allow up to one year or longer. Authorities have at times pressured shelters to shorten stays in order to increase turnover and serve more women. Services offered by shelters are broad and typically include casework, counselling and psychological support, legal and practical advice. Collaboration with external services is often required, especially for women with specific needs (e.g. disabilities), to secure adequate shelter placements.

All women-only shelters can accommodate older women, and most can accommodate women with children with disabilities. Some can be accessed by and are accessible to women with uncertain residence permits and undocumented women, refugee women, homeless women, women with physical, cognitive and/or sensory disabilities, women with substance abuse issues, transgender women and women with older sons. No women-only shelter is equipped to support unaccompanied girls under the age of 18.

³⁶ 2,720 is the number of family places. One family place in Germany corresponds to one bed for the woman and 1.54 beds for children. This is based on Germany's average birth rate. Data from WAVE Country Report 2023.

³⁷ Data from WAVE Country Report 2023.

Germany has several specialised women's shelters for specific groups. There are shelters specifically for survivors of trafficking, as well as girls' and young women's shelters (*Mädchenhäuser*) for survivors of sexualised violence within families or neighbourhoods. The National Network of Autonomous Girls' Shelters (*Bundesarbeitsgemeinschaft Autonome Mädchenhäuser*) currently operates girls' shelters in 14 different cities. These facilities are explicitly feminist, anti-racist, and anti-patriarchal in orientation and are increasingly addressing questions of gender diversity, though approaches vary across locations. In addition, there are shelters for women affected by honour-based violence, two facilities that provide specialised support for women with substance abuse issues, as well as specialised shelters for survivors of forced marriage, refugee women, and migrant women.

Women-only shelters are partially funded by the state, including regional authorities, municipalities, and federal states, and by religious institutions and through donations. Around one-third of shelters receive institutional funding and are therefore not fully dependent on daily-rate financing. In these cases, however, women still have to pay rent and apply for social welfare to cover living costs. About two-thirds of shelters are financed through a daily-rate system. In 2024, a draft law on financing the anti-violence structure was presented to the public, stating that there should be no costs for survivors of violence. Implementation is still pending, and the lack of available shelter places remains a major problem. Approximately 250 shelters are run by women's NGOs with a feminist or gender-specific approach, around 90 shelters are managed by faith-based organisations and 50 by welfare organisations. Women's shelters exist in all regions, but coverage is not sufficient, and the situation is particularly difficult in rural areas.

Main reasons for declining referrals were the lack of space or capacity to accommodate survivors with children, the women-only shelters not being adequately equipped for the needs of the women and/or children or no available space or capacity to support the survivor. The most common forms of violence reported by survivors were domestic, sexualised and physical violence. Survivors also frequently reported psychological violence, which remains a major issue as it is often not legally recognised, and women and children affected by domestic violence often face structural and institutional violence too. According to the Women's Shelter Coordination (*Frauenhauskoordination, FHK*), in their 176 shelters, 6,264 women and 7,043 children were accommodated in 2023. Scaled up to all 380–400 shelters, this corresponds to approximately 14,238 women and 16,008 children nationwide.

In addition to women-only shelters, there are other facilities that can support survivors of violence against women. These include support homes and groups within the youth welfare system.

❖ 3. WOMEN'S CENTRES

Total number of centres	526
Entities running women's centres	Women's NGOs
	Welfare organisations
	Other

Germany has approximately 526 women's centres. The Federal Association of Women's Counselling Centres and Rape Crisis Centres (*Bundesverband Frauenberatungsstellen und Frauennotrufe, bff*) reported 227 women's centres in 2023, including specialist counselling centres, rape crisis centres, intervention centres, and centres for survivors of gender-based violence. Centres working on human trafficking are represented by the German NGO Network against Trafficking in Human Beings (*Bundesweiter Koordinierungskreis gegen Menschenhandel e.V., KOK*), numbering about 81, some of which also operate shelters. Other centres are connected with shelters and represented by the Central Information Office of Autonomous Women's Shelters (*Zentrale Informationsstelle Autonomer Frauenhäuser, ZIF*) or the Women's Shelter Coordination (*Frauenhauskoordination, FHK*).

Women's centres provide counselling and psychological support, legal advice, financial and social welfare support and advocacy, housing advice, employment support, referrals and collaboration with other services, representation before courts, police and social services, and residential support services. Key challenges faced by women's centres in providing support to survivors include limited access to housing, childcare, employment, and support in custody and economic matters. Women's centres also provide specialist support for survivors of sexualised violence, including specialist psychological care, specialist advocacy services, community awareness and education for prevention work, as well as legal advice.

Most women's centres are equipped to support women with uncertain residence permits, undocumented women, and refugee women. Some women's centres can be accessed by and are accessible to women with physical, cognitive and sensory disabilities, older women, transgender women and unaccompanied girls under the age of 18. There are 102 women's centres specialised in supporting survivors (at risk) of forced marriage. Additionally, there are women's centres specialised in supporting survivors (at risk) of honour-based violence, female genital mutilation, trafficking in persons, women with uncertain residence permits and undocumented women, refugee women and Black and Minority Ethnic Women. Moreover, there are women's centres specialised in supporting women survivors with physical, cognitive and/or sensory disabilities. Older women survivors can be supported by 257 women's centres, and girls under the age of 18 by 176 women's centres. There are 44 women's centres that provide support to LGBTQI+ people, including transgender women and non-binary people.

Around 227 women's centres are run by women's NGOs. In addition, 299 centres are members of the Women's Shelter Coordination (*Frauenhauskoordination, FHK*) but are operated by welfare organisations. Main funding sources for women's centres include state funding, donations, religious institutions, and international funding. Overall, funding for centres is insufficient, and at the end of 2024, financial cuts affected the National Coordination Group Against Human Trafficking – KOK and its support for refugee women, with further cuts announced for 2025. Women's centres are present in all regions of Germany, but the coverage is inadequate. There are significant gaps in rural areas, and access to counselling for women with disabilities is insufficient both in rural and urban areas. According to bff internal statistics, the number of women accessing their centres is increasing over time: 77,583 persons were counselled in 2020, 78,711 in 2021, and 85,170 in 2022. Counselling contacts increased from 242,670 in 2020 to 295,553 in 2022. About 80% of clients were directly affected by violence, while the remaining 20% were counselling professionals and relatives.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are specialised services for women and girls who have experienced sexualised violence in Germany, including rape crisis centres, sexual violence referral centres, and a specialised helpline, as well as crisis and medical services. Services provided in the country to survivors of sexualised violence include specialist forensic and medical care, psychological care, specialist advocacy, community awareness and prevention, and legal advice. Additionally, about half of women's centres provide counselling for sexualised violence too.

A programme is underway to implement sexual violence referral centres across federal states, coordinated by a rape crisis centre in Hessen. The Sexual Abuse Helpline (tel.: +49 8002255530), anonymous and free of charge, provides help and support specifically for children affected by sexual abuse, their family members, concerned persons from their social circles, and relevant professionals. The Federal Working Group of Feminist Organisations against Sexual Violence against Girls and Women (*Bundesarbeitsgemeinschaft Feministischer Organisationen gegen Sexuelle Gewalt an Mädchen und Frauen e.V., BAG FORSA*) represents 22 organisations nationwide. BAG FORSA is based on a feminist perspective, viewing sexualised violence as a societal problem requiring coordinated strategies at multiple levels. It supports feminist organisations both during their establishment and when addressing later challenges.

Funding for sexualised violence support services comes from state authorities and donations. Institutional funding is insufficient, and many centres, such as rape crisis centres and organisations like *Wildwasser*, rely heavily on donations to sustain their work. BAG FORSA cooperates with

the German Association for Prevention and Intervention (DGPI) and with BKSf (nationwide coordination of specialised centres, not limited to feminist centres). Services in Germany support women over 18, as well as young women and children under 18.

5. PRIMARY PREVENTION SERVICES

Public authorities collected for the first time data on primary prevention through the study "Analysis for the Needs of Prevention of Gender-Specific and Domestic Violence" (*Bedarfsanalyse zur Prävention von geschlechtsspezifischer Gewalt gegen Frauen und von häuslicher Gewalt*), conducted by the institute Sozialwissenschaftliches Forschungsinstitut zu Geschlechterfragen (SoFFI) at the International Centre for Socio-Legal Studies (SOCLES), in cooperation with the German Youth Institute (Deutsches Jugendinstitut e.V., DJI). The project, funded by the Federal Ministry for Education, Family Affairs, Senior Citizens, Women and Youth (Bundesministeriums für Bildung, Familie, Senioren, Frauen und Jugend, BMBFSFJ), ran from 2022 to May 2025.

Germany has a National Action Plan against Violence against Women (*Aktionsplan II der Bundesregierung zur Bekämpfung von Gewalt gegen Frauen*), which was issued in 2012 and includes a definition of primary prevention of violence against women and girls. A new National Action Plan against Violence against Women will be implemented for 2025-2029. A budget for primary prevention activities exists, but programmes are mainly project-oriented. For instance, in Hessen, three ongoing prevention programmes are *#coolermove*, *Alle gegen K.O.-Tropfen*, and *HeRoes* (2020–2024).

Primary prevention activities conducted in Germany include feminist self-defence trainings, empowerment activities for girls and women, primary prevention programmes targeting men and boys, school-based prevention programmes, training for prevention practitioners, bystander intervention training, and self-care activities. An evaluation of prevention activities was published in May 2025 within the study "Analysis for the Needs of Prevention of Gender-Specific and Domestic Violence". It analysed prevention programmes related to gender-specific and domestic violence, as well as the expectations of practitioners working in prevention. A wide variety of activities and programmes are organised by NGOs, including women's centres and shelters, which strengthen support structures for women and address risks of violence.

GREECE



GENERAL COUNTRY INFORMATION

Population	10,400,720
Female population	5,303,827
Member of Council of Europe (year)	1949
Member of European Union (year)	1981
Member of United Nations (year)	1945
CEDAW ratified (year)	1983
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2018

SUMMARY

There are two state-wide women's helplines in Greece, which operate free of charge, are available 24/7 and offer multilingual support. There are 20 state-run women-only shelters that offer 400 beds, and 44 state-run women's centres. Additionally, there are other women's shelters and women's centres that are run by NGOs. There are no specialised services for women and girls who have experienced sexualised violence. There is a National Action Plan for Gender Equality, which includes specific provisions on primary prevention. The state collects data on state-operated women's specialist services, which is publicly available.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024 –	SOS Helpline 15900: 6,488 calls (between January and September 2024)

There are two state-wide women's helplines in Greece, both of which are free of charge and available 24/7, meeting the standards of the Istanbul Convention. The main helpline is called SOS Helpline 15900 (*Γραμμή SOS*, tel.: +30 15900) and is run by the General Secretariat for Equality and Human Rights. This helpline is state-funded and offers support in English and Greek. In 2024, Arabic and Farsi were also available for two hours per week. Psychologists and social workers provide emergency support for urgent cases, as well as information and counselling to survivors of all forms of gender-based violence. The second state-wide helpline is run by the Union of Women Associations of Heraklion Prefecture (UWAH) and is called Line of Hope (*Γραμμή Ελπίδας*, tel.: +30 8011116000), is free of charge and available 24/7. It is operated by psychologists working in the Survivor Support Unit of UWAH. This helpline offers support in Greek and English to survivors of different forms of domestic violence, gender-based violence and intimate partner violence. This helpline is self-funded through donations. Available data³⁸ from the SOS Helpline 15900 for the period January to September 2024 reported 6,488 calls, of which 4,250 for "incidents of violence" and 1,339 for "useful information". Of the calls concerning incidents of violence, 71% were made by female beneficiaries and 29% by third parties. The most common type of violence reported by callers in 2024 was domestic violence (84%), with the most frequent forms being psychological (71%), verbal (60%), physical (45%), economic (8%) and sexualised (5%) violence.

Additionally, there is another helpline developed by A21 (tel.: +30 1109) which is equipped to support survivors of trafficking in persons, offers multilingual support in over 200 languages through tele-interpretation, and is available 24/7.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	20
Total number of all shelters accessible to women	N/A
Existing number of beds	400 ³⁹
24/7 access	None

In Greece, there are twenty state-run shelters, as well as shelters run by NGOs, including UWAH in Heraklion, and Action for Women and Caritas in Athens. Most of these shelters can accommodate around 20 women; therefore, the 20 state-run shelters offer approximately 400 beds, but no official data is available about the number of beds in other shelters. None of the state shelters offer immediate access and women are not expected to pay for their accommodation. The state funds only state-owned shelters via EU resources. The length of stay is three months, with a possibility to prolong it for three more months. The most common types of services offered are counselling and psychological support. Some NGOs also collaborate with lawyers and provide support regarding employment,

38 General Secretariat for Equality and Human Rights, Annual Report on Violence Against Women 2024, accessible at: <https://isotita.gr/wp-content/uploads/2024/11/5%CE%B7-etisia-ekthesi-gia-ti-via-kata-twn-gynaikwn.pdf>

39 Number of beds in the 20 state-run shelters.

housing and healthcare through social workers or by collaborating with other services and organisations. Additionally, women’s shelters in Greece offer casework and referrals and collaboration with other services.

Most women-only shelters are equipped to support women with physical disabilities. Some shelters can be accessed by and are accessible to women with uncertain residence permits and undocumented women, refugee women, homeless women, women with cognitive and/or sensory disabilities and unaccompanied girls under the age of 18, while women with substance abuse, psychiatric disorders and transgender women cannot be accommodated in such shelters. In Greece, there are also other shelters specialised in supporting specific groups: the NGO Action for Women focuses on providing support to refugee and migrant women, including transgender women; Ananda Marge Universal Relief Team Ladies (AMURTEL) is equipped to support pregnant women and is based in Athens; A21 runs a short-term shelter for all survivors of trafficking in persons and is based in Thessaloniki.

Funding for shelters stems mainly from the state in the case of state-owned shelters. The remaining, NGO-operated shelters are not funded by the state. Women-only shelters are located in most regions of Greece, with state-run shelters available in 18 different municipalities and NGO-run shelters mostly available in Athens and Heraklion. Reasons for declining referrals were not having the space and/or capacity to support the survivor, the survivor being ineligible for support (e.g., women having a male child over the age of 12 or women with a transmissible disease, substance addiction or psychiatric disorder), and the shelter not being adequately equipped for the needs of the women and children. The most common forms of violence reported were domestic, physical and sexualised violence. From January to September 2024, according to data from the General Secretariat of Equality and Human Rights, 491 persons (248 women and 243 children) were accommodated in state-run shelters.

❖ 3. WOMEN’S CENTRES

Total number of centres	44 ⁴⁰
Entities running women’s centres	State (44)
	Women’s NGOs

There are 44 state-run women’s centres in Greece supporting survivors of violence against women and other women’s centres run by NGOs, although no official data is available on their availability. Women’s centres provide counselling and psychological support, social welfare support, housing advice, employment counselling, legal counselling and referrals and collaboration with other services; some centres, in cooperation with Bar Associations, provide representation at court, while police and residential support services are provided only for women who need interpretation.

All women’s centres are equipped to support refugee women and older women. Some centres can be accessed by and are accessible to women with uncertain residence permits and undocumented women, women with physical, cognitive and/or sensory disabilities, as well as transgender women. Additionally, there are NGO-run centres specialised in supporting survivors (at risk) of trafficking in persons, women with uncertain residence permits/undocumented women, refugee women and Black and minority ethnic women. Moreover, there are NGO-run women’s centres specialised in supporting girls under the age of 18, transgender women and non-binary people.

Only state-operated women’s centres are state-funded through EU resources; the remaining funding stems from donations and international funding. Ten of the state-run women’s centres are located in the Attica Periphery, and the remaining 34 are in the remaining 12 regions of Greece. Between January and September 2024, 3,626 women survivors of violence received support through Greece’s network of state-run counselling centres.

🚒 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no specific services for women and girls who have experienced sexualised violence. Women’s centres can provide counselling to survivors of sexualised violence, but no specialist support.

👤 5. PRIMARY PREVENTION SERVICES

In Greece, primary prevention of violence against women and girls is addressed in the National Action Plan for Gender Equality 2021–2025, which includes gender-based violence prevention as one of its four priority axes. The plan foresees training of professionals and awareness-raising campaigns, including targeting men and boys, although implementation has not yet taken place. School-based programmes are not introduced by the Ministry of Education, and there is no official definition of primary prevention. Women’s centres and NGOs conduct primary prevention activities, but these are carried out among other duties, without systematic implementation, and rely mainly on EU funding rather than state resources.

The National Action Plan is financed through national and European sources. Data collection and evaluation are the responsibility of the General Secretariat for Equality and Human Rights, and Law No. 4604/2019 mandates gender impact assessments and the collection of gender-disaggregated data. However, the effectiveness of data collection efforts is limited by weak coordination between public authorities, academic institutions, and NGOs. Direct funding of evaluations by national or regional authorities is limited, with most evaluations conducted through EU programmes or NGO-led initiatives.

40 Number of state-run women’s centres. Data about NGO-run women’s centres is not available.

HUNGARY



GENERAL COUNTRY INFORMATION

Population	9,584,627
Female population	4,961,249
Member of Council of Europe (year)	1990
Member of European Union (year)	2004
Member of United Nations (year)	1955
CEDAW ratified (year)	1982
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2014
Istanbul Convention ratified (year)	No

SUMMARY

There are two national women's helplines in Hungary, both are free of charge, and offer multilingual support, but are not available 24/7. There are also other helplines that can provide support to survivors of violence against women. There are 63 shelters accessible to women survivors of violence, four of which are women-only, with a total capacity of 460 beds. There are no women's centres in Hungary, nor are there services for survivors of sexualised violence. Public authorities in Hungary do not collect data on primary prevention activities nor is there a specific budget allocated to such activities. The term women's specialist service does not exist as an official category in Hungary, therefore, is not used by the state or in official statistics. The state can provide data on request regarding services that are available to the survivors of violence which collect their own data. There are only a few women's rights NGOs providing women's specialist services, and those that do exist undertake their own data collection as well.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	No
Multilingual support	Yes
Total number of calls/ callers in 2024	25,152 initiated calls/attempts 3,036 callers ⁴¹

There are two national women's helplines in Hungary, both are free of charge and offer multilingual support, but do not operate 24/7. The first helpline is run by the NANE Women's Rights Association and is called NANE Helpline (*NANE Segélyvonal*, tel.: +36 80505101). The NANE Helpline accepts calls from survivors of all forms of violence against women, their relatives or friends, as well as professionals, and offers confidential emotional support, information, and referrals. It is staffed by trained volunteers and available via phone on weekdays, as well as through an anonymous online chat on selected days. This helpline provides multilingual support in Hungarian, English, Russian, Polish and German, depending on the language skills of the helpline operators. The vast majority of helpline operators speak English well enough to provide support in this language.

The second national women's helpline is called PATENT Legal Aid (*PATENT Jogsegély*, tel.: +36 80808081) and is run by the PATENT (People Opposing Patriarchy) Association. PATENT's legal team operates the helpline, complemented by legal aid via email and chat. In addition, PATENT offers individual and in-person consultations and legal representation in gender-based violence-related criminal proceedings, family law matters, and child custody cases. The helpline focuses on both interpersonal and structural forms of violence against women, offers free legal support to women affected by violence and reproductive rights violations. This helpline also offers multilingual support and consultations in Hungarian and English, depending on the language skills of the helpline operators. Hungary therefore does not meet the Istanbul Convention minimum standards for the provision of a national women's helpline.

Both the NANE and PATENT Legal Aid helpline rely on a mix of funding sources, including international grants, individual donations, and limited funding comes from public sources, such as municipal (project) support, or taxpayers' offers.⁴² Both organisations also depend heavily on volunteer work and pro-bono expertise to operate sustainably. In 2024, the NANE Helpline received 19,513 initiated calls/attempts from 2,409 unique numbers, and was able to respond to 718 calls, 320 help requests via emails, 47 chat help requests and 181 help requests via Facebook.

⁴¹ Of these, 1,165 were answered by NANE and PATENT helpline operators.

⁴² There is a type of funding coming from personal tax offers of individual taxpayers as they can donate 1% of their personal income tax to a civil society organisation of their choice.

The PATENT Legal Aid helpline received 5,639 initiated calls/attempts from 627 unique numbers and was able to respond to 447 calls and 700 help requests via email. In addition to this, 150 chat inquiries were answered by PATENT's lawyers and volunteers and 140 clients received in-person consultations.

The most common forms of violence reported by callers to both helplines were domestic violence, physical violence, and psychological violence. Most callers reached out to the NANE Helpline regarding domestic violence, which typically includes a combination of verbal, psychological, economic, and physical abuse and in many cases, sexualised violence as well. A significant proportion of these situations also involve children. Most women contacting the PATENT Legal Aid Helpline seek legal guidance on divorce proceedings resulting from domestic violence and matters of child custody. Each case is unique, and intimate partner violence may manifest as physical, psychological, financial or sexual abuse, or a combination of these forms. In addition, many women fear losing custody of their child. Abusers often exploit legal proceedings to maintain control and to punish their former partners, prolonging harm and abuse, which makes continuous legal support provided through the helpline vital for PATENT's clients.

There are other helplines in Hungary that can provide support to survivors of violence against women. The National Crisis Management and Information Telephone Service (*Országos Kríziskezelő és Információs Telefonszolgálat – OKIT*, tel.: +36 80205520) is the statewide helpline for all survivors of domestic violence, child abuse, sexual exploitation, forced marriage, and human trafficking and has a central role in referring survivors to shelters. OKIT is available 24/7, free of charge, and offers multilingual support in Hungarian, English, and German. The helpline is run by the Hungarian Interchurch Aid (HIA) and is funded by the state. In 2024, it received approximately 10,300 contacts, including calls and emails, from around 10,000 callers. Of these, 4,598 were from women (of which 3,999 called in relation to their own case), and 1,040 from men (of which 762 called in relation to their own case). Additionally, OKIT initiated shelter placements for 2,117 survivors – 690 women, 1,401 children, and 26 men. The most common forms of violence reported by callers in 2024 were domestic violence, physical violence, and violence directly or indirectly affecting children. There are also helplines that provide support for survivors of specific forms of violence. For example, EMMA Association operates the EMMA Helpline (tel.: +36 80414565) for women facing difficulties with motherhood, primarily for those who had traumatising experiences during maternity or are survivors of obstetric violence. This helpline is free of charge, but not available 24/7.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	4
Total number of all shelters accessible to women	63
Existing number of beds	460⁴³
24/7 access	All

There are different types of services providing accommodation for all survivors of domestic violence and trafficking in Hungary. Access to shelter accommodation is available through the National Crisis Management and Information Telephone Service (*Országos Kríziskezelő és Információs Telefonszolgálat – OKIT*). Most shelters are open to both men and women, although there are institutions that only accept women survivors of violence. According to the information provided by the state, there is one crisis centre and one secret shelter only for women survivors of domestic violence as well as two protected accommodations for women survivors of human trafficking. These shelters offer in total 33 bed spaces for survivors of domestic violence and 18 bed spaces for survivors of human trafficking. Shelters offer 24/7 access and the state reports that women are not expected to pay for their accommodation. In terms of length of stay, women can stay in crisis centres for a maximum of eight weeks, in secret shelters for a maximum of six months. In the protected accommodation, the length of stay is tailored to the survivor's needs, meaning there is no pre-determined maximum length. The types of in-houses services most often provided by these shelters include casework, counselling/psychological support, referrals/collaboration with other services, practical advice, and legal advice as well as physical care (if necessary) and leisure activities.

According to the state, all the shelters that only accept women can accommodate homeless women and older women (65 years and above). Most of the shelters can be accessed by and are accessible to women with physical, cognitive, or sensory disabilities, women with substance abuse issues, transgender women, women with older sons (14 years and above), and women with children with disabilities. Some of the shelters are equipped to support women with uncertain residence permits/undocumented women and refugee women. However, the state notes that the admission of survivors into a crisis centre or shelter (as well as those in their care, such as children) is up to the institution. In the case of certain specific groups of women, their needs may be a deciding factor for admission into a shelter. Crisis centres, secret shelters, and protected accommodation facilities can only admit women who are capable of taking care of themselves and do not pose a threat to others (i.e., who can be integrated into the community). None of the shelters that accept only women can accommodate unaccompanied girls under the age of 18. There are no specialised shelters supporting particular groups of survivors nor women survivors (at risk) of

43 This figure excludes the six bed spaces in the protected accommodation only for male survivors of trafficking.

specific forms of violence, apart from the two protected accommodations for survivors of human trafficking.

Shelters only available to women in Hungary are funded by the state and run by other NGOs. These NGOs do not necessarily apply feminist principles and are not considered women's rights NGOs. Such shelters are available in the capital city, county seat and a smaller rural town. The main reasons for declining referrals to shelters are if the survivor did not experience domestic violence or human trafficking and if the survivor was not assessed to be in a crisis situation (as a result of domestic violence or human trafficking). Based on the experiences of women's NGOs, survivors are only assessed as being in a crisis situation in very serious cases of violence. The state claims that there is no waiting list for these services and that there are enough spaces for women survivors of violence. However, the entering condition "to be in a crisis situation" prevents many survivors from being eligible for shelter accommodation. In practice, the NANE Helpline has received complaints from survivors who could not find an available shelter space. That is often the case for women who seek shelter without children. According to the state, the most common forms of violence reported by survivors in 2024 were domestic violence, physical violence, psychological violence, and violence directly or indirectly affecting children. In 2024, 78 women and 170 child survivors of domestic violence were accommodated in women-only crisis centres and secret shelters.

In terms of other shelters that are available to both women and men survivors of violence, the state reports that there are 19 crisis centres and seven secret shelters for survivors of domestic violence with 286 available bed spaces, and 20 halfway houses with 80 beds for survivors after crisis intervention available in halfway houses. Additionally, there is one crisis apartment and three protected accommodations for survivors of human trafficking with 64 available bed spaces (one protected accommodation is only for male survivors of human trafficking with six bed spaces), and nine transitional houses/exit apartments with 36 beds for these survivors after crisis intervention. Survivors can stay in halfway houses for a maximum period of five years. There is no maximum length of stay in protected accommodation, it depends on the needs/recovery of survivors. The state reports that these shelters are run by church and municipal organisations and receive support from the state to provide shelter accommodation. In 2024, in total 613 women, 1,290 children, and seven men were accommodated in the domestic violence crisis services (which include crisis centres and secret shelters). The state notes that the number of male survivors receiving care is negligible. It also notes that when placing men in care, it is important to ensure that the host institution does not have a problem with accepting male survivors and that the presence of men does not have a negative impact on the recovery of female survivors receiving care.

❖ 3. WOMEN'S CENTRES

Total number of centres	0
Entities running women's centres (including	—

There are no women's centres in Hungary. However, there are non-residential services to support survivors of domestic violence which were established in 2018 – so-called "crisis ambulances". Currently there are seven crisis ambulances which are regionally spread throughout the country. Most of them are operated by the Hungarian Interchurch Aid (HIA). These services do not categorise themselves as women's centres; they are not operated by women's NGOs with a feminist and gender-specific approach. According to the information provided by the state, all crisis ambulances provide counselling/psychological support, legal advice, financial and social welfare support/advocacy (providing information and assistance in cases), referrals/collaboration with other services. Services such as housing advice, employment, representation at court/police/social services, and residential support services are available within other institutions.

The state reports that crisis ambulances are low threshold services and are available for all survivors and groups of women. One crisis ambulance provides specialised support to survivors of human trafficking. Apart from this, there are no specialised services supporting particular groups of survivors nor women survivors (at risk) of specific forms of violence in Hungary. According to the information provided by the state, the seven crisis ambulances are operated by civil society organisations with financial support from the state. Crisis ambulances are located in all regions, and an estimated 3,000-3,200 survivors were supported in 2024.

Several women's NGOs such as NANE and PATENT Association have different initiatives and programs that provide specialised services and support to survivors of violence against women through a feminist approach, although not in the form of a centre. The scope and outreach of these services is limited by available human and financial resources. NANE operates a trauma-response and empowerment support programme "Power to Change" which targets women survivors of intimate partner violence and domestic violence. In 2024, three "Power to Change" programmes were organised for 47 individuals. The "Integrated Client Service", a joint NANE-PATENT initiative, offers trauma-sensitive support to survivors including legal assistance, psychological, and social counselling. This initiative is delivered by a team of experts who regularly consult with each other to ensure balanced and coordinated cooperation. The "Helper's Network" is also a joint NANE-PATENT initiative which provided survivors with technical and practical support in relation to legal and administrative processes, in a trauma-informed and survivor-centred manner. The joint initiative ceased to operate in 2025.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no specialised support services for survivors of sexualised violence in Hungary. A specialised service for survivors of sexualised violence is available for children. It is based on the *Barnahus Model* from Iceland which was adapted and introduced in four cities in Hungary in the last decade; first, it was introduced in the city of Szombathely, then Debrecen, followed by two places in Budapest, and one in the city of Gyula. The *Barnahus Model* is recognised by the state, since the Act on Criminal Procedure now allows children to be involved in hearings with the support of specialist services to help assess and provide therapy to child survivors of neglect and abuse, and especially survivors of sexualised violence. Those five services now operate as Hearing and Therapy Centres. Women's rights NGOs have not been involved and are not taking part in this service provision process.

Organisations and state services available for survivors of domestic violence and other forms of violence against women can also support survivors of sexualised violence. For example, the NANE Helpline and PATENT Legal Aid offer support and consultations for cases involving sexualised violence, although not in the form of regular service provision for survivors of sexualised violence. These NGOs can provide specific sexualised violence services as well, if the capacity and resources allow it. For example, NANE organises support groups for adult women survivors of childhood incest. In addition, both organisations conduct different advocacy and awareness-raising work against sexualised violence.

5. PRIMARY PREVENTION SERVICES

Public authorities in Hungary do not collect data on primary prevention activities at a national, regional, or local level. Hungary also does not have a National Strategy or Action Plan on preventing and combating violence against women or specific forms of violence, with the exception of trafficking. In addition, primary prevention is not addressed in the goals and measures of the Action Plan developed for the European Union on "strengthening the role of women in the family and in society" for 2021-2030, and the first and second plan of measures for 2021-2022 and 2023-2024 did not include any aims related to primary prevention. There is also no definition of primary prevention at a national level, but there is a definition in the joint prevention programme developed by NANE and PATENT.

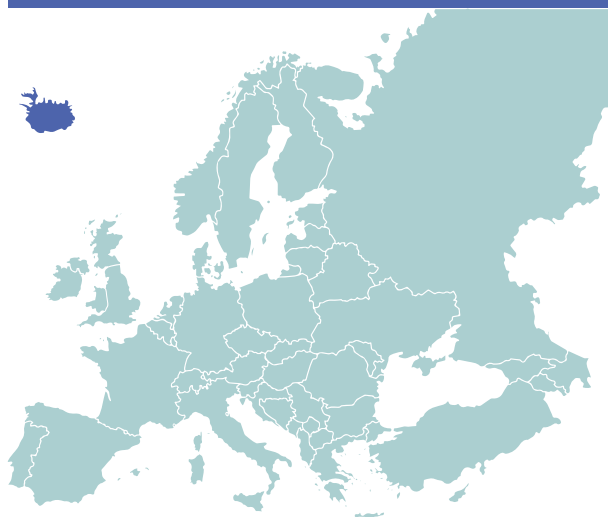
Available primary prevention programmes are run by women's NGOs. NANE and PATENT operate a joint intimate partner violence and sexualised violence prevention program for young people, composed of two stand-alone but complementary programmes. In 2024, 179 prevention workshops were organised, reaching nearly 800 participants. The two NGOs also manage a programme for teenage girls, called the "Rebellious Girl Camp", which

incorporates a series of interactive empowerment activities, and workshops connected to the topic of violence prevention. In 2024, one camp and one minicamp were organised.

NANE and PATENT apply for various, mostly international, tenders/calls for proposals to run these prevention workshops, but there are also related municipal cooperation projects. At least half of the workshops are financed by the client (e.g., school, community, etc.). The scope and reach of these programmes remain limited and there is currently no permanent or regular state funding for primary prevention programmes that focus on violence against women and girls in Hungary. There is no evaluation of the impact of primary prevention activities other than the analysis of participant feedback. None of the evaluations are financed by public authorities.

School-based prevention activities within the public education system have often been dependent on the commitment of individual teachers. Moreover, changes in the legislation in 2021 created an obstacle for NGOs conducting prevention activities to access schools: according to the new regulation, any person or organisation cooperating with an educational institution and holding a session on sexual culture, sex life, sexual orientation, sexual development, the harmful effects of drug use, the dangers of the internet and other physical and mental health promotion is required to receive a special authorisation by an official body designated by law. This body was only appointed in June 2025, and therefore organisations providing primary prevention programmes could not register as a provider of these activities before this. As a consequence, schools were noticeably less open to inviting external organisations, including violence prevention programmes organised by women's NGOs. In addition to this, in practice and based on the school curriculum, there is no space and time in the public education system to talk about sexual education and violence prevention.

ICELAND



GENERAL COUNTRY INFORMATION

Population	383,567
Female population	187,015
Member of Council of Europe (year)	No
Member of European Union (year)	1946
Member of United Nations (year)	1985
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2018

SUMMARY

Iceland has one national women's helpline that is free of charge, operates 24/7, and provides multilingual support. There are two women-only shelters in Iceland, with a total of 28 beds. Iceland has seven women's centres, as well as two rape crisis centres, which provide crisis and medical services for women survivors of sexualised violence. Provisions on primary prevention activities are included in official documents such as the Action Plan on the prevention of sexual and gender-based violence and harassment among children and young people 2021-2025, and there is a specific budget allocated to primary prevention activities. The state collects data on women's specialist services, which is publicly available, but it does not collect data on primary prevention.⁴⁴

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

There is one national women's helpline in Iceland called The Women's Shelter Helpline (*Neyðarsími Kvænnaathvarfsins*, tel.: +354 5611205), which is run by The Women's Shelter Association (*Samtök um Kvænnaathvarf*). The helpline is free of charge, available 24/7 and offers multilingual support in any language supported by the Language Line. Therefore, the helpline meets the minimum standards of the Istanbul Convention for the provision of a national women's helpline. The Women's Shelter Helpline provides support for survivors of domestic violence and intimate partner violence. Funding for the national helpline comes from the state as well as from donations. The most common forms of violence reported in 2024 were domestic violence, physical violence and psychological violence. There are no other regional helplines supporting survivors of violence against women in Iceland.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	2
Total number of all shelters accessible to women	2
Existing number of beds	28
24/7 access	All

In Iceland, there are two women-only shelters, one in Reykjavik and one in Akureyri, providing a total of 28 beds. The number of beds generally meets the country's needs, although in rare cases all places are occupied. As there are no legal regulations on the maximum number of residents, shelters are never considered "full," and solutions are found if all beds are taken. There is however still a shortage of women-only shelters in rural areas, as well as a lack of specialised services for survivors of trafficking, sexual exploitation, or women with substance abuse issues. Both shelters offer 24/7 access. The Reykjavik shelter has a consultant on duty around the clock, while in Akureyri night-time access is ensured through an emergency phone line. Women are not expected to pay for their accommodation. There is no set time limit for the length of stay, as each case is assessed individually. On average, women stay around 30 days. The main in-house services provided by women-only shelters include counselling and psychological support, referrals and cooperation with other services, practical advice on housing, social benefits, healthcare, and employment. General counselling is the predominant type of support provided.

All women-only shelters can accommodate women with uncertain residence permits/undocumented women, refugee women, women with sensory disabilities and older women. Most shelters can be accessed by and are accessible to women with physical or cognitive disabilities, transgender women, women with older sons and women with children with disabilities. Some shelters are equipped to support homeless women and women with substance abuse issues. There are no specialised shelters supporting particular groups of survivors nor women survivors (at risk) of specific forms of violence in Iceland.

⁴⁴ Data on women's specialist services is publicly available here: <https://www.112.is/>

Women-only shelters in Iceland are primarily funded by the state and through donations. Both shelters are run by women's NGOs with a gender-specific and feminist approach and are located in major cities. In 2024, the main reasons why women-only shelters had to decline referrals were that survivors were ineligible for support, or that shelters were not adequately equipped to meet the survivor's specific needs. Additionally, women-only shelters do not provide accommodation to women with active substance abuse issues. The most commonly reported forms of violence by survivors in 2024 were domestic violence, physical violence, and psychological violence. In total, 162 women and 117 children were accommodated in women-only shelters in 2024.

There are no other shelters which can provide support to survivors of violence against women in Iceland. There is one shelter providing shelter to survivors of violence which operates as a homeless shelter for women and is also accessible to women with substance abuse issues. This shelter has 12 available beds and is run by the NGO Rótin. This shelter is women-only but it is not explicitly for survivors of violence, although homeless women are very likely to be survivors of violence.

❖ 3. WOMEN'S CENTRES

Total number of centres		7
Entities running women's centres	Women's NGOs (3 centres)	Collaboration between state, municipalities and NGOs (4 centres)

There are seven women's centres in Iceland, two of which are only for women survivors of violence and five for all survivors of violence, including women. All centres provide counselling and psychological support, legal advice, housing advice, as well as referrals and collaboration with other services. Two centres also operate as women-only shelters, in Reykjavik and Akureyri, and therefore provide both residential and non-residential support. Women's centres also provide specialist support for survivors of sexualised violence, including specialist psychological care and community awareness and education for prevention.

All women's centres are equipped to support women with uncertain residence permits and undocumented women, women with sensory disabilities, older women and transgender women. Most women's centres can be accessed by and are accessible to refugee women, women with physical disabilities and women with cognitive disabilities. Some of them are equipped to support unaccompanied girls under the age of 18. In Iceland, all seven women's centres are equipped to support survivors of trafficking, but only one of them is specialised in providing such services.

Apart from that, there are no other centres specialised in supporting particular groups of survivors nor women survivors (at risk) of specific forms of violence.

Three women's centres are run by women's NGOs, and the four remaining ones are run in cooperation with NGOs, the state and municipalities. Funding for women's centres stems mainly from the state and donations and they are located in major cities of Iceland. Roughly 2,000 women were supported by all women's centres in 2024.

🚰 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Iceland, specialised services for survivors of sexualised violence are available and include rape crisis centres, crisis and medical services, and specialised helplines. Two rape crisis centres operate within national hospitals in the country's two main cities, where survivors can access both crisis response and medical care. The rape crisis centre in the north of the country was already in operation in 2022 but has now become fully functional. The national helpline can also provide support for survivors of sexualised violence, although it is not specialised for survivors of such violence. Additionally, the police helpline (112) has been developed over the last years to be better equipped to support survivors of sexualised violence.

There are education and counselling centres for survivors of sexual abuse and violence including *Stígamót* in Reykjavik, *Aflið* in Akureyri, *Sigurhæðir* in Selfoss and *Barnahús* for young survivors. *Stígamót* further provides free counselling for survivors of rape, sexual molestation, sexual harassment, and sexual exploitation. The organisation also conducts workshops and trainings on various aspects of sexualised violence and survivor support, aimed at professionals, individuals, and government officials.

The services offer a wide range of support, including specialist forensic and medical care, needs assessments, specialist psychological care, specialist sexualised violence advocacy services, legal advice as well as community awareness and prevention initiatives. Services are funded by the state and are available across all regions of the country. While women and girls are the primary target group, survivors of all genders can access these services.

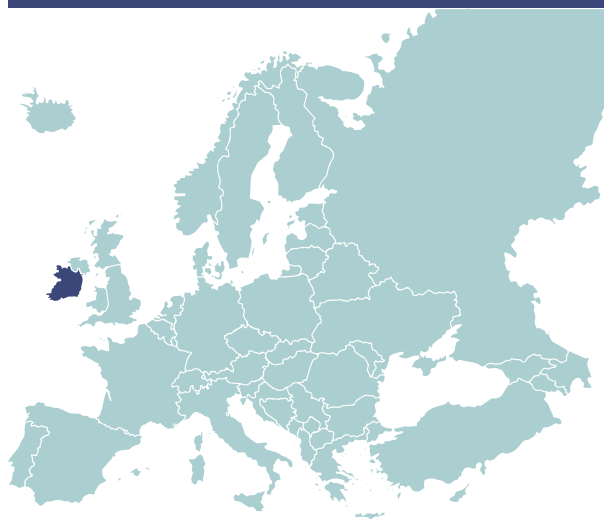
👤 5. PRIMARY PREVENTION SERVICES

Public authorities in Iceland do not systematically collect data on primary prevention activities, although provisions on primary prevention are included in official documents such as the Action Plan on the prevention of sexual and gender-based violence and harassment among children and young people 2021–2025.⁴⁵ Iceland allocates a

45 Government of Iceland, Action Plan on the prevention of sexual and gender-based violence and harassment among children and young people 2021–2025, <https://www.stjornarradid.is/verkefni/mannrettindi-og-jafnretti/jafnretti/forvarnir-medal-barna-og-ungmenna-gegn-kynferdislegu-og-kynbundnu-ofbel-di-og-areitni/>

specific, non-fixed budget to primary prevention activities, where specific projects receive funding, often applied annually or every three years. Primary prevention activities in the country include feminist self-defence trainings, school-based prevention programmes, training for prevention practitioners, bystander intervention training, and awareness-raising campaigns or programmes. However, the impact of these activities is not evaluated by public authorities. Further activities include *Sjúkást* (Crazy Love), a public awareness and prevention campaign run by *Stígamót*, focussing on healthy relationships and information for young people.⁴⁶ Public awareness campaigns are partially funded by national authorities and have increased in size from year to year. The Crazy Love campaign runs every year in cooperation with youth centres and schools.

IRELAND



GENERAL COUNTRY INFORMATION

Population	5,351,681
Female population	2,708,306
Member of Council of Europe (year)	1949
Member of European Union (year)	1973
Member of United Nations (year)	1955
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2015
Istanbul Convention ratified (year)	2019

SUMMARY

There is one national women's helpline in Ireland which is free of charge, available 24/7, and provides multilingual support. Additionally, there are 37 local helplines run by specialist domestic violence support services. There are 20 women-only shelters in Ireland with a capacity of 159 beds as well as 37 women's centres operating as specialised domestic violence centres providing a wide range of support services. There are 17 rape crisis centres in Ireland and six sexual assault treatment units providing support to survivors of sexualised violence. Provisions on primary prevention are included in the national strategy on domestic, sexual and gender-based violence and there is a specific budget allocated to such activities. Cuan, the State Agency for Domestic, Sexual & Gender-Based Violence, was established in February 2024 and collects data on women's specialist services. A new portal is currently under development to showcase this data which, when gathered, will give a more accurate picture of violence against women in Ireland. In addition to this, Safe Ireland is engaging with the Central Statistics Office on the first ever national prevalence survey.

⁴⁶ More info on the campaign available here: <https://sjukast.is/>.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	24,396 calls

There is one national women's helpline in Ireland, the 24hr National Freephone Helpline (tel.: +353 1800341900) which is run by Women's Aid. This helpline is free of charge, operates 24/7, and provides services in 240 languages through the telephone interpretation service. Therefore, Ireland does meet the Istanbul Convention minimum standards for the provision of a national women's helpline. A chat service is also available which operates Monday to Friday from 10:00–13:00 and 19:00–22:00 as well as Saturday from 12:00–15:00 and 19:00–22:00 and Sunday from 11:00–13:00 and 19:00–22:00. This helpline mainly provides support for survivors of domestic and intimate partner violence. Funding for the helpline comes from statutory and foundation support from Cuan, Pobal and the Community Foundation of Ireland. It is also supported by the public through donations and fundraising. The three most common forms of violence reported by callers to the women's helplines in 2024 were emotional/psychological abuse, which was the most common, followed by physical abuse, economic abuse, and sexualised abuse. The helpline received a total of 24,396 calls in 2024.

Additionally, there are 37 local helplines run by specialist domestic violence support services which are part of the Safe Ireland Network. Since the Women's Aid National Freephone Helpline is the only accredited 24/7 service for domestic violence, it acts as a direct referral to all other services when required. All other local services themselves offer helpline support to a greater or lesser extent, some of which are available 24/7. Most are funded through state funding in whole or in part.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	20
Total number of all shelters accessible to women	20
Existing number of beds	159
24/7 access	Some

There are 20 women-only shelters in Ireland with 159 available beds. In 2024, 12 new units were delivered and a further 54 units are under construction or due for refurbishment in 2025.⁴⁷ The Domestic, Sexual and Gender-based Violence Agency, Cuan, is coordinating the delivery of these additional shelter spaces. It is not known how many women-only shelters offer 24/7 access. Women are

expected to pay for their accommodation; this can range from approximately €30–€60 per week, but inability to pay is commonly considered by services.⁴⁸ The average length of stay in women-only shelters varies between services from six weeks to up to six months. Some services have a timeframe of up to three months while others will put no limit on it and instead will determine the length of stay depending on the need, or the availability of move-on options in the locality. There is often a demand to stay longer particularly because of the current housing crisis and the lack of move-on accommodation options. The types of in-house services most often provided by women-only shelters are counselling/psychological support, referrals/collaboration with other services, practical advice, casework, and legal advice.

All women-only shelters are equipped to support older women. Some women-only shelters can be accessed by and are accessible to homeless women, women with physical or cognitive disabilities, women with substance abuse issues, and women with older sons (14 years and above). None of the women-only shelters in Ireland can accommodate women with uncertain residence permits/undocumented women. There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Ireland. Women at risk of other forms of family violence, such as forced marriage and honour-based violence can request a place in a women-only shelter which is assessed on a case-by-case basis. Some services for women with disabilities are available but many shelters are in older buildings that require investment for physical accessibility, and many are not designed to accommodate neurodivergent people. Furthermore, if a woman is dealing with substance abuse issues, in most situations, women-only shelters will not accommodate her. Other shelters can accommodate a woman who is in a treatment programme, provided that she is not actively using drugs and/or alcohol.

Funding for women-only shelters comes from the state and donations. Currently, women-only shelter services are primarily funded by Cuan and are run by independent NGOs with a feminist and gender-specific approach. Women-only shelters are located in most regions of Ireland and currently, nine counties have been identified as having no shelter spaces. The main reason for having to decline referrals was no space or capacity to support the survivor. The most common forms of violence reported by survivors to women-only shelters in 2024 was emotional, physical and economic violence.

There are a number of other shelters in Ireland which can provide support to survivors of violence against women, such as homeless services that are not exclusive for women and provide accommodation more generally to families and individuals, but the exact number is unknown. There

⁴⁷ Of these, 22 are expected to be delivered in Dublin and Louth, according to Houses of the Oireachtas.

⁴⁸ This information comes from the WAVE Country Report 2023.

are two services that accommodate women (and their children) experiencing addiction and/or homelessness, but these are not specifically related to an experience of violence. Some services are short-term (night by night only) and some offer more medium to long-term accommodation. While most services support people with complex needs, including trauma, very few of these shelters are gender-sensitive and lack expertise on the dynamics of domestic violence or the risks and impacts of sexualised violence. Some of these shelters offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	37
Entities running women's centres	NGOs (37 centres)

There are 37 women's centres in Ireland which are all part of Safe Ireland. Regarding residential support services, 20 women's centres offer both non-residential and residential services and operate also as women-only shelters. All women's centres provide legal advice (although this may be more legal information rather than direct advice from lawyers), financial and social welfare support/advocacy, housing advice, referrals, and representation at court/police/social services. The High-Risk Support Project run by Women's Aid with the police provides support in post separation cases where there is a risk of serious harm or murder. Furthermore, 20 centres offer counselling/psychological support. Women's centres also provide specialist support for survivors of sexualised violence, including specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services and community awareness and education for prevention.

While all women's centres can support women with uncertain residence permits/undocumented women, there can be challenges to providing shelter accommodation, but other services can be provided, such as casework. The Immigrant Council of Ireland and Migrant Rights Centre Ireland provide support for survivors of trafficking. AkiDwA migrant women's organisation provides support to survivors of female genital mutilation.

All women's centres are run by NGOs and are located in most regions of Ireland. However, there continue to be some geographical places that are poorly served, and there is an ongoing piece of work to develop into these communities whereby a national services plan is to be developed by Cuan, the Domestic, Sexual and Gender-based Violence Agency. Women's centres are funded by the state as well as through donations. It should be noted that no centres are fully state funded; shortfalls, innovation and core roles not recognised by the state are funded from a variety of other sources.

🚫 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for survivors of sexualised violence in Ireland including 17 rape crisis centres (RCCs) and six sexual assault treatment units (SATUs). Since 2022, one new RCC has opened in West Cork. However, availability and consistency of services vary significantly depending on geographic location and local capacity. All RCCs provide legal advice, needs assessment, specialist psychological care, specialist sexualised violence advocacy series as well as community awareness and education for prevention. Delivery is often shaped by individual management decisions and local resources rather than through a coordinated national framework. This results in regional disparities in service access and quality. Survivors are able to access services in the location of their choosing, regardless of where they live. RCCs in Ireland receive funding from the state as well as donations. Whilst the majority of clients are women and girls, RCCs are open to everyone from the age of 14 onwards.

The six SATUs provide clinical, forensic and supportive care for those who have experienced sexualised violence. These units operate with varied opening hours, referral criteria, and availability across regions, with some only accepting police referrals. Where SATUs are not accessible, emergency departments can provide crisis medical care, but training on responding to sexualised violence is inconsistent and often inadequate. All units are operated by the state Health Service Executive which is financed entirely through public funds. Clients are also offered a needs assessment which may include, for example, assessing special measures in court or an International Protection Risk Vulnerability Assessment.

In addition, vital support is offered by grassroots community organisations whose core remit may not be sexualised violence but who nonetheless play a critical role, particularly for groups such as LGBTQ+ people, members of the traveller community, migrants, and others. These services often operate without formal recognition or integration into the national system, leaving key survivor support networks effectively invisible in official data and planning. While RCCs offer national geographic coverage, physical access remains a significant barrier in many rural areas due to poor public transport infrastructure. Survivors in isolated communities may be unable to reach in-person services, compounding trauma and exclusion. The national response to sexualised violence depends not only on recognised services like RCCs, but also on the broader ecosystem of informal, community-based supports and emergency healthcare services. A critical gap remains in the provision of national accompaniment services for survivors engaged in the justice process. While this service exists in some areas, it remains inequitable. Survivors in many parts of the country have no access to consistent court or police accompaniment or legal process support.

Rape Crisis Ireland has been delivering national standardised training and currently administers funding for a volunteer-based, ad-hoc accompaniment service that responds to need as it arises. In recent years, the sector has worked to transition to a more professional model with dedicated roles and there is one such service in the capital city that is funded.

The Dublin Rape Crisis Centre runs the national helpline for survivors of rape and sexualised violence, the National Rape Crisis 24hr Helpline (tel.: +353 1800778888). This helpline is free of charge, available 24/7 and fully funded by the state. For those who are deaf or hard of hearing, a text service operates Monday to Friday from 8:00-18:30 (tel.: +353 868238443) and a webchat service is available Monday to Friday, 10:00-17:00 (excluding Bank Holidays). All regional RCCs also operate a local helpline during working hours which is run by volunteers and funded through local fundraising. These local services are often the first point of contact for survivors, as well as for professionals such as the police, social workers, and SATUs. Furthermore, there are two helplines run by NGOs that specialise in services for child survivors of sexualised violence. The first one is run by the organisation CARI which runs a part-time helpline (tel.: +353 1890924567) and the second is run by the NGO One in Four (tel.: +353 16624070) which provides support for adult survivors of childhood sexual abuse.

There are also a number of other specialised services offering support to children, families, and adult survivors of sexualised violence in Ireland. Namely, there is one Barnahus children's service run by the state, which also runs an adult child sexual abuse counselling service. Additionally, there are two NGOs, One in Four and Ruhama, working with survivors of sexualised violence and broadly working from a feminist and gender-sensitive approach (although not only working with women). Ruhama works specifically with women in the sex industry and survivors of trafficking for the purposes of sexual exploitation and runs a dedicated helpline (tel.: +353 1800020202) as well as employment support service for women who have experienced commercial sexual exploitation (the Bridge to Work Programme).

5. PRIMARY PREVENTION SERVICES

Provisions on primary prevention are included in official documents such as the "Third National Strategy on Domestic, Sexual & Gender-Based Violence 2022–2026"⁴⁹ and "Implementation Plan 2024".⁵⁰ There is a specific budget allocated to primary prevention activities in Ireland, which are funded by Cuan the National State Agency for Domestic, Sexual & Gender-Based Violence on a rolling basis, and NGOs advise and collaborate on these programmes. In addition, there are a range of local or sector-specific initiatives that have partial state funding. Available primary

prevention activities include school-based primary prevention programmes, bystander intervention training, and awareness-raising campaigns or programmes. The programmes in schools are partially, but not wholly, related to primary prevention. Some primary prevention initiatives are at the early stages of analysis and so fall more firmly into awareness raising rather than structural change. Available evaluations are conducted at a national level by Cuan under the guidance of the Department of Justice. Safe Ireland liaises with other organisations to coordinate primary prevention activities such as MEND (Men's Development Network) and Move – Men Overcoming Violence, which provide information and assistance for people experiencing domestic violence in Ireland.

49 Available here: <https://assets.gov.ie/static/documents/third-national-domestic-sexual-and-gender-based-violence-strategy.pdf>

50 Available here: <https://assets.gov.ie/static/documents/dsgbv-implementation-plan.pdf>

ITALY



GENERAL COUNTRY INFORMATION

Population	58,971,230
Female population	30,124,502
Member of Council of Europe (year)	1949
Member of European Union (year)	1957
Member of United Nations (year)	1955
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2013

SUMMARY

There is one national women's helpline in Italy, operating 24/7 and free of charge, that offers multilingual support. There are 464 women-only shelters in the country, which, according to the latest data available, offer approximately 3,200 beds, and there are 404 women's centres. The women's centres that are part of the network D.i.Re. provide specialist and competent support for survivors of sexualised violence too, while medical and forensic services are provided in hospitals in the major cities. Primary prevention is included in the National Strategic Plan 2021–2023 (extended to 2025), but no data is collected about such activities, and no budget is allocated. The state collects data on services supporting survivors of violence, without differentiating between women's specialist services and general services. This data is public.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/ callers in 2024	65,064 calls 10,050 chat contacts 76,732 callers

There is one national women's helpline in Italy, called National Helpline Against Violence and Stalking (*Numero Nazionale Anti Violenza e Stalking*, tel.: +39 1522). The helpline is free of charge and available 24/7, and offers multilingual support in Italian, English, French, Spanish, Arabic, German, Albanian, Persian, Algerian, Syrian, Syro-Lebanese, Moroccan, Egyptian, Berber, and Chinese. The helpline does therefore meet the standards of the Istanbul Convention for the provision of a national women's helpline. It supports survivors of all forms of violence against women, with the most reported forms of violence being physical, psychological and economic violence. In 2024, the helpline was run by the NGO *Differenza Donna*, but in June 2025, this role passed to the organisation *Centro Veneto Progetti Donna*. The helpline is funded by the state, and in 2024, it received 65,064 valid calls and 10,050 chat contacts from 76,732 callers.

There are other regional and specialised helplines in Italy. For instance, the anti-trafficking helpline (tel.: +39 800290290) is free of charge and active 24/7, providing support to survivors of trafficking and exploitation with information and referrals to the local social welfare services. It is available in several languages, including Italian, English, Albanian, Russian, French, Spanish, Romanian, Hungarian, Arabic, Chinese, Polish, and Portuguese.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	464
Total number of all shelters accessible to women	N/A
Existing number of beds	3,200 ⁵¹
24/7 access	Most

There are 464 women-only shelters in Italy, which, according to the latest statistics available from 2023, offer approximately 3,200 beds. Most shelters provide 24/7 access, and women are not expected to pay for their accommodation. Nevertheless, shelter services are paid for by social services, and if a woman is not eligible to receive support from social services (e.g. migrant women), she might not be able to be accommodated. On average, women can stay in the shelter between three and six months. Due to the lack of adequate housing and social policies, the length of their stay is often extended, especially in

51 Data from the Country Report 2023, as the 2024 national statistics were not published at the time of writing.

disadvantaged regions. The types of in-house services most often provided by women-only shelters are case-work, counselling/psychological support, referrals/collaboration with other services, and practical and legal advice.

In terms of accessibility, all women-only shelters can accommodate refugee women, homeless women and older women; most shelters can accommodate women with older sons; some shelters can be accessed by and are accessible to women with uncertain residence permits and undocumented women, women with physical and with sensory disabilities, transgender women, women with children with disabilities and unaccompanied girls under the age of 18. There are also shelters specialised in supporting women survivors of trafficking, forced marriage and honour-based violence.

The main sources of funding are the state, donations, international and religious institutions, and they are run by different actors: women's NGOs with a gender-specific and feminist approach, other NGOs, faith-based organisations and municipalities. Shelters are located in most regions, although many areas, particularly rural ones, lack adequate coverage. If women-only shelters had to decline referrals, the main reasons were the lack of space and capacity to support the survivor, the survivor being ineligible for support, or the shelters not being adequately equipped for the needs of the women and children. The most common forms of violence reported by survivors in 2024 were domestic violence (including physical, sexualised, psychological, economic violence and violence directly/indirectly affecting children), stalking and technology-facilitated violence against women. In 2023, 3,574 women were accommodated in shelters, of which 3,054 were accommodated in women-only shelters.

There are also other shelters in Italy providing support to survivors of violence, but no data is available on their numbers and availability.

❖ 3. WOMEN'S CENTRES

Total number of centres	404
Entities running women's centres	Women's NGOs Other NGOs State

There are 404 women's centres in Italy, and 118 of these are run by the Women's Network against Violence D.i.Re. (Donne in Rete contro la Violenza). Women's centres provide counselling and psychological support, legal and housing advice, financial and social welfare support, employment referrals and collaboration with other services, representation at court, police, social services, and residential support services. Additionally, women's centres that are run by women's NGOs also provide specialist support for survivors of sexualised violence, including needs assessment, specialist psychological care, community awareness and education for prevention, and legal advice.

All women's centres can be accessed by women with uncertain residence permits, undocumented women, refugee women, and older women. Most women's centres can be accessed by and are accessible to women with physical, cognitive and sensory disabilities, while some centres can be accessed by transgender women and unaccompanied girls under the age of 18. Additionally, in Italy there are centres specialised in supporting survivors (at risk) of forced marriage, honour-based violence, female genital mutilation, trafficking in persons, and centres specialised in supporting women with uncertain residence permits, undocumented and refugee women, black and minority ethnic women, women with physical, cognitive, sensory disabilities, older women, girls under the age of 18 and transgender women.

👉 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no rape crisis centres or sexual violence referral centres in Italy. There are crisis/medical services for survivors of sexualised violence, since according to the Italian criminal law on sexual violence, public health institutions are obliged to collect and preserve forensic evidence of violence, independently from the circumstances and the decision of the survivor to file a complaint. Despite all emergency services in major hospitals being equipped to fulfil this obligation, there is no guarantee that the staff are trained and specialised to ensure proper medical and psychological care and trauma-sensitive support. Only some public hospitals in major cities apply an effective referral system and comply with the risk assessment standards. In other cases, survivors are referred to the local women's centres that are run by women's NGOs and provide specialist and competent support for cases of sexualised violence. Women's centres can accompany women during medical procedures and judicial proceedings, offer needs assessment, specialist advocacy services and community awareness and education for prevention. Specialist forensic and medical care is provided in medical services. In Italy, medical services for survivors of sexualised violence are funded by national/local authorities, are available only in major cities and are unevenly spread across the country. They support all survivors of sexualised violence, including women, minors, and other population groups.

There is no specialised helpline for cases of sexualised violence, and the national women's helpline provides support for these cases too.

👉 5. PRIMARY PREVENTION SERVICES

There is no centralised national database dedicated exclusively to primary prevention activities. The National Strategic Plan 2021–2023 (extended to 2025) mentions primary prevention and provides a definition, but does not include a dedicated strategy for primary prevention of violence against women and girls. Additionally, while national plans incorporate some primary prevention measures,

they primarily focus on awareness-raising activities. Only a few local authorities have their programmes specifically dedicated to primary prevention, which often focus on awareness-raising or are limited to short-term actions. There is no specific budget allocated to primary prevention activities, and no systematic national evaluation. Various activities are conducted in the country, mostly by feminist organisations or within the scope of individual projects, and often without funding from public institutions. These include feminist self-defence trainings, empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, trainings for primary prevention practitioners, bystander intervention trainings, self-care activities and awareness-raising campaigns or programmes. At the time of writing, the Ministry of Education is proposing a bill on prior informed consent of families for school activities related to sexuality. Moreover, the Ministry aims to exclude all children under the age of ten from primary prevention programmes on sexual-affective education, limiting the activities of feminist and LGBTQI+ organisations.

KOSOVO



GENERAL COUNTRY INFORMATION

Population	1,773,971 ⁵²
Female population	903,544 ⁵³
Member of Council of Europe (year)	No
Member of European Union (year)	No
Member of United Nations (year)	No
CEDAW ratified (year)	No ⁵⁴
CEDAW optional protocol ratified (year)	No
Istanbul Convention signed (year)	No
Istanbul Convention ratified (year)	No ⁵⁵

SUMMARY

There is one national women's helpline in Kosovo, which is free of charge and available 24/7. Ten women-only shelters offer 180 beds, and there is one shelter specialised in supporting survivors of trafficking in persons. There are three women's centres in the country and no specialised services for survivors of sexualised violence. Public authorities collect data on primary prevention, and the National Strategy for Protection from Domestic Violence and Violence against Women includes specific provisions on primary prevention. The state collects data on women's specialist services, which is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

⁵² The latest available Eurostat population data for Kosovo is from 2022.

⁵³ Ibid.

⁵⁴ CEDAW was signed while Kosovo was still part of former Yugoslavia, in July 1980, and ratified in February 1982. Art. 22 of the Constitution of the Republic of Kosovo includes CEDAW in the International Agreements and Instruments with direct applicability in the Republic of Kosovo and, in the case of conflict, have priority over provisions of laws and other acts of public institutions.

⁵⁵ Art. 22 of the Constitution of the Republic of Kosovo includes the Istanbul Convention in the International Agreements and Instruments with direct applicability in the Republic of Kosovo and, in the case of conflict, have priority over provisions of laws and other acts of public institutions.

There is one national women's helpline in Kosovo, called Helpline Number (*Numri i Linjës Ndhmëse*, tel.: +383 80011112). It is available 24/7, free of charge and provides multilingual support in Albanian and Serbian. Therefore, the helpline is meeting the standards of the Istanbul Convention. The Office for Protection and Assistance to Victims runs the helpline, which is funded by the state. The helpline provides support to survivors of all forms of violence. In 2024, the most common forms of violence reported by callers were domestic violence, physical violence and sexualised violence.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	10
Total number of all shelters accessible to women	11
Existing number of beds	180
24/7 access	All

There are 10 women-only shelters in Kosovo, offering 180 beds. All shelters offer 24/7 access, and women are not expected to pay for their accommodation. On average, women stay in the shelters between three and six months, depending on their needs. The types of support offered include casework, counselling/psychological support, referrals/collaboration with other services, practical advice and legal advice.

While all shelters can accommodate women with children with disabilities and unaccompanied girls under the age of 18, most shelters can also be accessed by and are accessible to women with physical disabilities and older women. Some shelters can also accommodate homeless women, women with cognitive and sensory disabilities, and transgender women, while no shelters are equipped to support women with older sons.

Funding for women-only shelters comes from state funding, donations and international funding. Women's NGOs run all shelters, which are located in all regions. If shelters had to decline referrals, the reason was that they were not adequately equipped for the needs of the women and children. The most common forms of violence reported by survivors in 2024 were domestic violence, psychological violence and technology-facilitated violence against women. In 2024, 596 women found support in the country's shelters.

There is another shelter with 15 beds in Kosovo, which specifically supports women survivors of trafficking in persons; it is funded by the state and offers 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	3
Entities running women's centres	Women's NGOs (3 centres)

There are three women's centres in Kosovo supporting women and girls survivors of gender-based violence. Additionally, women's shelters also provide specialised services for women and girls. Women's centres provide different forms of support, including counselling/psychological support, legal and housing advice, financial and social welfare support/advocacy, employment, referrals/collaboration with other services, representation at court and other institutions, and residential support services. Additionally, they also support survivors of sexualised violence with specialist psychological care, as well as specialist forensic and medical care in cooperation with other institutions.

In terms of accessibility, all centres can accommodate older women and unaccompanied minor girls, while access for women with disabilities cannot always be guaranteed. All centres are also specialised in supporting survivors (at risk) of forced marriage and of trafficking in persons, while half of the centres are specialised in supporting survivors with physical, cognitive and sensory disabilities, older women, girls under the age of 18 and transgender women. Women's centres are run by women's NGOs and receive funding from the state, donations, and international funding. They are located in all regions, and in 2024, they supported 800 women.

🚰 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no specific services for survivors of sexualised violence in Kosovo. Women's centres also provide support to survivors of sexualised violence through legal services, offering shelter and carrying out prevention activities.

👤 5. PRIMARY PREVENTION SERVICES

In Kosovo, public authorities collect data related to primary prevention activities aimed at addressing gender-based violence. Kosovo has implemented the National Strategy for Protection from Domestic Violence and Violence against Women (2022–2026), which includes a definition of primary prevention and outlines comprehensive measures for the prevention and response to gender-based violence, including provisions for data collection and monitoring of prevention activities at both national and local levels. At the local level, coordination mechanisms against domestic violence are responsible for implementing local action plans aligned with the national strategy, including activities related to primary prevention, such as awareness-raising campaigns and educational programmes. There is a specific budget allocated to primary prevention activities conducted in the country, which include empowerment activities for women and girls, school-based primary prevention programmes and awareness-raising campaigns and programmes.

In Kosovo, evaluations of the impact of primary prevention activities related to violence against women and girls are primarily carried out by civil society organisations rather

than public authorities. For instance, several women's organisations, such as the Kosovo Women's Network (KWN) and the Centre for Information, Criticism and Action (QIKA), actively monitor and evaluate their own primary prevention programmes. These evaluations often focus on awareness-raising campaigns, empowerment initiatives, and educational workshops, assessing changes in public attitudes, knowledge about gender-based violence, and participation rates. However, these evaluations are usually funded by the NGOs themselves or supported by international donors. Public authorities in Kosovo currently do not have a comprehensive, government-funded evaluation framework specifically for primary prevention activities. While the National Strategy emphasises prevention and coordination, systematic impact assessments financed and conducted by the government are still lacking. Some NGOs focus exclusively or predominantly on primary prevention, including advocacy, education, and community mobilisation. These organisations tend to report qualitative results and case studies, but often lack resources for large-scale quantitative impact evaluations.

LATVIA



GENERAL COUNTRY INFORMATION

Population	1,871,882
Female population	1,003,142
Member of Council of Europe (year)	1995
Member of European Union (year)	2004
Member of United Nations (year)	1991
CEDAW ratified (year)	1992
CEDAW optional protocol ratified (year)	No
Istanbul Convention signed (year)	2015
Istanbul Convention ratified (year)	2023

SUMMARY

There is no national women's helpline in Latvia, but there are three other helplines supporting women survivors of violence. There are 17 women-only shelters, although the number of beds is unknown, and three women's centres. No specialised services for women and girls who have experienced sexualised violence exist. Latvia has a National Action Plan for Preventing and Combating Violence Against Women and Domestic Violence (2024–2029), which includes prevention as one of its central pillars. The state collects data about women's specialist services on the provision of services and clients supported, and this data is available upon request.⁵⁶

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

There is no national women's helpline in Latvia. Therefore, the country does not meet the Istanbul Convention

⁵⁶ Information from the WAVE Country Report 2023.

minimum standard for a national women's helpline. There are three other helplines supporting survivors of violence against women in Latvia. The first is the national crisis helpline 'Krīzes tālrunis' which is run by the Crisis and Counselling Centre "Skalbes" (Krīžu un Konsultāciju Centrs "Skalbes", tel.: 116123 or +371 27722292) and operates 24/7. This is a generic crisis helpline which provides emotional and informational support to anyone in crisis or any survivor of a crime. The second available helpline is called Victim (Cietušajiem, tel.: 116 006) and operates every day from 12:00–22:00. This helpline is for all survivors of physical, mental, economic, and sexualised violence. In 2023, the helpline supported 423 individuals, and 302 individuals were supported from January to October 2024 (however, not all calls related to violence against women). The third helpline is run by the NGO Association "MARTA Centre" (Biedrība "Centrs MARTA", tel.: +371 67378539) and provides support to women survivors of physical, emotional, sexualised, and economic violence as well as human trafficking. The helpline is free of charge but is not available 24/7 (only available weekdays from 10:00–18:00). The helpline offers multilingual support in Latvian, Russian, and English.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	17
Total number of all shelters accessible to women	N/A
Existing number of beds	N/A
24/7 access	All

In Latvia, there are 17 women-only shelters, with an unspecified number of beds.⁵⁷ All women-only shelters have 24/7 access, and there are shelters for women with children facing a crisis or emergency situations. Women-only shelters are funded by the state as well as through donations and exist in all regions of Latvia. The in-house services most often provided by women-only shelters include counselling, legal advice, practical advice, casework, and collaboration with other services (such as the police).

All women-only shelters are accessible to homeless women, older women, and women⁵⁸ with older sons (14 years and above). Most women-only shelters are equipped to support women with physical or cognitive disabilities, and women with substance abuse issues. The NGO Shelter Safe House (Patvērums "Drošā māja") operates a rehabilitation programme for survivors of human trafficking, offering safe shelter, legal and psychosocial support, basic necessities, and a 24/7 helpline (tel.: +371 28612120). In addition, MARTA Centre provides anonymous shelter accommodation for women survivors of violence and/or human

trafficking. MARTA Centre also operates a rehabilitation service for women and children survivors of violence who have a high lethality risk and are in a vulnerable situation, in the form of state paid crisis apartments.⁵⁹ There are no other specialised centres supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Latvia.

One women-only shelter is run by a women's NGOs with a gender-specific/feminist approach and six women-only shelters are run by other NGOs.⁶⁰ These shelters are for women and girls and are funded and managed by the NGO Children's Fund of Latvia. If women-only shelters had to decline referrals, it was mainly because the survivor was ineligible for support. For example, in case the survivor cannot present an ID or does not have citizenship, the first night in the shelter will be provided, but she cannot receive other services or longer-term shelter accommodation.

There are other shelters in Latvia, besides the women-only shelters, which can provide support to survivors of violence against women. Support is delivered through crisis centres and family support centres, and in 2023, 15 crisis centres provided support to women and children survivors of violence.⁶¹ State-funded social-rehabilitation services are delivered by five crisis/family support centres run by NGOs, which are available to all survivors of violence regardless of gender or citizenship. In 2023, social rehabilitation in crisis centres was ensured for 70 women and 538 children. Additionally, since July 2023, the state has financed crisis apartments for survivors of violence.⁶² These provide safe housing for up to 180 days for women, their children, and household members at high risk of violence. Services are offered through municipal housing, NGO premises, hotels/guesthouses, or rented apartments. Between July 2023 and December 2024, 11 women and 15 children accessed this support. The service is free of charge, with state funding covering accommodation costs (up to €39.67 per adult and €19.67 per child per day). Crisis apartments are available nationwide via any local social service office, which decides on eligibility, administers the service, and ensures cooperation with NGOs to provide not only shelter but also social rehabilitation.

3. WOMEN'S CENTRES

Total number of centres	3
Entities running women's centres	Women's NGOs (3 centres)

There are three women's centres in Latvia, all are run by women's NGOs and located in most regions of the

⁵⁷ Information on women-only shelters is from the WAVE Country Report 2023.

⁵⁸ Information on shelter accessibility is from the WAVE Country Report 2023.

⁵⁹ The service can be used for a period of up to 30 days with a possible extension to 180 days.

⁶⁰ This information is from the WAVE Country Report 2023.

⁶¹ GREVIO Baseline Evaluation Report for Latvia, 2025: <https://rm.coe.int/state-report-of-latvia-addressed-to-grevio-received-on-25-march-2025/1680b4fa6f>, p.19

⁶² Ibid., p.21

country.⁶³ One women's centre is the Talsi Women's and Children's Crisis Center which is located in Talsi. Additionally, MARTA Centre provides support in three centres located in Rīga, Liepāja, and Rēzekne. All women's centres in Latvia provide counselling/ psychological support and legal advice services to women survivors of violence against women.

Two women's centres provide specialised support to survivors of forced marriage and trafficking, and one centre provides specialist support to women with uncertain residence status/undocumented women. Women's centres receive funding from the state as well as donations. Some of the women's centres also offer support for survivors of sexualised violence including needs assessment, specialist psychological care and community awareness as well as education for prevention.

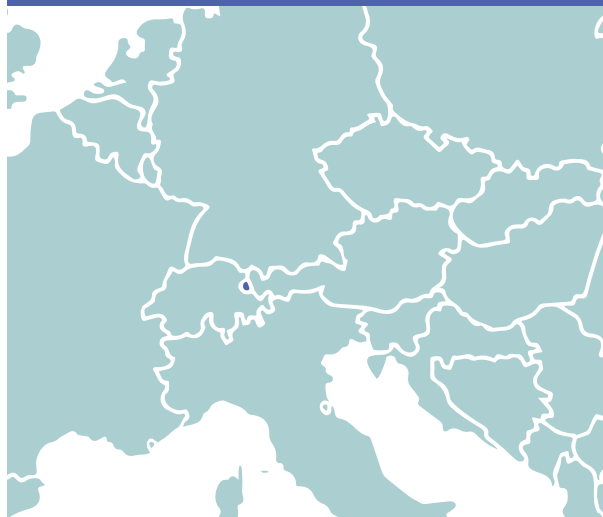
4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no services for women and girls who have experienced sexualised violence in Latvia. The country's "Plan for the Prevention and Combating of Violence Against Women and Domestic Violence 2024–2029" includes a measure relating to sexualised violence for which the Ministry of Health is designated as the responsible authority.⁶⁴ Measures include a pilot project to establish relevant support services for survivors of sexualised violence. Several NGOs (including Centre Dardedze for underaged girls and MARTA Centre) do however offer psychological and legal services for survivors of sexualised violence. The NGO Skalbes provides support to survivors of all forms of violence, including sexualised violence.

5. PRIMARY PREVENTION SERVICES

There is no official definition of primary prevention in Latvia. However, since 2024 the country has a National Action Plan for Preventing and Combating Violence Against Women and Domestic Violence (2024–2029), which includes prevention as one of its central pillars. Additionally, the state funds some primary prevention activities, namely, school-based primary prevention programmes, awareness-raising campaigns or programmes, and conferences on the prevention of violence.⁶⁵

LIECHTENSTEIN



GENERAL COUNTRY INFORMATION

Population	40,015
Female population	20,156
Member of Council of Europe (year)	1978
Member of European Union (year)	No
Member of United Nations (year)	1990
CEDAW ratified (year)	1995
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2016
Istanbul Convention ratified (year)	2021

SUMMARY

There is one national women's helpline in Liechtenstein which operates 24/7, is free of charge and offers multilingual support. There is one women-only shelter with three rooms (for three women and up to seven children), and two women's centres. There are no specific sexualised violence services. In Liechtenstein, there is no national strategy to prevent and combat violence against women, nor structured primary prevention programmes. The state collects data on women's specialist services, which is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

⁶³ This information is from the WAVE Country Report 2023.

⁶⁴ GREVIO Baseline Evaluation Report for Latvia, 2025: <https://rm.coe.int/state-report-of-latvia-addressed-to-grevio-received-on-25-march-2025/1680b4fa6f>, p.21

⁶⁵ This information is from the WAVE Country Report 2023.

There is one national women's helpline in Liechtenstein, called 24-hour Emergency Number (*24 Stunden Notruf*, tel.: +423 3800203). It is operated by the *Frauenhaus Liechtenstein*, is free of charge, and available 24/7. Therefore, it meets the standards of the Istanbul Convention for the provision of a helpline. In addition to German, callers can receive assistance in English, Italian, Spanish, Portuguese, French, Turkish, Bosnian/Croatian/Montenegrin/Serbian, Albanian, Czech, Ukrainian, Polish, Russian, and Hindi, with interpreters provided where necessary. It provides support across a broad spectrum of violence types, from physical abuse to economic and psychological violence. According to statistics from 2024, the most common forms of violence reported by callers were psychological violence, physical violence, and violence directly or indirectly involving children. The service is funded through state support as an officially recognised social service, complemented by donations and membership contributions.

In addition to the national women's helpline, other support lines are available. The Victims Assistance Centre (*Opferhilfestelle*, tel.: +423 2367696) provides legal, psychological, and social support to survivors and their relatives. The Information and Counselling Centre for Women (*Infra*, tel.: +423 2320880) offers counselling and legal advice. The Crisis Intervention Team (KIT, tel.: +423 2300506) provides immediate help in acute crises, while the Office for Social Services (tel.: +423 2367272) can be contacted for broader social support. The Helpline for children and young people (*Sorgentelefon für Kinder und Jugendliche*, tel.: +423 147) is a national helpline for children and youth, available free of charge 24/7, and the Helping Hand helpline (*Dargebotene Hand*, tel.: +423 143) offers telephone counselling for adults.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	1
Total number of all shelters accessible to women	1
Existing number of beds	3
24/7 access	Yes

There is one women-only shelter in Liechtenstein, the *Frauenhaus Liechtenstein*, which has three rooms with 10 beds in total, and can accommodate three women with their children. The shelter is accessible 24/7, and women are not expected to pay for their stay. Funding for the shelter comes from the state and donations. Women can stay in the shelter between one night and six months, but their stay can be extended if needed.

The women-only shelter is accessible to women with uncertain residence status, homeless women, older women, and women with older sons. Women with disabilities can find support in a specialised organisation called LBV (*Liechtensteiner Behinderten-Verband – LBV*). The shelter is not accessible to women with physical disabilities, and

women with substance abuse issues can find accommodation in a specific shelter called Association for Assisted Living (*Verein für betreutes Wohnen*).

The main reason why the women-only shelter had to decline referrals is that they do not have the space to accommodate survivors from other countries, as at least one-bed space must always be available for Liechtenstein residents, or they are not adequately equipped for the needs of women/children. The most common types of violence reported by clients were physical and psychological violence. In addition to accommodation, the shelter also offers counselling, referrals, and legal advice to survivors and their children.

In 2024, 19 women and 23 children were admitted to and supported at the Women's Shelter Liechtenstein. Three women were aged between 18–29, 16 were between 30 and 64 years old. In 17 cases the perpetrator of violence was the husband, in two the intimate partner. After their residential stay, two women and three children were taken into an external apartment and continued to receive support. The number of people seeking protection as well as the number of occupancy days has risen significantly since 2023 – in total 1,480 days in the women's shelter and 338 days in the external apartment. In 2024, the shelter recorded an average daily occupancy of 4.1 persons.⁶⁶

❖ 3. WOMEN'S CENTRES

Total number of centres	2
Entities running women's centres	Women's NGOs (2 centres)

There are two women's centres in Liechtenstein that support women survivors of violence against women: *Infra* (*Informations- und Beratungsstelle für Frauen – Infra*), which is run by a women's NGO, and a new counselling centre operated by *Frauenhaus Liechtenstein*. *Infra* offers women information and counselling services in areas such as legal advice, separation and divorce, marriage and cohabitation, sexual harassment, domestic violence, stalking, retirement planning, finances, and gender equality in the workplace. In addition, the women's centre offers legal counselling by female lawyers as well as counselling for migrant women in their native language. Since January 2024, the *Frauenhaus Liechtenstein* has operated its counselling centre for domestic violence from Mondays to Thursdays from 9:00–13:00 (and by appointment). It offers free, confidential support for women affected by domestic violence, their relatives, supporters, and professionals.

Additionally, the state operates one Victims Assistance Centre (*Opferhilfestelle*), which provides confidential counselling and comprehensive support to persons affected by crime and their relatives in coping with its consequences.

66 Data from the Annual Report 2024 of the Women's Shelter Liechtenstein, <https://www.frauenhaus.li/?page=10003>

The Victims Assistance Centre is funded by the state. In-fra relies both on state funding and donations, while the counselling centre of *Frauenhaus Liechtenstein* only relies on donations in 2024 and 2025, and will receive state funding starting from 2026.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Liechtenstein, there are no specific support services for survivors of sexualised violence. Medical services are provided at hospitals, and women survivors of sexualised violence are normally referred to the Immediate Help Centre after Sexual Violence at the Cantonal Hospital St. Gallen (Switzerland). Complementary services include forensic nursing at the Liechtenstein National Hospital (+423 2354416), which documents injuries (stemming from, for instance, domestic violence) without requiring a police report. Women's centres also support survivors of sexualised violence.

5. PRIMARY PREVENTION SERVICES

In Liechtenstein, there is a lack of a comprehensive national strategy, such as a National Action Plan, to prevent and combat gender-based violence against women.⁶⁷ Although some primary prevention measures are in place to empower women and reduce their vulnerability to violence, for instance, through awareness-raising campaigns and informing them about their rights, such measures are not offered regularly.⁶⁸ There are other measures in the country, such as the planned introduction of mandatory offender counselling, the Forensic Nursing Consultation at the Landesspital, and professional exchanges and training, but these focus mainly on intervention and documentation rather than on primary prevention.⁶⁹

LITHUANIA



GENERAL COUNTRY INFORMATION

Population	2,885,891
Female population	1,519,986
Member of Council of Europe (year)	1993
Member of European Union (year)	2004
Member of United Nations (year)	1991
CEDAW ratified (year)	1994
CEDAW optional protocol ratified (year)	2004
Istanbul Convention signed (year)	2013
Istanbul Convention ratified (year)	No

SUMMARY

Lithuania has one national women's helpline which is free of charge and operates 24/7 but does not offer multilingual support. There are two women-only shelters in Lithuania with 28 beds in total. There are 24 women's centres in Lithuania for all survivors of domestic violence, which operate as specialised assistance centres (SACs). There are services for survivors of sexualised violence, including five sexual violence referral centres and one specialised helpline. Lithuania has several action plans and programs that include primary prevention activities, but there is no official definition of primary prevention and no specific budget allocated to such activities. The state does collect data on women's specialist services, and this information is available upon request. However, there is no standardised data collection and services financed by the state or municipality are requested to provide differing statistics and data, depending on the funder.

⁶⁷ Committee on the Elimination of Discrimination against Women (CEDAW), Concluding observations on the sixth periodic report of Liechtenstein, published on 24 February 2025.

⁶⁸ GREVIO Baseline Evaluation Report for Liechtenstein, published on 4 December 2023.

⁶⁹ Annual Report 2023/2024 of the Coordination Group for the Implementation of the Istanbul Convention, published in November 2024.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	No
Total number of calls/callers in 2024	26,008 calls

There is one national women's helpline in Lithuania called Women's Helpline (*Pagalbos moterims linija*, tel.: +370 80066366) which is free of charge and available 24/7 but does not offer multilingual support. However, if the helpline consultants on duty speak other languages, they can sometimes provide multilingual support (for example in Russian, English or Polish). Lithuania therefore does meet the Istanbul Convention standards for the provision of a national women's helpline. The helpline provides support to women survivors of different forms of violence such as psychological violence, physical violence, sexualised violence and economic violence. The Women's Helpline is coordinated by three organisations: the public organisation Klaipeda Social and Psychological Service Center (*Klaipėdos socialinės ir psichologinės pagalbos centras*) in Klaipėda, Association "Kaunas Women's Line" (Association "*Kauno moterų linija*") in Kaunas, and Women's Issues Information Center (*Moteryų informacijos centras*) in Vilnius. The helpline receives partial funding from the Ministry of Social Security and Labour and supplements it with private donations. In 2024, there were 43,013 call attempts and a total of 26,008 calls, 1,374 online chat conversations, and 484 emails were answered. The most common forms of violence reported by callers in 2024 were domestic violence and psychological violence.

Specialised assistance centres – SACs (*Specializuotos kompleksinės pagalbos centras – SKPC*) also run a joint general helpline available Monday to Friday, 8:00-17:00 (tel.: +370 870055516). This helpline provides information, counselling assistance and emotional support to individuals who have experienced domestic violence or are at risk of violence. The helpline is operated by consultants from the Telšiai Crisis Center and Tauragė Women's Employment and Information Center (*Telšių krizių centro ir Tauragės moters užimtumo ir informacijos centro*), and live chat requests are answered by consultants from the Emotional Support and Counseling Center (*Emocinės pagalbos ir konsultacijų centro*). In 2024, assistance was provided to 535 individuals via the general telephone number and 98 individuals via online chat.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	2
Total number of all shelters accessible to women	N/A
Existing number of beds	28
24/7 access	All

There are two women-only shelters in Lithuania with 28 beds in total. The number of women-only shelters does not meet the needs of women survivors in Lithuania, as these shelters provide accommodation for the inhabitants of two cities in one district. Based on the assessed needs of the country, shelters are not the preferred support service for survivors of violence against women. The Lithuanian help provision model is focused on women's civic empowerment and proactive specialised complex help provision. An emphasis is placed on the legal framework that puts the responsibility on the perpetrator and strives to protect women's and their children's right to a safe home, by removing the perpetrator from the home. This however does not minimise the need for women-only shelters in Lithuania. Both women-only shelters offer 24/7 access, and women are not expected to pay for their accommodation. One women-only shelter covers one of the biggest cities in Lithuania, Panevezys, while the other shelter is only for residents of Klaipeda City. At the shelter in Panevezys, women can stay for up to six months, while in the shelter in Klaipeda, women are accommodated based on the individual case and needs, up to nine months. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services and legal advice.

Both women-only shelters in Lithuania can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, homeless women, women with physical, cognitive or sensory disabilities, older women (65 years and above), transgender women, women with older sons (14 years and above), women with children with disabilities, and unaccompanied girls under the age of 18. There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Lithuania.

Funding for women-only shelters in Lithuania comes from the municipality and from donations. One shelter is run by a women's NGO with a gender-specific and feminist approach and the other shelter is run by the state. The main reason for declining referrals to women-only shelters were no space/capacity to accommodate survivors with children, due to the limited number of available beds. Additionally, the women-only shelters do not accept women under the influence of alcohol or drugs – they can be admitted once they are sober. The three most common forms of violence reported by survivors to the women-only shelters in 2024 were domestic violence, physical violence, and psychological violence. Additionally, the women-only shelters provide humanitarian support as well as clothing, hygiene products, and food. A total of 55 women and 47 children were accommodated in women-only shelters in 2024.

There are other shelters in Lithuania which are accessible to women survivors of violence, although these are generic

and do not take a survivor-centred and feminist approach. There are 90 accredited social care providers, which offer intensive crisis intervention support on a municipal level. Among others, they support perpetrators and those at risk of committing domestic violence, formerly incarcerated individuals, individuals from vulnerable groups and survivors of domestic violence. Some of these shelters offer 24/7 and the exact number of beds is not known.

❖ 3. WOMEN'S CENTRES

Total number of centres		24
Entities running women's centres	Women's NGOs (15 centres)	
	Other NGOs (2 centres)	
	State (7 centres)	

There are 24 women's centres in Lithuania for all survivors of domestic violence, including women, which all operate as specialised assistance centres (SACs). All centres provide counselling and psychological support as well as referrals and collaboration with other services. Out of the 24 women's centres, 22 provide legal advice, financial and social welfare support, and housing advice. Two centres also organise psychotherapy sessions and other empowerment activities aimed at strengthening self-confidence for women. None of the centres can represent survivors at court, however, they can offer accompaniment to provide emotional support to the survivor. Women's centres also provide specialised support to survivors of sexualised violence in the form of specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice. All women's centres in Lithuania can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive or sensory disabilities, older women (65 years and above), and transgender women. None of the centres can support unaccompanied girls under the age of 18. All women's centres provide specialised support to women with uncertain residence permits/undocumented women. Furthermore, 20 centres provide specialised support to women with physical disabilities, 14 centres to women with cognitive disabilities, three centres to survivors (at risk) of trafficking as well as women with sensory disabilities, and two centres to survivors (at risk) of female genital mutilation.

Fifteen centres are run by women's NGOs, two by other NGOs and seven by the state. Funding for women's centres comes primarily from the state. SACs that do not receive state funding are limited in their activities, relying on existing projects or individual initiatives based on voluntary work. SACs cover all regions of Lithuania; however, they are not physically located in every municipality. This does not affect the provision of assistance, as centres offer support remotely online and via phone. When needed, in-person meetings are arranged either at the centre's office or by having a representative from the centre travel to the specific location. According to the State Data Agency,

a total of 21,180 women were supported by women's centres in 2024.

🚫 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for all survivors of sexualised violence in Lithuania including five sexual violence referral centres. Five major regional hospitals in Vilnius, Kaunas, Klaipėda, Šiauliai and Panevėžys are responsible for providing specialised services such as medical care, forensic examination, storage of evidence and crisis intervention to people who have experienced sexualised violence. However, the system is still under development and some of the hospitals do not yet fully provide all the necessary services. The five sexual violence referral centres are funded by the state and there is no additional funding to support survivors of sexualised violence. Sexualised violence services in Lithuania should cover all regions, however, on a local level there are few NGOs which can provide immediate assistance.

Beyond immediate medical needs, the network of specialised assistance centres (SACs) provides free, confidential support to individuals who have experienced domestic violence, including sexualised violence from intimate partners. This support includes psychological and intermediary assistance. There are 24 organisations performing SAC functions across Lithuania, 17 of which receiving funding from the state and 14 of which are run by NGOs, and SACs are located in every municipality.

Additionally, the National Centre for Information on Sexual Violence (*Nacionalinis informacijos apie seksualinį smurtą centras "Prabilk"*) runs a specialised helpline and provides information and guidance on available support services for individuals affected by or at risk of sexualised violence. The helpline is available Monday through Friday, 13:00–17:00. Both SACs and the specialised helpline provide preliminary legal advice and intermediary support with legal organisations. They also provide survivors with emotional support and refer them to professional psychological care.

👤 5. PRIMARY PREVENTION SERVICES

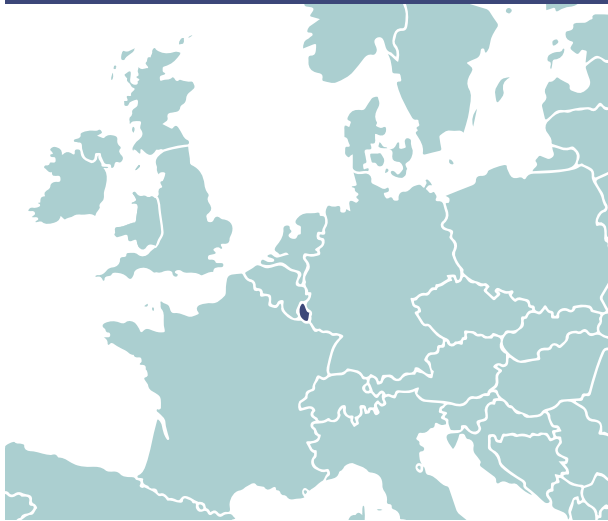
There is no systematised data collection on primary prevention in Lithuania, as the country's legal framework does not formally recognise gender-based violence as a distinct concept. Instead, various forms of violence are addressed in the Criminal Code or domestic violence law. Lithuania has several action plans and programs that include prevention elements; however, they focus on specific forms of violence and do not explicitly refer to primary prevention. These also do not include a definition of primary prevention nor is there a specific budget allocated to primary prevention activities on a national or local level. The primary prevention work of NGOs, if available, is financed through projects.

Available primary prevention activities in Lithuania include empowerment activities for girls and women, training for primary prevention practitioners, and awareness-raising campaigns or programmes. There is no comprehensive evaluation of national primary prevention activity impact, however, specific action plans do provide reports on their activities which are monitored by the Ministry of Social Security and Labour.

Official documents that include prevention are notably the Action Plan for the Prevention of Domestic Violence and the Provision of Support to Individuals Experiencing, Having Experienced, or Posing a Threat of Domestic Violence for 2024–2026. Its aim is to reduce the prevalence of domestic violence through enhanced prevention measures and improved support services. The plan outlines four key objectives: raising public awareness and education, strengthening the competencies of employees in state institutions and agencies, improving the quality and accessibility of support services, and fostering inter-institutional cooperation.⁷⁰ Additionally, the revised Law on Protection against Domestic Violence passed in 2023 holds municipalities responsible for the implementation of measures to prevent domestic violence and financing the work of NGOs working on preventing domestic violence.⁷¹

Furthermore, since 2023, the Ministry of Social Security and Labour has funded an NGO providing methodological support and organising activities aimed at preventing sexualised violence.⁷²

LUXEMBOURG



GENERAL COUNTRY INFORMATION

Population	672,050
Female population	333,776
Member of Council of Europe (year)	1949
Member of European Union (year)	1957
Member of United Nations (year)	1945
CEDAW ratified (year)	1989
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2018

SUMMARY

There is one national Domestic Violence Helpline accessible to all survivors of violence in Luxembourg. It is free of charge, offers multilingual support, but does not operate 24/7. In addition, the women's centre Femmes en détresse operates a women-only helpline on work-days. There are seven women-only shelters offering 196 beds and eight women's centres in Luxembourg. There is a sexual violence referral centre and crisis as well as medical services available for survivors of sexualised violence. Measures related to primary prevention are included in the National Action Plan on violence against women and girls, and the state finances such activities. The state collects some data on women's specialist services, which is publicly available.

⁷⁰ Minister of Social Security and Labour of the Republic of Lithuania, Action Plan for the Prevention of Domestic Violence and the Provision of Support to Individuals Experiencing, Having Experienced, or Posing a Threat of Domestic Violence for 2024–2026, 13 September 2023, No. A1-602, <https://e-tar.lt/portal/lt/legalAct/9ee69490820a11efabdbb4a1fc8b0b63>

⁷¹ Lietuvos Respublikos Seimas, Law of the Republic of Lithuania on protection against domestic violence no. XI-1425 Amendment Act, 15 March 2022, No. XIV-934, <https://www.e-tar.lt/portal/lt/legalAct/1e568520a9ee11ec8d9390588bf2de65>.

⁷² Minister of Social Security and Labour of the Republic of Lithuania, Order on the Approval of the Rules of Procedure for the Organisation of a Call for Proposals for the Organisation of a Project for the Provision of Methodological Support and the Organisation of Activities Aimed at the Prevention of Sexual Violence in 2023, 16 March 2023, No. A1-169, <https://www.e-tar.lt/portal/lt/legalAct/486c71a0c3ef11ed97b2975f7dad7488>.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	No
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

There is one national helpline in Luxembourg, the national Domestic Violence Helpline (*Helpline Violence Domestique*, tel. +352 20601060), which offers support to all survivors of violence, including women. The helpline was opened in 2020, during the first COVID-19 lockdown. It operates daily from 12:00–20:00, is free of charge and offers multilingual support in Luxembourgish, French, German and English. As it does not operate 24/7, it does not meet the standards of the Istanbul Convention for the provision of a national women's helpline. It is funded by the state and is run by the association *Fondation Maison de la Porte ouverte and Pro Familia*, which have signed an agreement with the Ministry of Equality between Men and Women to provide this service. Additionally, the women's centre Femmes en détresse operates a women-only helpline, available on workdays from 9:00–17:00.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	7
Total number of all shelters accessible to women	7
Existing number of beds	196
24/7 access	Some

Luxembourg has seven women-only shelters with a total of 196 beds. The capacity does not meet demand, as many women remain in the shelters for more than a year due to the shortage of affordable housing. Officially, stays are limited to three to six months, but in practice, the average stay is up to a year in some shelters and longer in most. Some shelters allow 24/7 access, but entry at night and on weekends is only possible with police accompaniment. Women with an income contribute a fixed percentage towards accommodation costs, while the state covers expenses for those without income. Shelters provide case-work, counselling and psychological support, referrals and cooperation with other services, as well as practical and legal advice.

Women-only shelters can be accessed by and are accessible to refugee women, women with cognitive and/or sensory disabilities, transgender women and women with older sons. Most shelters can accommodate homeless women, older women, and women with children with disabilities. Women with uncertain residence permits and undocumented women, women with physical disabilities and unaccompanied girls under the age of 18 can be accommodated by some shelters. Additionally, there are specific houses for all survivors of trafficking in persons,

which provide 24 beds in different cities of the country. No shelters can accommodate women with substance abuse issues.

Women-only shelters are funded by the Luxembourgish state and donations. Three women-only shelters are run by women's NGOs with a gender-specific and feminist approach, one by another NGO, and three by other entities. Two shelters are located in the South of Luxembourg, one in the North and the remaining ones in the centre. The reasons for declining referrals ranged from not having space and/or capacity to support the survivor and/or accommodate survivors with children to survivors being ineligible for support. The most common forms of violence reported by survivors were domestic, physical and psychological violence.

3. WOMEN'S CENTRES

Total number of centres	8
Entities running women's centres	Women's NGOs (4 centres) Other NGOs (1 centre) Other (3 centres)

There are eight women's centres in Luxembourg. Two of these centres operate for all survivors of human trafficking, one centre supports girls and young women up to the age of 21 (or up to 27 if they are students), one is an intervention centre for all survivors of gender-based violence, and the other centres support women survivors of intimate partner violence. All women's centres provide counselling and psychological support, referrals and collaboration with other services as well as residential support services. Seven centres in Luxembourg provide legal advice, financial and social welfare support and advocacy. Housing advice is given by six centres, two centres provide employment, and one women's centre can provide representation at court and other institutions. The women's centres offer only non-residential support services, but they can also organise and finance emergency housing accommodations.

All women's centres can be accessed by and are accessible to refugee women, women with physical, cognitive and/or sensory disabilities, as well as transgender women. Most centres are equipped to support women with uncertain residence permits and undocumented women, and older women. Moreover, two women's centres are specialised in supporting survivors of trafficking in persons.

Women's NGOs run four centres in Luxembourg, one centre is run by other NGOs and the remaining by other entities. Funding stems mainly from the state and donations. The women's centres cover most regions of the country, with one located in the North, two in the South and the remaining one in Luxembourg City. In 2024, 2,187 women were supported by women's centres in Luxembourg.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Luxembourg, there is one sexual violence referral centre and crisis as well as medical services available for women and girls who have experienced sexualised violence. They provide support such as specialist forensic and medical care, specialist psychological care, specialist sexualised violence advocacy services, as well as community awareness and education for prevention. The centre Umedo documents sexualised and physical violence and is open to both men and women. Moreover, all hospitals provide crisis and medical care. Funding for the centres is derived from the Luxembourgish State and donations. These services are located in major cities of Luxembourg and are available to all survivors of sexualised violence.

5. PRIMARY PREVENTION SERVICES

Luxembourg has a National Action Plan on violence against women and girls, which includes specific provisions on the primary prevention of violence against women, but no specific definition of primary prevention. The Luxembourgish State funds primary prevention activities. The activities available in the country are feminist self-defence trainings, school-based primary prevention programmes, training for primary prevention practitioners, bystander intervention training, self-care activities, awareness-raising campaigns, and prevention theatre sessions for young people. The Orange Week, a community event dedicated to combating violence against women and girls, offers additional events and workshops in line with the National Action Plan. In addition, there is a National Plan on sexual and emotional health, which focuses on the development of sexual and emotional education promoting the values of respect and equality between the sexes.

MALTA



GENERAL COUNTRY INFORMATION

Population	563,443
Female population	264,697
Member of Council of Europe (year)	1965
Member of European Union (year)	2004
Member of United Nations (year)	1964
CEDAW ratified (year)	1991
CEDAW optional protocol ratified (year)	2019
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2014

SUMMARY

There is no national women's helpline in Malta, but other helplines are available to survivors of different forms of crimes. There are three women-only shelters in the country and four other shelters accessible to women, with at least 90 beds available. In Malta, there are eight women's centres and one sexual violence referral centre. There is a National Strategy on Gender-Based Violence and Domestic Violence 2023–2028, which includes prevention measures. The state collects data on women's specialist services, which is available upon request, but there is a lack of coordination among different state bodies collecting data.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

There is no specialised national women's helpline in Malta, but two generic helplines that can provide support also to women survivors of gender-based violence. Therefore, the country does not meet the Istanbul Convention

standards for the provision of a 24/7 and free-of-charge specialised women's helpline. The first generic helpline available is the Support Line 179 (tel.: +356 179), which is run by the Foundation for Social Welfare Services. This helpline provides support, referrals and information to different groups of people, including survivors of domestic and intimate partner violence, sexualised violence, human trafficking and exploitation, stalking and harassment, child abuse linked to family violence, and psychological and economic violence. The second one is the Victims of Crime Helpline (tel.: 116006), which is run by the Victim Support Agency and covers all crimes, including different forms of gender-based violence. Both helplines are free of charge and available 24/7, and offer multilingual support primarily in Maltese and English, with interpretation services available for other languages. Funding comes from the state, and although helplines are not specifically for women, the staff receive training related to gender-based violence. In 2024, the most common form of violence reported to the Support Line 179 was domestic violence. Other frequently reported issues included child abuse and child protection concerns, often linked to family violence, followed by cases of sexual abuse and assault, as well as reports of human trafficking and exploitation. In 2024, this helpline received 14,838 calls from 11,068 callers.

Additionally, there are other helplines in Malta which can support survivors of violence against women, which are run by NGOs and targeted to specific groups. For instance, the Women's Rights Foundation helpline (tel.: +356 80062149) provides legal support about migrant women's rights and violence against women, and the LGBTIQ+ Wellbeing Service (tel.: +356 22959000) provides counselling for LGBTIQ+ individuals. The Kellmni.com helpline provides chat support to youth facing difficulties such as abuse, family conflicts and mental health issues, and the Life Line Malta (tel.: +356 99242490) supports women with pregnancy, parenting, emotional crises, and violence.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	3
Total number of all shelters accessible to women	7
Existing number of beds	90
24/7 access	Most

There are three women-only shelters in Malta, and four other shelters that are accessible to women survivors of violence, offering in total approximately 90 beds. Three of these are emergency shelters, while the others are second-stage shelters. Emergency shelters offer 24/7 access, and women are expected to pay for their accommodation if they can afford it; nevertheless, all women are accepted in the shelters and receive support regardless of their ability to pay. Access to shelters is often limited to women who meet specific criteria, such as filing a police report, holding legal status, or actively seeking help to leave abusive situations. There is a need for additional shelters and beds in

the country, especially for temporary accommodation for undocumented women and for those who need support in taking the first steps to escape abuse. In emergency shelters, women can stay for up to six months, while in second-stage shelters, they typically stay for more than a year. Women-only shelters provide different types of in-house services, such as casework, counselling and psychological support, referrals and collaboration with other services, and practical and legal advice.

All women-only shelters can be accessed by and are accessible to refugee women, women with physical disabilities and transgender women, while most shelters can also accommodate women with cognitive disabilities, and some are equipped to support women with substance abuse issues. There are no shelters which are tailored to the needs of specific groups of survivors.

Women-only shelters receive state funding through public social partnerships with NGOs. One women-only shelter is run by the state, while the other two women-only shelters are run by the church. Shelters are located in main cities, and due to the small size of the country, they can be reached by public transport. If women-only shelters had to decline referrals, the main reasons were the lack of space and capacity to support the survivor, the survivors being ineligible for support, or the shelters not being adequately equipped for the needs of women and children.

Besides the women-only shelters, there are other shelters in Malta that can be accessed by survivors. These include one homeless shelter run by the YMCA, where also women survivors of violence access to if they need accommodation, and a night shelter called Dar Papa Frangisku, which support all groups of women.

3. WOMEN'S CENTRES

Total number of centres	8
Entities running women's centres	Women's NGOs Other NGOs Police

There are both women-only centres and centres accessible to all survivors of violence in Malta, for a total of eight centres. Centres are run by different entities: one of them is run by a women's NGO, one state-run social work service supports survivors of domestic violence, one police-run centre supports survivors of crime, including domestic violence survivors, and other centres are run by NGOs.

Women's centres provide referrals, counselling, legal advice for survivors of domestic abuse, financial and welfare support, housing advice, employment, and representation at court/police. Four of the centres also provide residential support. One centre offers support for survivors of trafficking in persons, and one supports transgender women and non-binary people, including survivors of VAW. Funding for women's centres comes from state funding, donations,

and national projects. Women's centres are available in most regions of the country.

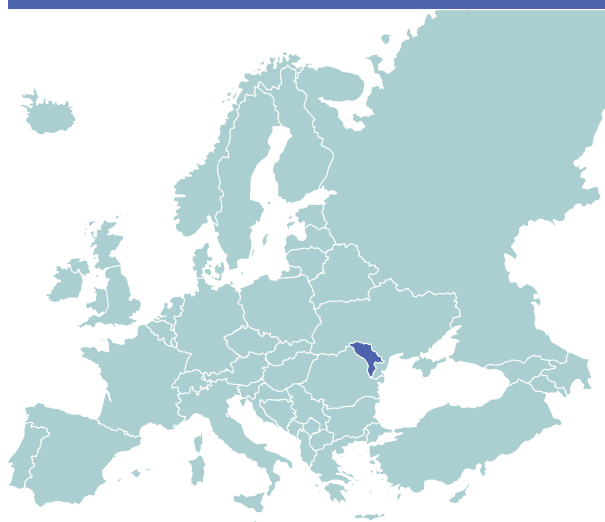
4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Malta, there is one sexual violence referral centre, the Care for Victims of Sexual Assault (CVSA) that is run by an NGO, Victim Support Malta. It operates 24/7 and provides forensic and medical care, needs assessment, psychological support, legal guidance, and referrals to police or shelters if needed. It supports all survivors of sexual assault. It is funded by the state and through donations.

5. PRIMARY PREVENTION SERVICES

Malta has issued its third National Strategy on Gender-Based Violence and Domestic Violence 2023 – 2028, called "Unite, Engage, Elevate". The strategy includes a chapter on prevention, but no specific reference to primary prevention. Measures addressed by the strategy include awareness-raising campaigns, educating boys and girls about gender equality and supporting the development of non-violent relationships, providing training programmes for professionals, and engaging the media and the private sector in addressing gender stereotypes.

REPUBLIC OF MOLDOVA



GENERAL COUNTRY INFORMATION

Population	2,423,287
Female population	1,293,798
Member of Council of Europe (year)	1995
Member of European Union (year)	No
Member of United Nations (year)	1992
CEDAW ratified (year)	1994
CEDAW optional protocol ratified (year)	2006
Istanbul Convention signed (year)	2017
Istanbul Convention ratified (year)	2022

SUMMARY

There is one national women's helpline in the Republic of Moldova, operating 24/7 and free of charge. There are 13 women-only shelters and two emergency rooms in the country offering 235 beds, as well as 16 women's centres and two sexual violence centres. Official documents refer to primary prevention, but with no budget specifically allocated to such measures. The state does not collect data on women's specialist services or prevention activities.

The region of Transnistria is presented in a separate section since the Moldovan government exercises no direct control over the territory of Transnistria. In Transnistria, there are two women's helplines which are free of charge, but not available 24/7, one women's shelter providing 12 beds for adults and two cot beds for children, 13 women's centres run by women's NGOs and no specialised services for survivors of SV, as well as no systemic primary prevention activities. The state does not collect data on women's specialist services.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	2,025 calls

There is one national women's helpline in the Republic of Moldova called Trust Line for Women and Girls (*Telefonul de Încredere pentru Femei și Fete*, tel.: +373 80088008), operating 24/7 and free of charge, thus meeting the standards of the Istanbul Convention. It offers multilingual support in Romanian, Russian, and, if necessary, in English. The helpline is run by the organisation La Strada and provides support for multiple forms of violence against women, including domestic violence, sexualised violence, psychological violence, economic violence, technology-facilitated violence, human trafficking and exploitation, stalking, and sexual harassment. It receives state funding and international funding, and in 2024, it was contacted 2,025 times by 1,104 women. The most commonly reported form of violence was physical violence, accounting for 65% of all cases, followed by psychological violence (22%), economic violence (7%), and sexualised violence (4%). Often, survivors face multiple, overlapping forms of abuse.

Additionally, the Law Women Centre runs a helpline providing free legal counselling, psychological and social support for women survivors of domestic violence and discrimination (tel.: +373 80080000). The helpline is not available 24/7, and it received 310 calls in 2024.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	13
Total number of all shelters accessible to women	13
Existing number of beds	235
24/7 access	Most

There are 13 women-only shelters in the country offering 235 beds, both in shelters and emergency rooms. This number does not meet the country's needs, particularly in rural areas. Additionally, some shelters operate at full capacity and cannot accommodate survivors for longer periods of time or handle complex cases. While most shelters and emergency facilities in the country are formally designated as 24/7 services, in practice, accessibility varies significantly by region. Many shelters face limitations, such as insufficient night/weekend staffing, inconsistent admission procedures outside working hours, and reliance on referrals that are not always working around the clock. The situation in Chișinău is better, as the capital hosts a specialised centre run by the General Inspectorate of Police, which offers emergency accommodation for up to 48 hours. This facility operates 24/7 and ensures immediate protection for survivors of violence. After the initial crisis

period, beneficiaries are referred to specialised shelters or support services for longer-term care, ensuring access to safety. A second emergency shelter in the capital is administered by the Ministry of Labour and Social Protection and offers emergency accommodation for up to three months. Nevertheless, more consistent implementation of 24/7 access across all shelters is needed, along with improved coordination between first responders, social services, and shelter providers. Shelter services for women and their children are provided free of charge. Most women stay in the shelters between three and six months, and extensions up to one year are possible, but available only in a few shelters. The length of stay varies depending on each shelter's capacity, available resources, and the individual needs of the women. Among the various in-house services provided by women-only shelters, counselling and psychological support emerge as the most predominant, followed by referrals and collaboration with other services, health-care, housing, employment assistance, social benefits, as well as legal advice. Some shelters offer casework and practical advice related to daily living and long-term reintegration, as well as child-focused support services.

Most women-only shelters can accommodate refugee women, older women and women with children with disabilities, while some shelters can also be accessed by and are accessible to women with uncertain residence permits, undocumented women, homeless women, women with physical and sensory disabilities, transgender women, women with older sons and unaccompanied girls under the age of 18. No shelters are accessible to women with cognitive disabilities. There are no shelters exclusively specialised in supporting specific groups of survivors, but some shelters and counselling centres can provide services tailored to specific groups.

Women-only shelters are funded through state funding, donations and international funding, and only one of them, Association "Casa Mărioarei", is run by a women's NGO with a gender-specific and feminist approach, while the others are run by other NGOs, religious organisations, public authorities and national institutions. Shelters and emergency facilities in Moldova are predominantly located in urban centres and regional hubs, which poses significant challenges for women and children living in rural and remote areas. While some regions have access to maternal centres or emergency rooms, many rural communities remain underserved due to limited geographic coverage, transportation barriers, and a lack of awareness about available services. In 2024, the main reason for having to decline referrals was the lack of space and capacity to accommodate survivors, especially those with children, as many shelters operate (close to) full capacity; additionally, another barrier was the lack of adequate facilities to support survivors with specific needs, e.g., with disabilities. For shelters run by NGOs, the lack of sustainable funding remains a major obstacle to expanding services. In 2024, the most common form of violence reported by 73% of survivors was psychological violence. Physical violence

was reported by approx. 40% of the survivors, while economic violence was reported by nearly 10% of them. In 2024, women-only shelters supported between 1,500 and 2,000 women and children. There are no other shelters in Moldova supporting survivors of gender-based violence.

❖ 3. WOMEN'S CENTRES

Total number of centres		16
Entities running women's centres	Women's NGOs (11 centres)	
	Other NGOs (1 centre)	
	State (4 centres)	

There are 16 women's centres in the Republic of Moldova, out of which ten are only for women, while the others support all survivors of gender-based violence. Women's centres in Moldova provide services such as legal advice, referrals and collaboration with other services, counselling, and advocacy. Many centres offer psychological support, including individual and group therapy, and empowerment and life skills workshops. Outreach and awareness-raising activities are common, targeting communities to prevent violence and reduce stigma. Some centres provide training on gender equality and violence prevention for professionals, including police, social workers, and healthcare providers. Additionally, several centres engage in policy advocacy to influence legislation and improve the overall response to violence against women. Additionally, women's centres also provide specialist support for survivors of sexualised violence, including specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention and legal advice.

In terms of accessibility, most centres are equipped to support women with uncertain residence permits, undocumented women, refugee women, and women with physical disabilities. Some centres are accessible to women with sensory disabilities, older women and unaccompanied girls under the age of 18. There are ten centres specialised in supporting girls under the age of 18, two centres specialised in supporting older women, and two centres specialised in supporting survivors (or those at risk) of trafficking in persons. Such centres provide expert counselling, legal aid, shelter, and empowerment programmes. Eleven centres are run by women's NGOs, one by another NGO, and four by the state. Funding comes from donations and international funding, and centres are mostly located in major cities. To address the geographical coverage gap, some centres have established regional branches or mobile outreach services to reach women in rural communities. However, challenges persist about transportation, awareness, and resource allocation. In 2024, between 15,000 and 20,000 women were supported by women's centres.

🚒 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In the Republic of Moldova, there are two rape and sexual violence crisis centres, and one specialised helpline for cases of sexualised violence. Forensic evidence in cases of sexualised violence is primarily collected in specialised medical institutions, such as the Forensic Medicine Centre in Chişinău, which operates with trained staff following trauma-informed protocols, as well as at the Family Justice Centre, subordinated to the General Inspectorate of Police, which operates 24/7 and provides all necessary services. Medical services collaborate closely with law enforcement and NGOs. Specialist services for survivors of sexualised violence in the country provide needs assessment, specialist psychological care, community awareness and education for prevention and legal advice. Funding for these services comes from state funding (the government allocates limited budgets to support state-run shelters, medical facilities, and some specialised centres), donations (many NGOs rely significantly on donations to sustain and expand their services), international funding (including the European Union, UN agencies, USAID, and other bilateral donors), and their work is further sustained through partnerships and volunteer work. These services support women, young women and children under 18. Specialised services for sexualised violence in Moldova are primarily concentrated in urban centres such as Chişinău and Ungheni. Access in rural and remote areas remains limited, and some NGOs and international partners have developed mobile outreach programs and established regional referral networks aimed at reaching survivors in remote locations. These initiatives provide critical first-line support, information, and referrals, but comprehensive services, including forensic examinations and specialised counselling, are often less accessible outside urban hubs.

👤 5. PRIMARY PREVENTION SERVICES

In the Republic of Moldova, although some progress has been made in systematising data related to the primary prevention of gender-based violence, public authorities still do not consistently collect it. Prevention efforts—such as awareness campaigns, gender education, and community engagement—are carried out mainly by civil society and international partners, with limited coordination and integration into national monitoring mechanisms. The Ministry of Labour and Social Protection and the National Agency for the Prevention and Combating of Violence Against Women and Domestic Violence occasionally report prevention initiatives; however, these are limited to ad-hoc activities and are not part of a structured, long-term approach based on partnerships involving multiple stakeholders. Official documents make reference to primary prevention but focus mainly on awareness-raising. The National Strategy for Preventing and Combating Violence Against Women and Domestic Violence (2018–2023) included prevention under Pillar I, but without providing evidence-based models or structural measures. National

Action Plans developed after Moldova ratified the Council of Europe's Istanbul Convention in 2021 also refer to prevention, but again, most actions prioritise communication rather than long-term interventions. Sectoral strategies in areas such as education or youth sometimes include gender equality or anti-discrimination elements, yet they lack a clear focus on preventing violence against women and girls. Moldova does not define primary prevention in law or in strategic documents. Although prevention is mentioned in policy frameworks, it is not distinguished from secondary or tertiary measures, and the concept remains underdeveloped. There is also no dedicated budget for primary prevention at the national, regional, or local levels.

Current activities include feminist self-defence courses, empowerment programmes for girls and women, initiatives dedicated to men and boys, optional school programmes, training for professionals, self-care activities, and general awareness-raising campaigns. However, the impact of these initiatives is not evaluated systematically. Public authorities do not coordinate or finance structured evaluations, and a national framework for monitoring effectiveness is lacking. As a result, evaluations are carried out on a small scale, mainly by NGOs within the projects they implement. Organisations such as the National Coalition "Life Without Violence", the Association Against Violence "Casa Mărioarei", the Women's Law Centre, the International Centre La Strada, and the Platform for Gender Equality have developed such evaluations, usually in line with donor requirements. Nevertheless, the findings remain fragmented, are not integrated into government systems, and have a limited impact on public policy development.

Primary prevention is mainly driven by women's organisations and civil society, often with international donor support. The National Coalition "Life Without Violence" and its members conduct gender education in schools, empowerment workshops, community campaigns, and training on positive masculinities. The association "Casa Mărioarei" is part of a regional project and has developed a programme specifically aimed at engaging men in challenging harmful gender norms.

TRANSNISTRIA

WOMEN'S SPECIALIST SERVICES

☎ 1. NATIONAL WOMEN'S HELPLINES

There are two state-wide women's helplines in Transnistria. The Free Informational Line on Domestic Violence in Transnistria (*Приднестровская платформа по предотвращению насилия в семье и продвижению прав женщин*, tel.: +373 80044000) is run by the Centre for Support and Development of Civic Initiatives "Resonance", it is free of charge, and it is available 24/7, meeting the standards of the

Istanbul Convention. The helpline does not offer multilingual support and assists survivors of domestic, physical, psychological, sexualised and economic violence. The other helpline is a trust line provided by the organisation Interaction (*Телефон Доверия по вопросам насилия в семье*, tel.: +373 80099800), which is free of charge, but it does not provide 24/7 support nor multilingual support. As the other helpline, it supports survivors of different forms of violence: domestic, physical, psychological, sexualised and economic violence. The helplines in Transnistria are supported by international funding, and in 2024, they received 2,029 calls. The most common forms of violence reported by callers were domestic violence, psychological violence and economic violence.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

There is one shelter in Transnistria providing 12 beds for adults and two cot beds for children up to the age of three. The shelter offers 24/7 support, and women are not expected to pay for their accommodation. On average, women can stay in the shelter up to one year, and the types of in-house services provided include casework, counselling, psychological support and legal advice. The shelter can accommodate women with uncertain residence permits, transgender women, women with disabilities, and women with children with disabilities. However, the shelter cannot accommodate women with substance abuse issues, as well as women with certain mental health issues that pose a risk to others.

The shelter receives international funding and is run by a women's NGO with a gender-specific approach. The shelter had to decline referrals due to the lack of space or capacity to support the survivor, if the survivors were ineligible for support, or in cases in which it was not adequately equipped to support the specific needs of the survivor. In such cases, the staff helps to find another organisation that can provide adequate support. The most common forms of violence reported by women were domestic violence, physical violence and psychological violence, but in most cases, women who seek support are victims of several types of violence at the same time. In 2024, the shelter supported 54 women. Additionally, in Transnistria, there is a maternity centre run by the Ministry of Health, Labour and Social Protection, which offers five beds, and there is a shelter for migrants.

❖ 3. WOMEN'S CENTRES

There are 13 women's centres in Transnistria, which provide counselling/psychological support, legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, representation at court/police/social services, and residential support services. There are women's centres specialised in supporting survivors (at risk) of trafficking in persons, women with uncertain residence permits and undocumented women, as well as refugee women. Women's

centres in Transnistria receive international funding, are present in all regions, and in 2024 supported 328 women.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no sexualised violence services in Transnistria, and survivors find support in women's shelters and centres.

5. PRIMARY PREVENTION SERVICES

There is no National Action Plan against violence against women available in Transnistria. Women's NGOs run primary prevention activities, in particular during the 16 days of activism against gender-based violence. Such activities include discussions with students, film screenings, advertising on local television about women's specialist services and awareness-raising campaigns.

MONTENEGRO



GENERAL COUNTRY INFORMATION

Population	623,680
Female population	316,854
Member of Council of Europe (year)	2007
Member of European Union (year)	No
Member of United Nations (year)	2006
CEDAW ratified (year)	2006
CEDAW optional protocol ratified (year)	2006
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2013

SUMMARY

There is a national women's helpline in Montenegro that operates free of charge and is available 24/7. There are four women-only shelters with 64 beds, seven women's centres, and one helpline specialised in sexualised violence. Although the National Plan for the implementation of the Istanbul Convention includes references to prevention, no budget is allocated, and state authorities do not conduct data collection and evaluation of such activities. The Ministry of Social Welfare, Family Care and Demography collects limited statistical data only on support services for women survivors of violence that receive state funding.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/ callers in 2024	2,144 calls 409 callers

There is a national women's helpline in Montenegro called the National SOS Hotline for Victims of Domestic Violence (*Nacionalna SOS linija za žrtve nasilja u porodici*, tel.: +382

80111111), which is run by the SOS Hotline for Women and Children Victims of Violence Nikšić. The helpline is free of charge and available 24/7, meeting the standards of the Istanbul Convention. It offers multilingual support in Montenegrin, Albanian, English, Russian and Ukrainian to survivors of all forms of violence against women, and the most common forms of violence reported by women in 2024 were domestic, physical and psychological violence. The helpline is run through state funding, and in 2024, it received 2,114 calls from 409 callers.

In addition to the National SOS Hotline for Victims of Domestic Violence, five additional local helplines can provide support to women survivors of violence. These include: SOS Hotline Podgorica, SOS Hotline Ulcinj, SOS Hotline Plav, SOS Hotline Bijelo Polje, and SOS Hotline Berane. The mobile application Be Safe, which was launched during the COVID-19 pandemic, is no longer active.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	4
Total number of all shelters accessible to women	4
Existing number of beds	64
24/7 access	All

There are four women-only shelters in Montenegro, offering 64 bed spaces. They offer 24/7 access, and women are not expected to pay for their accommodation. On average, women can stay in the shelters up to one year, but shelters can decide to extend the length of the stay based on the needs assessment. The types of in-house services most often provided by women-only shelters are case-work, counselling and psychological support, referrals and collaboration with other services, as well as practical and legal advice. Additionally, survivors can be accompanied to the police, social services, courts, as well as other institutions such as healthcare and educational facilities. This service is not limited solely to proceedings directly related to violence against women, but also includes all institutional procedures involving survivors, such as divorce proceedings, custody cases, child support, and visitation arrangements.

In terms of accessibility, out of the four shelters, one is not accessible to undocumented women, refugee women, homeless women and women with sensory disabilities, while another shelter is not accessible to women with physical disabilities, with sensory disabilities, to older women and women with children with disabilities. On the other hand, all shelters can be accessed by and are accessible to women with substance abuse issues and transgender women, while no shelter can accommodate women with older sons over the age of 14 and unaccompanied girls under the age of 18. There are no shelters in the country specialised in supporting particular groups of survivors.

Women-only shelters are funded by national/local authorities, donations, and international funding. Two of them are run by women's NGOs with a gender-specific approach, while the other two are run by other women's NGOs. Shelters are located in the central and southern regions of Montenegro. In the northern region, there are no shelters for women survivors of violence and domestic violence, as the shelter which was operating in Pljevlja is no longer operating. In some cases, shelters had to decline referrals due to the lack of space, and the women and children were redirected to other shelters that could accommodate and support them. The most common forms of violence reported by survivors were domestic, physical and psychological violence. In 2024, women-only shelters in Montenegro supported 154 women. There are no other shelters in the country that support survivors of violence against women.

3. WOMEN'S CENTRES

Total number of centres	7
Entities running women's centres	Women's NGOs (7 centres)

There are seven women's centres in Montenegro, supporting women and children who are survivors of violence. Women's centres provide counselling, psychological support, legal advice, housing advice, referrals and collaboration with other services, as well as representation at court/police/social services. Additionally, women's centres also support survivors of sexualised violence with legal advice, community awareness and education for prevention.

Although detailed accessibility information is not available for some women's centres, most centres can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive and sensory disabilities, older women, transgender women, and unaccompanied girls under the age of 18. There is one centre for girls and women survivors of forced/arranged marriage, operated by the NGO Centre for Roma Initiatives. In 2024, this NGO identified 26 potential victims of arranged marriage. There is also one shelter for women survivors of human trafficking, operated by the NGO SOS Hotline Nikšić. In 2024, this NGO identified six potential victims of human trafficking.

Women's NGOs run the centres in Montenegro; they receive state funding and international funding, and are present in all regions. Four centres that provided statistical data supported 815 women in 2024.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

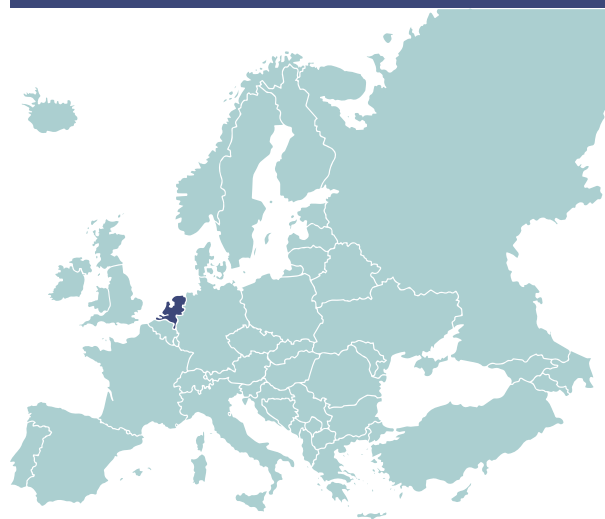
There are no specific support services, such as rape crisis centres for women and girls who have experienced sexualised violence, in Montenegro. There is a helpline providing specialist support for survivors of sexualised violence,

which is run by the NGO Montenegrin Women's Lobby (tel.: +382 20250750; mobile: +382 67998977).

5. PRIMARY PREVENTION SERVICES

In 2023, Montenegro adopted a National Action Plan for the implementation of the Council of Europe Convention on Preventing and Combating Violence Against Women and Domestic Violence (Istanbul Convention) for the period 2023–2027. The National Plan foresees efforts to conduct training for professionals and awareness-raising programmes about all forms of violence covered by the Convention, including reference to specific groups of women, such as Roma women and women with disabilities. Nevertheless, no specific budget line is allocated to such activities. The state did not engage in scaling up primary prevention measures to tackle violence against women, and there are no long-term campaigns and initiatives in the country. Primary prevention activities are carried out by women's NGOs based on projects usually funded by international donors. Such activities⁷³ include awareness-raising campaigns on topics such as sexism and different forms of gender-based violence, and training for professionals, students and young people. There is no evaluation on the impact of primary prevention activities in the country, nor data collection on the activities conducted.

THE NETHERLANDS



GENERAL COUNTRY INFORMATION

Population	17,942,942
Female population	9,022,174
Member of Council of Europe (year)	1949
Member of European Union (year)	1952
Member of United Nations (year)	1945
CEDAW ratified (year)	1991
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2015

SUMMARY

The Netherlands does not have a national women's helpline although there is a helpline for all survivors of domestic violence and child abuse which is available 24/7 and free of charge. There are also other helplines that can support women survivors of violence, including regional helplines that are run by four women-only shelter organisations. There are 20 women-only shelters with 986 beds as well as other shelters accessible to women, although the exact number is not known. There are no specialised women's centres in the Netherlands, however, all 20 women-only shelters also provide non-residential support. There are services for survivors of sexualised violence, including 16 rape crisis centres which operate as Sexual Assault Centres. Public authorities in the Netherlands do not collect data on primary prevention activities on a national level and there are no official national documents on primary prevention. A definition of primary prevention is included in the Plan "Stop Femicide!" although no specific budget is allocated to such activities. The state collects limited data on women's specialist services in order to monitor the impact and spending of public funding to those services. Data is also collected nationally by some networks and is made publicly available.

⁷³ Information from GREVIO's First thematic evaluation report "Building trust by delivering support, protection and justice – MONTENEGRO", published on 22 November 2024.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

There is no national women's helpline in the Netherlands, but there is a national helpline for domestic violence and child abuse, Safe Home (*Veilig Thuis*, tel.: +31 8002000) since 2015. While the helpline is available 24/7 and is free of charge, it provides support to all survivors of violence as well as for professionals who want to report cases of child abuse and/or domestic violence. Therefore, the Netherlands does not meet the Istanbul Convention standards for the provision of a national women's helpline. Safe Home operates from a predominantly gender-neutral framework, although awareness of the gender-based nature of violence has increased over the last two years. The helpline is an initiative of the Dutch government and was originally formed by merging the former Advice and Report Offices on Child Abuse and the former Support Centres on Domestic Violence. The 25 offices of *Veilig Thuis* work under a national helpline number and are run by regional NGOs who know the local networks and can refer survivors to the appropriate services. In three regions, *Veilig Thuis* is closely related to/part of a women's shelter organisation. There is the possibility to use the national interpretation centre, which is funded by the state and decentralised via municipalities, to receive support in any language when calling the helpline. The National Network of Women's Shelters, Valente, has been and still is campaigning for an independent women's helpline, supported by the Federation of Surviving Victims of Violence (*Federatie Nabestaanden van Geweldsslachtoffers*), Fier and several other women's organisations. In 2024, the 25 offices of *Veilig Thuis* collectively received 153,910 requests for advice and 129,220 reports on various forms of domestic violence. In total, *Veilig Thuis* received 232,724 calls. The most common forms of violence reported by callers in 2024 were domestic violence, physical violence, and violence directly/indirectly affecting children.

There are other helplines in the Netherlands that can support women survivors of violence, including regional helplines that are run by four women-only shelter organisations, which are available 24/7 in cases of emergency. These include *Fier* (tel.: +31 882080000), Strong House (*Sterk Huis*, tel.: +31 8008013), Stay Group (*Blijf Groep*, tel.: +31 882342450), and *Moviera* (tel.: +31 883744744).

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	20
Total number of all shelters accessible to women	N/A
Existing number of beds	986 ⁷⁴
24/7 access	All

There are 20 women-only shelters in the Netherlands with 986 beds for women and their children.⁷⁵ There are also 38 beds for male survivors of domestic violence, also operated by women-only shelters, but in different locations. As reliable data on the number of beds in women-only shelters has been lacking for years, Valente and the National Association of Municipalities (VNG) took the initiative to develop a national monitoring of shelters, financed by the Ministry of Health. The monitor provides data on the number of beds available, the duration of stay as well as some additional information. One of the aims of this project is to be better able to assess the need for more beds based on the monitoring of experiences and demand over time.⁷⁶ There are large differences between municipalities in terms of the number of women-only shelter beds per inhabitant. Interestingly, three of the four largest cities (Amsterdam, Utrecht, Rotterdam and The Hague) have more beds than the average in the Netherlands. This also applies to some smaller municipalities near those four cities. In 2023, 20 women-only shelter bed spaces were added in the Netherlands; this is equivalent to a 2% expansion of the total national amount. These shelters are all part of Valente, and every region has emergency or acute crisis beds available 24/7 for one to three nights. In general, most women-only shelters can be accessed 24/7.

Women are expected to pay for their accommodation in women-only shelters, these costs are however paid for through social welfare/income from benefits which the women-only shelters help survivors receive from the municipalities in order to pay for their expenses as well as those of their children. Women who already have (enough) income from their salary or other means must cover the costs for accommodation from their income. In no case are the shelter accommodation costs a barrier to accessing women-only shelters. If a woman is undocumented or has an irregular legal status, the costs are covered by the municipality and/or it is accepted as a 'loss of income' on the budget of the women-only shelter by the municipality, although these arrangements differ between regions. On average, women can stay in women-only shelters for three to six months. Averages can however vary depending on services provided by different shelters as well as specific funding conditions of municipalities. Based on the recent monitoring of women-only shelters (figures from 2022/2023), women stay in emergency beds on average 12 days, 64 days in crisis shelter beds, and 259 days in shelter beds for longer stays (approx. eight months). The

⁷⁴ This includes only the number of beds in women-only shelters.

⁷⁵ These figures are based on the years 2022 and 2023. The numbers for 2024 will be available only in 2026.

⁷⁶ This monitoring was published for the first time in 2025 (with data from 2022 and 2023) and is scheduled to repeat every year.

types of in-house services most often provided by women-only shelters are counselling, referrals and practical advice. Counselling and support in enhancing safety and minimising risks of recurring violence are the basis of all services, as well as working with other specialised services to diminish risk factors.

All women-only shelters can be accessed by and are accessible to refugee women, women with cognitive disabilities and/or sensory disabilities, older women (65 years and above), transgender women, women with older sons (14 years and above), and women with children with disabilities. Most shelters are equipped to support women with uncertain residence permits/undocumented women, women with physical disabilities, and women with substance abuse issues. Some shelters can accommodate homeless women and unaccompanied girls under the age of 18. Additionally, some members of Valente offer specialised shelter facilities for undocumented women and LGBTQI+ people. There are specialised shelters for women survivors of trafficking and for women and girl survivors of honour-based violence. All shelters accept women that have been in marital captivity⁷⁷ if there is acute violence and/or safety concerns.

Funding for women-only shelters comes from the state and all shelters are run by women's NGOs with a gender-specific and feminist approach. Thirty-five centre municipalities have been appointed that collaborate with the other regional municipalities in their area in the Netherlands. These centre municipalities receive financial resources from the Ministry of Health, Welfare and Sport which then goes (indirectly) to the women-only shelters. Centre and regional municipalities can, and in the majority of cases do, contribute extra funding to the women-only shelters for ambulatory care and sometimes for more beds and other services. With increasing living costs, there is a rising concern that funding used for women-only shelters might also be used for other purposes or go to other organisations in the field of domestic violence, such as non-specialist services. There are women-only shelters in all regions, some shelter organisations have more than one location and cover several regions, while others have just one location. The three main reasons for having to decline referrals to women-only shelters were no space to support the survivor, no capacity to support the survivor with her children, and the survivor being ineligible for support. Spaces in shelters are generally reserved for the most complex and unsafe situations; non-residential specialised services have been developed to assist women who want to escape their abusive situation but have other options than going to a women-only shelter. A safety assessment is always conducted to determine whether

this is a safe option. The most common types of violence reported by survivors in 2024 were intimate partner violence, often combined with patterns of stalking, and a mix of physical, psychological, and sexualised violence. Due to the policy of reserving shelter beds for the most complex and unsafe situations, coercive control involving different forms of (domestic) violence are usually reported in women-only shelters. In 2023, 2,934 women and 2,622 children were accommodated in women-only shelters.⁷⁸

There are other shelters in the Netherlands which can support survivors of violence, most of which offer 24/7 access. This includes three shelter organisations which participate in the meetings of the National Network of Women's Shelters and provide shelter accommodation for families, women and children, and people from the LGBTQI+ community that have lost their house/living arrangement due to family issues, sometimes linked to (risks of) domestic violence. The exact number of beds is not known or monitored. There are also more organisations that provide shelter accommodation specifically for women, girls and men as well as boys survivors of human trafficking (criminal or labour exploitation). The exact number of such shelters is not known, although specific research on the number of beds for survivors of human trafficking is expected in late 2025.

❖ 3. WOMEN'S CENTRES

Total number of centres	20
Entities running women's centres	Women's NGOs (20 centres)

All of the 20 women-only shelter organisations which are members of Valente provide both residential and non-residential services, such as individual or group counselling. All services are predominantly but not exclusively for women that experience domestic violence. Most centres also offer services to male survivors of domestic violence and LGBTQ+ survivors, and some also offer services to survivors of human trafficking. All women's centres offer counselling and psychological support as well as referrals and collaboration with other services. Most centres do not provide legal advice themselves but inform survivors about specialist legal services in the area with whom they collaborate, and this also goes for financial and social welfare support and housing advice. Only a limited number of centres offer services or programs on employment, as structural funding for these programs is limited.

All centres can be accessed by and are accessible to refugee women, women with physical disabilities, cognitive disabilities, and/or sensory disabilities, older women (65 years and above) and transgender women. Some centres

77 Marital captivity is a situation in which a woman in a religious marriage needs her husband's permission to divorce him, but he refuses to cooperate. In these situations, a secular court may have dissolved the civil marriage, but the religious marriage continues to exist. As long as the wife is tied to her religious marriage, she is seriously hampered in her participation in society. Marital captivity often occurs jointly with other forms of both domestic and honour-related violence. The organisation Femme for Freedom has more information available here: <https://www.femmesforfreedom.com/international/what-is-marital-captivity>.

78 Exact figures for 2024 are not available but according to Valente, in 2024 shelters accommodated 3,434 women and men (with over 90% being women) and 2,615 children.

are equipped to support unaccompanied girls under the age of 18. Most of the women's support centres are open to women from different groups and have received specialised training to work for example with elderly women (or men) and women with cognitive disabilities. Some have set up special consultation hours for specific groups, but a complete overview of specialised support offered is lacking. There are two specialised centres for survivors of honour-based violence as well as a specialist unit for case management, all other centres provide first care and risk assessment on all forms of domestic violence including honour-based violence. The National Contact Point for Forced Marriage and Abandonment (*Landelijk Knooppunt Huwelijksdwang en Achterlating*) is a specialised centre which exists since 2015 and provides information, advice and support for professionals dealing with cases of forced marriage and abandonment. This centre collaborates with both women-only shelters (if shelter is needed upon return in the Netherlands after abandonment), the national helpline and other relevant stakeholders.

All or most of the funding for women's centres comes from the appointed 35 centre municipalities, a (modest and varying) amount comes from donations by foundations and trusts and is generally aimed at specific projects. In the Dutch decentralised system, the Ministry of Health, Welfare and Sport finances 35 municipalities that provide women-only shelters as well as non-residential services offered by women's centres. The 35 centre municipalities collaborate with the other municipalities in their region and have a coordinating role. Both centre and regional municipalities contribute extra funding to the women's shelters for their non-residential support services. All centres are run by women's NGOs and are located in all regions of the Netherlands. Based on data from the annual reports of some of the largest members of Valente, it can be estimated that approximately 10,000 women are supported yearly by the 20 women's centres, with rather large margins between the larger member organisations that can support up to 1,800 women a year and the smaller ones that might support only 20 to 40 a year.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for women and girls who have experienced sexualised violence in the Netherlands, including 16 rape crisis centres that operate as Sexual Assault Centres. Support offered includes specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice. All centres run their own regional specialised helpline (tel.: +31 8000188). At the Sexual Assault Centres, a team of doctors, nurses, police officers, psychologists, social workers and sex therapists work together to provide specialist care to survivors of sexual assault and rape. Ideally,

help is provided within seven days after the event, as this timeframe reduces the chances of medical and psychological issues considerably and gives police a better chance of finding the perpetrator if forensic investigations are carried out. Survivors are met by a (forensic) nurse who will stay with them for the duration of their visit. Services are provided in such a way that survivors do not need to repeat their story more than necessary. The Sexual Assault Centres are coordinated nationally; the national office also develops tools and policies that apply to all of the 16 centres. Funding for sexualised violence support services comes from the state and they are located in all regions. Services are available to all survivors of sexualised violence, including non-residents visiting the Netherlands.

5. PRIMARY PREVENTION SERVICES

There is no comprehensive data collection on primary prevention activities in the Netherlands and there are no official national documents on primary prevention. A definition of primary prevention is included in chapter two of the Plan "Stop Femicide!"⁷⁹ as well as a list of primary prevention activities undertaken by different ministries. A coherent strategy and the impact of these initiatives in relation to each other and of efforts in different domains is however lacking. The prevention of sexualised violence is the core of the National Action Plan on Sexual Transgressive Behaviour and Sexual Violence (2022-2026). A Government Commissioner was appointed by the Ministers of Education, Culture and Science and of Social Affairs and Employment in 2022, who is pushing for a preventive approach in the National Action Plan on Sexual Transgressive Behaviour and Sexual Violence.

There is no specific budget nor for the execution of the Plan "Stop Femicide!" nor for primary prevention activities. A budget has been allocated to the Commissioner on Tackling Sexually Transgressive Behaviour and Sexual Violence, and the four participating Ministries invest on preventive measures from their regular budgets and some project funding. Smaller women's organisations have argued that the funding requirements from these alliances are such that they can hardly participate in those tenders. Budgets on regional and local levels allocated to primary prevention vary and are not structurally embedded, meaning they are subject to local political changes. Available primary prevention activities include feminist self-defence trainings, empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, bystander intervention training, and awareness-raising campaigns or programmes. There is no structural evaluation of the impact of primary prevention activities, only small-scale reports and evaluation but data on this is not collected or assessed in a thorough manner.

79 Available here: <https://open.overheid.nl/documenten/7428c2b8-01fb-4ff9-99d2-a86b789daf39/file>.

NORTH MACEDONIA



GENERAL COUNTRY INFORMATION

Population	1,826,247
Female population	921,508
Member of Council of Europe (year)	1995
Member of European Union (year)	No
Member of United Nations (year)	1993
CEDAW ratified (year)	1994
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2017

SUMMARY

There are three national women's helplines in North Macedonia operating 24/7, while one of them also operates free of charge and offers multilingual support, although with limited availability. There are 14 women-only shelters with a total of 67 beds, as well as two other shelters. In North Macedonia, there are 11 women's centres and three Sexual Violence Referral Centres. Public authorities collect data on primary prevention as part of a broader gender-based violence monitoring system, but there is no specific budget allocated for it. The state collects data only on support services that are licensed by the Ministry of Social Policy, Demographics and Youth, therefore excluding many women's specialist services which are run by women's organisations.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	3
Free of charge	One yes, two no
24/7 service	Yes
Multilingual support	One yes, two no
Total number of calls	760 calls
callers in 2024	360 callers ⁸⁰

80 Numbers of calls and callers refer to the Free SOS Mobile National Helpline.

81 Number of beds in the women-only shelters.

There are three national women's helplines in North Macedonia. The Free SOS Mobile National Helpline (*бесплатна СОС мобилна национална линија*, tel.: +389 141700) is run by the National Council for Gender Equality. It operates free of charge and is available 24/7; therefore, it meets the standards of the Istanbul Convention for the provision of a helpline. It offers support in Macedonian, Albanian, Roma and Turkish, although multilingual support is not available 24/7, but only when an operator who speaks one of these languages is on duty. The helpline supports survivors of various forms of violence against women, including domestic and intimate partner violence, sexualised violence, psychological, emotional and economic abuse, as well as stalking and cyberstalking. Main funding sources are the state and donations. In 2024, 760 calls by 360 callers have been registered, and the most common forms of violence reported were domestic, psychological and economic violence. The other two national women's helpline are the National SOS Line – Telephone of Trust (*Национална СОС линија – Телефон на доверба*, tel.: +389 15315), which is run by the Crisis Centre Hope, and the National SOS Line 15 700 (*Национална СОС линија 15 700*, tel.: +389 (02)15700), which is run by the Women's organisation of the city of Skopje. These helplines provide 24/7 support to survivors of domestic violence but do not operate free of charge.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	14
Total number of all shelters accessible to women	16
Existing number of beds	67 ⁸¹
24/7 access	All

There are 14 women-only shelters in North Macedonia, providing 67 beds. All shelters can be accessed 24/7 through police assistance or SOS helplines. Only 11 crisis centres offer 24/7 access, and women can find accommodation only if they report the violence to a centre for social welfare. Accommodation is free of charge for women survivors of violence. The permitted length of stay depends on the type of shelter. In crisis centres, women may stay up to 72 hours, while in long-term shelters, the standard period is three to six months, with the possibility of extension up to one year. In practice, most women stay for around three months. Women-only shelters most often provide casework, counselling and psychological support, referrals and collaboration with other services, as well as practical advice.

Women's shelters in North Macedonia are not intended to accommodate homeless women, who can be hosted in a specific shelter managed by the Ministry of Social Policy, Demography and Youth, with the support of the Red Cross of Skopje. However, if a homeless woman is also a survivor of gender-based violence, she can be accommodated in a women's shelter upon reporting to the competent

institution. Most shelters can be accessed by and are accessible to women with physical disabilities and women with children with disabilities. Some shelters are equipped to support women with uncertain residence permits and undocumented women, as well as refugee women. There are still no shelters equipped to support women with cognitive and/or sensory disabilities, women with substance abuse issues, transgender women and women with older sons. The shelter for survivors of trafficking in human beings also accommodates survivors of sexualised violence.

Women-only shelters are funded by the state, donations and international funding. Three shelters are run by women's NGOs with a gender-specific approach, while the others are run by the state and other NGOs. Shelters are located only in major cities of North Macedonia, and not every region provides such services. If shelters had to decline referrals, the main reasons were the lack of space and capacity to support survivors, or the shelters not being accessible due to, for instance, technical issues like flooding. The most common forms of violence reported were domestic, physical and psychological violence. In 2024, approximately 60 to 70 women and children were accommodated in shelters. Additionally, there are two other shelters accessible to women in the country: one supporting survivors of trafficking and one for LGBTQI+ people. The safe house for LGBTQI+ people was closed between February and September 2025 due to cuts in USAID funding.

❖ 3. WOMEN'S CENTRES

Total number of centres	11
Entities running women's centres	Women's NGOs (11 centres)

There are eleven women's centres in North Macedonia, with five being only for women survivors of violence against women and the others for all survivors of gender-based violence, including women. Women's centres in North Macedonia primarily provide counselling and psychological support, as well as referrals to other services. Most of the centres operated by the State are not fully functional, due to a lack of human resources. State-run centres do not directly provide legal assistance; instead, women are referred to the system of free legal aid, in line with the Law on Free Legal Aid. Representation at courts, police and social services is carried out only by NGO-run women's centres, which also provide specialist psychological care and legal advice for survivors of sexualised violence. State-run centres rarely provide psychotherapeutic services. Such services can primarily be accessed in counselling centres run by women's NGOs, where survivors receive accompaniment by a social worker, longer-term psychotherapeutic support, legal aid and the possibility of legal representation by a lawyer.

There are no specialised women's centres in North Macedonia dedicated exclusively to older women or to girls under 18. Some centres can be accessed by and are accessible

to women with uncertain residence permits and undocumented women, refugee women, women with physical disabilities and transgender women. There are no centres equipped to support women with cognitive disabilities.

The main funding sources are the state, donations and international funding. Women's centres are located in major cities in the country, and in 2024, approximately 750 women were supported by all women's centres. Additionally, there is another centre in North Macedonia that provides support both to survivors and to perpetrators of gender-based violence. Supported housing for women survivors of violence is offered in Skopje, Tetovo and Bitola through the Women Support Centres in these cities, all run by the National Network to End Violence Against Women and Domestic Violence. The centre in Bitola is fully supported by the local municipality, the centre in Tetovo receives a combination of funds from the local municipality and the Ministry for Social Policy, Demographics and Youth, and the centre in Skopje is supported by the Ministry for Social Policy, Demographics and Youth.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

North Macedonia has three sexual violence referral centres, located in Skopje, Tetovo, and Kumanovo, all of which are situated within healthcare facilities. The centres provide specialist forensic and medical care, psychosocial support, and needs assessments. They also offer legal advice, though free legal aid is limited. Survivors are entitled to free healthcare and psychosocial care upon arrival. However, healthcare access may be jeopardised for survivors without health insurance, as the Ministry of Health has not allocated specific funds for this purpose. Forensic evidence is collected by a forensic medical specialist within the centres, but only after an order is issued by the prosecutor. This requirement often leads to inconsistencies, delays, and re-victimisation, which in turn contribute to survivors withdrawing from proceedings. Centres are also required to notify the police, prosecutor's office, and social welfare services when the case involves minors or for specific types of violence.

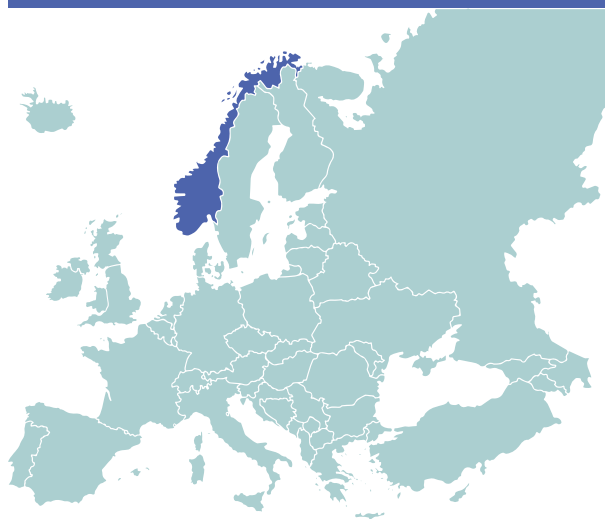
The sexual violence referral centres operate within the general healthcare system and lack dedicated state funding. The prosecution office usually covers the costs of evidence collection through the Institute of Forensic Medicine. Overall functioning is hindered by the absence of allocated funds for these services. Geographically, the centres are only available in the capital and two major cities. Survivors from rural areas are usually referred to Skopje. The centres provide support both to adult women and to girls under 18.

☞ 5. PRIMARY PREVENTION SERVICES

Public authorities collect data on primary prevention as part of a broader gender-based violence monitoring

system. North Macedonia does not currently have an official document dedicated to primary prevention. The previous National Action Plan (2018–2023) included a goal on the prevention of violence against women and domestic violence and defined prevention, though not primary prevention. A new eight-year National Strategy for the implementation of the Istanbul Convention is under preparation. There is no specific budget allocated to primary prevention in the country. Preventive activities are scattered across state and local budgets within broader programmes, such as those on social protection, equal opportunities, or local economic development. Available activities include empowerment programmes for girls and women, school-based programmes, and awareness-raising campaigns. An evaluation of the 2018–2023 National Action Plan was conducted by the Ministry of Social Policy, Demography and Youth, and monitoring was also carried out by the National Network to End Violence Against Women and Domestic Violence. These evaluations were conducted at the national level.

NORWAY



GENERAL COUNTRY INFORMATION

Population	5,550,217
Female population	2,754,489
Member of Council of Europe (year)	1949
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1981
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2017

SUMMARY

There is one national women's helpline in Norway which is free of charge, available 24/7, and offers multilingual support. Additionally, there are other helplines in Norway supporting survivors of specific forms of violence. Norway does not have any women-only shelters, but there are 41 shelters/crisis centres providing shelter accommodation which are accessible to women survivors of violence, offering an estimated 969 beds. All 41 crisis centres also offer non-residential support, and there are number of specialised services for specific survivors of violence. There are services for survivors of sexualised violence, including 22 sexual violence referral centres, two rape crisis centres, and 21 Centres for Victims of Incest and Sexual Abuse (SMISO/NOK). Norway has a National Action Plan addressing violence against women and girls, which includes specific provisions on primary prevention. The state collects data on women's specialist services, including primary prevention initiatives, and this information is publicly available. The Norwegian Directorate for Children, Youth and Family Affairs (*Bufdir*) provides annual statistics for both the crisis centres and Centres for Victims of Incest and Sexual Abuse.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	5,137

There is one national women's helpline in Norway called the VO-line-National Domestic Violence Helpline (*Vold-og overgrepslinjen*, tel.: 116 006) which is operated by the Secretariat of the Shelter Movement and *Indre Østfold Crisis Centre*. VO-line is also available via chat on weekdays from 9:00–20:00. The helpline is funded by the state, available 24/7 and offers support in Norwegian and English. It provides support for anyone experiencing domestic violence as well as relatives or friends of survivors and those working with survivors of domestic violence. In 2024, the helpline received 5,137 contacts (872 of callers were women and 126 were men) of which 3,439 were calls and 1,698 were chat requests. The most common forms of violence reported by survivors to the helpline were physical and psychological violence. Norway does meet the Istanbul Convention minimum standards for the provisions of a national women's helpline. Oslo Crisis Centre also run the Young Relations (*Unge Relasjoner*) chat for young people aged 16 to 25 who are in unhealthy and destructive relationships. The chat is free of charge, available Tuesdays from 12:00–20:00 and Fridays from 12:00–15:00 and offers support in Norwegian and English.

There are other national helplines in Norway supporting survivors of specific forms of violence. The helpline ROSA: Re-establishment – Places of Residence – Security – Assistance (*Reetablering – Oppholdssteder – Sikkerhet – Assistanse*, tel.: +47 22331160) provides support to survivors of trafficking in Norwegian, English, and Spanish. Furthermore, they offer guidelines and flyers in Polish, Russian, Vietnamese, Thai, Bulgarian, Romanian, Arabic, English, Spanish, and Norwegian. The helpline is free of charge and operates 24/7. In addition, Red Cross Norway operates a specialised helpline (tel.: +47 81555201) for survivors of forced marriage, female genital mutilation and honour-based violence. This helpline is available on weekdays from 9:00–16:00. The Expert Team for the prevention of forced marriage, female genital mutilation and negative social control (*Kompetanseteamet mot negativ sosial kontroll og æresrelatert vold*) also operates a helpline (tel.: +47 47809050). The helpline is available on weekdays from 9:00–15:00 and provides expertise in the fields of migration, child welfare, health, security (police), immigration law, as well as labour and welfare services (the Norwegian Labour and Welfare Administration – NAV).⁸² All crisis centres in Norway also operate a helpline for all survivors of violence looking for support.

tres in Norway also operate a helpline for all survivors of violence looking for support.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	0
Total number of all shelters accessible to women	41
Existing number of beds	969 ⁸³
24/7 access	All

There are 41 shelters accessible to women in Norway, referred to as crisis centres. There is no updated data on the number of available beds, based on the last available figures from 2019, the state estimated there were 969 beds in total. Approximately half of these shelters are part of the Secretariat of the Shelter Movement (*Krisesentersekretariatet*), the umbrella network of crisis centres for survivors of domestic violence and abuse. All shelters offer 24/7 access, and women are not required to pay for their stay. However, they are expected to provide their own food, clothing, and toiletries. Norwegian law does not specify a maximum duration of stay for survivors of domestic violence in order to ensure their safety; it only states that the length of stay should not exceed what is necessary. In some municipalities, funding is highly limited, which impacts the quality, resources, safety, and staffing of the services provided. The most common services offered by crisis centres in Norway include case management, counselling/psychological support, referrals and collaboration with other services, practical advice, and legal counselling. Although the Crisis Center Act (2010) states that accommodation for women and men should be separate, women's organisations have noted that, in some cases, women and men are housed in the same facilities but with separate entrances.

Some shelters can be accessed by and are accessible to women with uncertain residence permit and undocumented women, refugee women, women with sensory disabilities, and women with substance abuse issues. Twenty-nine shelters are equipped to support women with physical disabilities, while 10 can support women with psychosocial disabilities and eight women with cognitive disabilities. According to Norwegian authorities, some shelter accommodation is available in five municipalities for adults who are exposed to forced marriage and honour-based violence, with around 30–40 bed spaces. The Secretariat of the Shelter Movement is also responsible for running ROSA, which is a helpline and resource centre that works with survivors of human trafficking and cooperates with crisis centres. GREVIO notes with concern that, partly due to a lack of financial resources, many crisis centres do not provide shelter accommodation to certain groups of women exposed to intersectional discrimination, in particular women with disabilities and women with addiction issues.⁸⁴

⁸² The Expert Team can also be contacted with support requests via email at kompetanseteamet@bufdir.no.

⁸³ Number of available beds based on state data from 2019.

⁸⁴ GREVIO Baseline Evaluation Report on Norway (2022), p.39, <https://rm.coe.int/grevio-inf-2022-30-report-norway-eng-pour-publication/1680a923f8>.

Crisis centres are funded by municipalities, of which 29 are also run by municipalities, and 12 are run by foundations or non-profit organisations under contract with the municipalities. The Crisis Centre Act (2010) is meant to ensure the provision of a good, comprehensive crisis service centre for women, men and children who are subjected to domestic violence or threats of such violence in Norway. The Act related to municipal crisis centres uses gender-neutral language. Requirements for crisis centres regarding services provided stipulate that the centres may be used by any person who is subjected to domestic violence or threats of such violence and are in need of counselling or safe, temporary accommodation. The law obliges municipalities to ensure that crisis centres are available at a reasonable travel distance, however the number of crisis centres in some rural parts remains scarce. The issue of gender-specific services at crisis centres has been a major national debate in recent years. In 2026, a new law on crisis centres is expected to come into force in Norway, which emphasises that services should be gender-segregated. However, the legislation also allows crisis centres to apply for exemptions. This poses a significant risk to women's safety and their specific needs in the context of these services. If shelters have to decline referrals, it is mainly due to lack of space and capacity to support the survivor. In 2024, crisis centres accommodated a total of 2,340 adults, of which 2,123 were women and 1,704 children.

❖ 3. WOMEN'S CENTRES

Total number of centres	41
Entities running women's centres	State Women's NGOs Other NGOs

There are 41 crisis centres in Norway providing a range of services to all survivors of violence against women and domestic violence including counselling and psychological support, legal advice, and referrals and collaborations with other services. The crisis centres are funded and supervised by state municipalities. All of these centres also provide shelter accommodation and thus operate both as a women's shelter and centre. In 2024, 3,745 daytime clients visited a crisis centre, totalling 13,586 daytime visits.

There are also other centres in Norway that provide specialised support to women survivors of violence. For instance, the Mira Centre in Oslo provides counselling services to migrant as well as Black and minority ethnic women. Furthermore, Therese's House in Oslo offers emergency accommodation as well as medical and social assistance to women with substance abuse issues, as most crisis centres are not accessible to women with substance abuse issues. In addition, the NGO JURK in Oslo offers free legal aid and advice in criminal and civil proceedings to women survivors of violence. ROSA centre also provides support and information for survivors of trafficking such as legal

advice, counselling and healthcare. There is however a lack of specialised support services for Sámi⁸⁵ survivors of violence, and a lack of expertise from professionals on the Sámi language and culture. A number of centres also provide support to survivors of sexualised violence.

🏠 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are support services for survivors of sexualised violence in Norway, including Sexual Assault Reception Centres, Rape Clinics, and Centres for Victims of Incest and Sexual Abuse (SMISO, which are also called NOK). There are 22 sexual assault reception centres (*Overgrepsmottak*) which operate as sexual violence referral centres, as well as two rape clinics (*Voldtektsmottaket*) operating as rape crisis centres, located in clinics and hospitals around the country. These centres offer immediate medical support and crisis intervention; they can be accessed without referral and are free of charge. Survivors will receive help regardless of whether they wish to report the assault to the police. Forensic and gynaecological examination to secure evidence can be carried out in the centres and any evidence lifted off the survivors will be transmitted to the police if the survivor decides to press charges. Other services provided include counselling, medical examinations, pregnancy and STI testing, forensic examinations, help in contacting the police, help in contacting a lawyer regardless of reporting, and information about follow-up services, crisis shelters and other places to receive specialised support.

There are additionally 21 Centres for Victims of Incest and Sexual Abuse (SMISO/NOK) offering support to survivors of sexualised violence, including psychological counselling. The centres are primarily for adults who have been subjected to incest, sexual abuse or rape, as well as for their relatives. They provide services free of charge, function as a supplement to the public sector support system, and do not require a referral. Services provided by these centres include telephone counselling, one-on-one counselling, participation in self-help groups, and other thematic courses. Centres for Victims of Incest and Sexual Abuse are funded by national and local authorities and are run by inter-municipal agencies or NGOs. The exact services provided varies from centre to centre. There are 13 centres that employ professionals who have specialised expertise with working with children and youth. Furthermore, there are 16 centres that have tailored services to individuals with an immigrant background.⁸⁶ SMISOs are located in most regions of Norway and three out of the 21 are available 24/7.

The DIXI Resource Center against Sexual Assault is another free service for survivors of rape, as well as their relatives, located in Oslo. It provides individual and group counselling sessions for survivors, free legal aid, telephone and online counselling, and help in contacting health and

85 The Sámi are a group of indigenous people who inhabit Sápmi, their preferred name for Lapland, and adjacent areas of northern Norway, Sweden, and Finland as well as the Kola Peninsula of Russia.
 86 GREVIO Baseline Evaluation Report on Norway (2022), p.42, <https://rm.coe.int/grevio-inf-2022-30-report-norway-eng-pour-publication/1680a923f8>.

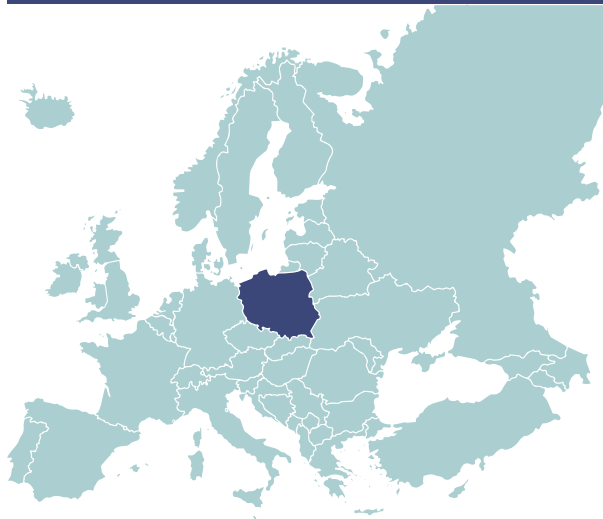
other support services. DIXI is funded by the state and local authorities and also operates a helpline available on weekdays (tel.: +47 22444050). Furthermore, there is a specialised helpline for survivors of sexualised violence in Norway, the helpline for victims of sexual abuse (*Hjelpe-telefonen for seksuelt misbrukte*, tel.: +47 80057000). The helpline is free of charge, available 24/7, and offers multi-lingual support in Norwegian and English. The helpline is run by the Centre for Victims of Incest and Sexual Abuse in Vestfold under the supervision of the Norwegian Directorate for Children, Youth and Family Affairs.

5. PRIMARY PREVENTION SERVICES

In Norway, several authorities are involved in data collection on different forms of violence. Norway has a National Action Plan (NAP) addressing violence against women and girls "Freedom from Violence: Action Plan Against Domestic Violence (2021–2024)", which includes specific provisions on primary prevention. Additionally, various initiatives aim to prevent violence generally and certain forms of violence against women more specifically. There is a dedicated chapter on targeted prevention in the National Action Plan on domestic violence. The National Action Plan provides a number of preventive measures on early detection of domestic violence and education initiatives targeting children. Separate National Action Plans for specific forms of violence against women exist, including honour-based violence, forced marriage, and female genital mutilation (2021–2024) as well as online sexual abuse (2021–2025).

There is a specific budget allocated to primary prevention activities in Norway, including for awareness-raising campaigns like "How Little Should One Tolerate?" which aims to increase knowledge of domestic violence and enable survivors to seek help.⁸⁷ Norway also offers other primary prevention activities, including empowerment activities for girls and women as well as bystander intervention trainings, training for primary practitioners, school-based primary prevention programmes and information guides for newly arrived migrants and refugees. Furthermore, there is a pilot programme called "TryggEst", which was tested in 10 municipalities with the aim of preventing and identifying violence against vulnerable individuals such as elderly women and women with cognitive disabilities. A number of initiatives are also carried out in schools and day-care centres, such as the programmes "SNAKKE" (talk) and "Jeg-vet" (I know) which aim to raise awareness about violence, including sexualised abuse among children, teachers, pre-school educators, and staff in services for children. Specific training courses also exist for professionals likely to come into contact with survivors of violence, including police officers, healthcare professionals, and social workers. However, as has been noted by GREVIO, the training programmes reflect a gender-neutral approach to the topic of domestic violence.⁸⁸

POLAND



GENERAL COUNTRY INFORMATION

Population	36,620,970
Female population	18,914,999
Member of Council of Europe (year)	1991
Member of European Union (year)	2004
Member of United Nations (year)	1945
CEDAW ratified (year)	1980
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2015

SUMMARY

There is one national women's helpline in Poland which is available 24/7, free of charge, and offers limited multi-lingual support. There are other regional helplines offering support to survivors of domestic violence, including multilingual support. There are 40 shelters accessible to women with approximately 605 beds, two of which are women-only shelters. There are women's centres in Poland and all 38 specialised support centres for survivors of domestic violence offer non-residential support. There is one rape crisis centre operated by a women's NGO with a gender-specific and feminist approach. However, there are no other specialised services for survivors of sexualised violence. In Poland, authorities do not recognise or collect data on primary prevention, and no specific budget is allocated to it. There is no available data on women's specialist services as the government does not distinguish between women's specialist services and general services providing support to survivors of domestic violence or other crimes. The available data presents all institutions and organisations available to survivors of violence as specialised services for women, although most services are gender-neutral and do not have a gender-specific approach. This data is available upon request.

87 GREVIO Baseline Evaluation Report on Norway (2022), p.26, <https://rm.coe.int/grevio-inf-2022-30-report-norway-eng-pour-publication/1680a923f8>.

88 GREVIO Baseline Evaluation Report on Norway (2022), p.28, <https://rm.coe.int/grevio-inf-2022-30-report-norway-eng-pour-publication/1680a923f8>.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

There is one national women's helpline in Poland called the Emergency Helpline for Women Victims of Violence (*Telefon zaufania*, tel.: +48 800107777), which is run by the NGO Women's Rights Centre (*Centrum Praw Kobiet – CPK*). It provides immediate specialist support to women survivors of violence, especially psychological, physical and sexualised violence, and offers advice to witnesses of these forms of violence. The helpline is available 24/7, free of charge and offers support in Polish, and on Tuesdays (10:00–14:00) and Thursdays (14:00–18:00) in Ukrainian. Therefore, Poland does meet the Istanbul Convention minimum standards for the provision of a national women's helpline. The helpline is funded through donations and international funding. The Women's Rights Centre additionally offers a separate helpline exclusively for legal support (tel.: +48 800107777), which is available on Thursdays from 16:00–21:00.

Poland has a national, gender-neutral helpline for all survivors of domestic violence, the Blue Line – Polish National Helpline for Victims of Domestic Violence (*Ogólnopolskie Pogotowie dla Ofiar Przemocy w Rodzinie "Niebieska Linia"*, tel. +48 800120002). It is run by the "Blue Line" Association on behalf of the National Centre for Combating Addictions. The service is free of charge, available 24/7, and provides support in Polish, with multilingual assistance in English and Russian available at specific times. In addition, there is a nationwide emotional support helpline (tel.: 116123), operated by the Institute of Health Psychology. This service is also available 24/7 and free of charge, offering crisis support to all individuals in need, including victims of crime and survivors of domestic violence. While it does not exclusively focus on violence against women, its website provides information on available support services and includes a chat option. The police headquarter also runs a Police Family Violence Hotline (*Policyjny Telefon Zaufania ds. Przeciwdziałania Przemocy w Rodzinie*, tel.: +48 800120226), available Monday to Friday from 9:30–15:30. This hotline is free of charge and offers advice on situations of domestic violence.

There are other helplines for survivors of domestic violence operating at a local level, which are run by city halls, local support services and NGOs. Additionally, local branches of the Women's Rights Center, in Krakow, Poznań, Gdańsk, also run helplines in Polish, Ukrainian, Russian and other languages. For example, the Feminoteka Foundation runs

a free helpline (tel. +48 888883388), available from Monday to Friday from 11:00–19:00, offering legal and psychological support for women who have experienced sexual or other forms of violence. Assistance in English is available on Thursdays from 15:00–19:00 and Fridays from 11:00–19:00. Support in Ukrainian and Russian is provided under a separate helpline (tel.: +48 888887988) on Mondays and Wednesdays from 14:00–19:00. Generally, it is difficult to collect data on regional women's helplines in Poland because the list of institutions running helplines is not publicly available.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	2
Total number of all shelters accessible to women	40
Existing number of beds	605 ⁸⁹
24/7 access	Most

Two women-only shelters exist in Poland, which are run by the Women's Rights Centre – a women's NGO with a gender-specific and feminist approach and they only exist in the country's largest cities. One is located in Warsaw (*Specjalistyczny Ośrodek Wsparcia dla Kobiet Doznających Przemocy, SOW*) with 20 available beds, the second one is located in Krakow (*Krakowski Ośrodek Siostrzeństwa, KOS*). Both offer immediate support in emergency situations; however, a previous call is preferred. If no places are available, referrals to other shelters are provided. Women are not expected to pay for accommodation, and they are allowed to stay for up to three months. The women-only shelters provide in-house services including casework, counselling and psychological support, referrals and collaboration with other services, as well as legal advice. Additionally, they provide essential items such as food and hygiene products when needed and ensure the security of the accommodation by keeping its exact address confidential. In 2024, 19 women and 21 children were accommodated in the shelter in Warsaw, and 15 women were accommodated in the shelter in Krakow. Additionally, the Women's Rights Centre offers protected apartments (*mieszkania chronione*) in Wrocław – where ten women and seven children were accommodated in 2024.

Additionally, there are 38 state-recognised shelters accessible to women which are referred to as specialised support centres for survivors of domestic violence (*Specjalistyczne Ośrodki Wsparcia, SOW*) with approximately 605 beds. These shelters were established by the National Program to Combat Domestic Violence and are financed by the state, local governments and donations. They follow a gender-neutral approach and are accessible to both women and men, although survivors in these shelters are predominantly women. All shelters are meant to be accessible 24/7, free of charge, and do not require a referral, although in practice this is not always the case and depends on the shelter. Most shelters are run by the

89 This excludes the beds in the women-only shelter in Krakow, as the number of beds is not available.

state and only a few are run by NGOs, and three are run by Caritas, a faith-based organisation. Shelters are located in all regions of Poland, including major cities, smaller towns and rural areas. The quality standards and services provided do vary across shelters, and most are not accessible to women with disabilities. There is also a specific shelter for survivors of human trafficking, but no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence.

❖ 3. WOMEN'S CENTRES

Total number of centres		38
Entities running women's centres	State	
	Women's NGOs (1 centre)	
	Other NGOs	
Faith-based organisation (3 centres)		

There are women's centres in Poland and all 38 specialised support centres for survivors of domestic violence offer non-residential support. Most of them are run by the state and local governments, and only the Women's Rights Centre branches in Warsaw, Poznań, Kraków, and Gdańsk apply a feminist and gender-specific approach. Types of services provided include counselling, legal, housing and employment advice, advocacy, and other support services such as representation at court. There are also so-called Crisis Intervention Centres in Poland, however, they are not specialised in supporting women survivors of violence and their children.

Additionally, most centres offer some psychosocial support to survivors of domestic violence, although the type of services provided differs widely across municipalities. The National Programme on Combating Family Violence in Poland has also encouraged local authorities to develop 'consultation points' for survivors of domestic violence. In rural municipalities, these services are often the only public services providing psychosocial support to survivors of domestic violence.⁹⁰ However, these services are available to all survivors of domestic violence, and there are no centres providing specialised support to particular groups of survivors nor women survivors or those at risk of specific forms of violence.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Poland, services for survivors of sexualised violence are extremely limited. There is one rape crisis centre which is run by Feminoteka in Warsaw, at a confidential location. It offers psychological, psychotherapeutic, legal, social, and medical support, as well as accompaniment to police or hospitals. The centre is accessible to women with disabilities, provides translation for foreign women, and follows World Health Organisation (WHO) survivor-centred standards. Access is available via Feminoteka's helpline (tel.: +48

888883388), which is also the only specialised helpline for survivors of sexualised violence. Both services operate exclusively for women over 18, are limited to the capital city, and rely on donations, grants, and international funding. In addition to direct assistance, Feminoteka trains police officers, while the Ministry of Health is developing new survivor-centred medical guidelines for hospitals.

Some organisations, such as the Women's Rights Centre, offer services to women who have experienced sexualised violence. Services are however only present in big cities like Warsaw, Gdańsk, Łódź, and Wrocław. There are other specialised services for survivors of sexualised violence. For example, the Foundation FEDERA (*Fundacja FEDERA – Telefon zaufania*), operates a helpline (+48 226359392) Monday to Friday from 16:00–20:00 (except public holidays) and offers confidential consultations on reproductive health provided by gynaecologists, pharmacists, midwives, psychologists, sexologists, and sexual educators. The Foundation Against Rape Culture – Margaret Hodgkinson (*Fundacja przeciw Kulturze Gwałtu im. Margaret Hodgkinson*) provides legal assistance and support for people who have experienced sexualised violence, offering consultations, guidance on procedures, and representation before prosecutors and courts, while prioritising listening and respecting survivors' choices. The anti-violence Helpline SEXEDPL (*Antyprzemocowa Linia Pomocy SEXEDPL*) focuses on preventing secondary victimisation, offering guidance for survivors and those supporting them, including information on medical care, emergency contraception, HIV post-exposure prophylaxis, psychological support, and legal aid.

Aside from this, there are no other specialised services for survivors of sexualised violence in Poland. Survivors must rely on non-specialised institutions such as the police, hospitals, or social welfare centres, where the quality of support varies greatly. Police and other officials are often untrained in survivor-centred approaches, leading to disbelief, victim-blaming, or indifference; as a result, only around 8% of women report sexualised violence. Because rape is prosecuted ex officio, survivors who seek medical care are often forced into police procedures without consent, and mandatory reporting obligations by doctors or social workers frequently cause secondary traumatisation. Legal protections are also insufficient, as special procedures to avoid re-traumatisation apply only in some cases.

👤 5. PRIMARY PREVENTION SERVICES

In Poland, public authorities do not recognise or collect data on primary prevention, and the term itself does not appear in legal or policy frameworks. The World Health Organisation (WHO) classification of primary, secondary, and tertiary prevention is not applied, and the gendered dimension of violence is almost entirely absent. The Government Programme for Combating Domestic Violence

⁹⁰ According to the GREVIO Baseline Evaluation Report on Poland, pg. 43. The report also notes that in 2019, there were 503 such consultation points.

2024–2030 devotes a chapter to “prevention,” but in practice focuses on secondary and tertiary measures such as victim support and perpetrator programmes. Budget allocations confirm this: over 90% of funds go to shelters, interdisciplinary teams, and perpetrator programmes, while just 0.16% is spent on awareness-raising campaigns. No funds are dedicated to primary prevention, and domestic violence is strongly linked to addiction in state policy, further obscuring its gendered nature. Local initiatives exist, such as Warsaw’s “Human Rights-Friendly School” programme and upcoming civic and health education curricula in schools, but they do not explicitly address gender-based violence.

Primary prevention activities are mainly carried out by feminist NGOs. Recent examples include: the Juniper Foundation and Plan International Poland project on girls’ safety in cities (2024); the Autonomia Foundation and trainers trained by Autonomia which offer WenDo feminist self-defence trainings, reaching over 2,000 women and girls in 2024; and the “Stand Up – Oppose Harassment in Public Places” campaign, implemented with L’Oréal Paris, Right To Be, Feminoteka, and the Women’s Rights Centre, which trained thousands in bystander intervention. These initiatives are innovative and survivor-centred but project-based, underfunded, and not integrated into state policy. Additionally, Krakow is part of the EU co-funded FEMACT-Cities Action Planning Network, which focuses on strengthening the local implementation of gender equality across urban planning and social cohesion.

PORTUGAL



GENERAL COUNTRY INFORMATION

Population	10,639,726
Female population	5,556,158
Member of Council of Europe (year)	1976
Member of European Union (year)	1986
Member of United Nations (year)	1955
CEDAW ratified (year)	1980
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2013

SUMMARY

There is one national women’s helpline in Portugal, free of charge and available 24/7, which does not offer multilingual support. There are 62 women-only shelters in Portugal with 1,024 beds available, and 140 women’s centres for survivors of violence against women and domestic violence. There are two rape crisis centres in Portugal, which offer support to women and girls who have experienced sexualised violence. Portugal has a National Action Plan for the period of 2023–2026. Certain primary prevention measures are included in the document; however, there is no specific budget allocated to such activities. The state, through the Commission for Citizenship and Gender Equality, collects data on women’s specialist support services and this data is public.

WOMEN’S SPECIALIST SERVICES

1. NATIONAL WOMEN’S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	No
Total number of calls/ callers in 2024	1,964 calls/ contacts

There is one national women's helpline in Portugal called Information Service for Domestic Violence Victims (*Serviço de Informação à Vítima de Violência Doméstica – SIVVD*, tel.: +351 800202148; SMS +351 3060), which is run by the Commission for Citizenship and Gender Equality. It operates free of charge, is available 24/7, meeting the standards of the Istanbul Convention, but it does not offer multilingual support. The helpline provides support to all survivors of domestic violence and gender-based violence from a gender-neutral perspective. The helpline is funded by the state and by international funding. In 2024, the helpline received 1,964 calls, emails and SMS. In 2024, the most common forms of violence reported by the callers to the state-wide helpline were domestic, physical and sexualised violence.

There are two other helplines available in the country. The Association for the Support of Victims (*Associação de Apoio à Vítima – APAV*, tel.: 116006) provides support for all victims of crime, including survivors of domestic violence. It is free of charge and operates from Mondays to Fridays from 8:00 to 22:00. The second helpline is called National Social Emergency Line (*Linha Nacional de Emergência Social*, tel.: +351 144), is run by the Social Security Services and operates 24/7. This helpline also provides support to all survivors of domestic violence and violence against women, but with a gender-neutral approach. Additionally, most centres supporting survivors of violence against women and domestic violence are accessible by phone, video calls or email and can provide remote support to survivors, but mostly only during working hours. Survivors can also contact women's centres, which can provide multilingual support depending on the language skills of staff members, or through a telephone translation service run by the High Commission for Migration.

The Association of Women Against Violence (*Associação de Mulheres contra a Violência – AMCV*, tel.: +351 213802160, SMS +351 0962048272) can be contacted by survivors during working hours and provides multilingual support in Portuguese, English, French and German, according to the staff available. The Women's Alternative and Response Union's helpline (*União de Mulheres Alternativa e Resposta*, tel.: +351 218873005) is also available to survivors.

The Portuguese government provides a mobile app (Ap-pVD) linked to the Resource Guide in the area of Domestic Violence (*Guia de Recursos na área da Violência Doméstica*, www.guiaderecursosvd.cig.gov.pt), which localises all services providing support in cases of violence against women and domestic violence, divided by regions.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	62
Total number of all shelters accessible to women	62
Existing number of beds	1,024
24/7 access	Some

There are 62 women-only shelters in Portugal, offering 1,024 beds. In the country, there are two types of shelter services available for women and children who are survivors of domestic violence: emergency accommodations (*Acolhimentos de emergência*) offer 24/7 access and can accommodate women for 72 hours; however, it is possible to extend this timeframe if the risk persists or according to the needs of the survivor. The shelters (*Casa de Abrigo*) can accommodate women for up to six months. There are 24 emergency accommodations with a total of 277 beds and 38 shelters with a total of 747 beds. Women are not expected to pay for their stay in women-only shelters. The services most often provided by the women-only shelters include casework, counselling/psychological support, referrals and collaborations with other services, and practical and legal advice.

All women-only shelters are accessible to women with uncertain residence permits, undocumented women, refugee women, homeless women, women with sensory disabilities, older women, women with children with disabilities and unaccompanied girls under the age of 18. Some women-only shelters are also accessible to women with physical disabilities, women with cognitive disabilities, women with substance abuse issues, transgender women and women with older sons (14 years and above). There are also shelters in Portugal that are specialised to support LGBTQI+ people, women survivors or those at risk of trafficking, older women and women with mental health issues.

Women-only shelters are funded by the state, donations, international funding, and religious institutions. Seven shelters are run by women's NGOs with a gender specific approach, while the others are run by other NGOs and faith-based organisations. Women-only shelters are located in all regions of Portugal, both in continental districts and in the autonomous regions of Azores and Madeira. If women-only shelters had to decline referrals, it is mainly because there is no space or capacity to support the survivor, no capacity to accommodate survivors with children, or inadequate equipment for the needs of women/children. In 2024, the most common forms of violence reported by the survivors to the women-only shelters were domestic violence, physical violence, and violence directly or indirectly affecting children. In 2024, women-only shelters accommodated 2,854 women and 2,685 children.

There are other shelters in the country, besides women-only shelters, that provide support for male survivors of domestic violence, LGBTQI+ people and for men and women survivors of trafficking.

❖ 3. WOMEN'S CENTRES

Total number of centres	140
Entities running women's centres	Women's NGOs Others

There are 140 centres for survivors of violence against women and domestic violence in the country. Most service structures also guarantee decentralised services, meaning that in total, 223 service points are available throughout Portugal. Two women's crisis centres are run by women's NGOs: AMCV in Lisbon and EIR (UMAR) in Porto. Women's centres provide non-residential support, such as counselling and psychological support, legal advice, social welfare and financial support, housing advice, referrals and collaborations with other services, and representation at court, police, and social services. The centres also offer specialised support for survivors of sexualised violence, including needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and prevention education, and legal advice.

All women's centres are accessible to women with uncertain residence permits, undocumented women, refugee women, women with disabilities (physical, cognitive, sensory), as well as older women. Most women's centres can be accessed by unaccompanied girls under the age of 18, and some are equipped to support transgender women. Additionally, there are services supporting specific groups of survivors, but they are often dependent on project-based funding.

Women's centres are funded by the state, donations, international funding, and religious institutions. The centres are located in most regions of Portugal.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are two rape crisis centres in Portugal, one located in Lisbon and the other in Porto. There are also crisis and medical services operated by the Institute of Forensic Medicine and hospitals. The rape crisis centres provide specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and prevention education, and legal advice. Sexualised violence services are funded by the state, as well as through international funding. These services support women, young women, and children.

5. PRIMARY PREVENTION SERVICES

The Commission for Citizenship and Gender Equality is responsible for collecting data on the public and private measures and actions outlined in the National Action Plans. Portugal has an Action Plan for the Prevention and Combating of Violence Against Women and Domestic Violence 2023-2026. The action plan does not provide a definition of primary prevention, but it does outline several prevention objectives, such as preventing and eradicating social tolerance of violence against women and domestic violence, raising awareness about their impact and promoting a culture of non-violence, human rights, equality, and non-discrimination. Different primary prevention

activities are conducted in Portugal, such as awareness-raising campaigns and programmes, school-based primary prevention programmes, and primary prevention activities specifically targeted to men and boys. Nevertheless, there is no specific budget allocated to primary prevention activities, and the National Action Plan mentions various funding sources such as the State Budget, the Demography, Qualifications and Inclusion Programme and other European and international funding sources.

ROMANIA



GENERAL COUNTRY INFORMATION

Population	19,067,576
Female population	9,796,206
Member of Council of Europe (year)	1993
Member of European Union (year)	2007
Member of United Nations (year)	1955
CEDAW ratified (year)	1982
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2014
Istanbul Convention ratified (year)	2016

SUMMARY

There are two national women's helplines Romania which are both free of charge and available 24/7, and one of which also offers multilingual support. There are 144 women-only shelters with a total of 1,260 beds. Romania also has 15 women's centres providing a range of services and 10 support services for survivors of sexualised violence located in emergency units of hospitals providing specialist forensic and medical care. Specific provisions on primary prevention are included in official documents such as National Action Plans, but it is not known if there is a specific budget allocated to such activities. The state does collect data on women's specialist services and most public institutions publish general information and occasionally share statistics or reports on their websites. For more detailed information, interested parties can request specific data in accordance with legislation that guarantees free access to information of public interest. Additionally, public and private institutions are periodically asked to provide statistical data about beneficiaries of assistance, which is collected and processed by the National Agency for Equal Opportunities between Women and Men (ANES).

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	9,285 calls ⁹¹

There are two national women's helpline in Romania, one which is run by the National Agency for Equal Opportunities between Women and Men (ANES), one run by the organisation Community Mediation and Security Center (*Centrul de Mediere și Securitate Comunitară – CMSC*). The first helpline is called the National Helpline for Victims of Domestic Violence (*HELPLINE național cu număr unic destinat victimelor violenței domestice*, tel.: +40 800500333) and is free of charge, available 24/7 and offers multilingual support in Romanian, English, Italian and Hungarian. Support in Ukrainian and Romani languages can also be provided on a voluntary basis, if needed. The helpline is funded by the state provides support to survivors of domestic violence, human trafficking, and sex discrimination. In 2024, the helpline received in total 9,285 calls, of which 6,534 were answered. Of these calls, 2,835 were related to domestic violence, 31 to human trafficking, 22 to sexualised violence, 15 to sexual harassment and workplace bullying, and nine calls were related to questions concerning equal opportunities/gender. The second helpline is the Call Center Hellena – Information and counselling center for victims of domestic violence (*Centrul de informare și consiliere pentru victimele violenței domestice de tip linie telefonică de urgență - Helena Helpline*, tel.: +40 800070017) which is also free of charge and available 24/7. This helpline was established in 2022 and is funded by the state, private funds/donations, as well as international funding, and offers support to women who are in situations of risk and concerned about their safety. The most common forms of violence reported to the helpline in 2024 were domestic, physical, and psychological violence. Romania does meet the Istanbul Convention minimum standards for the provision of a national women's helpline.

There are no other helplines supporting survivors of violence against women in Romania.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	144
Total number of all shelters accessible to women	144
Existing number of beds	1,260
24/7 access	Some

According to ANES, there are 144 women-only shelters in Romania with 1,260 available beds. These include 88 licensed shelters for survivors of domestic violence, human

⁹¹ This number only relates to one of the helplines, the National Helpline for Victims of Domestic Violence.

trafficking and sexualised violence with 875 bed spaces, as well as 56 maternity centres with 385 bed spaces that can also accommodate survivors of violence together with children under 2–3 years old or pregnant women in the last trimester of pregnancy. Most shelter beds are concentrated in Bucharest and other big cities in Romania and are therefore not distributed equally. Relocation of survivors (both for safety and access to specialised services) is not a widespread practice due to restrictions on expenses from local budgets. The types of in-house services most often provided by women-only shelters are counselling/psychological support, casework, and referrals/collaboration with other services, although counselling and psychological support are the most predominant service offered. Some of the women-only shelters have 24/7 access and women are not expected to pay for their accommodation in the shelters. In Romania, there are several types of women-only shelters, classified according to the services offered and the length of stay. The main types are emergency reception centres, temporary centres, and sheltered housing. Emergency reception centres provide immediate and temporary shelter to survivors (accessible 24/7 access, for two to six months). They usually also provide basic services such as food, primary health care and psychological counselling. Recovery centres provide a more stable environment, with more complex services such as social assistance, legal counselling and rehabilitation programs. The goal is to help survivors regain their autonomy and build a future without violence. The maximum length of stay is one year or more. Sheltered housing provide temporary, assisted housing facilities where survivors can live for a longer period (one year or more), with access to support and counselling services, while they look for permanent housing options.

Most women-only shelters can accommodate women with older sons (14 years and above), and some women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with cognitive disabilities, older women (65 years and above), and women with children with disabilities. According to national authorities, all women-only shelters are accessible to all survivors without discrimination, however, in reality this is not always the case.

Funding for women-only shelters comes from the state, donations, international funding and religious institutions. In terms of the entities running women-only shelters, the majority of domestic violence shelters and maternity centres are run by the state, and at least seven shelters are run by faith-based organisations. Some women-only shelters are also run by NGOs and women's NGOs with a gender-specific and feminist approach. Women-only shelters are located in most regions although state-run shelters located in rural areas do not strictly belong to the local community and usually serve the entire county. There is at

least one women-only shelter per county. Shelters run by NGOs are more flexible and can accommodate survivors from all over the country. The main reasons for declining referrals to women-only shelters were no space or capacity to support the survivor and the survivor being ineligible for support. The most common forms of violence reported to women-only shelters in 2024 were domestic, physical and psychological violence. According to data provided by ANES, in 2024 a total of 12,990 individuals accessed social services related to preventing and combating domestic violence, which includes shelter accommodation. However, data collection efforts are limited, and this number does not reflect the reality on the ground.

There are other shelters that provide support for survivors of violence in Romania, and which are accessible to women. This includes maternal centres which are not directly licensed as providers of social services for survivors of domestic violence but can accommodate women survivors if needed. The exact number of such centres as well as available beds is unknown. There are also shelters for survivors of different forms of trafficking.

❖ 3. WOMEN'S CENTRES

Total number of centres	15
Entities running women's centres⁹²	Women's NGOs (6 centres) Other NGOs (4 centres) State (6 centres) Faith-based organisation (1 centre)

There are 15 women's centres in Romania which provide a range of services including counselling/psychological support, legal advice, employment, referrals/collaboration with other services, and representation at court/police/social services. Other support services provided by women's centres include counselling for survivors' own safety and security and information on how to access services in order to obtain a forensic medical certificate and medical care. Women's centres also provide specialist support for survivors of sexualised violence including needs assessment, specialist psychological care, and legal advice. In recent years, a lot of women's centres have closed, especially during the COVID-19 pandemic period and due to losing their service license/accreditation. The number of available women's centres changes frequently and is updated monthly on the official website of the National Agency for Equal Opportunities between Women and Men (ANES).

Some women's centres can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical and/or cognitive disabilities, and older women (65 years and above). There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Romania. In general, existing centres seek to identify solutions and

92 Some women's centres are run by multiple entities (e.g., an NGO that is also a religious institution).

offer support to all survivors without discrimination, often in collaboration with other institutions.

Six women's centres are run by women's NGOs, six are run by the state, four by other NGOs, and one by a religious institution. It is important to note that some women's centres are run by multiple entities (e.g., an NGO that is also a religious institution). Women's centres are located just in a few major cities and are funded by the state from national/local authorities, donations as well as international funding.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are 10 support services for survivors of sexualised violence in Romania which operate as Intervention Centres for Victims of Sexual Violence and were established in 2021. These are integrated services located in emergency units of hospitals around the country and aim to provide specialist forensic and medical care, needs assessment, specialist psychological care and legal advice to all survivors of sexualised violence. Centres offer primary counselling regardless of the intervention offered, and survivors can receive free medical services and biological sample collection. When a survivor comes to the centre, the centre coordinator initiates the integrated intervention procedure by requesting the support of the other professionals involved, including the police and social assistance services. A police officer then registers the survivors' complaint and can also issue a temporary protection order. Through the intervention of the social worker, the survivor can benefit from support services offered by the General Directorate of Social Assistance free of charge, namely specialised psychological counselling, legal counselling, accommodation in sheltered housing or inclusion in support programs. These sexualised violence services are funded by the state as well as international funding from the Norwegian grants.

Unfortunately, survivors often do not receive adequate multidisciplinary support from the Intervention Centres for Victims of Sexual Violence.⁹³ Survivors have reported to NGOs that they encountered many shortcomings when seeking help. According to the National Agency for Equal Opportunities between Women and Men's Activity Report for 2024, 70 people received support from the Intervention Centres for Victims of Sexual Violence since they opened in 2021. There are no other services for survivors of sexualised violence in Romania.

5. PRIMARY PREVENTION SERVICES

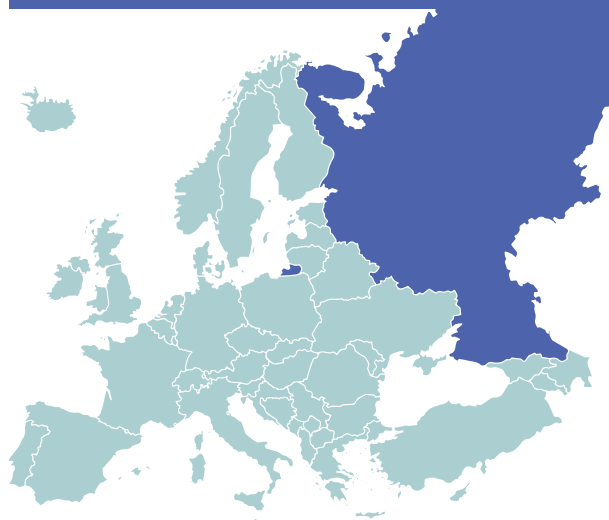
Specific provisions on primary prevention are included in official documents in Romania such as the National Strategy for the Promotion of Equal Opportunities and Treatment between Women and Men and for Preventing and Combating Domestic Violence 2022–2027,⁹⁴ and in the National Strategy for Preventing and Combating Sexual Violence "SINERGIE" 2021–2030.⁹⁵ SINERGIE refers to different strategic objectives which include the three forms of prevention: primary, secondary, and tertiary. The Government Decision No. 592/2021 also approved an Action Plan for the implementation of the SINERGIE for 2021–2030. A definition of primary prevention is also included in the Minimum Quality Standards for Social Services Organised as Population Information and Awareness Centres in the Field of Preventing and Combating Domestic Violence (Annex 5), however a clear definition in legislation does not exist. Due to the decentralisation of local public services in Romania, data on budget allocations for certain categories of expenditure are difficult to track. Many of the provisions of the strategies are the responsibility of local authorities, so it is not known if a specific budget is allocated to primary prevention activities. Available activities include feminist self-defence trainings, empowerment activities for girls and women, and awareness-raising campaigns or programmes. Many NGOs working in the field of violence against women do implement primary prevention activities, depending on their budget. They run and organise educational and informational activities for women and girls, as well as the general public (through awareness raising campaigns online and offline).

93 As revealed by a recent press investigation available (in Romanian) at: <https://pressone.ro/reportaj-prin-ce-trece-o-potentiala-victima-a-unui-viol-la-spital-ul-universitar-unde-ar-trebui-sa-functioneze-un-centru-special-pentru-femei>.

94 Government of Romania, National Strategy for Promoting Equality of Opportunities and Treatment between Women and Men and for Preventing and Combating Domestic Violence 2022–2027, <https://anes.gov.ro/wp-content/uploads/2023/11/Monitorul-Oficial-Partea-I-nr.-1239Bis.pdf>.

95 Government of Romania, National Strategy for Preventing and Combating Sexual Violence "SYNERGY" 2021–2030, <https://pcabucuresti.mpublic.ro/violenta-sexuala.htm>.

THE RUSSIAN FEDERATION



GENERAL COUNTRY INFORMATION

Population	143,666,931 ⁹⁶
Female population	77,120,043 ⁹⁷
Member of Council of Europe (year)	
<i>In March 2022, the Russian Federation ceased to be a member of the Council of Europe.</i>	
	No
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1981
CEDAW optional protocol ratified (year)	2004
Istanbul Convention signed (year)	No
Istanbul Convention ratified (year)	No

SUMMARY

The Russian Federation has one national women's helpline which is free of charge and operates 24/7, although it does not offer multilingual support. There are an estimated 30 to 40 women-only shelters in Russia, as well as an estimated 134 crisis centres offering shelter accommodation to women survivors of violence with 2,423 available beds. There are women's centres in Russia, although the exact number is unknown, but different estimations give an approximate number of 150-180 centres across the country. There are two rape crisis centres in Russia for survivors of sexualised violence as well as a specialised helpline. There is a National Strategy which does not focus on, but does address, violence against women and girls. However, it does not include specific provisions on primary prevention nor is there a budget allocated to such activities. The state does collect data on women's specialist support services and is data is publicly available, although it only refers to state-affiliated services and does not include primary prevention activities. Different NGOs also collect data on women's

specialist services, however, due to the size of the country and limited resources, this data is often not accurate enough or up to date.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	No
Total number of calls/callers in 2024	21,360 calls

The Russian Federation has one national women's helpline called the National Helpline for Women Suffering from Domestic Violence (*Всероссийский круглосуточный телефон доверия для женщин, пострадавших от домашнего насилия*, tel.: +7 8007000600). This helpline is run by the autonomous NGO for social rehabilitation and adaptation of women and children "Resource Assistance Center". The helpline is free of charge and available 24/7 but does not offer multilingual support. Therefore, the helpline does meet the Istanbul Convention minimum standards for the provision of a national women's helpline. It should be noted that although the helpline is available around the clock, in practice, calls are often not answered. This helpline provides support mainly to survivors of domestic violence. According to data published by the Resource Assistance Center, the helpline received a total of 21,360 calls in 2024.

There is another national women's helpline in Russia operated by POLINA – Crisis Center for Women (*Горячая линия организации «ПОЛИНА. Кризисный центр для женщин»*, tel.: +7 8123273000). The helpline is free of charge but is not available 24/7 nor does it offer multilingual support. This helpline provides psychological and legal support for women survivors of any form of domestic or sexualised violence, including commercial sexual exploitation, and also supports women experiencing cyberviolence and stalking. Due to the extremely complicated context and circumstances, communication between different women's NGOs in Russia is very limited. In this sense, information regarding sources of funding for the helpline is sensitive. POLINA Center specifically relies on various funding opportunities and for years their main sources of funding were international funds and donations. This helpline received a total number of 6,889 requests in 2024, which includes 1,561 calls and 5,328 online requests. The most common forms of violence reported by callers to the helplines in 2024 were physical violence and technology-facilitated violence against women.

There are other regional helplines supporting survivors of violence against women in Russia, but the exact number is unknown. It is important to note that in 2024, many

96 Latest available data on Eurostat's database is from 2014.

97 Ibid.

Russian women's organisations that continue to operate in Russia have reported a decreased number of calls, due to a sharp reduction in funding as well as a number of call operators having to leave the country. On the other hand, the number of help requests via chat and online channels has increased year after year as this service is more flexible and allows operators to cover a wider geographical area.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	30
Total number of all shelters accessible to women	134⁹⁸
Existing number of beds	2,423⁹⁹
24/7 access	Unknown

There are an estimated 30 to 40 women-only shelters in Russia, most of which are run by the municipality or faith-based organisations, but the exact number of beds is unknown. There are no official statistics on the number of shelters and beds available for women survivors of violence in Russia and the numbers reported are based on unofficial information provided by services supporting women in crisis situations. The number of beds is drastically insufficient to meet the needs for women survivors of violence, even in major cities, and in most other regions shelter accommodation is not an available option. Moreover, most of the shelters have a set of strict requirements (for example, the survivors must be legally registered to access the shelter), which makes the available services even less accessible. Women-only shelters generally do not offer 24/7 access as women are required to provide specific documents to enter a shelter. Women are not expected to pay for their accommodation in the women-only shelters, but in some shelters run by faith-based organisations, they may be required to do chores. In some state-run shelters, a woman must provide a document proving her income. If her income is higher than the official subsistence rate, she needs to pay for her accommodation. The types of in-house services most often provided by women-only shelters are casework, practical advice, and legal advice.

Some women-only shelters can accommodate and are accessible to women with sensory disabilities, women with substance abuse issues, and women with children with disabilities. No women-only shelters are equipped to support women with uncertain residence permits/undocumented women, refugee women, homeless women, women with cognitive disabilities, and older women (65 years and above). There are no specialised shelters supporting particular groups of survivors nor women survivors or

those at risk of specific forms of violence in Russia. An informal shelter for LGBTQI+ individuals exists, however, it is not specifically for survivors of violence. Additionally, as state oppression against civil society representatives grows, many services offering support to specific groups of survivors are underfunded and receive no governmental support.

Funding for women-only shelters comes from the state as well as donations. Women-only shelters exist in major cities only and women are allowed to stay there for up to three months. If women-only shelters had to decline referrals the three main reasons were no space/capacity to support the survivor, no space/capacity to accommodate survivors with children, and the survivor being ineligible for support. The three most common forms of violence reported by survivors to women-only shelters in 2024 were domestic violence, physical violence, and violence directly/indirectly affecting children.

There are other shelters in Russia which operate as crisis centres for women in difficult life circumstances and can provide support to survivors of violence. According to official data from the Department of Social Protection and Social Services of the Ministry of Labor of Russia, there are 134 crisis centres/units in 57 regions including 81 shelters run by state organisations, 19 shelters run by state crisis centres, 18 shelters run by non-governmental social service organisations, and 16 by non-profit organisations. In total, there are an estimated 2,423 available beds. In addition, the Russian Orthodox Church runs 75 crisis centres which includes shelters for teenage mothers, people with substance abuse issues and women released from prisons. Gender affirming medical procedures are banned in Russia, meaning that transgender and non-binary people face serious risks when accessing shelters, and there is a lack of services for LGBTQI+ survivors to receive support.

3. WOMEN'S CENTRES

Total number of centres	150¹⁰⁰
Entities running women's centres	N/A

There are women's centres in Russia, although the exact number is unknown, but different estimations give an approximate number of 150-180 centres across the country. The types of services provided by women's centres include counselling/psychological support, legal advice, referrals/collaboration with other services, and representation at court/police/social services. However, women's centres do not provide financial and social welfare support/advocacy, housing advice, and employment. Most women's centres provide only non-residential support services,

⁹⁸ This figure comes from the Department of Social Protection and Social Services of the Ministry of Labor of Russia and is repeatedly cited by officials in 2024–2025. However, it is not possible to know if this figure includes the estimated 30–40 women-only shelters based on unofficial information collected by NGOs. Additionally, this figure does not include the crisis centres run by the Russian Orthodox Church.

⁹⁹ Ibid.

¹⁰⁰ This figure is from the WAVE Country Report 2023. Mappings estimate that there around 200 services for survivors of violence (state/faith-based organisations/NGOs) available nationwide, but this figure lumps all services together including helplines, counselling centres, and shelters.

although all women-only shelters offer both residential and non-residential support. Some women's centres also provide specialist support to survivors of sexualised violence in Russia, including specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice.

Some women's centres are equipped to support women with uncertain residence permits/undocumented women, refugee women, women with physical disabilities, women with sensory disabilities, and older women (65 years and above). There are women's centres specialised in supporting survivors (at risk of) forced marriage, honour-based violence, and trafficking. Additionally, there are grassroots initiatives which provide support to transgender women and non-binary people.

Women's centres in Russia are located only in major cities and are funded by the state and through donations. International funding is no longer available. Information on which entities are running women's centres is not available, and it is furthermore not the case that all women's centres have a feminist and gender-sensitive approach. In fact, some centres may reinforce negative gender stereotypes and use victim-blaming language.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are two rape crisis centres in Russia, the Sexual Assault Recovery Centre Sisters (*Syostrí*), which also runs a specialised helpline (tel., +7 4999010201), and We Believe You (*Tebe Poveryat*). These centres provide psychological counselling and support to sexualised violence survivors. *Tebe Poveryat* works offline in St. Petersburg and online across Russia and other countries for Russian-speaking survivors. These services are located in the capital city and funded through donations. *Syostrí* supports all survivors of sexualised violence, no matter how long ago the violence took place, and *Tebe Poveryat* supports all survivors who experienced sexualised violence as a child and young adults. Additionally, some women's centres also provide online counselling and support to adult female survivors of sexualised violence, although these are not specialised sexualised violence centres. There are no other services in Russia for women and girls who have experienced sexualised violence.

5. PRIMARY PREVENTION SERVICES

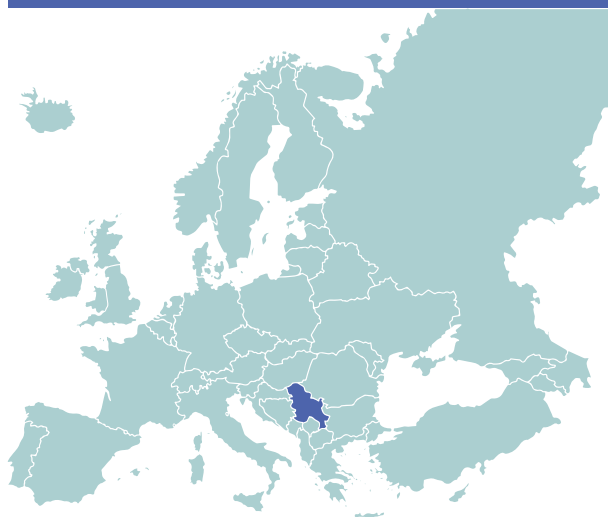
Russia has no stand-alone National Action Plan (NAP) on violence against women and girls. The main federal policy document is the National Strategy for Action in the Interests of Women, 2023–2030, plus a 2023–2026 implementation plan. These focus broadly on women's health, economic participation, "prevention of social disadvantage," political participation and improving official statistics, rather than setting out a dedicated violence against

women plan with prevention pillars. In practice, violence is referenced only indirectly—largely folded into the umbrella of "social disadvantage"—and not addressed as a separate prevention stream.

Public authorities do not publish consolidated data on primary prevention activities; available federal documents list actions and reporting duties but no national indicators specific to primary prevention, and international reviews have repeatedly flagged fragmented data systems in this area. The Strategy's narrative claims domestic violence figures have decreased, but this is misleading: the apparent drop follows the 2017 partial decriminalisation of first instance "light" domestic assault, which pushed many incidents out of criminal statistics and is widely associated with under-reporting.

There is no statutory definition of "domestic violence" in Russian law—and, consequently, no official legal definition of "primary prevention" of violence. The strategy/plan contain no earmarked budget lines for primary prevention measures, and implementation is left to the responsible bodies.

SERBIA



GENERAL COUNTRY INFORMATION

Population	6,605,168
Female population	3,392,796
Member of Council of Europe (year)	2003
Member of European Union (year)	No
Member of United Nations (year)	2000
CEDAW ratified (year)	2001
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2013

SUMMARY

There is one national women's helpline in Serbia, which operates 24/7 and is free of charge. Additionally, there are other national and regional helplines supporting survivors of violence against women. There are 11 women-only shelters in the country offering 178 beds, as well as one shelter for survivors of trafficking with six beds. There are 42 women's centres, and five support services for survivors of sexualised violence in the country. Prevention is included in the National Strategy for Preventing and Combating Gender-based Violence against Women and Against Violence in the Family, although there is no dedicated budget and no definition of primary prevention. The state collects data on women's specialist services licensed as social protection services, not on all specialist services for women, and not on those provided by non-governmental organisations that are not licensed. No data is collected on primary prevention activities. The data collected by the state is available upon request, but access to it is not guaranteed.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

There is one national women's helpline in Serbia called SOS Helpline for Women Who Experience Violence (*SOS telefon za žene sa iskustvom nasilja*, tel.: +381 0800222003). It is free of charge and operates 24/7, meeting the standards of the Istanbul Convention. Nevertheless, it does not offer multilingual support. The helpline is run by the Centre for Protection of Infants, Children and Youth in Belgrade, and it provides support for all types of domestic and intimate partner violence against women, including neglect, physical violence, sexualised violence (also technology-facilitated), sexual exploitation, psychological violence (including technology-facilitated), exploitation, economic violence, and bullying.

Additionally, there are eight other helplines which are state-wide and support women survivors of violence free of charge and 24/7. These helplines are run by organisations such as the Union of Associations Network SOS Vojvodina, the Citizens Association Women's Centre Užice and the Association Autonomous Women's Centre. In terms of funding, two SOS helplines (those run by the Centre for the Protection of Infants, Children and Youth in Belgrade and the Counselling Centre Against Family Violence in Belgrade) receive state funding from national and local authorities, while the other helplines are supported by donations and international funding. According to unofficial data, in 2024, the licensed helplines received approximately between 10,000 and 13,000 calls, and the number of callers was between 3,500 and 5,000. The most common forms of violence reported by callers were physical, psychological and economic violence.

In addition to the nine licensed national SOS telephone lines for women who have experienced violence, one of which is specialised in human trafficking, there are 15 non-licensed helplines: two specialised in supporting women with disabilities exposed to violence, one for the support of women survivors of sexualised violence (details in the dedicated section below), one for women from national minorities and 11 for all women.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	11
Total number of all shelters accessible to women	12
Existing number of beds	184
24/7 access	All

There are 11 women-only shelters in Serbia offering 178 beds, which falls short of the actual needs of the country. Seven of these shelters are licensed as services of social protection, while four are not licensed. Although all shelters have 24/7 access, most do not have a sufficient number of employees and specialised staff available around the clock. Moreover, shelters are not directly accessible to survivors, especially in emergencies, because access is managed by the Centre for Social Work. Women who accessed shelters in their municipality are not expected to pay, while the costs incurred by women coming from other municipalities are usually borne by social work centres from their place of residence; this is not possible when municipalities do not plan budget lines for these purposes. Women-only shelters are financed by local governments and donations from foundations and individuals. Ten shelters are run by local authorities, while one is run by an NGO. The territorial distribution of shelters is not adequate and covers 11 out of 25 districts and the City of Belgrade. On average, women stay in the shelters for three months.

The main reasons for declined referrals were the lack of space/capacity to accommodate survivors with children, survivors being ineligible for support, or the shelters not being adequately equipped for the needs of the women/children, for instance, in cases of women with disabilities or with substance abuse issues. There are cases of refusal to admit Roma women. The most common forms of violence reported by survivors in 2024 were domestic violence, physical violence, and violence directly/indirectly affecting children. The services provided by shelters include casework, counselling/psychological support, referrals/collaboration with other services, and practical and legal advice. Casework and referrals/collaboration with other services are predominant, because in most of the shelters, there is not enough professional staff employed to provide other services. On average, the licensed shelters support between 250-300 women per year.

While all shelters can accommodate older women, some of them can also be accessed by and are accessible to women with physical and sensory disabilities, women with older sons and women with children with disabilities. No shelters are accessible for women with uncertain residence permits and undocumented women, refugee women, homeless women, women with cognitive disabilities, women with substance abuse issues, transgender women and unaccompanied girls under 18. There is one licensed shelter for survivors of trafficking run by a women's NGO with a feminist approach in the country.

Additionally, there is one shelter for survivors of trafficking in persons, which had six beds in 2024.

❖ 3. WOMEN'S CENTRES

Total number of centres	42
Entities running women's centres	Women's NGOs (32 centres) Other NGOs (10 centres)

There are 42 women's centres in Serbia, 25 are only for women survivors of violence, and 17 for all survivors. Women's centres provide different types of services: 32 centres provide counselling and psychological support, referrals and collaboration with other services; 12 centres provide legal advice, eight provide representation at court/police/social services, six offer financial and social welfare support, and one also supports with employment. Additionally, women's centres can provide specialist support for survivors of sexualised violence, including needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice.

In terms of accessibility, seven centres are accessible for women with uncertain residence permits/undocumented women, six centres can be accessed by and are accessible to women with physical and sensory disabilities, five are equipped to support black and minority ethnic women and survivors (at risk) of forced marriage, three are accessible for refugee women, girls under the age of 18, transgender women and non-binary people, while two centres are equipped to support survivors (at risk) of trafficking, women with cognitive disabilities and older women.

Out of the 42 centres, 32 are run by women's NGOs and 10 by other NGOs. The main sources of funding are donations and international funding, as most NGOs do not receive support from the state. Women's centres are predominantly located in cities and much less in rural parts of the country, with some districts where there are no centres available. It is estimated that women's centres in Serbia provided support to 1,500 to 2,500 women in 2024.

🔑 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

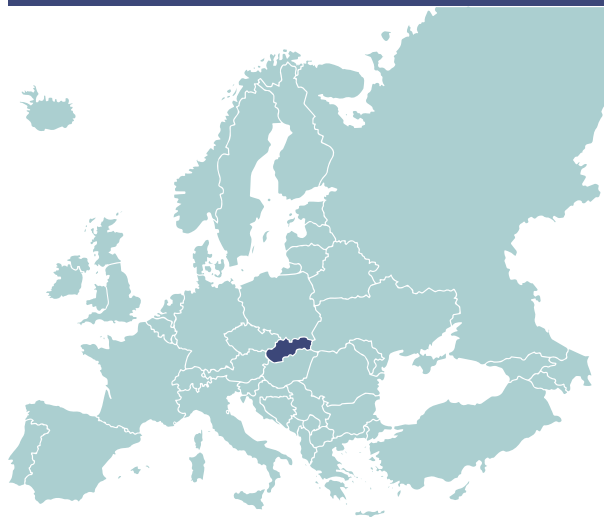
In Serbia, there are five centres for survivors of sexualised violence which operate 24/7. They are located in the hospitals of Kikinda, Zrenjanin, Novi Sad, Sremska Mitrovica, and Vrbas. The Centre for Victims of Sexual Violence at the General Hospital in Vrbas was opened in 2024. These centres are specialised in supporting women and girls over the age of 15 who have survived rape and attempted rape or other forms of sexualised violence. The centres provide survivors with specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention, and support for family members. In 2024, 67 cases of sexualised violence were reported, of which 22 survivors were minors. A total of 492 psycho-social support and psychological counselling services and 148 health and care services were provided. The most common perpetrators were acquaintances, friends (47%), then family members (19%), and then a married or extramarital partner (12%). All existing services are financed through a project supported by the United Nations Trust Fund to End Violence Against Women, and they are located only in major cities in the North of the country. In Serbia, there is

a SOS helpline for women survivors of sexualised violence run by the Association Women's Support Centre from Kikinda (tel.: +381 693044050); the helpline is not free of charge, does not offer multilingual support, and operates on working days from 8:00 to 16:00. In 2024, it received 31 calls.

5. PRIMARY PREVENTION SERVICES

In Serbia, there is a National Strategy for Preventing and Combating Gender-based Violence Against Women and Against Violence in the Family for the period 2021–2025. In the Strategy, there is a specific provision on prevention, even though the term "primary" is not used. The Strategy talks about the importance of and the forms of prevention, campaigns, information, and raising public awareness, as well as education and training of experts to act against various forms of violence, including that in the digital sphere. Nevertheless, only in the second half of 2024 the Action Plan for the period 2024–2025 was drafted, meaning that the activities planned for 2024 remain unrealised. In the National Action Plan, there is no definition of prevention. The funds allocated by the state for primary prevention are minimal and funds are not allocated for systemic actions aimed at prevention, but local and short-term actions such as campaigns, awareness-raising activities and educational activities. No evaluation of such activities is conducted. Most women's centres carry out activities aimed at empowering different groups of women to recognise violence and familiarise themselves with existing support services in the community, raising the capacities of institutional and non-institutional service providers, developing educational materials or advocating for educational programmes. Actions are mainly aimed at all types of violence, but there are also those specialised in the prevention of sexualised violence, digital violence, and violence at the workplace.

SLOVAKIA



GENERAL COUNTRY INFORMATION

Population	5,424,687
Female population	2,771,470
Member of Council of Europe (year)	1961
Member of European Union (year)	2004
Member of United Nations (year)	1993
CEDAW ratified (year)	1993
CEDAW optional protocol ratified (year)	2000
Istanbul Convention signed (year)	2022
Istanbul Convention ratified (year)	No

SUMMARY

There is one national women's helpline in Slovakia, which is free of charge, available 24/7 and offers multilingual support. There are ten women-only shelters with 273 beds and 22 women's centres in the country. Primary prevention is included in the National Action Plan for the Prevention and Elimination of Violence Against Women 2022–2027; Slovak public authorities collect limited output-based data, and no specific budget is allocated to primary prevention activities. The state collects data on women's specialist services, which is available upon request and is gathered by the Institute for Labour and Family Research together with the Coordinating-Methodical Centre for the Prevention of Violence against Women.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	2,527 calls 1,197 callers

There is one national women's helpline in Slovakia, called the National Helpline for Women Experiencing Violence (*Národná linka pre ženy zažívajúce násilie*, tel.: +421 800212212). It is run by the Institute for Labour and Family Research, operates free of charge, and is available 24/7. Therefore, the helpline meets the standards set by the Istanbul Convention. The helpline offers support in Slovak and English and provides assistance to women affected by all forms of gender-based violence, including physical, psychological, sexualised, and economic violence. It also receives calls from individuals seeking advice on supporting survivors or addressing related issues such as parental conflicts and mental health. Women experiencing intimate partner violence often contact the helpline repeatedly within a month. In 2024, the helpline received 2,527 contacts from 1,197 callers, with domestic violence being the most commonly reported form of violence. The helpline is funded through the European Social Fund under the Operational Programme Employment and Social Inclusion. In 2024, 17 women's centres reported operating a crisis helpline providing comprehensive services, charged at standard operator rates. Nine of them provided data on the number of calls, reporting a total of 1,543 calls. Most of these helplines operate only during working days.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	10
Total number of all shelters accessible to women	N/A
Existing number of beds	273
24/7 access	Most

There are ten women-only shelters in Slovakia, providing more than 273 beds.¹⁰¹ Several organisations have reported that the number of available places is insufficient, with four shelters specifically noting a shortage of capacity for women with multiple children. Most of the shelters offer 24/7 access. Accommodation is either free of charge for the first months or subject to small fees, and later contributions are usually adjusted according to the woman's income and financial situation. Women are generally allowed to stay in the shelters for up to one year, though this may be extended in exceptional cases if necessary. Providers often emphasise that longer stays can be counterproductive, as the aim is to support women in stabilising their lives and moving towards independent living. Services provided by shelters include casework, counselling and psychological support, legal advice, referrals and cooperation with external services, as well as practical advice.

Most women-only shelters can be accessed by and are accessible to women with uncertain residence permits and undocumented women, refugee women, homeless women, women with physical, cognitive and/or sensory disabilities, older women, transgender women, women with older sons and women with children with disabilities. Some shelters can accommodate women with substance

abuse issues. No shelters are equipped to support unaccompanied girls under the age of 18. There are no specialised shelters in Slovakia designated exclusively for specific groups of survivors. Most shelters provide support to all women in need, regardless of their background or circumstances, and some shelters report that their staff receive specialised training to work with certain groups, including women with disabilities, older women, migrants, and Black and minority ethnic women.

Women-only shelters in Slovakia are funded through the state, donations and international sources. Five shelters are operated by women's NGOs with a gender-specific or feminist approach, while the remaining five are run by other NGOs. Shelters are available in all regions of the country. In 2024, shelters reported having to decline referrals due to lack of capacity in accommodating women with children, no space to support the survivor, or because survivors were deemed ineligible for support. The most common forms of violence reported by survivors were physical, psychological, and economic violence. Data on the total number of women and children accommodated is incomplete, as not all shelters collect this information. Data provided by five shelters indicates that 454 clients were accommodated in 2024.

In addition to women-only shelters, other types of shelters are available in Slovakia that may provide support to survivors of violence against women.

❖ 3. WOMEN'S CENTRES

Total number of centres	22
Entities running women's centres	Women's NGOs (13 centres) Other NGOs (7 centres) State (1 centre) Other (1 centre)

There are 22 women's centres in Slovakia. Of these, 13 provide services exclusively for women survivors of violence against women, while nine are open to all survivors of gender-based violence, including women. The services most frequently provided by women's centres include counselling and psychological support (available in all centres) and referrals or collaboration with other services (20 centres). Legal advice is offered in 15 centres, financial and social welfare support or advocacy in 10, housing advice in nine, and employment-related support in seven. Eleven centres provide representation at court, police, or social services, while five centres also offer residential support services such as emergency accommodation. Four women's centres also provide specialised support for survivors of sexualised violence, including needs assessments, specialist psychological care, community awareness and prevention activities, and legal advice.

¹⁰¹ One shelter did not provide data on the number of beds.

Most women's centres can be accessed by and are accessible to women with uncertain residence permits and undocumented women, refugee women, women with physical, cognitive and/or sensory disabilities, older women, transgender women and unaccompanied girls under the age of 18. There are no women's centres in Slovakia specialised exclusively in supporting survivors of specific forms of violence nor dedicated to specific groups of women. Women's centres generally provide services to all women affected by gender-based violence, regardless of their background or specific vulnerabilities. While some centres report that staff have received training to better support such groups, these are exceptions, and the overall approach remains general rather than specialised.

Women's NGOs are running 13 women's centres in Slovakia, seven are run by other NGOs, one by the State and one intervention centre for survivors of domestic violence is operated by a law firm. Main funding sources are the state, donations and international funding and women's centres are located in all regions of the country. The total number of women supported by all women's centres in 2024 was 3,845.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are no services for women and girls who have experienced sexualised violence in Slovakia. Some women's centres can provide support to survivors of sexualised violence.

5. PRIMARY PREVENTION SERVICES

Slovak public authorities collect some data on primary prevention activities, but this is limited to outputs such as the number of campaigns or trainings, rather than systematic, outcome-based monitoring. The National Action Plan for the Prevention and Elimination of Violence Against Women 2022–2027 includes a dedicated prevention pillar, as well as provisions on integrated policies and data collection. However, it remains unclear to what extent the prevention component incorporates evidence-based strategies addressing structural issues such as gender inequality and power imbalances. The Coordinating-Methodical Centre (KMC) for Gender-Based and Domestic Violence is developing methodologies, standards, and systems for primary prevention, with the aim of embedding gender-sensitive and rights-based approaches across professional sectors.

The Slovak Republic, through the KMC and the national platform *Zastavme násilie* (Let's stop violence), has adopted an official definition of primary prevention, which is also referenced in the National Action Plan. No specific budget is allocated to primary prevention activities at the national, regional, or local level. Current prevention measures include school-based programmes, feminist self-defence training, and education of future professionals, with most activities organised by non-governmental organisations.

SLOVENIA



GENERAL COUNTRY INFORMATION

Population	2,118,697
Female population	1,055,520
Member of Council of Europe (year)	1993
Member of European Union (year)	2004
Member of United Nations (year)	1992
CEDAW ratified (year)	1992
CEDAW optional protocol ratified (year)	2004
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2015

SUMMARY

There is one national women's helpline in Slovenia, which is free of charge, operates 24/7, and offers multi-lingual support. There are 29 women-only shelters with 445 beds available and three other shelters accessible to survivors of violence with 28 beds. The country also has 16 women's centres, one rape crisis centre, and one crisis medical service for survivors of sexualised violence. Slovenia has a National Action Plan, which does not include a definition of primary prevention but outlines some primary prevention activities. The state collects data on women's specialist services, and this is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/ callers in 2024	2,929 calls/contacts 2,778 callers

There is one national women's helpline in Slovenia, called SOS Helpline for Women and Children-Victims of Violence (*SOS telefon za ženske in otroke – žrtve nasilja*, tel.: +386

801155). It operates free of charge and is available 24/7, meeting the standards of the Istanbul Convention. The helpline offers support in Slovenian, English, Serbian, Croatian and Bosnian. It provides support to survivors of psychological, physical, sexualised and economic violence, as well as stalking and cyberbullying. The helpline is funded by the Ministry for Labour, Family, Social Affairs and Equal Opportunities, as well as by the Employment Service of Slovenia, the Foundation for Financing Disabled and Humanitarian organisations and local municipalities. In 2024, the helpline provided a total of 2,929 counselling services, through telephone calls, emails, and chat service. The number of people who contacted the helpline was 2,778. In 2024, the most common forms of violence reported by the callers to the state-wide women's helpline were physical, sexualised and psychological violence. There are no other helplines supporting survivors of violence against women in Slovenia.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	29
Total number of all shelters accessible to women	32
Existing number of beds	473
24/7 access	Some

There are 29 women-only shelters in Slovenia, offering a total of 445 beds. The number of beds currently meets the country's needs. However, the cases are becoming more complicated, involving factors such as migration, mental health challenges, substance abuse and special needs, and requiring more specialised support. Therefore, individual housing solutions would be more suitable than the existing model of shared shelter accommodation. Only the three Crisis Centres for Victims of Violence can provide 24/7 access to accommodation. In other shelters, new admissions cannot take place outside the designated hours and procedures. Women are expected to pay for their accommodation in women-only shelters, based on a monthly contribution that amounts to 30% of the financial social assistance. An additional 10% is added for the second child, and 5% for the third child. However, if the survivors are experiencing financial hardships, the payment for accommodation can be adjusted according to their financial means.

The length of the stay is determined based on individual cases; however, it cannot exceed the maximum period of one year. Crisis Centres, which provide immediate accommodation and protection as a first response, can accommodate the survivors for up to three weeks. After this period, women are typically transferred to shelters that offer long-term housing support. Women-only shelters offer in-house services such as casework, counselling and psychological support, referrals and collaborations with other services and practice advice. Although legal advice is not formally provided within the shelters, counsellors support women by referring them to legal representatives.

All women-only shelters are equipped to support women with uncertain residence permits/undocumented women, refugee women, homeless women, women with cognitive disabilities, older women and women with children with disabilities. Women with sensory disabilities can access most of the shelters. Some women-only shelters are equipped to support women with physical disabilities, women with substance abuse issues, transgender women and women with older sons, while no shelters can accommodate unaccompanied girls under the age of 18. One shelter is specialised in supporting women with substance abuse issues who are survivors of violence.

The Ministry for Labour, Family, Social Affairs and Equal Opportunities is the main funder for 16 women-only shelters, while other funds come from the Foundation for Financing Disabled and Humanitarian Organisations, municipalities, and donations. Women's NGOs with a feminist approach run five women-only shelters; nine shelters are run by other NGOs, eight are run by the state (Centres for Social Work), and seven are run by faith-based organisations (Caritas). Women-only shelters are primarily located in cities or smaller urban areas, while rural areas do not provide such services. If women-only shelters had to decline referrals, it was primarily due to the lack of space/capacity to support the survivor or to accommodate survivors with children. In some cases, referrals were also declined if the survivor was ineligible for support. In 2024, the most common forms of violence reported by the survivors to the women-only shelters were physical, sexualised and psychological violence. In 2024, the estimated total number of accommodated women and children in women-only shelters is around 600.

There are three other shelters available in Slovenia, in addition to women-only shelters, offering a total of 28 beds. One is a shelter for students in social distress, including survivors of violence, that offers nine beds regardless of the gender of the survivor and is operated by a student NGO. The second one is a Crisis Centre for survivors of trafficking and is run by the Slovenian Caritas and provides accommodations to individuals of all genders, offering 13 beds in total. In the past two years, the centre has accommodated mainly male survivors of labour trafficking, whereas previously it mostly sheltered female survivors of forced prostitution. The third shelter also supports survivors of trafficking and offers six beds, available to all genders.

❖ 3. WOMEN'S CENTRES

Total number of centres	16
Entities running women's centres	Women's NGOs (8 centres) Other NGOs (3 centres) Faith-based organisations (3 centres) State (2 centres)

Slovenia has 16 women's centres, three only for women survivors of violence, and 13 for all survivors of

gender-based violence, including women. Women's centres provide counselling and psychological support, financial and social welfare support, housing advice, and referrals/collaborations with other services. Thirteen women's centres also provide representation at court/police/social services, while six centres provide residential support and emergency accommodation in safe houses. Women's centres also provide specialist support services for survivors of sexualised violence, such as needs assessment, specialist psychological care, legal advice, community awareness and education for prevention, and specialist sexualised violence advocacy services.

All women's centres are accessible to women with uncertain residence permits/undocumented women, refugee women and older women. Most women's centres are equipped to support women with cognitive disabilities, while some centres can be accessed by and are accessible to women with physical disabilities, women with sensory disabilities and unaccompanied girls under the age of 18. Some centres are also specialised in assisting specific groups, such as undocumented and refugee women, older women, and minors. In Slovenia, the *Stigma Safe House* is intended for women with substance abuse issues who are survivors of violence. Some organisations work with the Roma population and also address violence, providing support and assistance to survivors. In addition, the organisation Legebitra focuses on LGBTQI+ topics, and also offers legal and psychosocial counselling and addresses issues related to experiences of violence.

Eight women's centres are run by women's NGOs, three centres are run by other NGOs, three centres are run by religious institutions, while the state runs two centres. Women's centres are funded by the state, donations and international funding. They cover all regions of Slovenia and in 2024, they supported approximately 14,000 women.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

Slovenia has one rape crisis centre for survivors of sexualised violence called the Power Association (*Združenje za moč*). The association offers individual in-person counselling, telephone and online counselling, support groups and counselling services for minors and adolescents. The crisis centre also provides advocacy services, accompaniment during institutional procedures, assistance with document preparation and support with communication with institutions. All services are free of charge. Moreover, the Association for Nonviolent Communication has a specialised programme for survivors of sexualised violence, including counselling, accompaniment during institutional/court procedures and support in communication with institutions. In Slovenia, there is also a Clinical Forensic Medicine Ambulance, which is part of the Institute of Forensic Medicine. The ambulance offers services such as specialist forensic and medical care, and examinations are also later carried out at the emergency gynaecology

department. In Slovenia, survivors of sexualised violence must file a criminal report for the examination to be carried out, and medical professionals are obliged to report the case, which can prevent the survivors from accessing a forensic examination if they are not ready or do not wish to file a report. Women's organisations in the country are often the first contact point for survivors, and they provide support, counselling, and accompaniment to other services. The rape crisis centre is funded by the state and donations and is located in two major cities in Slovenia (Ljubljana and Maribor). The Clinical Forensic Medicine Ambulance is funded by the state.

5. PRIMARY PREVENTION SERVICES

Public authorities do not collect data on primary prevention activities. Slovenia has a National Action Plan that does not define primary prevention, but outlines primary prevention activities, such as inclusion of specific topics in the revised school curricula, including gender equality, mutual respect, non-stereotypical gender roles, non-violent communication, gender-based violence against women and girls, the right to personal integrity and the consequences of peer violence. In December 2024, the government also issued an Action Plan for 2024 and 2025, which provides a more detailed outline of prevention measures. Funds for primary prevention are limited and mostly project-based. Activities conducted in Slovenia include empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, bystander intervention training and training for practitioners, self-care activities and awareness-raising campaigns. Furthermore, the Association for Nonviolent Communication and similar organisations conduct workshops on multiple topics such as peer violence, sexualised violence, and online violence, which are primarily adapted to the needs of young people. These organisations carry out an internal evaluation for each workshop.

SPAIN



GENERAL COUNTRY INFORMATION

Population	48,619,695
Female population	24,792,824
Member of Council of Europe (year)	1977
Member of European Union (year)	1986
Member of United Nations (year)	1955
CEDAW ratified (year)	1984
CEDAW optional protocol ratified (year)	2001
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2014

SUMMARY

There is one national women's helpline in Spain, which is available free of charge, 24/7, and offers multilingual support. Additionally, there are other regional helplines in the country. There are 307 women-only shelters in Spain, offering 3,810 beds. There are approximately 475 women's centres in Spain, and rape crisis centres are available across the country, although their current distribution does not yet meet the planned objective of ensuring at least one centre in each province. Primary prevention is included in the State Pact against Gender Violence and in the State Strategy to Combat Gender Violence, along with a dedicated budget. The state collects data on women's specialist services and on primary prevention activities, which are available to the public.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	58,928 calls between January and June 2024

There is one national women's helpline in Spain called Service of attention to all forms of violence against women (*Servicio de atención a todas las formas de violencia contra las mujeres*, tel.: +34 016). The helpline is run by the Ministry of Equality, is free of charge and available 24/7, meeting the standards of the Istanbul Convention. It offers multilingual support in Spanish, Catalan, Basque, Galician, English, French, German, Portuguese, Mandarin Chinese, Russian, Arabic, Romanian, Bulgarian, Tamazight and 39 other languages through a tele-translation service. It provides help for all forms of violence against women and is funded by the state. It also provides a WhatsApp number for chats (tel.: +34 600000016) and is accessible to women with hearing disabilities through different channels, such as the application TOBMOVIL (tel.: +34 900116016), SVisual (www.svisual.org) and Telesor (www.telesor.es). Between January and June 2024, the helpline received 58,928 calls. The most common forms of violence reported by callers were domestic violence, sexualised violence and economic violence.

Additionally, there are other helplines supporting women survivors of violence against women in Spain. Among them is the helpline of the ANAR Foundation (*Fundación ANAR*, tel.: +34 900202010), which provides psychological, social, and legal assistance for children of women in intimate partner violent relationships or children in abusive relationships themselves, and a helpline for survivors of trafficking for the purpose of sexual exploitation (tel.: +34 900105090). Other regional helplines, which operate 24/7, include Andalusia's *Teléfono de atención a la mujer 24 Horas* (tel.: +34 900200999), Aragon's helpline (tel.: +34 900504405), Specialised Care and Support Service for Victims of Gender-Based Violence, run by *Institut Balear de la Donna* (tel.: +34 900178989, WhatsApp number: +34 639837476), Canary Islands' *Servicio de Atención a Mujeres Víctimas de Violencia de Género en el Centro Coordinador de Emergencias y Seguridad* (CECOES) which is run by the Canary Institute for Equality (tel.: +34 112), Catalonia's regional helpline (tel.: +34 900900120) which is free and confidential, Castille – La Mancha's emergency helpline (tel.: +34 900100114), the helpline of the community of Madrid (tel.: +34 012), and the one of the community of Valencia (tel.: +34 900 58 08 88), which operates 24/7 and is run by the *Centro Mujer* around the clock, the women's helplines of Castille and León (tel.: +34 012), Navarra (tel.: +34 112), Galicia (tel.: +34 900400273), which operates 24/7, La Rioja's information helpline for women (tel.: +34 900711010), the Basque Country's *Servicio de Atención Telefónica a las Mujeres Víctimas de la Violencia contra las Mujeres* (SATEVI) (tel.: +34 900840111), and the emergency telephone assistance service for abused women in Murcia (tel. +34 112).

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	307
Total number of all shelters accessible to women	N/A
Existing number of beds	3,810
24/7 access	Most

According to national statistics from 2022, there are 307 women-only shelters in Spain with 3,810 beds available, and most of them offer 24/7 access. These statistics only include public shelters and do not necessarily reflect all shelter services available in the country. There are three different kinds of shelters: emergency shelters/crisis centres (*Centros de Emergencia*), which have 24/7 access, temporary shelters/houses (*Casas de Acogida*) and supervised/transitional housing, which allows for a longer period of stay. Normally, in public shelters, women are not expected to pay for their accommodation. The average length of stay in shelters varies across regions and according to the kind of shelter. For instance, in 2024, 72.5% of users in the Valencian Community were able to stay with their children up to a full year. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services and legal advice.

While all shelters can accommodate older women, most of the shelters can also be accessed by and are accessible to refugee women, women with physical and sensory disabilities, women with substance abuse issues, transgender women and women with children with disabilities. Some shelters are equipped to support homeless women, women with cognitive disabilities and women with older sons. There are also shelters specialised in supporting specific groups of survivors, including migrant women and women without a residence permit, survivors of trafficking and sexual exploitation, women with disabilities, LGBTQI+ people, and migrant women survivors of domestic violence.

Public women-only shelters are funded by public authorities, while other women's shelters additionally rely on donations and international funding. Women-only shelters are run by various stakeholders, including women's NGOs with a gender-specific/feminist approach, as well as other NGOs, public authorities, as well as private companies. There are shelters in all autonomous communities, but in rural and mountain areas, some users are forced to travel 80-100 km to find a place. If women-only shelters had to decline referrals, the main reasons were the lack of space/capacity to support the survivor and to accommodate survivors with children, and shelters not being adequately equipped for the needs of the women/children. In some cases, women are not accommodated because they do not meet the criteria, or because the shelter is not prepared for severe addictions or severe behavioural disorders. The most common forms of violence reported by survivors to the women-only shelters in 2024 were domestic violence, physical violence and psychological violence. According to national statistics, the number of survivors supported by public women-only shelters in 2022 is approximately 7,874 women and 5,384 children, although some regional data is missing.

Additionally, other shelters in Spain can provide accommodation to survivors of gender-based violence, but they are not specialised women-only shelters.

❖ 3. WOMEN'S CENTRES

Total number of centres	475
Entities running women's centres	Women's NGOs Other NGOs State Private companies

According to national statistics, there are 475 women's centres in Spain, which specifically support women survivors of violence.¹⁰² Women's centres provide counselling and psychological support, referrals and collaboration with other services, legal advice, social welfare support/advocacy, housing and employment advice, needs assessment, specialist advocacy services, community awareness and education for prevention. Women's centres also provide support to survivors of sexualised violence.

All centres can provide support to older women; at the same time, most of them are also accessible to and can be accessed by women with uncertain residence permits/undocumented women, refugee women, and women with physical and sensory disabilities. Some centres are also equipped to support women with cognitive disabilities and transgender women, while no centre can support unaccompanied girls under the age of 16. Additionally, in Spain, there are centres specialised in supporting specific groups of survivors.

Women's centres in Spain are mostly run by women's NGOs, followed by other NGOs, public authorities and, in some cases, private companies. They are funded through public funding and are present in all regions. The available public statistics report that in 2024, women's centres supported 70,597 women.

☂ 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

By June 2025, 54 rape crisis centres operating 24/7 were expected to be established across the country. While many of these centres are confirmed to be functional, the status of others cannot be verified. According to NGO sources, at least 23 centres are currently operational. Rape crisis centres provide a range of services, including psychosocial and legal needs assessments, emergency psychotherapy, accompaniment to police stations, hospitals and courts, as well as awareness-raising activities in educational and community settings. Additionally, there are two specialised helplines belonging to two 24-hour rape crisis centres in Spain.

¹⁰² According to national statistics, the 2022 data indicate a substantial decrease compared to the figures reported in 2020 and cited in the previous WAVE Country Report. While 920 women's centres were recorded in 2020, 475 were documented in 2022. No explanation for this decline has been provided in the available sources.

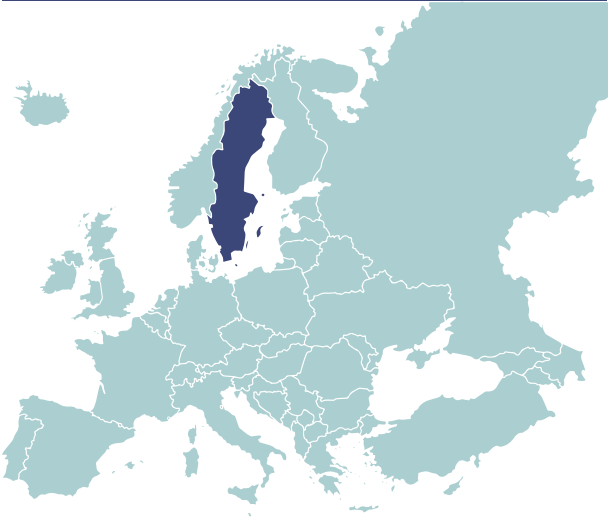
Most public hospitals and medical centres follow specific protocols for responding to cases of sexualised violence and can provide forensic and medical care for survivors. However, they generally lack dedicated facilities or specialised personnel for these procedures. Funding for the establishment of the 54 centres was provided by the European Union, while the responsibility for ongoing maintenance lies with public authorities.

In terms of geographical distribution, the model foresees one rape crisis centre in each Spanish province. Larger cities such as Madrid, Barcelona, Valencia, and Seville host several 24-hour centres, while in rural areas service availability remains limited. Currently, rape crisis centres provide support to women and girls aged 16 and above.

5. PRIMARY PREVENTION SERVICES

Public authorities collect data on primary prevention, although in a fragmented way, and they are included in the Monitoring System of the State Pact against Gender Violence and in the State Strategy to Combat Gender Violence 2022–2025, which has a system of indicators to measure progress, including on prevention. The State Pact against Gender Violence (2017, updated in 2024–2025) includes Axis 4 on prevention, with 90 specific measures for primary prevention. The State Strategy 2022–2025 addresses these measures and establishes lines of action for primary prevention, which go beyond awareness-raising. In the State Strategy, the definition of primary prevention from the Istanbul Convention is adopted. In Spain, there is a specific budget allocated to the implementation of primary prevention activities, while autonomous communities and local councils often co-finance prevention projects (school programmes, community workshops) with their own resources, which guarantees continuity and regional coverage. Primary prevention activities conducted in the country include feminist self-defence trainings, empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, training for primary prevention practitioners, bystander intervention training, self-care activities and awareness-raising campaigns or programmes. According to the State Pact Monitoring Report, the implementation of prevention activities is tracked; however, there is no detailed evaluation of either the activities themselves or their impact. In addition, specialised NGOs carry out evaluations of school and community projects with public co-funding.

SWEDEN



GENERAL COUNTRY INFORMATION

Population	10,551,707
Female population	5,239,188
Member of Council of Europe (year)	1949
Member of European Union (year)	1995
Member of United Nations (year)	1946
CEDAW ratified (year)	1980
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2014

SUMMARY

There is one national women’s helpline in Sweden which is free of charge, operates 24/7, and provides multilingual support. The exact number of women-only shelters in Sweden is unknown, however, according to the National Board of Health and Welfare there are 257 shelters accessible to women, with at least 2,219 beds. The number of women’s centres is also unknown but taking into account only Unizon’s and Roks’ member organisations, there are over 200 centres providing support to women and girls. Sweden has services for women and girls who have experienced sexualised violence. Measures related to primary prevention are included in the National Strategy to Prevent and Combat Men’s Violence against Women 2017–2026, and there is a specific budget allocated to such activities. The state collects data on women’s specialist support services, including primary prevention activities, and this data is publicly available. It should however be noted that the National Board of Health and Welfare focuses on shelters in general and the collected data is gender neutral.¹⁰³

103 The terms that the National Board of Health and Welfare uses in their mapping report are, for instance, “shelters for persons subjected to violence”, “adult(s) subjected to violence”, and “children subjected to violence”.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	48,369 calls

There is one national women's helpline in Sweden called the National Women's Helpline (*Kvinnofridslinjen*, tel.: 116 016) which is free of charge and available 24/7. Sweden therefore meets the Istanbul Convention minimum standards for the provision of a national women's helpline. The helpline offers multilingual support through interpreters in different languages. Support via chat is also offered (in Swedish) on Mondays and Thursdays from 15:00–21:00 and on Sundays from 11:00–17:00. The helpline is run by the National Centre for Knowledge on Men's Violence Against Women (NCK), on behalf of the Swedish Government. *Kvinnofridslinjen* provides support to survivors of threats or physical, psychological or sexualised violence. The helpline is funded by the state, and a total of 48,369 calls were answered in 2024, with an average of 133 calls per day.

There are other helplines supporting survivors of violence against women in Sweden, such as *Terrafem* (tel.: +46 20521010) which is a feminist women's organisation that offers support in over 70 languages primarily to immigrant women of all ages, including legal support. Additionally, women's crisis centres and empowerment centres for young women and youth in Sweden also provide support via phone and chat. They get support requests not only from their home regions/municipalities but also from other regions in the country.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	N/A
Total number of all shelters accessible to women	257
Existing number of beds	2,219 ¹⁰⁴
24/7 access	Some

There are women-only shelters in Sweden but the exact number is not known as there are no accurate statistics on women-only shelters. The National Board of Health and Welfare conducted a new mapping of shelters in Sweden which was published in December 2024 and, similarly to the previous report published in 2020, the report does not divide shelters into women-only and others. According to the new report, the overall number of shelters has decreased from 282 in 2019 to 257 in the 2024. A total of 190 shelters answered the questionnaire for the national

mapping report, offering 2,219 beds for adults and 2,516 beds for accompanying children. The number of available bed spaces has increased since 2019, which can be explained by the fact that shelters have become larger. All 190 shelters that responded to the questionnaire accept women, and 54% also accept men. In the previous report from 2019, it was indicated that 38% of the shelters accepted men, and in 2016, that figure was 16%. Those shelters that accept men subjected to violence state that they do not place men together with women – instead, men are placed in separate apartments. Some of the shelters in Sweden offer 24/7 access and beneficiaries are not expected to pay for their accommodation. All shelters offer casework, counselling/psychological support, referrals/collaboration with other services, practical advice, and legal advice, although the extent of provided services varies by shelter.

Most shelters in Sweden can be accessed by and are accessible to refugee women, homeless women, women with physical or cognitive disabilities, older women (65 years and above), transgender women, and women with older sons (14 years and above). Some shelters can accommodate women with uncertain residence permits/undocumented women, women with sensory disabilities, women with substance abuse issues, women with children with disabilities, and unaccompanied girls under the age of 18. In addition, there are specific shelters for women with substance abuse issues, women subjected to prostitution, pornography and trafficking,¹⁰⁵ and women survivors of honour-based violence.

Funding for shelters comes from the state as well as donations. However, it should be highlighted that donations are not common and that only very few shelters have donations as their main source of funding. Shelters in Sweden are run by NGOs, private profit-driven actors, and municipalities. Although there are no aggregated national statistics on women-only shelter services for women survivors of violence in Sweden, there are women's NGOs with a gender-specific and feminist approach that provide women-only shelter services. These primarily operate within two women's networks, Unizon and Roks (*Riksorganisationen för kvinnojourer och tjejjourer i Sverige*). In 2024, Unizon had 149 member organisations, including 95 crisis centres and shelters for women, 40 empowerment centres for young women and youth, 11 specialised support services for survivors of sexualised violence, one specialised support organisation for children, and two others. Roks had 64 member organisations in 2024, including 41 crisis centres and shelters for women and 23 empowerment centres for young women. There are also several women's NGOs outside of Unizon or Roks, for instance, Women's shelter Jamtland County (*Kvinnojouren Jämtlands län*), Uppsala Women's Shelter (*Uppsala Kvinnojour*),

¹⁰⁴ This number only includes the beds in the 190 shelters which answered the questionnaire for Sweden's National Board of Health and Welfare shelter mapping report.

¹⁰⁵ In Sweden, prostitution is considered a form of men's violence against women by law.

and Talita, which is a safe house for women in prostitution, pornography, and other forms of sexualised exploitation.¹⁰⁶ There has been an increase in the proportion of shelters run by private profit-driven actors in Sweden. In 2012, only 8% of shelters were run by private profit-driven actors. This figure grew to 37% in 2019, and in the latest mapping report, it stands at 47%. At the same time, the proportion of non-profit shelters has decreased from 54% in 2019 to 46% in the first quarter of 2024.

According to the general data on shelters available at the national level, there are shelters for women and other persons subjected to violence in all regions of Sweden, but not in all municipalities. In 2024, women stayed an average of 61 days in Unizon's shelters, compared to 50 days in 2022, 55 days in 2020, and 66 days in 2018. The main reasons why shelters had to decline referrals were no space/capacity to support the survivor, the survivor being ineligible for support, and the shelter not being adequately equipped for the needs of the women/children. For example, many shelters cannot accommodate or provide support to women with active substance abuse or severe mental health problems. According to Unizon's data, physical violence and psychological violence were the most common forms of violence reported by survivors to shelters. These forms of violence were commonly reported to other women's shelters not part of Unizon alongside other forms of violence such as sexualised and economic violence. Moreover, two of the most common requests that Unizon's member organisations receive from women survivors are housing support and child custody related support. There are no nation-wide statistics on this matter. In 2024, a total of 679 women, nine young adults, and 672 children were accommodated in Unizon's women's shelters.

❖ 3. WOMEN'S CENTRES

Total number of centres	200
Entities running women's centres	State Women's NGOs Other NGOs

There are women's centres in Sweden although the exact number is unknown as there is no aggregated data on women's centres on a national level. Taking into account only Unizon's and Roks' member organisations, as well as the data provided by the National Centre for Knowledge on Men's Violence Against Women (NCK), there are over 200 crisis and empowerment centres providing support to women and girls. Services provided by women's centres include counselling/psychological support, legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, representation at court/police/social services, and residential support services. Women's centres also provide specialised support for survivors of sexualised

violence, including needs assessment, specialist psychological care, specialist sexualised violence advocacy series, community awareness and education for prevention, as well as legal advice.

Most women's centres can be accessed by and are accessible to refugee women, women with physical or cognitive disabilities, older women (65 years and above), transgender women, and unaccompanied girls under the age of 18. Some women's centres can support women with uncertain residence permits/undocumented women and women with sensory disabilities. There are also women's centres specialised in supporting survivors of forced marriage, honour-based violence, female genital mutilation, and trafficking. Additionally, there are specialised women's centres for Black and minority ethnic women, women survivors of rape and incest, women subjected to prostitution, pornography and other forms of sexualised exploitation,¹⁰⁷ as well as women with substance abuse issues.

Funding for women's centres comes mainly from the state and donations, however, there are only a few organisations that have donations as their main source of funding. Women's centres are run by the state and NGOs, and there are women's centres in all regions of Sweden, but not in all municipalities. In 2024, Unizon's member organisations had nearly 147,000 support contacts, and the majority of those seeking support (88%) were women.

🚫 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for survivors of sexualised violence in Sweden including rape crisis centres, sexual violence referral centres, crisis/medical services, as well as specialised helplines. Although there is no aggregated data on the number of specialised services, women's shelters, crisis centres and empowerment centres for women and girls do provide specialised support for survivors of different forms of sexualised violence and exploitation. According to Unizon's statistics from 2024, there are 11 member organisations providing services for survivors of sexualised violence. Sexual violence referral centres exist in some dedicated hospital settings, for instance, Emergency Clinic for Rape Victims at Södersjukhuset (SÖS) in Stockholm, or out-patient clinic *Kvinnofridsmottagningen* at University Hospital. Available services include specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, as well as legal advice. Funding for sexualised violence services comes from the state and donations. That said, donations as the main source of funding are not common. Sexualised violence services are located just in major cities and are available to all survivors of sexualised violence. Specialised helplines include Big Sister (*Storasyster*)

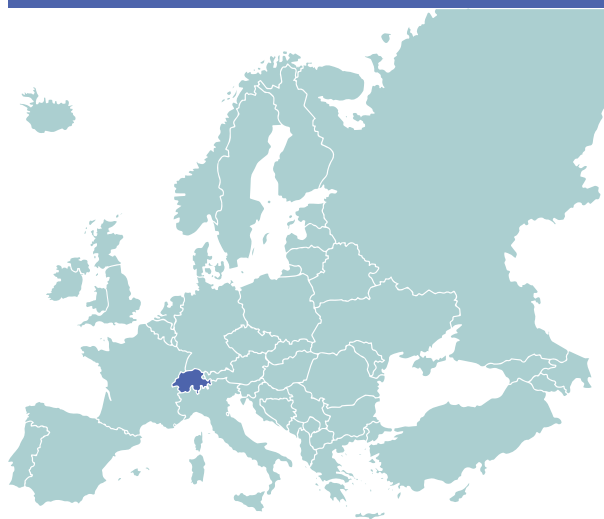
106 In Sweden, prostitution is considered a form of men's violence against women by law.
107 In Sweden, prostitution is considered a form of men's violence against women by law.

available via chat both for survivors of sexualised violence and for their relatives/close persons. Both chats are open daily, Sunday to Thursday from 19.00–21.00 and Friday to Saturday from 13.00–15.00. Additionally, Rise, the National Association against Incest and other Forms of Sexual abuse in Childhood, is a helpline and chat for survivors of sexualised violence and for their relatives/close persons. The chat is open every day except for Fridays from 20.00–21.00. The telephone helpline (tel.: +46 86960095) is open on Mondays from 21.00–22.00, Tuesdays and Thursdays from 19.00–20.00 and on Wednesdays from 14.00–15.00. *Ellencentret* is an organisation supporting survivors of sexualised exploitation between the ages of 15 and 25 which operates a chat that is open on Mondays from 13.00–15.00, Wednesdays from 15.00–17.00 and Thursdays from 17.00–19.00. Finally, *Alltid sedd* (Always Seen/Visible) provides support to survivors of sexualised exploitation via a chat that is open every Tuesday and Thursday from 19.00–21.00.

5. PRIMARY PREVENTION SERVICES

Public authorities such as the Gender Equality Agency in Sweden do collect data and follow up on different primary prevention programmes and efforts. Additionally, research institutes and universities conduct evaluation of primary prevention programmes and efforts on behalf of the Gender Equality Agency. The National Strategy to Prevent and Combat Men's Violence against Women 2017-2026 includes specific provisions on primary prevention. However, there is no official definition of primary prevention on a national level. The Gender Equality Agency, as well as other authorities and organisations, use different definitions that overlap to a large extent. There is a specific budget allocated to primary prevention activities and the state does fund such activities which include feminist self-defence trainings, empowerment activities for girls and women, primary prevention activities specifically targeting men and boys, school-based primary prevention programmes, trainings for primary prevention practitioners, bystander intervention training, and awareness-raising campaigns. Different activities and primary prevention programmes/efforts are common in Sweden, especially in schools. These programmes are usually run by NGOs, but NGOs do also train schoolteachers and other professionals working with children and youth to perform preventive work themselves. There are also programmes that specifically focus on consent-based and pornography-critical sex education.

SWITZERLAND



GENERAL COUNTRY INFORMATION

Population	8,962,258
Female population	4,510,726
Member of Council of Europe (year)	1993
Member of European Union (year)	No
Member of United Nations (year)	2002
CEDAW ratified (year)	1997
CEDAW optional protocol ratified (year)	2008
Istanbul Convention signed (year)	2013
Istanbul Convention ratified (year)	2017

SUMMARY

There is no national women's helpline in Switzerland, and support is provided by the helplines of women-only shelters and victim aid services. There are 23 women-only shelters with 462 beds accessible to women. In Switzerland, there are 24 women's centres supporting survivors of violence, as well as one rape crisis centre, around 60 sexual violence referral centres, and two crisis/medical services for survivors of sexualised violence. Switzerland has a National Action Plan on the implementation of the Istanbul Convention for the period 2022–2026, which includes provisions on primary prevention. The state does not collect data on women's specialist services. The Federal Statistical Office compiles statistics on victim assistance, while the umbrella organisation of women's shelters in Switzerland and Liechtenstein (*Dachorganisation der Frauenhäuser Schweiz und Liechtenstein, DAO*) collects data from its members.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	0
Free of charge	—
24/7 service	—
Multilingual support	—
Total number of calls/callers in 2024	—

Switzerland does not currently have a state-wide women's helpline; the introduction of a national 24-hour helpline is planned for May 2026, with implementation left to the individual cantons. Therefore, currently, the country does not meet the Istanbul Convention standards for the provision of a national women's helpline. Some cantons are considering cooperation with Helping Hand (*Dargebotene Hand*), a helpline providing emotional first aid by trained volunteers, though it is not specialised in violence-related issues. At present, support is provided through a range of helplines belonging to women's shelters and victim aid services. Victim aid helplines are available to all genders and for all types of survivors, but they only function during office hours. Outside of these hours, support is mostly provided by helplines operated by women's shelters. However, not every canton has a women's shelter helpline.

🏠 2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	23
Total number of all shelters accessible to women	N/A
Existing number of beds	462
24/7 access	All

Switzerland has 23 women-only shelters with a total of 462 beds. All shelters provide 24/7 access; while not all have a night team on-site, they maintain at least an on-call availability. Women are generally not expected to pay for accommodation if the survivor meets the criteria for victim aid, meaning the case involves domestic violence and a risk of danger. Costs are covered in line with the subsidiarity principle of victim aid law: if no other resources are available, victim aid covers all costs. Emergency assistance (*Soforthilfe*) ensures that all costs are covered for the first 35 days, after which financial support can be extended depending on the survivor's situation. Women-only shelters and counselling centres often help to facilitate this process to ensure swift and uncomplicated support. On average, women can stay in women-only shelters for up to three months, but in practice, there is no strict limit if the situation requires a longer stay, though cantonal practices differ. Victim aid services fund the initial 35 days of emergency assistance, and longer-term assistance can usually be approved. In 2024, survivors stayed in the shelters on average 49 days. Women-only shelters provide a range of in-house services, including casework, counselling and psychological support, legal advice, practical advice, as well as referrals and collaboration with other services.

Most women-only shelters in Switzerland can accommodate women with uncertain residence permits and undocumented women, refugee women, homeless women and women with older sons. Some can be accessed by and are accessible to women with physical, cognitive and/or sensory disabilities, women with substance abuse issues, older women, transgender women and women with children

with disabilities. There is only one shelter equipped to support unaccompanied girls under the age of 18, the *Mädchenhaus Zürich*. The NGO FIZ (*Fachstelle Frauenhandel und Frauenmigration*) in Zurich has eight shelters for all survivors of trafficking.

Women-only shelters in Switzerland receive funding from state contributions from cantonal or local authorities, though the level of funding varies significantly between regions. Most shelters receive basic or project-based funding, but they are often also reliant on other income sources such as donations. Only three women-only shelters are adequately funded by the public sector. The majority of shelters in the country are run by women's associations or foundations with a gender-specific or feminist approach, including the women-only shelters and the one girls' shelter that are members of the national umbrella organisation *Dachorganisation der Frauenhäuser der Schweiz und Liechtenstein* (DAO), as well as the specialised FIZ centre in Zurich. At least one shelter is run by a faith-based organisation, namely the Salvation Army. Shelters exist in most regions of the country, covering over 50% of the territory, but the level of provision differs by canton.

In 2024, the most common forms of violence reported by survivors to women-only shelters were domestic violence, physical violence, and psychological violence. If shelters had to decline referrals, it was due to a lack of space or insufficient capacity to support the survivor, and in some cases, because they were not adequately equipped to meet the specific needs of the survivors. According to women-only shelters' statistics, more than 72% of those seeking protection were referred elsewhere, including due to the lack of space (35%), high risk (2%), health issues (2%) or because their canton of residence did not grant cost coverage (8%).¹⁰⁸ In total, 2,676 women and children were accommodated in women-only shelters in 2024, based on figures reported by members of DAO.

❖ 3. WOMEN'S CENTRES

Total number of centres	24
Entities running women's centres	State Women's NGOs

Switzerland has 24 women's centres for survivors of violence, most of which only support women, while others are open to all survivors. Women's centres are present in all cantons, either as government-run information centres for gender issues and survivors of violence or as NGO-run counselling centres for women. All centres provide counselling support, legal advice, financial and social welfare support, and referrals or collaboration with other services. Other services provided are representation before the court and police, and psychological support. Centres also support survivors of sexualised violence through specialist psychological care and offer community awareness

108 DAO yearly report 2024, accessible at: https://www.frauenhaeuser.ch/sites/default/files/2025-06/250610_DAO_Jahresbericht_2024.pdf

and education for prevention. Seventeen centres are funded by the state, while other women's centres are mostly run thanks to donations.

Most women's centres can be accessed by and are accessible to women with uncertain residence permits and undocumented women, refugee women, women with physical disabilities and older women. Some women's centres are equipped to support women with cognitive and/or sensory disabilities, transgender women and unaccompanied girls under the age of 18. Additionally, five centres are equipped to support survivors (at risk) of honour-based violence, female genital mutilation, and/or trafficking in persons, and one women's centre is equipped to support survivors (at risk) of forced marriage.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Switzerland, there are services for women and girls who have experienced sexualised violence. These include one rape crisis centre, around 60 sexual violence referral centres, and two crisis/medical services. There is one sexual violence referral centre in the canton of Bern, while the canton of Zurich has introduced a model with forensic nurses. In most cases, forensic and medical care are provided in hospitals, with forensic nurses being an exception.

The available support includes specialist forensic and medical care, needs assessments, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention and legal advice. Funding comes mainly from state authorities and donations. Sexualised violence services are available in most regions of Switzerland, though they are less present in rural and remote areas. They support both young women and children (under 18) as well as adult women survivors. The association Violence Que Faire manages a website that offers free and anonymous online counselling services, which are available to survivors, perpetrators, or witnesses. A specific section of the website is dedicated to young people and LGBTQI+ people.

5. PRIMARY PREVENTION SERVICES

In Switzerland, public authorities do not collect data on primary prevention activities. There is, however, an official National Action Plan on violence against women and girls (2022–2026) that includes prevention, although not as a stand-alone concept. The National Action Plan addresses areas such as combating gender stereotypes, promoting non-violent education, and interventions in schools and sports. The focus remains on awareness-raising, education, research, and victim protection, while evidence-based primary prevention measures are largely absent. The National Action Plan does not provide a formal definition of primary prevention but it is outlined within the scope of the prevention pillar of the Istanbul Convention. No specific budget is allocated to primary prevention activities.

Existing initiatives include feminist self-defence trainings, empowerment activities for girls and women, and prevention programmes targeting men and boys. To date, there has been no evaluation of the impact of primary prevention activities in Switzerland.

TÜRKİYE



GENERAL COUNTRY INFORMATION

Population	85,372,377
Female population	42,638,306
Member of Council of Europe (year)	1949
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1985
CEDAW optional protocol ratified (year)	2002
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2012
<i>With effect of 1 July 2021, Türkiye withdrew from the Istanbul Convention and is no longer a state party.</i>	

SUMMARY

There are three national women's helplines in Türkiye, available 24/7, free of charge, and offering multilingual support. There are also five other helplines supporting survivors of violence against women. There are 150 women-only shelters in the country with a total of 3,677 beds, and 157 women's centres. Starting in 2024, there are 47 Women's Medical Support Units for survivors of sexualised violence. The National Action Plan (NAP) on violence against women and girls contains provisions on primary prevention, but state funding, as well as data collection, mostly focus on awareness-raising activities. The state collects data on women's specialist services, and only limited data is available upon request. Since 2014, no comprehensive official data has been shared with the public regarding the scope of domestic violence.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	3
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	246,555 calls

In Türkiye, there are three main helplines providing support in cases of violence. The first one is the national helpline, ALO 183 Hotline for Combating Violence (*ALO 183 Şiddetle Mücadele Hattı*, tel.: +90 183; +90 5493810183 for video calls; +90 5011830183 for WhatsApp chat), which is operated and financed by the Ministry of Family and Social Services. The hotline is free of charge and available 24/7, meeting the standards of the Istanbul Convention. It offers support in Turkish, Arabic, and Kurdish, and is open to various groups; however, it is not specialised in supporting women survivors of violence. In 2024, it received 235,406 calls, of which 39,781 were from women.

In addition, there is the Emergency Hotline for Domestic Violence (*Aile İçi Şiddet Acil Yardım Hattı*, tel.: +90 2126569696; +90 5496569696), which is run by the Federation of Women Associations of Türkiye. This helpline provides support in Turkish, English and Kurdish to survivors of all forms of violence against women, especially domestic violence. It is funded through international funds and donations. In 2024, it received 3,908 calls.

And lastly, there is the Woman Support Line (*Kadın Destek hattı*, tel.: +90 4448086), which is run and funded by the Istanbul Metropolitan Municipality, and provides support in Turkish, Kurdish, English and Arabic for all forms of violence against women. Between November 2020 and November 2024, it received 7,241 calls.

Additionally, there are five other helplines in Türkiye, which provide support to survivors of violence against women. The *Mor Çatı* Women's Shelter Foundation (tel.: +90 2122925231) provides social, legal, psychological and shelter support, and received 3,433 calls from 1,154 women in 2024. The KAMER Foundation runs a helpline (tel.: +90 5306644410) as well as social, legal, and psychological support. It also currently provides disaster relief support. The third helpline is provided by The Foundation for Women's Solidarity (tel.: +90 3124304005 and +90 3124320782). This helpline received 916 calls in 2024 and offers social, legal, and psychological support. The Women's Solidarity Foundation's helpline (tel.: +90 5464786917) provides social, legal, and psychological support. Finally, the *Mor Salkım* Women's Solidarity Association's helpline (tel.: +90 5310338844) provides social, legal, and psychological support.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	150
Total number of all shelters accessible to women	150
Existing number of beds	3,677
24/7 access	All

There are 150 women-only shelters in Türkiye, with 3,677 beds. The current capacity does not meet the overall needs of women, due to the inadequate conditions of shelters and the lack of space, often resulting in declined referrals. In Türkiye, all women who apply for a shelter must stay at “First Step Stations”, which tend to be overcrowded and provide basic accommodation and meals, but often with no additional social support. Women stay in these stations for a period of 15 days or more. Afterwards, the staff decides whether a woman needs shelter or not. Security measures are often inadequate, as some law enforcement officers provide information about the survivor’s location to the perpetrators. Shelters are available 24/7, and women are not expected to pay for accommodation. In state-run shelters, women can stay up to six months with a possible extension to one year. In *Mor Çatı*’s shelter, the duration of stay is determined according to the women’s needs and conditions. The types of in-house services provided by shelters are casework, counselling/psychological support, referrals/collaboration with other services, and practical and legal advice.

Most women-only shelters in Türkiye can accommodate refugee women and homeless women, while some shelters can also be accessed by and are accessible to women with physical, cognitive and sensory disabilities, women with substance abuse issues, women with older sons and women with children with disabilities. No shelters can support unaccompanied girls under the age of 18. There are no state-run, women-only shelters in Türkiye that accept women with uncertain residence permits or undocumented women, women aged 60 and above, or transgender women who have not undergone gender-affirming surgery and therefore do not hold the corresponding identity card. The only independent shelter in Türkiye, operated by *Mor Çatı*, does not impose such restrictive admission criteria. There are two shelters run by the Directorate of Migration specifically for migrant women, although they are mostly for those with official residence permits. Aside from women-only shelters, there are no other shelter facilities in the country that provide support specifically for violence against women.

Out of the 149 state-run shelters, 112 are run by the Ministry of Family and Social Services, 35 are run by municipalities, and two are run by the Directorate of Migration. These shelters are run with public funds. Only one shelter is independently run by a women’s NGO, the *Mor Çatı* Women’s Shelter Foundation, and is funded by donations and international project funds. This shelter operates with a feminist approach. In some municipalities, there are also some promising examples of feminist methods and

practices, largely driven by the personal initiatives of the women working within such shelters. Women-only shelters are located in all regions. The most common forms of violence reported by survivors to shelters were domestic, physical and sexualised violence. In 2024, a total of 37,454 women and 22,136 children received services from women’s shelters affiliated with the Ministry. Additionally, in the same year, 1,872 foreign women and 2,222 accompanying children, amounting to 4,094 individuals, were provided with services in these shelters.

3. WOMEN’S CENTRES

Total number of centres	157
Entities running women’s centres	Women’s NGOs (60 centres) State (97 centres)

According to the data available, as of 2023, there were 157 women’s shelters in Türkiye, 73 only for women, and 84 for all survivors of gender-based violence. Centres run by women’s NGOs are 60, Violence Prevention and Monitoring Centres (ŞÖNİM) affiliated with the Ministry of Family and Social Services manage 84 centres in 81 provinces, and at least 13 municipalities run women’s centres. Women’s centres provide different types of in-house services, although this might differ according to their resources. Services include counselling and psychological support, legal advice, financial and social welfare support, housing advice, employment, referrals and collaboration with other services, representation at court/police/social services, and residential support services. Women’s centres also provide specialist support for survivors of sexualised violence, namely needs assessment, specialist psychological care and legal advice.

In terms of accessibility, social support centres of The Ministry of Family and Social Services – not specifically dedicated to violence cases, but hosting several violence focal points – are accessible to older women, unaccompanied minor girls, most are equipped to support refugee women, women with physical, cognitive and sensory disabilities, transgender women, survivors (at risk) of forced marriage and honour-based violence. Nevertheless, although these groups of survivors can access these centres, the support provided is generally not grounded in specific training or specialised expertise, and no data is available on their number. There are various institutions run by municipalities or NGOs supporting specific groups, but only a few of them are specifically designed to support women who have experienced violence.

Centres run by the ministry and municipalities have public funds, while NGOs have different international funding sources and donations. Women’s centres are located in most regions in Türkiye.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Türkiye, there are 47 Women's Medical Support Units currently active in 39 provinces, mostly provided by hospitals, health institutions and law enforcement. The 2024 Action Plan of the Ministry of Health planned a pilot project to establish Women's Medical Support Units in selected provinces. These units are meant to operate 24/7 within hospitals that have obstetrics and gynaecology departments or emergency medicine clinics and are intended to include specialists in obstetrics and gynaecology to support women survivors of sexualised violence. According to the Ministry, these services are expected to provide medical interventions, consultations from relevant specialists and the collection of forensic evidence. Data collection reportedly began in April 2024 in facilities where the establishment process was completed, and 487 cases were reported in 2024. However, while these figures and plans reflect the Ministry's official framework, from the perspective of women's NGOs, a systematic and consistent implementation of this model has not yet been observed in practice.

Additionally, several feminist and LGBTQI+ organisations provide specialised support for survivors of sexualised violence, including migrants, and are funded through international funds and donations.

5. PRIMARY PREVENTION SERVICES

In Türkiye, public authorities only disclose the number of people reached through awareness-raising activities, such as educational sessions, conferences, and panels for different groups. No other public data is available. In 2024, a total of 297,107 people took part in these activities, including public officials, citizens, village heads, university students, conscripts and Turkish Armed Forces personnel, and gendarmerie and law enforcement officers. According to state statistics, since 2007, more than 4.5 million people have been reached. Under the National Action Plan, different awareness-raising activities are carried out, but there is no monitoring or evaluation of these activities, so their effectiveness is unknown. Further information is in the Parliamentary Commission Report, produced by a committee reviewing measures to prevent violence and discrimination against women. This report takes a family-centred approach, avoids the term "gender equality", and does not address gender inequality as the root cause of violence. It focuses on issues such as addiction, reflecting a depoliticised view of the problem. Data from public institutions covers primary prevention activities such as training for judicial personnel by the Ministry of Justice, anger management for perpetrators under probation and convicted offenders, brochures for women on legal rights, and non-violent communication training for children from the preschool level.

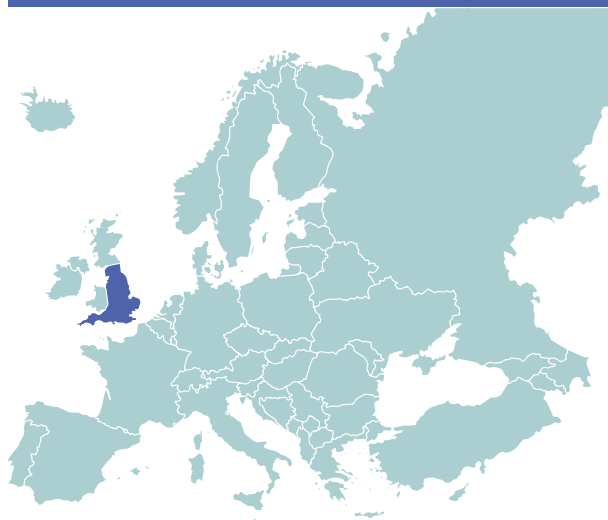
Türkiye's 2021–2025 National Action Plan on violence against women and girls contains provisions on primary

prevention, mainly awareness-raising and training for public service providers, as well as perpetrator programmes like anger management and rehabilitation. The National Action Plan also avoids the term "gender equality". The Parliamentary Commission Report also includes a series of recommendations on primary prevention, such as educating women on legal rights, running national and local campaigns, and addressing economic, digital, and campus-based violence. It also calls for sanctions against those promoting hatred and discrimination against women online, stricter regulation of digital games, and more perpetrator rehabilitation.

There is no official definition of primary prevention in Türkiye or in the National Action Plan. State funding mainly supports awareness-raising, practitioner training, anger management, and school-based prevention programmes. None of these activities is effectively monitored or evaluated. NGOs, including women's, feminist, LGBTQI+, and children's organisations, also carry out prevention work, usually without using the term "primary prevention". They inform women about their rights, raise awareness about male violence and the link between gender equality and violence, and run workshops for children and youth. Beneficiaries include the general public, minors, women, men, and LGBTQI+ people. Some groups, particularly LGBTQI+ individuals, are only addressed by NGO-led activities due to the state's "anti-gender" stance in public activities.

NGOs and women's groups cannot operate in public schools because their work is based on gender equality. As of May 2025, the Directorate General of Family and Community Services has banned the use of concepts such as "gender", "comprehensive sexuality education", and "gender equality" in relevant departments. This policy restricts civil society and women's groups by framing their initiatives as incompatible with current educational guidelines.

UNITED KINGDOM – ENGLAND



GENERAL COUNTRY INFORMATION

Population	57,690,323 ¹⁰⁹
Female population	29,407,249 ¹¹⁰
Member of Council of Europe (year)	1949
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1986
CEDAW optional protocol ratified (year)	1984
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2022

SUMMARY

There is a national helpline in England which is free of charge, available 24/7, and offers multilingual support. Additionally, there are 181 local domestic abuse services that run helpline services. There are 280 shelters available to women survivors of violence, 221 of which are women-only, with a total of 4,551 bed spaces. England has 308 women's centres for all survivors of violence, 116 of which are only for women. There are services for survivors of sexualised violence including 35 rape crisis centres, 48 sexual assault referral centres, several specialised helplines, and other services offered by women's centres and women-led services. There is no official document in England detailing data collection on, or strategic plans for, primary prevention specifically. There is also no specific budget allocated to primary prevention activities. The state collects data on women's specialist support services, as outlined in the Domestic Abuse Act (2021), and this information is publicly available. However, data collection is not comprehensive and consistent.

109 Office for National Statistics (ONS), Estimates of the population for the UK, England, Wales, Scotland, and Northern Ireland, mid-2023 edition, <https://www.ons.gov.uk/peoplepopulationandcommunity/populationandmigration/populationestimates/datasets/populationestimatesforukenglandandwales/scotland-and-northernireland>

110 Ibid.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	135,986 callers

There is one national women's helpline in England called the National Domestic Abuse Helpline (tel.: +44 8082000247). This helpline is specifically for survivors of domestic violence and is run by the organisation Refuge. The helpline is free of charge, available 24/7, and offers multilingual support in all languages, including British Sign Language interpretation (available weekdays 10:00 to 18:00). Funding for the helpline comes from the state as well as donations. The most common forms of violence reported by callers in 2024 were domestic violence, psychological or emotional violence, controlling behaviour, as well as threatening/intimidating behaviour. In 2024, the helpline received a total of 135,986 callers. England therefore does meet the Istanbul Convention minimum standards for the provision of a national women's helpline.

There are 181 local domestic abuse services that run helpline services for survivors of violence in England. These will usually be for local residents and callers will receive support over the phone, sometimes before accessing other support services. There are also national helplines supporting survivors of specific forms of violence including the National Stalking helpline (tel.: +44 808 8020300, available Monday to Friday from 9:30 to 16:00 and Wednesday until 20:00), the modern slavery and exploitation helpline (tel.: +44 8000121700, available 24/7), the 24-hour FGM helpline (tel.: +44 8000283550) for survivors of female genital mutilation, the Karma Nirvana helpline for those affected by honour-based violence (tel.: +44 8005999247, available Monday to Friday from 9:00 to 17:00), and the Galop's free helpline (tel.: +44 8009995428, available Monday to Thursday from 10:00 to 20:30 and Friday from 10:00 to 16:30) for LGBTQI+ people experiencing abuse or violence.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	221
Total number of all shelters accessible to women	280
Existing number of beds	4,551
24/7 access	Some

In England, there are a total of 280 shelters available to women survivors of violence, of which 221 are women-only. They include 4,551 bed spaces available for women, 4,177 of which are for women-only. Reports by Women's

Aid consistently show that demand outweighs the availability of bed spaces across the country. The most common reason why referrals into shelters are declined is that the service does not have the capacity, space or resources to accept the survivor into the shelter. Since 2022, some shelters which were previously women-only became shelters accessible to all survivors. Some women-only shelters offer 24/7 access. The types of in-house services most often provided by women-only shelters are casework, referrals/collaboration with other services, and practical advice. Most survivors are able to access benefits from the welfare security system to pay for their stay in a shelter. However, some women are unable to access these benefits, such as women with certain types of immigration status and who have no recourse to public funds, or those who own their own homes and/or have savings. In some of these cases funding can be accessed by other sources, such as children's services, to enable women without access to benefits to stay in a shelter. However, this isn't always possible, and the cost of shelters continues to be a barrier for some women. There is no limit to how long women can stay in shelters. The overall average length of stay is around seven months, but this has increased significantly over the past five years due to the lack of available accommodation that women can move into when they are ready to leave the shelter. From Women's Aid research, frontline workers have reported some of the women they support stay in services for up to two years because of the lack of available permanent housing, and this is significantly disrupting women's ability to recover and move-on from violence and further reduces the capacity of shelters to meet the demand for their service.

All women-only shelters can accommodate homeless women, and most are equipped to support older women (65 years and above). Most women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical or cognitive disabilities, women with substance abuse issues, transgender women, and women with older sons (14 years and above). In practice, space for some women can be very limited. For example, less than 0.7% of shelter spaces made available from 2023 to 2024 were suitable for a woman requiring wheelchair access.

There are 51 shelters for specific groups of women or survivors (at risk of) specific forms of violence. Availability of these services is very low and spaces in these dedicated services make up just 18.2% of all beds in shelters in England. This is however an increase from 2022 where 11.8% of beds were in dedicated services for specific groups of women. Not all these services are 'by and for' expert organisations, meaning they are not led by women from the group they support. There are 34 shelters run specifically for Black and minoritised women and three shelters for refugee women, trafficked women or women with insecure immigration status. There are three shelters for women with substance use support needs, three shelters for

young women and two shelters for older women. There are also two shelters for women with learning disabilities and three shelters for LGBTQ+ individuals.

Funding for women-only shelters comes from the state as well as donations. Women's NGOs with a gender-specific and feminist approach run 160 women-only shelters; 52 shelters are run by housing associations; eight by other NGOs; and one by the state. Women-only shelters are located in all regions of England. The most common forms of violence reported by survivors to shelters in 2024 were domestic violence, physical violence and psychological violence.

There are 59 other shelters which can provide support to all survivors of violence with 374 bed spaces for women, men, non-binary people and transgender women and men. Some shelters (including women-only shelters) can accommodate transgender individuals. This can be in shared accommodation or dispersed accommodation. Some of these other shelters offer 24/7 access.

❖ 3. WOMEN'S CENTRES

Total number of centres	308
Entities running women's centres	Women's NGOs (218 centres) Other NGOs (79 centres) State (11 centres)

There are 308 women's centres for survivors of violence in England, 116 of which are only for women. All women's centres provide legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, and representation at court/police/social services. Counselling and psychological support is provided by 145 women's centres. In total, 218 women's centres are run by women's NGOs, 79 by other NGOs, and 11 are run by the state. Most women's centres will offer some level of support around sexualised violence, and some run specific initiatives, such as a sexual violence advocacy project. Such centres provide specialist forensic and medical care, needs assessment, specialist psychological care, and specialist sexualised violence advocacy services along with community awareness, and education for prevention. Women's centres are more commonly referred to as "community-based services" and usually also offer services such as outreach, domestic abuse advocacy, floating support services, and prevention/education work.

Some women's centres can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive, or sensory disabilities, older women (65 years and above), transgender women, and unaccompanied girls under the age of 18. There are 27 women's centres providing specialised support to Black and minority ethnic women. Even though they are not exclusively supporting this group of survivors, some centres also offer specialist

support for Black and minority ethnic women, by, for example, having a dedicated specialist worker. Additionally, three centres provide specialised support to LGBTQ+ survivors, two centres to women with uncertain residence permits/undocumented women and refugee women, one centre to women with physical disabilities and one centre provides specialised support to women with cognitive disabilities.

Women's centres are funded by the state as well as through donations. They are located in all regions of England. However, geographical coverage is uneven. In 2024, a total of 138,955 women were supported by women's centres in England.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for survivors of sexualised violence in England, including 35 rape crisis centres and 48 sexual assault referral centres (SARCs). Some women's centres and women-led services also provide support to survivors of sexualised violence, but they may not be members of the network Rape Crisis England & Wales. Forensic evidence is collected by the SARCs which are funded by the Department of Health and have links with the police and women's services for onward support referrals. Support provided by sexualised violence services includes forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, as well as legal advice. These are provided in varying degrees, in some parts of the country. Due to funding arrangements, access to services varies widely across areas, creating a 'postcode lottery' for survivors, meaning that survivors only have access to sexualised violence services in their local area. Specialist services for survivors of sexualised violence are funded by the state and through donations and are located in most regions of England. Some rape crisis centres cover rural areas, but it is very dependent on local population and funding arrangements. Services may be commissioned by state or local authorities and receive funding and grants from charitable organisations and trusts. Many sexualised violence services are mixed sex, that is, they have provisions for men and boys within a wider service which is focused on women and girls. Additionally, many centres have services for young people, but support for children remains limited due to funding and commissioning arrangements.

The 24/7 Rape and Sexual Abuse Support Line (tel.: +44 8085002222) is free of charge and offers support for all those above 16 years who have been affected by rape, child sexual abuse, sexual assault, sexual harassment or any other form of sexualised violence. The helpline was opened in December 2022 and is run by Rape Crisis England & Wales. Other specialised helplines include The Survivors Trust helpline (tel.: +44 8088010818) which is free and confidential but has limited opening hours; the

National Association for People Abused in Childhood helpline (tel.: +44 8088010331) which also has limited opening hours, for all survivors of all child abuse, including sexual abuse; and the Revenge Porn helpline (tel.: +44 3456000459) available Monday to Friday from 10:00 to 16:00, for adults subjected to image-based abuse.

5. PRIMARY PREVENTION SERVICES

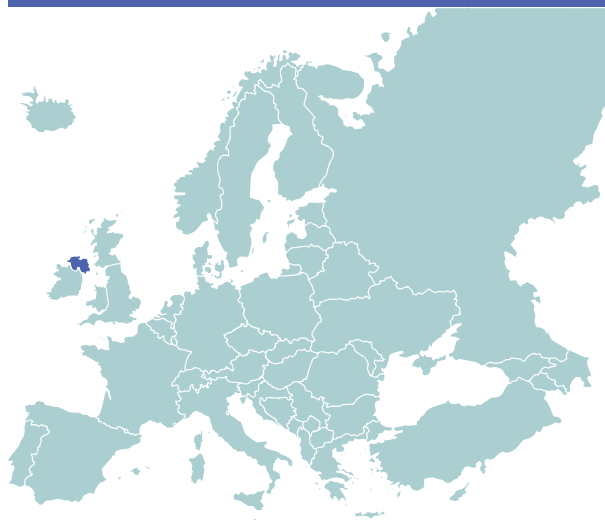
There is no official document in England detailing data collection on, or strategic plans for, primary prevention specifically. Since coming into office in July 2024, the Labour Government has been developing a violence against women and girls (VAWG) Strategy which is expected to be published before August 2025. A VAWG Advisory Board has been established to advise on the development of the Strategy and the content/provisions which should support the government's plan. While activities supporting primary prevention objectives are set out in the pre-existing Tackling VAWG Strategy, authored by the previous government, there is currently no definition of primary prevention in any official government document. Prevention more broadly, in the context of violence against women, is predominantly described as delivering long-term cultural and societal change. Available primary prevention activities include school-based primary prevention programmes, training for primary prevention practitioners, and awareness-raising campaigns or programmes.

There is no specific budget allocated to primary prevention activities, however a portion of the available funding for tackling violence against women will be committed to 'prevention' in its entirety, some of which will be specifically focused on primary prevention. There has also been limited progress in developing measures to prevent violence against women or establishing an understanding of what works to prevent violence, and its root causes. The existing VAWG Strategy states that while some initiatives, such as awareness-raising activities, showed promise, there was limited evidence on their effectiveness at reducing violence or changing perpetrator behaviour. Since 2021–2022, the Home Office has spent at least £4.2 million (€4.8 million) on new research into what works to prevent violence, but the projects funded have yet to be completed or to provide significant insights to inform any future interventions. Additionally, between 2021–22 and 2023–24, the Home Office underspent its own budget allocated to the VAWG strategy by an average of around 15%. This underspend came to around £22 million (€25.3 million), against a total £149 million (€171.6 million) budget for that period. It has also been noted that currently, the majority of the Home Office's VAWG-specific prevention activity focuses on addressing re-offending rather than preventing VAWG in the first instance.

The vast majority of prevention work in England, and the United Kingdom more generally, is focused on early intervention in schools and awareness campaign. A quarter of commitments relating to prevention were allocated to the

Department for Education (DfE), however DfE spent £0.52 million (€0.60 million) across all activities relating to the VAWG Strategy and Domestic Abuse Plan between 2021–22 and 2023–24, with much of its work on supporting survivors rather than prevention. Efforts to address primary prevention also occur at a local and regional level, with statutory agencies like Police and Crime Commissioners using funding at their discretion (allocated by national government) to implement prevention activities as they see fit. This can lead to variation in regional approaches to tackling violence against women. Examples of activities include funding allocated by the Mayor of West Yorkshire to programmes for young people on cybercrime awareness and education to target and stop exploitation and abuse facilitated online. The Mayor of London also implemented a region-wide campaign targeted at men and boys aimed at dismantling misogynist behaviours and attitudes.

UNITED KINGDOM – NORTHERN IRELAND



GENERAL COUNTRY INFORMATION

Population	1,920,382 ¹¹¹
Female population	974,897 ¹¹²
Member of Council of Europe (year)	1949
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1986
CEDAW optional protocol ratified (year)	1986
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2022

SUMMARY

There is one national domestic and sexual abuse helpline in Northern Ireland open to all genders that is free of charge, available 24/7, and provides multilingual support. There are 14 women-only shelters in Northern Ireland with 340 beds as well as 11 women's centres that provide support to all survivors of violence and one sexual violence referral centre. Measures related to primary prevention are included in official documents such as National Action Plans, and there is a specific budget allocated to such activities. The state does collect data on women's specialist services, including primary prevention activities, and this data is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	N/A

¹¹¹ Northern Ireland Statistics and Research Agency, population estimates for mid-2023, <https://www.nisra.gov.uk/publications/2023-mid-year-population-estimates-northern-ireland-and-estimates-population-aged-85>.

¹¹² Ibid.

There is one national helpline in Northern Ireland called the Domestic and Sexual Abuse Helpline (tel.: +44 8088021414). Northern Ireland therefore does meet the Istanbul Convention standards for the provision of a standalone national women's helpline. It offers support to survivors of all genders affected by domestic abuse such as psychological abuse, sexual abuse, physical abuse, coercive control, financial abuse and technological abuse. The helpline is run by Nexus Northern Ireland and is funded by the state. Previously, the helpline was established and run by the Women's Aid Federation Northern Ireland until 2019. The helpline is free of charge, available 24/7, and multilingual support is available on request for any language.

There is no other state or regional women's helpline supporting survivors of violence against women in Northern Ireland, although women survivors routinely contact Women's Aid services directly.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	14
Total number of all shelters accessible to women	14
Existing number of beds	340
24/7 access	Most

There are 14 women-only shelters in Northern Ireland with 340 beds in total. Northern Ireland is a small region with only two major cities, Belfast and Derry, both of which have 24/7 shelters and most other women-only shelters offer 24/7 access. The types of in-house services most often provided by women-only shelters are casework, referrals/collaboration with other services, practical advice, and legal advice. Women do not pay for their accommodation in a women-only shelter, they are asked to contribute a small donation if they can or wish to do so, but this is not expected of them. Women who have recourse to public funds have their shelter place funded through housing benefits covered by the state. For women with no recourse to public funds, women-only shelters try to cover the costs through specific pots of funding or swallow the cost of their stay in the shelter.

The average stay in women-only shelters varies from three months to one year, although very few women stay up to one year. The length of stay is determined on a case-by-case basis and depends on the area of the country the women-only shelter is in as well as access to safe housing after leaving the shelter. Northern Ireland has a housing shortage meaning that women sometimes have to stay longer in a shelter than expected.

All women-only shelters in Northern Ireland can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with physical, cognitive or sensory disabilities, older women (65 years and above), as well as women with children with disabilities. Some shelters can be accessed by women with substance abuse issues, transgender

women, and women with older sons (14 years and above). No women-only shelters can accommodate homeless women and unaccompanied girls under the age of 18. There are no specialised shelters supporting particular groups of survivors nor women survivors or those at risk of specific forms of violence in Northern Ireland.

All women-only shelters are run by women's NGOs with a gender-specific and feminist approach and are funded by the state. Women-only shelters are located in all regions of Northern Ireland. The main reasons for having to decline referrals to women-only shelters were no space/capacity to support the survivor and the survivor being ineligible for support. The three most common forms of violence reported by clients to the women-only shelters in 2024 were domestic violence, physical violence, and sexualised violence. A total of 528 women and 293 children were accommodated in women-only shelters in 2024.

There are no other shelters in Northern Ireland, besides the women-only shelters, which can provide support to survivors of violence against women.

3. WOMEN'S CENTRES

Total number of centres	11
Entities running women's centres	Women's NGOs (11 centres)

Northern Ireland has 11 women's centres for all survivors which are run by Women's Aid Services. Women's centres offer counselling/psychological support, financial and social welfare support and advocacy, housing advice, employment, referrals and collaboration with other services, representation at court/police/social services, and residential support. Women's centres only provide non-residential support services; however, all centres are linked to shelters run by the same organisation. Therefore, if women need a place in a shelter, the necessary arrangements can be made. Additionally, all centres also offer specialised support for survivors of sexualised violence in the form of community awareness and education for prevention.

All women's centres in Northern Ireland can be accessed by and are accessible to women with disabilities and women with children with disabilities. Some centres are equipped to support transgender women, but there are no centres available to unaccompanied girls under the age of 18. Additionally, all women's centres provide specialised support to women with uncertain residence permits/undocumented women, refugee women, Black and minority ethnic women, women with physical or sensory disabilities as well as older women (65 years and above). Furthermore, there are two women's centres specialised in supporting survivors of trafficking.

All women's centres are run by women's NGOs and are funded by the state as well as through donations. Women's centres are located in all regions of Northern Ireland. A

total of 7,318 women were supported by women’s centres in Northern Ireland in 2024.

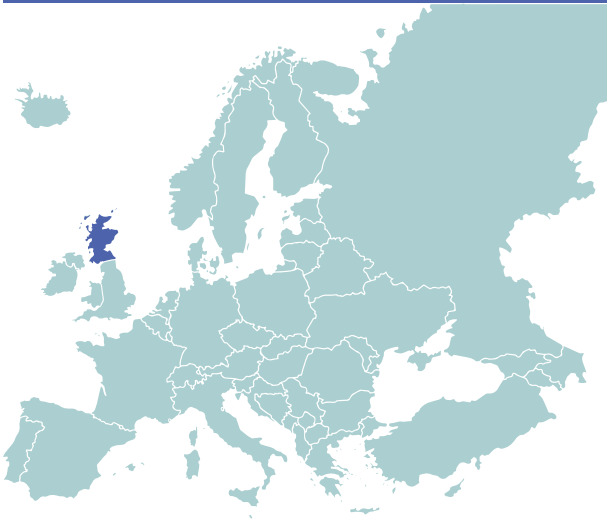
4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

Northern Ireland has one sexual violence referral centre, the Rowan Sexual Assault Referral Centre, which is located in Antrim. The Rowan Sexual Assault Referral Centre supports all survivors of sexualised violence, regardless of age. The centre provides specialist forensic and medical care and operates a specialised helpline (tel.: +44 8003894424). It is funded by the state and services the whole of Northern Ireland.

5. PRIMARY PREVENTION SERVICES

Public authorities in Northern Ireland do collect data on primary prevention activities. Official documents such as the Strategic Framework to End Violence Against Women and Girls 2024–2031¹¹³ and Domestic and Sexual Abuse Strategy 2023–2030 do include provisions on primary prevention.¹¹⁴ The Strategic Framework to End Violence Against Women and Girls also includes a definition of primary prevention focusing on changing attitudes, behaviours, and culture, building knowledge and skills of individuals to form healthy relationships, and ensuring that women and girls are and feel safe everywhere. There is a specific budget allocated to primary prevention activities in Northern Ireland; this amounted to approximately £3.2 million (€3.66 million) from September 2024 to March 2026. Activities include empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, training for primary prevention practitioners, bystander intervention training, and awareness-raising campaigns or programmes. The Strategic Framework to End Violence Against Women and Girls will be evaluated in late 2025.

UNITED KINGDOM – SCOTLAND



GENERAL COUNTRY INFORMATION

Population	5,546,900 ¹¹⁵
Female population	2,851,740 ¹¹⁶
Member of Council of Europe (year)	1949
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1986
CEDAW optional protocol ratified (year)	1986
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2022

SUMMARY

Scotland has one national women’s helpline which is free of charge, operates 24/7 and provides multilingual support. There are 34 women-only shelters with a total of 425 beds, as well as other shelters accessible to women survivors of violence. There are 37 women’s centres, most of which are part of the Scottish Women’s Aid and Rape Crisis Scotland networks. Scotland has services for survivors of sexualised violence, including 15 rape crisis centres, one sexual violence referral centre and a specialised helpline. Measures related to primary prevention are included in the Equally Safe national strategy, and there is a specific budget allocated to such activities. The state does collect data on women’s specialist services, including primary prevention activities, and this data is available upon request. Local authorities require data collection from all Women’s Aid groups, though the data submitted does not include any sensitive or personally identifying information about clients. However, one limitation is that the data requested typically only reflects support provided under the local authority’s funded services and therefore does not capture the full scope of support delivered through other funding streams.

113 Strategic Framework to End Violence Against Women and Girls (EVAWG) 2024–2031, <https://www.executiveoffice-ni.gov.uk/topics/ending-violence-against-women-and-girls>

114 Domestic and Sexual Abuse Strategy 2023–2030, <https://www.health-ni.gov.uk/consultations/domestic-and-sexual-abuse-strategy-2023-2030>

115 National Records of Scotland, mid-year population estimates 2024, <https://www.nrscotland.gov.uk/publications/population-estimates-time-series-data/>

116 Ibid.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	13,923 calls

There is one national women's helpline in Scotland called Scotland's Domestic Abuse and Forced Marriage Helpline (tel.: +44 8000271234, text: +44 7401288595) which is run by Scottish Women's Aid. The helpline is free of charge, available 24/7 and offers support to survivors of domestic abuse and forced marriage. The helpline also offers multilingual support via Language Line (a telephone interpreting service) and Google Translate (via web chat, text service, and email). Scotland therefore does meet the Istanbul Convention minimum standards for the provision of a national women's helpline. The helpline is funded by the government as well as through donations and received 13,923 calls in 2024. Fifty percent of callers contacted the state-wide women's helpline for the first time. The three most common forms of violence reported by callers were psychological, economic violence and violence directly/indirectly affecting children.

There are other helplines supporting survivors of violence in Scotland, including Shakti Women's Aid (tel.: +44 1314752399) and Hemat Gryffe Women's Aid (tel.: +44 1413530859), which provides specialised support to Black and Minority Ethnic women, as well as the National LGBT Helpline for LGBTQI+ survivors (tel.: +44 8009995428), Shelter Scotland for housing advice if fleeing domestic violence (tel.: +44 8088004444), Childline for children and young people experiencing or witnessing domestic violence (tel.: +44 8001111), and NSPCC Helpline for adults concerned about children experiencing domestic violence (tel.: +44 8088005000). The Scottish Women's Rights Centre (tel.: +44 8088010789), run by Rape Crisis Scotland and Just Right Scotland, also provides legal support and advocacy services around all forms of violence against women including domestic violence, stalking, forced marriage, honour-based violence, and sexualised violence. Additionally, local Women's Aid groups all provide local support and often offer local helplines.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	34
Total number of all shelters accessible to women	34
Existing number of beds	425
24/7 access	Some

There are 34 women-only shelters in Scotland offering approximately 425 bed spaces, which are run by Women's Aid groups, and all apply a gender-specific and feminist approach. Since 2022, two women-only shelters had to close

but they have been absorbed by other services. When women and families are in a shelter, they have 24/7 access to their accommodation. Otherwise, only three women-only shelters are able to offer shelter access outside their normal service hours. The number of available beds does not fully meet the needs of women, children and young people fleeing domestic violence. There is also an increasing demand as domestic violence reports have risen, especially since COVID-19, and cost of living pressures makes it harder for survivors to secure housing outside of shelters. Women are expected to pay for their accommodation in shelters, however most of them receive housing benefit payments that are used to cover the accommodation costs. Those who are not recipients of state benefits need to pay. On average, women are allowed to stay in a shelter more than one year which is assessed based on shelter availability, while some shelters either have longer-term residents or a consistent waiting list. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services, practical advice, and legal advice. Casework is the most predominant service offered. Other services include crisis support and emergency response, children and young people's support services, group work for women, parenting support, confidence and self-esteem building workshops, domestic abuse educational programmes, housing and homelessness prevention, financial inclusion and welfare advice as well as preventative work and public awareness raising. The focus of all in-house services is to provide long-term stability, safety, and empowerment.

All women-only shelters can accommodate homeless women, older women, women with older sons (14 years and above, although fewer options past the age of 18) and unaccompanied girls under the age of 18. Most women-only shelters are equipped to support women with uncertain residence permit/ undocumented women, such as support on seeking leave to remain and receiving funding from social work. Some shelters can be accessed by and are accessible to women with physical disabilities (18 shelters with full wheelchair access and 25 ground floor accommodation/support services), sensory disabilities (eight shelters with support such as flashing lights alarm system, induction loop, and/or British Sign Language-using staff) or cognitive disabilities (five shelters), women with substance abuse issues, transgender women (five shelters), and women with children with disabilities. Additionally, two women-only shelters are specifically for women of black and minority ethnic background (Shakti Women's Aid and Hemat Gryffe Women's Aid).

Women-only shelters are funded by the state as well as housing benefit payments received by the survivors. All 34 women-only shelters are run by women's NGOs with a gender-specific and feminist approach. There are women-only shelters in most regions of Scotland and are varied across cities, smaller urban areas and more remote locations. There are however geographical gaps, where some

rural, island and remote communities have no shelters at all. The main reasons why women-only shelters had to decline referrals were no space/capacity to support the survivor and shelters not being adequately equipped to support the needs of the women/children. Shelters also had to decline referrals from women with pets, due to the conditions of the letting agreement that does not allow pets on the shelter property. In 2024, the most common forms of violence reported by the survivors to the women-only shelters were domestic, psychological and economic violence. Besides the women-only shelters, there are other shelters that provide support to women survivors of violence, although the exact number of shelters and beds is not known. These services can support women survivors of violence, but they do not offer specialised domestic violence support.

❖ 3. WOMEN'S CENTRES

Total number of centres	37
Entities running women's centres	Women's NGOs (37 centres)

There are 37 women's centres located in Scotland, 36 of which are only for women survivors of violence and part of the Scottish Women's Aid and Rape Crisis Scotland networks. The remaining one is for all survivors of violence, including women. Since 2022, three Women's Aid services have either closed or merged with other Women's Aid services locally. Additionally, one other service outside of the Women's Aid network had to close. A range of services are offered by women's centres. All 37 centres provide counselling/psychological support, housing advice and referrals/collaborations with other services. Thirty-four women's centres provide residential support services, 32 provide advocacy services, and 30 provide support with financial and social welfare advocacy. Additionally, 20 women's centres provide access to legal advice services and 18 offer court support. Women's centres also offer coffee mornings, activities for mother and children, access to courses and trainings, access to pet fostering, and planning home security/ access to community alarms. Women's Aid is not allowed to represent survivors in court as they are considered biased in favour of survivors but can provide court advocates who can support women. In total, 34 centres also provide shelter accommodation in addition to non-residential services. Women's centres also provide support for survivors of sexualised violence. The services include forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, community awareness and education for prevention, and legal advice.

All women's centres can be accessed by and are accessible to women with uncertain residence permits/undocumented women, women with physical, cognitive or sensory disabilities, older women (65 years and above) and unaccompanied girls under the age of 18. Some centres

can support refugee women and transgender women. Women's centres also provide specialised support to survivors (at risk) of the following forms of violence: forced marriage (29 centres), honour-based violence (17 centres), and female genital mutilation (12 centres). Most centres that are part of the Scottish Women's Aid network can offer support around forced marriage and honour-based abuse. Some can offer support around female genital mutilation and trafficking, without necessarily having this as their specialism. There are two groups, between them covering approximately 10 areas of the country, who are specifically for black and minority ethnic women and more likely to have specialism around language and immigration needs; however ethnicity and religion are not barriers for women to access support from any centre.

Women's NGOs and are located in all regions of Scotland, including the North, Southwest, Southeast, Central Belt, Highlands and Islands. They are funded by the state as well as through donations and independent charitable funds and grants, for example Robertson Trust, National Lottery, Children in Need and Lloyds TSB. Thirty-two women's centres are run by women's NGOs, and the rest are run by other services such as housing services and local authorities.

🏠 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

In Scotland there are specialised services for survivors of sexualised violence including 15 rape crisis centres, and one sexual violence referral centre. Rape Crisis Centres (RCCs) are specialist, community-based NGOs which operate within a set of quality standards shared with Rape Crisis England & Wales. A range of services are offered by these centres, including needs assessment, specialist psychological care (including emotional and therapeutic support), specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice. Forensic evidence is primarily carried out within the public health service and in recent years, a specialised national referral pathway has been set up to facilitate this. Funding for sexualised violence services comes primarily from a national fund from the Scottish Government as well as private donations. However, the funding model is under threat, since the government is considering moving to localised, commission-based funding. RCCs are based in all regions of Scotland, covering three island archipelagos (Western Isles, Orkney and Shetland), as well as rural and remote Scottish Highlands. Furthermore, centres offer outreach and digital support services. Twelve centres support survivors of all genders, and three centres provide support only to women survivors of sexualised violence. Age ranges vary, however most rape crisis centres support survivors from the age of 13.

The National Advocacy Service is a network of specialist Rape Crisis Advocacy Workers across Scotland who help

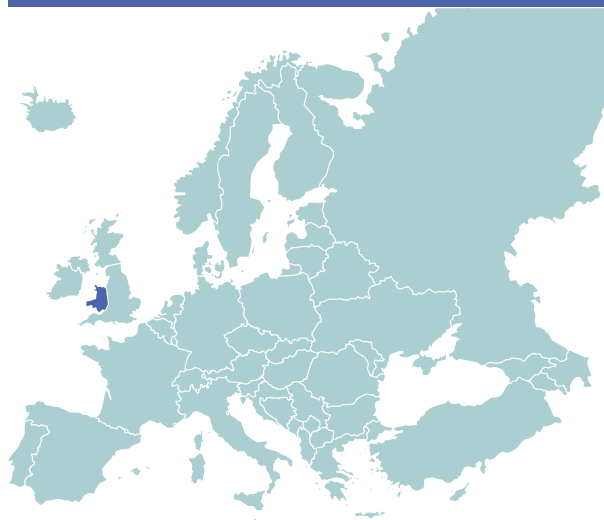
survivors to access justice. It was launched by Rape Crisis Scotland (RCS) in 2016 with financial support from the Scottish Government. This service provides dedicated advocacy support from RCS-trained advocacy workers to survivors of rape and sexualised violence throughout the criminal justice process. There is also a specialised helpline for survivors of sexualised violence, the Rape Crisis Scotland Helpline (+44 8088010302). It provides telephone advice, support and sign posting for all survivors of rape and sexual assault. The helpline is free of charge and offers multilingual support, although it is not available 24/7 (available every day 17:00 to midnight).

5. PRIMARY PREVENTION SERVICES

Public authorities in Scotland actively collect data on primary prevention activities aimed at reducing domestic violence. Scotland has an official national strategy that includes specific provisions on primary prevention: "Equally Safe: Scotland's strategy for preventing and eradicating violence against women and girls". This strategy, published in 2018, is developed by the Scottish Government and COSLA (the councillor-led, cross-party organisation that represents all 32 local authorities in Scotland) and includes a definition of primary prevention.¹¹⁷ One of the main priorities of the strategy is intervening early and effectively to prevent violence and maximising the safety and wellbeing of women, children and young people.

Scotland allocates specific funding for primary prevention activities through the Delivering Equally Safe (DES) Fund. As of the 2024–25 period, the fund has allocated approximately £19 million (€22 million) per year, supporting over 120 projects delivered by more than 100 organisations across Scotland. The Equally Safe strategy allocated approximately £5.2 million (€6 million) over two years to prevention-focused activities. The total DES Fund allocation over 3.5 years was £67 million (€77.7 million), and the proportion dedicated to primary prevention is approximately 7.8% of this amount. The DES Fund prioritises projects that prevent violence before it occurs by addressing the underlying causes of violence against women, support early intervention to stop violence at the earliest possible stage, and provide direct support to survivors, ensuring their safety and well-being. These initiatives cover a variety of activities, including educational programmes, community outreach, and support services, all aimed at creating a society free from violence. Furthermore, the initiative supports school-based primary prevention programmes, primary prevention activities targeted to men and boys, training for primary prevention practitioners and awareness-raising campaigns. Primary prevention initiatives are evaluated, and these evaluations are financed by public authorities, primarily through the Scottish Government.

UNITED KINGDOM – WALES



GENERAL COUNTRY INFORMATION

Population	3,164,400 ¹¹⁸
Female population	1,611,486 ¹¹⁹
Member of Council of Europe (year)	1949
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1986
CEDAW optional protocol ratified (year)	2004
Istanbul Convention signed (year)	2012
Istanbul Convention ratified (year)	2022

SUMMARY

There is one women's helpline in Wales which is free of charge, available 24/7, and offers multilingual support, as well as other helplines supporting survivors of gender-based violence. There are 20 women-only shelters offering an estimated 272 beds, all of which also offer non-resident support services. There are different services supporting survivors of sexualised violence, including one rape crisis centre, sexual violence referral centres, crisis and medical services and a specialised helpline. Primary prevention is addressed in the national strategy to counter violence against women, and the Welsh Government does fund prevention work through specific budgets. Data on women's specialist services is collected by Welsh Women's Aid on behalf of the state, and it is available to the public. National data collection is addressed by the state in the Violence Against Women, Domestic Abuse and Sexual Violence: Blueprint High Level Action Plan.

¹¹⁷ "Equally Safe: Scotland's strategy for preventing and eradicating violence against women and girls", <https://www.gov.scot/policies/violence-against-women-and-girls/equally-safe-strategy/>

¹¹⁸ Office for National Statistics (ONS), Estimates of the population for the UK, England, Wales, Scotland, and Northern Ireland, mid-2023 edition, <https://www.ons.gov.uk/peoplepopulationandcommunity/populationandmigration/populationestimates/datasets/populationestimatesforukenglandandwalescotlandandnorthernireland>

¹¹⁹ Ibid..

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	1
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	18,421 calls

There is one national women's helpline in Wales called Live Fear Free Helpline (*Llinell Gymorth Byw Heb Ofn*, tel.: +44 8088010800) which is run by Welsh Women's Aid. The helpline is free of charge, available 24/7, and offers multilingual support in English and Welsh, while other languages are available via a translation service. Therefore, the helpline meets the standards of the Istanbul Convention. It provides support for all types of violence against women and received a total of 18,421 calls in 2024. The most common forms of violence reported by callers were domestic, physical and psychological violence. Funding for the Live Fear Free Helpline comes from the Welsh Government and from donations.

Additionally, there are other helplines available to survivors of violence in Wales. The BAWSO helpline provides practical and emotional prevention, protection and support services to Black Minority Ethnic and migrant survivors of domestic abuse, sexualised violence, female genital mutilation, forced marriage, honour-based violence, modern slavery and human trafficking (tel.: +44 8007318147). Other helplines covering both England and Wales include the National Domestic Abuse Helpline (tel.: +44 8082000247) run by Refuge, Galop, the national helpline for LGBTQI+ victims and survivors of abuse and violence (tel.: +44 8009995428), the National Stalking Helpline (+44 8088020300), and Revenge Porn Helpline (+44 3456000459).

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	20
Total number of all shelters accessible to women	N/A
Existing number of beds	272 ¹²⁰
24/7 access	Most

There are 20 specialist organisations which are part of Welsh Women's Aid and provide emergency women-only shelter services to survivors of violence against women, with an estimated 272 beds (according to data from 2022). This figure includes shared refuge accommodation and dispersed housing units that are classed as Emergency Supported Accommodation, most of which offer 24/7 access. The number of beds available is not sufficient to cover the demand across Wales. While the majority of the accommodation cost is met by Housing Benefit, survivors also pay a minimal weekly service charge. On average,

women are allowed to stay in the shelters between three and six months, but this period can be extended until they find a new accommodation. There is a significant housing shortage in Wales, which means that while the planned maximum stay is six months, some shelters will allow women to stay for up to twelve months. The types of in-house services most often provided by the women-only shelters in Wales are casework, counselling and psychological support, referrals and collaboration with other services, and practical advice. Legal advice is regulated, so the services may signpost to solicitors but may not have the capacity to give legal advice.

All women-only shelters in Wales are equipped to support homeless women, women with sensory disabilities and older women, while most shelters can accommodate transgender women. Some women-only shelters can be accessed by and are accessible to women with uncertain residence permits, undocumented women, refugee women, women with physical and cognitive disabilities, women with substance abuse issues, women with older sons and with children with disabilities, and to unaccompanied girls under the age of 18. There is also a shelter specialised in supporting Black and Minority Ethnic women who may also have experienced honour-based violence, forced marriage and/or female genital mutilation.

Women's shelters are funded by the state, and the majority are run by women's NGOs with a feminist and gender-specific approach, while a few Housing Associations also have refuge provision. There is shelter provision in each county, including rural areas. If shelters had to decline referrals, the main reasons were the lack of space and capacity to support the survivors, or the shelter being unable to meet the complex needs of the survivors, often relating to mental health or drug dependency. The most common forms of violence reported by survivors to the women-only shelters in 2024 were domestic, physical and psychological violence. During this year, a minimum of 600 women and 560 children and young people were accommodated in women-only shelters in Wales.

There are other shelters in Wales besides women-only shelters which can provide support to survivors of violence, offering various forms of temporary and emergency accommodation, but the total number is not known. Such shelters include mental health crisis houses, probation-approved premises, residential women's centres (criminal justice related), and young people's supported accommodation. None of these services are provided because a client is a survivor of violence against women, but if a client has experienced such violence, they will be supported in a coordinated community response.

120 This figure is from the WAVE Country Report 2023.

❖ 3. WOMEN'S CENTRES

Total number of centres	20
Entities running women's centres	N/A

There are 20 women's centres in Wales supporting women survivors of violence. Women's centres provide financial and social welfare support, housing advice, referrals and collaboration with other services, counselling and psychological support, legal advice, representation at courts and other institutions, and residential support services. Additionally, women's centres also provide specialist support for survivors of sexualised violence, including specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services and community awareness and education for prevention.

All centres are equipped to support women with uncertain residence permits and undocumented women, refugee women, women with sensory disabilities, older women and unaccompanied girls. Most centres can be accessed by and are accessible to women with physical and cognitive disabilities and transgender women. Additionally, all centres are specialised in supporting survivors (at risk) of forced marriage, honour-based violence, female genital mutilation, as well as refugee women and Black and Minority Ethnic women. There are also centres specialised in supporting survivors (at risk) of trafficking in persons, women with uncertain residence permit and undocumented women, women with sensory disabilities, older women, girls under the age of 18, transgender women and non-binary people.

Women's centres are run thanks to donations as well as private and public fundraising and are available in most counties. In 2024, they supported a minimum of 8,000 survivors.

🕒 4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

There are services for women and girls who have experienced sexualised violence in Wales, including one rape crisis centre, at least nine sexual violence referral centres, at least eight crisis and medical services and one specialised helpline. Support provided by such services includes specialist forensic and medical care, needs assessment, specialist psychological care, specialist advocacy services, and community awareness and education for prevention. Services may signpost to solicitors for legal advice. Services are run thanks to jointly commissioned funding from the Health Service and the Police and Crime Commissioners, as well as through private and public fundraising. They are available in most regions and support all survivors of sexualised violence, including women. The 24/7 Rape & Sexual Abuse Support Line run by Rape Crisis England

& Wales (+44 8085002222) provides specialised helpline support to survivors of sexualised violence.

👤 5. PRIMARY PREVENTION SERVICES

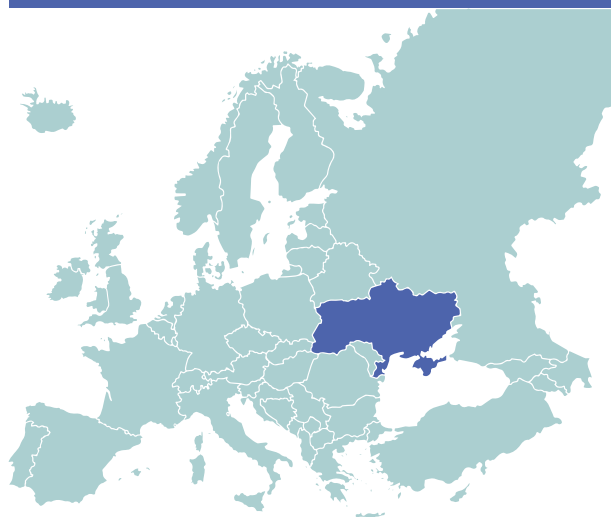
Public authorities do not collect data on primary prevention in Wales. The Violence Against Women, Domestic Abuse and Sexual Violence: Strategy 2022-2026 details how the Welsh Government plans to tackle such forms of violence and lists prevention as a priority. However, there is no official definition of primary prevention nor a dedicated budget for such activities in Wales. The Welsh Government does fund prevention work through specific budgets, but the total amount allocated to prevention is not known. Different primary prevention activities are conducted in the country: feminist self-defence trainings, empowerment activities for girls and women, primary prevention activities specifically targeted to men and boys, school-based primary prevention programmes, training for primary prevention practitioners, bystander intervention training, self-care activities and awareness-raising campaigns or programmes. Public authorities do not conduct evaluations of primary prevention activities, and organisations that provide primary prevention do their own analysis and evaluation.

Welsh Women's Aid conducts training and activities relating to bystander Intervention and its 'Ask Me' programmes.¹²¹ Additionally, Welsh Women's Aid's blueprint for prevention acknowledges that violence against women and girls is preventable.¹²² With a healthcare approach, it looks at the role of primary prevention, secondary prevention and tertiary prevention in stopping violence before it occurs. But more widely, Welsh Women's Aid promotes the *Change That Lasts* approach which looks at what role we can all play in preventing violence against women and girls. Programmes like *Trusted Professionals* support all professionals in ensuring that they can tackle violence properly and appropriately.

¹²¹ Further information available here: <https://welshwomensaid.org.uk/change-that-lasts/ask-me-project/>.

¹²² Available here: <https://welshwomensaid.org.uk/news/a-blueprint-for-the-prevention-of-violence-against-women-domestic-abuse-and-sexual-violence/>.

UKRAINE



GENERAL COUNTRY INFORMATION

Population	40,997,698 ¹²³
Female population	21,990,719 ¹²⁴
Member of Council of Europe (year)	1995
Member of European Union (year)	No
Member of United Nations (year)	1945
CEDAW ratified (year)	1981
CEDAW optional protocol ratified (year)	2003
Istanbul Convention signed (year)	2011
Istanbul Convention ratified (year)	2022

SUMMARY

There are two national women's helplines in Ukraine which are free of charge and available 24/7 but do not offer multilingual support. Other helplines supporting survivors of violence against women are available, including regional helplines. There are 60 women-only shelters for women and children survivors of domestic violence with around 1,035 available beds. There are also women's centres and services for survivors of sexualised violence in Ukraine although there is no available information on the overall number of such services. Measures related to primary prevention are included in the State Social Program for Prevention and Counteraction of Domestic Violence and Gender-Based Violence. There are however no official documents on primary prevention and there is no state definition of primary prevention. The state does collect data on women's specialist services and while some statistics are publicly available on government websites, it is often not concise and accurate. More detailed data on services is available upon request.

WOMEN'S SPECIALIST SERVICES

1. NATIONAL WOMEN'S HELPLINES

Total number	2
Free of charge	Yes
24/7 service	Yes
Multilingual support	Yes
Total number of calls/callers in 2024	44,805 calls ¹²⁵

There are two national women's helplines in Ukraine. One helpline is the National Hotline for the Prevention of Domestic Violence, Human Trafficking and Gender Discrimination (*Національна гаряча лінія з попередження домашнього насильства, торгівлі людьми та гендерної дискримінації*; tel.: +380 800500335 (from landline) or 116 123 (from mobile)). This helpline is run by the women's organisation "La Strada-Ukraine" and has been in operation since 1997. It is free of charge, available 24/7, and offers multilingual support in Ukrainian, Russian and English (upon request). Consultations are also available via online channels such as Telegram and Instagram. The helpline provides support to survivors of domestic violence, sexualised violence, conflict-related sexualised violence, sexual harassment, discrimination, and human trafficking/exploitation. The helpline is funded by international organisations as well as through donations. In 2024, the helpline received 44,805 calls.

The second national helpline is the National Hotline for Combating Violence in Ukraine (*Національна лінія з протидії насильству в Україні, 1547*) which was opened in 2020. This helpline is run by the Ministry for Social Policy and is also free of charge, available 24/7 and offers multilingual support in Russian. This helpline is financed by the state and provides support to survivors of domestic violence, human trafficking, and gender-based violence. The most common forms of violence reported by callers to both national helplines in 2024 were domestic violence, physical violence, and psychological violence.

In Ukraine, there are other helplines supporting survivors of violence against women. For example, local community organisations often operate local helplines. According to the Ministry for Social Policy, as of December 2024 there were 29 call centres and hotlines available for survivors of all forms of violence. Some examples of available helplines include *Jurfem* (tel.: +380 681455590, available via Telegram, Viber, and Signal) which is an initiative of the Ukrainian Women Lawyers Association to support survivors of sexualised violence, rape, and sexual harassment as well as survivors of all forms of gender discrimination. Additionally, *Marsh Zhinok* (Instagram: @Marshzhinok_bot) provides psychological support and support in finding shelter accommodation for survivors. The NGO Divchata

¹²³ The most recent available data on Eurostat is from 2022.

¹²⁴ Ibid.

¹²⁵ This only includes the number of calls to the National Hotline for the prevention of domestic violence, human trafficking, and gender discrimination.

also provides helpline support (tel.: +380 734603860 or email: help@divchata.org) such as psychological and legal aid or referrals.

2. SHELTERS ACCESSIBLE TO WOMEN

Total number of women-only shelters	60
Total number of all shelters accessible to women	60
Existing number of beds	1,035
24/7 access	Some

According to the National Social Services of Ukraine, there are 60 women-only shelters for women and children survivors of domestic violence with around 1,035 available beds (equalling approximately 345 family places). It should be noted that the service responsible for reporting on shelter capacity counts bed spaces, while family places (comprising of three beds for one adult and two children) is used to assess compliance with the recommended standards. Some women-only shelters run by NGOs are available 24/7, however, access to government and municipal shelters is more complicated due to complex bureaucratic procedures. Usually, the police can bring a survivor to a women-only shelter which runs 24/7, but such shelters are not available in every town. Women are not expected to pay for their accommodation in women-only shelters, although it can be the case that only basic food items are covered, and the rest needs to be covered by the survivor. The types of in-house services most often provided by women-only shelters are casework, counselling/psychological support, referrals/collaboration with other services, practical advice, and legal advice. On average, women are allowed to stay in women-only shelters up to three months (90 days).

According to data provided by the Department for Compliance with Equality Standards of the National Social Service of Ukraine, as of July 1, 2024, less than half of the available beds in shelters (396 out of 1,035 beds) were occupied. This signifies that shelters are not being used at full capacity and suggests that women do not have access or have difficulties accessing the necessary services. Some women-only shelters can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, women with cognitive disabilities, women with substance abuse issues, older women (65 years and above), women with children with disabilities, and unaccompanied girls under the age of 18. There are specific shelters supporting survivors of conflict-related sexualised violence. After the full-scale invasion, additional shelters were built for these survivors. Funding for women-only shelters in Ukraine comes from the state, international funding and religious institutions. Women-only shelters are run by women's NGOs with a gender-specific/feminist approach, faith-based organisations, and the state. Women-only shelters are located in

most regions of Ukraine: six are located in Odesa region, five in Lviv region, four in Zhytomyr, Ivano-Frankivsk, Kyiv and Chernivtsi regions, three in Vinnytsia, Volyn, Mykolaiv, Zakarpattia, Poltava, Khmelnytskyi regions and the city of Kyiv, two in Dnipropetrovsk, Kirovohrad, Kharkiv and Cherkasy regions, and one in Zaporizhia, Sumy, Rivne and Kherson regions. There are no women-only shelters in Luhansk, Ternopil and Chernihiv regions. The main reasons why women-only shelters had to decline referrals were no space/capacity to support the survivor, the survivor being ineligible for support, or the shelter not being adequately equipped for the needs of the women/children. The most common forms of violence reported by survivors to women-only shelters in 2024 were domestic violence.

There are other shelters in Ukraine which can provide support to women survivors of violence and, according to the Ministry of Social Policy, there are 100 crisis rooms providing emergency accommodation for up to 10 days. There are different types of shelters run by the state (also on a local level), NGOs, international organisations, and religious organisations. These include shelters predominantly accommodating internally displaced women (and children and older people), survivors of conflict-related sexualised violence, women with substance abuse issues, women survivors of human trafficking, and LGBTQI+ survivors.¹²⁶

The full scale-invasion has had a major impact on all services including shelters accessible to women survivors of violence in terms of infrastructure. As many shelters were either closed or destroyed, some new ones opened, and a number of shelters also re-oriented their work to accommodate internally displaced people and others who were relocated due to the war. Some shelters which are located in occupied territories were destroyed or damaged in the conflict, were too close to active conflict zones, or have been repurposed to accommodate internally displaced people.

3. WOMEN'S CENTRES

Total number of centres	N/A
Entities running women's centres	N/A

There are women's centres in Ukraine although there is no available information on the overall number of centres for women survivors of violence or for all survivors. According to the Ministry for Social Policy, there are 101 day centres providing social and psychological assistance, 110 specialised primary social and psychological counselling services for survivors of domestic violence and/or GBV, 715 mobile socio-psychological assistance teams, and 25 other institutions and organisations providing assistance to survivors (such as, social apartments, centres for social and psychological assistance, centres for emergency anonymous medical and psychological assistance). The

¹²⁶ This information is from the WAVE Country Report 2023.

types of services offered by women's centres include counselling/psychological support, legal advice, financial and social welfare support/advocacy, housing advice, employment, referrals/collaboration with other services, representation at court/police/social services, and residential support services. Women's centres also provide support to survivors of sexualised violence in the form of legal advice as well as needs assessment, specialist psychological care, specialist sexualised violence advocacy services, and community awareness and education for prevention. Some women's centres can be accessed by and are accessible to women with uncertain residence permits/undocumented women, refugee women, and women with physical disabilities. There also are women's centres specialised in supporting survivors of trafficking. However, there are no women's centres providing specialised support to girls under the age of 18.

Before the full-scale invasion, regions in Ukraine received state subventions in order to develop services for survivors of domestic violence and gender-based violence, including centres providing assistance to women. However, the full-scale invasion impacted the situation of services drastically. There are now a number of different centres and services for women, many of which are funded by international organisations. Some centres are run by international organisations, but exact numbers are not available, as well as information on centres run by NGOs. Additionally, the suspension of USAID funding has led to the closure of some services. Funding for women's centres comes from the state, donations, international funding and religious institutions. Women's centres are located in most regions of Ukraine.

4. SPECIALIST SERVICES FOR SURVIVORS OF SEXUALISED VIOLENCE

Ukraine has services for women and girls who have experienced sexualised violence, but there is no available data on the exact number of services. Since the full-scale invasion in Ukraine, more services for women survivors of conflict-related sexualised violence have been established. Some of these have been opened by new actors or those already working in the field who extended or specified their services for women survivors of such violence. Available services include rape crisis centres, sexual violence referral centres and there is a specialised helpline. Support provided by sexualised violence services includes specialist forensic and medical care, needs assessment, specialist psychological care, specialist sexualised violence advocacy services, community awareness and education for prevention, and legal advice. Funding for these services comes from the state, donations, international funding, and religious institutions. The majority of services are for women over the age of 18, some are for minors, and very few are for male survivors of sexualised violence (in most

cases for war-related trauma). Sexualised violence services are located in most regions of Ukraine.

5. PRIMARY PREVENTION SERVICES

The State Social Program for Prevention and Counteraction of Domestic Violence and Gender-Based Violence, in place until 2025, contains a section on the prevention of domestic violence. This includes provisions on overcoming negative stereotypes in Ukrainian society and forming an intolerant attitude toward violent behaviour. There are however no official documents on primary prevention in Ukraine and there is no state definition of primary prevention. The state does fund primary prevention activities, although such funding is limited.¹²⁷ Primary prevention activities are primarily run by NGOs with the support of donor institutions or funding from international organisations.¹²⁸ Such activities include feminist self-defence training, empowerment activities for girls and women, and awareness-raising campaigns or programmes. The Ministry for Social Policy annually publishes information on the implementation of tasks and measures of the State Social Program for Prevention and Counteraction of Domestic Violence and Gender-Based Violence. However, it is impossible to objectively assess the effectiveness and quality of these measures in relation to prevention.

¹²⁷ Information from the WAVE Country Report 2023.

¹²⁸ Ibid.

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Glossary

By-and-For

By-and-for organisations are those that are led by women with intersecting identities and for women with intersecting identities. Their work is grounded in the lived experiences of women who hold multiple, intersecting identities and whose experiences of violence and abuse are shaped by these interlocking factors. The commitment to serving women with intersecting identities is woven through their vision, policy frameworks, organisational culture, and frontline practice. Within this wider definition, there are multiple specialist forms of by-and-for provision, each rooted in the lived experiences and cultural knowledge of particular groups of women, such as Black and minority ethnic women, women with disabilities, and survivors with other protected characteristics.

Crisis/medical services for survivors of sexualised violence

Crisis/medical services for survivors of sexualised violence provide immediate medical care, psychological support, and legal assistance. These services can include forensic examinations, treatment for injuries, STI prevention, pregnancy-related care, and trauma-informed mental health support. They can also offer legal guidance, evidence collection, and referrals for ongoing care, ensuring survivors receive survivor-centred assistance.

Domestic violence

Domestic violence means all acts of physical, sexual, coercive, psychological, or economic violence that occur within the family or domestic unit or between former or current spouses or partners, whether or not the perpetrator shares or has shared the same residence with the victim.¹

Femicide

The term 'femicide' refers to the gender-related killing of women and girls. In broad terms, these can be understood as killings of women and girls because of their gender. These crimes are often perpetrated by men and are ultimately connected to stereotyped gender roles associated with women and historically unequal power relations between women and men, which instil a sense of possessiveness and superiority in men over women.

Gender backlash

Gender backlash can be understood as deliberate, organised attempts to roll back established commitments, rights and achievements in gender equality, women's rights and women's empowerment. It usually takes the form of orchestrated, often virulent political opposition to the rights of women, girls and people with diverse sexual orientations and gender identities and to the actors who

champion them. Those behind the backlash not only rally to oppose feminist framings but also trumpet their own narrow definitions of human rights. Backlash tends to operate in cycles. In many instances, it responds to prior waves of feminist activism and/ or advances. It can lead to policy backsliding that removes formal commitments or dismantles policies to advance gender equality. It can also involve more gradual processes that undermine the implementation of commitments. Such attacks on established rights are not always immediately successful, but they narrow policy debates, making progress more difficult or impossible.²

This backlash increasingly manifests through gender-neutral policy frameworks that have moved beyond fringe far-right politics into mainstream discourse, obscuring the gendered nature of violence and presenting gender equality measures as either unnecessary expenditure or potentially harmful to men and society.

Gender-based violence

Gender based violence against women means violence that is directed against a woman because she is a woman or that affects women disproportionately.³ Gender-based violence is understood to be a form of discrimination and a violation of the fundamental freedoms of the victim and includes violence in close relationships, sexual violence (including rape, sexual assault and harassment), trafficking in human beings, slavery, and different forms of harmful practices, such as forced marriages, female genital mutilation and so-called 'honour crimes'. Women victims of gender-based violence and their children often require special support and protection.⁴

Gender neutrality

Gender neutrality is defined as a set of policies and regulations imposed by the state and governments to provide services to both men and women. Gender neutral policies and practices hide or diminish the root causes of gender inequality and gender-based violence by denying the need for women-only services and by failing to recognise the prevalence of male violence against women. Gender neutrality specifically targets women-only feminist organisations by imposing access to services to both men and women without considering historically unequal power relations between men and women that produce inequality. Gender neutrality threatens to dismantle the ethos of women-only feminist organisations that challenge patriarchy as a root cause of women's inequality and violence against women. Gender neutrality denies women's access to safe women-only spaces, which impacts diverse groups of women in terms of their access to secure

¹ Article 3(b) of the Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention).

² UNRISD (2025) *Understanding backlash against gender equality: Evidence, trends and policy responses*: <https://knowledge.unwomen.org/en/digital-library/publications/2025/09/understanding-backlash-against-gender-equality-evidence-trends-and-policy-responses>

³ Article 3(d) of the Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention).

⁴ Recital 17, Directive 2012/29/EU of the European Parliament and of the Council of 25 October 2012 establishing minimum standards on the rights, support and protection of victims of crime, and replacing Council Framework Decision 2001/220/JHA.

housing, health, employment, education and training and other rights and opportunities critical to women in rebuilding their lives after violence.

Gender-specific approach

Article 18 §3 of the Istanbul Convention recognises violence against women as gender-based violence. As such, all measures to eliminate violence against women must be implemented using a gender-specific approach, meaning a gendered understanding of the violence experienced by women, its specific dynamics and consequences, and should focus on the survivors' empowerment.

Intersectionality

Intersectionality recognises multiple forms of inequality and oppression as overlapping and existing simultaneously. Central to intersectionality is the experience of racism by Black and minority ethnic women. The focus on racism within intersectionality stems from a recognition that racism is a foundational system of oppression that interlocks with other forms of inequality, such as sexism, patriarchy, white supremacy, classism, ableism, transphobia, and others. To deepen the understanding of intersectionality, it is important to frame it appropriately as oppression rooted in structural inequality and how diverse identities can be used by institutions and structures to deny women equality. An intersectional framework ensures that women's diverse experiences are recognised, and their needs are met through more inclusive services and policies. It moves beyond single-issue approaches, addressing the interconnectedness of social inequalities and the specific challenges faced by those with intersecting identities.

National women's helpline

A national women's helpline is a dedicated service for women that serves exclusively or predominantly women survivors of violence. It should operate 24/7, be free of charge, and support survivors of all forms of violence against women. To ensure accessibility, the helpline should offer assistance in multiple languages and be available to women across the entire country, with staff who are well-trained, skilled communicators, and knowledgeable about regional contexts and relevant services. Women's helplines provide crisis counselling, practical support, and referrals to other services, such as women's specialist services.

Prevention

Prevention refers to actions that prevent gender-based violence from occurring by addressing its root causes, namely gender inequality, systemic discrimination and unequal power relations between women and men, as well as people with diverse sexual orientations and gender identities.⁵

Primary prevention

Primary prevention refers to all approaches that aim to prevent violence before it occurs by tackling root causes like gender inequality and stereotypes. Actions may include community organising for gender equality and against VAWG, school-based programmes on preventing child abuse or dating violence, feminist self-defence, and programmes challenging harmful masculine behaviour and attitudes.

Rape crisis centres

Rape crisis centres are specialist services for survivors of rape, sexual assault, and all forms of sexualised violence. They provide immediate, medium-, and long-term support grounded in survivor-centred empowerment, offering counselling, advocacy, information, and accompaniment, including support at police stations, in court, and throughout legal proceedings. Many centres also run helplines that offer specialist counselling, explain survivors' rights, and connect them with other support services. In addition to direct care, rape crisis centres often work with communities and partner agencies to strengthen responses to sexualised violence and advance prevention. The Istanbul Convention recommends at least one centre for every 200,000 people, with equal accessibility in both urban and rural areas.

Sexual violence referral centres

A sexual violence referral centre specialises in immediate medical care, forensic practice, storage of evidence and crisis intervention, and can be placed in hospital settings to respond to survivors of recent sexualised violence. These centres can also carry out medical assistance and refer survivors to other specialist community-based centres.

Specialist services for survivors of sexualised violence

Specialist services for survivors of sexualised violence include rape crisis centres, offering long-term support including counselling and therapy, support groups and help navigating other services, and sexual violence referral centres, specialised in immediate medical care, high-quality forensic practice and crisis intervention. Other specialised services, such as crisis/medical services and helplines specialised in sexualised violence, also form part of this support. Many women's centres also offer a range of support services to survivors of sexualised violence within their holistic approach to ending violence against women. In some countries, this is based on clear political principles and an operational analysis of the continuum of violence, and specific services for sexualised violence survivors are not present.

⁵ UNHCR (2020) *UNHCR Policy on the Prevention of, Risk Mitigation and Response to Gender-based Violence*, pag. 9, <https://www.unhcr.org/media/unhcr-policy-prevention-risk-mitigation-and-response-gender-based-violence-2020-pdf>.

Survivor/victim

This report uses the term “survivor” as a preferred term, to empower women by recognising that the woman has survived the violence and is not defined by it. The term victim is an international legal term, which means a natural person who has suffered harm (including physical, mental or emotional harm or economic loss) that was directly caused by a criminal offence.⁶

Survivor-centred approach

A survivor-centred approach prioritises the rights, needs and wishes of survivors. It ensures that all actions promote their safety, confidentiality, respect and non-discrimination, and that they are empowered to make informed decisions about their lives.

Technology-facilitated violence against women

Technology-facilitated violence against women is any act that is committed, assisted, aggravated or amplified by the use of ICTs (information and communications technology) or other digital tools, that results in or is likely to result in physical, sexual, psychological, social, political or economic harm, or other infringements of rights and freedoms.⁷

Violence against women

Violence against women is understood as a violation of human rights and a form of discrimination against women and shall mean all acts of gender-based violence that result in or are likely to result in, physical, sexual, psychological or economic harm or suffering to women, including threats of such acts, coercion or arbitrary deprivation of liberty, whether occurring in public or private life.⁸

Women's centres

Women's centres are non-residential specialist services that support women survivors of violence and their children (if any). Grounded in feminist, gender-specific, and human-rights-based practice, they provide empowering short- and long-term assistance while prioritising women's safety and autonomy. The term includes women's counselling and crisis centres, regional domestic violence crisis centres, specialist services for Black and minority ethnic, migrant, or refugee women, specialist services for women survivors of trafficking, outreach services, and services offering independent domestic or sexual-violence advisors. Typical support includes information, advice, advocacy, counselling, practical help, and accompaniment to courts, police, or social services, as well as proactive and outreach support, among other services.

Women's specialist services

Women's specialist services are feminist services that support women and their children experiencing gender-based violence. These services include, but are not limited to, women's support centres, women's shelters, helplines, rape crisis or sexual violence referral centres, as well as primary prevention services.

Women's specialist services empower and support women and girls throughout the cycle of violence by putting their needs at the centre of all interventions, applying an intersectional approach, and working together with them, recognising their agency. Women's specialist services are typically run by non-governmental feminist organisations that aim to advance women's and girls' human rights to enjoy a life free from all forms of violence.

Women's specialist services have for decades been agents of social, cultural, and political change, promoting women's equality in the wider society and challenging the patriarchal system, which is the root cause of violence against women and girls. Not only do women's specialist services provide vital services to women and their children, but they also serve as a laboratory for continuous innovation and development of practices and are the first to identify gaps in legislation and policy that affect women and areas for improvement. They are, therefore, vital partners to governments, policymakers, as well as to all other stakeholders working to end violence against women.

Women-only shelters

Women-only shelters are specialised services that provide immediate, safe accommodation for women experiencing violence and their children (if any). Rooted in feminist, gender-specific, and human-rights-based practice, these shelters offer far more than emergency accommodation: they work to empower women, prioritise their safety, and offer long-term support. Services provided often include counselling, legal assistance, help accessing employment, and support in securing stable housing. To be considered a women-only shelter, the service must exclusively support women and their children. The Istanbul Convention recommends that specialised women's shelters should be available in every region with at least one shelter place per 10,000 people, where one family place counts as one bed space in WAVE's methodology.

⁶ Directive 2012/29/EU of the European Parliament and of the Council of 25 October 2012 establishing minimum standards on the rights, support and protection of victims of crime, and replacing Council Framework Decision 2001/220/JHA.

⁷ UN Women and World Health Organization (2023) “Technology-facilitated Violence against Women: Taking stock of evidence and data collection”.

⁸ Article 3(a) of the Council of Europe Convention on preventing and combating violence against women and domestic violence (Istanbul Convention).

List of WAVE Members (as of 15 December 2025)

NAME OF ORGANISATION (English)	NAME OF ORGANISATION (national language)	WEBSITE	COUNTRY
1. Albanian Women Empowerment Network (AWEN)	Rrjeti i Fuqizimit të Grave Shqiptare	awenetwork.org	Albania
2. Counselling Line for Women and Girls	Linja e Këshillimit për Gra dhe Vajza	hotlinealbania.org	Albania
3. Gender Alliance for Development Center (GADC)	Qendra Aleanca Gjimore për Zhvillim	gadc.org.al	Albania
4. Human Rights in Democracy Center (HRDC)	Qendra "Të Drejtat e Njeriut në Demokraci" (QDNJD)	hrdc.al	Albania
5. Women's Association "Refleksione"	—	refleksione.org	Albania
6. Women's Forum Elbasan	Forumi i Gruas Elbasan	forumigruaselbasan.org	Albania
7. Woman to Woman	Gruaja tek Gruaja	gtg.al	Albania
8. Center for Legal Civic Initiatives	Qendra për Nisma Ligjore Qytetare	qag-al.org	Albania
9. Psycho-Social Centre "Vatra"	Qendra Psiko-Sociale "Vatra"	qendravatra.org.al	Albania
10. Women Center "Light Steps"	Qendra-Sociale "Hapa te Lehte"	hapatelehte.org	Albania
11. Center for Equality and Inclusion	Qendra "Barazi dhe Përfshirje"		Armenia
12. Women's Support Center (WSC)	Կանանց աջակցման կենտրոնը (ԿԱԿ)	womensupportcenter.org	Armenia
13. Women's Rights Center (WRC)	Կանանց իրավունքների կենտրոնը (ԿԻԿ)	womensrightshouse.org	Armenia
14. Sexual Assault Crisis Center (SACC)	Սեռական բռնության ճգնաժամային կենտրոն	saccarmenia.org	Armenia
15. Women's Resource Center NGO	Կանանց ռեսուրսային կենտրոն ՀԿ	womenofarmenia.org	Armenia
16. Impact Innovations Institute's Initiatives Foundation (IMINI)	Իմփակտ Ինովացիաների Ինստիտուտի Նախաձեռնություններ Հիմնադրամ	safeyou.space	Armenia
17. Women's Rights House NGO	Կանանց իրավունքների տուն ԿՀ (ԿԻՏ)	womensrightshouse.org	Austria
18. Association of Austrian Autonomous Women's Shelters (AÖF)	Autonome Österreichische Frauenhäuser (AÖF)	aoef.at	Austria
19. Gewaltschutzzentrum Wien	—	gewaltschutzzentrum.at	Austria
20. Network of Austrian Counselling Centres for Women and Girls	Netzwerk österreichischer Frauen- und Mädchenberatungsstelle	netzwerk-frauenberatung.at	Austria
21. Ninlil – Empowerment and Counselling for Women with Disabilities	Empowerment und Beratung für Frauen mit Behinderun	ninlil.at	Austria
22. FEMA – Association of Feminist Single Mothers	Verein Feministische Alleinerzieherinnen	verein-fema.at	Austria
23. Rosa Logar (Honorary Member)			Austria
24. Renate Egger (Individual Member)			Austria
25. Clean World AID to WOMEN Social Union	—	aidsactioneurope.org	Azerbaijan
26. Gender Hub Azerbaijan	Azərbaycan Respublikasının Gənclərin Gender Bərabərliyi Mərkəzi "Gender Hub"	genderhubaz.org	Azerbaijan
27. International Public Association "Gender Perspectives"	Международное общественное объединение «Гендерные перспективы»	—	Belarus
28. Legal Initiative – Commission on Women's Rights	Правая ініцытыва	—	Belarus
29. Collective Against Family Violence and Exclusion (CVFE)	Collectif contre les Violences Familiales et l'Exclusion (CVFE)	cvfe.be	Belgium
30. Garance ASBL		garance.be	Belgium
31. Foundation "United Women" Banja Luka	"Udružene žene" Banja Luka	udruzene-zene.org	Bosnia and Herzegovina
32. "Medica" Zenica Association	Udruženje "Medica" Zenica	medicazenica.org	Bosnia and Herzegovina
33. CURE Foundation	FONDACIJA CURE	fondacijacure.org	Bosnia and Herzegovina
34. Association of citizen Vive Žene	Udruženje građana Vive Žene	vivezene.ba	Bosnia and Herzegovina
35. Association "Novi put"	Udruženje "Novi put"	newroadbih.org	Bosnia and Herzegovina
36. Foundation "Lara"	Fondacija "Lara"	fondacijalara.com	Bosnia and Herzegovina
37. Alliance for Protection against Gender-Based Violence	Алианс за защита от насилие, основано на пола	alliancedv.org	Bulgaria
38. Bulgarian Gender Research Foundation	Български център за джендър изследвания	bgrf.org	Bulgaria
39. DA Foundation – united against violence	Фондация „Джендър алтернативи“	genderalternatives.org	Bulgaria
40. Autonomous Women's House Zagreb	Autonomna Ženska Kuća Zagreb	azkz.hr	Croatia
41. B.a.B.e. Be active. Be emancipated	Budi aktivna. Budi emancipiran.	babe.hr	Croatia
42. Centre for Women War Victims – ROSA	Centar za Žene Žrtve Rata (CŽŽR) – ROSA	czzzr.hr	Croatia
43. Women's Room	Ženska Soba	zenskasoba.hr	Croatia

NAME OF ORGANISATION (English)	NAME OF ORGANISATION (national language)	WEBSITE	COUNTRY
44. domine – Organization for the Promotion of Women's Rights	Udruga domine – Organizacija za promicanje ženskih prava	domine.hr	Croatia
45. SOS Rijeka – Center for Nonviolence and Human Rights	SOS Rijeka – centar za nenasilje i ljudska prava	sos-rijeka.org	Croatia
46. Mediterranean Institute of Gender Studies (MIGS)	—	medinstgenderstudies.org	Cyprus
47. Association for the Prevention and Handling of Violence in the Family (SPAVO)	Συνδέσμου για την Πρόληψη και Αντιμετώπιση της Βίας στην Οικογένεια (ΣΠΑΒΟ)	domviolence.org.cy	Cyprus
48. Association of Women to Support Living (KAYAD)	Kadından Yaşama Destek Derneği (KAYAD)	kayad.org	Northern part of Cyprus
49. ProFem – Center for Victims of Domestic and Sexual Violence	Centrum pro Oběti Domáčního a Sexuálního Násilí	profem.cz	Czech Republic
50. ROSA Centre for Women	ROSA Centrum pro Ženy	stopnasili.cz	Czech Republic
51. L.O.K.K – National Organization of Women's Shelters in Denmark	Landsorganisation af Kvindekrisecentre	lokk.dk	Denmark
52. Danner	—	danner.dk	Denmark
53. Center for Violence Prevention	Center for Voldsforebyggelse (CFV)	centerforvoldsforebyggelse.com	Denmark
54. The Joan-Sisters	Joan-Søstre	joan-soestrene.dk	Denmark
55. Women's Support and Information Centre	Naiste Tugi- ja Teabekeskus	wwsic.ee	Estonia
56. Pärnu Women's Support Centre	Pärnu Naiste Tugikeskus	naistevarjupaik.ee	Estonia
57. International Women's Network in Estonia (IWNE)	—	iwne.ee	Estonia
58. Women's Line Finland	Naisten Linja – Kvinnolinjen	naistenlinja.fi	Finland
59. Federation of Mother and Child Homes and Shelters	Ensi- ja Turvakotien Liitto	ensijaturvakotienliitto.fi	Finland
60. National Federation of Women Solidarity	Fédération Nationale Solidarité Femmes (FNSF)	solidaritefemmes.org	France
61. Women for Women France	—	womenforwomenfrance.org	France
62. Association Protect Children	Protéger l'enfant	protegerlenfant.fr	France
63. Anti-Violence Network of Georgia (AVNG)	ძალადობისგან დაცვის ეროვნული ქსელი	—	Georgia
64. Women Fund Sukhumi (WFS)	ქალთა ფონდი “სოხუმი”	fundsokhumi.ge	Georgia
65. Sakhli Advice Center for Women	ქალთა საკონსულტაციო ცენტრი “სახლი”	sakhli.org	Georgia
66. Democrat Women's Organization of Samtskhe-Javakheti	—	—	Georgia
67. Sapari	საფარი	sapari.ge	Georgia
68. Association Merkuri	ასოციაცია „მერკური“	—	Georgia
69. BIG e.V. – Berlin Initiative against Violence against Women	Berliner Initiative gegen Gewalt an Frauen	big-berlin.info	Germany
70. Association of Women's Shelters	Frauenhauskoordinierung e.V.	frauenhauskoordinierung.de	Germany
71. bff – Federal Association of Rape Crisis Centres and Women's Counselling Centres	Bundesverband Frauenberatungsstellen und Frauennotrufe – Frauen gegen Gewalt e.V.	frauen-gegen-gewalt.de	Germany
72. GESINE Network “Gesundheit.EN”	GESINE Netzwerk Gesundheit.EN	gesine-intervention.de	Germany
73. kofra – Communication Center for Women's Life and Work Situation	Kommunikationszentrum für Frauen zur Arbeits- und Lebenssituation	kofra.de	Germany
74. papatya – Crisis Facility for Young Migrant Women	Kriseneinrichtung für junge Migrantinnen	papatya.org	Germany
75. ZIF – Central Information Center for Autonomous Women's Shelters	Zentrale Informationsstelle Autonomer Frauenhäuser	autonome-frauenhaeuser-zif.de	Germany
76. Prof. Carol Hagemann-White (Honorary Member)	—	—	Germany
77. Karin Heisecke (Individual Member)	—	—	Germany
78. European Anti-Violence Network (EAVN)	Ευρωπαϊκό Δίκτυο κατά της Βίας (Ε.Δ.κ.Β.)	antiviolence-net.eu	Greece
79. Union of Women Associations of Heraklion Prefecture	Σύνδεσμος Μελών Γυναικείων Σωματείων Ηρακλείου	kakopoiisi.gr	Greece
80. NANE – Women's Rights Association	Nők a Nőkért Együtt az Erőszak Ellen	nane.hu	Hungary
81. PATENT Association	PATENT Egyesület	patent.org.hu	Hungary
82. Stigamót – Counselling and Information Centre on Sexual Violence	—	stigamot.is	Iceland
83. Women's Shelter Association	Samtaka um Kvænnaathvarf	kvennaathvarf.is	Iceland
84. Sexual Violence Centre Cork	—	sexualviolence.ie	Ireland
85. Rape Crisis Network Ireland	—	rcni.ie	Ireland
86. Safe Ireland	—	safeireland.ie	Ireland
87. Women's Aid Ireland	—	womensaid.ie	Ireland

	NAME OF ORGANISATION (English)	NAME OF ORGANISATION (national language)	WEBSITE	COUNTRY
88.	D.i.Re – National Women's Network against Violence	Donne in Rete contro la Violenza	direcontrolaviolenza.it	Italy
89.	Differenza Donna – Women and Girls against Violence	—	differenzadonna.org	Italy
90.	BeFree Social Cooperative against trafficking, violence, and discrimination	BeFree Cooperativa Sociale contro tratta, violenza e discriminazioni	befreecooperativa.org	Italy
91.	Marcella Pirrone (Honorary Member)			Italy
92.	Paola Degani (Individual Member)			Italy
93.	Dr. Cristina Gamberi (Individual Member)			Italy
94.	Silvia Menecali (Individual Member)			Italy
95.	Women's Wellness Centre (WWC)	Qendra për Mirëqenien e Gruas (QMG)	qmg-ks.org	Kosovo
96.	Kosovar Gender Studies Center	Qendra Kosovare për Studime Gjinore	kgscenter.net	Kosovo
97.	Kosovo Women's Network	Rrjeti i Grave të Kosovës	womensnetwork.org	Kosovo
98.	Crisis and Counselling Centre "Skalbes"	Križu un Konsultāciju Centrs "Skalbes"	skalbes.lv	Latvia
99.	Women's NGO Cooperation Network of Latvia	Latvijas Sieviešu nevalstisko organizāciju sadarbības tīkls	sadarbibastikls.lv	Latvia
100.	Association "MARTA Centre"	Biedrība "Centrs MARTA"	marta.lv	Latvia
101.	Women's Shelter Liechtenstein	Frauenhaus Liechtenstein	frauenhaus.li	Liechtenstein
102.	Centre for Equality Advancement (CEA)	Lygių galimybių plėtros centras	lgpc.lt	Lithuania
103.	Women's Issues Information Center (WIIC)	Motery informacijos centras	lygus.lt	Lithuania
104.	Women in Distress ASBL	Femmes en Détresse A.S.B.L	fed.lu	Luxembourg
105.	Commission on Gender-Based Violence and Domestic Violence	—	domesticviolence.gov.mt	Malta
106.	Network Forum Malta	—		Malta
107.	Migrant Women Association Malta	—	migrantwomenmalta.org	Malta
108.	Dr. Marceline Naudi (Individual Member)			Malta
109.	Association against Violence "Casa Marioarei"	Asociația Împotriva Violentei în Familie „Casa Marioarei"	antiviolența.md	Moldova
110.	Center for Support and Development of Civic Initiatives "Resonance"	Центр развития и поддержки гражданских инициатив «РЕЗОНАНС»	resonancengo.org	Moldova
111.	Women's Law Centre	Centrul de Drept al Femeilor	cdf.md	Moldova
112.	National Coalition "Life without Violence in the Family"	Coaliția Națională "Viața fără Violență în Familie"	sdvmd.net	Moldova
113.	SOS Hotline for Women and Children Victims of Violence Nikšić	SOS Telefon za Žene i Djecu Žrtve Nasilja Nikšić	sosnk.org	Montenegro
114.	Women's Safe House	Sigurno Ženska Kuća	szk.co.me	Montenegro
115.	Association "Valente"	Vereniging "Valente"	valente.nl	Netherlands
116.	Femmes for Freedom	—	femmesforfreedom.com	Netherlands
117.	National Council for Gender Equality (NCGE)	Национален совет за родова рамноправност (СОЖМ)	sozm.org.mk	North Macedonia
118.	National Network to End Violence against Women and Domestic Violence – Voice against Violence	Националната мрежа против насилство врз жени и семејно насилство	glasprotivnasilstvo.org.mk	North Macedonia
119.	Crisis Center in Salten	Krisesenter i Salten	bodo.kommune.no	Norway
120.	Secretariat for the Crises Centre Movement in Norway	Krisesentersekretariatet	krisesenter.com	Norway
121.	Women's Rights Centre	Centrum Praw Kobiet	cpk.org.pl	Poland
122.	Autonomy Foundation	Fundacja Autonomia	autonomia.org.pl	Poland
123.	Juniper Foundation	Fundacja Juniper		Poland
124.	Feminoteka Foundation	Fundacja Feminoteka	feminoteka.pl	Poland
125.	AMCV – Association of Women against Violence	Associação de Mulheres contra a Violência	amcv.org.pt	Portugal
126.	A.L.E.G Association for Liberty and Equality of Gender	Asociația pentru libertate și egalitate de gen	aleg-romania.eu	Romania
127.	Anais Association	Asociația Anais	asociația-anais.ro	Romania
128.	Necuvinte Association	Asociația Necuvinte	necuvinte.ro	Romania
129.	Filia Center	Centrul Filia	centrulfilia.ro	Romania
130.	Dr. Max Foundation	Fundația Dr. Max	—	Romania
131.	Nizhny Novgorod Women's Crisis Center	Нижегородского Женского Кризисного Центра	woman-nnov.ru	Russia
132.	Interregional Non-Governmental Organization for the Support of Family, Motherhood, and Childhood "Doctors to Children"	"Врачи детям"	vd-spb.ru	Russia
133.	P.O.L.I.N.A – Crisis Center for Women	П.О.Л.И.Н.А – Кризисный центр для женщин	crisiscenter.ru	Russia
134.	We Believe You	Tebe Poveriat «Тебе поверят»	verimtebe.ru	Russia
135.	Autonomous Women's Center (AWC)	Autonomi Zenski Centar Beograd	womenngo.org.rs	Serbia
136.	Association FenomenA	Udruženje Fenomena	fenomena.org	Serbia

NAME OF ORGANISATION (English)	NAME OF ORGANISATION (national language)	WEBSITE	COUNTRY
137. Oasis of Safety	Oaza Sigurnosti Kragujevac	oazasigurnosti.rs	Serbia
138. The Association of Women Sandglass	Pescanik Udruzenje Zena	udruzenjepescanik.org	Serbia
139. Daje - Roma Center for Women and Children	Romski Centar za Žene i Decu	romadaje.org	Serbia
140. Human Rights Committee Vranje	Odbor za Ljudska Prava Vranje	hrcvr.org	Serbia
141. ...IZ KRUGA - VOJVODINA, organization for the support of women with disabilities	IZ KRUGA – VOJVODINA organizacija za podršku ženama s invaliditetom	izkrugavojvodina.org	Serbia
142. Biljana Brankovic (Honorary member)			Serbia
143. Alliance of Women in Slovakia	Aliancia Zien Slovenska	alianciazien.sk	Slovakia
144. Fenestra	—	fenestra.sk	Slovakia
145. Association SOS Helpline for Women and Children – Victims of Violence	Društvo SOS	društvo-sos.si	Slovenia
146. Association for Nonviolent Communication	Društvo za nenasilno komunikacijo	društvo-dnk.si	Slovenia
147. Association Another Time	Asociación Otro Tiempo	otrotiempo.org	Spain
148. Aspacia Foundation	Fundacion para la Convivencia Aspacia	fundacion-aspacia.org	Spain
149. HELIA – Women Association	Associació de suport a les Dones que pateixen Violència de Gènere	heliadones.org	Spain
150. Federation of Women's Associations Arena and Laurisilva	Federacion de Asociaciones de Mujeres Arena y Laurisilva	arenaylaurisilva.org	Spain
151. Association of Feminist Psychotherapy and Psychology	Asociación de Psicología y Psicoterapia Feminista	psicofeminista.com	Spain
152. For You Woman Association	Asociación Por Ti Mujer	asociacionportimujer.org	Spain
153. Unitary platform against gender violence	Plataforma unitària contra les violències de gènere	violenciadegenere.org	Spain
154. Association Of Women Lawyers Themis	Asociación De Mujeres Juristas Themis	mujeresjuristasthemis.org	Spain
155. Roks – National Organisation for Women's and Young Women's Shelters in Sweden	Riksorganisationen för kvinnojourer och tjejjourer i Sverige	roks.se	Sweden
156. Unizon	—	unizon.se	Sweden
157. Uppsala Women's Shelter	Uppsala Kvinnojour	uppsalakvinnojour.se	Sweden
158. Women's Shelter Jamtland county	Kvinnojouren Jämtlands län	kvinnojour.nu	Sweden
159. Hultsfred Vimmerby Women's Shelter	Hultsfred Vimmerby Kvinnojour		Sweden
160. Jenny Westerstrand (Individual Member)			Sweden
161. Women's Solidarity Organization of Switzerland and Liechtenstein	Dachorganisation der Frauenhäuser der Schweiz und Liechtenstein	frauenhaus-schweiz.ch	Switzerland
162. Violence What to do?	Violence que Faire	violencequefaire.ch	Switzerland
163. Brava – Together against Violence Against Women	Brava – Gemeinsam gegen Gewalt an Frauen	brava-ngo.ch	Switzerland
164. Gender Equality Foundation	Fondation pour l'égalité de genre	fondationegalitedegenre.ch	Switzerland
165. Foundation for Women's Solidarity	Kadın Dayanışma Vakfı	kadindayanismavakfi.org.tr	Türkiye
166. Mor Çatı Women's Shelter Foundation	Mor Çatı Kadın Sığınağı Vakfı	morcati.org.tr	Türkiye
167. Mor Salkım Women's Association	Mor Salkım Kadın Dayanışma Derneği	morsalkim.org.tr	Türkiye
168. Kadriye Bakırcı (Individual Member)			Türkiye
169. International Migrant Women's Solidarity Association (MIWOSA)	Uluslararası Göçmen Kadınlar Dayanışma Derneği (UGKDD)	ugkdd.org	Türkiye
170. Civil Society Organization "La Strada-Ukraine"	Ла Страда-Україна	la-strada.org.ua	Ukraine
171. Women's Information Consultative Center (WICC)	Інформаційно-консультативного жіночого центру (ІКЖЦ)	wicc.net.ua	Ukraine
172. Women's Perspectives Center	Центр Жіночі Перспективи	women.lviv.ua	Ukraine
173. Imkaan	—	imkaan.org.uk	UK
174. Latin American Women's Aid (LAWA)	—	lawadv.org.uk	UK
175. London Black Women's Project	—	lbwp.co	UK
176. Refuge	—	refuge.org.uk	UK
177. Scottish Women's Aid	—	womensaid.scot	UK
178. Welsh Women's Aid	—	welshwomensaid.org.uk	UK
179. Women's Aid Federation of England	—	womensaid.org.uk	UK
180. Women's Aid Federation Northern Ireland	—	womensaidni.org	UK
181. Latin American Women's Rights Service (LAWRS)	—	lawrs.org.uk	UK
182. Standing Together against Domestic Abuse	—	standingtogether.org.uk	UK
183. Rape Crisis Scotland	—	rapecrisisscotland.org.uk	UK
184. Women's Support Project	—	womenssupportproject.org.uk	UK
185. Hibiscus (for Social Justice)	—	hibiscusinitiatives.org.uk	UK
186. FILiA	—	filia.org.uk	UK
187. Greta Squire (Individual Member)			UK
188. Shazia Choudhry (Individual Member)			UK
189. Beverley Gilbert (Individual Member)			UK
190. Lily Greenan (Individual Member)			UK



* as of 15 December 2025



The Women Against Violence Europe (WAVE) Network is the largest European network of women's specialist services, with 190 members based in 46 European countries. Since its foundation in 1994, WAVE has been working to prevent and combat violence against women and their children.

WAVE Network and European Information Centre Against Violence

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